MEDIMIND / COMPLETE CHAT WORK ARCHIVE

Everything we built.
One place to find it.

Marketing, positioning, landing-page copy, research, customer journeys and workshop briefs.

142 original files preserved | 125 indexed sections | 739 pages in the complete PDF

Start here

01 ALL SESSION WORK.pdf contains the complete reading archive, including this guide. Open its bookmarks/sidebar to jump to a section, or search the text. The table of contents below uses that PDF's page numbers.

Original files/ keeps the HTML, editable Markdown, workshop PDFs, supplied libraries, technical files and visual QA images. These are byte-for-byte copies. Nothing was moved, deleted, deployed or published for this archive.

Archive inventory/ records original locations, checksums, PDF section/page mappings and the coverage check. Some Desktop files were found in Trash and copied here; the originals remain there.

What to read first

Current working framework: the three October workshop briefs.
Copy choices: the latest five HTML additions and the ten full-screen hero previews. Option 5 preserves your preferred proof-led opening.
Deep strategy: all 20 B11 prompt answers, followed by earlier copy and research versions.

Read historical material as historical

Dates, assumptions, evidence warnings and proposed offers are preserved. Earlier drafts can conflict with later decisions. In particular, the later focus metric is the first hospital hands-on test; older chapters still discuss qualified demos. Archived claims are not newly validated or approved for publication.

Scope: all recoverable saved deliverables identified for this chat, plus the original B11 inputs and supporting source snapshots. This is not a verbatim conversation transcript or an export of unrelated repositories. Duplicate copies are retained in originals; distinct versions remain in the PDF.

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Coverage & preservation notes

All 25 consolidated HTML chapters

The 20 prompt answers and five later additions are included in full. Hidden tabs were expanded so the copy alternatives are not lost in print. Navigation and copy/print controls are omitted from the reading edition.

All ten website-style screens

Each original hero preview has its own landscape page. The copy is also present in the text edition and saved Markdown. A logo rasterization is used for reliable PDF rendering; original HTML remains unchanged.

Earlier drafts, notes and alternate editions

The original favorite, ten earlier alternatives, UVP/JTBD strategy, live-copy snapshot, pain-led draft, evidence register, interview guide, first-week guide, positioning snapshots and prior HTML/Markdown versions are included.

Workshops and source inputs

Blocks 1, 2 and 3 are included as their original PDFs. The two supplied B11 libraries appear in a separately labelled reference section; they are not presented as work authored here.

Technical and visual appendix

Build scripts, JSON catalogs, audit records and saved visual QA images are preserved. Very long embedded image data in code listings is represented by a note, with the complete bytes retained in the original file. Text extractions of the supplied libraries are covered by the full reference PDFs.

Private by default

This folder is local on your Desktop. No new share link was created. It contains internal strategy and source notes; review it before sharing publicly.

Core positioning & customer

A clear offer, a focused buyer and a reason to choose MediMind.

01 USP / value proposition - the one-liner

MediMind replaces disconnected hospital software with one tailored, AI-assisted system - with a free parallel evaluation before you commit to buy.

The offer: we build the parallel system free; the hospital tests its quality and the improvement before deciding. No obligation to buy. For our next **10 hospital clients**, we match their current software price when they switch. Ongoing development is included within agreed scope.

Today: connected workflows and AI assistance. **Next:** the foundation for validated, hospital-wide AI, with clinicians retaining control of medical decisions.

02 Ideal customer profile (ICP) - who we pursue first

Initial focus: Georgian private multi-profile hospitals and hospital groups with disconnected clinical and business software, repeated cross-team work, and a leadership reason to improve it.

Strong fit: a recurring problem we can demonstrate improvement on, an accessible decision sponsor, a potential internal champion, and willingness to allocate staff time to an evaluation. **Not the first priority:** organizations seeking only a standalone AI chatbot, with no relevant pain, or outside a scope we can support.

03 Buyer personas + customer value propositions

Person / role	Pain they recognize	Value we put in front of them
CEO / owner / group operator Economic buyer	Pressure to improve performance without making a disruptive, expensive mistake.	Evaluate a tailored hospital-wide replacement before buying; matched software pricing for the next ten clients.
Operations or clinical lead Internal champion	Staff repeat work and chase patient information across teams.	Connected workflows and AI assistance in daily work; staff test whether the new system makes the work better.
Finance lead / CFO Financial evaluator	Billing gaps, corrections and reports that take too much work to reconcile.	Connect clinical activity with financial records and test a specific billing or reporting improvement.
IT / data owner Technical approver	Migration risk, access concerns and the burden of supporting another system.	A separate, scoped evaluation while the existing system stays in place; agree controls and responsibilities before rollout.

A **connector** opens the door. A **champion** mobilizes hospital colleagues. A **buyer** authorizes the purchase. These may be different people.

Which leads deserve attention?

Working MQL and SQL definitions for a founder-led hospital sale.

04 MQL + SQL - clear gates, not vague interest

MQL

Marketing-qualified lead

A good-fit hospital that has engaged and is ready for discovery.

1. Fits our target hospital profile and supportable scope.
2. A relevant contact is identified and engaged.
3. The contact agrees to discovery or another dated next step.

SQL

Sales-qualified lead

Discovery confirms a credible opportunity to plan an evaluation.

1. A recurring pain and reason to act are confirmed.
2. The buyer/sponsor and approval path are identified.
3. An internal owner will coordinate the evaluation.
4. A credible budget path exists if the test succeeds.
5. Scope is feasible; evaluation planning has a date.

Does not count: a page visit, content download, unaccepted introduction or friendly reply with no meaningful next step. Missing SQL criteria stay in discovery or nurture; they do not become a forecasted sale.

05 Keep qualification separate from evaluation

Target account — Fit + route in → **MQL** — Discovery next → **SQL** — Plan the evaluation → **Evaluation-ready** — Scope + team + approvals → **First real test** — Hospital staff test it

A SQL is **not** a granted-access milestone or a paying customer. Agree evaluation scope, hospital participants, permitted data, success criteria and dates before building. Count one buying organization once, not each contact or each site in a centrally purchasing group.

Current priority metric: new hospital buying organizations outside the anchor partner completing their first hands-on parallel-system evaluation session in the last 30 days. SQLs feed this metric; they are not the metric itself.

06 What we should agree together

1. Confirm the target segment and strongest buyer pain. **2.** Approve these MQL/SQL gates and one follow-up owner. **3.** Finalize the free-evaluation scope, next-ten-client price-match terms and development boundaries.

Basis: the supplied Block 1 workshop slide, MediMind marketing work and founder offer. Buyer pains and these qualification rules are working hypotheses to validate; no clinical or financial uplift is claimed as proven.

Acquisition & sales foundation

A realistic first-month plan for hospitals beyond the anchor partner.

01 Lead generation plan: trust first, volume later

First buyer: a Georgian private hospital/group leader with a recurring cross-team problem and staff willing to test an alternative. Sell the hospital-wide vision; agree a manageable first evaluation.

Priority / channel	Approach + weekly action
1. Warm introductions	Ask clinicians, advisers and real professional contacts for a specific introduction. Confirm the relationship; do not imply an endorsement.
2. Targeted founder outreach	Contact a researched hospital leader by work email or LinkedIn. Ask about one pain using the drafts on page 2.
3. Follow-up + proof	Follow up personally with a relevant synthetic-data workflow example. Skip paid ads, purchased lists and broad campaigns for now.

Budget: 5 team hours/week: research, introductions, outreach, follow-up and review - 1h each. Calls and engineering are additional. Founder owns conversations; a coworker can maintain the list.

02 Initial lead list: build 15 accounts, not 150 names

Prioritize **private multi-profile hospitals and groups with an accessible sponsor**. Rank warm access and a verified reason to talk above size. Several sites may share one buying decision.

Where to start	What to verify before calling it a lead
Founder / clinician network	Name the hospital, connector and intended buyer. A connector is not automatically an internal champion.
Official hospital sites + professional profiles	Confirm services, current leadership, buying group and work contact route. Never infer software problems from a website.

Four research seeds, not qualified leads: New Hospitals; American Hospital Tbilisi; Caucasus Medical Centre; Todua Clinic. Official sites checked 2 October 2026. Their pain, interest, access and purchasing independence are unconfirmed; deduplicate against existing relationships.

One tracker row per buying organization: hospital/group; source + date; fit evidence; buyer; connector; possible champion; pain (hypothesis or confirmed); stage; owner; last touch; next action + date.

30-day experiment - planning targets, not a forecast

Week 1: verify 15 accounts and request 5 introductions. **Weeks 2-3:** send 10 tailored first messages, avoiding duplicate approaches; follow up and hold discovery calls. **Week 4:** aim to scope one evaluation. **Learning targets:** 4 held discovery calls, 2 SQLs, 1 evaluation-ready hospital. Review what blocks progress; a completed test may fall outside this month.

Basis: Block 2 workshop slide, founder offer and existing MediMind marketing work.

Outreach that opens a conversation

Ready-to-adapt drafts. One clear next step; no pressure to switch.

03 Outreach content: three usable drafts

Send personally, in the recipient's preferred language. If you add a hospital-specific opening, use only a fact you have checked. These are drafts for outreach, not messages that have been sent.

A. Warm-introduction request - to a real connector

Do you know a hospital CEO or operations leader whose team is doing too much work between disconnected systems?
MediMind brings that work into one tailored, AI-assisted system. We build a parallel version free for their team to test, with no obligation to buy.
Would you be comfortable introducing us for a 25-minute conversation?

B. Direct email - subject: Test the alternative before you switch

Is your team re-entering information, chasing updates or fixing gaps between hospital systems?

MediMind brings your hospital into one tailored system with AI tools in daily workflows. We build an agreed parallel version free, alongside your current system. Your team tests it before any obligation to buy.

For our next 10 hospital clients, we match their current software price when they switch, under agreed terms.

Could we spend 25 minutes next week choosing one workflow worth testing?

C. Short message - LinkedIn or an existing business chat

When hospital systems do not connect, staff become the connection. Does that sound familiar?
We build a tailored, AI-assisted parallel system free. Test it before any purchase commitment; your current system stays in place.
Open to a 25-minute call to see if this fits?

A simple follow-up rhythm

Day 4: "Which would be more useful to improve first: repeated entry, chasing patient information, or billing corrections? If none is a priority, no problem."
Day 10: "I will close the loop here. If a parallel comparison becomes useful later, reply and we can pick one workflow."
Stop after a decline or opt-out. If they respond, follow the agreed next step rather than this schedule.

Keep the offer accurate: the next-ten-client offer is not a countdown. Record eligibility, matched cost items, duration and development scope in the proposal. Do not promise a launch date before readiness is checked, or proven revenue/clinical gains before they are measured.

Turn a conversation into a real test

A 25-minute discovery call, followed by a clear decision and owner.

04 Discovery call script - diagnose before demonstrating

Time / purpose	Questions to ask, in order
0-2 min Set the frame	"Let's identify one problem worth testing in a free parallel system. No decision to switch today. What would make this call useful?"
2-8 min Find the real pain	"Describe the last difficult handoff between teams. Where did someone re-enter, call, wait or correct something? How often does it happen, and who deals with it?"
8-12 min Find impact + urgency	"What did that cost in staff time, delays or billing corrections? What have you tried? Why act now? What improvement would make a test worthwhile?"
12-17 min Map the buying path	"Who approves software, organizes testing and reviews clinical, finance and IT concerns? When does your contract renew? If the test works, how would budget and approval happen?"
17-22 min Check fit + explain offer	"We build a separate test free while yours stays in place, with no obligation to buy. Which workflow matters most? What cannot change? Who will test it and approve data use?"
22-25 min Agree the next step	"Have I understood the problem, desired improvement and constraints correctly? Shall we book a scoping session with your operations/clinical and IT owners? Who joins, and when?"

Example test: compare one task on comparable cases in both systems. Measure time, repeated entries and errors against a threshold agreed in advance. This does not prove clinical outcomes or revenue gains.

After the call: advance, nurture or disqualify

MQL: fits the target, a relevant contact engages, and discovery/another meaningful next step is dated.
SQL: pain + reason to act, sponsor + approval path, internal owner, credible budget path, feasible scope and a dated planning step are confirmed.
Not ready: agree a specific follow-up trigger. **No fit:** close the lead with the reason; do not force a demo.

The handoff: interest is not permission to build

SQL → Evaluation-ready: confirm scope, hospital testers, permitted data/access, security approvals, success criteria and dates. **Then:** build + internal checks → hospital hands-on test → joint results review → explicit buy / revise / stop decision. A purchase leads to a separately agreed rollout.
Timing: a two-week build is a planning target only after readiness and feasibility are confirmed - not two weeks from the first email.

Measure the real milestone: distinct new hospital buying organizations outside the anchor partner completing their first hands-on parallel-system evaluation session in the last 30 days. Track introductions, held calls, SQLs, readiness, build delivery and stalled days to explain that number.

Funnel, metrics & tracking

From finding the right hospital to a real evaluation and a buying decision.

01 The funnel: each step needs evidence

Find the fit → earn trust → prove the alternative

Acquire: Target + route in → MQL → SQL
Prove: Evaluation-ready → Parallel delivered → First hands-on test
Convert: Results review → Purchase agreement → Approved rollout

Stage	What must be true before recording it	Owner
1. Target + route in	Hospital/group fits our focus. Map buyer, possible connector and contact route; direct outreach is also valid.	Founder / researcher
2. Engaged / MQL	A relevant contact responds meaningfully and agrees to discovery or another dated next step.	Founder
3. Qualified / SQL	Discovery confirms pain, reason to act, sponsor, approval/budget path, internal owner and feasible evaluation scope; next step dated.	Founder
4. Evaluation-ready	Scope, testers, baseline, success criteria and dates agreed. Required data/security approvals complete and approved data access works.	Hospital owner + IT + MediMind
5. Parallel delivered	Agreed scope passes internal checks. Test access and instructions reach the named hospital evaluators; existing system stays in place.	MediMind delivery lead
6. First hands-on test	Hospital staff complete an agreed workflow in the parallel system and feedback is recorded. A founder-only demo does not count.	Hospital champion + founder
7. Results reviewed	Compare against the agreed baseline; record gaps and a dated buy, revise or stop decision. A first test is not a passed evaluation.	Buyer + evaluators
8. Purchase agreed	Authorized purchase agreement is signed, with scope, price and rollout terms. Invoice/payment is tracked separately.	Buyer + founder
9. Approved rollout	Hospital accepts the go-live plan and agreed scope goes live. Training, support and cutover are handled as a separate delivery plan.	Hospital + delivery lead

MQL = marketing-qualified lead. **SQL =** sales-qualified lead. A connector opens the door; a champion organizes colleagues; a buyer approves spending. They need not be the same person.

Branches: no fit → closed with reason; not now → nurture with a review date; test gaps → revise and retest, or stop. Retests never create another first-test count. An introduction alone is not an MQL.

Basis: supplied Block 3 slide, Blocks 1-2 and the founder's parallel-evaluation offer.

A dashboard that tells us what to do

A proposed structure - no live CRM data or performance figures are assumed.

02 The one metric that matters right now

New hospitals completing their first real parallel-system test

Count distinct new **hospital buying organizations outside the anchor partner** whose first hands-on test happened in the last 30 days.
Why this one: it shows a hospital moved beyond interest, our team delivered, and its staff actually tried the alternative. It does **not** prove clinical benefit, a successful full evaluation or a sale.

03 Dashboard layout - review every Monday

FIRST REAL TESTS
Not recorded
Last 30 days - main metric

EVALUATION-READY
Not recorded
First became ready in last 30 days

PARALLEL DELIVERED
Not recorded
First delivery in last 30 days

OVERDUE ACTIONS
Not recorded
Open accounts needing attention now

Below the cards: funnel by current stage → stuck-account list (blocker, owner, next action/date) → outcomes (reviewed, revised, stopped, signed). Show last-updated time. Unknown is not zero.

04 KPI definitions - tie every number to a stage

KPI / stage	Definition and window	Decision it supports
First real tests Stage 6	Distinct eligible organizations with first-test date in the rolling 30-day window.	Are we winning real evaluations?
New SQLs Stage 3	Distinct organizations first meeting every SQL criterion in the last 30 days.	Is outreach finding a credible fit?
Evaluation-ready Stage 4	Distinct organizations clearing all readiness gates in the last 30 days; retain data-access date separately.	Are hospital-side approvals moving?
Parallel delivered Stage 5	Distinct first deliveries in the last 30 days. Also report median calendar days from readiness to delivery; list open-build ages separately.	Are we keeping our side of the offer?
7-day test uptake Stage 5 → 6	Of first deliveries at least 7 days old, % first tested within 7 calendar days. Use a named delivery cohort; show n/N.	Are hospital staff actually trying it?
Overdue actions Any open stage	Open organizations whose next-action date has passed. Separately flag missing dates/owners. Snapshot, not a 30-day total.	What needs attention this week?
Reviewed / signed Stages 7 / 8	Separate counts of first results reviews and signed purchases in the last 30 days. Signed value is not cash collected.	Do evaluations lead to a decision?

Basis: supplied Block 3 slide, Blocks 1-2 and the founder's parallel-evaluation offer.

Track the work. Keep the rules simple.

One shared tracker, milestone dates and a short weekly review.

05 One shared tracker is enough to start

Use one row and stable ID per **buying organization**, not per contact or demo. Centrally purchased sites count together. Keep the anchor partner and its expansion separate.

Keep these fields	What goes in them
Account + route	ID; hospital/group; source (warm intro, direct outreach or inbound); connector; buyer; champion; internal owner; anchor/existing flag.
Current work	Current stage; active/nurture/closed status; pain and fit evidence; blocker; next action + due date; responsible person.
First milestone dates	MQL; SQL; approved data access verified; evaluation-ready; delivery; first hands-on test; first results review; signed agreement; go-live.
Evidence + decision	Link to call note, approval, delivery checklist or test feedback. Record baseline, agreed success criteria, review outcome and reason for loss/delay.

Hospital commitment: verify approved read-only backup access or the agreed data route works. Access alone is **not** readiness or evaluation. Store approval references, never credentials or patient data, in this tracker.

06 Counting rules that prevent misleading numbers

Dates: latest 30 calendar dates, including today, in Asia/Tbilisi; timestamp the report. Count each first milestone once; preserve dates when accounts pause.
Do not mix: current-stage counts are a snapshot; milestone counts measure progress during a period.
Conversion: use deliveries from the same named week, not unrelated monthly totals. Once every delivery has had 7 days, calculate tests within 7 days / all deliveries in that group. Show n/N, %, and cohort dates. Zero denominator = N/A; immature groups = pending.

07 Timing: start the clock when the hospital is ready

Proposed working targets - not a universal launch guarantee

Ready → delivered: aim for 14 calendar days after all readiness gates pass and scope/feasibility are confirmed.
Delivered → first hospital test: aim for the next 7 calendar days. Book the session before delivery.
Combined goal: first real test within 21 days of readiness. Track elapsed time and blockers; never rush approvals to meet the date.

Block 2's first-month targets: 4 held calls, 2 SQLs, 1 ready hospital - not reported results. A first test may fall next month. Establish a baseline before setting a recurring volume target.

Weekly review: 15 minutes, three decisions

Founder: update conversations and next steps. **Delivery lead:** verify readiness, delivery and test evidence. **Tracker owner:** refresh the dashboard.
Ask: Where did progress stop? Who owns the next action and date? What did hospital testing prove - and what still needs work? Record one corrective action for the biggest bottleneck.

Basis: supplied Block 3 slide, Blocks 1-2 and the founder's parallel-evaluation offer.

A Practical synthesis · 23 September 2026

Buyer pain points

What the hospital CEO needs to fix.

Money that is hard to account for. Staff time consumed by repeat work. Decisions made without a clear picture. Care that depends on people connecting the dots.

The central buying problem: improve how the hospital runs without putting care, revenue, or daily operations at risk during the change.

Working priorities-not a measured ranking of Georgian buyers. The six pains below come from our saved buyer research and the complete library's problem analysis. No new buyer interviews were conducted. Different hospitals may rank them differently.

01 / Money & margins

Care delivered. Payment delayed or missed.

Documentation, charges, payment records, and actual costs do not always line up. Finance spends time finding gaps and correcting records.

Why the CEO cares: uncertain collections and avoidable costs leave less room to invest in care.

What would prove it?Actual payment corrections, collection delays, and comparable case costs-not just charges identified.

02 / Staff capacity

Skilled people doing the same work twice.

Teams re-enter information, search across screens, call other departments, and check what someone else already knows.

Why the CEO cares: scarce staff attention goes into keeping work moving instead of caring for patients.

What would prove it?Observed repeat entries, minutes per task, overtime, and total workload before and after.

03 / Leadership control

A hospital picture that arrives too late.

Departments report different numbers. Leadership waits for spreadsheets and reconciliation before it can trust the answer.

Why the CEO cares: problems can grow before leaders see them; sites are hard to compare.

What would prove it?Time to prepare reports, disputed figures, data freshness, and decisions delayed.

04 / Patient care

Doctors chasing the patient's story.

Relevant history, results, and handoff information are scattered or arrive late. People fill the gaps with calls and repeated checking.

Why the CEO cares: care becomes harder to coordinate, with concern about delays and missed information.

What would prove it?Information-retrieval time, review delays, and handoff completeness. Clinical outcomes need separate evaluation.

05 / Ability to improve

Every improvement becomes another wait.

Change requests bring new quotes, uncertain timelines, or suppliers saying another system is responsible.

Why the CEO cares: improvement plans stall while workarounds multiply.

What would prove it?Real change requests, delivery times, accepted changes, backlog behavior, and total costs.

06 / Reporting & accountability

Rebuilding the record when someone asks.

Staff manually reconstruct what happened for management, payment, or external review.

Why the CEO cares: reporting consumes attention and creates uncertainty about whether records are complete and consistent.

What would prove it?A real reporting task, preparation effort, missing records, and traceability checks-not a blanket compliance claim.

Two fears can stop the purchase-even when the pain is real.

"Will changing make things worse?"

Missing history, training, extra parallel work, downtime, and payment disruption can outweigh the expected benefit.

> **Our response:** agree on scope, effort, success criteria, and a controlled switch decision with clinical and IT owners.

> ### “Can one young supplier support all this?”
>
> One system reduces vendor coordination but concentrates dependency. Support, continuity, data access, and exit arrangements matter.
>
> **Our response:** make responsibilities, service expectations, development terms, and data access explicit.

The two questions are illustrative buyer-language hypotheses, not customer quotations. AI readiness is a strategic opportunity to test, not yet a proven leading buying pain.

How the pains should shape the page.

Buyer concern	Message to lead with	Where it belongs
Repeat work + financial blind spots	A better-running hospital, with less wasted work and clearer financial control.	Hero and first problem section.
Scattered patient information	Keep the patient’s information connected as care moves between teams.	Care benefit and workflow demonstration.
Slow, costly changes	A system-and a development team-that keep improving with you.	Differentiation and early-partner offer.
Risk of switching + supplier dependency	See the difference before deciding to switch. Know who owns the work.	Evaluation, support, and trust sections.
Readiness for future AI	Solve today’s problems while building the foundation for broader AI.	Hero’s future promise and clearly labeled roadmap.

Read the full six-pain research and the complete library’s problem stack. Its repeat-work campaign hook is useful for discovery; the homepage below retains the broader hospital-wide promise.

B Practical synthesis · 23 September 2026

Recommended landing-page copy

Lead with the hospital they want to run.

The recommendation: present-day value first, the AI destination second, and a practical way to evaluate the change. Keep the full hospital ambition; use a specific workflow to demonstrate it.

Proposed public copy appears in the panels below. Editorial notes sit outside them. This is a review draft, not a conversion-tested winner or an update to the live website.

01 / Hero · One intelligent hospital operating system

A better-running hospital today.
Your AI future starts here.

Replace disconnected hospital software with one system built around your teams-designed for better-connected care, less wasted work, and clearer control of costs and revenue.

- Patient information and daily work, connected.
- Useful AI, with your people in control.
- A development team that keeps improving your system.

Request a hospital demo / See how switching works

See the system. Discuss your needs. Decide whether a side-by-side evaluation is right for your hospital.

> 02 / Dedicated public pain section
>
> ### Your people should care for patients.
> ### Not hold disconnected software together.
>
> When information stops at each department, the work does not. Your teams fill the gaps-with repeated entries, phone calls, spreadsheets, and billing corrections.
>
> - **Staff time goes into repeat work.**
> - **Payment gaps and avoidable costs become harder to see.**
> - **Care teams have to chase the information they need.**
>
> Adding another tool does not always fix what happens between them.

> 03 / The replacement

One hospital.
One connected way to work.

MediMind brings clinical and business work into one tailored hospital system. Keep patient information connected, make daily work easier to follow, and give leadership a clearer view of how the hospital is performing.

04 / Continuing development

Your hospital keeps evolving.
Your software should too.

Work directly with the team building your system. Shape improvements around your hospital's needs, test changes with your staff, and keep making the system better.

05 / The AI destination

The future is a hospital that can anticipate.
Not just react.

Start with connected information and AI that assists daily work. Our roadmap builds toward a hospital-specific AI system that helps anticipate risks and coordinate clinical and business workflows-with your clinical team retaining control of medical decisions.

06 / Evaluation

See the difference.
Then decide to switch.

Start with an agreed workflow. Compare a tailored MediMind setup beside your current system. Review the results, staff effort, and transition plan with your team before deciding on a wider rollout.

07 / Early-partner offer · subject to final terms

A new standard.
At your current software price.

For eligible early hospital partners, we match current software pricing and include unlimited development requests under agreed delivery terms.

Explore the early-partner offer and shape a system around the hospital you want to run.

Offer note-not public copy: “unlimited development requests” is a proposed definition for review, not a replacement for the founder’s intended unlimited-hours offer. Confirm included delivery, prioritization, exclusions, price-match scope, term, and future AI costs before publishing. Do not publish scarcity unless the actual offer boundary is defined.

08 / Final invitation

Build your hospital’s future.
Start with what needs to work better today.

Tell us where your current software gets in the way. We’ll show you the relevant workflow and discuss what an evaluation would involve.

Request a hospital demo

Why this is the recommended version.

- **It keeps the full promise:** a hospital-wide replacement, ongoing development, and an AI future.
- **It names recognizable value:** connected care, less repeated work, clearer financial control-not architecture alone.
- **It answers the switching objection:** evaluate a real workflow before a wider rollout.
- **It separates today from tomorrow:** proactive clinical coordination is a roadmap, not a claimed current autonomous hospital.

Before launch: add only verified product/customer proof; confirm the offer and actual delivery targets; verify the demo form receives requests. The complete library identified a source-level submission concern, not a confirmed deployed failure. No form or website changes are made here.

The three-business-day change and two-week parallel-test promises are held out of this core draft until scope, readiness, and delivery conditions are agreed. No guaranteed revenue uplift, patient-outcome improvement, or “live in 25 hospitals” badge is invented. Open the editable copy and rationale →

C Latest copy exploration · 23 September 2026

Five stronger ways to make the value clear.

One hospital-wide offer. Five different reasons to care. These are complete opening-screen options-not five headlines attached to the same paragraph.

My first choice: Option 1. It makes the hospital-wide problem recognizable, explains the replacement, and gives the CEO a concrete reason to look closer. Option 5 is the strongest alternative when fear of switching dominates. This is editorial judgment, not a conversion-test result.

Drafts for your review. No live website changes. The previous copy is preserved. Navy panels contain proposed visitor-facing copy; the notes beneath them explain the strategy. “Copy this option” copies only that option’s public wording.

01 · The whole-hospital promise

Recommended starting point
MediMind · One intelligent hospital system

Run your hospital.
Not a patchwork of software.

Replace disconnected programs with one system built around your hospital-so patient information, daily work, and financial records work together.

- **Less repeated work.** Less retyping, searching, and chasing other departments.
- **Clearer financial control.** Make payment gaps and avoidable costs easier to find.
- **Built to keep improving.** Your needs shape development and the next steps toward hospital-wide AI.

See it beside your current system. Decide before you switch.

Show me a better way to run my hospital →

Copy this option Add the early-partner offer ↓

Why it works: “patchwork” names the problem without listing modules. The body explains what replaces it. The benefits connect directly to staff effort, money, and the ability to improve. It sells a hospital system-not just a workflow consultation.

02 · The hidden operating cost

For a financially focused CEO

MediMind · Connected care and business operations

Your software bill is only *part of what it costs you.*

The rest can hide in repeated work, billing corrections, and hours spent making reports agree. MediMind brings clinical and business work into one tailored hospital system.

- **Connect care to financial records.** See where documentation and payment need attention.
- **Give staff a shared way to work.** Reduce the need to move the same information between programs.
- **See the hospital more clearly.** Bring the information behind management decisions together.

Compare the work, the cost, and the change involved-before committing to a switch.

Show me what could work better →

Copy this option Add the early-partner offer ↓

Why it works: moves the conversation beyond the subscription price to the cost of running the hospital. Use it where the buyer recognizes these issues. It does not claim that every loss comes from software or promise a recovery amount.

03 · A system that moves with you

For leaders tired of waiting on vendors
MediMind · Hospital software built around you

Your next improvement *shouldn’t need another vendor.*

Replace disconnected hospital software with one tailored system-and work directly with the team that keeps building it around your needs.

- **One connected foundation.** Patient information, clinical work, and business operations together.
- **Your priorities shape the product.** Test improvements with the people who will use them.
- **A path to your AI future.** Add useful AI as capabilities are developed and validated.

Not just new software. A team helping your hospital keep moving forward.

Let’s talk about what your hospital needs →

Copy this option Add the early-partner offer ↓

Why it works: makes continuing development a central reason to choose MediMind. It is particularly relevant after a stalled change request or an expensive integration proposal. The headline is an aspiration, not a promise that no external supplier will ever be needed.

04 · The future, with a first step

For an AI-focused hospital leader
MediMind · The foundation for your hospital's AI future

Build a hospital that can anticipate.
Start with what slows it down today.

Replace disconnected software with one hospital system built around your teams. Connect patient information, simplify daily work, and bring useful AI into the same environment.

- **Connect the hospital first.** Bring clinical and business work onto a shared foundation.
- **Improve the work today.** Help staff find information, avoid repeat entry, and see what needs attention.
- **Build toward proactive AI.** Our roadmap is hospital-specific AI that helps anticipate risks and coordinate care and operations-with clinicians in control of medical decisions.

Start with a real workflow. Build toward a more capable hospital.

Show me the path for my hospital →

Copy this option Add the early-partner offer ↓

Why it works: keeps your future-facing ambition while explaining what the buyer gets first. Choose it when AI is a real leadership priority. It does not present autonomous medical decisions as available today or imply replacement is the only way to use any AI.

05 · The proof-before-switching offer

For an interested but cautious buyer
MediMind · A tailored replacement you can evaluate

Don't switch on a promise.
See the difference first.

Let your team compare MediMind beside your current system: connected patient information, clinical and business work in one place, and software tailored to your hospital.

- **Your workflow.** Start where your current software gets in the way.
- **Your team's judgment.** Compare the information, effort, and results together.
- **Your decision.** Review the full costs and transition plan before a wider rollout.

A hospital-wide system. A practical first step. A decision based on what you see.

Discuss a side-by-side evaluation →

Copy this option Add the early-partner offer ↓

> **Why it works:** answers the fear that can block all the other benefits. Best for warmer prospects who already understand the product. A scoped parallel evaluation still takes planning and staff effort; this is not a no-risk or instant-migration promise.

The offer beneath your chosen hero

Give this its own short strip. The headline earns attention; the commercial offer makes a closer look more attractive.

Early hospital partners

A new hospital system.
At your current software price.

For eligible early partners, we match current software pricing and include ongoing development under agreed terms.

Explore the early-partner offer →

Copy this option

Your stronger development promise, once its terms are defined:
“Unlimited future development included.”
This preserves your intended offer, rather than silently turning unlimited delivery into unlimited requests. Before using it publicly, define included work, prioritization, delivery capacity, exclusions, and future AI costs. Do not imply unlimited instantaneous engineering capacity.

What I would put on the homepage first

1. **Option 1 as the opening screen:** replacement, staff effort, financial control, and continued improvement.
2. **The early-partner strip underneath:** matched pricing and development, with approved terms.
3. **Show the actual product and verified proof:** one workflow that makes the connected system tangible.
4. **Use Option 5’s promise for the evaluation section:** “Don’t switch on a promise. See the difference first.”
5. **Explain the AI destination lower down:** a clear today-versus-roadmap progression, not a competing headline.

Why not force every promise into the headline? A CEO should understand the first screen on one reading. Care, money, staff effort, development, price, evaluation, and proactive AI can all belong on the page without fighting for the same sentence.

Publication checks: verify available AI workflows and deployment scope; agree on price-match and development terms; substantiate any clinical or financial results; confirm the demo route receives requests. Three-day changes and a two-week evaluation can be added after their prerequisites and scope are settled. No artificial deadline, invented customer result, or guaranteed hospital failure is used here.

Inputs: the existing 20-prompt analysis, six buyer pain priorities, and your requested whole-hospital, development, price-match, and AI-future offer. These options are original editorial drafts, not validated buyer quotations or measured conversion winners. Earlier versions remain in this document.

D Metrics workshop · 24 September 2026

From the right hospital to a lasting customer.

Find the right hospital. Earn access to the right people. Prove the product in their world. Win the decision. Make the change work.

Prepared for Nicholas Ho's "Unit Economics & Metrics Tracking" workshop: design your flowchart and select OMTM. This is a proposed operating model, not a report of completed sales, granted access or measured results.

The priority metricWho qualifies?People & introductionsFull flowchartStage rulesMeasurementWorkshop answer

The current bottleneck, in your words: "Winning new hospital evaluations beyond the anchor partner."
This means the journey starts *before* a demo or database request: with account selection, relationships and a credible reason to change. The earlier chapters remain intact; for this stage, qualified demos are a supporting measure-not the main success metric.
One Metric That Matters · current recommendation

New hospitals that actually start evaluating MediMind.

Exact measure: the number of new hospital buying organizations outside the anchor partner that complete their first hands-on parallel-system evaluation session in the last 30 days.

Count the hospital's first real test-not an introduction, a booked demo, a promise to send data, or a system that is ready but unused.

Milestone 1 · their commitment
Usable backup access granted

The hospital authorizes access; the agreed data can actually be read and used for the evaluation.

Milestone 2 · our delivery
Parallel product ready

MediMind delivers and checks the agreed evaluation workflow in a separate environment.

Milestone 3 · counts toward OMTM
The hospital tests it

Its assigned team completes the first agreed workflow test and records feedback.

Why the third milestone wins: database access proves willingness; a ready system proves our delivery; an actual evaluation proves both have turned into customer engagement. Keep the first two visible so we know whether the blockage is hospital approval, our delivery, or hospital participation.

1. Find hospitals worth pursuing.

A hospital can be a promising target without yet being a qualified opportunity. Use two gates, not one.

Build the first list from concrete sources: hospital and hospital-group websites, your existing market knowledge, professional relationships and permission-based referrals from the anchor partner. For each account, capture the ownership group, likely buyer, evidence of a relevant problem or trigger, possible connector and next research step. Verify the buying structure in conversation; a hospital directory entry is not a qualified lead.

> Gate A · researched fit
>
> ### Should we pursue this organization?
>
> - Matches the initial market focus: a Georgian private hospital or hospital group with meaningful cross-department workflows.
> - Has a plausible problem MediMind can address and a scope we can realistically support.
> - Has an identifiable leadership team and a possible route to a conversation.
> - Has a relevant trigger worth investigating: expansion, a software renewal, reporting pressure, repeated work, or a change in leadership.
>
> Output: a prioritized account, a reason to approach it, and an outreach owner. Public clues are hypotheses-not evidence of budget, dissatisfaction or purchase intent.

> Gate B · confirmed in discovery
>
> ### Should we invest in an evaluation?
>
> - The hospital confirms a specific recurring problem, affected team and reason to act.
> - A decision sponsor and internal champion can mobilize clinical/operations and IT counterparts.
> - There is a credible budget and approval path if the evaluation succeeds.
> - Data access and the proposed scope look feasible, with unresolved issues made explicit.
> - The hospital agrees to contribute people, time and a dated next step.
>
> Output: a qualified opportunity. A friendly introduction, prestigious name or enthusiastic demo alone does not pass this gate.

Prioritize fit + urgency + access + ability to deliver. Start with a small named-account list that the founder can actively manage. Do not assign invented numerical scores to unknowns. Mark each fact as confirmed, inferred or unknown. A group with one central purchasing decision is one account, even if it operates many hospitals.

2. Find the route in-and the people who move it forward.

A connector opens the door. A champion keeps the work moving. A buyer can authorize the purchase. One person may fill several roles, but never assume they do.

Connector / mediator

A trusted person who can introduce us: an anchor-partner contact, founder relationship, adviser, clinician, professional association or relevant industry contact. Verify the relationship and ask for a permission-based introduction. They are not automatically authorized to approve a purchase or data access.

Internal champion

Someone inside the hospital who wants this problem solved and actively helps: brings the right colleagues, gets the evaluation scheduled, resolves internal obstacles and presents the findings. “Likes the idea” is not enough.

Economic buyer / decision sponsor

The CEO, owner, group executive or budget holder who can sponsor the change and navigate approval. Confirm who recommends, who signs, and whether a board or procurement team also needs to agree.

Clinical / operations evaluator

The people who actually perform the chosen workflow. They define what a good result looks like, test the product and tell us whether it fits real work-not just a presentation.

IT, data and security approver

The hospital’s authorized counterpart for access, environment separation, technical feasibility and data handling. A champion’s enthusiasm does not replace this approval.

MediMind accountable owners

The founder/account owner manages the opportunity and next steps. The delivery owner manages the parallel system. The rollout/support owner manages adoption. These are responsibilities to assign, not an assumption that three separate hires exist.

Find the connector before the introduction; build the champion through discovery. A known insider can be both, but a warm connection is not mandatory. Direct, relevant outreach, an inbound enquiry or a professional event can reach the same discovery stage. Do not leave a good-fit account waiting indefinitely for the perfect introduction.

Suggested introduction request

“Do you know who owns hospital operations and software decisions at [hospital]? We help hospitals evaluate a tailored parallel system using authorized read-only access to a backup, without replacing their current system during the evaluation. If [specific problem] is relevant, would you be comfortable introducing us for a short fit conversation?”

Personalize the problem; do not ask a connector for credentials or patient data. Any referral fee or conflict of interest would need separate review-not an informal shortcut.

3. The full journey, with decisions and return paths.

This is the business flowchart for the assignment: **how a hospital moves from prospect to customer**, where it can stall, and what we do next. It is not a diagram of the software or a patient-care pathway.

Phase A · find and reach

Earn a relevant conversation.

> **01 · Identify and prioritize suitable hospital accounts**
>
> Research fit, a plausible problem and a trigger. Record one account owner and the evidence behind the hypothesis.

Fit gate: a credible fit moves forward. Clearly unsuitable → close with a reason. Relevant but not ready → nurture with a specific revisit trigger.

↓

> **02 · Map the buying people and the best route in**
>
> Identify the likely sponsor, possible internal champion and IT/clinical counterparts. Check who in our network genuinely knows them.

↓

> **Warm route available**
>
> Ask the connector for a relevant, permission-based introduction. Track whether the introduction is actually made-not only requested.

> **No warm route available**
>
> Use tailored direct outreach, a relevant event or an inbound route. Lead with the hospital's likely problem and a small first conversation.

↓ both routes converge

> **03 · Hold the first decision-maker conversation**
>
> Confirm relevance and identify missing stakeholders. A booked meeting becomes progress only when the conversation happens.

No reply / no-show: follow up within an agreed, bounded outreach sequence; try another legitimate route if appropriate. Then nurture or close-do not count silence as an active opportunity.

↓
Phase B · qualify and commit

Turn interest into an owned evaluation.

> **04 · Confirm the problem and the buying path**

> Ask what repeatedly goes wrong, who feels it, what it costs in work or missed opportunities, and why they would change now. Identify approval steps, budget ownership and timing.

Opportunity gate: a real problem, an accountable sponsor and a plausible route to a purchase move forward. No urgency → nurture. No product fit or feasible approval path → stop or explicitly re-scope.

↓

> **05 · Demonstrate relevance and activate a champion**
>
> Show the workflow that matters to this hospital. The champion brings the evaluators and IT counterpart; the sponsor agrees what decision a successful evaluation would unlock.

↓

> **06 · Agree a short evaluation plan**
>
> Write down scope, participants, permitted data, baseline, success criteria, delivery date, first test session, decision-review date and possible commercial terms. Reserve a delivery slot.

Readiness gate: without an owner, test time, data-approval path and decision date, this remains interest-not a committed evaluation. Start procurement and security discussions here, not after the trial is over.

↓
Phase C · earn trust through delivery

Access → delivery → real testing.

> **07 · Hospital authorizes usable read-only backup access**
>
> Its data owner approves the agreed use. MediMind verifies connectivity, permissions, backup freshness and enough relevant data for the chosen scope.
>
> M1 · hospital commitment

Data gate: if authorization or data is insufficient, resolve the specific issue or pause. A synthetic-data demonstration can continue the conversation, but does not count as this parallel-system evaluation. Never bypass an approval to keep the funnel moving.

↓

> **08 · MediMind delivers the agreed parallel product**
>
> Prepare a separate evaluation environment; check the data mapping and agreed workflow; confirm the hospital’s testers can log in. Keep production use and the current system separate.
>
> M2 · our delivery

↓

09 · The hospital completes its first hands-on evaluation session

The assigned hospital team tests the agreed workflow with the agreed data, records its feedback and confirms the next review. An engineer testing alone or a CEO watching a sales demo does not count.

M3 · OMTM counting event · once per new buying organization

↓

10 · Review the evidence and make a decision

Compare the agreed baseline and success criteria. The sponsor and evaluators decide whether to move forward, resolve a defined gap, or stop.

Meets the agreed criteria

The sponsor accepts the evaluation findings and advances the purchase decision. A positive evaluation is not yet a signed sale.

Continue to commercial approval ↓

A fixable, bounded gap

Agree the change, owner and one next review date. Re-test against the same criteria; avoid an indefinite free development cycle.

Return to delivery / re-test ↰

No fit, no decision or no value

Record the reason, close or explicitly nurture, and end access / retain or delete evaluation data according to the agreed terms.

Use the learning to improve targeting ↰

Ready but nobody tests? The champion owns scheduling; the sponsor resolves priority. Rebook with a named participant and date, or pause. Do not count delivery as customer evaluation. Re-tests never add a second OMTM event for the same organization.

↓ proceed branch
Phase D · convert, adopt and retain

Turn demonstrated value into a lasting relationship.

11 · Approve the purchase and sign the agreement

Confirm scope, price, responsibilities, support and development terms, rollout plan and payment schedule. Complete procurement and contractual approvals.

↓

12 · Roll out in controlled stages

> Plan migration, training, clinical/operational checks, acceptance and a fallback path. Obtain the hospital's explicit go-live approval for each agreed scope.

↓

> **13 · Establish sustained use and review real value**
>
> Check actual workflow use, unresolved issues, agreed operational measures and payments when due. Investigate adoption gaps instead of treating installation as success.

↓

> **14 · Renew, expand and earn introductions**
>
> Review results with the sponsor. Renew on the agreed cycle; expand to additional sites or workflows when justified; ask satisfied customers for permission to introduce us to another suitable organization.

Renewal risk → diagnose and address adoption/value problems at stage 13. **Expansion** → re-scope and evaluate as needed, tracked separately. **Referral** → return to stage 01 for a genuinely new buying organization. Payment follows the contract schedule and may start before rollout; the drawing is not a rule to defer payment.

4. Give every stage an exit condition, owner and next action.

These are recommended responsibilities and tracking rules. No current conversion rates, deadlines or counts have been established from a verified pipeline.

▼ **Stage-by-stage operating checklist**

Stage / accountable lead	Evidence required to move forward	Supporting measurement / stalled action
01 · Account selection Founder / account owner	One buying-organization record; fit hypothesis, source, trigger and next action. Distinguish network membership from an independent purchasing decision.	New fit-confirmed targets; accounts with no owner or next step. Remove poor fits; investigate important unknowns.
02 · Relationship mapping Account owner	Named target roles and best route; connector relationship verified where relevant. Warm and direct routes are alternatives.	Accounts with a credible route; introduction requests → introductions actually made. Switch routes when the warm path stalls.
03 · First conversation Account owner	A relevant conversation held; the correct decision participants identified; next step recorded.	Outreach / introduction → held conversation; no-show rate and days waiting. Follow up, rebook, nurture or close.

Stage / accountable lead	Evidence required to move forward	Supporting measurement / stalled action
04 · Discovery qualification Account owner + sponsor	Hospital-confirmed pain and priority; buying process, budget path, timing and plausible delivery fit.	Held discovery → qualified opportunity; reasons for rejection. Do not keep unqualified interest in the active forecast.
05 · Champion and relevance Account owner + champion	Champion performs an observable action; evaluators and IT counterpart join the plan; sponsor agrees the decision being tested.	Qualified opportunities with an active champion and buyer access. Ask for a concrete internal action, not another generic demo.
06 · Evaluation commitment Account owner + sponsor	Evaluation plan accepted by both sides; team, scope, data requirements, dates, success criteria and delivery capacity recorded.	Qualified opportunity → committed evaluation; age while waiting for approvals. Resolve the missing commitment or keep the opportunity out of build.
07 · Usable data access · M1 Hospital data owner + delivery owner	Authorization and permitted use recorded; read-only backup connection or agreed read-only backup provision verified; sufficient relevant data available.	Committed evaluation → usable access; days from commitment to access. Track approval and data-quality blockers separately.
08 · Parallel ready · M2 MediMind delivery owner	Agreed scope passes the readiness checklist; data mapping reviewed; separate environment usable; hospital credentials issued securely.	Usable access → ready; elapsed delivery days; ready by agreed date / all deliveries due. If late, disclose cause, owner and recovery date.
09 · First hospital test · M3 Champion + delivery owner	Hospital users complete the agreed first workflow test; dated session note, feedback and next review recorded.	**OMTM: new qualifying organizations in the last 30 days.** Also track ready → first session and waiting time; escalate scheduling gaps.
10 · Evaluation decision Sponsor + evaluator	Criteria reviewed; proceed, bounded re-test, stop or dated deferral recorded. Positive findings require the hospital's acceptance.	Started evaluations → completed decision reviews; accepted criteria and re-test cycles. Close open-ended evaluations or explicitly extend with a reason.
11 · Commercial decision Buyer + account owner	Authorized agreement and payment terms-not a verbal "yes." Any conditions to purchase are recorded.	Evaluation cohort → signed customers; time to signature; first payment separately. Escalate approval blockers or record loss reason.
12 · Production acceptance Rollout owner + hospital approvers	Agreed rollout checks, training, fallback and acceptance completed; hospital signs off the live scope.	Signed → accepted go-live; time to live use; critical issues. Hold or reverse the affected rollout step if acceptance fails.
13 · Sustained use and value Support owner + champion	Agreed workflows used by the intended team; operational scorecard reviewed; issues and due invoices tracked.	Workflow use, time to first accepted value, support burden, payments due/received. Fix adoption problems before selling expansion.

Stage / accountable lead	Evidence required to move forward	Supporting measurement / stalled action
14 · Renewal / expansion / referral Account owner + sponsor	Renewal decision, separately approved expansion scope, or consented introduction to a new account.	Renewals / contracts due; expansion separately; referred new accounts. Learn from churn; return suitable referrals to stage 01.

The evaluation plan: the small document that prevents a vague “pilot.”

- **One decision:** “If the agreed evaluation succeeds, what will the hospital be able to approve next?”
- **One bounded starting scope:** the workflow, participants and data needed to test the claim. This is not a promise to rebuild an entire hospital before a purchase decision.
- **A baseline and scorecard:** for example, time to complete a repeat task, repeated entry steps, or time to reconstruct a case or report. Agree the method and a meaningful threshold with the hospital before testing; do not invent results.
- **Owners and dates:** data approval, delivery, first hospital test and decision review; include the hospital’s time commitment.
- **Data and exit rules:** permitted use, access controls, retention/deletion and what happens when testing stops.
- **Commercial boundaries:** confirm evaluation costs, purchase assumptions, any price-match scope and ongoing development terms in writing. Interest in an offer is not acceptance of undefined terms.

Your “parallel product in two weeks” promise needs a precise clock.

Before making it contractual, agree what “ready” includes, whether the period means calendar or business days, and the prerequisites. Track both *evaluation commitment → ready* and *verified usable access + agreed scope → ready*. Otherwise a fast build can hide weeks of access delays.

Read-only is not the same as risk-free. It prevents writes through that access, but copied patient information still needs authorized handling and protection. A backup may be stale or incomplete; the agreed evaluation must account for that. Parallel evaluation is not production replacement or permission for live clinical decisions.

5. Make the priority metric impossible to misunderstand.

Rule	MediMind definition
Name	**New qualified parallel evaluations started - rolling 30 days.** “Started” is counted only after the hospital completes its first substantive hands-on session.
Unit and count	Count distinct new **buying organizations**, not contacts, beds, users, sessions or sites. A group with one purchasing decision counts once. Count an organization only once, at its first qualifying session.

Rule	MediMind definition
Qualifying event	An agreed workflow and success criteria; assigned sponsor, clinical/operations evaluator and IT counterpart; authorized usable data; a ready parallel environment; hospital users complete the first agreed test and record feedback plus the next review.
Window	At each weekly review, count first-session events in the immediately preceding 30 days. Record timestamps consistently in Asia/Tbilisi and use the same weekly cutoff. Do not add overlapping rolling-window totals together.
Exclude	The existing anchor organization, current-customer expansions, repeat sessions, revived evaluations already counted, scheduled but missed sessions, synthetic-data demos, data access alone and MediMind-only tests.
Evidence / owner	A unique account ID, date, attending hospital roles, tested scope, feedback and next review in the opportunity record. The account owner maintains the record; the delivery owner confirms the technical milestone. No patient records or credentials belong in this marketing document.
Baseline and target	**Not yet established from a verified pipeline.** First reconcile actual accounts and events, then set a 30-day target against qualified opportunities, likely access dates and real delivery capacity. Unknown is not zero. No invented benchmark is needed to select the metric.
When to change it	If acquisition reliably feeds real evaluations but they do not convert, move focus to evaluated organizations becoming paying customers. If paying adoption or retention becomes the constraint, move focus there. Change the priority deliberately; keep the funnel history.

This is the one focus metric, not the only number we track. It fits the stated bottleneck better than introductions or access grants, while converting sooner than a long sales cycle's final revenue. It is not a claim that the product has improved clinical outcomes or hospital profit.

The weekly review: find the queue, not just the total.

What to review	What it tells us	Decision
OMTM: new qualifying evaluation starts in 30 days	Are new organizations getting into real evaluation?	Keep this as the headline. Baseline and current value remain unverified until the pipeline is reconciled.
Fit accounts → held conversations → qualified opportunities	Whether the issue is targeting, access to people or relevance.	Improve the account list, introduction route or discovery-not the product indiscriminately.
Committed evaluations waiting for usable access	Hospital approval, missing ownership or data feasibility.	Assign the precise approval/data blocker and a dated resolution step.

What to review	What it tells us	Decision
Usable access waiting for a ready system	MediMind delivery capacity or scope problems.	Fix delivery before promising more starts than we can support.
Ready systems waiting for hospital testing	Weak internal ownership, scheduling or priority.	Activate the champion and sponsor; rebook or pause.
Evaluation cohorts → decisions → signed and paying customers	Whether trials become business, rather than an expanding free-pilot backlog.	Resolve value, scope, buyer or commercial objections; inspect lost reasons.
Live adoption, unresolved serious issues, delivery/support effort	Whether growth is creating durable use at a supportable cost.	Protect customer delivery and safety; address issues before expansion.

Always look at counts and waiting time together. For each stage, show entered, advanced, still open, paused and closed. Keep stalled accounts visible; do not improve conversion artificially by deleting them. Use the median and oldest waiting time when there are enough cases; with very few hospitals, review each account directly.

Use matching groups for conversion rates. For example: among the organizations granted usable access in a particular period, how many subsequently reached their first test? Display the numerator, denominator, observation cutoff and still-waiting cases. Do not divide this week's tests by this week's access grants when they belong to different hospitals. Compare equivalent time windows and treat small samples cautiously.

▼ **Minimum opportunity record-without building a new CRM**

Use a simple account sheet or the existing customer relationship tracker. One row per buying organization, with milestone history rather than repeatedly overwriting dates.

- Account ID, group/site relationship, new versus anchor/existing customer, accountable owner.
- Fit evidence, confirmed pain, urgency trigger, budget/approval path.
- Source/route, connector where applicable, sponsor, champion and evaluator/IT roles.
- Current stage; date entered; last meaningful interaction; blocker; next action, owner and due date.
- Dates for held discovery, evaluation agreement, usable access, parallel readiness, first qualifying session and decision review.
- Evaluation criteria and findings; proceed/re-test/stop decision; signed date; payment and go-live milestones.
- Renewal date, expansion separately, lost/paused reason and permitted referral source.

Store sensitive contact details only in an appropriate access-controlled business system. Keep patient data, access tokens, connection strings and database credentials out of the marketing tracker.

Keep the unit-economics lesson connected.

The lecture also asks whether each sale is economically sustainable. Track founder/sales time and evaluation delivery cost-including evaluations that do not convert-plus rollout, infrastructure, AI usage and ongoing support effort. Report the cost per evaluation separately from customer acquisition cost. A signed contract, an issued invoice and collected cash are different events.

For an early, low-volume hospital business, show actual costs and cash alongside clearly labelled assumptions. Do not claim a reliable acquisition cost or payback period before there are enough paying customers and consistent cost records. A price-match or unlimited-development offer should not hide the work required to serve each organization.

6. How this matches the assignment.

Lecture reference	What this answer supplies
Slide 24 · “Power of Flow Chart in Problem Solving”	A full acquisition-to-renewal flow with decision gates, follow-up, re-test and referral loops, plus owners and supporting measurements. This adapts the lecture’s example to an enterprise hospital sale.
Slide 25 · “One Metrics that Matters”	One selected focus metric, one unambiguous counting event, and a rationale tied to the founder’s stated bottleneck-not a list of equally important metrics.
Slide 26 · “Workshop: Design Your Flowchart & Select OMTM”	The flowchart above and the short submission below directly answer both tasks.
Supporting lecture themes · context, time lag and unit economics	Stage waiting times, matched conversion groups, downstream sales/adoption checks and delivery-cost visibility. These strengthen the answer; they are not presented as extra mandatory assignment deliverables.

Slide references use the deck’s reading order, not Google’s internal slide IDs. The lecturer did not prescribe MediMind’s metric, targets or hospital qualification criteria; those are recommendations based on your business and stated bottleneck.

7. Your assignment-ready answer.

Our current constraint: MediMind needs to win real hospital evaluations beyond its anchor partner.

Our flow: identify suitable hospitals → map the buyer and introduction route → hold discovery → qualify the problem and buying path → activate an internal champion → agree the evaluation → obtain authorized usable read-only backup access → deliver the parallel system → hospital team completes its first test → review the evidence → sign the agreement → controlled rollout → sustained use and payment → renewal, expansion and referrals.

Our One Metric That Matters: the number of new hospital buying organizations outside our anchor partner that complete their first hands-on parallel-system evaluation session in the last 30 days.

Why this metric: introductions show access to people; database access shows hospital commitment; parallel readiness shows our delivery. The first real hospital test shows those steps have produced an actual evaluation. We count each new buying organization once, and separately check whether evaluations become paying, active customers.

How we improve it: inspect the stage where suitable hospitals are waiting-introductions, qualification, data approval, our delivery or hospital testing-and fix that blockage. Failed-fit accounts exit; fixable evaluation gaps loop back to delivery; satisfied customers can introduce the next account.

Copy assignment answer Back to the flowchart ↑

Basis and limits

- Nicholas Ho · 500 Eurasia B11 · Unit Economics & Metrics Tracking: lecture flowchart and OMTM workshop. Read the deck in order.
- Your explicit current bottleneck and read-only-backup parallel-evaluation offer, clarified in this conversation.
- Existing MediMind research: market focus, product mechanics, buyer pain hypotheses, buyer roles, tracking and reporting. These inform the proposed flow, not proof that each step has already happened.
- Lean Analytics · The One Metric That Matters: supplementary rationale for choosing a focus appropriate to the business's current stage.

Added to the original consolidated reading document on 24 September 2026. No prospect was contacted, no database was accessed, no tracking system was installed, and no live landing-page copy was changed by preparing this section.

E Latest homepage copy · 24 September 2026

Your ten options. On the actual opening screen.

Switch between the ten versions below. The typography, blue accents, three benefit columns and primary CTA follow your website’s existing hero design. Option 5 starts with your preferred proof-led opening.

05 / 10 · Prove the alternative Website-style preview · not a live page

For our next 10 hospital clients

One hospital. Too many systems.
Too much wasted time.

Your people shouldn’t have to hold disconnected software together. Replace it with one system built around your hospital.

- **One connected hospital.**
  Patient information, daily work and finances together.
- **AI in everyday workflows.**
  AI assistance where your teams do their work.
- **The foundation for your AI future.**
  Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build your parallel system free. Test the quality and improvements yourself before making any purchase commitment.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

For our next 10 hospital clients

Your best people deserve better than fighting software.

Less retyping. Less chasing answers. Give your teams a hospital system built around the work they actually do.

- **Give your people time back.**
  Connect information and simplify repeated work.
- **AI in everyday workflows.**
  AI assistance where your teams do their work.
- **The foundation for your AI future.**
  Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build a free parallel system for your team to test. Let them judge the quality and the difference. No obligation to buy.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

For our next 10 hospital clients

You shouldn’t have to chase answers about your own hospital.

Bring scattered information together in one tailored hospital system. See where work gets stuck and where money gets missed.

- **A clearer view of your hospital.**
  Care, operations and financial information connected.
- **AI in everyday workflows.**
  AI assistance where your teams do their work.
- **The foundation for your AI future.**
  Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

Start with a free parallel system built around your hospital. See the quality, measure the improvement and decide whether it earns the switch.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

For our next 10 hospital clients

Still waiting for the software changes your hospital needs?

Get one hospital system built around your workflows-and the team that keeps developing it around your priorities.

- **Keep improving.**
  Ongoing development included within agreed scope.
- **AI in everyday workflows.**
  AI assistance where your teams do their work.
- **The foundation for your AI future.**
  Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build your parallel version free, before asking you to buy. Your team tests it. You judge the quality and improvement. You decide.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

For our next 10 hospital clients

You know your hospital could work better.
Let us prove the alternative.

Replace disconnected software with one intelligent system built around your hospital. Give your teams a better way to work.

- **Built around your hospital.**
  Connected workflows, patient information and finances.
- **AI in everyday workflows.**
  AI assistance where your teams do their work.
- **The foundation for your AI future.**
  Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build and launch your parallel system free. Let your team test the quality and see the improvement for themselves. No obligation to buy.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

For our next 10 hospital clients

Your software needs to change.
Your hospital can’t afford a blind leap.

Move to one connected hospital system-without buying the replacement before you’ve tested it.

- **Your current system stays in place.**
  Evaluate the alternative alongside what you use today.
- **AI in everyday workflows.**
  AI assistance where your teams do their work.
- **The foundation for your AI future.**
  Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build your tailored parallel system free. Your team tests the quality and improvements before you make any purchase commitment.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

For our next 10 hospital clients

Your hospital has a future.
Your software should help you reach it.

Solve today’s disconnected workflows with one tailored hospital system. Start building the connected foundation your next generation of AI will need.

- **Solve today’s problems.**
  Connected information and simpler daily work.
- **AI in everyday workflows.**
  AI assistance where your teams do their work.
- **The foundation for your AI future.**
  Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build your parallel version free. Test the quality and improvements it delivers today before committing to the next step.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

For our next 10 hospital clients

You’re paying for software.
You shouldn’t keep paying for its problems.

Repeated work. Scattered information. Hours spent chasing answers. Replace disconnected systems with one tailored hospital platform.

- **Find the friction.**
  Make repeated work and financial gaps easier to see.
- **AI in everyday workflows.**
  AI assistance where your teams do their work.
- **The foundation for your AI future.**
  Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build your parallel alternative free. Compare the quality and improvements with your own team. No obligation to buy.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

For our next 10 hospital clients

Your AI future needs more than another disconnected tool.

It needs connected patient information, connected workflows and software built around your hospital. MediMind creates that foundation.

- **Connect the hospital first.**
  One shared system for clinical and business work.
- **AI in everyday workflows.**
  AI assistance where your teams do their work.
- **The foundation for your AI future.**
  Connected data to build toward proactive hospital-wide AI.

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We build your parallel system free. Test it alongside your current software and judge the quality and improvement before making a purchase decision.

Get my free parallel system →

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For our next 10 hospital clients

A better hospital today.
The foundation for an AI-powered hospital tomorrow.

Replace disconnected software with one tailored system. Put AI tools to work now and build toward more capable hospital-wide AI.

- **One tailored hospital system.**
  Built around your teams and the way they work.
- **AI in everyday workflows.**
  AI assistance where your teams do their work.
- **The foundation for your AI future.**
  Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build and launch your parallel version free. Let your people use it, challenge it and judge the improvement. Commit only if it earns your confidence.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

Preview only. No live page changes or evaluation requests are sent.

01One hospital. Too many systems. Too much wasted time.**02**Your best people deserve better than fighting software.**03**You shouldn’t have to chase answers about your own hospital.**04**Still waiting for the software changes your hospital needs?**05**You know your hospital could work better. Let us prove the alternative.**06**Your software needs to change. Your hospital can’t afford a blind leap.**07**Your hospital has a future. Your software should help you reach it.**08**You’re paying for software. You shouldn’t keep paying for its problems.**09**Your AI future needs more than another disconnected tool.**10**A better hospital today. The foundation for an AI-powered hospital tomorrow.

The ten-client price-match offer and free parallel evaluation reflect your requested offer. Proactive hospital-wide AI is presented as the direction being built toward-not as autonomous clinical decision-making available today. Scope and commercial terms still need to be finalized before publication. Earlier drafts and the full marketing analysis are preserved.

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01 Part 1 · Prompt 1

Describe Your Business & Market

Source: Complete library, pages 5-7 · 2,416 words · 7 requested steps / sections

> **Status and input (22 September 2026).** Working strategy for MediMind, a Georgian hospital operating system built on a shared FHIR R4 foundation. The first economic buyer is the CEO, owner, or group operator of a private multi-profile hospital or network; clinical, finance, operations, and IT leaders shape the decision. GeoHospitals is a founder-reported signed anchor agreement, not proof of a particular number of live sites or measured outcomes. Georgia is the first market; Kazakhstan and Uzbekistan are possible later markets. The founder describes the company as pre-revenue pending integration. Buyer pains below are hypotheses to test, not invented interview findings.

Step 1 - Restate it back to me sharply

MediMind gives private hospital leaders one operating system for clinical and business work, so they can test a clearer patient pathway, reduce repeated coordination, and gain a better basis for improving care and operations.

That sentence deliberately names the buyer, the mechanism, and the change they can inspect. It does not claim an outcome has already been measured. The shortest version for a conversation is: “We help a hospital see and improve the work that one patient creates across departments.” The broad destination is a hospital whose information and operations work together, with AI assistance that staff review and a longer-term path toward more proactive coordination. That destination still needs proof one workflow at a time.

The product is a hospital operating system, not a single documentation tool or management dashboard. Its shared record matters because the same patient journey touches registration, orders, care, medicines, results, billing, and reporting. A buyer does not purchase the data architecture for its own sake. They buy a credible way to make those handoffs easier, with acceptable transition risk and cost.

Step 2 - Name the real market I am in

The behavioral market is **hospital leaders already paying to compensate for work that fragments across clinical and business teams**. Their actual alternatives are doing more with the current supplier, asking staff to reconcile records, adding a focused tool, buying a broader established system, funding internal customization, or postponing replacement. This is the budget and management attention MediMind must win.

What the hospital does now	Why it is rational	When a new operating system becomes relevant
Keeps its current hospital software and asks that vendor for changes	Familiar staff routines and known support avoid a disruptive decision	A recurring cross-department problem remains unresolved at an acceptable cost or speed

What the hospital does now	Why it is rational	When a new operating system becomes relevant
Adds coordinators, spreadsheets, phone calls, and manual checks	People handle exceptions flexibly	The same patient information is recreated so often that the work can be observed and measured
Adds reporting, integration, or a narrow AI product	Targets a specific pain while preserving the core	Another layer makes a task easier but leaves the underlying handoffs and data disagreement intact
Replaces the core with a major vendor or open-source implementation	Broad scope, references, or code access may reduce perceived risk	The actual proposal fails the hospital's local, commercial, adaptation, or transition requirements
Waits	Avoids immediate change to care and revenue processes	Leadership can name an ongoing cost or clinical workflow burden large enough to justify evaluating a change

The practical first segment is not “every hospital in Georgia.” It is an organization with a named recurring pathway problem, an executive owner, a clinical or operations owner, an IT counterpart, and enough readiness to compare an agreed workflow. A 200-800-bed profile appears in internal buyer notes, but bed count alone is an unreliable qualifier. A multi-site group can be one buying organization with many facilities, while a single site can have a complex decision committee.

The first job is to learn what these buyers are trying to fix now. Internal notes suggest reimbursement gaps, avoidable staff effort, and delayed management visibility, but no local interview corpus establishes their frequency or ranking. At the point of sale, the relevant question is: **What was the last concrete patient pathway that forced people to switch screens, copy information, chase an answer, or reconcile a bill?**

Step 3 - Surface my 3 biggest assumptions

1. **The pain is important enough to change the core.** A hospital can dislike its software and still prefer a small fix or the known incumbent. If independent buyers cannot point to a recent costly problem, an owner, and an evaluation window, whole-system replacement is a poor first ask. Test by comparing a scoped MediMind pathway proposal with the narrower remedy the hospital would otherwise buy.
2. **The shared system changes real work, not just the look of the screens.** Better data flow can plausibly reduce repeated entry or reveal gaps, but policy, payer contracts, staff habits, and clinical complexity also drive results. If a measured pathway takes no less time, creates new work elsewhere, or does not improve information reliability, the strategic premise weakens. Test task time, repeat entries, handoff completeness, error/rework, and actual collected revenue where appropriate; do not count an identified charge as cash received.
3. **The attractive offer can be delivered repeatedly at sustainable cost.** Matching current software price, including continuing development, presenting requests for test in three business days, and arranging a tailored parallel evaluation in two weeks are powerful only if scope, resources, and economics work at more than one organization. Test engineering effort, hospital staff effort, support load, infrastructure and AI usage costs, reuse of changes, and acceptance. A testable feature is not a production-approved clinical release.

Each assumption has a disconfirming observation. A buyer who praises a demo but will not supply a workflow owner has not validated demand. A faster screen without less total pathway work has not validated the benefit. One unusually cooperative anchor deployment has not validated the repeatable business model.

Step 4 - Market sizing I can actually use

The defensible starting count is **232 hospitals and about 15,200 hospital beds in Georgia in 2025**, reported by Geostat in July 2026. This counts facilities, not purchasing organizations, qualified prospects, or buyers currently searching. It cannot be multiplied by a guessed annual contract value to produce a useful near-term market number.

Build the reachable market from identifiable organizations, one decision unit at a time:

Filter	How to verify it	Count now
Georgian hospital facilities	Official annual hospital statistics	232
Distinct organizations that buy hospital software	Deduplicate ownership and purchasing authority, including chains	Unknown
Private, multi-profile organizations whose operational complexity fits the product	Review actual service mix and workflow ownership	Unknown
Reachable through a lawful, named route to a decision-maker	Record introduction, contact route, and last verification date	Unknown
Actively seeking a solution	Record recent trigger, alternatives, budget route, owner, and timeframe from a real buyer	Unknown
Able to evaluate during MediMind's available delivery window	Confirm access, counterpart staff, data readiness, scope, and delivery capacity	Unknown

Direct answer: A verified count of reachable businesses actively looking for a solution is unavailable. The answer is not 232 and not zero. Treat the number as a research output, not a slide assumption.

A useful first pass is a **15-organization research list**, including the anchor separately, and **8-10 conversations across at least four independent organizations**. These are proposed discovery targets, not an estimate of live demand. Publicly findable candidates in the prior research include Georgia Healthcare Group/Georgia Capital's healthcare operations, Aversi Clinic, and Vivo Medical Group; their existence does not imply interest or access. GeoHospitals belongs in an anchor/reference column, not the new-customer pipeline. A historical HelsiCore mention needs reconfirmation before it is treated as an active opportunity.

Named organization for the first research list	Why it is findable	Demand and reachability status
GeoHospitals	Founder-reported signed anchor agreement	Existing relationship; exclude from new-buyer count and verify approved reference scope
Georgia Healthcare Group / Georgia Capital healthcare services	Public operating-group reporting	No verified access, current software pain, or purchase intent
Aversi Clinic	Public hospital/clinic website	No verified access, current software pain, or purchase intent

Named organization for the first research list	Why it is findable	Demand and reachability status
Vivo Medical Group	Public group website	No verified access, current software pain, or purchase intent
HelsiCore	Historical founder-side lead in internal notes	Status and relationship need reconfirmation; not a live opportunity count

For each organization record the purchasing unit, facility count, named buyer and evaluators, current remedy, specific observed problem, trigger date, urgency, contract window, willingness to allocate an evaluation owner, and the next verified action. Only then calculate a bottom-up annual opportunity: sum the quoted recurring fees of eligible organizations, show implementation and optional AI separately, and attach explicit timing and conversion assumptions. A signed network agreement must not be inflated into a count of independently won facilities.

Step 5 - Identify the key players

Official vendor descriptions establish the alternatives a buyer can hear about; they do not prove Georgian availability, local price, head-to-head performance, or a vendor's weakness at a particular site. “Fast-growing” cannot honestly be assigned without a growth dataset, so the map groups established and emerging alternatives by the job they compete for.

Player or option	What it does well or offers	Limitation to test, not assume	Why a buyer may choose it now
Current supplier, including MEDSOFT/MEDWORK where installed	Local familiarity; the vendor describes hospital and government-system functions	Can the installed version solve this hospital's chosen pathway problem at acceptable speed and cost?	No migration, familiar support, and known work patterns
InterSystems IntelliCare	Markets a broad clinical, administrative, financial, and AI-connected proposition	Compare local availability, implementation responsibility, adaptation, and total cost in an actual proposal	Breadth, vendor resources, and category credibility
Epic	Broad clinical and enterprise scope with an established ecosystem	Check fit, localization, procurement path, and total cost for this buyer	Established references and institutional familiarity
Oracle Health	Broad EHR and embedded-AI direction	Distinguish available functionality from roadmap and check local fit	Familiar supplier scale or an existing relationship
Bahmni and implementation partners	Open-source hospital-system model and room for adaptation	Who pays for localization, hosting, maintenance, security, and support?	Greater code control or a trusted implementation partner
Commure	Focused clinical documentation and revenue-cycle automation	Does a targeted layer solve the recurring cross-department problem?	A narrower change can be easier to approve

Player or option	What it does well or offers	Limitation to test, not assume	Why a buyer may choose it now
Qventus	Operational automation, including actions in existing systems	Its scope and local fit must be evaluated against the specific pathway	The hospital may improve operations without replacing the core
Staff, spreadsheets, integration projects, and delay	Familiar, immediately controllable, often reversible	What is the continuing cost and residual error?	Avoids an uncertain vendor and transition

The closest message-level challenge is not whether another vendor says “one system” or “AI”: several do. The competitive test is whether MediMind can show a hospital-specific pathway, a credible local implementation, clear ongoing adaptation, and a safe decision process at a cost the buyer can defend. There is no evidence here to claim that all incumbents are disconnected, cannot use AI, or are categorically slower.

The prompt asks for “fast-growing startups,” but a growth label needs revenue, customer, or deployment evidence that this research did not establish. Commure and Qventus represent newer adjacent options competing for attention and budget; the table describes their publicly stated scope without asserting a growth rate. This is a more useful buying comparison than an unsupported startup ranking.

Step 6 - Pinpoint the gap

The underserved buyer is a private hospital leader whose clinical and business teams share a patient but still spend effort stitching that patient's work together, and who cannot justify a risky core replacement on a generic demo alone. That buyer needs a **visible, measurable first improvement** plus a way to understand the complete transition, not another abstract claim that software will “transform care.”

MediMind's proposed response has five parts. First, take an actual pathway across departments as the unit of change. Second, use a shared patient and operational foundation so information can follow the work. Third, let staff see the proposed pathway in a tailored parallel evaluation before a cutover decision. Fourth, offer continued development around the hospital's priorities. Fifth, reduce the initial subscription objection through the founder's early-partner price match. The exact coverage, total transition cost, request boundaries, and ongoing commercial terms must be made explicit per proposal; none is assumed to be zero or unlimited in delivery capacity.

This is a **gap hypothesis**, not a claim that no other company can deliver it. Its defensibility would come from repeatable local implementation, reusable care and business pathways, trusted support, and verified improvements. FHIR R4 is the shared technical base, like common grammar between departments; the hospital buys the easier conversation, not the grammar itself.

Step 7 - Market positioning statement

MediMind is the hospital operating system for Georgian private hospitals and networks that need clinical and business work to move together. Built on a shared FHIR R4 foundation, it lets a hospital start with one costly patient pathway, see how work and information can connect across teams, and evaluate a tailored version beside its existing system before deciding on a wider change. The early-partner offer matches current software pricing and includes continuing development, while the long-term product path adds more useful AI under the hospital's clinical control. The reason to act now is practical: repeated work and delayed visibility continue until the hospital measures and fixes a real pathway.

The investor-deck version should append actual evaluation results when available, rather than presenting an intended benefit as achieved. “Why now” should be anchored to the prospect's recent event-contract renewal, cross-site expansion, an unresolved billing handoff, or staff time lost to a pathway-not to a fabricated market deadline.

Sources and handoff to Prompt 2

Internal facts and limits: evidence register, buyer-pain synthesis, canonical scope, and the founder offer captured in the live-hero working draft. External sources are linked at the claims above. Next, map the complete discovery-to-evaluation-to-live-use sequence and pick the first pathway where a buyer can feel and measure value.

02 Part 1 · Prompt 2

Map Your Product Mechanics & JTBD

Source: Complete library, pages 8-10 · 2,080 words · 6 requested steps / sections

> **Status and input (22 September 2026).** Prompt 1 established the buyer (a private Georgian hospital or group), the real alternative (current software plus staff workarounds), and the central test (can one scoped patient pathway improve without an unacceptable transition?). This is Prompt 2's working product journey, assembled from founder terms, canonical scope, and internal positioning; it is not an account of one completed customer's path. The main conversion recommendation is a founder-led, 25-minute pathway demo, followed by a proposed measured evaluation. The founder offers requested features testable in three business days and a tailored parallel evaluation in two weeks, subject to readiness and a defined request. Actual production sign-off is separate. The source for the existing `/request-demo` form appears to show no backend lead submission; live behavior has not been independently tested.

Step 1 - Map the customer journey in steps

Use one cross-department example throughout: an ordered test is performed, the result reaches the clinician, and the related work reaches billing. The actual first pathway must be chosen with each hospital. The patient example in marketing should use synthetic data; any real evaluation requires the hospital's authorization, privacy review, and agreed handling.

#	What the customer does	What MediMind does	Result and information still needed
1. Recognize a recurring problem	CEO or operator names a recent case of repeated entry, delay, missing context, or disputed numbers	Explains the cross-department operating-system proposition through one pathway rather than a wall of capabilities	A reason to investigate. Need the buyer's actual incident, frequency, and current workaround.
2. Request a conversation	Provides contact and basic hospital context through `/request-demo` or a direct founder introduction	Should capture and route the request, acknowledge it, and prepare a relevant demonstration	**Conversion gap:** the current source for `src/components/Demo/RequestDemo.tsx` appears to simulate submission with a timer and log, then show success, rather than saving/sending the lead. This needs implementation and a live test before advertising a reliable inbound path.

#	What the customer does	What MediMind does	Result and information still needed
3. Map the first pathway	In a proposed 25-minute founder-led demo, explains where the patient journey creates repeat work and who owns it	Shows the corresponding registration, chart, ordering, results, coordination, and business handoffs with synthetic data	Buyer sees the shape of a possible solution. Need a named owner, systems touched, and the first comparison metric.
4. Decide whether evaluation is worth the effort	CEO, clinical/operations owner, and IT counterpart agree on one pathway, baseline, access requirements, staff time, success criteria, and a decision date	Produces a short evaluation plan, product-scope map, responsibilities, cost view, and readiness check	An actionable evaluation proposal. Missing details vary by site; there is no universal readiness or security approval.
5. Prepare a tailored parallel environment	Hospital authorizes the agreed data or synthetic substitutes, mapping, and staff participation	Configures the relevant workflows and connects or imports the agreed inputs where feasible; staff check data meaning and permissions	A comparable pathway beside current work. The founder's **two-week** offer is a readiness-dependent target, not a promise of whole-hospital migration in two weeks.
6. Compare the first end-to-end pathway	Frontline staff complete the agreed tasks and record friction, safety concerns, time, and exceptions	Shows patient context and linked work in the shared FHIR R4 foundation; provides AI assistance where the specific function is available and approved for use	The first observable difference. Clinical decisions remain with clinicians. Do not confuse a demo click path with accepted real-world performance.
7. Review evidence and decide	Clinical, finance, operations, and IT owners compare baseline with the parallel path and assess burden, errors, data quality, total cost, support, and transition risk	Resolves issues, presents what is supported versus pending, and proposes a staged scope and commercial terms	A go/adjust/stop decision. Even if a request is testable in three business days, production requires separate clinical, operational, security, and contractual sign-off.
8. Expand and operate if approved	Hospital approves controlled rollout, training, responsibilities, and cutover criteria	Deploys agreed capabilities, supports staff, reconciles data, and iterates on prioritized needs	The long-term outcome must be measured across real work: less repeated effort, more reliable information, and any legitimate financial improvement actually realized. Exact rollout and support commitments need a contract.

The product mechanic is a shared work context, not merely “integrations.” Patient data and hospital tasks live in a common model; a doctor, coordinator, pharmacist, lab worker, nurse, and finance colleague act on their respective parts of a connected journey. The hospital still needs interfaces to outside payers, ministries, devices, and other external services wherever required. Scope is verified per site.

Step 2 - Identify the moment of value

Aha moment: the hospital's own team completes its chosen pathway in the parallel environment, then can point to one verified handoff that required less reconstruction of patient information or less repeated work.

The aha is not signing up, seeing a beautiful dashboard, or watching AI text appear. It happens when the coordinator no longer has to reassemble what the clinician, lab, and billing team already know-or when the clinical team can find the needed context in the agreed place-and the hospital can compare this with its baseline. The first moment can be qualitative ("we no longer need this phone call") before it is statistically persuasive; record it without converting it into a general outcome claim.

This matters for retention because a hospital keeps a system that makes its routine work reliably easier and understandable across roles. It matters for marketing because a single, visible pathway is easier to demonstrate and substantiate than an unmeasured promise to improve an entire hospital. The first proof package should show the pathway before and after, staff effort, exceptions, and what still required manual work.

Step 3 - Identify friction points

Journey point	Why the buyer may drop or lose trust	Specific repair or question
Initial message	"Hospital operating system" sounds too broad to believe; AI can sound clinically autonomous	Open with a familiar patient pathway and specify that AI drafts or flags while staff decide.
Demo request	The existing three-step, nine-required-field form asks for effort before value; the source-observed simulated submit could lose leads	Shorten the request to what scheduling needs, implement reliable submission/notification, and test the live route. The exact public form change belongs to implementation work.
Founder demo	A generic feature tour may miss the hospital's real problem	Ask for the last concrete failure and map that journey before showing the relevant capability.
Scope selection	"One operating system" may be heard as a promise to replace every external tool instantly	Make a site-specific scope map: included work, necessary outside interfaces, unverified functions, and later phases.
Data and technical readiness	Unclear access, source quality, terminology, permissions, and security review can stall a two-week evaluation	Name data owner and IT counterpart, test mapping, and define synthetic versus authorized real data before starting the clock.
Staff burden	Parallel work can double tasks temporarily; clinicians may see it as a risky distraction	Limit the pathway and sample, document extra work, let clinical owners stop unsafe use, and collect staff feedback.

Journey point	Why the buyer may drop or lose trust	Specific repair or question
Outcome comparison	A polished demo can be mistaken for proof; an incomplete baseline can make measurement impossible	Agree on a small metric set and method before evaluation. Mark missing denominators and record adverse effects too.
Commercial decision	Price match and continuing development sound open-ended; total cost and priorities are unclear	Put the founder's attractive offer in writing with the actual eligible scope, request process, third-party costs, term, and renewal treatment. These are commercial definitions to settle, not pre-existing exclusions.
Rollout and support	A young supplier concentrates operational dependency	Review support, data access/export, downtime approach, change control, and exit provisions with IT and clinical owners.

Step 4 - Apply the JTBD lens

“Jobs to be done” means the progress the buyer wants at each moment, beyond the screen they click. The executive's functional job is economic and operational; frontline and IT roles determine whether that progress is real and safe.

Major step	Functional job	Social job	Emotional job
Recognize and request	Find a practical option for an unresolved cross-department problem	Show the board and team that the leader is addressing a known bottleneck	Feel there may be a way forward rather than another expensive presentation
Map a pathway in the demo	See whether the proposed system understands the hospital's actual work	Be seen as a leader who listens to staff and asks hard questions	Feel understood and retain control of the conversation
Scope the evaluation	Decide what is safe and worthwhile to test	Demonstrate discipline to clinical and IT colleagues	Feel able to investigate without committing the whole hospital
Prepare parallel work	Keep ordinary operations running and protect data meaning	Be a responsible steward of patients, people, and information	Reduce fear of an uncontrolled change
Complete and compare the pathway	Prove whether a task is clearer, faster, or more reliable	Show a defensible result to leadership and staff	Feel relief that improvement may be tangible, or confidence to stop if it is not
Decide and roll out	Buy only the scope the hospital can operationalize; expand based on evidence	Be the leader who improved work without a careless switch	Feel accountable and confident about the next step

Across these steps the core job is: **“Help me improve how this hospital handles a patient, while giving me enough evidence and control to defend the change.”** Different committee members put weight on different parts. A clinician wants patient context and review authority. Finance wants reconciliation and actual collections, not speculative uplift. IT wants feasible mapping, support, and an exit path.

Step 5 - Identify my market sophistication level

Working assessment: Level 4 of 5, “New Mechanism.” This is an editorial score, not a survey of Georgian buyers. Hospital buyers already hear claims about unified records, less administration, and embedded AI from substantial vendors such as InterSystems and Oracle Health. A fresh list of generic benefits will blend in. “Experience” (Level 5) may apply to an individual buyer fatigued by failed implementations, but cannot be assumed for the whole segment.

The message therefore needs a concrete, believable mechanism: **choose one patient pathway; show the shared work across roles; compare it beside the current system; decide from observed evidence.** Lead with the work the CEO recognizes, explain the shared FHIR R4 foundation only as a reason the handoffs can connect, and put AI in its proper role as assistance with human review. The founder's price match, included continuing development, three-business-day test target, and two-week parallel offer make the mechanism more tangible when written with site-specific scope and readiness. Avoid “first,” “fully autonomous,” “zero risk,” and guaranteed clinical or financial outcomes.

Step 6 - Rewrite the product description

Customer-facing description (112 words):

> One patient can create work across reception, clinicians, the lab, pharmacy, coordinators, and finance. MediMind brings that work into one hospital operating system, so each team can act with the patient context it needs and leaders can see where the journey stalls. Start with one pathway your hospital wants to improve. We map it with your team, show how it works in MediMind, and propose a tailored evaluation beside your current system. Your clinicians remain responsible for care decisions; AI helps with defined tasks for them to review. Compare the work, cost, and staff experience before deciding how far to go. As your needs change, the system can keep developing with you.

Sources and handoff to Prompt 3

The business and market answer, canonical scope, evidence register, and live offer snapshot ground this journey. The `/request-demo` observation comes from the current source file `src/components/Demo/RequestDemo.tsx`; it needs a live browser check before describing deployed behavior. Prompt 3 should translate each documented domain into a buyer benefit, while keeping code presence, deployment, acceptance, and outcomes distinct.

03 Part 1 · Prompt 3

Build Your Features, Benefits & Guarantees

Source: Complete library, pages 11-12 · 4,068 words · 6 requested steps / sections

Status and input (22 September 2026). This answers all seven input questions and all six requested steps using the canonical 25-domain scope inventory, the canonical product description, Prompts 1-2, the founder offer, and the evidence register. “Documented” means listed in internal positioning with a named code path or architectural assertion; it does **not** mean a feature has passed a fresh code audit, is deployed at every hospital, has been accepted in production, or has produced measured outcomes. The internal positioning claims the ambient scribe and coordinator desk are live; current site, acceptance, and use details were not verified for this document. AI assistance requires human review in clinical work. A guarantee below is either an existing founder-stated offer needing written scope or a proposed evaluation trust signal-not an invented service level.

The seven required inputs

1. **What are all the features?** The canonical scope names 25 hospital domains, reproduced in Step 1, plus two cross-cutting architecture/language capabilities in the canonical product description. Each domain includes smaller actions, from registering a patient to routing a pharmacy queue or reviewing a clinical event. The source is a product inventory, not a release-by-release acceptance register.
2. **What does the customer get from each?** The benefit is less reconstruction of a patient or operational story, clearer handoffs, and a more inspectable path from care to business work. Step 2 translates every domain separately; each is a potential benefit to test in the hospital's own pathway.
3. **Single most important outcome?** The hospital completes a real patient pathway with less avoidable handoff work and a trustworthy shared account of what happened, while preserving clinical judgment and operational control. This is a target outcome, not a measured result. Financial gains, if any, must be measured as appropriate collections or costs, not simply billing opportunities or minutes assumed to be cash.
4. **Top three objections?** (a) “Will switching disrupt care and revenue?” (b) “Can we trust a young supplier with critical operations, data, and support?” (c) “What exactly is included in price, development, and AI as needs grow?” A broad demo without a staged, costed, clinically reviewed evaluation will not answer these.
5. **What proof exists?** A founder-reported GeoHospitals agreement; internal product scope and code-path inventory; a Medplum-hosted case study authored by a founder; and a founder-stated early-partner offer. These establish narrative, technical work, and a relationship, but they do not independently establish site-wide deployment, a customer endorsement, quantified savings, better outcomes, a certified security posture, or repeatable delivery time. No public testimonial or attributed outcome is supplied.
6. **What guarantees can honestly be offered?** The founder has stated: match current software price for early hospital partners, include continuing development, make requested features testable in three business days, and prepare a tailored parallel evaluation in two weeks. These need scope, eligibility, readiness, and total-cost definitions in writing. The hospital should retain the decision after a parallel evaluation; production

deployment requires its separate approval. No blanket money-back promise, zero-risk promise, result guarantee, permanent price lock, fixed scarcity, or unbounded AI inclusion has been established.

7. **What do customers say about why they chose MediMind?** No traceable buyer quote was supplied. Do not write an invented first-person testimonial. The founder-reported anchor agreement signals that at least one buying organization signed, but its exact selection reasons require a buyer interview and permission before public attribution. Working hypotheses to test are the connected scope, local adaptation, founder access, and an opportunity to evaluate before switching.

Step 1 - List every feature

Rows 1-25 below are the **complete canonical domain list**, organized by the patient and hospital work they support. Rows 26-27 add the two cross-cutting capabilities asserted in the canonical product description. “Scope-listed” is the appropriate common evidence label for domains. It does not make every subfunction live at every site. Before using a row in a proposal, confirm exact workflow, configuration, interfaces, data quality, permissions, localization, site deployment, and acceptance.

#	Category and documented domain	Concrete capability to demonstrate or specify	Current evidence boundary
1	Front door: Patient registration	Patient identity, coverage, related people	Scope-listed; local identity and payer rules need checking
2	Clinical: Ambient documentation	Draft clinical documentation linked to encounter and provenance	Scope-listed; internal live assertion, no fresh acceptance audit
3	Clinical: Computerized physician order entry (CPOE)	Clinical orders for services and medicines	Scope-listed; order catalogs and safety checks need site validation
4	Clinical: Medication administration record (MAR)	Record medicine administration at point of care	Scope-listed; high-risk workflow requires clinical approval
5	Clinical: Pharmacy queue and dispense	Route prescriptions and record dispense work	Scope-listed; inventory and formulary fit need checking
6	Diagnostics: Laboratory	Orders, specimens, observations, diagnostic reports	Scope-listed; instruments and result flow are site-specific
7	Diagnostics: Imaging/PACS viewer	Imaging studies and linked documents available to review	Scope-listed; image archive and viewer fit need checking
8	Capacity: Bed management	Locations and encounter placement	Scope-listed; real-time census accuracy needs validation
9	Access: Appointments and scheduling	Schedules, slots, and booked appointments	Scope-listed; local booking rules need checking

#	Category and documented domain	Concrete capability to demonstrate or specify	Current evidence boundary
10	Coordination: Coordinator/care navigation	Tasks, care teams, and communications	Scope-listed; internal live assertion, no fresh acceptance audit
11	Finance-adjacent: Prior authorization and appeals	Work queues and records for approval/appeal tasks	Scope-listed; payer-specific rules and human review required
12	Finance: Billing and revenue cycle	Charges, definitions, accounts, billing work	Scope-listed; pricing, contracts, and code mapping need checking
13	Finance: Claims processing	Claims, responses, and coverage information	Scope-listed; payer integration and actual collections unverified
14	Supply: Inventory and warehouse	Stock quantities, supplies, deliveries, requests	Scope-listed; physical counts and purchasing rules need checking
15	Workforce: HR and staff management	Practitioner roles and schedules	Scope-listed; payroll and local employment scope not implied
16	Management: Analytics	Read-only views across relevant clinical and operating data	Scope-listed; definitions, completeness, and freshness need checking
17	Public obligations: Ministry submissions/compliance	Prepare/route relevant reporting data	Scope-listed; no blanket regulatory-compliance claim
18	External records: Government EHR integration	Exchange bundles/parameters with government system	Scope-listed; interface availability and approval are external dependencies
19	Group operations: Multi-branch orchestration	Organization hierarchy and branch context	Scope-listed; cross-site data access and reporting need testing
20	Resilience: Offline-first progressive web app	Local cache, sync queue, service worker	Scope-listed; data sync/conflict behavior needs site testing
21	Nursing: Nursing workstation	Nursing tasks, observations, medicine-administration work	Scope-listed; nurse review and handoff fit need checking
22	Coordination: Clinical event engine	Event/subscription-driven tasks and signals	Scope-listed; never imply autonomous patient surveillance

#	Category and documented domain	Concrete capability to demonstrate or specify	Current evidence boundary
23	Teamwork: Messaging and staff inbox	Communications and assigned tasks	Scope-listed; delivery, privacy, and escalation rules need checking
24	AI: Clinical assistant	Assistance within the shared clinical context	Scope-listed; specific functions and clinical review rules need checking
25	Clinical: Unified patient card and history	Patient, encounter, observation, and related history in one chart	Scope-listed; completeness depends on migrated/connected source data
26	Foundation: Shared FHIR R4 data structure	Clinical and business work uses a common health-data model on Medplum	Internal architecture assertion; interoperability still depends on mapping and interfaces
27	Localization: Georgian, Russian, English architecture	Product is architected for three languages	Internal architecture assertion; translation completeness and workflow fit need review

FHIR R4 is the common data foundation behind the rows. The scope document names resource types and code paths for the 25 domains; that is useful technical traceability, not a deployment certificate. Some hospitals will keep necessary outside services and device interfaces. “One operating system” describes the intended core, not a promise that every external dependency disappears.

Step 2 - Convert every feature to a benefit

These are **customer-language benefit hypotheses**. Each should be tied to a demonstration and, where material, an agreed baseline. The grammatical form is intentionally “I can…” rather than an unearned claim that all hospitals already achieved it.

#	Direct customer benefit
1	“I can start the patient's journey with identity and coverage details the next team can use.”
2	“My clinicians can spend less time drafting routine notes from scratch, then review and own the final record.”
3	“Orders can follow the clinical decision into the next team's queue without staff rebuilding the request.”
4	“Nurses can see and record what medicine was actually given in the same care context.”
5	“Pharmacy staff can work from an organized prescription queue and record what was dispensed.”

#	Direct customer benefit
6	“The care team can follow a test from order to result and review, with fewer calls to discover its status.”
7	“A clinician can reach the relevant imaging information from the patient's work context.”
8	“My team can place and track patients with a clearer view of available beds.”
9	“Scheduling staff can offer and manage appointments without rebuilding the patient's context.”
10	“Coordinators can see the next task and who owns it, instead of carrying the pathway in memory.”
11	“Approval and appeal work can be assembled and tracked more coherently, with staff reviewing what goes out.”
12	“Finance can relate legitimate charges to the care documented and identify records that need correction.”
13	“The team can see where a submitted claim stands and what response needs action.”
14	“Departments can see and request needed supplies with less manual stock chasing.”
15	“Managers can connect staff roles and schedules to the care and work that must be covered.”
16	“Leaders can inspect agreed operating measures without waiting for a fresh spreadsheet assembly.”
17	“Reporting staff can prepare required submissions from more consistent records, subject to local validation.”
18	“The hospital can exchange agreed information with a government record system where the interface is available.”
19	“A group operator can view and manage branches with a shared structure and site-specific context.”
20	“Staff may be able to keep selected work available during connectivity loss and reconcile it when service returns.”
21	“Nurses can see assigned care tasks and observations in a focused work area.”
22	“Relevant events can create a visible task or signal for a responsible person to review.”
23	“The right staff member can receive a message or task within the hospital's work context.”

#	Direct customer benefit
24	“Clinicians can ask for defined assistance while keeping the judgment and final action with the care team.”
25	“A clinician or coordinator can see more of the patient's story in one place, depending on available source data.”
26	“My teams can work from a shared data structure, making agreed handoffs and future changes easier to design.”
27	“Our Georgian, Russian, and English-speaking staff can evaluate whether the system fits their daily language needs.”

No benefit here automatically establishes cash savings, a clinical outcome, or use by a specific customer. For example, recording an unbilled charge can help legitimate billing, but only adjudicated and collected amounts establish realized financial gain. A minute freed for a nurse is capacity, not automatically a reduced wage bill.

Step 3 - Add the emotional benefit layer

The emotional and social job varies by user. Below, “feel” names an intended experience to test, and “be seen” names the professional identity at stake. These are not customer quotations.

#	Emotional and social job behind the benefit
1	Reception feels confident that the journey starts correctly; colleagues see a reliable handoff.
2	Clinicians feel heard rather than buried in typing; peers see a complete, clinician-owned note.
3	Prescribers feel confident that an order reached the next role; receiving teams see fewer ambiguous requests.
4	Nurses feel accountable and supported; clinical leaders see a traceable administration record.
5	Pharmacy feels in control of the queue; clinicians see a dependable medicines handoff.
6	Clinicians feel less need to chase; patients and colleagues see a coordinated diagnostic journey.
7	Clinicians feel prepared to decide; peers see the relevant imaging context when it matters.
8	Bed managers feel less reactive; leadership sees capacity decisions made from a coherent picture.
9	Scheduling staff feel organized; patients see a hospital that remembers their context.

#	Emotional and social job behind the benefit
10	Coordinators feel less alone carrying the journey; teams see clear accountability.
11	Approval teams feel prepared, not rushed; payers and leaders see a reviewable record.
12	Finance leaders feel able to explain the gap; clinicians see legitimate, accurate documentation respected.
13	Claims teams feel in control of follow-up; executives see disciplined revenue work.
14	Supply teams feel less surprised by shortages; clinical teams see dependable preparation.
15	Managers feel better able to cover work; staff see fairer, clearer responsibility.
16	Executives feel less exposed in decisions; boards see a leader who can explain the numbers.
17	Reporting staff feel ready for scrutiny; the organization appears more orderly and accountable.
18	IT feels able to meet an external obligation; leadership sees a manageable connection.
19	Group operators feel in command of multiple sites; owners see consistent oversight.
20	Staff feel less helpless when connectivity fails; leadership sees preparedness, if sync is verified.
21	Nurses feel their shift is organized around care; colleagues see dependable follow-through.
22	Responsible staff feel informed without surrendering judgment; leaders see clear event ownership.
23	Team members feel included in the same work; colleagues see timely handoffs.
24	Clinicians feel supported rather than replaced; peers see responsible AI use.
25	Care teams feel oriented to the patient; patients see professionals who know their story.
26	IT feels less trapped by incompatible data structures; leadership sees a foundation they can inspect.
27	Multilingual staff feel included; managers see a more locally usable system if translations are complete.

The highest-leverage emotional job for the economic buyer is **confidence to improve a hospital without gambling today's care, budget, or reputation on an untested switch**. That job links the broad product to the parallel evaluation.

Step 4 - Build the guarantees and trust layer

Only the first three entries describe founder-stated commercial offers. They should be written as agreed terms for eligible partners before public use at scale. The final two are proposed trust signals and evaluation practices, not contractual warranties. “In two weeks” and “in three business days” require a defined start condition and scope; no such detail is invented here.

ID	Guarantee or trust signal; what it promises	Why it reduces buying risk	Where it belongs
T1	**Tailored parallel evaluation in two weeks**, when the agreed pathway and readiness conditions are met; hospital decides after comparing the work.	Makes the first decision an inspectable, limited evaluation rather than a blind cutover.	Hero support line, founder demo close, evaluation plan.
T2	**Current software price matched for eligible early hospital partners**; exact scope, term, and total transition costs to be written in the proposal.	Removes an immediate subscription-budget objection without pretending total cost is unchanged.	Pricing conversation and proposal, after the pathway value is clear.
T3	**Continuing development included, with requested features ready to test in three business days** under an agreed request process.	Makes adaptation feel practical rather than another open-ended vendor queue.	Demo follow-up and contract schedule; clarify testable versus production-approved.
T4	**Visible product mechanism:** demonstrate the actual scoped pathway and relevant FHIR-based record/task flow; give IT a documented scope map.	Lets clinical, operational, and technical evaluators examine what is real instead of accepting an architectural slogan.	Demo, technical diligence, and scope appendix.
T5	**Measured, buyer-controlled decision:** agree baseline, responsibilities, exceptions, clinical sign-off, and go/adjust/stop criteria before rollout.	Preserves decision rights and exposes missing proof early.	Evaluation plan, review meeting, and staged rollout decision.

Do not replace this evidence with a fake testimonial, unverified certification badge, or promised outcome percentage. The anchor agreement can be described accurately when approved for external use; any named logo, customer quote, live-site count, or quantified result needs separate confirmation and permission.

Step 5 - Build the BFG master table

This is the copy source of truth for the canonical 25 domains and two documented cross-cutting capabilities. A row is a **message candidate**, not permission to claim the capability is deployed or beneficial in every hospital. The trust ID resolves to the precise wording and status in Step 4; T4 and T5 are process evidence, while T1-T3 are founder-stated offers awaiting final written definitions.

Feature	Benefit	Emotional Job	Guarantee or Trust Signal
1. Patient registration	Start with reusable identity and coverage context	Reception feels dependable; staff trust the first handoff	T4 scoped pathway demo; T5 data check
2. Ambient clinical documentation	Draft a note for clinician review with less blank-page work	Clinician feels present with the patient and owns the record	T4 live function demonstration; T5 clinical review
3. CPOE	Send an order into the next role's work with context	Prescriber feels the request will not disappear	T4 order-to-receipt demo; T5 safety sign-off
4. MAR	See and record medicine administration in care context	Nurse feels accountable and supported	T4 task demonstration; T5 clinical sign-off
5. Pharmacy queue and dispense	Organize prescription-to-dispense work	Pharmacy feels in control of demand	T4 queue demonstration; T5 site formulary check
6. Laboratory	Follow order, specimen, result, and review	Clinician feels less need to chase the answer	T4 end-to-end demo; T5 instrument/result check
7. Imaging/PACS viewer	Reach imaging context during care	Clinician feels prepared to decide	T4 viewer demonstration; T5 archive/interface check
8. Bed management	See patient placement and capacity in context	Bed manager feels less reactive	T4 capacity demo; T5 census comparison
9. Appointments/scheduling	Manage slots and bookings with patient context	Scheduling staff feel organized	T4 booking demo; T5 local rules review
10. Coordinator/care navigation	Keep tasks and owners visible across a pathway	Coordinator feels less alone carrying handoffs	T1 parallel pathway; T4 task demo
11. Prior authorization/appeals	Track approval work with a reviewable record	Team feels prepared and accountable	T4 task demo; T5 human review
12. Billing/revenue cycle	Connect documented work to legitimate charge review	Finance feels able to explain gaps	T1 parallel comparison; T5 actual-collections measure
13. Claims processing	See submission, response, and follow-up work	Claims team feels in command of exceptions	T4 claims demo; T5 payer validation
14. Inventory/warehouse	See and request supplies more coherently	Supply team feels ready rather than surprised	T4 stock demo; T5 physical reconciliation

Feature	Benefit	Emotional Job	Guarantee or Trust Signal
15. HR/staff management	See roles and schedules related to work	Managers feel prepared to cover shifts	T4 role/schedule demo; T5 scope review
16. Analytics	Inspect agreed operating measures sooner	Executive feels better able to defend decisions	T1 pathway comparison; T5 definitions and baseline
17. Ministry submissions/compliance	Prepare required data more consistently	Reporting team feels ready for scrutiny	T4 report demo; T5 local requirements check
18. Government EHR integration	Exchange agreed records with an external system	IT feels able to meet a data obligation	T4 interface specification; T5 external approval
19. Multi-branch orchestration	Coordinate sites with shared structure and local context	Group leader feels oversight across hospitals	T4 branch demo; T5 site access review
20. Offline-first PWA	Continue selected work during lost connectivity, then sync	Staff feel less helpless during outages	T4 offline/sync demo; T5 conflict test
21. Nursing workstation	See assigned tasks and observations in one work area	Nurses feel the shift is manageable	T4 nurse walkthrough; T5 clinical review
22. Clinical event engine	Route a defined event to a responsible reviewer	Staff feel informed while retaining judgment	T4 event demo; T5 escalation rules
23. Messaging/staff inbox	Keep messages and assignments within the work context	Team feels connected and accountable	T4 delivery demo; T5 privacy rules
24. AI clinical assistant	Get defined assistance with human final decisions	Clinician feels supported, not replaced	T4 function demo; T5 clinical governance
25. Unified patient card/history	See more of the patient story where work happens	Care team feels oriented and prepared	T1 pathway evaluation; T5 completeness check
26. Shared FHIR R4 structure	Design agreed handoffs and changes on common data	IT feels able to inspect the foundation	T4 data-model walkthrough; T5 mapping check
27. Georgian/Russian/English architecture	Evaluate daily work in staff languages	Staff feel included in a local fit	T4 language walkthrough; T5 translation review

The founder offer T2-T3 applies to the buying relationship, so it should accompany the table as a whole rather than be falsely attached as a technical performance guarantee to each clinical feature. If a claim must appear in public copy, lead with a row already demonstrated in the intended hospital setting and add the exact proof available.

Step 6 - Identify my strongest BFG combination

Strongest combination: The unified patient card plus coordinator tasks and the selected downstream handoff (feature), a pathway the team can follow with less reassembly (benefit), confidence that leadership can improve work without losing control (emotional job), and a tailored parallel evaluation with buyer-owned success criteria (T1

+ T5). This connects the widest product story to one experience the hospital can actually judge. It is stronger than using an unmeasured financial or clinical outcome as the headline.

- **Rational:** “Choose one patient pathway, then compare how its information and tasks move across your teams in MediMind beside your current system.”
- **Emotional:** “See your people spend less effort piecing together a patient's journey before you ask them to change how they work.” *Use only as an aspiration until a site comparison shows less effort.*
- **Identity-based:** “Lead a hospital that improves from evidence: start with one pathway your own clinical and operations teams can inspect, measure, and approve.”

Recommended reusable line: “One patient. Too much repeat work. See the first pathway your hospital could improve.” The accompanying CTA is **“Map my hospital's first improvement.”** The line invites the hospital to identify the actual bottleneck; it does not assert that MediMind has already removed a measured amount of work.

Sources and handoff to Prompt 4

Core sources: canonical product scope, canonical language, evidence register, founder-offer hero snapshot, and Medplum's founder-authored case study. Prompt 4 should test direct brand promises against three executive buying situations, with the parallel pathway evaluation as the leading reason to believe.

04 Part 1 · Prompt 4

Generate Your UVPs & Map Your JTBD

Source: Complete library, pages 13-14 · 2,576 words · 6 requested steps / sections

Status and required inputs (22 September 2026). This is Prompt 4 of the complete B11 sequence. Prompt 1 defines MediMind as a hospital operating system for Georgian private hospitals and groups, competing first with current software and workarounds. Prompt 2 maps discovery, a founder-led pathway demo, a readiness-dependent parallel evaluation, evidence review, and staged rollout; the first value moment is the hospital's own team seeing one handoff work with less reconstruction. Prompt 3 inventories all 25 canonical domains and finds the strongest BFG combination in a connected patient pathway plus buyer-controlled parallel comparison. These are completed working answers, not validated buyer research. The ten UVPs and scores below are editorial options, not observed conversion results. The three buyer types are executive buying situations; finance, clinical, operations, and IT leaders remain essential members of each committee.

Required customer-type inputs

Buyer type	Description in 2-3 sentences	Single most important outcome
A. Private multi-profile hospital CEO or owner	Runs a hospital where clinical quality, staff capacity, and financial performance collide in one operating decision. May be close to daily management but must defend a major software change to clinicians, managers, and owners.	Improve one costly cross-department pathway with a transition and total cost the organization can justify.
B. Private hospital-group CEO or operator	Oversees more than one facility and needs a reliable view of comparable work without erasing necessary local differences. A new site, inconsistent numbers, or repeated implementation effort makes the limits of separate systems visible.	Establish a repeatable way to improve and oversee patient pathways across the network.

Buyer type	Description in 2-3 sentences	Single most important outcome
C. Clinician-owner or medically led hospital executive	Has purchasing influence and sees how software interrupts care and creates duplicate clinical work. Wants useful AI and connected context while preserving clinician judgment and staff trust.	Give clinicians a more coherent patient journey without asking them to accept unreviewed clinical automation.

Step 1 - Map the full JTBD for each customer type

The jobs below are **hypotheses drawn from internal positioning and hospital-context research**, not verbatim statements from interviews. “Job” means the progress a buyer is trying to make when an event forces a decision.

A. Private multi-profile hospital CEO or owner

Layer	Job	Trigger moment	Previous solution or workaround
Functional	Identify a pathway that wastes staff effort, obscures legitimate revenue, or delays decisions; fix it without destabilizing the hospital.	A contract renewal, repeated billing rework, staff overtime, or a report with irreconcilable numbers.	Ask the current vendor for a change, add checks and coordinators, buy another tool, or defer replacement.
Social	Be seen by owners, doctors, and managers as the leader who improved operations and protected care.	A board or clinical leadership meeting asks why the known problem persists.	Present another improvement plan or select a familiar vendor for reassurance.
Emotional	Feel confident that the change is worth the money and will not become a public operational failure.	The proposed replacement feels larger and riskier than the known problem.	Demand more presentations and references, then postpone a decision.

The CEO's job is not to “adopt AI.” It is to deliver a better-run hospital with evidence and control. Price matching may open the conversation, but a visible pathway and credible transition make it a decision.

B. Private hospital-group CEO or operator

Layer	Job	Trigger moment	Previous solution or workaround
Functional	See comparable performance and spread an improvement across sites without rebuilding it from scratch each time.	A new acquisition, branch opening, unexplained variation, or a late group report.	Separate site suppliers, spreadsheet consolidation, local process owners, and repeated integration projects.
Social	Show owners and site leaders that growth comes with oversight and a practical standard of care.	A board asks why similar facilities cannot report or operate consistently.	Mandate more reporting and meetings while each site keeps its own workflow.
Emotional	Feel in command of the network while still respecting site realities.	A cross-site issue becomes visible only after its cost or clinical impact grows.	Rely on calls to local managers and after-the-fact reconciliation.

For this buyer, one pathway is the prototype for a repeatable operating pattern. The proof must show both what can be standardized and what must remain local; a shared dashboard alone is not enough.

C. Clinician-owner or medically led hospital executive

Layer	Job	Trigger moment	Previous solution or workaround
Functional	Give care teams the relevant patient context and reduce documentation and handoff work while keeping clinical decisions with clinicians.	A missed result, a slow handoff, a near miss, clinician complaints, or another request the current system cannot accommodate.	Phone calls, duplicate notes, manual checks, separate tools, and personal oversight of exceptions.
Social	Be seen by clinicians and patients as a leader of safe, useful modernization.	Clinicians resist a new system or ask whether AI is being introduced responsibly.	Trial isolated AI tools or announce an innovation initiative without changing the underlying work.
Emotional	Feel proud of a care environment where technology supports staff rather than second-guesses them.	A clinician describes losing attention to screens or an AI proposal sounds autonomous.	Delay rollout or restrict it to low-risk pilots with little effect on the wider pathway.

This buyer wants the path from “interesting technology” to “my staff trust using it.” The message must keep review rights and clinical governance visible. A claimed clinical outcome needs its own study, beyond a process demo.

Shared core job: “Help me improve how this hospital handles a patient, with enough evidence and control to defend the change.” It carries a functional outcome (less reconstruction and more reliable information), a social outcome (responsible leadership), and an emotional outcome (confidence rather than anxiety about a switch).

Step 2 - Confirm market sophistication level

Working score: Level 4 of 5, “New Mechanism.” The label is a copywriting diagnosis, not a measured Georgian-market statistic. Established vendors such as InterSystems IntelliCare and Oracle Health already describe broad connected systems with AI. Familiar claims like “one record,” “better care,” and “less admin” can establish relevance but cannot carry a distinctive message alone. Individual buyers who have endured failed replacements may react more like Level 5 (“Experience”), so the copy should invite firsthand evaluation early.

The right language shows a concrete mechanism and a controlled next step: **one patient → too many repeated handoffs → map the hospital's version → compare a connected path beside current work → decide from evidence.** Use the operating-system category, then plain language about the work people do. The FHIR R4 foundation can substantiate why data can share a structure, but should not replace the buyer's problem in the headline. The offer-matched current software price for eligible early partners, continuing development, three-business-day test target, and a readiness-dependent two-week parallel evaluation-supports the mechanism. These terms need written definitions. AI remains assistance under clinician review, with broader coordination a product direction.

Step 3 - Generate 10 UVPs as brand promises

Every line below speaks directly to the buyer about a valuable change. Wording that says “can” or “see whether” is intentional where performance has not yet been measured. Some lines suit a specific pathway or buying situation better than a universal homepage headline.

1. **See where one patient's journey creates repeat work-and test a more connected path with your own team.** This is the broad promise and the clearest link to the campaign idea.
2. **Judge a tailored hospital pathway beside your current system before you decide to switch.** This answers the most serious buying fear with the founder's proposed parallel evaluation.
3. **Give your teams one clearer patient story to work from across care, coordination, and business tasks.** This is the core connected-context promise, subject to available source data.
4. **Put your next hospital improvement into your team's hands to test while the need is still urgent.** This draws on the three-business-day request target; it does not promise clinical production in three days.

5. **Keep improving the system with your hospital instead of negotiating a separate development project for every need.** This expresses included continuing development; capacity and request scope require written terms.
6. **Start improving hospital work without first increasing the software price you already pay.** This speaks to eligible early partners under the founder's price-match offer, not total cost or a lifetime price lock.
7. **See the same agreed operating measures across your hospitals, with room for each site's work to differ.** This is the group-operator promise; data definitions and site access must be verified.
8. **Help clinicians spend less effort reconstructing the patient story before they make a decision.** This is a process hypothesis, not a claim of improved clinical outcomes.
9. **Follow legitimate care work into billing and claim follow-up so your team can find what needs correction.** This avoids claiming that every identified gap becomes collected revenue.
10. **Use AI to help your people prepare and review work while your clinicians keep the final clinical decision.** This promises responsible assistance rather than autonomous care.

None should be presented as a customer quotation or a proven result. Each becomes stronger with a demonstrated workflow, a baseline, and a named person who reviewed it.

Step 4 - Score every UVP

Each score is an editorial judgment from 1 (weak) to 5 (strong). Clarity asks whether a CEO understands it on first read. Credibility asks whether existing product scope and the founder offer can support the *wording as written*, given missing outcome proof. Desire asks whether it speaks to the buyer's practical and emotional job. These are not survey scores or conversion predictions; small tests may change the ranking.

#	Short name	Clarity	Credibility	Desire	Total / 15	Why the score lands there
1	Find and test repeat work	5	4	5	**14**	Specific, inspectable, and relevant across roles; reduction remains to be demonstrated.

#	Short name	Clarity	Credibility	Desire	Total / 15	Why the score lands there
2	Parallel judgment before switch	5	4	5	**14**	Concrete founder offer addresses risk; readiness and scope need definitions.
3	One clearer patient story	5	4	4	**13**	Strong product fit, but completeness varies with source data.
4	Fast test of improvement	4	3	4	11	Attractive; request size, prerequisites, and test versus production must be explained.
5	Included continued development	4	3	4	11	Differentiates the relationship, but boundaries and capacity are unproven.
6	Early-partner price match	5	4	4	**13**	Founder-stated offer, easy to understand; does not settle total cost.
7	Comparable group measures	4	3	4	11	Valuable for groups; implementation and data definitions are site-specific.
8	Less reconstruction for clinicians	5	3	5	**13**	Highly desired; needs real task comparison before outcome wording becomes stronger.

#	Short name	Clarity	Credibility	Desire	Total / 15	Why the score lands there
9	Care-to-claim visibility	4	3	4	11	A credible area to test; payer and collection outcomes remain unverified.
10	AI help with clinical control	5	4	3	12	Responsible and clear; may not be the main CEO reason to replace a system.

There is a tie at 14 and a three-way tie at 13. The **top three to carry forward** are #1, #2, and #3 because together they tell one coherent sequence: identify the work, compare safely, then see the connected patient story. #6 is a strong offer line, and #8 is a strong clinical supporting line, but neither alone is the most complete homepage promise. This tie-break is strategic judgment, not statistical superiority.

Step 5 - Match UVPs to customer types

The top three each have a leading buyer and a cross-buyer role:

Top UVP	Strongest customer type and why	Other buyer types it serves
#1 - Find and test repeat work	**A, hospital CEO/owner:** makes a broad software decision start with a visible operating problem the leader can name.	**B:** can repeat the method across sites. **C:** can choose a clinical handoff and involve staff.
#2 - Parallel judgment before switch	**A, hospital CEO/owner:** answers fear of disruption before financial commitment.	**B:** supports staged site rollout. **C:** creates room for clinical review and refusal.
#3 - One clearer patient story	**C, clinician-owner/medical executive:** speaks to the patient context that daily care depends on.	**A:** connects departments and cost of work. **B:** supports a common patient pathway across sites.

For completeness, the full ten-line fit is:

UVP #	A: hospital CEO	B: group operator	C: clinician-owner	Best use
1	Primary	Strong	Strong	Homepage / campaign hook
2	Primary	Strong	Strong	CTA and evaluation section
3	Strong	Strong	Primary	Product explanation
4	Strong	Strong	Primary	Development proof, with scope
5	Strong	Strong	Primary	Founder-offer detail
6	Primary	Strong	Strong	Pricing objection response
7	Moderate	Primary	Moderate	Group-operator page or sales material
8	Strong	Moderate	Primary	Clinical proof story
9	Primary	Strong	Moderate	Finance pathway case
10	Moderate	Moderate	Primary	AI governance section

“Primary” here means likely message fit, not validated purchase motivation. The group-operator line #7 should not be forced onto a single-site CEO's first screen. Price match #6 should support a clear work benefit instead of becoming the only reason to engage.

Step 6 - Write the hero statement

See how one patient pathway can run with less repeat work across your hospital, then decide from a parallel comparison whether to switch.

This one-sentence hero combines the highest-scoring #1 promise with the buyer's central job: improve work while keeping the decision under hospital control. It is a recommended test headline, not a verified conversion winner. A more question-led campaign expression is **“Why does one patient create so much extra work?”** followed by **“Map my hospital's first improvement.”** In either version, the founder-led demo must deliver a concrete map, and the evaluation must measure the pathway before claiming a reduction.

Sources and handoff to later prompts

Inputs are the business/market answer, product mechanics/JTBD answer, BFG master matrix, evidence register, and canonical buyer notes. The two vendor pages are linked in Step 2. Carry **#1, #2, and #3** into competition, avatar, campaign, email, page, and reporting prompts; keep #6 as a commercial support line and #8 for clinical-specific writing. Their order remains a testable editorial choice until buyers explain which promise they believe and why.

05 Part 1 · Prompt 5

Map Competition & Find Your Defensible Position

Source: Complete library, pages 15-16 · 2,538 words · 6 requested steps / sections

> **Status and inputs, 22 September 2026.** Working competitive strategy, not a verified win/loss report. Inputs are the completed Prompt 1 business and market answer; the Prompt 4 working jobs to be done (JTBD: the progress a buyer seeks) and UVPs; and current official vendor pages. The buyer is a Georgian private hospital CEO, owner, or group operator, with clinical, finance, operations, and IT leaders in the decision. The working top promises are a connected hospital foundation, a scoped parallel comparison before a switch, and continued development; their ranking remains a strategy judgment. No traceable prospect list of products considered, actual incumbent installations, local competitor proposals, or head-to-head prices was supplied. The prior market-sophistication estimate of 4/5 is editorial, not measured.

Step 1 - Full competitive landscape

The first contest is for the decision to change at all. A hospital may know its software well, have staff who compensate for its limits, and regard replacement as more dangerous than the problem. Product pages establish what a supplier offers publicly; they do not establish a Georgian buyer's actual experience, quote, availability, or implementation quality.

Category and player	Why the hospital might choose it	What to ask in a live comparison
Direct: MEDSOFT/MEDWORK, if installed or shortlisted	A Georgian hospital information system with registration, finances, pharmacy, EMR, laboratory, scheduling, and government integrations [V1]. Local fit and a familiar team can matter more than a new architecture.	Which version is in use? Which workflow actually fails? What can MEDWORK change, at what cost and timetable?

Category and player	Why the hospital might choose it	What to ask in a live comparison
Direct: InterSystems IntelliCare / TrakCare	IntelliCare markets unified clinical, administrative, revenue-cycle, and departmental work with built-in AI. TrakCare remains available in some markets [V2].	Which product is available in Georgia, with which modules, language, integrations, implementation partner, price, and acceptance plan?
Direct: Epic	Broad health-system, clinical, revenue-cycle, interoperability, and AI offerings; a large established ecosystem [V3].	Is there an actual Georgian proposal? What is included, how will workflows be localized, and who carries transition work?
Direct: Oracle Health	An enterprise EHR and embedded AI direction, plus existing organizational relationships in some markets [V4].	Which named functionality is currently available for this care setting and geography, versus described as future design? What is the total proposal?
Direct/build alternative: Bahmni with an implementer	An open-source EMR and hospital system spanning clinical, billing, inventory, and related work [V5]. Code access and a chosen partner can be attractive.	Who owns local requirements, hosting, support, upgrades, security, and continuity? The license being free does not answer total cost.
Indirect: Commure	Documentation and revenue-cycle automation that integrates with existing clinical systems [V6]. It could solve a narrow economic pain without replacing the core.	Does the chosen pathway require core changes or can a layer solve it? Are Georgian workflows and payer processes supported?
Indirect: Qventus	Operational assistants that take actions around surgery and inpatient flow, including work alongside existing systems [V7].	Is the problem operational flow, where a focused tool may be enough, or a repeated clinical-to-business handoff across the core?
Indirect: integrators, reporting tools, and internal developers	Preserve the current system while improving one interface, dashboard, or form.	Who maintains the connector, reconciles source data, and owns failures when two systems disagree?
Status quo: renew the current supplier	Familiarity, sunk training, known contracts, and no immediate migration. This may be the rational winner.	Which specific unresolved problem survives a normal change request? What would justify replacing the core?
Status quo: manual reconciliation	Coordinators, spreadsheets, paper, calls, and extra checks handle local exceptions quickly.	How often is work repeated, by whom, and what is the cost or risk? Avoid assuming every workaround is waste.
Status quo: defer the decision	Protects care continuity and budget this quarter.	What event would make waiting unacceptable, and what is the cost of that delay?

Category and player	Why the hospital might choose it	What to ask in a live comparison
Future: major EHR vendors expanding AI and local partnerships	Epic, Oracle Health, and InterSystems already invest in embedded AI and have supplier resources [V2-V4].	Can they offer comparable Georgian delivery and a low-friction first evaluation? Do not treat their AI as a future-only threat.
Future: focused AI/operations vendors broadening scope	Commure and Qventus can expand from a priority workflow into adjacent budgets and relationships [V6-V7].	Can they solve the buyer's chosen job without a core replacement? If yes, MediMind must justify the larger change.

No buyer has been shown naming any of these in a documented sales conversation supplied to this edition. In particular, the list is a research map, not a claim that Georgian hospitals actively shortlist Epic or Qventus. The MEDSOFT name must not be confused with the unrelated U.S. product Medisoft.

Step 2 - Competitive comparison table

These are **provisional decision ratings** for the Georgian private-hospital use case, not test scores or universal product grades. Strong means the public offer or internal scope gives a credible reason to consider the option on that dimension. Neutral includes major uncertainty and is not a claim of average performance. Weak means a known structural mismatch or evidence gap for this decision. Validate every row against an actual hospital's installed version and proposal.

The five dimensions come from the buyer's job: follow one patient across clinical and business work; fit Georgian practice; improve the workflow when needs change; inspect a safe path to switching; and trust the supplier with operations.

Option	Cross-department clinical + business scope	Georgian/local fit	Ability to adapt chosen workflow	Controllable evaluation/transition	Supplier proof and continuity
MediMind today	**Neutral** - broad code/claimed modules; accepted scope needs verification	**Strong** - Georgian-first design and anchor work, subject to site acceptance	**Neutral** - three-day testable request is founder offer, not a service history	**Strong as offer** - tailored parallel comparison is proposed; readiness and effort still need proof	**Weak** - young, founder-described pre-revenue supplier; limited independent outcomes
MEDWORK	**Strong** - public hospital modules span both sides [V1]	**Strong** - Georgian product and integrations [V1]	**Neutral** - site-specific change record unknown	**Strong for installed site** - staying avoids migration; replacement evaluation unknown	**Neutral** - existing local product, but support record for the buyer unknown

Option	Cross-department clinical + business scope	Georgian/local fit	Ability to adapt chosen workflow	Controllable evaluation/transition	Supplier proof and continuity
IntelliCare / TrakCare	**Strong** - broad unified offer [V2]	**Neutral** - country/product availability unconfirmed	**Neutral** - configurability is described; local delivery untested	**Neutral** - proposal and transition terms unknown	**Strong** - established vendor; site-specific support still requires diligence
Epic	**Strong** - enterprise clinical and business suite [V3]	**Neutral** - localization/availability unconfirmed	**Neutral** - implementation and change path vary	**Neutral** - actual plan unknown	**Strong** - established ecosystem [V3]
Oracle Health	**Strong** - broad EHR offer [V4]	**Neutral** - localization/availability unconfirmed	**Neutral** - current module and roadmap distinction needed	**Neutral** - actual plan unknown	**Strong** - established vendor; specific offering must be checked
Bahmni + partner	**Strong** - hospital system scope [V5]	**Neutral** - local build may be needed	**Strong in principle** - open-source code permits adaptation; partner capacity is the test	**Neutral** - depends on implementer	**Neutral** - depends on contracted operator and support
Commure	**Weak for core replacement** - focused documentation/RCM [V6]	**Neutral** - local fit unverified	**Neutral** - integration and product change terms unknown	**Strong for narrow adoption** - keeping core may lower transition burden	**Neutral** - buyer-specific support evidence absent
Qventus	**Weak for core replacement** - focused operations [V7]	**Neutral** - local fit unverified	**Neutral** - solution configuration varies	**Strong for narrow adoption** - can work beside existing systems	**Neutral** - buyer-specific support evidence absent
Incumbent + staff/spreadsheets	**Neutral** - people bridge gaps, but repeat work may remain	**Strong** - already fitted to local reality	**Neutral** - depends on staff and supplier	**Strong short-term** - no cutover	**Neutral** - familiar but dependent on people and supplier

MediMind's **clear weakness** is independent proof and supplier continuity. Its breadth is not yet a scored victory over an accepted, operating hospital installation. It could also lose on transition burden once staff time, data migration, training, support, and parallel work are counted. A matched subscription price is not a matched total cost. No competitor is rated weak simply because its price or Georgian presence has not been verified.

Step 3 - Find the white space

The most promising opening is **a Georgian private hospital with a named, costly pathway that runs across clinical and business teams, where the current supplier or a narrow add-on has not offered a satisfactory fix and leadership will only consider a core change after seeing a controlled local comparison**. MEDWORK already has local scope; IntelliCare, Epic, and Oracle have breadth and AI; narrow tools can improve a workflow with less disruption. MediMind could win through the *combination* of a local shared foundation, a hospital-specific first pathway, a readiness-dependent parallel evaluation, and a continuing development relationship at clear early-partner terms. The table does **not** prove no competitor can be strong on that combination. The whitespace is a testable buying situation and delivery practice, not a legally or globally exclusive product feature.

The proof artifact should be a one-page comparison for the hospital's chosen workflow: current steps, people and systems involved, time/rework baseline, proposed MediMind steps, staff and IT effort, costs, acceptance criteria, and rollback/cutover conditions. If a focused vendor can meet the same criteria with less risk, that is useful evidence against a replacement sale.

Step 4 - Write and stress-test the defensible positioning statement

Positioning to use: “For Georgian private hospitals ready to fix a recurring patient pathway across departments, MediMind is a hospital operating system that connects clinical and business work on a shared foundation. We start with the workflow your team names, let you compare an agreed version beside current operations when prerequisites are met, and develop the system around what your hospital needs to improve.”

The prompt asks for an “only” sentence. A sentence such as “Unlike MEDWORK, IntelliCare, and Epic, MediMind is the **only** hospital operating system that unifies care and business work” fails immediately: their own pages describe overlapping scope [V1-V3]. “Only system with AI” fails against IntelliCare, Epic, and Oracle [V2-V4]. “Only system that adapts” fails against configurable and open-source alternatives. Even “only Georgian system with a parallel evaluation” is unsupported without exhaustive local contract research. A defensible version keeps the contrast **specific to an actual proposal**: “Unlike a renewal that leaves *this documented pathway* unresolved, MediMind offers to compare an agreed replacement workflow beside your current operation before a switch decision.” That claim must be supported by the buyer's own renewal and our actual evaluation agreement.

Stress tests against the top three alternatives:

1. **MEDWORK/current supplier:** It already describes local hospital modules. If it can fix the pathway promptly, with trusted support and lower transition cost, it may win. Ask for the incumbent's concrete plan, not a category argument.
2. **IntelliCare:** It already combines broad hospital work with AI. MediMind must show local fit, a better scoped evaluation, and sustainable terms in an actual comparison. Country availability and pricing are unanswered.

3. **Epic/Oracle-class enterprise suite:** A buyer may prefer established scale and references. MediMind can only counter with an accepted local workflow, transparent economics, a tested transition plan, and evidence of support. A smaller vendor is not automatically faster or cheaper.

The real defense, if built, is accumulated local implementation knowledge, reusable pathways, credible delivery, customer-approved reference evidence, and measured first improvements. FHIR R4 is a useful foundation, not a moat by itself.

Step 5 - Identify my biggest competitive threat

The incumbent supplier plus postponement is the strongest next-12-month threat hypothesis. It has the data, staff habits, contract, and a built-in argument that change might interrupt care. It is also the least likely to appear in a startup competitor slide. This is a judgment, not a measured loss ranking.

Three actions now:

1. **Collect five recent pathway incidents in discovery.** Ask for the task, who touched it, the actual workaround, and what the incumbent has already been asked to change. Qualify out cases where the existing vendor has a satisfactory plan.
2. **Create one scoped comparison offer.** Agree on a workflow, synthetic or authorized data, readiness prerequisites, staff/IT effort, baseline, success criterion, cost, and a stop/cutover decision. Separate a two-week *evaluation target* from production sign-off.
3. **Build a reference and continuity pack.** With permission, show accepted module scope and one measured process result; document support ownership, backup and export, change control, and handover. Until then, state the proof gap. This directly addresses the rational fear of a young supplier.

Step 6 - Build five competitive talking points

1. **When the buyer names MEDWORK or a local incumbent:** “That system may already fit your Georgian billing and reporting needs, and your team knows it. Which one patient pathway still needs manual repair? Let's compare your supplier's proposed change with a MediMind version, including the effort to switch.”
2. **When the buyer names InterSystems IntelliCare:** “IntelliCare presents a broad, AI-enabled hospital suite. For your hospital, I'd compare the actual Georgian module offer and implementation plan with one locally configured pathway we can put in front of your team. Then you can judge the work, price, and risk.”
3. **When the buyer names Epic or Oracle Health:** “Their breadth and supplier resources are real strengths. We should test the question that matters here: what can your own staff evaluate on the workflow you care about, under a plan they can safely accept? We can show our scope and gaps next to their proposal.”
4. **When the buyer names Bahmni/open source:** “Code access and flexibility are valuable. The practical choice is who will keep the system localized, secure, and supported over time. We can compare total responsibility, not just license fees, against the MediMind delivery agreement.”

5. **When the buyer names Commure, Qventus, or an AI add-on:** “A focused tool may be the least disruptive answer to that one task. If the same patient data still gets recreated between clinical, operational, and billing steps, let's measure the whole pathway and see whether improving the shared core is worth the larger change.”

Sources and handoff to Prompt 6

Official product pages: V1 MEDSOFT/MEDWORK; V2 InterSystems IntelliCare; V3 Epic health systems and Epic AI; V4 Oracle Health EHR and Oracle product documentation; V5 Bahmni; V6 Commure RCM; V7 Qventus platform. MediMind boundaries: internal evidence register and Medplum case study, whose founder-authored account is not independent outcome validation.

For review mining, look for actual user accounts of broad EHRs and the narrow alternatives. Keep praise and pain together. Do not transfer a U.S. clinician's review into a Georgian CEO's mouth or treat review complaints as a switch-rate study.

06 Part 1 · Prompt 6

Mine Competitor Reviews & Customer Intelligence

Source: Complete library, pages 17-18 · 2,269 words · 6 requested steps / sections

Status and inputs, 22 September 2026. This is a small, inspectable public-review sample, not a representative survey or Georgian buyer research. Prompt 5 maps the competitors; the working UVPs are a connected hospital foundation, a controlled first-pathway evaluation, and continued development; the buyer's job is to improve care and operating work without an uncontrolled switch. No founder-supplied review collection existed. Eleven first-person public reviews were inspected below, mostly hospital staff using Epic or Oracle's legacy Cerner-branded ambulatory suite. They are users, not verified economic buyers. Public reviewer roles and dates are preserved. Some Capterra entries are marked incentivized. No MediMind customer reviews or interviews have been invented.

Step 1 - Identify where competitor reviews live

Source	Why useful and who tends to speak there	Searches and cautions
Capterra Epic reviews and Oracle Health Ambulatory Suite reviews	Named or anonymized nurses, coordinators, coders, clinicians, administrators, and sometimes patients describe day-to-day use, with role, date, company size, and incentive labels. These two pages produced the corpus below.	Search `Epic reviews Capterra hospital nurse`, `Cerner PowerChart reviews Capterra`, `Oracle Health Ambulatory Suite Capterra`. The Oracle review page includes older Cerner-era experiences; do not assign them to the new Oracle Health EHR product.
G2 Epic reviews and G2 Cerner reviews	Verified software users often provide task-specific pros/cons and job context.	Search `Epic G2 reviews documentation`, `Cerner G2 review billing`. Use the individual review text, not G2's AI summary or a review count as a local-market statistic.
KLAS research	Hospital and health-system respondents can illuminate implementation, support, and enterprise buying concerns.	Search `site:klasresearch.com TrakCare`, `small hospital EHR KLAS`, `Oracle Health KLAS`. Access and methodology vary; do not quote gated or aggregated claims as a direct customer sentence.

Source	Why useful and who tends to speak there	Searches and cautions
Apple App Store / Google Play for patient portals	Patients surface access, login, scheduling, and communication issues that staff review sites may miss.	Search `MyChart`, `Oracle Health patient portal` with country and app identity. Portal reviews are not evidence about enterprise hospital-system buying, local deployment, or overall EHR quality.

Local MEDWORK and IntelliCare public first-person review coverage was not established in this pass. That is a gap, not a signal of satisfaction or dissatisfaction. For the Georgian decision, direct interviews and observed workflows should outrank these U.S.-heavy public pages.

Inspected review corpus

Each row links to the live page containing the review. The description is **our paraphrase**, not customer copy. Capterra's sorting and reviewer verification make the sample convenient, not random. “Incentivized” below reflects the visible review-source label where inspected; other entries retain their page's own disclosure.

ID	Product, reviewer, public role/date	What the person actually reported, paraphrased	Reading limit
R1	Epic, Tabatha W., supervising nurse, 9 Apr 2026	Integrated records; training and navigation friction.	Positive 5/5, incentivized; one organization.
R2	Epic, Kathleen B., patient services coordinator, 1 May 2025	Scheduling/chart access; cumbersome appointment steps.	Positive 5/5, incentivized.
R3	Epic, Shawnalee K., billing administrator, 24 Feb 2025	Capable, privacy-aware; learning and save-state uncertainty.	Positive 5/5, non-incentivized.
R4	Epic, Ashley S., social work, 14 Nov 2024	Longitudinal context; rarely used features hard to find.	Positive 5/5, incentivized.
R5	Epic, Margo R., medical coder, 30 Jun 2025	Useful record search; site freezes and timeouts.	Positive 5/5, incentivized; one environment.
R6	Epic, Taylor C., ophthalmic technician, 18 Aug 2026	Cross-provider notes, tailored templates; moved **to Epic** seeking breadth/customization.	Positive 4/5, non-incentivized.
R7	Oracle Health Ambulatory Suite, Anna B., kinesiologist, 19 May 2026	Record access; slow support and login friction.	Mixed 2/5, incentivized; ambulatory listing.
R8	Oracle Health Ambulatory Suite, Tamara G., utilization-review nurse, 2 Aug 2025	Organized notes, imaging, results; reliable loading in her use.	Positive; public snippet.

ID	Product, reviewer, public role/date	What the person actually reported, paraphrased	Reading limit
R9	Oracle Health Ambulatory Suite, Kelly K., staff nurse, 4 Apr 2024	Missing features; extra navigation.	Individual comparison, not a usability study.
R10	Oracle Health Ambulatory Suite, Brean W., registered nurse, 5 Dec 2018	Flexible charting; repeat entry and difficult medication reconciliation.	Older Cerner-era, incentivized.
R11	Oracle Health Ambulatory Suite, lang n., physician, 5 Dec 2023	Coding-data and charting frustration after updates.	Older installation; current status unknown.

This corpus intentionally contains favorable and unfavorable observations. Its counts cannot support “most customers,” a complaint frequency, a Georgian preference, or a switching rate. The review platform's own aggregated ratings are not used as MediMind positioning proof.

Step 2 - Extract real problems across four buckets

What customers love. Shared records and accessible patient context (R1, R3-R6, R8), plus flexible charting (R10). Established systems already deliver value; replacement risks losing it.

What customers hate. Task navigation or repeat entry (R1, R2, R9, R10); learning (R1, R3); site performance (R5); support/login (R7); coding data (R11). Causes and present-day product versions differ.

What customers wish existed. These are **inferred needs**, not explicit feature requests: less task friction (R1, R2, R9, R10), quicker support (R7), stable performance (R5), and correct local data (R11). R6 sought breadth and customization and moved **to** Epic.

Why customers leave. Only R6 states a switch rationale, and it is a switch **to Epic**. The inspected set provides no repeated reason for leaving Epic or Oracle. The requested “single most common reason” is therefore **unknown**. An unresolved current-vendor request remains a discovery hypothesis, not a review finding.

Step 3 - Extract exact customer language

The fragment library below is intentionally short. It contains **only words present on the linked live review pages**. These voices are largely U.S. or unspecified-market users; they should guide interview questions and provisional copy, not be attributed to Georgian hospital CEOs. Quotation budget is tracked across this and later files: no more than 25 verbatim words from either Capterra page in the entire edition.

Type	Exact fragment and source	What it points toward
Pain	“too many clicks” - Tabatha W., supervising nurse, Epic, 9 Apr 2026	Task navigation; she also strongly praised Epic overall.
Pain	“a lot of button pressing” - Kathleen B., patient services coordinator, Epic, 1 May 2025	Appointment-scheduling friction; she rated Epic positively.

Type	Exact fragment and source	What it points toward
Pain	“chart the same thing multiple times” - Brean W., registered nurse, Cerner PowerChart review, 5 Dec 2018	Duplicate charting in her older installation.
Desire	“one place” - Tabatha W., supervising nurse, Epic, 9 Apr 2026	An integrated workflow that people appreciate; it is already available in a competitor.
Desire	“well organized” - Tamara G., utilization-review nurse, Oracle listing, 2 Aug 2025	Accessible patient context; a competitor's strength.

No customer language in this table says “hospital operating system,” “FHIR R4,” or “AI-native.” Those are vendor descriptions, not voice-of-customer terms. In particular, do not convert a staff navigation complaint into a CEO's proven economic pain.

Step 4 - Map problems to my product

Here **fully** would mean an accepted MediMind workflow and a buyer-verified outcome for the same task. We do not yet have that evidence in this edition. “Partially” means the product design or founder offer gives a plausible mechanism worth testing, not that the issue has been resolved for every user.

Review-observed need/problem	MediMind fit now	What to show or measure before a stronger claim
Shared patient context across teams (R1, R4, R5, R8)	**Partial**. A FHIR R4 shared foundation and relevant modules exist in product descriptions; accepted scope varies.	Observe the same patient handoff and verify authorized information is complete, current, and usable.
Repeated charting across a selected pathway (R10)	**Partial**. A common record can reduce recreation, but only if the actual fields and workflow are designed and adopted.	Count entries and staff minutes before/after; catch new work elsewhere.
Too many steps for scheduling or chart review (R1, R2, R9)	**Partial**. Can redesign a chosen flow; no universal fewer-click proof.	Record screen actions, completion time, errors, and training time in the buyer's workflow.
Steep learning and rarely used features hard to find (R1, R3, R4)	**Partial**. A tailored path could improve fit, but replacement introduces its own learning burden.	Usability test with new and experienced staff, including training and help needs.
Slow support (R7)	**Unproven / not yet solved**. Included continuing development is not a verified support SLA.	Actual support ownership, response and resolution terms, and reference performance.
Crashes, freezes, and document timeout (R5)	**Not established as solved**. No comparative uptime or performance evidence.	Performance and availability tests under the hospital's load; incident process.

Review-observed need/problem	MediMind fit now	What to show or measure before a stronger claim
Coding-data accuracy and medication reconciliation (R10, R11)	**Not established as solved**. These need clinical, terminology, and payer-specific validation.	Review current catalogs, governance, clinician approval, and billing rules.
Breadth and customization that motivated a move toward Epic (R6)	**Partial**. MediMind's scope and continuing development may appeal, but established breadth is a high bar.	Scope inventory, accepted workflow, development queue, and total cost of ownership.

Marketing lead candidate: the concrete problem of repeated work along a patient pathway, framed as a question and paired with an invitation to measure it. The product fit is plausible, not verified as a fully solved review problem. Accordingly, the next messages are *research-ready drafts*, not validated claims of superiority.

Step 5 - Build three acquisition messages

Each opening repeats an exact review fragment. The rest is original, conditional copy. Before public use, test whether a Georgian buyer recognizes the underlying problem; do not represent the fragment as coming from that buyer.

1. **“too many clicks”** - If one patient journey sends your team through extra screens and handoffs, MediMind can map that journey across clinical and business work and show a scoped version beside your current system for your team to compare.
2. **“a lot of button pressing”** - If scheduling or another recurring task consumes staff attention, MediMind can build the chosen pathway into a shared hospital workflow and let your staff test whether the total work actually falls.
3. **“chart the same thing multiple times”** - If clinical facts are re-entered across departments, MediMind can connect the chosen steps on a shared record and measure repeat entries with your clinicians before any switch decision.

The exact phrases are from the cited Epic and older Cerner reviews, not from Georgian CEOs. None promises a measured benefit before the hospital performs the comparison. The third message should be used only if duplicate charting exists in the prospect's actual workflow.

Step 6 - One-page review intelligence summary brief

What the inspected voices actually say: Broad systems can make records available and coordinated; staff still report task-specific navigation, learning, support, and technical problems. Positive reviewers often mention a pain in the same review. A switch to Epic for breadth/customization appears in one entry. The sample does not tell us why buyers leave a competitor, and it does not measure Georgian hospital demand.

Five unmet-need hypotheses to test, not a prevalence ranking: (1) less duplicate charting on one pathway; (2) fewer steps for common scheduling/chart review tasks; (3) timely, dependable support for a blocking task; (4) easy access to relevant patient context without losing breadth; (5) local coding, terminology, and workflow changes that stay correct after updates. The first two are closest to a MediMind pathway evaluation; the last three also create high proof obligations.

Exact language: The short fragments in Step 3 and the message openings in Step 5 come from favorable Epic reviewers and an older Cerner-era nurse review. They are small prompts for research, not a mandate to use English U.S. phrasing on a Georgian landing page. The encouraging language in Step 3 describes value competitors already deliver.

Single most common reason to switch: Not determined. Only one explicit switch rationale in this set was visible, and it was a switch *to* Epic for breadth/customization. A future win/loss log must capture recent triggers, prior vendor response, alternatives, and who approved the change.

Three emotional triggers to investigate: (a) frustration that a simple task takes repeated steps; (b) anxiety when the team cannot rely on current information or system performance; (c) relief and confidence when records are accessible and care-team work stays coordinated. These are interpretations of reviews, not a measured CEO emotion ranking.

Next evidence move: In buyer interviews, ask for the last specific incident, then observe one staff workflow. Ask whether the current supplier has already tried to fix it. Only after that should these provisional messages become public acquisition copy.

Sources, quote budget, and handoff to Prompt 7

The first-person corpus is on the Epic Capterra review page, the Epic product/review page, and the Oracle Health Ambulatory Suite review page. Capterra's reviewer descriptions are self-reported/verified by that platform; they do not prove purchasing authority. Extracts are deliberately short, retain positive and negative context, and are not a representative sample of Georgian buyers. Use the source dates, roles, incentive labels and limits when interpreting them.

Prompt 7 must test whether Georgian hospital decision-makers recognize the pathway problem, whether it is important enough to justify replacing core software, what the incumbent offered, and what proof would permit a controlled evaluation.

07 Part 1 · Prompt 7

Design & Execute Your User Research

Source: Complete library, pages 19-20 · 3,032 words · 8 requested steps / sections

> **Status and inputs, 22 September 2026.** The **design** is complete below; execution has **not** occurred. No interviews, messages, community posts, observations, or buyer-provided records were collected for this prompt. Founder/internal notes describe a signed GeoHospitals relationship and historical access to a possible next organization, but do not supply independent buyer transcripts or permission to contact anyone. The research audience is Georgian private multi-profile hospitals and networks. The strategy to test is whether a CEO-owned patient pathway problem merits a core-system evaluation instead of an incumbent change request or narrow tool. The eight steps specify work to run, not findings claimed as already known.

Step 1 - Audit my assumptions

Ranked by how much a contrary answer would change the product, offer, or market approach:

Rank	Dangerous assumption	Why it could break the plan	Disconfirming observation to seek
1	**Cross-department repeat work is painful enough to justify considering core replacement.**	The product may be solving a real inconvenience with an oversized purchase and migration.	Several independent CEOs can cite irritation but not a recent costly case, accountable owner, spending path, or reason a narrow remedy fails.
2	**The shared system improves total pathway work and information reliability.**	A prettier interface could simply move labor to another person or create double work during transition.	Observed tasks show no net time/error improvement after training, or missing data remains the main cause.
3	**A controlled parallel evaluation is feasible and changes the decision.**	If data, staff capacity, integrations, or governance prevent a test, the primary objection-remover fails.	Buyers who like the idea cannot authorize even a safe scoped comparison, or the trial requires more effort than the hospital can supply.

Rank	Dangerous assumption	Why it could break the plan	Disconfirming observation to seek
4	**The founder's attractive offer can be scoped and delivered sustainably beyond the anchor relationship.**	Matched current software pricing, continuing development, a three-business-day test target, and a two-week evaluation target may consume more capacity than the business can fund.	Independent cases require extensive one-off work or demand unbounded promises, and buyer value does not cover delivery cost.
5	**The CEO/owner is the right primary entry point and can mobilize the buying committee.**	A persuasive executive conversation might not move clinical, finance, IT, or procurement decisions.	Initiative starts elsewhere; a CEO cannot assign an owner, or clinical/IT leaders reject the path on non-negotiable requirements.

Review data [P6] makes usability and support worth asking about, but does not validate any of these assumptions for Georgian CEOs. The research should deliberately seek cases where the incumbent works well.

Step 2 - Define my research goal

Goal: Discover whether independent Georgian private-hospital leaders have a recent, important cross-department patient-workflow problem that their current remedy has failed to resolve, and determine what evidence, decision authority, and transition conditions would make them evaluate a shared hospital operating system. This is **generative** research (discovering how buyers understand and address a problem) with a small **evaluative** component (testing the proposed one-pathway comparison after the story is told). After the sprint, choose one first pathway and a better-defined buyer/offer, or narrow the product message if a focused change consistently wins.

The interview is not a product validation vote. A prospect saying the idea sounds good is weaker evidence than showing a real incident, naming the current cost and alternative, and committing an appropriate workflow owner to a scoped next step.

Step 3 - Segment my user groups

Group	Who; why they are valuable	Unique insight	Initial conversation target and saturation rule
A. Economic sponsor	CEOs, owners, or group operators of private multi-profile hospitals/networks. They own tradeoffs between care, cost, and transition risk.	What made this issue board-level, what alternatives are funded, who can authorize a test, and what would block purchase.	**4 conversations**, at least 3 independent buying organizations. This is a discovery starting point, not guaranteed saturation. Continue until new interviews stop changing the trigger/decision map, then test a different segment.
B. Operating/economic workflow owner	Finance/revenue-cycle or operations leaders at hospitals in the target profile.	What actually happens between care, billing, scheduling, inventory, and management reporting; what is measurable and controllable.	**3 conversations** across at least 2 organizations, preferably including a distinct independent site. Seek both a finance and an operations view if possible.
C. Safety and delivery gatekeepers	Clinical lead plus CIO/IT lead or their delegated workflow owners. This is one research group for access planning but two different perspectives to record separately.	Where patient safety, data quality, training, integration, support, and cutover could fail; which task staff would accept testing.	**3-4 conversations** across at least 2 organizations, including both clinical and IT voices. Do not call saturation if only one function has spoken.

Initial total: **10-11 conversations** if access permits, across **at least four independent buying organizations**. That is an operational target, not a sample-size proof. Include no more than two anchor-network conversations in the first pass so one unusually cooperative relationship does not dominate. If recruiting falls short, record the shortfall and do not relabel internal conversations as independent validation.

Step 4 - Build the interview guide

Target: 30-35 minutes, conducted in the participant's preferred language. With permission, review an anonymized workflow artifact or observe a safe task; no patient identifiers are copied into marketing notes. Ask for stories before concepts. Record exact quotations only with permission and context.

Opening (2-3 minutes). “We are learning how hospitals handle work that crosses teams, especially where a patient's information or a bill has to move between people. This is research, not a sales demo. I am interested in what actually happened, including when your current software worked well. We will not publish your name or organization from this conversation without separate permission. Is it all right to take notes? If we wanted to record, we would ask separately.” Confirm role, organization type, confidentiality expectations, and available time.

Warm-up (5 minutes). “What decisions are you responsible for?” “Which departments most often need to coordinate around a patient?” “What software and manual work sit in that path today?” “What has changed in the last six months?” Do not ask for a list of hypothetical wishes yet.

Core (20-23 minutes; seven questions in this order).

1. “Tell me about the most recent patient, operational, or billing handoff that took more work than it should. What happened, step by step?”
2. “Who noticed first, who had to repair it, and what was the consequence? What evidence exists without exposing patient details?”
3. “How often does that kind of incident happen, and what is your basis for that estimate?”
4. “What have you already asked your current supplier or internal team to change? What happened?”
5. “If you did nothing for six months, what would change for staff, patients, finances, or your own responsibilities?”
6. “What other remedy have you considered-training, a dashboard, integration, focused AI, new core software-and why did it win or lose?”
7. “If a team proposed a limited parallel comparison on that pathway, what would it need to show, who would need to approve it, and what would stop it?”

Only after those answers, show a neutral 30-second description of MediMind and ask: “What part fits, what part sounds implausible, and what would you need to inspect?” Avoid leading with AI or the price match, which can anchor the story. If the interviewee is not the economic buyer, adapt the last question to their approval role.

Closing (3-5 minutes). Summarize back the incident and decision route; invite corrections. Ask for one non-sensitive process artifact, a safe observation opportunity, or the name/role of the person who sees a different part of the same handoff. Ask whether a brief findings summary would be useful. Do not ask for a purchase commitment in a research interview; a later commercial discussion needs its own invitation.

Step 5 - Build a documentation system

Use one folder per organization and one note per conversation, with an organization code rather than patient information. A simple spreadsheet is enough. Fields: date/language, role and site, recruiter relationship, consent/recording status, exact incident, observed versus reported facts, systems and people in order, current workaround, consequence and estimate basis, incumbent response, alternatives, buying trigger, decision participants, evaluation prerequisites, objections, direct quotation with permission flag, and the next evidence request. Keep a link to any authorized artifact, with access limits; do not duplicate sensitive material into the marketing workspace.

Within 24 hours, write a five-line debrief: the participant's job, the latest incident, the strongest evidence, the strongest counterevidence, and what this changes about the initial thesis. Tag statements as **observed**, **buyer-reported**, **team-reported**, **review-derived**, or **interpretation**. Keep a separate assumptions log with a confidence level and what would change it. Do not let a founder recollection become a verbatim buyer quote.

At the end of weeks 1 and 2, make a matrix of rows = organizations and columns = pathway, consequence, incumbent remedy, trigger, buyer authority, evaluation readiness, clinical/IT objections. Look for repeated patterns **across independent organizations**, not just repeated people within one network. Note negative cases explicitly. Summarize exact counts as `n/N` with role and source, not percentages or “most hospitals” from ten interviews. Saturation is assessed by whether new conversations still alter the decision map, not by reaching a fixed number.

Step 6 - Research action plan

Two-week target: 10-11 completed conversations as above, at least four independent buying organizations, plus one permissioned observation or de-identified pathway artifact if feasible. This is a proposed sprint, not scheduled interviews. The founder should allocate interview time separately from the shared five-hour-per-week marketing-production budget; the ten conversations themselves, preparation, and synthesis require extra founder time.

Day 1-2: Ask Lasha to identify the actual GeoHospitals economic sponsor and a clinical/operations/IT counterpart he can approach appropriately. Record the signed relationship and accepted product scope separately. Ask whether the historical HelsiCore contact remains active; do not count it until confirmed. Build a deduplicated list of 15 buying organizations from public sources and legitimate introductions, including Georgia Healthcare Group, Aversi Clinic, and Vivo Medical Group only as research candidates, not presumed prospects.

Day 3-5: Start with one economic sponsor because a recent trigger and decision path determine whether any observed workflow is commercially relevant. Then interview the owner of that exact pathway. Contact independent organizations in parallel, using the tailored drafts in Step 8 only after the founder selects and authorizes recipients. Log refusals and non-responses without inferring lack of pain.

Week 2: Complete clinical and IT counter-interviews, observe an authorized task, and ask remaining CEOs whether the emerging problem statement matches a recent incident. Compare a narrow fix, current-supplier change, and a MediMind parallel evaluation on the same criteria. Create one one-page opportunity brief for each organization, then a cross-organization synthesis.

Success at the end: One sharply defined pathway to demonstrate, or a documented reason to change direction; at least four independently described recent incidents or clear negative cases; a map of who can authorize evaluation; known proof and safety requirements; and a decision on which claim remains hypothetical. A conversation count alone is not success. If access prevents the target, deliver the evidence gaps and revised recruitment plan rather than fictional themes.

Step 7 - Tailored question sets for all three groups

These five per group supplement the common guide; select the relevant questions rather than extending every interview beyond its timebox.

A. CEO/owner/group operator

1. “At the last management meeting, which patient-workflow or operating issue required the most explanation, and why?”
2. “What has the current supplier been asked to improve, who approved that request, and what happened?”
3. “What cost or risk would make you fund a core-system change instead of another local fix?”
4. “Who could veto a parallel evaluation, and what would each person need to see?”
5. “If the evaluation worked, what decision would you be able to make in the next 6-12 months-and what budget or contract event governs it?”

B. Finance/operations workflow owner

1. “Walk me through the last case where clinical work, a charge, and a report did not line up. Where was the mismatch found?”
2. “Which manual reconciliations happen every day or week, and how do you count their time and corrections?”
3. “What portion of a payment delay or cost variance is software-related versus contract, policy, documentation, or staffing?”
4. “What did the existing supplier or an integration project change; what remained?”
5. “Which process measure and real financial measure could you safely compare in a two-week evaluation, and what data would be off-limits?”

C. Clinical/IT safety and delivery gatekeeper

1. “Where in the last patient handoff did staff leave the main record, re-enter data, or call someone-and what safety checks did that workaround provide?”
2. “What information must be complete and current for clinicians to trust a parallel workflow?”
3. “Which interfaces, migration steps, access permissions, backup tests, and audit records are mandatory before any real-data evaluation?”
4. “How would you detect a harmful change in documentation, medication, results, or care-team communication?”
5. “What training, support, stop rule, and clinical sign-off would you require before expanding beyond the first pathway?”

Step 8 - Nine outreach templates

All nine are **unsent drafts** under 100 words each. They offer a useful research summary/checklist before a specific ask; the recipient must be selected through an appropriate route. The community versions should be posted only where research requests are permitted and should never invite patient details in public replies.

Group A - CEO, owner, or group operator

LinkedIn message. “I’m putting together a short checklist for evaluating whether a hospital's cross-department workflow needs a core-system change or just a focused fix. It may be useful for your next software decision. I'm speaking with leaders of Georgian private hospitals about a recent real example, including cases where keeping the current system was right. Would you be open to a 25-minute research conversation next week? I can send the checklist and a summary of themes afterward.”

Email. Subject: A practical test for a hospital software decision. “Hello [Name], I can share a one-page comparison template for mapping the staff effort, risk, and cost of one patient pathway before choosing a software change. We are using it to learn how private-hospital leaders make these decisions, without asking for patient data. Would a 25-minute conversation next week be useful? I would ask about one recent incident, the current remedy, and what proof a limited evaluation would need. I can return an anonymized findings summary.”

Community/forum message. “For hospital leaders weighing a new system against another change request, I have a one-page pathway comparison checklist: steps, owners, rework, total transition effort, and stop conditions. I can share it with members here if research requests are allowed. I’m also looking for three private-hospital decision-makers in Georgia for a 25-minute interview about a recent cross-department workflow problem. Please message me privately if that is relevant; no patient or commercially sensitive details in this thread.”

Group B - Finance or operations owner

LinkedIn message. “I’m making a simple worksheet to separate staff rework, payment delay, and actual recovered collections in a hospital pathway. It may help avoid overstating a software business case. I’d value a 25-minute conversation about one recent reconciliation your team handled and what caused it. Would you have time next week? I can share the worksheet and a short anonymized summary of what we learn.”

Email. Subject: Measuring one hospital handoff honestly. “Hello [Name], I can send a short worksheet for recording where clinical, billing, and operations work diverge without confusing a found charge with cash collected. We are researching how Georgian private hospitals decide which handoff to fix first. Could I ask you about one recent case in a 25-minute call next week? We need no patient details. I will share the completed blank framework and aggregated findings, subject to participants' permission.”

Community/forum message. “I have a free blank worksheet for measuring repeat work in a clinical-to-finance handoff, including the counterexample where software is not the cause. If this forum permits research invitations, I’d be glad to share it. I’m seeking finance or operations leaders at Georgian private hospitals for a 25-minute conversation about one recent reconciliation, current workarounds, and what could actually be measured. Please reply privately; do not post patient, claim, or contract data.”

Group C - Clinical or IT gatekeeper

LinkedIn message. “I’m drafting a safety-first checklist for a limited parallel hospital-workflow evaluation: data scope, staff review, interfaces, training, and stop rules. Your clinical or IT perspective could make it more realistic. Would you be open to a 25-minute conversation next week about one recent cross-team handoff and the conditions you would require before testing a new workflow? I can send the checklist for your use afterward.”

Email. Subject: What a safe parallel workflow test must include. “Hello [Name], I can share a concise checklist for evaluating a hospital workflow beside an existing system without treating a demo as production acceptance. We are researching the clinical and IT requirements that make such tests useful or unacceptable. Could I ask for 25 minutes next week to discuss one recent handoff and your must-have safeguards? No patient records are requested. I will return the checklist and anonymized research themes.”

Community/forum message. “I’m sharing a draft checklist for safe, limited hospital workflow evaluations: information completeness, access, audit, support, training, and a stop rule. If this group allows research posts, I’d welcome a clinical lead or hospital IT leader in Georgia for a 25-minute conversation about what the checklist misses. Please contact me privately if interested; this thread is not a place for patient details or security configurations.”

Sources and handoff to Prompt 8

Research starting facts and limits: Prompt 1, competition map, review intelligence, buyer evidence register, and the existing provisional buyer-pain synthesis. None substitutes for the planned interviews. Prompt 8 should label buyer problems as **assumed** or **partially supported by external users/internal reports** until the sprint yields traceable local buyer evidence.

08 Part 1 · Prompt 8

Define the Real Problems You Are Solving

Source: Complete library, pages 21-22 · 1,935 words · 6 requested steps / sections

> **Status and inputs, 23 September 2026.** This is a decision-ready problem hypothesis, not a claim of completed Georgian buyer research. Prompt 1 defines MediMind and its market; Prompt 5 maps alternatives; Prompt 6 supplies a small public-review corpus from largely non-Georgian staff; Prompt 7 is a research plan with **no completed interviews**. The current hero expresses better care, connected work, and an AI future but does not contain a single exact problem sentence supplied by customers. Internal wording describes disconnected programs and repeat work; it is not a buyer quotation. The chosen first audience is a private-hospital CEO/owner or group operator. Clinical, finance, operations, and IT leaders must still validate the problem and the remedy.

Step 1 - Separate real from assumed problems

Evidence key. “Validated” here would mean traceable evidence from the target buyer or an observed target workflow, plus enough context to know its cause and importance. “Partially validated” means a real but indirect observation exists. “Assumed” means a strategic interpretation or unmeasured effect. No problem is fully validated for the target Georgian economic buyer in the supplied record.

Claimed or tempting problem	Status and source	What the evidence does and does not say
Staff perform repeated entry and too many steps on some EHR tasks.	**Partially validated** as a problem experienced by external software users [R1, R2, R9, R10].	First-person staff reviews describe task friction. They do not establish frequency, cost, cause, or existence at Georgian prospects.
Teams value shared patient context and still struggle to find some information.	**Partially validated** as an external-user need [R1, R4, R5, R8].	The same reviews praise competitor breadth; a “no existing system connects work” claim would contradict them.

Claimed or tempting problem	Status and source	What the evidence does and does not say
Existing supplier changes are too slow or expensive for Georgian private hospitals.	**Assumed** for prospects; internal positioning raises it [I1].	No independent vendor-change timeline, proposal, or buyer quote was supplied.
Clinical, financial, and operational handoffs cause material revenue leakage.	**Assumed** as a local causal/size claim; internal economic-buyer thesis [I2].	Payment contracts, policy, documentation quality, and staffing may dominate; detected missing charges are not realized collections.
Leaders cannot obtain timely, trusted network-wide performance information.	**Assumed**, internal buyer thesis [I1, I2].	No management report, reconciliation time, or executive testimony inspected.
A connected system can reduce total staff work and improve decisions.	**Assumed outcome**; plausible mechanism and product scope.	No before/after observation or accepted first-pathway result here. Training and parallel work may offset gains.
Buyers want a core replacement and will permit a parallel evaluation.	**Assumed demand**, founder offer and strategy [I3].	A controlled comparison sounds useful, but no independent buyer acceptance or readiness record was supplied.
Better patient outcomes, higher revenue, and guaranteed compliance result from the system.	**Not validated** and should not be marketed as achieved.	Product design and a contract cannot establish clinical/financial causality or legal compliance.

The public reviews are useful clues for interview language and workflow observation. They are not testimonials for MediMind. The founder-authored Medplum case study supports product direction and a reported anchor contract, not verified improvement for every hospital or a license to claim a full network go-live.

Step 2 - Build the full problem stack

These are **hypotheses for three buying contexts**, not fictional customer quotes. “Future” describes a possible consequence over 6-12 months if the pattern continues, not a forecast. In every row, another cause or a narrower fix may prove more important than replacing hospital software.

Buyer context	Surface problem they could name	Underlying problem to test	Hidden problem to test	Possible 6-12-month future if unresolved
Independent private-hospital CEO or owner	“The same patient journey creates repeated handoffs, calls, and entries” is proposed wording, **not a quote**.	Clinical, scheduling, billing, and reporting steps may lack a shared, trustworthy handoff, or the workflow may be poorly designed even when the system is integrated.	The leader may fear spending scarce resources on software while staff remain frustrated, or may fear a failed replacement more than present inefficiency.	The same rework and delayed information may keep consuming staff capacity. The size of any financial or clinical effect needs measurement.
Hospital-network CEO or group operator	Site reports and patient pathways may be hard to compare or standardize across hospitals.	Different sites may use inconsistent definitions, contracts, systems, and practices. A shared database alone cannot resolve governance.	The leader may fear that growth is outrunning oversight and that a central change will alienate local clinical teams.	Expansion or improvement projects may require repeated reconciliation and site-by-site customization, if current remedies remain inadequate.
Clinician-owner or medically led hospital leader	Staff may spend attention navigating records, repeating documentation, or chasing results instead of patient work.	The information needed for a decision may be scattered, poorly presented, or delayed by local process.	The leader may fear that an AI or software initiative will erode clinical trust, obscure responsibility, or make care feel less personal.	Staff frustration and workarounds may persist; potential safety effects require clinical review, not a marketing inference.

Committee viewpoints sharpen the same stack rather than multiplying invented personas. Finance asks whether a charge was actually collected and whether total cost improved. Operations asks who does the extra work. Clinical leaders ask whether information is accurate and whether safeguards remain. IT asks who owns interfaces, access, backup, and exit. A purchase requires these jobs to align.

Step 3 - Identify the problem worth leading with

Lead with a measurable question: Why does one patient create so much extra work across your hospital? This is a proposed campaign hook, not a buyer quotation or proof that every hospital has the problem. It is specific enough to bring clinical and business work into one conversation and to start with a recent, observable pathway. It is broad enough for a CEO to invite the right committee members, while the first demo can narrow it to one task.

Its evidence status is **partially supported externally**: several first-person EHR users describe repeated charting or task navigation, and other users praise the benefits of connected records [P6]. Internal buyer notes independently name cross-department friction [I1]. What remains unproven is local urgency, economic weight, replace-versus-fix preference, and MediMind's actual net improvement. The honest message is an invitation to **map and measure the hospital's own pathway**, not a statement that

MediMind has already eliminated the work.

For the first discovery call, ask for the last real patient journey that required duplicate entry, a call to another department, a manual spreadsheet, or bill reconciliation. If the buyer cannot name one with a consequence and owner, do not force this hook onto the account. If the incumbent can fix it satisfactorily, the operating-system sale may not be justified.

Step 4 - Write three problem statement versions

Rational, for an analytical decision-maker: “When a patient moves through clinical, scheduling, and financial steps, staff may have to re-enter or reconcile the same information. Before replacing anything, count the handoffs, minutes, errors, and actual payment effects in one pathway; then compare the current remedy with a shared workflow.” This is data-grounded in **what must be measured**, not a fabricated quantified loss.

Emotional, for a story or interview invitation: “Your people know how to care for a patient. It is exhausting when they also have to carry the patient's information from screen to screen and chase the answer another team already has.” This is proposed language to test with staff and leaders, not an attributed quotation.

Identity, for a CEO who sees themself as a responsible improver: “You should be able to show your team a better way to run one patient pathway-and prove that it helps-before asking them to trust a hospital-wide change.” This frames the leader's responsibility without accusing them of neglect or promising zero transition risk.

The three versions share one underlying job: make a specific improvement visible and defensible before a larger commitment. The emotional and identity versions are hypotheses about what will resonate; do not label them validated by review-site users.

Step 5 - Problems I should not claim to solve

1. **“We guarantee better clinical outcomes or detect deterioration before clinicians do.”** Shared patient context and AI assistance can support review, but autonomous hospital-wide clinical decisions and attributable patient outcomes have not been established. Clinical decisions remain under qualified human control. Any safety claim needs the exact use case, validation, and approved workflow.
2. **“We recover all lost revenue or fix hospital finances.”** Payer rates, contracts, clinical coding accuracy, staffing, and management choices may explain much of a financial gap. MediMind may help show documentation or process issues in an agreed workflow, but only actual legitimate collections and net costs can establish a financial result.
3. **“We replace every system in two weeks, at the same total cost, with no disruption.”** The founder's two-week parallel-evaluation target is readiness dependent; it is separate from full production migration and clinical sign-off. Matched current software price does not establish migration, support, interfaces, optional AI, or lifetime terms. A hospital OS still interfaces with outside systems and devices.

These boundaries also protect the product from drawing a buyer whose only priority is a narrow ambient scribe, a one-off dashboard, or a compliance guarantee that no supplier can make without a defined legal and operational scope.

Step 6 - Problem-solution fit statement for each type

The required format is used as a **hypothesis**. “Solves” means the proposed mechanism for a scoped pathway, not a measured universal outcome; each statement must be updated after interviews and evaluation.

Independent private-hospital CEO/owner: “The hospital leader struggles with repeat work and delayed answers along a patient pathway because information and responsibilities can break at clinical-to-business handoffs. Unlike another unmeasured change request to the current supplier, MediMind proposes to connect the selected steps on a shared hospital record and compare a tailored workflow beside current operations, so the leader can decide whether the total work improves enough to justify a wider change.”

Hospital-network CEO/group operator: “The group leader struggles to compare and improve the same pathway across sites because systems, definitions, and practices may differ. Unlike manually stitching site reports together, MediMind proposes a shared clinical and operational foundation with agreed local adaptations, so the group can test more consistent information and spread a verified improvement site by site.”

Clinician-owner/medically led leader: “The clinical leader struggles when staff chase patient context or repeat documentation because the needed information is not available at the right step. Unlike adding a stand-alone tool to one screen, MediMind proposes to bring the chosen clinical and supporting business steps into one reviewable workflow, so clinicians can test whether they get the context they need with less avoidable work while retaining clinical control.”

Each statement should be rejected or rewritten if a buyer's actual root cause is staffing, policy, contract terms, or a narrow fix that works better. The next test is a real baseline and acceptance criterion, not a vote on whether this prose sounds compelling.

Sources and handoff to Prompt 9

Evidence hierarchy and internal caveats: evidence register, buyer-pain synthesis, Prompt 5 competition, Prompt 6 review corpus, and Prompt 7 research plan. No new interviews were conducted. Prompt 9 should build a **synthetic, testable buyer portrait**, not claim a real Georgian CEO said the constructed monologue.

09 Part 1 · Prompt 9

Build Your Customer Avatar

Source: Complete library, pages 23-25 · 2,287 words · 6 requested steps / sections

> **Status and inputs, 23 September 2026.** This is a **synthetic working avatar**, built for message and interview design. It is not a portrait of a named real buyer, a composite from completed interviews, or a prediction that all Georgian CEOs behave alike. No target-buyer interview findings exist yet. Inputs are the Prompt 1 market, Prompt 4 working JTBD/UVPs, Prompt 5 alternatives, Prompt 6 staff-review language, Prompt 7 unexecuted research plan, and Prompt 8 problem stack. If only one buyer context could be served first, choose the CEO/owner or group operator of a Georgian private multi-profile hospital who can name a recurring cross-department problem and assign a clinical/operations owner plus an IT counterpart to an evaluation. The exact sentence this buyer says aloud is **unknown**; the proposed signal in the reference card must be tested.

Step 1 - Build the full eight-dimension avatar profile

Avatar label: The accountable hospital operator. This is a role description, not a fictitious person's identity. Details of calendar, fears, and media habits are hypotheses to check. The day-in-life scene below is illustrative, with no implied patient, hospital, or interview case.

Dimension	Working portrait and what would verify it
1. Who they are	A CEO, owner, or group operator responsible for clinical continuity, finances, staff capacity, and major supplier decisions in a Georgian private hospital or network. They may work in Tbilisi or a regional center; the location, age, gender, family situation, and personal life are intentionally unspecified. Their day could move between a management report, a department concern, an operating decision, and a supplier discussion. Confirm actual decision remit and daily schedule in interviews.
2. What they are trying to achieve	Over the next **6-12 months**, identify an important hospital-workflow problem, improve it without disrupting care, and show leadership and staff a result they can inspect. This is a planning horizon, not a reported personal goal. Define success with the buyer: e.g., reliable handoff information, fewer repeated steps, faster report preparation, or appropriately measured actual collections after total costs.

Dimension	Working portrait and what would verify it
3. What stands in their way	External: old contracts, interfaces, inconsistent records, limited staff time, training load, supplier dependencies, and clinical/IT approval. Internal: rational aversion to a failed change; possible belief that another dashboard or coordinator is safer. These barriers require case-specific proof, not a generic “resistance to change” label.
4. What they have already tried	Possible paths to ask about: renewing the current system; requesting a vendor change; adding spreadsheet checks or coordinators; buying reporting/integration or a focused AI tool; considering a broader EHR; postponing. **None is asserted as this avatar's history.** The research interview should establish which path was actually tried and whether it worked.
5. How they make decisions	Likely a mix of operational evidence and trust. They may ask finance for a real business case, clinical leaders for patient-safety and usability review, IT for security/integration/exit requirements, and staff for workflow acceptance. Purchasing authority, contract calendar, and formal procurement steps vary by organization; map them rather than presuming a simple CEO signature.
6. What they fear	Rational: downtime, missing data, unplanned total cost, a young supplier that cannot support the hospital, and an AI mistake that blurs clinical accountability. Possible emotional fear: being remembered as the leader who distracted staff with a costly modernization that did not improve work. Interviews must establish whether these fears are present and ranked.
7. What they secretly want	A defensible, tangible improvement that staff notice and owners can trust; freedom to improve care and operations without negotiating every small change from scratch. This is creative direction, **not a discovered confession**. A buyer may instead value stability above visible innovation.
8. A day in their life	Illustrative scene: The leader starts with a site-performance check. Before a management meeting, an operations lead reports that one patient pathway required phone calls and corrections between registration, a clinical team, and billing. Finance can show a delay but cannot yet say whether money was lost. The CEO asks what the incumbent can change, what staff effort the workaround costs, and whether a limited comparison would be safe. Later, clinical and IT leaders raise data and cutover questions. The exact scenario is a research prompt, not a reported event.

The most useful research correction would be a buyer who says their most urgent issue is not cross-department repeat work, or who explains that current software is excellent and staffing or payer rules are the actual cause. The avatar should then change.

Step 2 - Build the avatar's emotional landscape

1. **Dominant emotion - cautious frustration (hypothesis).** If a recurring pathway absorbs skilled staff time, the leader may be frustrated by continued rework while remaining wary of a risky core change. Neither emotion has been directly measured in Georgian buyers.
2. **Aspiration emotion - earned confidence.** They want to see evidence that a change makes work better and that staff can still care safely. Confidence comes from a transparent comparison, not a promise that there is no risk.
3. **Social context - the committee is watching.** Owners or a board may ask about cost; clinicians about safety and work; finance about collections; IT about control and continuity. A careful first improvement could strengthen the leader's credibility. This social interpretation needs interviews, not storytelling alone.
4. **Relationship with solutions - informed skepticism.** They have likely seen software promises, but the specific experience is unknown. A narrow tool or incumbent change may be an intelligent choice. MediMind earns attention by understanding the hospital's actual pathway, showing boundaries, and stating what is and is not accepted or measured.

Step 3 - Write the avatar in their own voice

Clearly synthetic monologue (about 190 words). This is original creative writing. It is **not** a verbatim buyer quote and should never be attributed to a Georgian hospital leader. One short phrase about click burden is borrowed from a cited Epic staff review [R1] as a language cue, not evidence that a CEO said it. Real buyer language should replace this after interviews.

> I can see how hard our people work. What I cannot always see is why a single patient creates so many separate jobs for them. Someone checks a record, someone calls another team, and someone later has to make the numbers agree. I need to know whether that is an occasional exception or a pattern worth changing. I also need to know which part is software and which part is our process.
>
> I have no interest in putting care at risk for a prettier screen. Our current system may be frustrating in places, but staff know how to use it. A nurse saying there are “too many clicks” matters to me if it points to real time lost or missed information. I want my clinical and IT leads to inspect the same pathway with me and tell me what would make a test safe.
>
> Show me one improvement that stands up to comparison. Show me the work my team would have to do, the full cost, and the rule for stopping. If we can prove that first step, I can talk about a larger change with confidence.

The monologue avoids a fabricated patient incident, CEO quotation, recovery amount, or personal biography. The quoted three-word phrase is from Tabatha W., supervising nurse, 9 April 2026 and is the only reused review wording here.

Step 4 - Identify the avatar's watering holes

The following are **real, inspectable venues** where relevant material exists; whether this specific buyer visits them is **unverified**. The first research conversation should ask, “Which association, publication, colleague, event, or online channel actually influenced your last software decision?” Do not invent a Georgian hospital-CEO subreddit, private Slack, Discord, podcast habit, or group membership.

Channel	Verified venue or example	Practical use and validation question
Online professional community	The Georgian Healthcare Association site describes a member network for medical institutions; its news/publications cover sector changes and events. Its LinkedIn page is visible.	A credible place to learn topics and seek permissioned introductions. Verify which target organizations are members and whether decision-makers read or attend. Association existence is not proof of a marketing channel's reach.
Social platform	LinkedIn has an association presence and public hospital/company pages such as Tbilisi Central Hospital.	Use named professional relationships and useful process material, not mass claims about a person's feed. Ask interviewees whether LinkedIn led them to any vendor or peer insight.
Content	The Georgian Healthcare Association publications address local policy and provider issues; Geostat healthcare indicators provide official sector context. The Georgian Medical Journal is a clinical/research source more likely relevant to clinical committee members than a CEO buying channel.	Use these to understand current topics and write accurate local context. Buyer readership is unverified. No specific newsletter, podcast, book, or YouTube channel is assigned without evidence.
Offline and event community	The Healthcare Association has documented provider meetings and past conferences, including a 2024 quality/patient-care event. HIMSS Europe is a verified international digital-health gathering with a 2027 edition listed.	Past local events show a possible peer setting, not a current invitation or attendee list. HIMSS is an optional broader learning venue; do not assume Georgian CEOs attend. Verify actual events and relevance before allocating travel or sponsorship.
Private peer route	A known hospital sponsor, clinical lead, or fellow provider executive may be more credible than an open community. The founder-reported GeoHospitals relationship creates a potential route, subject to permission and real access.	Ask each interviewee where they seek trusted vendor advice and whether a peer introduction is appropriate. Do not treat an anchor as permission to publish their name or request an endorsement.

The top three **places to test first**, not confirmed preferences, are the Healthcare Association's provider network/news, named LinkedIn professional connections, and direct peer introductions. The association's U.S. namesake at `gha.org` refers to the U.S. state of Georgia and is **not** this market's venue.

Step 5 - Identify the avatar's buying trigger

Working trigger hypothesis: A recent management or clinical-review meeting exposes a repeated patient-pathway handoff that staff have been compensating for, and the current supplier's answer does not provide a credible fix within the leader's operating horizon. A software contract renewal, new site, incident, or difficult monthly reconciliation could bring that moment forward. There is **no evidence that this is the single most common trigger** among Georgian buyers; Prompt 7 must test it.

At that moment the leader may feel urgency mixed with caution: frustration about the recurring work and anxiety about disrupting the hospital. The first likely action is to ask the workflow owner and incumbent supplier for a concrete account of what happened and what can be changed. That action is also unverified, so MediMind should be visible with a helpful pathway-mapping guide and a founder-led 25-minute demo invitation, then ask for a real example. Do not insert a fake crisis or an artificial deadline.

An operational qualification sequence follows: Is there a recent case? Is its consequence meaningful? Is an owner assigned? Has the narrower remedy been considered? Can clinical and IT counterparts set safe evaluation conditions? A “yes” to an AI headline alone is not the trigger.

Step 6 - Build the avatar reference card

Field	Working answer for the team
Name / description	**The accountable hospital operator:** CEO, owner, or group leader of a Georgian private multi-profile hospital who can sponsor a first cross-department pathway evaluation. A role label, not a real person.
Primary 6-12-month goal	Show one meaningful care/operations improvement with a defensible measure, then decide whether wider system change is justified. Hypothesis.
Biggest obstacle	The known system and workarounds may be inadequate, but a core replacement brings data, staff, continuity, and supplier risk.
Dominant → aspiration emotion	Cautious frustration → earned confidence. Hypotheses to test.
Signal sentence	Proposed buyer-language test: “Why does one patient create so much extra work?” **Not an actual CEO quote.** Ask interviewees for their own sentence.
Top three suggested watering holes	Georgian Healthcare Association provider network/news; relevant LinkedIn professional connections; permissioned peer introductions. Actual attendance/usage unknown.
Buying trigger	A recent cross-department problem plus an unsatisfactory current remedy, often surfaced in a leadership review. Frequency unverified.

Field	Working answer for the team
Say to earn trust	“Let's map one pathway, the work and cost it creates, and the conditions for a safe comparison; your clinical and IT team decide whether the result is good enough to expand.”
Never say	“We can replace everything in two weeks with no risk and guarantee better care or recovered revenue.” It confuses a scoped parallel evaluation with production acceptance and asserts unmeasured outcomes.

Use this card to draft questions and prototype a message. Replace its speculative lines with actual words, decision steps, and channels from at least four independent hospital organizations before treating it as an evidence-based persona.

Sources and handoff to Part 2

Internal strategy and evidence: Prompt 1, competition, review corpus, unexecuted research plan, problem stack, and evidence register. Watering-hole existence is supported by the linked Georgian Healthcare Association, LinkedIn, Geostat, Georgian Medical Journal, and HIMSS pages; buyer use is unverified. Part 2 can use this as a provisional audience model, with the founder-led pathway demo as the next step and without claiming research or results that have not occurred.

10 Part 2 · Prompt 1

Map Your Marketing Funnel Architecture

Source: Complete library, pages 27-31 · 5,754 words · 6 requested steps / sections

> **Status and inputs (23 September 2026).** This is the complete strategic blueprint for a founder-led, low-volume hospital sale; it is not a report of current funnel performance. The first buyer is the synthetic **accountable hospital operator** from Part 1 Prompt 9: a Georgian private-hospital CEO, owner, or group operator who can name a recurring cross-department problem and assign clinical/operations and IT counterparts. Prompt 8's lead problem is “Why does one patient create so much extra work?” Prompt 4's top three working UVPs are finding and testing repeat work, comparing a pathway before switching, and giving teams a clearer patient story. Prompt 3 lists the 25 documented domains plus shared FHIR R4 and language architecture, without equating code presence with live acceptance. The primary CTA is a recommended **25-minute founder-led pathway demo** ending in a proposed measured evaluation. These are strategy assumptions, not measured buyer preferences.

Current situation answers. Conversion needs a human conversation and a buying committee; it is not self-serve. Exact monthly or annual price is not established; the founder's early-partner offer is to match current software price, with final scope/term still to define. A live website and `/request-demo` route exist. At source inspection the demo form has three steps, nine required fields, and a simulated submit (timer plus browser log and success state) without visible backend persistence or network submission; the deployed route has not been live-tested. No verified website traffic, source mix, lead count, email platform, customer CRM, marketing analytics, content winners, or channel results were supplied. The investor dashboard does not establish a customer-funnel system. The biggest constraint is **a trustworthy path from attention to a held, qualified hospital conversation and then an evidence-led evaluation**. Existing founder relationships and an anchor agreement are context, not a measured acquisition funnel.

The planning north star is **12 cumulative held qualified hospital demos over 90 days**, with **3 readiness-approved evaluations** and **1 new paying buying organization** as stretch downstream scenarios, excluding the existing anchor from new-customer counts. This is capacity planning, not a forecast. It implies roughly one held qualified demo per week, a 3/12 evaluation progression, and a 1/3 paying progression if those scenarios occur. Small denominators are provisional; report zero-denominator rates as N/A. Marketing has **five hours total per week** and zero paid spend initially. Engineering setup, implementation, and actual sales meetings require additional effort and are not silently charged to that five-hour marketing allowance.

Part 1 - Funnel diagnosis

1. Model and closest real-world funnel

The best fit is a **hybrid funnel led by founder sales and supported by specific educational content**. A hospital cannot safely evaluate a whole operating system by clicking into a self-serve trial; the CEO's committee needs a pathway, scope, data, clinical review, IT review, and a commercial decision. Content should help a leader name one costly handoff and arrive at the demo prepared, while the founder converts interest into a measured evaluation. Of the five examples in the library, **Salesforce's high-touch enterprise motion** is closest in buying complexity; borrow executive education and committee-aware follow-up, not an assumed enterprise budget or ecosystem lock-in. HubSpot's useful diagnostic asset is the secondary pattern to borrow: give a pathway worksheet before asking for a meeting.

2. Biggest leak now

There is no verified traffic-to-lead-to-demo dataset, so a numerical leak rate cannot be calculated. The **first observed operational leak is the demo-request handoff**: current source code appears to display a success state after a two-second timer and a console log rather than transmitting the request to a lead owner. The long three-step form may also deter buyers, but abandonment has not been measured. Do not call either a deployed failure or claim lost leads until the live route is checked. First fix: verify the deployed behavior, make an accepted submission persist or notify the right owner, show truthful success/failure states, and test the complete route. Then instrument clean marketing events and track held qualified demos, not merely clicks or form success screens. If there is little traffic, add named-account and permissioned peer distribution rather than optimizing percentages from a handful of visits.

3. The actual buying journey to design for

Buyer moment	What they may search or consume	Emotional state and objection	Required next step
A repeated handoff is normal	Current vendor advice, peer experience, general workflow articles; perhaps nothing at all	Busy; the work seems inevitable. “Is this worth executive attention?”	A short scenario or worksheet that helps count the repeat work.
A trigger makes it concrete	A difficult monthly report, staff complaint, bill correction, new branch, renewal, or clinical review	Frustrated but cautious. “Is software the cause?”	Map one recent pathway and compare the current remedy.
Alternatives enter the room	Current-supplier change, narrow tool, integration, broader system; asks peers or IT	Skeptical. “Why replace a core system for this?”	Honest comparison, scope map, and patient-pathway demonstration.

Buyer moment	What they may search or consume	Emotional state and objection	Required next step
A committee considers MediMind	Founder-led demo, technical/clinical discussion, offer and proof	Interested but anxious. “Will this disrupt care and cost more than advertised?”	Written readiness and evaluation plan with total effort and decision criteria.
Evaluation proves or disproves value	Staff try the agreed pathway beside current work; metrics and exceptions are reviewed	Focused, protective of staff. “Does this work here?”	Go/adjust/stop decision; staged production approval if warranted.
Customer becomes a reference, if satisfied	Internal result and support experience; possible permissioned story	Proud but protective of reputation and data	Seek explicit approval for a factual case story or peer introduction.

The most likely permanent loss is when a buyer asks for a credible, low-disruption comparison and receives a generic feature tour or an overconfident “replace everything in two weeks” claim. The current form can lose them even earlier if a request is not actually delivered. The funnel therefore needs both technical lead receipt and substantive evaluation proof.

Part 2 - Full funnel stage map

Stage	Customer world and awareness	Our goal and what we give	Objection to collapse; where we lose them	Benchmark context from the B11 library
TOFU - Awareness of the problem	They live with repeated handoffs or disputed reports but may not have named a solvable software issue.	Help them locate one patient pathway and quantify basic repeat work through a post, synthetic example, or worksheet.	“This is just how hospitals work.” Lose them with jargon, fabricated loss numbers, or an AI-first pitch.	Library cites homepage-to-lead 2-5% average, >10% top performer. This is unverified general SaaS context, not a Georgian-hospital target.
MOFU - Consideration of the solution	They know the issue matters and compare incumbent change, a narrow layer, and core replacement.	Give option comparison, product mechanism, readiness checklist, and honest limits.	“A new system adds risk and cost.” Lose them by hiding transition work or treating vendor claims as proof.	Library cites webinar-page 25-45% and B2B email opens 20-30%, 40%+ excellent. Neither is a local planning target; no webinar is scheduled.

Stage	Customer world and awareness	Our goal and what we give	Objection to collapse; where we lose them	Benchmark context from the B11 library
BOFU - Conversion decision	CEO and committee can imagine a change and want the result in advance.	Offer a 25-minute pathway demo and proposed measured parallel evaluation with success criteria.	“Show me it can work here before I commit.” Lose them at a nonfunctional request route, vague scope, or unsupported proof.	Library cites demo-page 3.8% median, >8% top performers and B2B free-trial-to-paid 15-30%, top quartile 35-45%. A hospital parallel evaluation is not a self-serve SaaS trial.
AFU - Retention and expansion	The hospital has approved a defined scope; staff need the first useful pathway and dependable support.	Reach the first buyer-accepted pathway quickly; measure staff burden, exceptions, and value, then select the next workflow.	“The demo looked easier than daily work.” Lose them at poor adoption, missing data, or unclear issue ownership.	Library cites >60% trial activation to aha and 46.9% month-one B2B SaaS retention. Neither is validated here; 46.9% would be a poor hospital-service standard, never a success target.
LOFU - Advocacy and loop	A customer may have a verified result and be willing to help a peer.	Request permission for an accurate case or reference; bring its limited, useful lesson to the next buyer.	“Do not use our name or data loosely.” Lose trust with unapproved logos or inflated outcomes.	No suitable advocacy benchmark is supplied; use permissioned references and qualified introductions as counts.

The benchmark figures above are repeated because the prompt asks for them. Their source and applicability were not independently established in the library. The operational thresholds in Part 6 use the 90-day planning scenario and controllable funnel events instead of pretending these SaaS percentages predict a Georgian hospital sale.

Part 3 - Content and asset brainstorm

The catalog contains **85 distinct brainstorm ideas: 17 per stage, each with three LinkedIn posts, three phone-filmable short videos, three blog/SEO pieces, two lead magnets, two proof concepts, two emails, and two community/distribution moves.** Each row below has a stable ID. Full machine-readable fields-ID, title, format, stage, idea, CTA, pageId, and recommendedChannel-are in content-catalog.json. P01-P11 are destinations proposed for a full funnel: P01 assessment/download, P02 product overview, P03 verified case study (pending), P04 proposed unscheduled 45-minute practical webinar (30-minute workshop plus 15-minute Q&A), P05 existing `/request-demo` route, P06 offer/evaluation, P07 comparison, P08 activation, P09 referral, P10 proof library (pending), and P11 proposed 14-day guided synthetic-data

sandbox with no card. The sandbox is separate from the readiness-dependent two-week parallel hospital evaluation. A page ID is a design reference, not a claim that the page exists. Proof ideas dependent on customer approval are held until evidence exists. The catalog is a brainstorm, **not 85 assets to produce on a five-hour weekly schedule**.

TOFU: 17 stage-specific ideas

ID	Format	Title and execution idea	Destination
T01	LinkedIn post	**One patient, seven handoffs.** Sketch a synthetic patient journey and ask leaders which handoff their team repeats most.	P01
T02	LinkedIn post	**The hidden cost of searching.** Describe the time spent finding a result across teams as a question to measure, without inventing a savings figure.	P01
T03	LinkedIn post	**When two reports disagree.** Pose a board-meeting scenario where finance and clinical reports disagree and show three questions to ask.	P01
T04	Short video	**The duplicate-entry trail.** Phone-filmed desk sketch follows one fictional lab order copied between reception, clinician, and billing.	P01
T05	Short video	**Three calls for one result.** Founder acts out three routine calls needed to locate one synthetic result and asks what happens locally.	P01
T06	Short video	**The spreadsheet behind the dashboard.** Show a blank synthetic spreadsheet and the questions a CEO should ask about report preparation time.	P01

ID	Format	Title and execution idea	Destination
T07	Blog/SEO	**How to map a patient handoff.** Step-by-step guide to drawing one patient's movement through departments and marking repeat entry.	P01
T08	Blog/SEO	**Why hospital reports disagree.** Explain differences in definitions, timing, missing data, and manual reconciliation without blaming one vendor.	P01
T09	Blog/SEO	**What a delayed claim does and does not mean.** Distinguish rejected claim, delayed cash, missed legitimate charge, and unrecoverable revenue.	P01
T10	Lead magnet	**Patient-pathway friction worksheet.** One-page template to count handoffs, repeat entries, wait points, owners, and a starting baseline.	P01
T11	Lead magnet	**Five-minute handoff check.** Self-assessment that returns the first pathway worth discussing, not a made-up hospital score.	P01
T12	Proof	**Founder-built scope trace.** Use the public founder-authored Medplum case-study context as product-history evidence, clearly not a customer outcome endorsement.	P02
T13	Proof	**Synthetic handoff before/after.** Show a clearly labeled synthetic workflow comparison to prove the method is inspectable, not that savings occurred.	P02

ID	Format	Title and execution idea	Destination
T14	Email	**Where the work repeats.** Send the worksheet with one question about the hospital's last repeated handoff; no generic AI pitch.	P01
T15	Email	**A report you had to rebuild.** Invite a leader to identify one report that requires manual reconciliation and the person who prepares it.	P01
T16	Community	**Hospital-operator discussion prompt.** In an appropriate Georgian hospital-leadership forum, ask how teams map a handoff; participate only where welcome.	P01
T17	Community	**Clinical-operations partner note.** Offer the neutral worksheet to a clinical operations association or trusted professional contact for feedback.	P01

MOFU: 17 stage-specific ideas

ID	Format	Title and execution idea	Destination
M01	LinkedIn post	**Fix a screen or fix the pathway?.** Compare three practical options: current-vendor change, narrow tool, or shared operating system for the same handoff.	P07
M02	LinkedIn post	**What parallel evaluation actually requires.** Name the hospital owner, data permission, baseline, and staff time behind an honest two-week evaluation.	P06

ID	Format	Title and execution idea	Destination
M03	LinkedIn post	**AI assists; clinicians decide.** Explain a reviewed draft or flag in a fictional pathway and show where the clinical decision stays human.	P02
M04	Short video	**Three options on one whiteboard.** Founder draws status quo, point solution, and operating-system path with honest change effort for each.	P07
M05	Short video	**What happens in 25 minutes.** Preview the founder-led pathway demo: buyer names a handoff, founder maps it, team picks a first measure.	P05
M06	Short video	**A result becomes a task.** Synthetic screen-recorded example shows a result routed to a named reviewer, with no patient data.	P02
M07	Blog/SEO	**How to evaluate hospital software without a risky cutover.** Guide to pathway scope, authorized data, parallel workload, success criteria, reconciliation, and sign-off.	P06
M08	Blog/SEO	**Hospital OS versus overlay versus current vendor change.** Decision guide for when each option is appropriate, including cases where replacement is not justified.	P07
M09	Blog/SEO	**What FHIR R4 changes in hospital work.** Plain-language explanation of a shared data structure and the interface work still required.	P02

ID	Format	Title and execution idea	Destination
M10	Lead magnet	**Parallel-evaluation readiness checklist.** Checklist for named owners, agreed pathway, data permissions, baseline, IT review, and stop criteria.	P06
M11	Lead magnet	**Three-option decision worksheet.** Template comparing incumbent fix, narrow add-on, and MediMind against burden, risk, cost, and scope.	P07
M12	Proof	**Documented domain map.** Publish the 25-domain scope as an internal product map with clear deployed, accepted, and pending fields after verification.	P10
M13	Proof	**Live-pathway evidence template.** Show the exact baseline and after fields a customer-approved case study would need; label outcome cells pending.	P10
M14	Email	**The change-risk question.** Explain what a parallel comparison can answer and ask which transition failure the hospital fears most.	P06
M15	Email	**The current-vendor alternative.** Invite the buyer to compare a current-vendor fix with a connected pathway using the same criteria.	P07
M16	Community	**Peer discussion on evaluation design.** Ask consenting hospital operators what evidence they require before changing a core workflow.	P06

ID	Format	Title and execution idea	Destination
M17	Community	**IT and clinical review session.** Offer an educational walk-through of data, safety, and cutover questions with no sales claims.	P06

BOFU: 17 stage-specific ideas

ID	Format	Title and execution idea	Destination
B01	LinkedIn post	**What your 25-minute demo will settle.** List the three outputs: first pathway map, first metric, and evaluation readiness decision.	P05
B02	LinkedIn post	**Match price, define total cost.** Describe early-partner software-price match and the need to itemize transition and third-party costs.	P06
B03	LinkedIn post	**The honest go/no-go test.** Show how leadership can decide after staff compare one pathway and its extra work or risk.	P05
B04	Short video	**Demo in sixty seconds.** Founder shows synthetic map, shared task, and comparison worksheet as the demo preview.	P05
B05	Short video	**Parallel beside current work.** Draw existing and proposed pathways side by side and explain readiness, workload, and sign-off.	P06

ID	Format	Title and execution idea	Destination
B06	Short video	**Who should join the first call?.** Name CEO or operator plus optional clinical/operations and IT counterparts, depending on decision stage.	P05
B07	Blog/SEO	**Questions to ask before a hospital OS demo.** Give buyers questions about scope, human review, data rights, deployment proof, and total cost.	P05
B08	Blog/SEO	**What happens after a parallel evaluation.** Explain measured review, reconciliation, approval, and staged rollout; no automatic production switch.	P06
B09	Blog/SEO	**What current-software price match should include.** Explain the questions a hospital should settle about eligible spend, term, development, and total cost.	P06
B10	Lead magnet	**Pathway demo preparation card.** One-page card for an executive to bring a recent incident, current workaround, owner, and desired measure.	P05
B11	Lead magnet	**Evaluation decision sheet.** Go/adjust/stop template that records baseline, staff burden, risk, cost, and sign-off owner.	P06
B12	Proof	**Verified implementation timeline.** After authorization, publish dated scope, readiness, and acceptance milestones from a real customer without claiming broad outcomes.	P03

ID	Format	Title and execution idea	Destination
B13	Proof	**Buyer-approved workflow result.** Reserve case study for a measured result and customer permission; until then show evidence fields as pending.	P10
B14	Email	**Your pathway before our demo.** Ask attendee for one recent handoff and offer the preparation card; do not request patient-identifying details.	P05
B15	Email	**What we will compare.** After demo, send a proposed baseline and evaluation scope with unresolved requirements visibly marked.	P06
B16	Community	**Private buying-committee walkthrough.** Convene named clinical, finance, operations, and IT evaluators around one pathway, by invitation.	P05
B17	Community	**Procurement questions session.** Offer a structured discussion of costs, responsibilities, support, and exit needs with the buying group.	P06

AFU: 17 stage-specific ideas

ID	Format	Title and execution idea	Destination
A01	LinkedIn post	**First pathway, first baseline.** With permission, describe the method for recording a baseline after sign-up, without showing confidential data.	P08

ID	Format	Title and execution idea	Destination
A02	LinkedIn post	**What staff should report in week one.** Explain how to capture repeat work, exceptions, and safety concerns during a controlled evaluation.	P08
A03	LinkedIn post	**One improvement, then the next.** Show an anonymized learning loop: observe, prioritize, test, approve, measure; no unverified success claim.	P08
A04	Short video	**How to log a handoff issue.** Phone walkthrough of an issue form with synthetic patient data and an owner for follow-up.	P08
A05	Short video	**Five-minute pathway review.** Show how clinical and operations owners compare the agreed metric and exceptions each week.	P08
A06	Short video	**How a testable change becomes approved.** Explain request, test, clinical/IT review, and production approval with a generic example.	P08
A07	Blog/SEO	**Measuring repeat work without double counting.** Define staff time, task count, training burden, and why minutes do not automatically equal cash savings.	P08
A08	Blog/SEO	**Running a safe pathway review.** Guide to issue ownership, clinical escalation, baseline comparison, and go/adjust/stop decisions.	P08

ID	Format	Title and execution idea	Destination
A09	Blog/SEO	**Selecting the second pathway.** Criteria for choosing a next workflow after the first is accepted: frequency, burden, risk, and readiness.	P08
A10	Lead magnet	**First-pathway measurement sheet.** Customer worksheet for baseline, tasks, exceptions, staff burden, and sign-off.	P08
A11	Lead magnet	**Change-request decision card.** Template for request owner, test criteria, clinical impact, priority, and production approval.	P08
A12	Proof	**Accepted-workflow log.** Internal customer-approved ledger of which pathway elements were accepted and which remain open.	P08
A13	Proof	**Measured improvement record.** Record an actual agreed before/after comparison only after data quality and buyer sign-off; no public release by default.	P08
A14	Email	**Your first review agenda.** Send the clinical/operations owner a concise agenda for metric, exceptions, staff feedback, and next decision.	P08
A15	Email	**A request ready for test.** Tell the named owner what changed, where to test it, what remains unapproved, and how to respond.	P08
A16	Community	**Clinical-IT issue huddle.** Short private working session for clinicians and IT to resolve one observed pathway issue.	P08

ID	Format	Title and execution idea	Destination
A17	Community	**Cross-site learning exchange.** With a group customer, invite site owners to compare process lessons without patient details.	P08

LOFU: 17 stage-specific ideas

ID	Format	Title and execution idea	Destination
L01	LinkedIn post	**The buyer's permission comes first.** Explain what a responsible public hospital story needs: verified result, named scope, and permission.	P10
L02	LinkedIn post	**A pathway another leader can inspect.** After approval, share a measured pathway story with method and limits rather than a sweeping transformation claim.	P03
L03	LinkedIn post	**What we changed after feedback.** Publish a consented example of customer feedback becoming a tested product change, with dates verified.	P10
L04	Short video	**Customer pathway walkthrough.** With customer approval, have staff explain a measured before/after task using non-identifying visuals.	P03
L05	Short video	**What a reference actually verified.** Founder shows the evidence standard and a permissioned result; hold production until proof exists.	P10

ID	Format	Title and execution idea	Destination
L06	Short video	**Why we chose the first pathway.** Customer-approved owner explains why the initial workflow was selected and what remains unsolved.	P03
L07	Blog/SEO	**How to read a hospital software case study.** Teach readers to distinguish contracted sites, live sites, accepted workflow, and measured outcome.	P10
L08	Blog/SEO	**From one hospital pathway to another.** Permissioned account of what transferred across sites and what required local redesign.	P03
L09	Blog/SEO	**What changed in a claim handoff.** Only after verification, explain actual claim-process result and separate identified charges from collections.	P03
L10	Lead magnet	**Reference-story evidence checklist.** Template a hospital can use to evaluate any vendor story: baseline, denominator, site, dates, approval.	P10
L11	Lead magnet	**Peer evaluation guide.** Guide for hospital executives to ask a reference about transition burden, support, and continuing development.	P09
L12	Proof	**Buyer-approved measured case study.** Publish only after customer permission and audited baseline, method, result, and limitations.	P03

ID	Format	Title and execution idea	Destination
L13	Proof	**Reference conversation.** Offer a customer reference only after that customer's explicit agreement, with scope and availability confirmed.	P09
L14	Email	**Share the method, not a claim.** Ask a satisfied authorized customer whether they would review a draft factual pathway story.	P09
L15	Email	**A peer may have this problem.** After evidence and consent, offer a short forwardable note about the measured pathway and its limits.	P03
L16	Community	**Peer operator exchange.** With permission, convene an existing customer and one peer operator to discuss implementation lessons.	P09
L17	Community	**Hospital-association learning session.** Invite a permissioned customer to teach the pathway method; verify event and affiliation before promotion.	P10

Three highest-leverage pieces for this founder: T10, the one-page patient-pathway friction worksheet; M10, the parallel-evaluation readiness checklist; and B04, the synthetic one-minute demo preview. **This week, create T10.** It helps a leader articulate a real problem, gives a useful result before a meeting, and supplies the first 25-minute demo question. The most powerful result-in-advance asset is a **guided first-pathway map**: in a short founder conversation, the buyer leaves with the chosen handoff, who owns it, a baseline metric, and a go/no-go question for evaluation. That is a useful result even if they do not buy. A guided synthetic sandbox (P11) is a later option; it must not be confused with the founder's two-week tailored parallel evaluation or a live clinical deployment.

Part 4 - Funnel wiring diagram

A Georgian private-hospital CEO first encounters us when a founder post or permissioned peer discussion asks why one patient creates repeated work across departments. The CEO opens the patient-pathway worksheet (T10), names a recent handoff, and receives one useful follow-up email rather than a broad AI brochure. An option-comparison article (M08) and readiness checklist (M10) answer why a current-vendor fix, narrow tool, or new operating system might be right. A short synthetic demo preview (B04) leads to the `/request-demo` route (P05), which must record a real accepted request before any “received” message is shown. The founder runs a 25-minute call, maps the hospital's first pathway, and proposes an evaluation only if the buyer can name an owner and a readiness path. Clinical, finance, operations, and IT counterparts agree on baseline, access, effort, and success criteria; they compare the tailored path beside current work before any production decision. If a defined scope is approved, the customer reaches a buyer-accepted first pathway, reviews the measure and exceptions, and chooses whether to expand. If a meaningful result is measured and the customer consents, MediMind publishes an accurate case or arranges a reference, **and they become an advocate who brings in another private-hospital CEO or group operator.**

Part 5 - Build order

The calendar below describes **marketing work** within five hours total per week. Engineering for lead receipt, page instrumentation, and sandbox setup; the 25-minute sales meetings; customer evaluation; and clinical/IT delivery need separately planned time and owners. No paid promotion is assumed.

Phase 1 - Weeks 1-2: minimum viable funnel, three assets

Asset	Why first	It must achieve
P05 reliable pathway-demo request route	No funnel exists if a request appears successful but is not received.	Real delivery to a named owner, honest error/success behavior, and one logged qualified/held-demo trail. Engineering work is separate from marketing hours.
T10 one-page pathway worksheet with P01 download	Gives a relevant result before asking for time and sharpens the founder's discovery.	A CEO can name one handoff, its owner, and a measure without sharing patient data.
T04 synthetic duplicate-entry trail plus founder outreach note	Shows the repeated work visually and gives a reason to bring a real pathway to the founder.	A relevant executive can recognize the problem and choose a slot or reply for coordination.

Single MVF signal: a held qualified hospital demo sourced through a traceable contact path, with the first pathway and next decision recorded. A completed form alone is not a demo. Qualification requires a hospital organization, relevant authority or sponsor, a named recent problem, and an evaluation owner or a clear path to one.

Phase 2 - Weeks 3-6: seven conversion assets

“Expected conversion improvement” below means the **specific behavior the asset is designed to improve**, not an invented uplift percentage.

Asset	Stage fixed	Behavior to look for
M08 option-comparison article / P07	MOFU	More buyers can explain why a core evaluation is or is not warranted.
M10 readiness checklist / P06	MOFU	More demos end with named clinical/operations and IT counterparts.
B10 demo preparation card	BOFU	More booked calls arrive with a recent handoff and a useful example.
B11 evaluation decision sheet	BOFU	More qualified demos yield an explicit go/adjust/stop proposal.
B14 confirmation email	BOFU	Fewer bookings are missed; attendees know what to prepare.
B15 post-demo scope email	BOFU	Next actions, owners, and unresolved requirements are recorded rather than lost.
M12 documented-domain map / P10	MOFU	IT and clinical reviewers can separate documented scope from accepted deployment.

Readiness for Phase 3: a working lead route and a small traceable sequence of held qualified demos with follow-up ownership; preferably at least one readiness-approved evaluation before expanding content production. The 3-of-12 downstream scenario is a hypothesis to monitor, not a percentage threshold to declare a win after one or two calls.

Phase 3 - Months 2-3: growth layer

Asset or system	How it compounds	Manual effort it can replace
Reusable patient-pathway content set (T07/M07/B07)	Each real buyer question improves an evergreen answer and a better demo.	Repeating the same explanation in each first call.
Permissioned proof library P10, later verified case P03	A measured, approved story answers a recurring risk question for later committees.	Unstructured founder claims and ad hoc evidence requests.
Clean public-site source and event tracking	Shows which named distribution paths produce accepted requests and held qualified demos.	Guessing from page views or button clicks.
Opt-in email follow-up (T14/M14/B14)	Gives useful next steps at the right buying stage.	Remembering every nurture step manually.
Customer activation measurement and a permitted reference path	The next buyer can learn from an actual accepted pathway, with its limits.	Rebuilding trust entirely from founder description.

Compounding signal: new qualified hospital conversations begin from a reusable asset or permissioned peer introduction without a fresh one-off founder explanation. Count them by source and held demo; do not infer “organic growth” from impressions alone.

Part 6 - Weekly funnel scorecard

Use a Monday review of **counts first**, with rates only when denominators exist. A green/amber/red threshold is an internal action rule for the 90-day scenario, not a validated industry benchmark. The library's 2-5% homepage, 3.8% demo-page, 25-45% webinar, 15-30% trial-paid, >60% activation, 20-30% email-open, and 46.9% month-one-retention figures are in Part 2 as unverified generic context. They should not override low-volume account evidence or define a hospital service standard. Track source and stage with no patient data or personal information in analytics events.

Stage	Metric and how to measure	Green - on track	Amber - needs attention	Red - act now	Likely red cause	First fix
TOFU	Relevant named-account or permissioned-peer conversations begun this week; record source and problem in a secure lead log	2+ relevant conversations	1	0 for two weeks	Message or distribution misses the buyer	Ask known contacts which handoff matters; test T10 with one leader
MOFU	Buyers who complete a pathway map or option discussion with a named owner	1+ this week	Interest without a named pathway	0 from four relevant conversations	Generic content or unclear first result	Use T10 live and ask for the last incident
BOFU	Held qualified hospital demos, not booked calls; verify attendance, role, problem, owner	1+ this week, accumulating toward 12 in 90 days	1 booked but not held, or held without qualification	0 for two weeks despite relevant conversations, or request route fails	Broken route, weak call promise, or no follow-up	Verify submission and contact owner; send B14 preparation note
BOFU → evaluation	Readiness-approved evaluations / held qualified demos, with named IT and clinical/operations owners	1 approved as the series matures; scenario is 3 of 12	Proposal pending missing prerequisite	Repeated demos with no explicit decision	Evaluation feels too risky or vague	Use M10 and B11 to expose missing readiness and scope
AFU	First accepted pathway milestone and agreed metric/exception log for each active customer	Milestone met or openly re-scoped with customer approval	Behind schedule with named remedy	No owner, baseline, or clinical/IT sign-off	Delivery or data readiness gap	Joint issue review; narrow the pathway safely

Stage	Metric and how to measure	Green - on track	Amber - needs attention	Red - act now	Likely red cause	First fix
LOFU	Permissioned case/reference or qualified introduction from an accepted customer	1 permissioned action after verified value exists	Eligible story identified, approval pending	An unapproved name/outcome is published	Proof governance failure	Remove or correct claim and seek explicit permission
Overall	**Cumulative held qualified hospital demos in rolling 90 days**; one record per buying organization meeting	Pace at or above 12/90-day scenario	Below pace but improving with recorded causes	No held qualified demo for two consecutive weeks once route/distribution is live	Earlier stage leak or qualification mismatch	Trace the last four accounts from first touch to outcome

If a denominator is zero, show **N/A**, not 0%. The stretch downstream markers are three readiness-approved evaluations and one new paying buying organization by day 90; report them as counts, not a forecast. The anchor account is a separate evidence and expansion line. A single week of red can be noise, but a broken request route is actionable immediately.

Sources and handoff

Part 1 inputs: business/market, product journey, BFG matrix, top UVPs, problem stack, and synthetic avatar. The research addendum records the demo-form source inspection. The B11 benchmark list is reproduced from the assigned main library, whose figures do not supply current local baselines. Prompt 2 should prioritize the pathway hook, controlled evaluation, and clearer patient story within the same five-hour weekly marketing budget.

11 Part 2 · Prompt 2

Define Your Marketing Angles

Source: Complete library, pages 33-35 · 3,031 words · 6 requested steps / sections

> **Status and required inputs (23 September 2026).** Prompt 1's funnel is a founder-led hybrid, with a 25-minute pathway demo and a proposed measured evaluation as the principal transition from interest to a decision. The synthetic avatar is the accountable Georgian private-hospital operator; Prompt 8's lead question is “Why does one patient create so much extra work?” The top three working UVPs are: find and test repeat work, compare beside the current system before switching, and give teams a clearer patient story. The BFG matrix documents 25 hospital domains and two cross-cutting capabilities but no independently measured outcome. Competition includes the incumbent, narrow tools, broad vendors, and workarounds; the small public-review corpus is mostly non-Georgian staff and cannot supply a CEO quote. No target-buyer interview language has been collected. This document uses **proposed buyer language**, not an attributed customer's exact words.

Four current-situation answers. The leading fear to test is that a core switch disrupts care or revenue and leaves the leader responsible for an expensive mistake. What the buyer most wants to be able to say is, “Our team tested one important pathway, saw the real work and cost, and made a careful decision.” This is synthetic language, not a quotation. A live site and founder-authored material exist, but no verified marketing traffic, lead, or channel-performance series was supplied; the demo form's source-observed simulated submission needs a live check and repair before promotion. The founder did not provide brands they admire. As editorial references only, borrow HubSpot's useful diagnostic asset, Salesforce's committee-aware enterprise education, and Monday.com's easily understood visual demonstration; none establishes a suitable acquisition benchmark for Georgian hospitals.

Part 1 - Master marketing angle

1. Hook

Why does one patient create so much extra work across your hospital?

The question gives a busy executive something they can test against a recent case. It points to one patient journey, not a claim that every hospital is broken. A reader who does not recognize the problem can still learn whether it exists by mapping one pathway; a reader whose problem is staffing or payer policy should not be pushed toward core replacement.

2. Differentiator

The template in the library demands “the only.” There is no global comparison that supports exclusive ownership of a hospital operating system, FHIR R4, AI, or parallel evaluation. The defensible one-sentence version keeps its structure without fabricating exclusivity:

Unlike adding another point solution to a repeated handoff, MediMind is a hospital operating system that lets a private hospital map and compare a tailored patient pathway across clinical and business work before deciding on a wider switch.

The difference to demonstrate is the combination of shared scope, site-specific path, founder development relationship, and controlled comparison. It is a testable positioning claim, not a declaration that no competitor could offer any of those elements.

3. Primary psychological trigger

Commitment and consistency is the most useful initial trigger: ask the leader to name a recurring problem they already agree should be fixed, then make the next action a small, consistent step-map one pathway. This avoids asking them to endorse a wholesale replacement before they have evidence. Loss aversion is present in the fear of failed transition, but the campaign should answer it with control and proof so the desired emotion becomes earned confidence from a hospital-owned comparison.

Part 2 - Angle selection and prioritization

“Yes” means suitable for the strategy, not necessarily an asset to produce this month. Priority 1-3 are the **only active angles in the first 30 days**; the rest can be tested after the route and core assets work. “No” means hold until the stated missing evidence, channel access, or resources exist.

Angle	Use it - yes or no	Why it fits or does not fit	Funnel stage	Priority rank	Specific message to lead with
Encourage audience to sign up for a demo	Yes	The buying decision requires a human, pathway-specific conversation.	BOFU	3	“Bring one handoff; leave with a first pathway map.”

Angle	Use it - yes or no	Why it fits or does not fit	Funnel stage	Priority rank	Specific message to lead with
Target audience based on language and region	Yes	Start with Georgian private-hospital decision-makers; validate exact language preferences.	TOFU-MOFU	5	“Let's map the work as your Georgian hospital actually runs it.”
Book sales calls	Yes	Named executive introductions may outperform anonymous traffic at this stage.	BOFU	6	“Could we spend 25 minutes on the last handoff your team had to repair?”
Offer rewards to increase product usage	No	Clinical software use should follow patient/work value and governance, not incentives that distort behavior.	AFU	-	No reward promise.
Try a new low-risk CTA offer	**Yes**	A pathway map gives a useful result before a purchasing decision.	TOFU-BOFU	**1**	“Map my hospital's first improvement.”
A sneak peek at pricing options	Yes, after written terms	Current-software price match is meaningful; total cost and eligibility must be clarified.	BOFU	7	“Early partners can compare the matched software fee alongside the full evaluation and transition costs.”
Promote product launches and new features	No now	A feature announcement is weaker than a buyer pathway and needs verified readiness.	MOFU-AFU	-	Publish only accepted, relevant changes later.
Highlight a use case offer	**Yes**	One patient pathway makes broad product scope concrete.	TOFU-MOFU	**2**	“Follow one test from order to result to bill; count where the work repeats.”

Angle	Use it - yes or no	Why it fits or does not fit	Funnel stage	Priority rank	Specific message to lead with
An inside look at your product dashboard	Yes, carefully	A synthetic walkthrough can show mechanics; a dashboard alone does not solve source-data quality.	MOFU	4	“See the shared patient and task context in one agreed example.”
Display testimonials	No now	No verified, permissioned MediMind customer quote is supplied.	MOFU-LOFU	-	Reserve a slot only after a real approved quote.
Use video ads to demonstrate benefits	No paid ads now	Zero paid budget and no local creative baseline; an organic founder video supports the use-case angle.	TOFU-BOFU	-	Film the pathway organically, then evaluate paid distribution later.
Share case studies	No public outcome case now	Founder-authored implementation context exists, not an independent measured customer outcome.	MOFU-LOFU	-	Publish a buyer-approved measured pathway case when available.
Promote blog posts	Yes	Searchable decision guides can support a long, committee-led purchase.	TOFU-MOFU	8	“How do you compare a current-vendor fix with a parallel hospital-system evaluation?”
Promote exclusive research	No now	No original hospital-buyer dataset or permissioned workflow study exists.	TOFU	-	Publish methods and source-labeled findings, not invented exclusivity.
Promote webinars	No now	A 45-minute practical session is proposed but unscheduled and requires an audience.	MOFU	-	Hold until registration route and speaker/agenda are real.

Angle	Use it - yes or no	Why it fits or does not fit	Funnel stage	Priority rank	Specific message to lead with
Promote online and offline events	No now	No specific suitable event, slot, or partner is confirmed.	TOFU-MOFU	-	Verify an association opportunity before promotion.

Priority 3 is the demo invitation framed as a controlled parallel comparison: “See your pathway beside today's work before deciding.” The first three active angles therefore form one story: **map the problem → show a patient-pathway use case → compare safely**.

Part 3 - Angle-to-channel mapping

The table covers every selected “yes” angle. It specifies an initial asset and a countable success event. Channel use is proposed until the buyer's actual media and referral habits are observed. There is no paid campaign in the first month.

Selected angle and channel	First content/format	CTA	What success looks like
Low-risk CTA / founder LinkedIn	T01: post with a synthetic seven-handoff sketch and invitation to name the local version	Map my hospital's first improvement (P01)	A relevant operator requests or discusses a pathway map.
Low-risk CTA / existing professional introductions	One-to-one note offering T10 worksheet, only to a legitimate contact	Use the pathway worksheet (P01)	Recipient returns a real handoff or introduces its owner.
Use-case offer / founder phone video	T04: duplicate-entry trail using synthetic information	Map my hospital's first improvement (P01)	A relevant leader watches enough to ask about their own pathway; count replies, not views alone.
Use-case offer / website article	T07: how to map a patient handoff	Download the worksheet (P01)	Qualified downloads or a pathway question from a hospital.
Controlled comparison within low-risk CTA / founder follow-up	M10 readiness checklist after an interested response	Discuss an evaluation (P06)	Named clinical/operations and IT counterparts plus agreed next step.

Selected angle and channel	First content/format	CTA	What success looks like
Demo / existing `/request-demo` page	B04 synthetic demo preview, after real submission receipt is verified	Request a pathway demo (P05)	Held qualified demo, not a success-screen view.
Language/region / Georgian professional connections	Localized T01 with reviewed terminology, not automatic translation	Map my hospital's first improvement (P01)	A Georgian hospital operator recognizes and corrects the scenario.
Direct sales call / permissioned peer introduction	Short founder note about a recent handoff, not a generic pitch	Request a pathway demo (P05)	Relevant executive accepts a 25-minute conversation.
Pricing peek / founder proposal	One-page offer comparison including matched software fee and visible total-cost questions	Discuss the early-partner offer (P06)	Buyer understands what is and is not yet priced and remains in evaluation.
Inside look / synthetic screen recording	M06 result-to-review task flow, with no patient data or implied clinical autonomy	See the operating system (P02)	Clinical/IT evaluators ask about their own equivalent workflow.
Blog promotion / founder LinkedIn and targeted follow-up	M08 decision guide comparing incumbent fix, point solution, and new system	Compare the approaches (P07)	An evaluator can name why a narrow fix is or is not sufficient.

The catalog's proposed community entries identify places to test, not pre-approved posting rights. The Georgian Healthcare Association has a public provider network; whether the actual target buyer reads a channel or welcomes a post requires verification and permission.

Part 4 - Ten-hook library

These are original lines in the **synthetic avatar's working language**. They are not recorded Georgian buyer quotations. For a Level-4 market, each should lead toward a mechanism or proof step rather than stopping at a vague “better care” claim.

Approach	Hook	Best first use
1. Pain	“Your team cared for the patient. Why did they also have to carry the information between departments?”	LinkedIn opener

Approach	Hook	Best first use
2. Future	“Imagine a hospital where the next team starts with the patient's story instead of rebuilding it.”	Product explainer
3. Mechanism	“Pick one patient pathway; compare its handoffs beside the system you use today.”	Demo page
4. Number	“One patient. One pathway. A 25-minute map of where your hospital repeats the work.”	Founder video thumbnail; 25 minutes is the proposed demo length
5. Question	“Why does one patient create so much extra work across your hospital?”	Master campaign
6. Story	“The result was ready. Three teams still had to ask where it went.”	Synthetic scenario post, labeled illustrative
7. Provocation	“A faster screen can still leave the hospital doing the same work twice.”	Comparison article
8. Social proof	“A signed network agreement shows a hospital willing to explore this path; your own team's comparison is the proof you need.”	Warm follow-up only, after founder-approved reference wording; no customer quote or outcome implied
9. Loss	“Every repeated handoff uses staff attention you could put toward the next patient.”	Email subject, as a plausible cost to measure, not quantified loss
10. Identity	“Lead the hospital that checks the pathway before it changes the whole system.”	Executive invitation

For #8, the agreement is founder-reported and the Medplum case study is founder-authored. Verify external naming and permission before publication. If that cannot be done, omit the social-proof hook rather than inventing a customer success story.

Part 5 - Message by funnel stage

Stage	Core one-sentence message	Tone	Never say here	Three actual example messages
TOFU	“A single patient can reveal where your hospital's teams repeat work; map one journey to see whether the problem is worth fixing.”	Observant, useful, curious	“Your hospital loses a known amount of money” without data	LinkedIn: “Where does one result turn into three phone calls?” · Email subject: “The handoff your team repeats” · Page: “Map one patient pathway.”
MOFU	“Compare a current-vendor fix, a point solution, and a shared hospital path against the same work, cost, and risk.”	Fair, explanatory, specific	“Every other system is disconnected”	LinkedIn: “Would a new dashboard fix the handoff-or just display it?” · Email subject: “Fix the screen or the pathway?” · Page: “Compare the three practical options.”
BOFU	“Bring one recent handoff to a founder-led demo and decide whether a scoped parallel comparison is worth your team's time.”	Calm, direct, transparent	“Replace your hospital in two weeks with no risk”	LinkedIn: “Your first demo should end with a pathway, not a feature list.” · Email subject: “What to bring to our 25 minutes” · Page: “See your first pathway before you switch.”
AFU	“Use the first agreed pathway, measure staff work and exceptions, and approve the next step only when your owners accept it.”	Supportive, accountable	“Launch equals success”	Customer note: “What did staff still have to repeat?” · Email subject: “Your first pathway review” · Activation page: “Measure the work; record the exceptions.”
LOFU	“A verified, permissioned pathway story can help another hospital judge whether the approach fits its own work.”	Factual, generous, respectful	“Our customer guarantees your result”	LinkedIn: “What this pathway result actually measured” · Email subject: “Would you review a factual story?” · Proof page: “Scope, baseline, result, and limits.”

Part 6 - 30-day angle execution plan

Five hours below is the **total marketing allocation each week** across this prompt, the funnel plan, and campaign work. The binding weekly clock reserves **85 minutes** for routine work: five five-minute weekday checks (25 minutes total), a 30-minute Monday scorecard review, and a 30-minute amortized monthly cleanup reserve. That leaves **215 minutes (3 hours 35 minutes)** for the creative and outreach actions in each row. The founder's 25-minute sales meetings are additional sales time; reliable form submission and analytics setup require engineering time. Do not launch traffic to a simulated request route. Week 1 contains a route verification gate, while drafting and direct discovery can continue independently.

Week and leading angle	Create and publish within five marketing hours	Measure	If the previous week misses its signal
1 (days 1-7): low-risk pathway CTA	**215 min active:** 75 min build T10 one-page worksheet; 40 min write/publish T01 post; 60 min send five individually relevant, permissioned notes; 40 min respond and ask for the last handoff. Publish only when P01 works. **85 min routine** as above.	Relevant replies, completed maps, and accepted route submissions-not impressions.	If no relevant reply, ask two known operators which wording describes a real handoff; revise T01 rather than making more formats.
2 (days 8-14): use-case offer	**215 min active:** 85 min film T04 phone video with a synthetic pathway; 40 min caption/edit; 10 min publish with T10 link; 80 min follow up with interested leaders. **85 min routine.**	A hospital leader names a pathway or asks for a demo.	If week 1 had no signal, use a simpler recent incident question, test it in direct conversation, and publish only the version a buyer recognizes.
3 (days 15-21): controlled comparison	**215 min active:** 85 min make M10 readiness checklist; 40 min write M02 post; 30 min publish/share with prior responders; 60 min answer readiness questions. **85 min routine.**	Named clinical/operations and IT counterparts, evaluation interest, held qualified demo.	If week 2 gets attention but no next step, show what the buyer leaves with after 25 minutes and check the actual request path.
4 (days 22-28): combine and learn	**215 min active:** 85 min create B04 synthetic demo preview; 15 min publish with verified P05 link; 75 min personally follow up; 40 min inspect the last four account journeys and choose one correction. **85 min routine.**	Cumulative held qualified demos versus the 12/90-day scenario, plus evaluation readiness.	If week 3 yields demos without evaluations, ask exactly which prerequisite is missing; change the offer or qualification, not the volume of content.

Days 29-30 use the routine checks and next Monday review from the following weekly allocation; they add no fifth creative block to the 30-day plan. There is no paid budget and no fabricated webinar or event date. The first month succeeds when the system can receive a real inquiry, the founder can hold a qualified pathway demo, and a hospital can clearly accept or decline an evaluation. A small sample cannot validate one hook over another; use buyer explanation and the next step they take.

Sources and handoff to Prompt 3A

Working inputs: funnel blueprint, synthetic avatar, problem stack, UVPs, BFG, competition, and review intelligence. The live-route caveat comes from the research addendum. The campaign should film the question hook and one synthetic handoff, then invite a founder-led pathway map.

12 Part 2 · Prompt 3A

Build Your Campaign Concept & Video Script

Source: Complete library, pages 36-38 · 1,887 words · 3 requested steps / sections

> **Status and required inputs (23 September 2026).** The first buyer is the synthetic accountable Georgian private-hospital operator from Prompt 9. Prompt 8's working surface problem is repeat work along one patient pathway; the hidden problem is fear that a replacement will make care and operations worse. The strongest working UVP is to **find and test** that repeat work, at market-sophistication Level 4 ("New Mechanism"). Prompt 2's top angle is a low-risk, useful first action: map the hospital's own pathway, then compare a tailored version beside current work before switching. The campaign must drive a **held, qualified founder-led pathway demo**, initiated through a verified request route or direct founder reply. Production reality is **one founder, a phone camera, paper cards or a simple synthetic screen recording, natural light, and no paid production budget**. The most painful moment to dramatize is staff having to ask or re-enter what another department already knows while a patient waits for the next step. There is no traceable buyer quote explaining why a customer chose MediMind; no line below is attributed to a customer. The founder reports a signed GeoHospitals agreement; no customer outcome or live-site count is implied.

Part 1 - Campaign concept

Premise: This campaign is for a private-hospital leader who sees one patient create repeated work across departments, and it shows them that a safer first step is to map and compare one pathway before considering a wider software change.

Pattern interrupt - the question they may already be asking. Open the main asset with the founder holding three simple cards labeled "order," "result," and "bill," then moving the same blank patient card between them. Exact first five seconds: **"Why does one patient create so much extra work?"** The question is original proposed copy, not a buyer quotation. The cards make the problem visible without using patient information or pretending a fabricated case is real. The two A/B variations deliberately test different openings against this main question-led version.

Campaign CTA (five words): "Map my hospital's first improvement." The promised outcome is a concrete pathway map and next measurement question, not an instant hospital-wide transformation. The first conversation is recommended as 25 minutes, led by the founder. A tailored parallel evaluation in two weeks is the founder's readiness-dependent offer after scope and approval, not what every viewer automatically receives.

Three-act path: Problem-one patient story gets recreated across roles. Mechanism-the team follows one shared path from task to task on a FHIR R4 foundation and can compare it beside current work. Relief-the leader can inspect what changed and decide with clinical and IT colleagues whether a broader move is justified. AI may assist with defined tasks for human review, but the story does not need an autonomous clinical claim to be compelling.

Part 2 - Video script: founder on camera, phone format

Target running time: 78-85 seconds at an ordinary speaking pace. Film vertically with a clean desk and daylight; crop a horizontal version from a wider phone frame if needed. The founder may speak as a physician and CEO, but should sound like a colleague mapping work, not a narrator advertising a miracle. Cards and screens contain invented names or no names at all. Every onscreen product capture must come from a synthetic/demo environment, with no actual patient, staff, payer, or contract data.

Time / act	Exact founder words	What the camera shows	On-screen text
0-5s / opening	**"Why does one patient create so much extra work?"**	Close view of one blank patient card passing from a paper "order" card to "result" to "bill." Founder looks up at camera on "work."	"ONE PATIENT. TOO MUCH REPEAT WORK."
5-17s / problem	"A doctor orders a test. Someone calls to check whether it happened. Someone else finds the result. Finance still has to work out what to bill."	Founder places a small question mark between each card. No product visible.	"Order → chase → result → reconcile"
17-27s / hidden problem	"Each person is doing their job. But the journey between them can become another job for your staff."	Stack blank paper slips under the same patient card. Pause so the image lands.	"The handoff becomes work."
27-41s / mechanism	"So we start with one real pathway at your hospital. We map who needs to act, what information they need, and where the same work is repeated."	Whiteboard/paper diagram with three role labels and arrows; founder circles one repeated step. If a screenshot is used, switch only to synthetic demo data.	"Map one pathway."
41-57s / mechanism	"MediMind brings the clinical and business steps into one hospital operating system. Your team can compare that path beside the way it works today."	Show two paper lanes labeled "today" and "proposed"; reveal one shared task and patient context on a synthetic screen if available. Do not show a flashy dashboard unrelated to the example.	"Compare beside today's work."
57-69s / relief	"Then we measure the handoff, the staff effort, and the exceptions. Your clinicians and IT team help decide what is ready-and whether a wider change makes sense."	Founder places a simple three-column card: "work," "exceptions," "decision." A handwritten checkbox remains empty.	"Measure. Review. Decide."
69-72s / transition	"Bring me one handoff your hospital keeps repeating."	Direct to camera; keep the circled pathway visible.	"ONE HANDOFF"

Time / act	Exact founder words	What the camera shows	On-screen text
72-82s / closing ten seconds	**“In 25 minutes, we'll map the first improvement worth testing. Map my hospital's first improvement.”**	Hold a clean CTA card with the verified public route or a direct founder contact path.	“MAP MY HOSPITAL'S FIRST IMPROVEMENT”

The spoken script is about 155 words, leaving time for visual pauses. A fast read would undercut the tone; if the founder reaches the CTA too early, let the two-lane comparison remain on screen rather than adding claims. If the full cut runs past 90 seconds, cut the line about billing in the first act before accelerating speech.

Director's three instructions:

1. Keep the opening quiet and slightly puzzled; speak directly to one hospital operator, not to a crowd. Let the paper-card movement supply the interruption.
2. Make each cut coincide with one handoff. Use large, readable labels and high contrast; mobile viewers should understand the visual with sound off and captions on.
3. End on the founder's face and one CTA. Do not place an unverified customer logo, “25 hospitals live,” a performance percentage, or an artificial deadline on screen.

15-second organic cut-down (same question-led campaign):

Time	Exact words	Visual
0-4s	“Why does one patient create so much extra work?”	Same three cards and blank patient card.
4-9s	“Pick one handoff. We'll map what repeats.”	Circle one repeated arrow on paper.
9-15s	“Compare beside today's work. Map my hospital's first improvement.”	Two lanes, then CTA card.

The cut-down invites the same action and does not introduce an unsupported result or a second destination.

Part 3 - Two shorter A/B script variations

These are **ready-to-film creative tests**, not instructions to buy ads now. They can be posted organically first; paid LinkedIn/YouTube distribution is conditional on a real budget, a functioning lead route, and evidence that the message attracts qualified hospital operators. Each follows problem → mechanism → relief, but changes the opening so the team can learn which explanation earns a useful reply.

Variation A - visual contrast interrupt, 22-25 seconds

Audience: cold relevant hospital leaders; **trigger:** commitment and consistency-the leader can check one visible example without endorsing a replacement. The contrast appears in the opening split screen.

Time	Exact script	Visual / act
0-5s	“Same patient. Two very different ways to handle the handoff.”	Left: three separate blank cards and arrows; right: one connected path. No MediMind logo yet. **Problem.**
5-12s	“Here, staff ask and copy. Here, the next task stays with the patient context.”	Founder points to each lane; label the right lane “proposed,” not “proven result.” **Mechanism.**
12-23s	“Which path works better for your hospital? Map one handoff, compare both, then decide. Map my hospital's first improvement.”	Founder circles a single handoff, then CTA card. **Relief and CTA.**

This variant tests whether seeing a concrete alternative before hearing the question improves comprehension. It does not show actual patient data or assert that MediMind has already reduced a real customer's work.

Variation B - honest social-proof interrupt, 25-29 seconds

Audience: warm retargeting or existing leads; **trigger:** social proof that lowers the perceived risk of engaging, immediately bounded by an invitation to verify the buyer's own case. Social proof is a founder-reported procurement fact, **not** a customer testimonial or measured outcome. Before public release, confirm the approved wording about the signed network agreement and avoid using the customer's name or logo without permission.

Time	Exact script	Visual / act
0-5s	“A Georgian hospital network signed with MediMind.”	Founder on camera; text “Signed network agreement - founder-reported.” No logo, facility count, or invented result. **Social-proof opening.**
5-15s	“That is a reason to look, not proof for your team. Pick one handoff your staff repeats.”	Founder lays out the same blank “order → result → bill” cards. **Problem.**
15-28s	“We'll map a connected path, set a measure, and let your clinical and IT team decide. Map my hospital's first improvement.”	Two-lane paper diagram and CTA card. **Mechanism and relief.**

If the agreement cannot be referenced publicly with approved wording, **hold this variation**. Replace the social-proof opening only with a source-checked, customer-approved implementation fact; never substitute a fabricated quote, outcome, or “live at 25 hospitals” badge. The main and Variation A remain filmable without this variant.

Production and destination checklist

Keep this as a short execution checklist, not a new approval workflow: phone at eye level, natural light, quiet room, paper labels in large type, clean synthetic screen capture if used, accurate captions, one CTA, and no patient information. The proposed destination is the existing `/request-demo` route, which currently says 30 minutes in source; align that page with the recommended 25-minute offer if the strategy is adopted. Source inspection found a

simulated form submission, so verify deployed behavior and implement actual receipt before linking a public campaign to it. Track accepted request, qualified booking, and held demo as separate events. The video itself does not claim a scheduled webinar, self-serve clinical trial, response-time SLA, or clinical outcome.

Sources and handoff

The avatar, problem stack, UVPs, angle strategy, and funnel blueprint establish the narrative. The research addendum records the source-code status of `/request-demo`; Medplum's founder-authored case study is background on the product and agreement, not independent outcome proof. Prompt 3B can distribute the main question-led film first, use Variation A as a comprehension test, and hold Variation B until reference wording is approved. All campaign routes beyond the existing demo path are proposals.

13 Part 2 · Prompt 3B

Build Your Campaign Distribution

Source: Complete library, pages 39-41 · 4,520 words · 5 requested steps / sections

> **Status and inputs, 23 September 2026.** Complete copy package for a proposed, unlaunched campaign. Prompt 3A fixes the concept **"One patient. Too much repeat work."**, the opening question **"Why does one patient create so much extra work?"**, the paper-card order → result → bill visual, and the CTA **"Map my hospital's first improvement."** The action offered is a recommended 25-minute founder-led pathway demo that ends in a proposed measured evaluation, not an instant replacement. The funnel moves relevant hospital leaders from problem awareness through consideration to a held qualified demo. The avatar is synthetic; no buyer-language interview or customer outcome quote is available. The first media budget is zero; paid ads below are ready-to-test copy, not a paid launch instruction. Existing audience size and opt-in email list are unknown. The five-hour weekly marketing ceiling is shared with distribution and reporting; engineering work and live sales meetings are additional.

This campaign drives traffic to: the existing `/request-demo` page. The page exists, but source inspection found a simulated submission handler and current "30 minutes" text. A public campaign needs verified request receipt, a named owner, truthful success/error states, and aligned 25-minute copy before this route can be treated as a working lead destination. If a verified direct founder reply path is arranged first, it can serve as a temporary campaign response route; no response-time promise is made. All other proposed pages below remain drafts until built and tested.

Part 1 - Ad copy

The three LinkedIn assets address different awareness states. Each body is under 150 words, has one emotion, and uses the exact campaign CTA. Paid audience definitions and retargeting require consent, working measurement, and an approved budget; the same copy can be tested in founder posts or permissioned introductions first.

LinkedIn sponsored content - three versions

A. Cold / TOFU - curiosity.

- **Hook, first two lines:**

  Why does one patient create so much extra work?
  Follow the handoff, not just the chart.

- **Body:** “An order becomes a result. The result needs a decision. The decision reaches billing. If staff have to ask, copy, or reconcile at every step, that journey creates another job alongside care. We draw one patient pathway with your team, show where work repeats, and agree what would be worth measuring before anyone discusses a wider system change. Start with the pathway you already know is difficult.”
- **Primary copy CTA: Map my hospital's first improvement** → verified `/request-demo` route. **Native sponsored button:** use a platform-supported preset such as “Request demo” if available; verify the actual options in the account.
- **Visual:** Founder moves a blank patient card across three paper cards: order, result, bill. No patient data.
- **Psychological trigger:** Recognition of a familiar task, followed by a small, inspectable first step.

B. Warm / MOFU - cautious hope.

- **Hook, first two lines:**

  Why does one patient create so much extra work?
  What would a fair comparison look like?

- **Body:** “A focused tool may fix one step. Your current supplier may be able to change another. If the same handoff keeps crossing departments, compare the whole pathway. In a 25-minute founder-led walkthrough, we map the current work, show a connected proposed path using synthetic data, and name the staff and IT conditions for any parallel evaluation. You keep the decision. The first result is a clear question to test, not a promise that the hospital changes overnight.”
- **Primary copy CTA: Map my hospital's first improvement** → `/request-demo`. **Native sponsored button:** supported “Request demo” preset, if available.
- **Visual:** Two paper lanes labeled “today” and “proposed,” with one repeated arrow circled.
- **Psychological trigger:** Control over change and transparency about the test.

C. BOFU visitor who has not requested a demo - confidence.

- **Hook, first two lines:**

One patient. Too much repeat work.
Bring us the handoff that matters most.

- **Body:** “You may already know which workflow makes your team call another department, re-enter a result, or reconcile a bill. Bring that example to a 25-minute pathway demo. We will leave with a map of the current steps, one candidate improvement, and the first measure your clinical, operations, and IT colleagues could review. If the current supplier can solve it with less disruption, that belongs in the comparison too.”
- **Primary copy CTA: Map my hospital's first improvement** → `/request-demo`. **Native sponsored button:** supported “Request demo” preset, if available.
- **Visual:** Founder holds the three-column card from the video: “work / exceptions / decision.”
- **Psychological trigger:** A defensible next decision; no implied tracking of an individual's visit in the copy.

YouTube six-second bumper ads - three versions

Read each line at a natural, quick pace and cut at six seconds; caption every spoken word. These are scripts, not a recommendation to buy YouTube media with a zero budget.

Version / one emotion	Exact spoken words	On screen	Text overlay and single CTA
1. Puzzlement	“One patient. Too much repeat work. Map your first improvement.”	Blank patient card moves across order, result, bill cards.	“ONE PATIENT. TOO MUCH REPEAT WORK.” then “Map my hospital's first improvement.”
2. Relief	“Three handoffs. One patient. Map your first improvement.”	Founder circles a repeated arrow on the paper pathway.	“MAP ONE PATHWAY” then “Map my hospital's first improvement.”
3. Confidence	“Before switching, compare one path. Map your first improvement.”	Side-by-side lanes labeled “today” and “proposed.”	“COMPARE BEFORE A WIDER CHANGE” then “Map my hospital's first improvement.”

The longer on-screen CTA can remain on the final frame after the short spoken line. No bumper asserts a real customer result, a three-day production release, or an autonomous clinical action.

Retargeting ad - one lower-friction version

Channel: LinkedIn if a consented audience and enough eligible traffic ever exist; otherwise reuse as a founder post or appropriate follow-up. **Copy:** “A new hospital system is a serious decision. Start with a pencil, not a cutover plan. Draw one patient journey: who gets the order, who sees the result, who has to ask again, and what finance must reconcile. Our free one-page pathway worksheet helps you spot the first question worth testing. If it looks like a core-system problem, we can map the next step together.” **CTA: Use the patient-pathway worksheet** → proposed P01 page, only after it is built and its delivery works. **Visual:** a simple blank worksheet and three paper cards. This addresses the risk-of-change objection without saying the reader visited the website or claiming the worksheet proves MediMind wins.

Part 2 - Four-email campaign sequence

These are complete **unsent** drafts for an opted-in or otherwise permissioned audience. Source-list size, email service, and opened/clicked events are unverified. A message should only be sent when its segment and delivery are real. Subject lines are eight words or fewer; preview text is 40 characters or fewer. The product is absent from Email 2. The third email uses a *competitor customer's public review* and labels it as such; it is not MediMind social proof.

Email 1 - Campaign launch, day 1

Subject: Why does one patient create extra work?
Preview: Trace one handoff with us.
Opening line: Why does one patient create so much extra work?

Full body:

Hello [First name],

A patient can move from an order to a result to a bill while the information is still carried by calls, copied notes, and checks between teams. Each person may be doing the right thing. The handoff itself can become extra work.

We are starting with one practical question: which patient pathway does your team repeat most often? In a 25-minute founder-led walkthrough, we can draw that pathway, show a proposed connected version with synthetic data, and name one measure worth checking with your team. It is a first look, not a request to switch systems.

CTA link text: Map my hospital's first improvement

Email 2 - Value delivery, day 3, opened but did not click

Subject: Map the handoff before changing software
Preview: A five-minute pathway check.
Opening line: Here is a simple way to see where the work repeats.

Full body:

Hello [First name],

Take one recent patient journey and draw four boxes: the first request, the result, the next decision, and the financial close. Under each box, write who owns it and what information they need. Put a circle around every place someone calls, copies, waits, or checks an earlier step.

Then ask two questions: Which circle happens repeatedly? Which one has a cost or safety consequence you can actually observe? The answer may point to training, a supplier change, a narrow tool, or a wider workflow issue.

You can use the blank worksheet without sharing patient details.

Soft CTA link text: Use the pathway worksheet - proposed P01 route; replace the link only after it exists and works.

Email 3 - Process proof, day 6, opened but not converted

Subject: What a real reviewer valued
Preview: Breadth matters. So does fit.
Opening line: A hospital-software review gave us a useful counterexample.

Full body:

Hello [First name],

An ophthalmic technician described moving **to Epic** because the previous system did not provide the breadth and customization her team wanted. In her August 2026 public review, she reported easier access to notes across providers and useful tailoring after the switch. That is a competitor's success story, not a MediMind result.

It is also a good test for any new system: does it make the *whole* job easier for your staff, without taking away the parts they already value? Bring one pathway to our founder-led walkthrough. We will map the current work and the evidence needed for a fair comparison.

CTA link text: Map my hospital's first improvement

Email 4 - Truthful urgency, day 10; under 100 words

Subject: The handoff will repeat next month
Preview: Start with one measurable path.
Opening line: If the same pathway keeps creating extra work, waiting has a cost worth measuring.

Full body:

Hello [First name],

If staff repeat the same handoff next month, the calls and reconciliation will still need an owner. We cannot tell you the size of that cost without looking at your workflow. We can spend 25 minutes mapping one path and agreeing what your team should measure before any wider change.

CTA link text: Map my hospital's first improvement

No expiry date or limited place is claimed. If there is no permissioned email segment or working destination, these remain drafts.

Part 3 - Two-week social content calendar

All ten are complete LinkedIn post drafts. Times are **proposed tests in Asia/Tbilisi time**, not validated audience benchmarks; the library's Tuesday/Thursday morning rule is a generic starting point. Post only where the founder has a relevant audience and can respond. “Format” is executable with a phone, a blank card, or text. The day-8 public review is clearly a competitor story, not a MediMind outcome. The day-14 close uses process proof because a buyer-approved result is unavailable.

Day / format / test time	Opening hook	Full post copy	CTA
1 / text + paper photo / 09:00	**A patient does not create the extra work. The handoffs do.**	“A doctor orders. A result comes back. The next person still has to ask whether it arrived. Finance still needs to reconcile what happened. Each team can be doing its job while the journey between them becomes another job. Before buying another dashboard, draw one real pathway and mark every call, copied value, and wait. What would your team find?”	None.

Day / format / test time	Opening hook	Full post copy	CTA
2 / text + three-card sketch / 09:00	**Why does one patient create so much extra work?**	“Try following one patient from order to result to bill. Where does someone look up the same detail? Where does a call replace a visible handoff? Where does the bill depend on a note another team cannot see yet? This is a question for a hospital's own workflow, not a claim that every hospital has the same failure. The important number is not how many screens you own. It is how much work the next person has to recreate.”	None.
4 / sourced data card / 09:00	**Georgia counted 232 hospitals in 2025.**	“That is Geostat's national facility count, not 232 software buyers. Some facilities share a purchasing group. Each hospital has its own people, pathways, and transition limits. A useful software conversation starts with one actual handoff and its owner, not a national number on a market slide. What would you count first in your hospital?”	Question in post.
5 / campaign video / 09:00	**Why does one patient create so much extra work?**	“An order. A result. A bill. The work between those steps deserves a closer look. Our founder maps one pathway before asking anyone to consider a wider system change. [Watch the synthetic-data walkthrough.]”	“Map my hospital's first improvement” → verified demo route, only after request receipt works.

Day / format / test time	Opening hook	Full post copy	CTA
7 / paper diagram carousel / 09:00	**One shared path starts with one honest map.**	“Choose one patient journey. Mark each person who needs to act. Mark the information each person needs. Circle where the same fact is entered twice or someone calls to ask what happened. Then put a proposed path beside today's path and count the work on both sides, including the extra work of a parallel test. A new system is worth discussing only if the comparison holds up for your people. That is the method behind MediMind's first-pathway demo.”	None.
8 / text + source link / 09:00	**A competitor review that made us pause.**	“A technician wrote that her organization moved **to Epic** seeking a broader, more customizable record. After the switch, she valued access to notes across providers (Capterra, 18 August 2026). That is real praise for an established competitor. The lesson for us: a hospital must keep the useful parts of today's system while fixing the handoff it cannot live with. A fair evaluation asks staff to name both.”	None.
9 / text / 09:00	**Myth: a new interface fixes the handoff.**	“A cleaner screen can help. It cannot by itself resolve unclear ownership, missing data, a payer rule, or a staff process that does not match the screen. Even broad EHRs can have task friction; their staff reviews also praise shared records. The test is specific: follow a patient pathway, compare all steps, and include training and exception work. If the current supplier can solve it with less disruption, that should count.”	None.

Day / format / test time	Opening hook	Full post copy	CTA
11 / synthetic micro-story / 09:00	**Imagine a result arriving at the wrong moment.**	“This is a fictional example, with no patient data: A lab result is ready. The clinician needs it for the next decision. The coordinator calls to confirm it is there. Finance later checks which service was completed. In one pathway review, the team can ask whether the result, task, and billing record move together or whether people bridge the gaps. The first improvement is the one staff can inspect and measure.”	None.
12 / text + side-by-side sketch / 09:00	**The best reason to wait is safety.**	“A hospital cannot stop for a software experiment. A serious parallel evaluation therefore needs an agreed pathway, an owner, authorized data or synthetic data, clinical and IT review, a baseline, and a stop rule. MediMind's two-week tailored-evaluation target depends on readiness; production change needs separate sign-off. Ask any vendor for the work your team must do and the conditions for saying no.”	None.
14 / founder photo + process card / 09:00	**Here is what 25 minutes should produce.**	“Bring one handoff your hospital keeps repeating. We will draw the current path, sketch a connected option, and name the first measure and owners for a fair test. You will know what MediMind can show now, what needs more work, and which questions your clinical and IT team should ask before a wider change.”	**Map my hospital's first improvement →** verified demo route. No deadline.

The ten posts form a **library and recommended sequence**, not a mandate to create ten new media assets inside two five-hour weeks. Reuse the same paper-card photo/video and schedule text drafts; if time is tight, select the highest-fit posts in the Part 5 calendar. No social proof count or clinical outcome is fabricated to satisfy a posting cadence.

Part 4 - Campaign destination page: precise edits to existing `/request-demo`

Source inspected: RequestDemo.tsx. The page is not a verified lead-receipt path. These are **copy and behavior instructions**, not implemented site changes. They must be coordinated with the full funnel-page copy and the product owner before publication. The source library's claimed conversion lifts for shorter forms, proof, grade-level text, or video are not independently established here; early hospital traffic will be too small for precise rate claims. Count valid received requests and held qualified demos first.

Page location	Current source copy / behavior	New copy or required change	Why
Eyebrow and headline	“Enterprise Demo Request”; “See MediMind on your hospital's workflows”	“FOR GEORGIAN HOSPITAL LEADERS”; **“Map your first patient-pathway improvement”**	Match the video promise and identify the right buyer.
Subheadline	“30 minutes. One login, one chart, one patient - live. We'll tailor the walkthrough to your hospital.”	“In 25 minutes with our founder, trace one recurring handoff and choose what your team should measure before a wider change.”	Align duration and deliverable; avoid “live” implying a customer environment.
Above form	Three-step progress and form starts with “Personal Information”	“Bring one handoff your team repeats. No patient details needed.” Then one short form.	Explain the task and lower the first-step burden.
Required fields	First/last name, email, company name and size, industry, job title, facility type, provider count; phone optional	Required: name, work email, hospital/group name. Optional: “Which patient pathway creates repeat work?” No patient identifiers.	Ask only what is needed to route a hospital conversation.
Submit button	“Request Demo”	**“Map my hospital's first improvement”**	Exact campaign CTA.
Success state	“We've received your demo request… contact you within 24 hours”; repeated 24-hour promise	Only after real receipt: “Your request was sent. Our team will review it and contact you to arrange the 25-minute pathway demo.” If send fails: “We could not send your request. Please try again or use the verified contact option.”	Current timer/console log is not receipt; no confirmed response-time SLA.

Page location	Current source copy / behavior	New copy or required change	Why
Trust badges	“Live in 25 hospitals”; government startup badge	Remove the live-site claim until accepted scope and permission are checked. Keep startup status only after current documentation and approved wording are verified. Use “Founder-led review of one pathway” as process reassurance.	Contracted network is not proof every facility is live; trust should rest on accurate evidence.
Alongside CTA	No approved measured customer story in the supplied evidence	“You will leave with a pathway map, a first measure, and the questions your clinical and IT leads should review.” Reserve quote/logo slots until documented approval.	Provides useful proof of process without inventing testimonials.
Form plumbing	`handleSubmit` waits, logs, then displays success	Implement durable lead receipt, ownership notification, error handling, and end-to-end test before public links.	A clicked CTA is not a received lead. Engineering time is outside the marketing schedule.

Do not add an embedded calendar until availability, owner, and booking integration are real. If it becomes available, place it directly on the page and confirm timezone and confirmation delivery. The current form should not promise a calendar that is not there.

Part 5 - 14-day launch plan within the shared five-hour weekly budget

When this clock starts: these are campaign days, not a second company-wide work plan. Start day 1 only after the required destinations and receipt checks work- normally no earlier than implementation week 3. During these two weeks, this sprint replaces the ordinary creative/outreach slots in Prompts 2 and 5; it is not added to them. If the sprint is not launched, continue the ordinary 30-day plan. All ten posts are drafted, so reuse the single filmed card sequence instead of producing ten new assets.

The plan uses **300 marketing minutes per week**: 25 minutes of brief daily checks across the week, 30 minutes of weekly review/report, 30 minutes reserved toward the monthly deep dive, and **215 minutes of actual campaign/distribution work**. In a monthly-report week the full 120-minute review replaces the 30-minute reserve and **90 minutes of execution must be cut**. Live demo calls, site engineering, video post-production beyond basic phone editing, and delivery evaluations need additional named capacity. No paid spend is assumed. “Publish/send” below is conditional on a working destination, permissioned audience, and content approval; holding an asset is a valid action when a gate fails.

Day	Campaign work and asset used	Marketing minutes	Publish/send and end-of-day measure	If weaker than the practical gate
1	Verify deployed `/request-demo` submission/receipt with owner; mark copy edits for engineer. Prepare Email 1 and Day 1 post.	35	Post Day 1 only if relevant founder channel exists. Record unique relevant replies and form-test result.	If receipt fails, hold all destination links; use no false success claim.
2	Share the pathway worksheet draft with founder for one useful example; publish Day 2 text using same card photo.	25	Record qualified comments or replies, not reach alone.	If readers discuss other pains, capture exact themes for research.
3	Prepare Email 2 and permissioned segment; draft neutral follow-up to interested replies.	25	Send only if opted-in list and working P01 exist; otherwise keep draft. Log delivery/clicks if sent.	If no list/page, use worksheet in a permissioned one-to-one conversation.
4	Source-check and schedule Day 4 Geostat data card; film main card sequence in one phone take if founder available.	45	Post data question; log relevant replies and source/caption QA.	If filming needs a production crew, use still cards rather than adding time.
5	Trim/caption the existing phone clip; schedule Day 5 caption and copy-check `/request-demo`.	45	Publish video only after receipt verified; count accepted requests, qualified bookings, and video questions separately.	If no receipt, post educational clip without an inactive CTA link or hold it.
6	Prepare Email 3 with source link; queue warm ad B as an organic text variant, not paid media.	20	Send only to actual permissioned opened/non-converted segment. Track real replies.	If open data is unavailable, do not infer the segment; keep draft.
7	Publish mechanism post from prepared text; review the first week's distinct hospital conversations.	20	Count held qualified demos and named next steps (N/A rate if denominator zero).	If no qualified interest, inspect route and audience fit before changing creative.

Day	Campaign work and asset used	Marketing minutes	Publish/send and end-of-day measure	If weaker than the practical gate
	Week 1 campaign execution	**215**	**+85 baseline = 300 minutes total.**	
8	Publish sourced competitor-review story; use already checked R6 paraphrase.	30	Record relevant questions or corrections.	If audience thinks it is MediMind proof, clarify visibly and revise.
9	Publish myth-bust text and prepare one three-option follow-up note.	25	Track replies naming a current remedy.	If none name a real pathway, ask for one incident in the next conversation.
10	Prepare/send Email 4 only to real permissioned segment; align any booked-demo preparation card.	25	Record delivered emails and accepted requests separately.	If no segment, do not send; use time for a named-account research note.
11	Publish the synthetic micro-story with explicit fictional label.	25	Count responses that describe a real workflow.	If synthetic scene is misunderstood, simplify labels and wording.
12	Publish the transition-risk post; audit one actual buyer question with clinical/IT counterpart.	30	Record safety objections and any qualified demo scheduled.	If concern is supplier trust, prepare verified scope/support answers before more promotion.
13	Inspect visit-to-received-request handoff, source attribution, and held-demo log; use prepared copy only.	40	Compare counts of relevant visitors, accepted requests, booked demos, held qualified demos.	If requests are zero, first prove submission works; avoid percentage theater.
14	Publish honest process-proof close; write two-week learning note and decide next asset.	40	Cumulative held qualified hospital demos and readiness-approved evaluations; count distinct buying organizations.	If qualified demos are absent after a working route and genuine reach, retest the hook and target-account selection.

Day	Campaign work and asset used	Marketing minutes	Publish/send and end-of-day measure	If weaker than the practical gate
	Week 2 campaign execution	**215**	**+85 baseline = 300 minutes total.**	

By day 7, three useful signals: (1) at least one valid, received request or verified direct founder reply from a relevant hospital; (2) at least one qualified booking with a named pathway and buyer role; (3) one held qualified demo or a dated, owned path to it. These are provisional gates, not statistical benchmarks. If none occurs, diagnose route receipt and qualified reach before declaring the concept wrong.

Day-14 budget-increase metric: fully loaded cost per **held qualified demo from a distinct new buying organization**, counting founder marketing time at an agreed internal hourly cost and any paid spend. If no qualified demo is held, the rate is **N/A** because its denominator is zero. Paid-only acquisition cost is **N/A** while no paid campaign runs. No increase is automatic. Consider a small paid test only after a working destination, approved budget, consent/measurement setup, a qualified-demo signal from organic distribution, and enough capacity to answer leads. The north star remains cumulative held qualified hospital demos toward the 90-day planning scenario of 12.

Concept-wrong signal: Several independent, relevant hospital operators can describe recent handoff problems but consistently say the cost is minor, the incumbent or narrow tool resolves them, and a first-pathway comparison is not worth an executive/committee conversation. One weak post or a broken form does not disprove the concept.

Sources and handoff to Prompt 4

Campaign source: Prompt 3A script. Position and evidence boundaries: avatar, problem stack, review corpus, funnel, traffic routine, and current demo source. The Geostat 2025 hospital release supplies the Day 4 facility count. Capterra's Taylor C. Epic review supplies the clearly labeled competitor story; it does not support MediMind outcomes. Prompt 4 must give `/request-demo` an actual received-request path and must keep optional worksheet, webinar, and sandbox pages distinct from the two-week parallel evaluation.

14 Part 2 · Prompt 4

Build Your Funnel Pages

Source: Complete library, pages 42-45 · 7,436 words · 4 requested steps / sections

> **Status and inputs, 23 September 2026.** Complete page-copy and wiring specification, **not implemented website work**. The 85-item content catalog from Prompt 1 is the source of content-piece IDs, channels, CTAs, and destinations. The provisional buyer is a Georgian private-hospital CEO/owner/group operator; the campaign is **"One patient. Too much repeat work."** with the question **"Why does one patient create so much extra work?"** and the CTA **"Map my hospital's first improvement."** The first sale step is a recommended 25-minute founder-led pathway demo and a possible measured, readiness-dependent parallel evaluation. There are no approved customer testimonials, outcome numbers, or logos in this edition. The site homepage `/` and `/request-demo` route exist, but the catalog's proposed product, proof, worksheet, offer, comparison, activation, and referral destinations do not yet exist as verified routes. Source inspection of `/request-demo` found a simulated submission rather than durable lead receipt. Do not publish campaign links to unbuilt routes or claim that a form request is received until tested.

Conversion-rate note. The source library states broad SaaS uplift/rate claims for clear headlines, short forms, proof, video, and calendars without a verifiable source in the supplied text. They are design prompts, not MediMind benchmarks. Every page below names its actual conversion event and a practical **pre-launch gate**. With tiny hospital volumes, report counts and N/A when a denominator is zero; do not optimize to a borrowed percentage. A calendar should only be embedded when it really books, respects local time, and confirms an owner. A form should only promise a privacy or response practice after operations adopts it.

Step 1 - Map every content piece to its destination

Route key. P01 `/pathway-assessment` = awareness worksheet and five-minute check; P02 `/hospital-os` = product overview; P03 `/case-study` = verified case study, held pending evidence; P04 `/webinar` = optional webinar, unscheduled; P05 `/request-demo` = existing demo route needing repair; P06 `/early-partner` = offer and evaluation; P07 `/compare` = alternatives comparison; P08 `/activation` = private activation; P09 `/referrals` = private advocacy; P10 `/evidence` = evidence method and later approved stories; P11 `/sandbox` = optional guided synthetic-data trial, unlaunched. Only P05 currently corresponds to a catalog destination already in the site. P04 and P11 are included because the separate page library asks for those page types; no content-catalog item points to them yet.

The table is the **complete 85-piece map**. The CTA text is the action from the catalog; the destination key resolves to the route/type above. `No` means the proposed destination is not verified as live; `Yes, repair` applies only to P05. The page-action map in Part 2 supplies the next step after landing, so each row forms a complete content → page → action flow.

Content piece (ID and name)	Stage	CTA action → destination/type	Exists?
T01 One patient, seven handoffs	TOFU	Map my hospital's first improvement → P01 worksheet	No
T02 The hidden cost of searching	TOFU	Use the pathway worksheet → P01 worksheet	No
T03 When two reports disagree	TOFU	Get the reconciliation checklist → P01 worksheet	No
T04 The duplicate-entry trail	TOFU	Map my hospital's first improvement → P01 worksheet	No
T05 Three calls for one result	TOFU	See the pathway map → P01 worksheet	No
T06 The spreadsheet behind the dashboard	TOFU	Get the checklist → P01 worksheet	No
T07 How to map a patient handoff	TOFU	Download the worksheet → P01 worksheet	No
T08 Why hospital reports disagree	TOFU	Get the reconciliation checklist → P01 worksheet	No
T09 What a delayed claim does and does not mean	TOFU	Map one claim pathway → P01 worksheet	No
T10 Patient-pathway friction worksheet	TOFU	Download the worksheet → P01 worksheet	No
T11 Five-minute handoff check	TOFU	Run the check → P01 worksheet/check	No
T12 Founder-built scope trace	TOFU	See the operating system → P02 product	No
T13 Synthetic handoff before/after	TOFU	See the pathway map → P02 product	No
T14 Where the work repeats	TOFU	Use the pathway worksheet → P01 worksheet	No
T15 A report you had to rebuild	TOFU	Get the checklist → P01 worksheet	No
T16 Hospital-operator discussion prompt	TOFU	Read the handoff guide → P01 worksheet	No
T17 Clinical-operations partner note	TOFU	Download the worksheet → P01 worksheet	No

Content piece (ID and name)	Stage	CTA action → destination/type	Exists?
M01 Fix a screen or fix the pathway?	MOFU	Compare the approaches → P07 comparison	No
M02 What parallel evaluation actually requires	MOFU	See the evaluation plan → P06 offer	No
M03 AI assists; clinicians decide	MOFU	See how it works → P02 product	No
M04 Three options on one whiteboard	MOFU	Compare the approaches → P07 comparison	No
M05 What happens in 25 minutes	MOFU	Request a pathway demo → P05 demo	Yes, repair
M06 A result becomes a task	MOFU	See the operating system → P02 product	No
M07 How to evaluate hospital software without a risky cutover	MOFU	Get the evaluation plan → P06 offer	No
M08 Hospital OS versus overlay versus current vendor change	MOFU	Compare the approaches → P07 comparison	No
M09 What FHIR R4 changes in hospital work	MOFU	See the product path → P02 product	No
M10 Parallel-evaluation readiness checklist	MOFU	Download readiness checklist → P06 offer	No
M11 Three-option decision worksheet	MOFU	Compare the approaches → P07 comparison	No
M12 Documented domain map	MOFU	Inspect product scope → P10 evidence	No
M13 Live-pathway evidence template	MOFU	See the evidence method → P10 evidence	No
M14 The change-risk question	MOFU	See the evaluation plan → P06 offer	No
M15 The current-vendor alternative	MOFU	Compare the approaches → P07 comparison	No
M16 Peer discussion on evaluation design	MOFU	Get the readiness checklist → P06 offer	No
M17 IT and clinical review session	MOFU	See the evaluation plan → P06 offer	No

Content piece (ID and name)	Stage	CTA action → destination/type	Exists?
B01 What your 25-minute demo will settle	BOFU	Request a pathway demo → P05 demo	Yes, repair
B02 Match price, define total cost	BOFU	Discuss the early-partner offer → P06 offer	No
B03 The honest go/no-go test	BOFU	Request a pathway demo → P05 demo	Yes, repair
B04 Demo in sixty seconds	BOFU	Request a pathway demo → P05 demo	Yes, repair
B05 Parallel beside current work	BOFU	Discuss an evaluation → P06 offer	No
B06 Who should join the first call?	BOFU	Request a pathway demo → P05 demo	Yes, repair
B07 Questions to ask before a hospital OS demo	BOFU	Request a pathway demo → P05 demo	Yes, repair
B08 What happens after a parallel evaluation	BOFU	See the evaluation plan → P06 offer	No
B09 What current-software price match should include	BOFU	Discuss the early-partner offer → P06 offer	No
B10 Pathway demo preparation card	BOFU	Prepare my demo → P05 demo	Yes, repair
B11 Evaluation decision sheet	BOFU	Discuss an evaluation → P06 offer	No
B12 Verified implementation timeline	BOFU	Review verified evidence → P03 case study	No; evidence held
B13 Buyer-approved workflow result	BOFU	See the evidence method → P10 evidence	No; result held
B14 Your pathway before our demo	BOFU	Prepare my demo → P05 demo	Yes, repair
B15 What we will compare	BOFU	Review the evaluation plan → P06 offer	No
B16 Private buying-committee walkthrough	BOFU	Request a pathway demo → P05 demo	Yes, repair
B17 Procurement questions session	BOFU	Discuss the early-partner offer → P06 offer	No
A01 First pathway, first baseline	AFU	Review the activation plan → P08 activation	No, private

Content piece (ID and name)	Stage	CTA action → destination/type	Exists?
A02 What staff should report in week one	AFU	Use the activation checklist → P08 activation	No, private
A03 One improvement, then the next	AFU	Review next-pathway options → P08 activation	No, private
A04 How to log a handoff issue	AFU	Open the activation guide → P08 activation	No, private
A05 Five-minute pathway review	AFU	Review the activation plan → P08 activation	No, private
A06 How a testable change becomes approved	AFU	Use the change checklist → P08 activation	No, private
A07 Measuring repeat work without double counting	AFU	Use the measurement sheet → P08 activation	No, private
A08 Running a safe pathway review	AFU	Open the activation guide → P08 activation	No, private
A09 Selecting the second pathway	AFU	Review next-pathway options → P08 activation	No, private
A10 First-pathway measurement sheet	AFU	Download the measurement sheet → P08 activation	No, private
A11 Change-request decision card	AFU	Use the change checklist → P08 activation	No, private
A12 Accepted-workflow log	AFU	Review the activation plan → P08 activation	No, private
A13 Measured improvement record	AFU	Review the measurement sheet → P08 activation	No, private
A14 Your first review agenda	AFU	Review the activation plan → P08 activation	No, private
A15 A request ready for test	AFU	Use the change checklist → P08 activation	No, private
A16 Clinical-IT issue huddle	AFU	Open the activation guide → P08 activation	No, private
A17 Cross-site learning exchange	AFU	Review next-pathway options → P08 activation	No, private
L01 The buyer's permission comes first	LOFU	See approved stories → P10 evidence	No; proof gated
L02 A pathway another leader can inspect	LOFU	Read the case study → P03 case study	No; evidence held

Content piece (ID and name)	Stage	CTA action → destination/type	Exists?
L03 What we changed after feedback	LOFU	See approved stories → P10 evidence	No; proof gated
L04 Customer pathway walkthrough	LOFU	Read the case study → P03 case study	No; evidence held
L05 What a reference actually verified	LOFU	See approved stories → P10 evidence	No; proof gated
L06 Why we chose the first pathway	LOFU	Read the case study → P03 case study	No; evidence held
L07 How to read a hospital software case study	LOFU	See approved stories → P10 evidence	No; proof gated
L08 From one hospital pathway to another	LOFU	Read the case study → P03 case study	No; evidence held
L09 What changed in a claim handoff	LOFU	Read the case study → P03 case study	No; evidence held
L10 Reference-story evidence checklist	LOFU	Download the checklist → P10 evidence	No; proof gated
L11 Peer evaluation guide	LOFU	Get the guide → P09 referral	No, private
L12 Buyer-approved measured case study	LOFU	Read the case study → P03 case study	No; evidence held
L13 Reference conversation	LOFU	Request a reference → P09 referral	No, private
L14 Share the method, not a claim	LOFU	Review the story request → P09 referral	No, private
L15 A peer may have this problem	LOFU	Share the approved case study → P03 case study	No; evidence held
L16 Peer operator exchange	LOFU	Request a reference → P09 referral	No, private
L17 Hospital-association learning session	LOFU	See approved stories → P10 evidence	No; proof gated

Step 2 - Identify which pages need to be built and edit what exists

I will now build the following page-copy specifications: P01 worksheet; P02 product overview; P03 case-study shell held for evidence; P06 evaluation/offer; P07 alternatives comparison; P08 private activation; P09 private referral; P10 evidence hub. P05 is an existing route with edits below. P04 webinar and P11 guided sandbox are

optional copy-ready additions from the separate page library, **not current campaign destinations**. No code or site route is changed by this document.

Page	Visitor/job	Required state before promotion	Conversion event to count
P01 worksheet	TOFU visitor identifies a first pathway and receives a useful worksheet	Page, actual asset delivery, consent text, and analytics verified	Worksheet received; subsequent qualified demo counted separately
P02 product	TOFU/MOFU visitor understands mechanism and boundaries	Product scope and screenshots checked against accepted current capability	Click to demo from a relevant visitor
P03 case study	BOFU/LOFU reader evaluates a real customer result	Customer permission, baseline, method, after measure, limitations, and quote approval	Qualified demo or reference request; **hold until evidence exists**
P05 demo	MOFU/BOFU visitor requests a founder-led pathway conversation	Actual lead persistence/notification, owner, error handling, and tested confirmation	Valid received request → booked → held qualified demo
P06 offer	MOFU/BOFU buyer understands evaluation and commercial questions	Founder-defined terms and readiness boundaries confirmed per proposal	Demo request or readiness discussion
P07 comparison	MOFU buyer chooses among current supplier, narrow tool, and MediMind	Fair current information; no unsupported competitor weakness	Demo request with named alternative
P08 activation	AFU customer starts accepted pathway work	Private access, named owner, agreed scope/data and safety plan	First accepted task/baseline recorded
P09 referral	LOFU customer chooses whether to introduce a peer	Permission, customer satisfaction, no implied incentive	Permissioned introduction; no unsolicited send
P10 evidence	MOFU/BOFU/LOFU reader sees what is documented and pending	Accurate scope statuses and separate public approvals	Evidence inspection → qualified demo; approved story views separately
P04 optional webinar	MOFU attendee registers for a real practical session	Actual date, host, platform, reminder/recording policy	Real registration; **N/A while unscheduled**
P11 optional sandbox	BOFU evaluator requests a guided synthetic-data trial	Working safe environment, owner, scope, terms, data handling	Trial accepted and first task completed; **N/A while unlaunched**

Existing P05 `/request-demo`: precise edit instructions

The inspected RequestDemo.tsx currently displays “Enterprise Demo Request,” “See MediMind on your hospital's workflows,” a “30 minutes” subheadline, a three-step form with nine required fields, “Request Demo,” a timer/console-log success path, a 24-hour contact promise, and “Live in 25 hospitals.” These are source observations, not a deployed-browser audit.

Location	Current copy/behavior	Replacement	Reason
Eyebrow and H1	“Enterprise Demo Request”; “See MediMind on your hospital's workflows”	“FOR GEORGIAN HOSPITAL LEADERS”; “Map your first patient-pathway improvement”	Connect the campaign promise to the landing page.
Subhead	“30 minutes. One login, one chart, one patient - live…”	“In 25 minutes with our founder, trace one recurring handoff and choose what your team should measure before a wider change.”	Match the offered call and avoid implying a real patient's live record.
Form introduction	Three stages headed Personal, Company, Healthcare	“Bring one handoff your team repeats. No patient details needed.” One compact step.	Keep focus on the buyer's job.
Fields	Nine required fields across three steps, phone optional	Required: name, work email, hospital/group. Optional: one-line pathway question.	Enough to route a relevant conversation without collecting unnecessary data.
Submit	“Request Demo”	“Map my hospital's first improvement”	Exact campaign CTA.
Success/error	Simulated success and 24-hour contact promise	Real receipt success: “Your request was sent. We will contact you to arrange the 25-minute pathway demo.” Failure: “We could not send your request. Please try again or use the verified contact option.”	Never say a lead was received unless it was delivered; no unconfirmed response SLA.
Trust and proof	“Live in 25 hospitals”; government status badge	Remove live-site count pending verified accepted scope and public approval. Use specific process copy below. Retain startup badge only after wording/documentation check.	A signed network agreement does not establish all-site go-live.

Final consolidated P05 demo page copy (publish only with repaired behavior):

Eyebrow: For Georgian hospital leaders
Headline: Map your first patient-pathway improvement
Subheadline: In 25 minutes with our founder, trace one recurring handoff and choose what your team should measure before a wider change.
Hero body: Bring one moment where information had to move between clinical, operations, and finance teams. We will follow the work together using a synthetic example and mark what your hospital would need to check.
What you will see: (1) a pathway map from the first task to the next responsible person; (2) a proposed connected version beside today's work; (3) a first measure, owners, and readiness questions for a possible parallel comparison.
Form headline: Which handoff should we map?
Fields: Name; work email; hospital/group; optional one-line pathway. Do not enter patient information.
Button: Map my hospital's first improvement
What happens next: After a successful request, our team contacts you to arrange the founder-led call. No meeting is booked by merely submitting this form.
Process proof alongside form: A physician founder will map one pathway with you. The first output is a question your clinical, operations, and IT colleagues can inspect-not a claim of measured hospital results.
Closing CTA: Bring one repeated handoff. Leave with a first improvement worth testing. **Map my hospital's first improvement.**

If an actual calendar integration is introduced, its slots, timezone, confirmation, and ownership must be tested before the copy says “book.” Customer-quote, result, or logo columns remain empty until permitted. The primary conversion is a **held qualified demo**, not a displayed success screen.

P01 `/pathway-assessment` - lead capture for T01-T11, T14-T17 / TOFU

Visitor arrives from: founder LinkedIn/video, a guide, a permissioned email, or a professional discussion. **Job:** identify one repeated patient-workflow handoff. **Conversion event:** successful worksheet delivery; do not infer purchase intent.

Headline (7 words): Find the handoff your team repeats
Subheadline: A five-minute worksheet to trace one patient journey, mark avoidable handoffs, and choose a first question to measure.
Hero body: Start with an actual recent pathway, but do not enter patient details. The worksheet makes the work visible before anyone argues for a new system.
What you get: (1) four boxes for request, result, next decision, and financial close; (2) a place to mark calls, copied facts, waits, and owners; (3) one question for your clinical and operations leads to test.
Form headline: Send me the pathway worksheet
Fields: First name; work email.
Button: Send the pathway worksheet
What happens next: We email the blank worksheet and show an immediate download link after successful delivery.
Privacy copy, proposed pending policy review: We use your work email to deliver the worksheet and respond to your request. Separate opt-in is required for campaign emails. Do not include patient information.
Optional next step after value is delivered: If a recurring handoff matters to your hospital, **map my hospital's first improvement** in a 25-minute founder-led call.

Actual worksheet body, ready to place in a one-page PDF or on-page form: “Patient pathway: _____ (describe the kind of journey, not a person). Date observed: _____. (1) First request: who creates it, where is it recorded, and who needs it next? (2) Result or response: where does it arrive, who notices, and what happens if it is late? (3) Next clinical/operations decision: what facts must be visible, who confirms them, and what gets re-entered? (4) Financial/administrative close: what must reconcile, what does staff check manually, and what remains unknown? Circle each call, copied fact, wait, or recheck. Count the circles; note the staff time and any exception you can verify. Ask: which circle repeats enough to be worth fixing, and could the current supplier or a narrow change remove it?” This is a worksheet, not a diagnostic score. The P01 five-minute check uses these four boxes and returns the chosen pathway, not an invented maturity grade.

P02 `/hospital-os` - product overview for T12-T13, M03, M06, M09 / TOFU-MOFU

Visitor arrives from: a synthetic pathway visual, a mechanism post, or a FHIR explainer. **Job:** understand what MediMind does today, what needs local setup, and what is future direction. **Conversion event:** qualified move to a demo request.

> **Eyebrow:** One hospital pathway at a time
> **Headline:** Clinical and business work, in one path
> **Subheadline:** MediMind is a Georgian hospital operating system built around a shared FHIR R4 record and workflows that cross departments.
> **Opening:** A patient journey does not end at a chart. An order, result, task, medicine, bill, and report may pass through different teams. MediMind gives those teams a shared foundation so the next responsible person can work with relevant context. What the system can demonstrate or deploy at your hospital depends on the agreed modules, interfaces, data, and review.
> **How it works:** (1) Map the current pathway and people who own each step. (2) Show a proposed connected workflow using synthetic data. (3) Set a baseline and compare, with clinical and IT sign-off before any production change.
> **What FHIR R4 means:** It is a common structure for health information-like agreed labels on every handoff. It helps information move and be understood, but does not automatically solve interface, data-quality, or training work.
> **AI's role:** Defined assistance can help draft or surface information for human review. The future direction is broader coordination; no autonomous clinical decision or guaranteed patient outcome is claimed.
> **Scope note:** Clinical and business domains are described in the product map; ask which are demonstrated, configured, accepted, or planned for your institution. External payers, ministries, devices, and other systems still require appropriate interfaces.
> **Proof panel:** Founder-authored Medplum implementation account describes the architecture and a signed anchor-network agreement. It is not an independently measured outcome study or a statement that every facility is live.
> **CTA:** Bring one handoff; **map my hospital's first improvement** → P05.

Visual placement: Use the campaign's paper-card path or a clearly synthetic product screen with current capability labels. A customer logo or result card is held until documented permission and measurement.

P03 `/case-study` - verified case study for B12, L02, L04, L06, L08, L09, L12, L15 / BOFU-LOFU

Visitor arrives from: a verified implementation timeline, approved pathway walkthrough, or permissioned referral. **Job:** inspect a real before/after result. **Conversion event:** qualified demo or approved reference request. **Publication state: hold** until customer approval and the fields below are filled with evidence. A fabricated case is

not a usable page.

> **Working headline slot (under 10 words):** How [hospital] improved [verified pathway measure]
> **Subheadline slot:** [Hospital and team] compared [defined workflow] over [period], including training, exceptions, and total staff effort.
> **Before:** [Buyer-approved description of the actual workflow, systems, and baseline; date and denominator.]
> **Moment that made them act:** [Documented event or decision, in approved language; do not create a patient case.]
> **What changed:** [Accepted MediMind scope, interfaces, staff training, parallel work, and who signed off.]
> **After:** [Measured process and/or financial outcome, same unit as baseline, limitations, and what else changed.]
> **Pull quote:** [Exact customer-approved quotation, name/title/hospital, with publication consent. If absent, omit the block.]
> **Three result numbers:** [baseline], [after], [net change]-each with unit, dates, denominator, and method. These are **fields to fill**, not numbers to publish.
> **Social-proof bar:** [approved name/logo] · [accepted scope] · [measurement period]. Omit until approved.
> **CTA after results:** See what a comparable first-pathway test would require at your hospital. **Map my hospital's first improvement** → P05.

Useful live alternative until case evidence exists: On P10, publish “How we will measure a first pathway” with blank baseline/after fields and a synthetic example explicitly labeled synthetic. Do **not** publish P03 with bracketed placeholders and call it a customer case.

P06 `/early-partner` - offer and readiness for M02, M07, M10, M14, M16-M17, B02, B05, B08-B09, B11, B15, B17 / MOFU-BOFU

Visitor arrives from: a risk/change article, preparation card, demo follow-up, or price-match question. **Job:** see the evaluation process, commercial offer, and unresolved terms without hidden promises. **Conversion event:** a qualified demo or a named evaluation-readiness discussion.

> **Headline (7 words):** Do not switch on a promise alone
> **Subheadline:** Compare one agreed patient pathway beside current work, measure the full effort, and decide with clinical and IT owners.
> **The offer:** For eligible early partners, the founder proposes matching current software pricing and including continuing development. Requested features are proposed to be testable in three business days; a tailored parallel evaluation is proposed within two weeks when prerequisites are met. These are commercial and delivery intentions requiring written scope. Testing a feature is separate from clinical validation and production release; a parallel evaluation is separate from a full cutover.
> **What is included in the first discussion:** (1) map one current pathway; (2) identify information and people needed; (3) show a relevant synthetic workflow; (4) select a first process measure; (5) list access and integration prerequisites; (6) name clinical/IT review and stop conditions.
> **Risk reversal:** Your current operation keeps running during a properly scoped parallel evaluation. Your team reviews the comparison and can choose go, adjust, or stop under the agreed plan. There is still staff work, data work, and possible cost; we will itemize it.

> **Trust/proof:** See the method and exact accepted scope for a proposed evaluation. Customer quotes and measurable outcomes will appear only with source records and permission.
> **CTA: Map my hospital's first improvement** → P05.

Five buyer FAQs:

1. **"Do we have to replace everything to start?"** No. Start with a chosen pathway and decide wider scope after an agreed comparison. The eventual replacement and external-interface map is hospital-specific.
2. **"Does two weeks mean production go-live?"** No. It is the founder's readiness-dependent target for a tailored parallel evaluation. Data access, owners, interfaces, and acceptance criteria must be agreed; production sign-off is separate.
3. **"Are all requested features live in three business days?"** The founder's proposed target is a **testable** change, subject to defined size and prerequisites. Clinical, security, and production approvals take their own review.
4. **"Does matched price cover every cost and future AI?"** The matched current software price is an early-partner offer; total cost, term, included development, implementation, interfaces, support, and advanced AI must be specified in the proposal. No lifetime lock or blanket inclusion is established.
5. **"What if the current supplier can solve our problem?"** Compare that solution on the same pathway, including risk, time, total cost, and observed work. MediMind should win only where the hospital can see a better route.

Actual readiness checklist content: "Name the pathway and one recent instance without patient identifiers; name executive, clinical/operations, and IT owners; list required source systems and interfaces; agree whether synthetic or authorized real data may be used; record baseline task count/time/errors and any financial metric with its definition; estimate hospital and MediMind staff effort; define privacy/security and clinical-review requirements; define the go/adjust/stop criteria and who signs off." This is the content for M10 and the P06 download. Do not make it a medical checklist or claim regulatory compliance.

P07 `/compare` - comparison for M01, M04, M08, M11, M15 / MOFU

Visitor arrives from: three-option guide or current-vendor question. **Job:** see when a narrow fix or incumbent is better, and when a core comparison is warranted. **Conversion event:** demo request with named current alternative.

> **Headline:** MediMind or your current hospital software?
> **Subheadline:** Compare the same patient pathway across three real options: change the current system, add a focused tool, or evaluate a shared operating system.
> **Opening:** The lowest-risk answer may be to keep what works. Start with the problem, the current supplier's proposed fix, and the work your staff still must do. This page is a decision guide, not a claim that every competitor is deficient.

Buyer question	Current supplier change	Focused add-on/integration	MediMind operating-system evaluation
Can it address **this whole pathway**?	Often plausible; inspect proposal and installed version.	Strong when pain sits in one step; may leave handoffs.	Intended to cover clinical/business work; inspect accepted scope.

Buyer question	Current supplier change	Focused add-on/integration	MediMind operating-system evaluation
How much **change** does staff face?	Usually less core migration.	New tool, interface, and training still add work.	Larger potential transition; parallel plan and staff effort must be explicit.
Who owns **local adaptation**?	Existing vendor and hospital under contract.	Vendor, integrator, and current-system team may share it.	MediMind proposes continuing development; capacity and terms need agreement.
What is the **total cost**?	Quote and internal staff time needed.	License, integration, support, and maintenance needed.	Price-match offer does not cover unknown transition/optional costs; obtain proposal.
What **proof** is available?	Current staff experience and a proposed fix.	Pilot or site evidence if provided.	First scoped pathway comparison; independent measured outcomes pending.

> **Honest assessment:** MEDWORK publicly offers Georgian hospital modules and government integrations; Epic, InterSystems, and Oracle also offer broad EHR and AI capabilities. A current supplier may be the right answer for a narrow, fixable issue. MediMind is worth evaluating where the problem follows the patient across teams and a shared workflow can be shown with acceptable risk.
> **Migration-story slot:** [No customer-approved migration story exists; do not publish a before/after anecdote.] Instead show the process: map → readiness → parallel comparison → measure → clinical/IT go/adjust/stop decision.
> **CTA:** Bring the current option and one recent handoff. **Map my hospital's first improvement** → P05.

Comparison claims are grounded in official vendor pages, InterSystems, Epic, and Oracle, with nuance detailed in Prompt 5. No head-to-head performance, Georgian availability, or competitor price is asserted.

P08 `/activation` - private activation for A01-A17 / AFU

Visitor arrives from: an accepted evaluation invitation or authenticated customer update. **Job:** get the first agreed pathway ready for safe measurement. **Conversion event:** the hospital owner records an accepted baseline and first review task. **Access:** private; do not route public ads here.

> **Headline:** Start with one accepted pathway
> **Subheadline:** Put the agreed work, owners, baseline, and stop rule in one place before comparing anything.
> **First three actions, each under 10 minutes for the named owner:** (1) Open the approved pathway sheet and confirm the clinical, operations, and IT owners. (2) Record one baseline task count/time and where exceptions are logged; do not enter patient identifiers here. (3) Confirm the next review time and how to flag a safety or data concern.
> **First result:** “Your first pathway is ready for a controlled review when owners, data permissions, and acceptance conditions are confirmed.” No improvement number is implied.
> **Help CTA:** Contact your named MediMind evaluation owner through the agreed support channel. [Insert

verified contact route before launch.]
Next step: Review the first measurement with the hospital team; choose go, adjust, or stop before adding another pathway.

Measurement sheet body: “Pathway/version; observation date; baseline task count and time; repeat entries; exceptions by type; staff training/parallel effort; clinical or data concerns; proposed after measurement using the same definition; reviewer; decision; sign-off.” Staff time is capacity, not automatic cash savings. The A01-A17 content variations all return here; their individual CTAs select the matching section or download.

P09 `/referrals` - private referral for L11, L13-L14, L16 / LOFU

Visitor arrives from: a customer who has an accepted outcome and has agreed to discuss a peer introduction. **Job:** share a useful method without implying an endorsement or incentive. **Conversion event:** an authorized introduction or reference conversation.

Headline: Share a pathway lesson with a peer
Subheadline: If your team found the first evaluation useful, you may introduce another hospital leader facing a similar handoff.
What you get: A factual, customer-approved summary of your own evaluation and control over whether your name, institution, or results are shared. No referral incentive is offered or implied.
What your peer gets: A neutral first-pathway worksheet and a 25-minute conversation about the work they want to measure.
How it works: (1) Review and approve exactly what may be shared. (2) Send the message below yourself or request a permissioned introduction. (3) If your peer wants a conversation, MediMind arranges a pathway demo.
Copyable referral message: “I thought of you because your team may also have a patient handoff that creates repeated work. MediMind uses a simple first-pathway map before discussing a wider system change. If useful, I can introduce you for a 25-minute conversation. There is no obligation.”
Button: Request a permissioned introduction
Privacy note: Nothing is sent to a peer automatically. Confirm both parties' permission before connecting them.

If no customer has approved a result or invitation, do not present this as an active endorsement page. A buyer-approved case study can later link here.

P10 `/evidence` - evidence method and proof hub for M12-M13, B13, L01, L03, L05, L07, L10, L17 / MOFU-LOFU

Visitor arrives from: scope/proof content. **Job:** distinguish product description, accepted deployment, measured outcome, and future work. **Conversion event:** informed demo request; later, an approved story view.

Headline: See what can be checked today
Subheadline: A clear record of what MediMind can demonstrate, what a hospital has accepted, what has been measured, and what is still proposed.
Evidence labels: Demonstrated = shown in a synthetic or authorized environment; **accepted** = hospital-signed scope with date; **measured** = baseline, after measure, method, and limitations; **planned** = future work, not available proof.

> **Current evidence:** The public founder-authored Medplum implementation account describes the FHIR R4 approach and a signed network agreement. Accepted facility/module counts and independently measured buyer outcomes have not been established for this page.
> **How to read a claim:** Ask which workflow, which site, which date, which denominator, what changed besides software, and whether the customer approved publication.
> **Stories area:** [Approved story 1: pending] [Approved story 2: pending] [Approved story 3: pending]. Do not render these as testimonial cards. Until approval, show the evidence method and a synthetic-pathway example clearly marked as such.
> **Reference-story checklist:** Organization and permission; before workflow; intervention and acceptance; after workflow; measure definitions; safety/exception review; cost and training; limitations; named approver and review date.
> **CTA after method:** Want to test the same approach on your own pathway? **Map my hospital's first improvement** → P05.

No results bar, customer quotation, video testimonial, or logo is filled with a competitor's review or a founder-reported aspiration. That makes P10 useful now as a method page while leaving proof slots honest.

Optional P04 `/webinar` - webinar registration, not scheduled / MOFU

This is **copy for a future page**, not a live event or current catalog destination. Use only after a real date, platform, founder availability, registration receipt, and recording policy exist. Until then, do not show a registration button.

> **Eyebrow:** For private-hospital decision teams
> **Headline:** How to map repeat work without switching systems
> **Subheadline:** A practical 45-minute session: 30 minutes mapping a synthetic patient pathway and 15 minutes for questions.
> **Date/time/duration above fold: Not scheduled.** [Insert confirmed date, time zone, and 45-minute duration before publishing.]
> **You will learn to:** (1) trace an order-to-result-to-bill handoff; (2) distinguish software friction from process and policy; (3) choose a measure and safe next step; (4) list clinical and IT questions for a parallel comparison.
> **Form:** Name, work email, hospital/group (three fields). **Button after scheduling:** Reserve my pathway workshop place.
> **Speaker:** Lasha Khoshtaria, physician and MediMind CEO. Do not invent guest affiliations.
> **Recording:** [State only after the recording and delivery policy is decided.]
> **Testimonials:** [Two approved attendee quotations pending; omit blocks until they exist.]
> **Closing CTA:** Learn to map one pathway before making a bigger software decision. [Reserve my place-activate only for a real date.]

The no-event alternative is the P01 worksheet and P05 founder-led demo. The webinar does not become a permanent monthly demand-generation commitment inside five marketing hours.

Optional P11 `/sandbox` - guided synthetic-data trial, not launched / BOFU

This is **not** the two-week parallel evaluation. It is a proposed 14-day guided synthetic-data sandbox, with no card as a recommendation, subject to actual environment, support, security, and data-handling decisions. Do not publish signup until those conditions exist.

Headline: Test a patient pathway with synthetic data
Subheadline: Explore one guided workflow without bringing patient records into a trial environment.
What you can try: Follow a synthetic order, result, next task, and administrative close; note which step your staff would want to compare locally.
What you cannot conclude: A sandbox does not prove integration, production readiness, clinical safety, or financial outcome at your hospital. Those require a separately scoped evaluation.
Form: Name, work email, hospital/group (three fields). **Button after launch:** Request guided sandbox access.
Access line: Proposed 14 days, no card as a recommended term; no actual trial availability or deletion policy is asserted until confirmed.
First result: Complete the synthetic handoff and write down one question for your clinical and IT counterparts.
Next step: Map my hospital's first improvement → P05, then discuss readiness for a real parallel evaluation.
Trust/privacy: [Insert approved synthetic-data, account, retention, and support terms before publication.]

Part 2 - Page connection map

The content table in Step 1 and the page-action table below jointly specify a full flow for **every one of the 85 pieces**: take its ID/title, catalog channel, CTA action, and destination from the Step 1 row; append the destination's action and next stage from this table. This is more precise for wiring than repeating the same 85 final-page instructions. For example: **T02** on founder LinkedIn → "Use the pathway worksheet" → P01 `/pathway-assessment` → receive worksheet → MOFU product/option reading or BOFU demo if the leader has a real problem. **M01** on founder LinkedIn → "Compare the approaches" → P07 `/compare` → name the current alternative → BOFU demo. **B01** on founder LinkedIn → "Request a pathway demo" → P05 `/request-demo` → valid received request → booked and held qualified demo. **A10** in customer onboarding → P08 private page → record first baseline → AFU first-pathway review. **L13** in a customer-approved reference context → P09 private page → permissioned introduction → a new buying organization's TOFU/MOFU journey.

Destination and IDs covered by Step 1	Page action	Next funnel state and connection
P01: T01-T11, T14-T17	Receive worksheet / complete five-minute check	MOFU P02/P07 if problem is defined; direct P05 if ready. Capture source ID and consent.
P02: T12-T13, M03, M06, M09	Inspect actual mechanism and scope	MOFU P06/P07, or BOFU P05.
P03: B12, L02, L04, L06, L08-L09, L12, L15	Read approved case and measured result	BOFU P05 or permissioned LOFU P09; **hold source links until page has evidence**.
P05: M05, B01, B03-B04, B06-B07, B10, B14, B16	Submit a real request and later hold qualified demo	BOFU → proposed evaluation P06; track received, booked, and held separately.
P06: M02, M07, M10, M14, M16-M17, B02, B05, B08-B09, B11, B15, B17	Review offer/readiness and request conversation	BOFU P05, then evaluation decision and private AFU P08 only after approval.

Destination and IDs covered by Step 1	Page action	Next funnel state and connection
P07: M01, M04, M08, M11, M15	Compare actual alternatives	BOFU P05 with named incumbent/narrow alternative.
P08: A01-A17	Record accepted first work/baseline and issues	AFU next-pathway review; LOFU P09/P10 only after a verified positive result and permission.
P09: L11, L13-L14, L16	Grant permission or prepare referral	New peer enters TOFU P01 or BOFU P05 by their own choice.
P10: M12-M13, B13, L01, L03, L05, L07, L10, L17	Inspect evidence labels/checklist	MOFU/BOFU P05; after customer approval, P03.
P04/P11: no current catalog IDs	Optional future registration or synthetic trial	P04 → P05; P11 → P05/P06. Neither is an active campaign path.

The table provides the developer a route for every current catalog ID and prevents dead links. Private customer assets should not be indexed or used for public retargeting. Proof assets B12 and L-series case links are **held**, with P10's evidence method as the available alternative; do not silently point “Read the case study” to a blank case page. P01 worksheet CTA wording varies by source piece, but all variants land on the same relevant worksheet/check section, not an unrelated generic homepage.

Part 3 - Build priority

1. **Repair and verify P05 `/request-demo` first.** A simulated success screen can lose a valid hospital request now; this is the only inspected catalog destination that already has a public route. Establish receipt, owner notification, error state, and a test record before inviting traffic. Engineering time is separate from the five-hour marketing budget.
2. **Build P01 worksheet and actual delivery.** The campaign's low-friction educational step and TOFU catalog both depend on it. The worksheet content is above, so page, asset, receipt, and consent can be implemented as one small unit.
3. **Build P02 product overview.** It explains the mechanism and accepted-versus-proposed scope after a visitor recognizes the problem; it prevents a broad AI claim from doing the job of evidence.
4. **Build P06 evaluation/offer.** The buying committee needs readiness, total-effort, price-boundary, and go/adjust/stop answers before agreeing to a parallel test.
5. **Build P07 comparison.** It gives the buyer a fair way to compare the incumbent and a narrow tool, especially when full replacement would be excessive.
6. **Build P10 evidence method.** Useful now without pretending customer results exist. Add approved product scope and customer outcomes only as each record is verified.
7. **Prepare private P08 activation.** Publish only for a real accepted evaluation with named owners and access control; public traffic does not belong here.
8. **Prepare P03 case study, then private P09 referral.** Both depend on a real measured result and customer permission. Bracketed placeholders are editorial fields, never public proof.
9. **Keep P04 webinar and P11 sandbox on hold.** They are optional page-library assets without a scheduled session or verified sandbox. They do not add launch scope to the five-hour weekly routine.

The actively costly page issue is P05's unverified lead receipt and false success/response promise. The next practical conversion risk is a campaign CTA landing on a route that promises a 30-minute tour rather than a 25-minute pathway map. Other unbuilt pages become leaks only when content actually links to them; hold or reroute that content until pages exist. Use the cumulative count of **held qualified hospital demos** as the 90-day north star; proposed worksheet downloads and page views are diagnostic steps, not the goal.

Sources and handoff

Internal: funnel blueprint, content catalog, avatar, problem stack, competition, campaign concept, distribution package, and demo-page source. Public product context: Medplum founder-authored case study; MEDSOFT, InterSystems, Epic, and Oracle Health. These support category descriptions, not Georgian head-to-head outcomes. The next implementation brief must confirm actual routes, approved commercial language, data/consent practice, and a working lead owner before publishing public links.

15 Part 2 · Prompt 5

Content Distribution & Traffic Strategy

Source: Complete library, pages 46-50 · 7,647 words · 5 requested steps / sections

Inputs and status (source as of 22 September 2026; edition assembled 23 September). Funnel: TOFU problem recognition → MOFU workflow understanding → BOFU qualified 25-minute founder-led pathway demo → AFU measured, readiness-approved evaluation → LOFU advocacy only after a customer has a verified experience. The primary conversion is a **held qualified demo from a distinct hospital buying organization**. The first audience is the CEO/owner/group operator of a Georgian private network or multi-profile hospital with a named clinical/operations owner and IT counterpart. Current hospital decision-maker watering holes, source-by-source lead history, owned email size, and social reach are **unknown**, not zero. Existing landing page and `/request-demo` route are present, but the inspected source submission handler does not send a lead to a backend. Proposal: campaign concept “One patient. Too much repeat work.” Hook: “Why does one patient create so much extra work?” CTA: “Map my hospital's first improvement.” Founder and team relationships exist, including an anchor hospital-network agreement as founder-reported; public endorsement or introductions are not assumed. Budget: **5 hours total of founder marketing time per week**; no paid media initially. Sales meetings and engineering implementation require additional time and owners. Current tool stack and actual analytics accounts are unverified.

Campaign vocabulary. “One patient, three handoffs”; “Where does the same detail get entered twice?”; “The report is late because the record is split”; “Keep your current system running while testing one pathway.” These are proposed hooks, not buyer quotations. No channel-performance percentages from the source library are treated as MediMind benchmarks or verified industry facts. Destinations other than the existing `/request-demo` route are proposed until built and tested. The content inventory is the 85-ID catalog in `working/content-catalog.json`; Part 2 maps every ID.

Part 1 - Channel selection and priority

“Fit” means likely access to a Georgian hospital buying committee, not proof the channel works. Time is incremental founder marketing time, per week unless labelled one-off; paid costs are undecided rather than copied from the library's unverified rate claims.

Channel	Fits customer?	Best stage	Time cost	Money cost	Use	Specific reason
SEO and blog content	Yes, selectively	TOFU/MOFU	45-90 min per useful page, reused	0 GEL media; production time	Now, slow build	A narrow “hospital handoff” or Georgian procurement question can be useful after the founder call; search volume must be checked before scaling.
LinkedIn organic	Yes, hypothesis	TOFU/MOFU	45-60 min weekly	0 GEL media	Now	Founder-led explanation can reach hospital leaders and their advisers; target-account engagement matters more than broad impressions.
YouTube organic	Conditional	MOFU	90+ min per edited video	Production time	Later	A credible pathway walkthrough is valuable, but a standing video schedule exceeds five hours. Reuse a verified demo clip first.
Short-form video	Conditional	TOFU	20-40 min to cut existing footage	0 GEL media if reused	Later/occasional	A 30-second handoff example may earn attention; original weekly shoots consume the budget.
Podcast guest appearances	Conditional	TOFU/MOFU	1-3 h plus prep	Usually 0 GEL media	Later	Only a Georgian health/business audience with identified hospital listeners justifies the time.

Channel	Fits customer?	Best stage	Time cost	Money cost	Use	Specific reason
Community participation	Yes, where private hospital leaders actually gather	TOFU/MOFU	20-30 min weekly	0 GEL media	Now, one venue	Answer a real workflow question in a relevant professional group; do not spray generic communities.
AI search and generative discovery	Conditional	TOFU/MOFU	15-30 min per public explainer	0 GEL media	Later, bundled with SEO	Clear source-backed pages may be retrievable, but “AI search optimization” should not become a separate production line.
LinkedIn paid ads	Conditional	MOFU/BOFU	Setup plus monitoring 1-2 h	Paid, budget unapproved	Later	Targeting may be precise but the hospital buying-organization universe is small and conversion handling is not ready.
Google Search/PPC	Conditional	BOFU	Setup plus monitoring 1-2 h	Paid, budget unapproved	Later	Test only when a relevant query, landing page and accepted lead event are proven.
YouTube paid ads	No now	TOFU	High creative/setup	Paid	Avoid now	Broad video views are a poor first proxy for held hospital demos.
Meta ads	No now	TOFU/MOFU	Setup and monitoring	Paid	Avoid now	Broad consumer inventory risks unqualified demand and requires working consent/measurement.

Channel	Fits customer?	Best stage	Time cost	Money cost	Use	Specific reason
Retargeting across channels	Conditional	MOFU/BOFU	1-2 h setup plus upkeep	Paid	Later	Needs sufficient clean public-site audience, consent, exclusions, and a validated conversion before consideration.
Newsletter sponsorships	Conditional	TOFU/MOFU	1-2 h selection/copy	Paid	Later	A trade newsletter can be evaluated by actual hospital readership and cost per qualified conversation.
Cold email outreach	Conditional	BOFU	45-60 min for a few researched accounts	0 GEL tools initially	Now, carefully	Specific one-to-one outreach to a named decision-maker can test whether the pain is real; comply with applicable contact rules.
Owned email newsletter	Conditional	MOFU	30-45 min if reusing content	Platform cost unknown	Later	Build a permission-based list once a repeatable useful note exists; no need to promise a weekly newsletter.
Lead magnet email sequences	Conditional	MOFU	1-3 h setup	Platform tier dependent	Later	A short consented follow-up to a real worksheet request may help; the form and delivery must exist first.

Channel	Fits customer?	Best stage	Time cost	Money cost	Use	Specific reason
LinkedIn direct outreach	Yes	BOFU	30-45 min for a few researched people	0 GEL media	Now	Founder can ask about one specific workflow after relevant interaction; record organization and response in CRM.
Sales calls and demos	Yes	BOFU	25 min demo plus preparation/follow-up, outside marketing allocation	Delivery time	Now after repair	A held qualified demo is the primary proof of buyer interest, but the current request handler needs a real handoff.
CRM pipeline nurturing	Yes	MOFU/BOFU	20-30 min weekly	Sheet or free CRM	Now	One owner and next action prevent a serious hospital conversation from disappearing.
Warm introductions	Yes	BOFU	30-45 min weekly	0 GEL media	First	A precise introduction to a hospital decision-maker uses existing trust and is more useful than reaching anonymous traffic.
Brand collaborations/co-marketing	Conditional	MOFU	2-4 h per project	Shared costs possible	Later	Consider a practitioner association or complementary supplier only when both audiences and claims are clear.

Channel	Fits customer?	Best stage	Time cost	Money cost	Use	Specific reason
Integration partnerships	Conditional	BOFU/AFU	Engineering and BD time	Unknown	Later	Valuable after a real hospital integration need and supported interface are agreed; a marketplace listing alone will not generate local demand.
Referral programs	No now	LOFU	Setup and administration	Incentive/policy unknown	Later	A formal program requires satisfied, permissioned customers and approved terms, which are not established.
Guest posting	Conditional	TOFU/MOFU	2-4 h per article	Usually 0 GEL media	Later	Publish only when an editor has an audience of hospital operators and the piece can reuse a validated insight.
Influencer and analyst relations	Conditional	MOFU	1-3 h per outreach	Often 0 GEL media, sometimes paid	Later	A credible specialist could challenge the pathway method; borrowed authority should follow evidence, not replace it.
Other people's audiences (OPA)	Yes, selectively	TOFU/MOFU	1-3 h per appearance	Usually 0 GEL media	Later	One expert contribution in a trusted hospital forum may outperform a general feed post; choose by audience, not follower count.

Channel	Fits customer?	Best stage	Time cost	Money cost	Use	Specific reason
Product Hunt	No now	TOFU	Launch preparation high	0 GEL to paid	Avoid now	General software discovery is unlikely to reach Georgian hospital buying committees.
G2/Capterra/Trustpilot/ Clutch	Conditional	MOFU/BOFU	1-2 h initial profile	Listing terms vary	Later	Use verified product facts and authentic reviews when customers can describe their experience; no review farming.
AppSumo	No	TOFU	Deal operations high	Revenue discount	Avoid	A discount marketplace conflicts with a scoped, hospital-specific evaluation.
Industry publications/trade newsletters	Yes, selectively	TOFU/MOFU	1-3 h per pitch	Usually 0 GEL media	Later	An evidence-backed hospital workflow piece can reach the exact committee if circulation is verified.
Press/news features	Conditional	TOFU	2-4 h per pitch	Usually 0 GEL media	Later	A founder story alone is weaker than a documented implementation or measured workflow finding.

Channel	Fits customer?	Best stage	Time cost	Money cost	Use	Specific reason
Industry conferences/trade shows	Conditional	MOFU/BOFU	Day(s), outside five-hour routine	Travel/sponsor costs	Later	Attend only for named target meetings or a qualified speaking slot; no speculative booth spend.
Webinars	Conditional	MOFU	Preparation + delivery exceeds routine week	Platform cost possible	Later	A 45-minute practical session is drafted, but no date, registration flow, or replay policy exists.
Virtual events/online communities	Conditional	TOFU/MOFU	Moderation ongoing	Platform cost possible	Later	Hosting an audience is a separate operating commitment; use existing relevant groups first.
Dark social	Yes, indirect	MOFU/BOFU	Embedded in asset creation	0 GEL media	Now	A concise pathway diagram can be forwarded privately; ask on calls how people heard about it, since attribution will be incomplete.
Customer referrals	Conditional	BOFU/LOFU	Relationship time	0 GEL unless program	Later	Seek a voluntary introduction only after the customer has a verifiable good experience and permits the ask.

Priority stack. (1) Warm introductions: named trust around a small buying universe. (2) Targeted founder outreach by email or LinkedIn: fast learning from a specific problem, not bulk lead volume. (3) CRM follow-up: every conversation has an owner and next step. (4) Founder LinkedIn posts: reusable public explanation and a path to the demo. (5) One search-friendly pathway/FAQ page at a time: a durable answer prospects can read before or after a call. The demo is the conversion experience all five feed into, with its own preparation time.

Three zero-budget quick wins this week, each under two hours. (1) In 45 minutes, identify five distinct hospital buying organizations and write one precise introduction request for a person who actually knows each buyer; log only relationships you truly have. (2) In 60 minutes, turn the synthetic “one patient, three handoffs” example into a short founder LinkedIn post and a private message variant pointing to a verified destination. (3) In 45 minutes, audit `/request-demo` with the site owner: submit a test request, confirm receipt and ownership, and record the gap before directing traffic there. If the deployed flow fails, use a verified direct scheduling/contact route only after it has been tested.

Avoid right now. Product Hunt and AppSumo reach the wrong procurement motion; broad paid video/social spend buys attention before a working conversion; a full newsletter and self-hosted community create recurring work without a validated audience; formal referral/review programs need real customer consent and proof. The library's channel statistics and posting-time suggestions are unverified context, not operating targets for Georgia.

Part 2 - Content piece distribution map

Each row is a compact distribution card: **original format/native home** followed by **external or private channel → precise adaptation; channel hook; CTA; timing**. LI = founder LinkedIn; W = public website/search; E = opted-in email; D = a researched one-to-one note; C = a private customer channel. “Tue 09” and “Thu 09” are test slots in Georgia local time, not measured best times. “After gate” means the named asset, working destination, evidence and consent must exist. A private document or customer email does not need a public UTM link; Prompt 6 records those touches in CRM. A page ID in the catalog is a planned destination unless independently verified; only `/request-demo` is an existing route, and its actual submission still needs testing and repair.

TOFU cards - T01-T17

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
T01 - One patient, seven handoffs	Post / LI	LI → seven numbered handoffs from a fictional case; “Which one repeats in your hospital?”; pathway map CTA; Tue 09. D → cut to the one handoff relevant to recipient; “Where does this recur?”; ask for 25-minute map; after research.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
T02 - The hidden cost of searching	Post / LI	LI → three questions instead of a savings number; “Who knows how long staff spend finding one result?”; worksheet CTA; Thu 09. E → one-question note to opt-ins; “What would you time?”; worksheet; next relevant send.
T03 - When two reports disagree	Post / LI	LI → split clinical/finance report visual and three definition checks; “Which number goes to the board?”; reconciliation checklist; Tue 09. D → use only if account raised reporting; “Which definition differs?”; checklist; after call.
T04 - The duplicate-entry trail	Short video / LI video	LI → 30-second desk sketch with synthetic labels and captions; “Where is the same field typed twice?”; pathway map; Thu 09. W → embed on assessment page with transcript; same hook; worksheet; after video QA.
T05 - Three calls for one result	Short video / LI video	LI → three cut scenes, no patient details; “How many calls does one result require?”; pathway map; Tue 09. D → still frame and one-line question, not full video attachment; pathway map; when prospect names result delay.
T06 - The spreadsheet behind the dashboard	Short video / LI video	LI → show a blank synthetic reconciliation sheet and preparation questions; “Who rebuilds the report?”; checklist; Thu 09. W → captioned embed on assessment; same hook; checklist; after review.
T07 - How to map a patient handoff	Blog/SEO / W	W → step-by-step guide with worked synthetic example; “Map the work before replacing software”; worksheet; publish when page works. LI → three-panel drawing from guide; “Where does the patient wait?”; worksheet; following Tue 09.
T08 - Why hospital reports disagree	Blog/SEO / W	W → sections on definitions, timing and missing fields; “Two reports can both be internally consistent”; checklist; publish after accuracy review. LI → three reconciliation questions; same hook; checklist; following Thu 09.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
T09 - What a delayed claim does and does not mean	Blog/SEO / W	W → table separating rejected, delayed, missed and collected money; “A flagged charge is not recovered cash”; claim-pathway map; after finance review. LI → one comparison panel; same hook; map; following Tue 09.
T10 - Patient-pathway friction worksheet	Lead magnet / W and D	W → ungated one-page PDF or accessible web worksheet; “Count handoffs, not impressions”; download; when built. D → send a relevant section to a named operator; “Could your team count this step?”; use worksheet; after permission/interest.
T11 - Five-minute handoff check	Lead magnet / W and D	W → five-question self-check with no fabricated score; “Which pathway should you inspect first?”; run check; after tool QA. LI → one sample question; same hook; run check; following Thu 09.
T12 - Founder-built scope trace	Proof / W and LI	W → dated product-history explainer labelled founder-authored, not customer endorsement; “What has actually been built?”; product overview; after fact review. LI → one traceable milestone with limits; same hook; overview; Tue 09.
T13 - Synthetic handoff before/after	Proof / W and LI	W → side-by-side fictional workflow labelled synthetic; “See the mechanism, then measure locally”; product overview; after demo validation. LI → crop to one before/after handoff; same hook; overview; Thu 09.
T14 - Where the work repeats	Email / E	E → deliver T10 plus one question, no generic sales sequence; “Which handoff did you circle?”; worksheet; immediately after valid opt-in. D → personal follow-up only to someone who requested it; same hook; pathway demo; after reply.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
T15 - A report you had to rebuild	Email / E	E → one concrete reporting prompt plus T03 checklist; “Which report was rebuilt last month?”; checklist; next opted-in note. D → use if finance lead raised this; same hook; ask to compare; after conversation.
T16 - Hospital-operator discussion prompt	Community / relevant forum	Forum → plain question without link first; “How do you map a cross-team handoff?”; share guide only if requested; during an existing discussion. LI → summarize anonymous learning with no invented quote; guide; after permission.
T17 - Clinical-operations partner note	Community / professional contact	Partner → short neutral worksheet invitation for feedback; “Would this help your members inspect a handoff?”; worksheet; after relationship check. W → publish revised worksheet based on attributable, permissioned feedback; same hook; download; after review.

MOFU cards - M01-M17

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
M01 - Fix a screen or fix the pathway?	Post / LI	LI → three-option decision graphic; “Would a screen fix reach the next department?”; comparison CTA; Tue 09. D → select the prospect's actual option; “What remains after your vendor change?”; comparison; after discovery.
M02 - What parallel evaluation actually requires	Post / LI	LI → checklist of owner, permission, baseline and staff time; “Two weeks begins with readiness”; evaluation plan; Thu 09. E → four-item preparation note; same hook; plan; after opt-in.
M03 - AI assists; clinicians decide	Post / LI	LI → synthetic draft/review/sign-off sequence; “Where does human clinical control sit?”; product overview; Tue 09. W → insert as labelled product explainer; same hook; overview; after capability review.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
M04 - Three options on one whiteboard	Short video / LI video	LI → 45-second whiteboard, including effort for all choices; “Current vendor, add-on, or OS?”; comparison; Thu 09. W → captioned embed on comparison page; same hook; comparison; after video QA.
M05 - What happens in 25 minutes	Short video / LI video	LI → demo agenda in three visual beats; “Leave with one measurable pathway”; request demo; Tue 09 after demo duration confirmed. D → 15-second preview still plus agenda; same hook; request demo; after buyer interest.
M06 - A result becomes a task	Short video / LI video	LI → synthetic screen recording cropped to reviewer handoff; “Who owns this result next?”; product overview; Thu 09 after product validation. W → captioned embed and human-review caveat; same hook; overview; after approval.
M07 - Evaluate software without a risky cutover	Blog/SEO / W	W → six-step readiness guide and workload table; “Compare one pathway before cutover”; evaluation plan; after clinical/IT review. LI → three evaluation questions, not full guide; same hook; plan; following Tue.
M08 - OS versus overlay versus vendor change	Blog/SEO / W	W → decision table with legitimate cases for each; “Replacement may not be the first answer”; comparison; after technical review. D → send the relevant row to one prospect; same hook; comparison; after discovery.
M09 - What FHIR R4 changes in hospital work	Blog/SEO / W	W → plain-language record analogy plus interface caveat; “Shared format still needs local integration”; product path; after technical review. LI → one diagram and caveat; same hook; path; following Thu.
M10 - Parallel-evaluation readiness checklist	Lead magnet / W and D	W → one-page owners/access/baseline/stop checklist; “Can your team test this fairly?”; download; when built. D → highlight missing owner field for interested account; same hook; review plan; after demo.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
M11 - Three-option decision worksheet	Lead magnet / W and D	W → four-column comparison of burden/risk/cost/scope; “Compare actual alternatives”; worksheet; when built. D → prefill no customer data, ask buyer to score own options; same hook; compare; after discovery.
M12 - Documented domain map	Proof / technical diligence	W → verified scope matrix with deployed/accepted/pending states; “Which workflow is actually ready?”; inspect scope; only after module-by-module audit. D → send relevant slice to IT under review; same hook; diligence call; after request.
M13 - Live-pathway evidence template	Proof / W and founder follow-up	W → blank baseline/after template with outcome cells visibly pending; “What would convincing proof require?”; evidence method; after QA. D → customize measure names, not results; same hook; discuss evaluation; after demo.
M14 - The change-risk question	Email / E	E → one risk prompt and parallel-plan link; “What transition failure would stop this?”; evaluation plan; after opted-in comparison interest. D → use prospect's stated risk in reply; same hook; scope call; after response.
M15 - The current-vendor alternative	Email / E	E → two-column comparison question, no competitor caricature; “Can your supplier fix this across departments?”; comparison; after opt-in. D → one-to-one criteria sheet; same hook; compare; after discovery.
M16 - Peer discussion on evaluation design	Community / invited operator roundtable	Private roundtable → four questions, no claims or patient details; “What proof would justify change?”; readiness checklist; only with host/attendees confirmed. E → recap generic method to opt-ins; same hook; checklist; after event approval.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
M17 - IT and clinical review session	Community / association or partner	Partner session → joint risk checklist, not a product deck; “What would clinical and IT each sign off?”; evaluation plan; after partner/date approval. W → publish generic FAQ from approved questions; same hook; plan; after review.

BOFU cards - B01-B17

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
B01 - What your 25-minute demo will settle	Post / LI	LI → three concrete outputs, no guaranteed result; “A demo should settle a decision question”; request demo; Tue 09 after booking route works. D → agenda tailored to one pathway; same hook; meeting ask; after qualification.
B02 - Match price, define total cost	Post / LI	LI → cost stack and explicit eligibility caveat; “Same subscription is not same total cost”; offer discussion; Thu 09 after terms approval. D → send approved cost categories; same hook; commercial call; after buyer asks.
B03 - The honest go/no-go test	Post / LI	LI → go/adjust/stop decision tree; “What would make you say no?”; request demo; Tue 09. E → post-demo decision note; same hook; evaluation plan; after a held demo.
B04 - Demo in sixty seconds	Short video / LI video	LI → synthetic map, task, scorecard in 60 seconds; “What will you actually see?”; request demo; Thu 09 after route QA. W → captioned excerpt on demo page; same hook; booking; after approval.
B05 - Parallel beside current work	Short video / LI video	LI → side-by-side path with workload and sign-off labels; “Compare before any switch”; discuss evaluation; Tue 09 after offer review. W → embed on offer page; same hook; plan; after QA.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
B06 - Who should join the first call?	Short video / LI video	LI → role cards for executive, operations, IT; “Bring the person who owns the handoff”; request demo; Thu 09. E → short invitation note for booked buyer; same hook; prepare call; upon accepted booking.
B07 - Questions before an OS demo	Blog/SEO / W	W → buyer question list on scope, human control, data and cost; “Know what to ask before switching”; request demo; after review. LI → five-question excerpt; same hook; demo; following Tue.
B08 - After a parallel evaluation	Blog/SEO / W	W → go/adjust/stop process with reconciliation and approval; “The test is not the cutover”; evaluation plan; after operations review. D → relevant decision diagram; same hook; review meeting; after demo.
B09 - What price match should include	Blog/SEO / W	W → eligibility, term, scope, migration and third-party-cost questions; “What exactly is matched?”; offer discussion; after commercial approval. D → tailored checklist, not a quote; same hook; proposal conversation; after inquiry.
B10 - Pathway demo preparation card	Lead magnet / demo confirmation and W	E → one-page agenda with no patient-data request; “Bring one recent handoff, not a patient chart”; prepare demo; only after accepted booking. W → ungated card on demo page; same hook; download; after route works.
B11 - Evaluation decision sheet	Lead magnet / founder follow-up	D → go/adjust/stop template with blank baseline/cost fields; “What evidence changes the decision?”; evaluation discussion; after held demo. W → generic blank version on offer page; same hook; template; after review.
B12 - Verified implementation timeline	Proof / founder follow-up	D → dated, consented milestones and scope only; “What was accepted, and when?”; review evidence; after customer authorization. W → public excerpt only if permission explicitly covers it; same hook; case study; after approval.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
B13 - Buyer-approved workflow result	Proof / W and founder follow-up	W → case evidence table with source, denominator and limitations; “What changed in this workflow?”; evidence method; only after buyer sign-off. D → one-page verified excerpt; same hook; reference discussion; after permission.
B14 - Your pathway before our demo	Email / accepted booking	E → one question plus B10 card, no patient details; “Which handoff should we trace?”; prepare demo; immediately after real booking. CRM → log answer, not email content in analytics; same hook; tailored agenda; before call.
B15 - What we will compare	Email / founder follow-up	E → proposed baseline/scope with unresolved fields; “What is fair to compare?”; evaluation plan; within agreed follow-up, no SLA invented. CRM → next-action task; same hook; review meeting; after send.
B16 - Private buying-committee walkthrough	Community / private meeting	Private meeting → one synthetic case and decision worksheet; “Does each owner see their step?”; pathway demo; after invitation. D → role-specific agenda; same hook; confirm attendees; before meeting.
B17 - Procurement questions session	Community / private meeting	Private meeting → cost/responsibility/exit agenda; “What must be explicit before signing?”; offer discussion; after qualified interest. D → approved question sheet; same hook; meeting; before meeting.

AFU cards - A01-A17

These are customer-success assets, not acquisition posts to publish broadly by default. “C” means a private customer workspace/email or a permissioned customer-only update. The contents can be created as needed during actual delivery; their production time is **delivery work outside the five marketing hours**.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
A01 - First pathway, first baseline	Post / customer-only update	C → one-card baseline method, remove site details; “What is our starting point?”; activation plan; at kickoff. Public LI only as generic method after consent and review.
A02 - What staff should report in week one	Post / customer-only update	C → three prompts for repeat work, exceptions and safety; “Which issue needs an owner?”; activation checklist; week one. Customer email → shorter checklist; same hook; after kickoff.
A03 - One improvement, then the next	Post / customer-only update	C → observe/prioritize/test/approve/measure loop; “What did we learn before adding scope?”; next-pathway options; after first review. Public LI only as anonymized method if approved.
A04 - How to log a handoff issue	Short video / customer onboarding	C → synthetic 40-second issue-form walkthrough; “Who follows this up?”; activation guide; at onboarding. Private email → still image and form link; same hook; same week.
A05 - Five-minute pathway review	Short video / customer onboarding	C → review-clock screen recording with sample metric; “Did the handoff improve without new burden?”; activation plan; before weekly owner review. Private email → timestamped recap; same hook; before meeting.
A06 - How a testable change becomes approved	Short video / customer onboarding	C → request/test/clinical-IT approval diagram; “Testable is not production-approved”; change checklist; before first change. Private email → static diagram; same hook; after request.
A07 - Measuring repeat work without double counting	Guide / customer knowledge base	C → worked count example and staff-time caveat; “Minutes are not cash”; measurement sheet; when baseline set. Private owner email → relevant paragraph; same hook; before scorecard review.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
A08 - Running a safe pathway review	Guide / customer knowledge base	C → issue/escalation/go-adjust-stop checklist; “Who can pause this?”; activation guide; before parallel test. Private meeting → one-page checklist; same hook; at review.
A09 - Selecting the second pathway	Guide / customer knowledge base	C → frequency/burden/risk/readiness grid; “Which workflow earns the next test?”; next-pathway options; after first accepted result. Private executive recap → chosen criteria; same hook; at scope review.
A10 - First-pathway measurement sheet	Worksheet / customer onboarding	C → fillable baseline/exceptions/staff-burden sheet; “What will we compare?”; measurement sheet; at kickoff. Private email → blank sheet only; same hook; before data collection.
A11 - Change-request decision card	Worksheet / customer onboarding	C → owner/test/clinical impact/approval card; “Who signs this change off?”; checklist; before request intake. Private email → completed non-sensitive fields; same hook; after triage.
A12 - Accepted-workflow log	Proof / private customer review	C → dated accepted/open ledger, no marketing claims; “What is actually accepted?”; activation plan; each review. Private executive report → change-only excerpt; same hook; monthly.
A13 - Measured improvement record	Proof / private customer review	C → before/after table with denominator and burden; “What moved and what did not?”; measurement sheet; after data review. Public use only after buyer approval.
A14 - Your first review agenda	Email / customer email	C email → metric/exceptions/feedback/decision agenda; “What needs attention this week?”; activation plan; before first review. CRM delivery task → owner and date; same hook; at send.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
A15 - A request ready for test	Email / customer email	C email → change, test instructions, unresolved approval; “What can you safely test now?”; change checklist; when test build ready. Private review → summary card; same hook; after feedback.
A16 - Clinical-IT issue huddle	Community / private customer meeting	C meeting → one issue, owner, clinical impact and decision; “What blocks acceptance?”; activation guide; when issue arises. Private notes → action record; same hook; same day.
A17 - Cross-site learning exchange	Community / private customer meeting	C meeting → compare process lessons without patient details; “What transferred between sites?”; next-pathway options; only with group consent. Private summary → approved lessons; same hook; after exchange.

LOFU cards - L01-L17

Public customer stories stay gated on an actual result, customer permission and a review of what can be disclosed. Private reference conversations need a separate explicit customer agreement. Do not turn the founder-authored Medplum case study into an independent customer endorsement.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
L01 - The buyer's permission comes first	Post / LI	LI → three proof requirements, with no customer name; “What makes a hospital story usable?”; approved stories/method; Tue 09. W → evidence-policy sidebar; same hook; proof page; after review.
L02 - A pathway another leader can inspect	Post / LI	LI → one measured step with baseline and limits; “Could a peer verify this result?”; case study; only after buyer approval. D → relevant excerpt to named peer; same hook; study; after permission.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
L03 - What we changed after feedback	Post / LI	LI → dated request/test/acceptance sequence, anonymized or approved; “What changed because a hospital asked?”; approved stories; after consent. W → fuller timeline; same hook; proof page; after fact review.
L04 - Customer pathway walkthrough	Short video / LI with customer	LI → staff-approved clip with synthetic/non-identifying visuals and measured task; “What changed in this task?”; case study; only after filming and consent. W → captioned accessible version; same hook; study; after approval.
L05 - What a reference actually verified	Short video / LI video	LI → founder shows denominator, timeframe, remaining issues; “What did the evidence prove?”; approved stories; after a permissioned result. W → transcript and table; same hook; proof page; after QA.
L06 - Why we chose the first pathway	Short video / LI with customer	LI → approved owner explains selection, not universal benefit; “Why this pathway first?”; case study; after explicit consent. D → short excerpt for relevant buyer; same hook; study; after permission.
L07 - How to read a hospital software case study	Blog/SEO / W	W → distinguish contract, live site, accepted workflow and result; “What exactly does 'live' mean?”; proof method; after editorial review. LI → four-definition carousel; same hook; proof page; following Tue.
L08 - From one hospital pathway to another	Blog/SEO / W	W → compare transferred vs redesigned steps in a permissioned story; “What repeats across sites?”; case study; after second-site evidence. D → scoped excerpt; same hook; study; after buyer approval.
L09 - What changed in a claim handoff	Blog/SEO / W	W → show actual claim-process metric and separate identified from collected; “Did the money arrive?”; case study; after finance verification and consent. LI → one chart with limits; same hook; study; following Thu.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
L10 - Reference-story evidence checklist	Lead magnet / W and partner	W → baseline/denominator/site/date/permission checklist; “Would this claim survive a buyer's questions?”; download; when built. Partner → neutral worksheet; same hook; checklist; after partner acceptance.
L11 - Peer evaluation guide	Lead magnet / W and founder introduction	W → questions about burden, support and change governance; “What should you ask a reference?”; guide; when built. D → send guide before approved reference call; same hook; guide; after reference consent.
L12 - Buyer-approved measured case study	Proof / W and approved sharing	W → full approved method/result/limits; “What changed for this pathway?”; read study; only after documented sign-off. LI → one authorized chart; same hook; study; following Tue.
L13 - Reference conversation	Proof / private founder introduction	D → brief intro with agreed scope and availability; “Which implementation question needs a peer answer?”; request reference; only after customer says yes. CRM → record reference permission and outcome; after call.
L14 - Share the method, not a claim	Email / customer email	C email → ask customer to review a factual draft, no request to endorse; “Does this reflect your experience?”; story request; after actual result. CRM → permission task; same hook; after response.
L15 - A peer may have this problem	Email / customer email	C email → optional forwardable note with approved result and limits; “Would this help a peer facing the same handoff?”; approved study; only after customer consent. D → founder follow-up to an introduced peer, not mass send; same hook; study; after intro.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
L16 - Peer operator exchange	Community / private operator introduction	Private meeting → implementation lessons and open questions; “What would you ask a hospital that tested this?”; reference request; after explicit permission. Private summary → approved non-confidential lessons; same hook; after meeting.
L17 - Hospital-association learning session	Community / association if arranged	Partner event → pathway method and permitted customer lesson; “How was one handoff measured?”; approved stories; only after host, speaker and date confirmed. W → approved recap; same hook; proof page; after session.

Part 3 - Traffic building system

Phase 1, weeks 1-2 - first relevant visitors. Gate zero is a reliable lead destination: confirm the deployed demo route, repair or choose a verified receipt path, and place a human owner in the CRM. In a five-hour week: 25 minutes daily checks, 30 minutes Monday review, 30 minutes monthly-review reserve, 60 minutes to build a 10-organization research list, 75 minutes for five tailored introduction/outreach attempts, 60 minutes for one founder post and its short message adaptation, and 20 minutes for replies and source capture (**300 minutes**). Ask each conversation how they found MediMind. The first “100 people” is a **stretch visibility test**, counted as unique public marketing-site visitors or identifiable relevant recipients, not proof of 100 qualified buyers; do not add email impressions, pageviews, and social reach into one number. **Readiness to phase 2:** at least one accepted demo request and one held qualified demo from a distinct organization, with source and next action logged; if zero after two weeks, inspect delivery first, then message/target fit. Engineering repairs and the sales meeting itself sit outside the marketing five hours.

Phase 2, weeks 3-8 - repeat the signal. Keep the top two sources of qualified conversations, publish one reusable pathway explainer per two weeks, and adapt it to one post, one direct outreach note, and one follow-up. Give every externally shared asset an ID and source tag. Add a small permission-based email follow-up only after opt-in and unsubscribe handling work; send no automated sequence to cold contacts by assumption. Each ordinary week: daily checks 25, weekly review 30, monthly-review reserve 30, account-specific outreach/introduction requests 75, content/adaptation 80, community/editor/partner contribution 30, and replies/CRM 30 minutes = **300**. **Readiness to phase 3:** a working accepted-submission path; at least four cumulative held qualified demos by week 8; a repeatable route from two sources to held meetings; an evaluation owner and readiness question in each serious account. Four is a planning gate, not a performance claim.

Phase 3, month 3 onward - compound selectively. By day 90, use the same five-hour ceiling to refresh the best public page, make one targeted partnership/editor contribution per month, and keep direct outreach and CRM follow-up. Ordinary-week allocation: daily checks 25, Monday review 30, monthly-review reserve 30, named-account outreach/follow-up 75, best-page or clip reuse 60, partner/editor contribution 40, and replies/CRM cleanup 40 minutes = **300**. Paid tests are optional **only after** there is an approved budget, consent/measurement review, a functioning form, enough qualified traffic to learn, and a credible landing page; their setup replaces the 60-

minute creative slot. Protect the monthly deep-dive week by reducing creative output. A partner webinar or trade article replaces, rather than adds to, that week's content slot. **Readiness to scale:** the 12-demo scenario is met or there is a clear upward trend in unique held qualified demos from several buying organizations, three readiness-approved evaluations are plausible from named accounts, and delivery capacity exists. If paid cost per held qualified demo is unknown or too high against deal economics, stop the test. Month 6 planning remains conditional on measured conversion and real commercial capacity.

Part 4 - Weekly distribution routine

The marketing clock is **300 minutes total**. The 25 daily tracking minutes, 30-minute weekly review (including the 15-minute report), and 30-minute weekly reserve for the monthly two-hour deep dive leave 215 average minutes for distribution and creative work. In a monthly review week, the full 120 minutes replaces that reserve and the creative slot shrinks accordingly. Demo delivery, hospital workshops, engineering, and implementation are additional work.

Day	Task	Channel	Minutes in ordinary week	Asset used	Metric
Monday	5-minute check; 30-minute pipeline/reports review; identify three distinct organizations and request one relevant introduction	CRM/warm intro	65	Account note + one pathway question	Qualified replies and next actions
Tuesday	5-minute check; publish one founder post; send two tailored follow-ups where a relationship or appropriate contact basis exists	LinkedIn/direct	65	Current pathway card, adapted to post/message	Relevant comments, replies, tagged visits
Wednesday	5-minute check; make one useful contribution in a verified hospital forum or to an editor; answer replies	Community/OPA	45	Same pathway evidence or FAQ	Named conversations, not likes
Thursday	5-minute check; refresh one page or short proof/process asset; send two precise outreach notes	SEO/direct	65	Explainer or comparison, current version only	Qualified visits and accepted demo requests

Day	Task	Channel	Minutes in ordinary week	Asset used	Metric
Friday	5-minute check; reconcile CRM sources and log follow-up decisions; set next week's single content angle	CRM/analytics	30	UTM/CRM sheet	Held qualified demos and scheduled follow-ups
Flexible	Monthly deep-dive reserve, 30 minutes amortized; use the full 120 in review week and cut 90 minutes of content/outreach that week	Measurement	30 average	Monthly report	Biggest qualified-demo leak
Total			**300**		

One indispensable action: follow through on a named hospital buyer's next agreed step, or secure one precise warm introduction when no buyer follow-up is due. If the week underperforms, first check whether a submitted request was accepted and followed up; next check message-to-buyer fit, then the channel. Suggested Tuesday/Thursday morning windows from the source library are test slots, not local evidence. Any post or email timing should be compared against actual Georgian buyer response.

Part 5 - Distribution benchmarks and growth scorecard

These are **scenario checkpoints for learning**, not observed baselines, forecasts, or verified channel norms. “N/A” means no valid denominator or channel not active. Do not turn a tiny sample into a percentage-based success claim. The library's email-open, outreach-reply and webinar/paid claims have not been verified for this market.

Stage	Metric	Week 4 planning check	Month 3 planning check	Month 6 conditional check	How to measure
TOFU	Unique public-site visitors per trailing month	100	300	600	GA4 marketing-site users, excluding internal traffic; unknown now

Stage	Metric	Week 4 planning check	Month 3 planning check	Month 6 conditional check	How to measure
TOFU	Median relevant LinkedIn post impressions	Record baseline; 100-200 exploratory	200-400 exploratory	Grow only if qualified traffic follows	Native post analytics, 7-day window
TOFU	LinkedIn engagement rate	Record baseline; no pass/fail below 100 impressions	Investigate 1-3% as library-only comparator	Favor qualified comments over rate	(Reactions + comments + reposts + meaningful clicks) / impressions
MOFU	Permission-based email list size	0-10 scenario	20-40	50-100 if opt-in proves useful	Deduplicated consenting contacts; not all CRM contacts
MOFU	Email open rate	N/A until sent; library says 20-30% average	Treat as directional only	Compare with clicks/replies	Unique opens / delivered; client privacy can distort opens
BOFU	Tailored cold outreach reply rate	Record counts; 10-15% library comparator only	Compare by buyer segment after enough sends	Keep if qualified replies follow	Human replies / delivered tailored messages; exclude bots
MOFU	New inbound leads per month from organic	0-2 scenario	3-5	5-8 if evidence supports	Accepted forms or manually logged inbound distinct organizations with organic first touch
BOFU	Leads from paid	N/A; 0 GEL spend	N/A unless gated test approved	Scenario only after budget; no target yet	Accepted leads plus actual spend and held demos; do not count clicks as leads
LOFU	Referral/word-of-mouth leads	Record source, possibly 0	1-2 scenario	2-3 conditional	Ask on first call; de-duplicate buying organizations
TOFU/MOFU	Community contributions that become conversations	1-2 useful answers; no conversion promise	2-4 named conversations/month	Continue only if qualified	Manual source note linked to actual counterpart

Stage	Metric	Week 4 planning check	Month 3 planning check	Month 6 conditional check	How to measure
North star	**Cumulative held qualified demos from distinct hospital buying organizations**	**2 scenario**	**12 scenario**	Set only after day-90 evidence	CRM held date + qualification fields + distinct organization ID

The leading indicator is **held qualified hospital demos**, counted once per buying organization in this 90-day acquisition scenario. A booked calendar slot, repeat meeting with the same group, webinar registration, or anchor-account expansion discussion is tracked separately. More traffic is useful only when it produces these meetings.

Sources and limits: shared writer brief, research addendum, source demo route. Channel percentages and posting windows in the library were supplied as prompts, not independently validated for this buyer. Handoff to Prompt 6: use the same content IDs, distinct-organization north star, form acceptance gate, and 300-minute weekly ceiling.

16 Part 2 · Prompt 6

Tracking, Mapping & Implementation

Source: Complete library, pages 51-55 · 7,985 words · 6 requested steps / sections

Inputs and status (evidence snapshot 22 September 2026). The funnel has five stages: TOFU problem recognition, MOFU evaluation, BOFU held qualified pathway demo and scoped evaluation decision, AFU activation/retention after a customer agreement, and LOFU permissioned advocacy. The 85 stable content IDs T01-L17 and page IDs P01-P11 are defined in `working/content-catalog.json`; the distribution priorities and 300-minute weekly routine are in Prompt 5. Primary conversion: **one held qualified 25-minute demo per distinct hospital buying organization**. The 90-day planning scenario is 12 such demos, then three readiness-approved evaluations and one *new* paying buying organization as stretch assumptions. Baselines, marketing analytics, CRM, opted-in email platform and founder technical comfort are unverified. The website is a React app; `/request-demo` exists, but source inspection found a three-step form whose submit handler waits, logs data locally, and shows a receipt message without a backend request. This is a code finding, not a deployed-site test or proof of lost leads. It is the first implementation risk. Assume a founder can use a spreadsheet; one-time tagging/form engineering needs a technical owner. No analytics, CRM, email, pixels or external pages were installed by this document.

Read the metrics correctly. The source library supplies benchmark percentages for broad B2B funnels. They are **unverified library comparators**, not MediMind's historical rates, Georgian hospital norms, or automatic targets. Below, absolute gates protect the launch; percentages become directional only after a meaningful denominator. A zero denominator is **N/A**, never 0% or red. Report both numerator and denominator and label any rate from fewer than 20 qualified organizations or fewer than 100 relevant page sessions **provisional**. Site traffic, LinkedIn impressions, booked calls, held meetings, trials and paying organizations are different events.

Part 1 - Funnel tracking system

Event and entity definitions. A public-site visit is a unique GA4 user/session on the marketing site, after internal-traffic filtering. An accepted lead has a successful backend receipt and CRM record; a button click or client-side “thank you” state is not one. A qualified hospital organization is a deduplicated legal buying group with a relevant cross-department need, a decision/influence route, a candidate owner, and a plausible evaluation path. A *held qualified demo* has an actual meeting date, the

defined qualification fields, and a unique organization ID; reschedules, repeats, anchor-account conversations and two attendees from the same group do not inflate the north star. An evaluation is readiness-approved only after scope, owners, access, baseline and stop criteria are agreed. Paid means a signed commercial agreement **and first revenue recognized/collected as the business defines it**, with contracted versus received amounts separately labelled. Store organization identity and contact details in an access-controlled CRM, never in GA4 parameters.

Stage	Metric and what it tells you	How to measure	Green	Amber	Red	Fix
TOFU	Relevant content reach	LI impressions and named target-account interactions by asset ID	Qualified interaction rises over 4 weeks	Reach rises but no relevant interaction	No relevant interaction after 4 consistent posts	Narrow topic and audience
TOFU	Engagement rate	Meaningful LI interactions / impressions	Direction improves and target buyers respond	Rate only, buyer fit unclear	0 relevant replies/comments across 4 posts	Rewrite hook around observed problem
TOFU	Marketing-site visitors from content	GA4 sessions by external source/content ID	Source yields a qualified inquiry	Visits only	Tagged external links produce no visits after QA	Check links, placement, message
TOFU	Accepted new leads	Backend-confirmed, deduplicated organization records	≥1 new qualified inquiry/week in scenario	Inquiry exists but fit uncertain	0 after verified traffic and working form	Inspect form and CTA, then channel
TOFU	Cost per lead, if paid	Actual spend / accepted leads	Within approved acquisition economics	Cost unknown or sample tiny	Spend with no accepted lead after pre-set test limit	Stop/pause test; inspect tracking
MOFU	Lead → demo request	Unique organizations requesting / eligible new-lead organizations	Rising with named buyer need	Flat, provisional	No requests after five qualified conversations	Improve proof and invitation
MOFU	Email opens	Unique opens / delivered opted-in emails	Directional only; library says 20-30% broadly	Low opens but clicks/replies exist	No opens and no clicks after deliverability check	Check consent/delivery, subject; do not over-read pixel

Stage	Metric and what it tells you	How to measure	Green	Amber	Red	Fix
MOFU	Email clicks	Unique clickers / delivered	Useful replies or demo requests; library says 2-5% broadly	Clicks without action	No click after relevant sends	Match CTA to promise
MOFU	Webinar registration	Accepted registrations / unique webinar page sessions	Event-specific baseline improves	Visits but date/form unclear	No accepted registration after launch QA	Fix event details or form
MOFU	Webinar attendance	Unique live attendees / accepted registrants	Attendance and useful questions	Registrations but many no-shows	Event did not run or attendee data absent	Reminder and relevance; N/A before event
MOFU	Case-study visits and CTA clicks	Verified P03 sessions and CTA events	Buyers inspect a permissioned proof asset	Visits without demo moves	No valid case study exists	Keep placeholder; do not fabricate proof
MOFU	Lead capture → demo request time	Median days, CRM timestamps by organization	Shorter with fit intact	Long but next action owned	Contact has no owner/next date	Assign follow-up
BOFU	Demo page conversion	Accepted demo requests / unique P05 sessions	Improving with qualified fit	Library 3.8% median is only a comparator	Form acceptance fails at all	Repair backend first; then simplify friction
BOFU	Held demo → approved evaluation	Unique approved evaluations / unique held qualified-demo organizations	Scenario reaches 3 from 12 by day 90	Scope questions open	No evaluation owner after several demos	Refine qualification and scope
BOFU	Sandbox signup → activation	Activated synthetic workspaces / accepted sandbox accounts	Directional; library says 60%+ for mature trial	Opened but no completed pathway	Signup flow or pathway broken	Improve guided first session

Stage	Metric and what it tells you	How to measure	Green	Amber	Red	Fix
BOFU	Time to first value	Median from accepted access to first synthetic pathway map	Within first guided session, proposed	Later than first session	Never reaches map	Remove setup steps; review support
BOFU	Sandbox → paid	New paying organizations / trial organizations with completed observation window	Measured, not benchmark-driven	N/A until launched and matured	Trial creates no buyer evaluation	Reassess fit/offer; 15-30% library figure unverified here
BOFU	Conversion-page conversion	Accepted form completions / unique relevant sessions on P04/P05/P11 separately	Page-specific improvement	Traffic without enough denominator	False success or broken handler	Fix event acceptance and page first
AFU	Month-one retention	Active contracted organizations at day 30 / contracted organizations eligible for day-30 check	Customer retained and using agreed workflow	Usage unclear	Exit or non-use attributable to unresolved issue	Customer-success review; library 46.9% not local norm
AFU	Activation rate	Organizations accepting first agreed pathway / eligible signed organizations	First pathway accepted with sign-off	Work open with owner	No sign-off or unmanaged safety issue	Fix delivery and acceptance criteria
AFU	Expansion revenue	Additional recurring amount signed/recognized, separate from base	Evidenced growth without hidden cost	Discussion only	Churn/contraction	Address value and support

Stage	Metric and what it tells you	How to measure	Green	Amber	Red	Fix
AFU	Satisfaction/NPS	One standardized post-milestone question + denominator	Positive qualitative signal with specific reason	Mixed/too few answers	Repeated unresolved criticism	Close loop; do not calculate NPS on tiny sample
AFU	Support burden	Open/aged tickets per active organization, severity	Issues owned, safety escalated	Backlog growing	Critical issue without owner	Assign and escalate
LOFU	Referral rate	New qualified buying organizations explicitly introduced / eligible satisfied customers	Permissioned introductions emerge	Satisfied users but no ask yet	Dissatisfaction or pressure to refer	Improve customer experience first
LOFU	Review submission rate	Authentic posted reviews / customers eligible and asked with consent	A real detailed review	No public review, customer satisfied	Incentivized/inauthentic request	Stop request, clarify consent
LOFU	Case-study participation	Customers consenting to measured story / eligible customers with verified result	One complete authorized story	Evidence/permission pending	Claim drafted without approval	Hold publication
LOFU	Word-of-mouth leads	Unique buyer organizations naming a peer/private share	Source noted in CRM	Source “unknown” but inquiry real	Repeated unattributed traffic with no source question	Ask “How did you hear?”

Library comparator note. The supplied prompt also lists homepage lead capture 2-5%, webinar page 25-45%, trial-to-paid 15-30%, trial activation 60%+, LinkedIn engagement 1-3%, outreach replies 10-15%, referrals 20-30% of new customers, and other “top performer” rates. These have no cited research method in the library. Use them only to form questions after a real local baseline; never make a red/green decision from them alone.

Part 2 - Tool setup guide

The current stack is not confirmed, so the tools below are **recommendations**. No-code work should be possible for daily use; engineering is needed for reliable React-route events and accepted form submissions. Use the minimum set that records a real lead and next action before adding behavior tracking.

Priority / category	Recommended tool; what it tracks	Plain setup and acceptance check	Free alternative
Must now - website analytics	GA4 for public marketing page visits, source, CTA intent and accepted conversion	Create account/property/web stream, choose Georgia reporting timezone, add one tag to public marketing routes, verify in Realtime. Check React history-route page views once (avoid duplicate manual pageviews). Record `cta_click` with safe `content_id` and `destination_id` for each live public CTA as **intent**, not conversion. Register distinct accepted events for `lead_magnet_download`, `email_signup`, `webinar_registration_accepted`, `demo_request_accepted`, `sandbox_signup_accepted`, and `sandbox_pathway_completed`; later paid status comes from reconciled CRM and may be represented by a carefully designed offline `paid_customer_recorded` event, never a browser button. Send success events only after the real system accepts them. Build a simple path exploration from landing → P05 → accepted request. Never send names, email, free text, patient information, or organization IDs.	Privacy-conscious aggregate server log or manual source/accepted-form tally until analytics is ready; no false precision.

Priority / category	Recommended tool; what it tracks	Plain setup and acceptance check	Free alternative
Must now - CRM	HubSpot default deal pipeline **or** a controlled spreadsheet for unique organizations, source ID, stage, owner, next action, held demo, evaluation and payment	Make one organization record per buying group; use fields `first_source`, `latest_source`, `content_id`, `qualification`, `demo_held_date`, `evaluation_readiness`, `next_owner`, `next_date`. Start from the default pipeline or a sheet; verify one test request creates a record and task. Do not assume custom-pipeline creation or automation is free.	Protected spreadsheet with a weekly follow-up filter and manual tasks.
Must now when sending permissioned email - email platform	Mailchimp or equivalent for opted-in delivery, unsubscribes, opens and clicks	Import only consenting contacts; segment interested prospects versus customer onboarding; write a short delivery/one-follow-up sequence before automation; test unsubscribe, delivery and link tracking. Platform automation and plan limits vary. Cold one-to-one outreach stays in the CRM, not an opted-in nurture list.	Manual permission-based email plus consent log and reply counts.
Must now - LinkedIn analytics	Native personal-post analytics for impressions, reactions, comments, reposts and relevant conversations	Record asset ID, date, impressions at seven days, meaningful buyer interactions, and tagged external-link visits weekly; use an external UTM link on an actual CTA, not on LinkedIn's internal features.	Weekly manual post log.
Month 2 only after privacy review - heatmaps/recordings	Microsoft Clarity, public marketing pages only, for drop-off and usability	Limit tag placement to approved public routes. Verify masking, consent handling, page exclusions, and a synthetic test recording before viewing any real session. Never install on EMR, clinical app, investor/staff private routes or patient-data pages; disable if exclusion cannot be guaranteed.	Manual moderated usability test with a synthetic form; no recording script.

Priority / category	Recommended tool; what it tracks	Plain setup and acceptance check	Free alternative
When paid is approved - ad tracking	Google Ads/LinkedIn Campaign Manager as applicable, spend and accepted conversions	Create one conversion for true accepted lead, use one tag implementation, test source/UTM and deduplication; do not count button click or client-side success as a conversion. Check consent and public-route audience exclusions before any retargeting.	CRM campaign ID and manual spend/held-demo log for tiny tests.
Month 2 or later, evidence-ready - review/proof tracking	G2/Capterra/Trustpilot listing where relevant, permissioned reviews and proof requests	Claim a listing only after verifying official product identity and public claims; ask a satisfied customer for an honest review after an agreed milestone, not automatically on day 30. Log request/permission/publication and unresolved criticism.	Internal permission and reference log; publish no public review claim until one exists.

Fresh official setup and limitation references: GA4 web setup, GA4 enhanced page-view behavior, GA4 recommended lead events, GA4 PII policy, HubSpot pipeline limits, Mailchimp automation and plan caveat, Mailchimp open tracking limitations, LinkedIn post analytics, Clarity setup, Clarity masking, Google Ads conversion setup.

Part 3 - UTM tracking system

A UTM tag is a short set of words added to a **link shared outside the site** so analytics can tell whether a visitor arrived from a founder post, email, partner or ad and which piece sent them. It does not identify a person or prove that channel caused the purchase. Keep identifiable names, hospital names, email addresses, clinical details and free-text answers out of URLs and analytics. Google explains how manually tagged campaign dimensions appear in GA4 here.

Naming convention. Lowercase ASCII and underscores only. `utm_source` = platform or sender class (`linkedin`, `email`, `partner`, `hospital_forum`, `direct_outreach`, `google`, `referral`); `utm_medium` = `social`, `email`, `partner`, `community`, `outreach`, `cpc`, or `referral`; `utm_campaign` = `one_patient_2026q4` for this campaign, never a person/account name; `utm_content` = catalog ID plus version (`t01_v1`, `m05_v1`) and optional approved variation (`_a`, `_b`). Source and medium describe *arrival*; content identifies the specific creative. Preserve both first-touch and latest external touch in CRM; if the buyer says a colleague forwarded it privately, record “self-reported peer forward” too. Do not overwrite a first source with an internal click.

Destinations. P01 `/pathway-assessment`; P02 `/hospital-os`; P03 `/case-study`; P04 `/webinar`; P05 `/request-demo`; P06 `/early-partner`; P07 `/compare`; P08 `/activation`; P09 `/referrals`; P10 `/evidence`; P11 `/sandbox`. These are **planned path names** except P05, the existing route. Even P05 must not be promoted until real backend acceptance, receipt and scheduling are checked. Do not share a planned URL until its page, form, analytics and privacy behavior work. For organic search landings and internal navigation, the URL is clean, without UTMs. For private customer documents, use a controlled link and CRM source note rather than a public tracking URL.

Complete 85-piece channel-link library. A row with a public external channel supplies a ready link structure; "private/none" is a deliberate no-UTM rule. The table is keyed to the exact catalog ID and its planned destination, with one line for every catalog piece. The "Native…" line shows the **clean destination URL** for search, direct access or internal navigation; it does not mean an externally shared LinkedIn, email, outreach, community or partner CTA should use that clean link. Use the matching tagged link on every controllable external CTA. Where a second public distribution channel is approved, use the same destination and ID with its own listed `source/medium`; this keeps the content identity while distinguishing the sender. Links are **planning URLs**, not a statement that the page exists.

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T01 - One patient, seven handoffs	li + dir	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t01_v1` dir: `https://medimind.md/pathway-assessment?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=t01_v1`	T01 source/medium visits and accepted downstream action; P01 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T02 - The hidden cost of searching	li + email	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t02_v1` email: `https://medimind.md/pathway-assessment?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=t02_v1`	T02 source/medium visits and accepted downstream action; P01 destination
T03 - When two reports disagree	li + dir	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t03_v1` dir: `https://medimind.md/pathway-assessment?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=t03_v1`	T03 source/medium visits and accepted downstream action; P01 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T04 - The duplicate-entry trail	li	Native Short video / Founder LinkedIn video: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t04_v1`	T04 source/medium visits and accepted downstream action; P01 destination
T05 - Three calls for one result	li + dir	Native Short video / Founder LinkedIn video: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t05_v1` dir: `https://medimind.md/pathway-assessment?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=t05_v1`	T05 source/medium visits and accepted downstream action; P01 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T06 - The spreadsheet behind the dashboard	li	Native Short video / Founder LinkedIn video: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t06_v1`	T06 source/medium visits and accepted downstream action; P01 destination
T07 - How to map a patient handoff	li	Native Blog/SEO / Website search: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t07_v1`	T07 source/medium visits and accepted downstream action; P01 destination
T08 - Why hospital reports disagree	li	Native Blog/SEO / Website search: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t08_v1`	T08 source/medium visits and accepted downstream action; P01 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T09 - What a delayed claim does and does not mean	li	Native Blog/SEO / Website search: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t09_v1`	T09 source/medium visits and accepted downstream action; P01 destination
T10 - Patient-pathway friction worksheet	dir	Native Lead magnet / Website and founder outreach: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/pathway-assessment?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=t10_v1`	T10 source/medium visits and accepted downstream action; P01 destination
T11 - Five-minute handoff check	li	Native Lead magnet / Website and founder outreach: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t11_v1`	T11 source/medium visits and accepted downstream action; P01 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T12 - Founder-built scope trace	li	Native Proof / Website and founder LinkedIn: `https://medimind.md/hospital-os` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/hospital-os?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t12_v1`	T12 source/medium visits and accepted downstream action; P02 destination
T13 - Synthetic handoff before/after	li	Native Proof / Website and founder LinkedIn: `https://medimind.md/hospital-os` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/hospital-os?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t13_v1`	T13 source/medium visits and accepted downstream action; P02 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T14 - Where the work repeats	email + dir	Native Email / Opt-in email: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/pathway-assessment?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=t14_v1` dir: `https://medimind.md/pathway-assessment?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=t14_v1`	T14 source/medium visits and accepted downstream action; P01 destination
T15 - A report you had to rebuild	email + dir	Native Email / Opt-in email: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/pathway-assessment?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=t15_v1` dir: `https://medimind.md/pathway-assessment?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=t15_v1`	T15 source/medium visits and accepted downstream action; P01 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T16 - Hospital-operator discussion prompt	forum + li	Native Community / Relevant hospital-leadership forum: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) forum: `https://medimind.md/pathway-assessment?utm_source=hospital_forum&utm_medium=community&utm_campaign=one_patient_2026q4&utm_content=t16_v1` li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t16_v1`	T16 source/medium visits and accepted downstream action; P01 destination
T17 - Clinical-operations partner note	partner	Native Community / Professional association or introduction: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) partner: `https://medimind.md/pathway-assessment?utm_source=partner&utm_medium=partner&utm_campaign=one_patient_2026q4&utm_content=t17_v1`	T17 source/medium visits and accepted downstream action; P01 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
M01 - Fix a screen or fix the pathway?	li + dir	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/compare` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/compare?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m01_v1` dir: `https://medimind.md/compare?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m01_v1`	M01 source/medium visits and accepted downstream action; P07 destination
M02 - What parallel evaluation actually requires	li + email	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/early-partner?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m02_v1` email: `https://medimind.md/early-partner?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=m02_v1`	M02 source/medium visits and accepted downstream action; P06 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
M03 - AI assists; clinicians decide	li	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/hospital-os` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/hospital-os?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m03_v1`	M03 source/medium visits and accepted downstream action; P02 destination
M04 - Three options on one whiteboard	li	Native Short video / Founder LinkedIn video: `https://medimind.md/compare` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/compare?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m04_v1`	M04 source/medium visits and accepted downstream action; P07 destination
M05 - What happens in 25 minutes	li + dir	Native Short video / Founder LinkedIn video: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/request-demo?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m05_v1` dir: `https://medimind.md/request-demo?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m05_v1`	M05 source/medium visits and accepted downstream action; P05 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
M06 - A result becomes a task	li	Native Short video / Founder LinkedIn video: `https://medimind.md/hospital-os` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/hospital-os?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m06_v1`	M06 source/medium visits and accepted downstream action; P02 destination
M07 - How to evaluate hospital software without a risky cutover	li	Native Blog/SEO / Website search: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/early-partner?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m07_v1`	M07 source/medium visits and accepted downstream action; P06 destination
M08 - Hospital OS versus overlay versus current vendor change	dir	Native Blog/SEO / Website search: `https://medimind.md/compare` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/compare?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m08_v1`	M08 source/medium visits and accepted downstream action; P07 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
M09 - What FHIR R4 changes in hospital work	li	Native Blog/SEO / Website search: `https://medimind.md/hospital-os` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/hospital-os?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m09_v1`	M09 source/medium visits and accepted downstream action; P02 destination
M10 - Parallel-evaluation readiness checklist	dir	Native Lead magnet / Website and founder outreach: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/early-partner?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m10_v1`	M10 source/medium visits and accepted downstream action; P06 destination
M11 - Three-option decision worksheet	dir	Native Lead magnet / Website and founder outreach: `https://medimind.md/compare` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/compare?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m11_v1`	M11 source/medium visits and accepted downstream action; P07 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
M12 - Documented domain map	dir	Native Proof / Technical diligence and website: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/evidence?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m12_v1`	M12 source/medium visits and accepted downstream action; P10 destination
M13 - Live-pathway evidence template	dir	Native Proof / Website and founder follow-up: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/evidence?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m13_v1`	M13 source/medium visits and accepted downstream action; P10 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
M14 - The change-risk question	email + dir	Native Email / Opt-in email: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/early-partner?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=m14_v1` dir: `https://medimind.md/early-partner?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m14_v1`	M14 source/medium visits and accepted downstream action; P06 destination
M15 - The current-vendor alternative	email + dir	Native Email / Opt-in email: `https://medimind.md/compare` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/compare?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=m15_v1` dir: `https://medimind.md/compare?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m15_v1`	M15 source/medium visits and accepted downstream action; P07 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
M16 - Peer discussion on evaluation design	private + email	Native Community / Invited operator roundtable: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only email: `https://medimind.md/early-partner?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=m16_v1`	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
M17 - IT and clinical review session	partner	Native Community / Professional association or partner session: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) partner: `https://medimind.md/early-partner?utm_source=partner&utm_medium=partner&utm_campaign=one_patient_2026q4&utm_content=m17_v1`	M17 source/medium visits and accepted downstream action; P06 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
B01 - What your 25-minute demo will settle	li + dir	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/request-demo?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=b01_v1` dir: `https://medimind.md/request-demo?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b01_v1`	B01 source/medium visits and accepted downstream action; P05 destination
B02 - Match price, define total cost	li + dir	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/early-partner?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=b02_v1` dir: `https://medimind.md/early-partner?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b02_v1`	B02 source/medium visits and accepted downstream action; P06 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
B03 - The honest go/no-go test	li + email	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/request-demo?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=b03_v1` email: `https://medimind.md/request-demo?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=b03_v1`	B03 source/medium visits and accepted downstream action; P05 destination
B04 - Demo in sixty seconds	li	Native Short video / Founder LinkedIn video: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/request-demo?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=b04_v1`	B04 source/medium visits and accepted downstream action; P05 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
B05 - Parallel beside current work	li	Native Short video / Founder LinkedIn video: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/early-partner?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=b05_v1`	B05 source/medium visits and accepted downstream action; P06 destination
B06 - Who should join the first call?	li + email	Native Short video / Founder LinkedIn video: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/request-demo?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=b06_v1` email: `https://medimind.md/request-demo?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=b06_v1`	B06 source/medium visits and accepted downstream action; P05 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
B07 - Questions to ask before a hospital OS demo	li	Native Blog/SEO / Website search: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/request-demo?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=b07_v1`	B07 source/medium visits and accepted downstream action; P05 destination
B08 - What happens after a parallel evaluation	dir	Native Blog/SEO / Website search: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/early-partner?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b08_v1`	B08 source/medium visits and accepted downstream action; P06 destination
B09 - What current-software price match should include	dir	Native Blog/SEO / Website search: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/early-partner?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b09_v1`	B09 source/medium visits and accepted downstream action; P06 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
B10 - Pathway demo preparation card	email	Native Lead magnet / Demo confirmation email and website: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/request-demo?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=b10_v1`	B10 source/medium visits and accepted downstream action; P05 destination
B11 - Evaluation decision sheet	dir	Native Lead magnet / Founder sales follow-up: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/early-partner?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b11_v1`	B11 source/medium visits and accepted downstream action; P06 destination
B12 - Verified implementation timeline	dir	Native Proof / Founder sales follow-up: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/case-study?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b12_v1`	B12 source/medium visits and accepted downstream action; P03 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
B13 - Buyer-approved workflow result	dir	Native Proof / Website and founder sales follow-up: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/evidence?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b13_v1`	B13 source/medium visits and accepted downstream action; P10 destination
B14 - Your pathway before our demo	email	Native Email / Demo-confirmation email: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/request-demo?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=b14_v1`	B14 source/medium visits and accepted downstream action; P05 destination
B15 - What we will compare	email	Native Email / Founder follow-up email: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/early-partner?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=b15_v1`	B15 source/medium visits and accepted downstream action; P06 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
B16 - Private buying-committee walkthrough	private + dir	Native Community / Private hospital meeting: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only dir: `https://medimind.md/request-demo?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b16_v1`	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
B17 - Procurement questions session	private + dir	Native Community / Private hospital meeting: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only dir: `https://medimind.md/early-partner?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b17_v1`	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
A01 - First pathway, first baseline	private + li	Native LinkedIn post / Customer-only LinkedIn or direct update: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only li: `https://medimind.md/activation?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=a01_v1`	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A02 - What staff should report in week one	private + li	Native LinkedIn post / Customer-only update: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only li: `https://medimind.md/activation?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=a02_v1`	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
A03 - One improvement, then the next	private + li	Native LinkedIn post / Customer-only update: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only li: `https://medimind.md/activation?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=a03_v1`	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A04 - How to log a handoff issue	private	Native Short video / Customer onboarding: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A05 - Five-minute pathway review	private	Native Short video / Customer onboarding: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A06 - How a testable change becomes approved	private	Native Short video / Customer onboarding: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
A07 - Measuring repeat work without double counting	private	Native Blog/SEO / Customer knowledge base: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A08 - Running a safe pathway review	private	Native Blog/SEO / Customer knowledge base: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A09 - Selecting the second pathway	private	Native Blog/SEO / Customer knowledge base: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A10 - First-pathway measurement sheet	private	Native Lead magnet / Customer onboarding: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
A11 - Change-request decision card	private	Native Lead magnet / Customer onboarding: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A12 - Accepted-workflow log	private	Native Proof / Private customer review: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A13 - Measured improvement record	private	Native Proof / Private customer review: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A14 - Your first review agenda	private	Native Email / Customer email: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A15 - A request ready for test	private	Native Email / Customer email: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
A16 - Clinical-IT issue huddle	private	Native Community / Private customer meeting: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A17 - Cross-site learning exchange	private	Native Community / Private customer meeting: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
L01 - The buyer's permission comes first	li	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/evidence?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l01_v1`	L01 source/medium visits and accepted downstream action; P10 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
L02 - A pathway another leader can inspect	li + dir	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/case-study?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l02_v1` dir: `https://medimind.md/case-study?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=l02_v1`	L02 source/medium visits and accepted downstream action; P03 destination
L03 - What we changed after feedback	li	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/evidence?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l03_v1`	L03 source/medium visits and accepted downstream action; P10 destination
L04 - Customer pathway walkthrough	li	Native Short video / Founder and customer LinkedIn: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/case-study?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l04_v1`	L04 source/medium visits and accepted downstream action; P03 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
L05 - What a reference actually verified	li	Native Short video / Founder LinkedIn video: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/evidence?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l05_v1`	L05 source/medium visits and accepted downstream action; P10 destination
L06 - Why we chose the first pathway	li + dir	Native Short video / Founder and customer LinkedIn: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/case-study?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l06_v1` dir: `https://medimind.md/case-study?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=l06_v1`	L06 source/medium visits and accepted downstream action; P03 destination
L07 - How to read a hospital software case study	li	Native Blog/SEO / Website search: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/evidence?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l07_v1`	L07 source/medium visits and accepted downstream action; P10 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
L08 - From one hospital pathway to another	dir	Native Blog/SEO / Website search: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/case-study?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=l08_v1`	L08 source/medium visits and accepted downstream action; P03 destination
L09 - What changed in a claim handoff	li	Native Blog/SEO / Website search: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/case-study?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l09_v1`	L09 source/medium visits and accepted downstream action; P03 destination
L10 - Reference-story evidence checklist	partner	Native Lead magnet / Website and partner sharing: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) partner: `https://medimind.md/evidence?utm_source=partner&utm_medium=partner&utm_campaign=one_patient_2026q4&utm_content=l10_v1`	L10 source/medium visits and accepted downstream action; P10 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
L11 - Peer evaluation guide	dir	Native Lead magnet / Website and founder introduction: `https://medimind.md/referrals` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/referrals?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=l11_v1`	L11 source/medium visits and accepted downstream action; P09 destination
L12 - Buyer-approved measured case study	li	Native Proof / Website and customer-approved sharing: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/case-study?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l12_v1`	L12 source/medium visits and accepted downstream action; P03 destination
L13 - Reference conversation	private + dir	Native Proof / Private founder introduction: `https://medimind.md/referrals` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only dir: `https://medimind.md/referrals?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=l13_v1`	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
L14 - Share the method, not a claim	private	Native Email / Customer email: `https://medimind.md/referrals` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
L15 - A peer may have this problem	email + dir	Native Email / Customer email: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/case-study?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=l15_v1` dir: `https://medimind.md/case-study?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=l15_v1`	L15 source/medium visits and accepted downstream action; P03 destination
L16 - Peer operator exchange	private	Native Community / Private operator introduction: `https://medimind.md/referrals` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
L17 - Hospital-association learning session	partner	Native Community / Professional association, if arranged: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) partner: `https://medimind.md/evidence?utm_source=partner&utm_medium=partner&utm_campaign=one_patient_2026q4&utm_content=l17_v1`	L17 source/medium visits and accepted downstream action; P10 destination

Tracking rule and QA. Tag every controllable **external** link into a public funnel page. Do not append UTMs to internal site navigation, organic search results, in-app paths, private customer records or links carrying identifiers. Keep natural/direct traffic as “direct/unknown” rather than creating fake attribution. Before release, open one test link for every active source/medium, confirm that it resolves to the intended live page, appears once in GA4 source/campaign/content, and survives the React navigation. Send an end-to-end test form and check backend receipt, CRM record, owner/task, true accepted event, failure error, consent/unsubscribe where applicable, and no PII in network analytics requests or browser URL. Only then mark a link “ready.”

Part 4 - Weekly tracking routine

Daily, five minutes (Monday-Friday; 25 minutes/week). Look first at the CRM count of cumulative unique **held qualified demos** versus the 13-week pace, and at yesterday's accepted requests and owner assignment. A day with no demo is normal; a new unowned request or a form test that fails is not. If the pace is behind, inspect real accepted leads and next actions before looking at impressions. The CRM is the source of truth for the north star; GA4 is the source for site traffic. This five-minute block is included in Prompt 5's 300-minute total.

Weekly, 30 minutes Monday (includes Prompt 7's 15-minute report). In 5 minutes, record TOFU relevant reach, tagged site visits and accepted new leads; 5 minutes, MOFU lead-to-demo, permissioned email and any webinar/case asset (N/A if unlaunched); 5 minutes, BOFU held demos, evaluation decisions and sandbox first value (N/A if absent); 3 minutes, AFU activation/retention and support (N/A before customers); 2 minutes, LOFU referrals/reviews (N/A before eligible customers); 5 minutes, reconcile UTM first/latest source with self-reported source and check CRM owners/aging; 5 minutes, write the single biggest leak and next action. All rates show numerator/denominator. An absent product stage is N/A, not red.

Monthly, two hours on first Monday, replacing ordinary creative work that week. 20 minutes verify raw accepted submissions, distinct organizations and source completeness; 25 minutes compare asset ID → qualified conversations → held demos; 20 minutes compare the top two channels by *qualified* meetings, not clicks; 20 minutes inspect stage drop-offs, median lead-to-demo time and support/evaluation workload; 20 minutes reconcile month-over-month revenue and cohort eligibility; 15 minutes choose one asset to improve, one channel to reduce, and one experiment with a clear stop rule. Prompt 7's 30-minute monthly review is the summary produced inside this two-hour analysis, not additional founder time. In the first-month deep-dive week, the 120 minutes leaves 125 of the 300 weekly minutes for distribution after daily and weekly checks; no second 5-hour allocation is assumed.

Part 5 - Week-by-week implementation plan

| Window | Deliverable and sequence | Acceptance gate and dependency | |---|---| | Week 1 - foundation | Verify the actual deployed form; implement a real accepted request and explicit failure state with an owner; create the minimal organization CRM/sheet; define IDs and link rules; install GA4 on public marketing routes only; test React pageviews and PII exclusion. Claim a review listing only if verified and low effort; it is not a gate. No broad campaign publication. | A synthetic request appears in backend/CRM exactly once, reaches the right human, records a next action, and does **not** emit an accepted event on failure. Technical work is separate from founder marketing time. | | Week 2 - minimum viable funnel | Bring three priority assets into a verified path: working P05 demo page, P01 pathway worksheet/assessment if built, and T04 synthetic phone clip if filming and review capacity exists. Prepare permissioned follow-up email T14 only if opt-in exists. | Every external link resolves, source ID arrives, accepted lead is recorded, receipt is truthful, and a founder can conduct the 25-minute meeting. If P01 or T04 is late, use the verified P05 route and a plain pathway sketch; do not claim three assets are live. | | Week 3 - first campaign | Add M10 readiness checklist and adapt M02 into a founder post, each subject to review and a functioning destination. Begin a 14-day campaign test **only after** P05 receipt/owner, the needed landing page, the assets, UTMs, privacy wording and actual campaign capacity pass QA. | No real patient data, approved claims, accurate 25-minute promise, and one source-to-held-demo path tested. If not ready, continue direct outreach and the worksheet; the 14-day clock has not started. | | Week 4 - routine | Run Prompt 5's five-hour schedule, record each live content ID and external UTM link, and fill the first weekly scorecard. Produce B04 60-second synthetic demo preview only if capacity and P05 QA allow. | No missing source or unowned request; 2 cumulative unique held qualified demos is a **scenario check**, not an asserted outcome. | | Weeks 5-6 - conversion layer | Prioritized backlog from Prompt 1: M08, M10, B10, B11, B14, B15, M12. Reuse M10 if already produced in week 3; build the next needed items only as buyer questions, evidence and production capacity justify. Tighten the demo page; add the proposed sandbox only after product, support, billing and data policies are approved. | This is a sequence, **not a seven-asset production quota** or extra five-hour allocation. Held meetings, form reliability and customer delivery take priority. | | Weeks 7-8 - review | Run the first monthly deep dive, identify the biggest verified leak, improve one page/offer or source, and stop a channel that consumes time without qualified conversations. | Four cumulative held qualified demos is the planning pace; record sample size and reasons for any gap. | | Month 3 - selective growth | Start a partner contribution or OPA appearance if audience is confirmed. Activate paid only with budget, clean consent, working conversion, adequate audience, a test cap, and capacity for follow-up. Add review/reference material only from permissioned verified evidence. | Day-90 scenario: 12 held qualified demos from distinct organizations, 3 readiness-approved evaluations, 1 new paying organization as stretch. If prerequisites fail, remain in targeted organic/direct motion. |

Part 6 - 90-day success dashboard

The following is **illustrative funnel math**, not a forecast: research 60 distinct target organizations over roughly 13 weeks (about 5/week); if 30 engage in a substantive exchange (50%), 18 accept a demo slot (60%), and 12 hold a qualified demo (67% of booked), the north star reaches 12. If 25% of those 12 approve an evaluation after readiness checks, that is 3; if one of those three becomes a *new* paying buying organization, that is 33% of evaluations. None of these rates is measured yet. Website signups can contribute, but outbound and introductions mean page-conversion benchmarks cannot be used alone to derive the target.

Metric	Week 4 scenario	Week 8 scenario	Day 90 scenario	Current baseline	Gap to close
Distinct researched hospital buying organizations	20	40	60	Unknown	Establish list and dedupe
Substantive two-way exchanges	8	18	30	Unknown	Test access/message
Accepted demo bookings, unique organizations	3	7	18	Unknown; current source handler is not proof of accepted leads	Repair capture, qualify
Held qualified demos, unique organizations	**2**	**4**	**12**	**Unknown**	Record held dates and organization IDs
Readiness-approved evaluations	0-1	1	3 stretch	Unknown	Scope, owners, access, baseline
New paying buying organizations	0	0	1 stretch	Founder reports pre-revenue; anchor agreement not counted as new paying	Agree scope and close independently
Public-site visitors, trailing month	100	200	300	Unknown	Tag and measure relevant sources
Accepted lead → CRM owner/task coverage	100% after repair	100%	100%	Source handler not connected	Test every path

One number that matters most: cumulative held qualified demos from distinct hospital buying organizations. Check against a staircase of 2 by week 4, 4 by week 8, and 12 by day 90, adjusting only as a labelled plan after evidence.

Three early warnings in the first 30 days: (1) a test request shows success without a stored lead and named owner; (2) researched, relevant outreach yields no substantive buyer reply across the first ten distinct organizations; (3) no held qualified demo by week 4 despite confirmed capture and conversations. Resolve instrumentation before declaring message failure.

Three growth signals by day 90: (1) 12 distinct held qualified demos, not repeated meetings; (2) at least three named evaluations pass readiness checks with a baseline and hospital owner; (3) one *new* paying organization plus delivery capacity, with acquisition source and commercial terms recorded. If only the first two occur, continue learning rather than claiming revenue traction.

Sources: source form, research addendum, Google GA4 manual campaign dimensions, and official tool docs linked in Part 2. Handoff to Prompt 7: the weekly one-page report draws from the same distinct-organization CRM and never reports N/A as zero.

17 Part 2 · Prompt 7

Reporting

Source: Complete library, pages 56-58 · 2,097 words · 4 requested steps / sections

> **Inputs and current state (evidence snapshot 22 September 2026).** The five-stage funnel is TOFU awareness, MOFU consideration, BOFU held demo/evaluation/commercial decision, AFU customer activation, and LOFU customer advocacy. The primary conversion and north star are **cumulative held qualified 25-minute pathway demos from distinct hospital buying organizations**. The 90-day **scenario**, not forecast, is 12 such demos; three readiness-approved evaluations and one *new* paying organization are stretch downstream assumptions. Prompt 6 defines accepted leads, qualification, source tracking, eligibility and N/A rules. Current baseline for traffic, leads and held qualified demos is unknown. Founder reports pre-revenue, and the inspected `/request-demo` source does not submit to a backend; verify the deployed route before counting accepted forms. The anchor agreement is neither a new paying organization nor proof of network-wide deployment. This report is a template; no investor update was sent. The library's broad conversion percentages are unverified for Georgian hospitals and do not appear as facts in the scorecard.
>
> **Time budget.** The weekly report takes 15 minutes **inside** the Monday 30-minute tracking review, and the monthly 30-minute summary is produced **inside** Prompt 6's two-hour first-Monday deep dive. Those blocks and five daily 5-minute checks are already part of the same five hours of founder marketing time per week. Demo meetings and customer delivery require additional time.

Part 1 - Weekly funnel report

This is one page with **three sections only**. Use a rolling Monday-Sunday week in Georgia local time. Every count is by distinct buying organization where relevant; a rescheduled or repeat meeting does not become a new demo. When an activity has not launched or no organization is eligible, enter **N/A**, not zero. Include count and denominator beside any rate from a small sample.

Section 1 - The numbers

Week ending [date]; owner [name]. The “benchmark” column is a planning gate or due milestone, not a claimed industry norm.

Funnel stage	Metric	This week	Last week	Benchmark / eligibility	Status
TOFU	New accepted leads from distinct relevant hospital organizations	[count]	[count]	Planning check ≥1/week **after** capture works	[Green/Amber/Red/N/A]
MOFU	Eligible lead organizations newly moving to an accepted demo request or guided-trial request	[count]	[count]	Planning check ≥1/week when eligible leads exist; track demo and sandbox separately in detail	[Green/Amber/Red/N/A]
BOFU	Eligible evaluation organizations becoming **new paying** buying organizations	[count]	[count]	Decision due this week? If no, N/A; stretch 1 by day 90	[Green/Amber/Red/N/A]
AFU	New contracted organizations completing their agreed first-pathway activation milestone	[numerator]/[eligible] = [%]	[same]	Compare with agreed due date and sign-off; N/A before any eligible customer	[Green/Amber/Red/N/A]
LOFU	Permissioned referrals or authentic reviews generated by eligible satisfied customers	[count]	[count]	N/A before a satisfied eligible customer or agreed ask date	[Green/Amber/Red/N/A]

Automatic colour logic for Google Sheets. Keep this visible six-column report in A:F. In hidden G put `Eligible?` as `1` only when the stage is live, measurement is verified, and at least one relevant organization is eligible for the week; otherwise `0`. In hidden H put the numerical gate for that row: TOFU `1`, MOFU `1` when eligible, BOFU `1` only if a commercial decision is due, AFU `1` as a 100% fraction of due activations (or the specific agreed milestone), LOFU `1` only when a permissioned referral/review ask is due. Put numeric values in C and D (AFU as a fraction, e.g. `1/2 = 0.5`) and enter N/A only through G. In F2, copy down:

```
=IF(G2=0,"N/A",IF(C2="","N/A",IF(C2>=H2,"Green",IF(C2>0,"Amber","Red"))))
```

For a due AFU cohort with 0/2 completed, this is red; 1/2 amber; 2/2 green. BOFU and LOFU do **not** become red just because a long sales cycle has no payment/referral this week: they are N/A until a real due decision/eligible customer exists. Use conditional formatting on F2:F6 with text equal to `Green`, `Amber`, or `Red` for the three colours; keep `N/A` grey. If data has not been reconciled, leave C blank and the formula returns N/A. This avoids a fake 0% when the denominator is zero, but an eligible, due

opportunity with zero progress is correctly red. **Provisional:** if a rate has fewer than 20 eligible organizations or a page rate fewer than 100 relevant sessions, append “small sample” in the underlying notes and do not infer a market benchmark from colour alone. The 90-day demo pace is shown in Part 4 and discussed in the Monday decision even though the library's five-stage table uses paid conversion for BOFU.

Section 2 - What it means

1. **Where is the funnel healthy this week?** “[Stage] worked because [count of distinct organizations and observed buyer action], with [source/content ID].”
2. **Where is it leaking?** “[Stage] had [numerator]/[denominator] move forward; [named friction or ‘unknown until tested’] is the likely cause.”
3. **What is the single biggest constraint?** “The next held qualified demo is most constrained by [capture / access / trust / evaluation readiness / delivery capacity], evidenced by [one observation].”

Section 3 - The one decision

Next week we will [one owned action] by [date], and know it worked when [one observable result]. If TOFU is red, inspect lead receipt and distribution; if MOFU is red, test content, proof and invitation; if BOFU is red *with a due eligible decision*, fix offer, page or friction; if AFU is red, fix onboarding and first value; if LOFU is red with eligible customers, resolve customer experience before asking for advocacy. If multiple stages are red, fix the earliest confirmed break first. If the `/request-demo` handler still shows success without confirmed receipt, repairing that path is the decision regardless of post reach.

Part 2 - Monthly funnel review

Complete this four-part summary in 30 minutes using the already reconciled two-hour monthly tracking session. Month-over-month percentages use `([this]-[last])/[last]` only when last month is nonzero; otherwise write **N/A (new baseline)**. Compare qualified organizations, not duplicate contacts. For revenue, distinguish contracted recurring value from collected cash and do not count the anchor as a new close.

1. Month in numbers

Metric	Last month	This month	Change	Trend
Unique public-site visitors	[count or N/A]	[count]	[delta; % if denominator >0]	[↑/→/↓/N/A]
New accepted leads, distinct buying organizations	[count or N/A]	[count]	[delta; % if valid]	[↑/→/↓/N/A]

Metric	Last month	This month	Change	Trend
Held qualified demos, distinct buying organizations; guided trials separately	[demos] / [trials]	[demos] / [trials]	[deltas]	[↑/→/↓/N/A]
New paying buying organizations	[count]	[count]	[delta; % if valid]	[↑/→/↓/N/A]
Collected revenue / contracted MRR, labelled separately	[amount] / [MRR]	[amount] / [MRR]	[delta; % if valid]	[↑/→/↓/N/A]
Best content piece by held qualified-demo contribution	[ID or N/A]	[ID plus organization count]	[changed?]	[repeat/test]
Best channel by held qualified-demo contribution	[channel or N/A]	[channel plus organization count]	[changed?]	[repeat/test]

2. What worked

- **[Source/content ID]** generated **[number] held qualified demo(s) from distinct organizations** because **[buyer question or observed behavior]**; [repeat/expand one adaptation next month].
- **[Offer/page or follow-up change]** moved **[number]/[eligible denominator]** relevant organizations forward because **[observed reason]**; [keep or test once more].
- **[Customer/evaluation step, or "N/A-no eligible customer yet"]** achieved **[specific milestone]** because **[verified condition]**; [repeat only if appropriate].

3. What did not work

- **[Source/content ID]** consumed **[minutes]** and produced **[relevant conversations and held demos]**; likely issue **[target/message/landing]**; [fix once or cut].
- **[Funnel step]** lost **[numerator]/[eligible denominator]** because **[confirmed friction or unknown]**; [one repair/test].
- **[Implementation or evidence gate]** stayed open **[days]** and blocked **[asset/channel]**; [owner/date or stop the promise].

4. Next-month priorities

1. **Protect the next held demo:** [highest-impact repair or source action, owner, date, expected observable result].

2. **Improve the largest measured leak:** [one content/page/form/evaluation change, owner, date, success test].
3. **Compound only proven reach:** [one reuse, introduction or partner contribution, with minutes taken from the same five-hour budget].

Part 3 - Investor and advisor update

Use the following one-page draft after replacing brackets with reconciled records. **Current known state for this edition:** founder reports pre-revenue; the baseline number of held qualified demos is unknown; no new paying customer or measured acquisition conversion is asserted. Source inspection revealed a form acceptance risk, so reporting a “demo request” from the client-side success screen would be premature. This is an internal draft and has not been sent.

[X] cumulative held qualified hospital demos from [X] distinct buying organizations - [month and 90-day scenario pace; currently **unknown** until the CRM/meeting record is reconciled].

What happened this month

- **Win:** [One verified buyer action or implementation milestone; if none yet, “No verified held-demo count available this month.”]
- **Learning:** [One observed buyer question and the content/offer change it prompted; no anonymous quote presented as a customer quotation.]
- **Challenge:** The inspected `/request-demo` source currently shows a receipt state after a local console log; [owner] is verifying deployed behavior and implementing confirmed delivery before campaign traffic is sent there.

The funnel in one line: “[X] accepted leads from [Y] distinct hospital buying organizations led to [Z] held qualified demos this month; [A] readiness-approved evaluations and [B] new paying organizations followed-or N/A where the stage has not launched.”

What we learned: “[The single strongest observation], based on [numerator]/[denominator] distinct buying organizations and [source]. We will [one change] next month; this is provisional while the sample is small.”

What we need

- One introduction to a decision-maker at a **Georgian private multi-profile hospital or hospital group** who owns a recurring cross-department patient pathway and can involve a clinical/operations owner and IT counterpart in an evaluation discussion. Confirm relationship and relevance before making any introduction request.

Part 4 - North star dashboard

The north star is the **cumulative number of distinct hospital buying organizations that have held a qualified demo**, not booked slots or repeat calls. Calculation: the 90-day scenario asks for 12 in 13 weeks. With the first two weeks used to fix capture and begin outreach, the average required across the next 11 weeks is `12 ÷ 11 = 1.09` held demos/week. The deliberately back-loaded plan below is **harder** than that average: 2 by week 4, 4 by week 8, then 8 more in weeks 9-13 (`8 ÷ 5 = 1.6`/week). To make that plausible as a planning exercise, Prompt 6 models 60 distinct researched organizations → 30 substantive exchanges → 18 accepted bookings → 12 held qualified demos, then 3 readiness-approved evaluations and 1 new paying organization as a stretch. None of those rates is observed. If early volume does not support the last-five-week pace, revise the plan openly; do not relabel bookings as held demos.

For the last column in a sheet, put the target in B, the reconciled actual count in C, and copy down `=IF(ISNUMBER(C2),IF(C2>=B2,"Yes","No"),"N/A")`; an unknown actual is N/A rather than zero.

Week	Cumulative north-star target	Actual distinct held qualified demos	On track - yes or no
1	0	[count or unknown]	[Yes/No/N/A]
2	0	[count or unknown]	[Yes/No/N/A]
3	1	[count or unknown]	[Yes/No/N/A]
4	2	[count or unknown]	[Yes/No/N/A]
5	2	[count or unknown]	[Yes/No/N/A]
6	3	[count or unknown]	[Yes/No/N/A]
7	3	[count or unknown]	[Yes/No/N/A]
8	4	[count or unknown]	[Yes/No/N/A]
9	5	[count or unknown]	[Yes/No/N/A]
10	7	[count or unknown]	[Yes/No/N/A]
11	9	[count or unknown]	[Yes/No/N/A]

Week	Cumulative north-star target	Actual distinct held qualified demos	On track - yes or no
12	10	[count or unknown]	[Yes/No/N/A]
13 / day 90	12	[count or unknown]	[Yes/No/N/A]

On-track signal: actual cumulative distinct held qualified demos meets or exceeds the current week's scenario target, with source and next action recorded.

Off-track signal: actual is below the target for two consecutive weeks or accepted requests lack a named owner; inspect the weekly report and repair the first confirmed leak.

Pivot signal: after ten distinct relevant buying organizations have had substantive exchanges, none accepts or holds a qualified demo **despite a verified working request/booking path**; revisit the segment, problem and offer before scaling output.

18 Library #0 · Prompt 1

Book a Demo Page

Source: Library #0, pages 3-7 · 1,345 words · 13 requested steps / sections

Status and completed inputs. This is proposed page copy for a founder-led 25-minute pathway demo, not a description of the current `/request-demo` experience. The current source calls it 30 minutes, requires nine fields over three steps, and its submit handler logs data locally before displaying a receipt message; a real delivery and scheduling path needs verification before this page launches. The privacy sentence in Section 12 is proposed policy copy requiring adoption. Pain lines are buyer-language hypotheses, not customer quotes.

Product: MediMind. One-line description: a hospital operating system that brings clinical and business work into a shared patient and operational record. Core mechanic: follow a patient pathway across roles → complete and review work in connected workflows → inspect where information, time, or legitimate charges need attention. Differentiator: founder-led local adaptation plus an agreed, measured parallel evaluation of the hospital's own priority workflow. The FHIR R4 foundation is a technical design choice, not a promised result.

Buyer: CEO, owner, or group operator of a private hospital network or multi-profile hospital in Georgia, with clinical, operations, finance, and IT counterparts. Proposed pain 1: “Why does one patient make so many people enter the same thing?” Pain 2: “I get three versions of the same number.” Pain 3: “Every useful change becomes another request to another vendor.” Likely workarounds to test: spreadsheets, calls, manual reconciliation, current supplier changes, and point tools. Booking fear: an unqualified sales pitch or a risky rip-and-replace proposal. Proposed 90-day success: one agreed pathway measured against its baseline for repeat entries, handoff time, or another hospital-selected process measure; no outcome is promised.

Alternatives: The hospital's current supplier, a focused integration or automation layer, and a broad replacement platform are the relevant choices; named vendors depend on the account. Their gap is **conditional**: a local evaluation may show that a needed cross-department change is still hard, costly, or slow. MediMind's honest position is that the buyer can compare an agreed workflow and transition effort before deciding whether a broader change is justified.

Proof: Customer/user count, result statistic, attributed testimonials, review score, and logo permissions: not verified for public use. Founder credential: Lasha Khoshtaria is MediMind's physician CEO; the team has founder-led hospital implementation experience, with current scope and outcomes to be substantiated separately. GeoHospitals' signed agreement is founder-reported, not an authorized logo, deployment-count, or outcome claim.

Demo: Proposed duration 25 minutes. Host: Lasha Khoshtaria, physician CEO, subject to his calendar. Show: a synthetic patient's shared pathway, the handoff from clinical work to operational/financial follow-up, and a sample baseline-to-evaluation scorecard. Leave with: one candidate pathway, proposed measure, required hospital owners, and next-step questions. Recording: no recording promise; ask before recording any call. Response time: no stated service promise. **Recommended form:** first name, work email, hospital/group name; one optional question, “Which patient pathway creates the most repeated work?” Current live source instead requires first and last name, email, company name and size, industry, job title, facility type, and provider count.

> **Variant selection:** This Library #0 page is the evergreen, detailed demo-page alternative; its three Section 4 button options all state the 25-minute time commitment required by this prompt. For the proposed **"One patient. Too much repeat work."** campaign, the selected P05 `/request-demo` version in Part 2 Prompt 4 takes precedence: headline **"Map your first patient-pathway improvement"** and single campaign CTA **"Map my hospital's first improvement."** Use one coherent headline and CTA set per live page, not the evergreen button and campaign button together.

Section 1 - Headline

1. **See where one patient creates repeat work** [RECOMMENDED]
2. **Find the handoff slowing your hospital down**
3. **Map a clearer path through your hospital**

Section 2 - Subheadline

For Georgian hospital leaders, a 25-minute founder-led walkthrough reveals one candidate pathway to measure and improve with your team.

Section 3 - Above-the-fold trust line

MediMind is led by a physician CEO who has worked on hospital implementation, so the conversation begins with the work your staff actually does.

Section 4 - CTA button text

1. **Map my pathway in 25 minutes** [RECOMMENDED]
2. **Find my first improvement in 25 minutes**
3. **See the 25-minute hospital walkthrough**

Section 5 - What happens on the demo

- **Trace** one synthetic patient journey from registration through a clinical handoff and the records the next team needs.
- **Inspect** where repeat entry, waiting, or disconnected information could be measured in your hospital's version of that journey.
- **Choose** one candidate pathway, the hospital owner, and a fair baseline for a possible parallel evaluation.

Section 6 - Pain section

"Why does one patient create so much extra work?"

“My team is moving information between systems instead of moving the patient forward.”

“I want to know where the delay starts, not hear another promise about a dashboard.”

Section 7 - How it works

1. Choose one pathway. Name a recurring patient journey where clinical, operational, and business work cross.

2. Follow the work. See how MediMind presents the same case and its handoffs to the people responsible for each step.

3. Agree on a measure. If the fit looks promising, define a baseline, scope, and owners for a readiness-dependent parallel evaluation.

Section 8 - Differentiation section

Your current supplier may already support much of your hospital, and a focused tool may fix a single bottleneck. MediMind is worth comparing when the problem follows the patient across departments and separate fixes leave people reconciling the same work. In the demo, we follow one pathway and name what the product can show, what would need adaptation, and what should be measured. A production change happens only after your hospital's review and sign-off.

Section 9 - Testimonial block

See the work before you ask for a reference. We will show a synthetic pathway and discuss the implementation steps with the founder; a public customer result is not available for this block yet.

[PLACEHOLDER: insert only an approved verbatim customer quote that names the specific workflow and observed result → full name, title, hospital; obtain publication permission and a documented measure first.]

Section 10 - Logo bar caption

Follow a patient pathway, then decide what evidence your hospital would need before changing it.
[PLACEHOLDER: display only hospital logos with written public-use permission; until then, omit the logo images and keep this line.]

Section 11 - Objection FAQ

Will this turn into a sales pitch? We use the 25 minutes to trace one pathway and hear where your hospital's work differs. You can end with a measurement question and no commitment to an evaluation.

Will this fit our workflows? We will mark which steps we can demonstrate now and which need configuration, integration, or development. Bring one clinical or operational colleague if their view would help test the fit.

How much work would an evaluation take? The demo needs only a pathway you can describe; an evaluation needs agreed scope, owners, access and readiness checks. A tailored parallel evaluation is a proposed two-week path when those prerequisites are met, with production sign-off separate.

What would it cost? For eligible early partners, the founder's proposed offer is to match current software pricing within agreed terms and include continuing development within an agreed scope. We will discuss implementation, integrations, support, and any advanced AI costs explicitly before a proposal; no total-cost or permanent price-lock promise is implied.

Section 12 - Privacy line

We will not sell your work email or give it to third-party marketers. **[Proposed privacy promise: publish only after operations and policy adopt it.]**

Section 13 - Bottom repeat CTA

See where one patient creates repeat work and leave with a candidate pathway your team can measure. **Map my pathway in 25 minutes**

Sources: source demo component, source hero, evidence register, buyer pain synthesis. Handoff: verify submission and receipt delivery, confirm demo duration/host availability, set the early-partner commercial scope, and obtain permission and measured evidence before replacing either proof placeholder.

19 Library #0 · Prompt 2

Webinar Registration Page

Source: Library #0, pages 8-13 · 1,293 words · 14 requested steps / sections

Status and completed inputs. This is a proposed webinar page. No live session, registration flow, date, time, replay, or guest speaker has been confirmed. Replace the date/time placeholder and verify registration, reminder, and consent handling before publication. The privacy sentence in Section 13 is proposed policy copy requiring adoption; the pain lines are buyer-language hypotheses, not quotations.

Webinar: Working title: “One patient. Too much repeat work.” Outcome: a hospital leader can map a recurring patient pathway, identify one handoff worth measuring, and set a fair first evaluation question. Counterintuitive hook: work often multiplies at the handoff between teams, even when each individual screen seems usable; this is a hypothesis to investigate together, not a measured universal finding. Live date/time: **[PLACEHOLDER: approved day, date, local time and timezone]**. Duration: **45 minutes total**, with a 30-minute practical workshop and 15-minute live Q&A. Format: proposed founder-led online workshop using synthetic data. Recording: undecided; the secondary CTA collects interest only if a replay is approved. Seat cap: none established.

Speaker: Proposed host Lasha Khoshtaria, physician CEO of MediMind. Relevant experience: clinical perspective plus founder-led hospital implementation work; exact public examples require his approval. No borrowed employer, association, or institutional affiliation is asserted. Speaker 2: none planned.

Audience: CEOs, owners, and group operators of Georgian private hospitals and multi-profile hospitals, with clinical, operations, finance, and IT colleagues welcome. Proposed pain: “We already know the work is duplicated, but I cannot see where it starts or which fix is worth the disruption.” Workarounds to discuss: more coordinators, spreadsheets, calls, and one-off software changes. Fear: a generic product webinar disguised as an operations session. Proposed 30-day success: choose one real pathway, record a baseline (repeat entries, handoff delay, or another suitable process measure), and assign a clinical/operations owner and IT counterpart. No improvement is guaranteed.

Proof and takeaways: Registration count, past attendance/satisfaction statistic, attendee testimonial, and permitted company logos: unavailable. Takeaways: a four-column pathway map; one baseline and one decision rule for an evaluation; a list of readiness questions for a parallel comparison. Proposed resource: a one-page pathway worksheet, to be created and reviewed before promising a download. Suggested agenda: 0:00 problem framing; 0:08 trace a synthetic patient case; 0:20 choose a measure and evaluation boundary; 0:30 live Q&A. **Recommended registration form:** name and work email; one optional question, “Which handoff costs your team the most repeat work?” This form is proposed and has no verified live handler.

Section 1 - Page headline

1. **Find the handoff making one patient everyone's extra work** [RECOMMENDED]
2. **Map repeat work hiding in one patient journey**
3. **Find one hospital bottleneck before buying another tool**

Section 2 - Subheadline

For Georgian hospital leaders, this 45-minute workshop on **[PLACEHOLDER: approved day and date]** turns one synthetic patient journey into a pathway map, a baseline question, and a sensible first evaluation.

Section 3 - Date / time / format bar

[PLACEHOLDER: Day, Date] · [PLACEHOLDER: Time + Timezone] · 45 minutes (30-minute workshop + 15-minute Q&A) · Live founder-led online workshop · Free

Section 4 - CTA button text

1. **Save my place in the pathway workshop** [RECOMMENDED]
2. **Send me the patient-pathway session details**
3. **Reserve my seat for the handoff workshop**

Section 5 - Above-the-fold trust line

Led by MediMind physician CEO Lasha Khoshtaria, who brings a clinician's view to hospital implementation questions.

Section 6 - Speaker block

Lasha Khoshtaria - physician CEO, MediMind. Lasha brings a physician's view of the patient journey to the operational questions that follow it through a hospital. As a founder involved in hospital implementation, he can show how to turn a messy handoff into a specific workflow question that clinical, operations, and IT leaders can test together.

Relevant authority: physician and CEO of the company building the hospital operating system discussed in the session; no external affiliation is claimed.

Why this speaker: he can connect the clinical moment to the implementation choices the hospital would actually have to make, and distinguish a useful demonstration from a measured result.

Section 7 - What you will learn

- **Draw** a four-column map of one patient journey: action, responsible team, information needed, and where the handoff stalls.
- **Spot** repeated entry or reconciliation that looks small in each department but grows along the whole path.
- **Choose** a baseline measure, such as entries repeated per case or minutes from one handoff to the next, that your team can observe without claiming a patient outcome.
- **Set** a narrow evaluation boundary: the case type, owners, readiness checks, and evidence needed before considering a production change.

Section 8 - Pain section

“Every department says its part is working. Why is the whole journey still slow?”

“We keep fixing screens and still make people carry the patient story between them.”

“I need one problem we can measure, not another transformation presentation.”

Section 9 - Agenda block

SUGGESTED AGENDA - confirm before publishing.

0:00 - Where the extra work appears - Start with a single patient journey and the ordinary handoffs that can hide repeat effort.

0:08 - Trace the case across teams - Follow a synthetic case through clinical and business steps and mark the information each person needs.

0:20 - Pick a fair first test - Turn one suspected bottleneck into a baseline, evaluation scope, and readiness question.

0:30 - Live Q&A - Bring a real workflow question; we will discuss how to test it without sharing patient information in the session.

Section 10 - Social proof block

The test is whether the method fits your hospital's work. The session uses a synthetic patient pathway so you can judge the questions and measures for yourself.

[PLACEHOLDER: add a permitted, verbatim past-attendee quote mentioning a specific useful takeaway → full name, role, organization. No past-webinar testimonial or attendance statistic is currently verified.]

Section 11 - Objection FAQ

Is this just a sales pitch? The 30-minute workshop produces a pathway map and a measurement question you can use with or without MediMind. We will identify where MediMind might fit, and where your existing tools may already answer the need.

What if I can't make the live time? Use the replay-interest option below if you want an update. A recording is not yet approved or promised.

Is this too basic or too advanced for me? You do not need a technical background; bring an example of a handoff that feels harder than it should. Clinical and IT colleagues can use the same map to ask more detailed questions.

How long is it really? Plan for 45 minutes total: 30 minutes of practical mapping and 15 minutes of live Q&A. You can leave with the mapping method even if you cannot stay for questions.

Section 12 - “Can't make it?” secondary CTA

If the live time does not work, tell us you are interested in a replay. We will email you only if a recording is approved and available; there is no recording guarantee. **Email me if a replay is available**

Section 13 - Privacy line

We will not sell your work email or share it with third-party marketers. **[Proposed privacy promise: publish only after operations and policy adopt it.]**

Section 14 - Bottom repeat CTA

On **[PLACEHOLDER: approved day, date and time + timezone]**, use 45 minutes to turn one patient journey into a pathway map and a measurable first question. **Save my place in the pathway workshop**

Sources: shared writer brief, buyer pain synthesis, source hero. Handoff: approve the event date and host, create the worksheet, implement registration/reminders, decide recording policy, and collect real permissioned proof before replacing the placeholder.

20 Library #0 · Prompt 3

Free Trial Page

Source: Library #0, pages 14-20 · 1,532 words · 15 requested steps / sections

Status and completed inputs. This is proposed copy for a guided synthetic-data sandbox, not a currently verified self-service product or an approved commercial policy. The recommended offer is **14 days, no credit card, no automatic charge**, with an option to stop at any time; the team must implement and approve those terms before publication. This is separate from a readiness-dependent, two-week parallel evaluation inside a hospital and from live clinical deployment. Trial-end access, retention/deletion rules, support capacity, pricing, security controls, and any uptime commitment need operational decisions. The privacy sentence in Section 15 is proposed policy copy requiring adoption. Pain lines are buyer-language hypotheses, not customer quotes.

Product: MediMind, a hospital operating system connecting clinical and business work around a shared patient and operational record. First-session “aha”: use a supplied synthetic patient to follow one pathway across roles and mark where repeated entry or a missing handoff could be measured. Recommended time to this first result: the first guided session, assuming the sandbox is provisioned and the workflow is validated; no minute-level setup promise. Three core activities: trace a synthetic patient case, inspect the handoff between teams, and mark one evaluation question.

Trial structure: Opt-in guided sandbox, 14 calendar days, no card, synthetic cases only; this is a recommendation. Included scope: a reviewed sample pathway, role views needed to follow it, a simple pathway worksheet, and one guided orientation if capacity is approved. Excluded from the sandbox: real patient data, production integrations, hospital-specific configuration, clinical use, and a guaranteed production-ready feature set. At trial end: **[PLACEHOLDER: approved pause, export, retention, and deletion policy]**; no paid conversion without a separate proposal and explicit agreement is the recommended billing rule. Cancel/stop anytime: recommended. Free plan after trial: undecided; do not advertise one.

Customer: Georgian private hospital CEO/owner or an assigned clinical/operations lead and IT counterpart evaluating a cross-department workflow. Company type: private network or multi-profile hospital, with fit assessed by workflow complexity and evaluation readiness rather than a fixed bed count. Proposed pain: “I cannot ask people to change systems until I see what the patient journey looks like.” Workaround: screenshots, slides, vendor demos, spreadsheets, and manual walkthroughs. First fear: hidden billing or effort. Second fear: synthetic success may be mistaken for safe real-world readiness. Proposed day-seven milestone: document one pathway, name its owner, and choose one baseline measure; this is a user task, not a predicted product outcome.

Proof: Verified user/company count, customer result, attributable testimonials, third-party review score, and authorized logos: unavailable. Founder-led implementation experience can be described, but it is not a substitute for independent outcome data. Security certifications and uptime/service-level numbers: none verified for this page. Proposed data handling rule: use only supplied synthetic cases and do not enter patient information; any storage, access, deletion, and incident promises need technical and policy approval.

Sign-up and price: Proposed required fields: work email, name, and hospital/group name; optional question after email: “Which handoff do you want to explore?” SSO/social login: none verified, so do not advertise. Public post-trial starting price: unavailable. Proposed path after sandbox: discuss a scoped hospital

> evaluation and a separate commercial proposal. Founder-defined early-partner intent includes matching current software price and continuing development within agreed terms; it does not establish a fixed public plan, total implementation price, advanced AI costs, or a free plan.

Section 1 - Headline

1. **Trace one patient journey before changing your hospital** [RECOMMENDED]
2. **See where your hospital's repeat work begins**
3. **Map the handoff your team keeps chasing**

Section 2 - Subheadline

Georgian hospital teams trace a synthetic patient pathway in their first session: 14-day trial, no credit card required.

Section 3 - Primary CTA button text

1. **Open my synthetic pathway** [RECOMMENDED]
2. **Explore one patient journey**
3. **Map my first hospital handoff**

Section 4 - Trust strip

No credit card required · Stop anytime without a charge · 14-day guided sandbox

Section 5 - Social proof bar

A clinician-led team built this walkthrough around the work that follows a patient between departments.

[PLACEHOLDER - add logos when available and written public-use permission is recorded. Until then, show the credibility line above without logo images.]

Proof to seek: a documented, permissioned result from a real hospital pathway, with its baseline and evaluation scope. No public outcome statistic is verified today.

Section 6 - The problem section

“The patient moves on, but the information has to be carried after them.”

“We enter the same detail again because the next team cannot see it.”

“I need to know whether this fits our work before I ask anyone to change it.”

Start with one synthetic case and see exactly which question your team should test next.

Section 7 - What you get in the trial

Patient path / Follow one synthetic case across roles, so you can point to the handoff that currently depends on a call, re-entry, or spreadsheet.

Shared context / See what each responsible team would need in the same journey, instead of guessing whether another department has the latest detail.

First measure / Leave with one baseline question your hospital could verify before deciding whether a broader change is worth the effort.

Section 8 - Up and running in 3 steps

1. Request access. Enter your work details and choose the handoff you want to explore; the sandbox uses synthetic cases.

2. Follow the case. Open the guided patient journey and move through the roles and handoffs shown in the sample pathway.

3. Mark the handoff. Identify one repeated step or missing piece of context and write down a measure your hospital could test.

Section 9 - Social proof block

Use the first week to assess a real question: can the sample journey help your team name one workflow to measure? This is a suggested test of usefulness, not a reported customer result.

[PLACEHOLDER: “Here's what hospital leaders say after their first week” followed by an approved, verbatim trial quote naming the observed result → full name, title, hospital, with consent and documented context. No qualifying testimonial is verified.]

Section 10 - Product visual caption

One synthetic patient crosses clinical and operational handoffs, making repeat steps visible before you ask your staff to change a live workflow. **[PLACEHOLDER: use only an approved screenshot or GIF of this actual sandbox state.]**

Section 11 - Trust and security block

Synthetic-data security. Use supplied cases only, and give IT verified hosting, access-control, logging, and data-handling details before approval **[PLACEHOLDER: verified controls; no unearned certifications].**

Reliability terms. Ask for the current service-status process and any agreed availability terms; **[PLACEHOLDER: insert a verified uptime figure or approved SLA only if one exists].**

Trial-end data. The proposed sandbox has no automatic paid conversion; **[PLACEHOLDER: publish the approved access, export, retention, and deletion policy before launch].**

Human help. A MediMind team member can guide the first pathway review if guided-trial support capacity and contact route are approved; **[PLACEHOLDER: add confirmed support channel and availability].**

Section 12 - Objection FAQ

Will I need a card or be charged automatically? The recommended 14-day sandbox asks for no card and does not convert into a paid plan by itself. Publish that answer only after sign-up and billing behavior are implemented and checked.

Will setup consume my team's week? Begin with a supplied synthetic journey and a guided path through it; no integration or patient-data migration is needed for this proposed sandbox. A real hospital evaluation needs separate scoping and time from your clinical/operations and IT owners.

What happens to my data if we do not continue? The sandbox should contain supplied synthetic cases, and no real patient information should be entered. **[PLACEHOLDER: state the approved account access, export, retention, and deletion policy here before publication.]**

Is this right for my hospital and role? It is designed for a private hospital or group leader and their clinical, operations, or IT counterparts who can name a cross-department pathway. If your need is a single isolated task, use the trial to test whether a narrower fix would serve you better.

Section 13 - Pricing context line

After the 14-day sandbox, there is no automatic purchase: ask for a hospital-specific proposal covering subscription, implementation, integrations, support, and any advanced AI scope. For eligible early partners, current software pricing may be a reference point under agreed terms; **[PLACEHOLDER: publish approved eligibility, scope, term, and post-trial policy before launch].**

Section 14 - Bottom repeat CTA

Follow one synthetic patient journey and name the handoff your team should measure next. **Open my synthetic pathway · No credit card required · Stop anytime without a charge · 14-day guided sandbox**

Section 15 - Privacy line

We will not sell your work email or share it with third-party marketers. **[Proposed privacy promise: publish only after operations and policy adopt it.]**

Sources: shared writer brief, evidence register, source demo component. Handoff: validate the guided sandbox and support capacity; approve no-card, no-conversion, access/deletion, security and pricing terms; build a working sign-up route; capture a real approved product visual and permissioned testimonial before replacing proof placeholders.

Source: Original files/Marketing workspace/archive/hero-copy-alternatives-2026-09-21.md. Historical wording preserved; not a new approval or factual revalidation.

MediMind landing-page hero copy

Archive note (22 September 2026): Preserved for reuse, not final positioning. The user considers these directions acceptable but not strong enough yet. The exact later deployed version is saved in `../copy/2026-09-22-live-hero-working-draft.md`. The status below records the original drafting stage.

Date: 21 September 2026

Status: Copy exploration. The user selected the version below as the favorite so far. Preserve its wording unless the user selects a revision. No website implementation or publication is included in this document.

Favorite so far - exact user-approved wording

Your hospital deserves a future.
Your patients deserve better today.

Replace disconnected software with one intelligent system built around your hospital.

- **Better care. More revenue. Less waste.** Give your teams connected information and simpler ways to work.
- **Your own hospital AI brain.** Built around your patients and needs. Start with AI in everyday work. Advance toward AI that anticipates problems, makes clinical and business decisions, and coordinates care-with your team setting the boundaries.
- **A team that keeps building for you.** Unlimited future development. New features ready to test in three business days.

Move first. At the price you already pay.

We match your current software price for early hospital partners. **This offer won’t last indefinitely. Start building your advantage now.**

See it beside your current system in two weeks. Keep running. Commit after you see the difference.

[Build my hospital’s future]

Alternative 1 - A better present and an ambitious future

Build tomorrow’s hospital.
Make today work better.

One intelligent system replaces disconnected software and grows around your hospital.

- **Start with what matters.** Better patient care, fewer wasted hours, and more revenue captured.
- **Build toward an AI-run hospital.** From everyday assistance to AI that anticipates needs and coordinates care and business decisions under your team’s control.
- **Keep improving.** Unlimited future development. New features ready to test in three business days.

Early partners get their current software price matched. That opportunity won't last indefinitely.

Test it beside your current system in two weeks. Decide after you see the difference.

[Start my hospital's next chapter]

Alternative 2 - The value of the existing budget

Your current software budget.
A completely different future.

Put what you already spend toward one intelligent system built for your entire hospital.

- **Get more from every day.** Connected information helps teams improve care, capture missed revenue, and reduce waste.
- **Get a team behind your ideas.** Unlimited future development, with new features ready to test in three business days.
- **Get ready for AI that runs deeper.** Progress toward a hospital AI brain that helps direct care and operations within your rules.

We match current software pricing for early partners. Start while the offer is available.

See your tailored system in two weeks, alongside the one you use today.

[See what my budget could deliver]

Alternative 3 - People and purpose

Give your people more power.
Give your patients better care.

Replace disconnected software with one hospital system that helps your teams do their best work.

- **More attention for patients.** Less searching, repeated typing, and missing information.
- **More value for your hospital.** Help capture missed revenue and reduce the cost of wasted work.
- **More intelligence with every step.** Start with everyday AI. Build toward an AI brain that anticipates risks and coordinates decisions with your team in control.

Unlimited future development included. New features to test in three business days.

Early partners get matched pricing. Test beside your current system in two weeks before committing.

[Give my team a better system]

Alternative 4 - Partnership and customization

A hospital system built around you.
A team that keeps building with you.

Your priorities shape one intelligent system that replaces your disconnected software.

- **Make better care easier.** Connect information and simplify how your teams work.

- **Make improvement continuous.** Unlimited future development to help reduce waste, capture revenue, and solve the next problem.
- **Make AI your advantage.** Build an AI brain around your hospital's needs, advancing toward coordinated clinical and business decisions under your rules.

New features to test in three business days. A parallel system to try in two weeks.

Join early for pricing matched to what you pay today.

[Build around my hospital]

Alternative 5 - Leadership and competitive opportunity

Lead the next chapter of healthcare.
Start with your hospital.

Build your advantage with one intelligent system that replaces disconnected software and improves how your hospital works.

- **Raise the standard today.** Help your teams deliver better care, capture missed revenue, and cut waste.
- **Prepare for what comes next.** Introduce AI throughout daily work, then progress toward an AI-run hospital with people setting the boundaries.
- **Keep moving forward.** Unlimited future development. New features ready to test in three business days.

Early partners get their current software price matched. Take the opportunity to start ahead.

Test beside your current system in two weeks. Commit after your team evaluates it.

[Start building my advantage]

Alternative 6 - Confidence through direct comparison

See your hospital's future.
Before you commit to it.

In two weeks, test a tailored MediMind system beside the software you use today.

- **See what connected care changes.** One system built to improve patient care, recover missed revenue, and reduce waste.
- **See your ideas become software.** Unlimited future development. New features ready to test in three business days.
- **See a path to an AI-run hospital.** Everyday AI grows toward a hospital brain that coordinates clinical and business decisions within your team's boundaries.

Keep your hospital running. Let your team judge the difference.

Current software pricing is matched for early partners. Begin while that offer is available.

[Arrange my side-by-side trial]

Alternative 7 - Information becoming action

Turn everything your hospital knows into better care.

Replace disconnected software with one system that brings your hospital's information together.

- **Help your people see more.** Connected patient information supports better decisions and smoother teamwork.
- **Help your hospital keep more.** Find missed charges, repeated work, and avoidable waste.
- **Build intelligence that acts.** Progress toward an AI brain that sees patterns, anticipates needs, and coordinates care and business decisions under your rules.

Unlimited future development. New features ready to test in three business days.

Early partners get matched software pricing. See it beside your current system in two weeks before committing.

[Put my hospital's information to work]

Alternative 8 - Capacity and growth

Give your hospital room to do more.

More attention for patients. More time for your people. More value from the work your hospital already does.

- **Remove the daily friction.** Replace disconnected software with one system designed around your hospital.
- **Put AI to work.** Help staff reduce routine work, capture missed revenue, and make informed decisions.
- **Grow into an AI-run hospital.** Build toward intelligence that anticipates needs and coordinates care and operations, with your team in control.

Unlimited future development included. New features ready to test in three business days.

Match your current software price as an early partner. Test in parallel in two weeks before deciding.

[See what my hospital could do]

Alternative 9 - Proactive intelligence

Help your hospital see what's coming.
Start by fixing what slows it down.

One intelligent system replaces disconnected software and creates the foundation for your hospital's AI brain.

- **Today: better daily work.** Connected information helps improve care, recover missed revenue, and reduce waste.
- **Next: AI throughout the hospital.** Tools that help your people understand, decide, and act.
- **The future: intelligence that takes initiative.** AI that anticipates risks and coordinates clinical and business decisions within boundaries you set.

Unlimited future development. New features to test in three business days.

Early partners get matched pricing. Try a tailored parallel system in two weeks.

[Build my hospital’s AI future]

Alternative 10 - Raising standards across the hospital

Raise the standard.
Across your entire hospital.

One connected system replaces your separate programs and brings modern medical standards into everyday work.

- **Better support for every decision.** Help teams improve patient care with connected information and simpler work.
- **A stronger business.** Help capture missed revenue, reduce waste, and see where your hospital can improve.
- **A future built around you.** Unlimited development and a path from everyday AI to an AI brain that helps run your hospital under your team’s rules.

New features to test in three business days. A system to compare in two weeks.

Early partners get their current software price matched. Start while the opportunity is available.

[Raise my hospital’s standard]

Source: Original files/Marketing workspace/archive/uvp-jtbd-strategy-2026-09-22.md. Historical wording preserved; not a new approval or factual revalidation.

MediMind - upgraded value propositions and buyer jobs

Archive note (22 September 2026): This is an earlier strategic hypothesis, not completed customer research or final positioning. Preserved without rewriting its proposals. See `../research/2026-09-22-hospital-buyer-pain-points.md` for the subsequent problem-first analysis and `../copy/2026-09-22-live-hero-working-draft.md` for the exact deployed copy. The status below records the original drafting stage.

Date: 22 September 2026

Status: Proposed strategy and copy for review. The favorite in `hero-copy-alternatives-2026-09-21.md` remains unchanged and selected as the favorite so far. No website content has been changed.

Inputs and scope

Applied the six-step UVP/JTBD framework supplied in the user's two screenshots. UVP means unique value proposition: the valuable change promised to the buyer. JTBD means jobs to be done: the progress the buyer hires the product to achieve.

Used the conversation, current landing-page source, investor deck, Brain positioning and buyer notes, August deployment and fundraising notes, and September diligence status. Formal outputs from worksheet Prompts 1-3 and the BFG master table were not supplied; no claim is made that those exercises were completed. Existing research supplies a working baseline for this exercise. Customer motivations, triggers, market sophistication, and scores below are strategic hypotheses to test, not customer-research findings.

The screenshot's memory command is not evidence that unavailable previous answers are stored. Its request to stop for missing worksheets does not override the user's instruction to proceed using the information already collected.

The commercial offer comes from the founder's instructions in this conversation: match current software pricing for early hospital partners; include unlimited future development; requested features ready to test in three business days; a tailored parallel trial in two weeks; commitment follows evaluation. These are proposed delivery commitments, not independently measured service performance. A testable feature is not a clinically validated production release. A parallel trial is not a full hospital cutover. No expiration date, number of available places, or lifetime price lock has been established.

Step 1 - Customer types and complete JTBD map

The primary audience remains hospital decision-makers. The following are three useful buying situations, not mutually exclusive market segments. CFOs and IT leaders participate in the decision; they are not invented as separate customer markets.

A. Independent hospital CEO or owner

Leads an established hospital and carries responsibility for care quality, financial performance, staffing, and major spending decisions. Needs a credible improvement path that can be defended to staff and owners.

Most important outcome: improve the hospital's clinical and business performance while keeping the transition manageable.

Job	Desired progress	Trigger moment	Previous solution or workaround
Functional	Reduce wasted work and missed revenue, connect patient information, and modernize the hospital within a budget leadership can defend.	A budget review exposes weak margins; software renewal or an expensive change request forces a decision.	Separate programs, spreadsheets, manual reconciliation, and paying vendors for individual fixes.
Social	Be recognized as the leader who made a thoughtful improvement and protected the hospital.	The board asks for an improvement plan; staff challenge the current tools.	Preserve familiar systems, defer the decision, or choose a familiar vendor for reassurance.
Emotional	Feel confident that change will help and that there is a practical way to evaluate it.	A proposed replacement brings fears about disrupted care, staff resistance, or hidden costs.	Postpone replacement; require repeated demonstrations and references without an operational trial.

B. Hospital-chain CEO or group operator

Leads several hospitals and needs reliable visibility across them. Wants consistent ways of working without losing necessary local differences or making every improvement a separate project.

Most important outcome: run the network as a coordinated whole and spread successful improvements across its hospitals.

Job	Desired progress	Trigger moment	Previous solution or workaround
Functional	Get comparable information, reduce duplicated work, and introduce improvements across multiple hospitals.	A new acquisition, branch expansion, or large performance difference exposes disconnected systems.	Local vendors, separate reporting teams, spreadsheets, and repeated implementation projects.
Social	Demonstrate that the group can grow while maintaining oversight and consistent care standards.	Owners ask why similar hospitals deliver different results or why expansion is difficult.	Demand more reports, add reporting layers, and appoint more coordinators.
Emotional	Feel in control of the whole network and confident that problems are visible.	A late report or cross-site incident shows that leadership learned about a problem too slowly.	Escalation calls, frequent meetings, and manual checking with each hospital.

C. Clinician-owner or medically led hospital

An owner or leader closely involved in clinical care sees how software affects staff attention and patient decisions. Wants the hospital to adopt AI in a way that supports good medicine and earns clinicians' trust.

Most important outcome: equip clinicians to deliver better care while making the hospital a credible early adopter of useful AI.

Job	Desired progress	Trigger moment	Previous solution or workaround
Functional	Give teams a complete patient context, reduce paperwork, and turn clinical improvement ideas into tools they can use.	A handoff problem, missed information, excessive paperwork, or unanswered software request exposes a gap.	Staff compensate with phone calls, repeated checking, extra documentation, and separate AI tools.
Social	Be seen as a leader in modern care whom clinicians trust.	Colleagues or patients ask about AI; a rival announces a new capability.	Trial isolated tools or announce an innovation initiative without changing the shared foundation.
Emotional	Feel proud of the care environment and confident that technology supports clinical judgment.	Frustration that capable staff are held back by tools, or concern about AI acting without sufficient context.	Personally supervise exceptions, improvise workflows, or delay broader adoption.

Cross-cutting emotional job: **Help me move the hospital forward without putting today's care, budget, or my reputation at risk.**

Step 2 - Market sophistication

Working assessment: **4/5 - familiar promises require a concrete reason to believe.** This is an editorial assessment, not a measured market statistic. Familiarity will vary by hospital and region.

Current primary-source messaging supports the competitive premise:

- InterSystems IntelliCare markets a unified system with integral AI and shared patient information.
- Oracle Health EHR markets an all-in-one system with AI embedded in workflows.

Therefore, whole-hospital software and built-in AI are useful category descriptions but do not establish uniqueness by themselves. MediMind's promising message combines the desired transformation with a concrete offer: a tailored parallel evaluation, matched current pricing for early partners, included continuing development, and a phased path toward proactive AI. This combination has not been proven globally unique.

Messaging implications:

1. Lead with the valuable change for the hospital and its people.
2. Explain the mechanism simply: replace disconnected programs with one shared system.
3. Make evaluation feel controllable: compare on the hospital's own workflows before switching.
4. Present matched pricing and included development as early-partner commercial terms.
5. Describe the proactive hospital brain as the destination being built toward, distinct from currently deployed workflows.
6. Use the opportunity to start early as urgency. Do not invent deadlines, customer disappearance, malpractice timelines, or guaranteed financial/clinical results.

The Medplum case study is a useful public implementation reference. It does not establish guaranteed clinical outcomes, financial gains, or a universal two-week delivery guarantee. Current live footprint must be checked separately before publishing a deployment number.

Step 3 - Ten UVPs as direct brand promises

1. Better care and an AI future within the current software budget

Give your hospital better care today and an AI-run future-at the software price you already pay.

Replace disconnected software with one system built to improve care, capture missed revenue, and reduce waste. Early partners get matched pricing and a clear path toward proactive AI.

Primary customer fit: A; also B and C.

2. Proof before the commitment

See a better way to run your hospital before you commit to changing it.

Test a tailored system beside yours in two weeks. Keep current operations running while your own team compares the work and decides whether to switch.

Primary customer fit: A; also B and C.

3. Continuous improvement without repeated development bills

Keep improving your hospital without paying a new development bill for every idea.

Unlimited future development is included. Your priorities shape the system as your hospital grows and its needs change.

Primary customer fit: C; also A and B.

4. More value from the care already delivered

Get paid for more of the care you already deliver-and lose less to avoidable waste.

Bring care and billing information together so your team can find missed charges, repeated work, and unnecessary spending.

Primary customer fit: A; also B. The desired financial improvement requires customer-specific measurement; no amount is established.

5. Time and attention returned to care

Give your people more time for patients and less work fighting software.

Connected information and AI assistance help reduce searching, repeated typing, and administrative work that competes with care.

Primary customer fit: C; also A and B.

6. A stronger basis for care decisions

Give every care decision a more complete patient picture.

Bring patient information together across the hospital, so teams and AI tools can work with relevant context instead of repeatedly rebuilding it.

Primary customer fit: C; also B and A.

7. A coordinated hospital network

Run your hospitals as one coordinated network while preserving what each hospital needs.

Give leadership a shared view and teams connected ways of working, with room to adapt the system to each hospital.

Primary customer fit: B. Secondary: A when expanding to multiple sites.

8. Improvement while the need is still urgent

Get working improvements into your team's hands in days, while the need is still urgent.

Requested features are ready to test in three business days. Your staff can evaluate the change and help shape what comes next.

Primary customer fit: C; also A and B. Testable work is distinct from validated clinical production deployment.

9. A hospital that anticipates and responds

Move toward a hospital that sees problems coming and acts early, within the rules your teams set.

Start with connected information and everyday AI. Build toward a hospital AI brain that anticipates risks and coordinates clinical and business decisions.

Primary customer fit: C; also B and A. This is a future direction, not a claim of a currently deployed hospital-wide brain or proven earlier detection.

10. A head start on attractive commercial terms

Start building your hospital's AI advantage while early-partner pricing is still available.

Begin improving today's work and preparing for tomorrow's care, with current software pricing matched and future development included.

Primary customer fit: B; also A and C. No fixed deadline or guaranteed advantage over competitors has been established.

Step 4 - Scores

Scores are editorial judgments, not interview results or conversion forecasts. Each criterion uses a 1-5 scale. Clarity measures immediate understanding; credibility measures support in the reviewed evidence and realism of the commitment; desire measures fit with the emotional job. Credibility 3 means a plausible, relevant offer that needs demonstrated delivery; 2 means the important outcome or future capability is not yet established. No unsupported outcome receives a credibility score of 5.

#	Proposition	Clarity	Credibility	Desire	Total / 15
1	Better care + AI future + current price	5	4	5	14
2	See it before committing	5	4	4	13
3	Improvement without repeated development bills	5	3	5	13
4	More revenue captured, less waste	5	2	5	12
5	More time for patients	5	3	4	12
6	More complete patient context	5	3	4	12
7	Coordinated hospital network	4	3	4	11
8	Working improvements in days	5	3	4	12
9	Proactive hospital	4	2	5	11
10	Early AI advantage and partner pricing	5	3	4	12

Why #1 leads: it joins ambition, present-day value, and the budget objection in one understandable promise. Its credibility is supported by the unified-system direction and founder-defined early offer, but measured outcomes and the full AI destination remain unproven.

Why #2 is strong: parallel evaluation answers fear of change with a tangible process. Internal deployment notes support the approach, while the two-week timeframe remains the founder's proposed offer.

Why #3 is strong: it sells an ongoing relationship in terms a buyer understands. The unlimited scope is compelling but should not be mistaken for proven unlimited delivery capacity.

Step 5 - Top-three customer matches

Top UVP	Best customer	Why it resonates	Cross-customer use
#1	A: independent hospital CEO/owner	Lets the leader pursue a meaningful upgrade within a familiar software budget.	Broadest homepage message; relevant to B and C as well.
#2	A: independent hospital CEO/owner; B during a cautious rollout	Gives leadership and staff a way to judge the system before making the switch.	Strong supporting message and call to action across all three.
#3	C: clinician-owner or medically led hospital	Makes frontline ideas actionable without a fresh custom-development negotiation every time.	Also valuable to B for recurring network needs and A for financial predictability.

Use #1 as the main promise, #3 as the distinctive ongoing value, and #2 as the reason to take the next step. These work together; they need not compete for headline space.

Step 6 - Single-sentence hero statement

We turn your current software budget into one connected hospital system built for better care, stronger finances, and an AI-run future.

This combines the highest-scoring UVP with the emotional job of moving forward within controllable financial and operational commitments. The matched-price claim applies to the early-partner offer.

Proposed hero using the favorite's voice

Your hospital deserves a future.
Your patients deserve better today.

Turn your current software budget into one connected hospital system built for better care, more revenue, and less waste.

- **Improve today.** Replace disconnected programs with simpler work and a clearer patient picture.
- **Build your AI future.** Start with AI in daily work. Advance toward your own hospital AI brain that anticipates risks and coordinates clinical and business decisions within your team's rules.
- **Keep getting better.** Unlimited future development included. New features ready to test in three business days.

Early partners get their current software price matched. This offer won't be available indefinitely.

Test beside your current system in two weeks. Keep running. Commit after you see the difference.

[See my hospital's future]

This proposed revision is not automatically the new favorite. The previously approved favorite remains preserved verbatim.

Evidence and validation priorities

Internal sources reviewed in this conversation:

- `src/components/Landing/LampHeroSection.tsx` - existing first-screen message.
- `public/deck.html` - category, product scope, vision, and older traction language.
- `/Users/toko/Desktop/Brain/01 – Projects/MediMind/Positioning/06 – Buyers.md` - buyer pains and role distinctions.
- `/Users/toko/Desktop/Brain/01 – Projects/MediMind/Investor Conversations/2026-08-19 – 500 Global B11 – Document Pack Drafts.md` - buyer committee, missed reimbursement, parallel deployment.
- `/Users/toko/Desktop/Brain/01 – Projects/MediMind/2026-08-16 – Investor Pitch Q&A.md` - phased AI journey, pricing context, parallel-readiness qualification.

- `/Users/toko/Desktop/Brain/01 - Projects/MediMind/500 Global Due Diligence/00 - Master Diligence Status and Decision Log.md` - newer commercial and rollout status.

External primary sources consulted on 22 September 2026:

- https://www.intersystems.com/products/intellicare/
- https://www.oracle.com/health/clinical-suite/electronic-health-record/
- https://www.medplum.com/blog/medimind-case-study

For customer testing, show #1, #2, and #3 separately with the same next step. Ask the buyer to explain what they think they get, which promise they value most, and what proof they need. Track qualified trial interest rather than selecting the loudest headline. The most valuable new evidence would be measured time saved, revenue recovered, waste reduced, and real delivery times at a named reference hospital.

Source: Original files/Marketing workspace/copy/2026-09-23-pain-led-landing-page-draft.md. Historical wording preserved; not a new approval or factual revalidation.

Pain-led landing-page recommendation

Date: 23 September 2026 Status: Proposed copy for review. Not approved, published, or conversion-tested. Existing live copy and archived favorites remain unchanged.

Why this direction

Lead with the hospital-wide result, name tangible problems underneath, and use a scoped evaluation as the practical next step. The complete library's patient-pathway hook is a useful campaign/discovery entry point; it is too narrow to carry the entire hospital OS vision alone. This recommendation is an editorial synthesis, not a claim that it outperformed the earlier options.

Working pain priorities: lost/delayed payment and opaque costs; repeat staff work; late or inconsistent leadership information; fragmented patient context; slow vendor changes; reporting reconstruction. Switching risk and supplier trust are separate purchase objections. These priorities are hypotheses, not validated Georgian buyer rankings. AI readiness is a strategic opportunity, not established as the primary purchasing motive.

1. Hero

Eyebrow: One intelligent hospital operating system

A better-running hospital today.

Your AI future starts here.

Replace disconnected hospital software with one system built around your teams-designed for better-connected care, less wasted work, and clearer control of costs and revenue.

- Patient information and daily work, connected.
- Useful AI, with your people in control.
- A development team that keeps improving your system.

Primary CTA: Request a hospital demo Secondary CTA: See how switching works

See the system. Discuss your needs. Decide whether a side-by-side evaluation is right for your hospital.

2. Dedicated pain section

Your people should care for patients. Not hold disconnected software together.

When information stops at each department, the work does not. Your teams fill the gaps-with repeated entries, phone calls, spreadsheets, and billing corrections.

- Staff time goes into repeat work.
- Payment gaps and avoidable costs become harder to see.
- Care teams have to chase the information they need.

Adding another tool does not always fix what happens between them.

3. The replacement

One hospital. One connected way to work.

MediMind brings clinical and business work into one tailored hospital system. Keep patient information connected, make daily work easier to follow, and give leadership a clearer view of how the hospital is performing.

4. Continuing development

Your hospital keeps evolving. Your software should too.

Work directly with the team building your system. Shape improvements around your hospital's needs, test changes with your staff, and keep making the system better.

5. The AI destination

The future is a hospital that can anticipate. Not just react.

Start with connected information and AI that assists daily work. Our roadmap builds toward a hospital-specific AI system that helps anticipate risks and coordinate clinical and business workflows-with your clinical team retaining control of medical decisions.

6. Evaluation

See the difference. Then decide to switch.

Start with an agreed workflow. Compare a tailored MediMind setup beside your current system. Review the results, staff effort, and transition plan with your team before deciding on a wider rollout.

7. Early-partner offer - conditional draft

A new standard. At your current software price.

For eligible early hospital partners, we match current software pricing and include unlimited development requests under agreed delivery terms.

Explore the early-partner offer and shape a system around the hospital you want to run.

8. Final invitation

Build your hospital’s future. Start with what needs to work better today.

Tell us where your current software gets in the way. We’ll show you the relevant workflow and discuss what an evaluation would involve.

CTA: Request a hospital demo

Editorial notes - not public copy

- The offer is conditional. “Unlimited development requests” proposes an operating definition for review; it does not silently replace the founder's requested unlimited development hours. Set delivery obligations, priorities, exclusions, pricing term, and advanced AI treatment before publication.
- Price matching refers to agreed software scope, not necessarily unchanged total transition costs or a lifetime price lock. No artificial deadline or limited number of places is created.
- Three-business-day testable changes and the two-week parallel-evaluation offer are omitted from the core copy until readiness, request scope, and timing conditions are agreed. A testable change is not automatically an accepted production release.
- Full hospital replacement remains the ambition. External devices, portals, and services may require integration and a hospital-specific scope assessment.
- Proactive AI is labeled as roadmap. No autonomous clinical operation, inevitable malpractice deadline, or hospital-disappearance prediction is asserted.
- Outcomes are design aims, not guaranteed immediate financial or clinical results. Real proof should distinguish staff time, actual legitimate collections, net cost, and patient outcomes.
- Add verified, permissioned product/customer proof after the solution and before the evaluation. Do not fill that space with an unverified network go-live number or invented testimonial.
- CTA labels are copy proposals, not functioning controls in the report. Before launch verify real demo receipt, lead ownership, confirmation, and error behavior. The complete-library page analysis identified a source-level concern, not a confirmed deployed failure; this task does not repair or deploy the form.
- This document recommends a page narrative, not new standalone routes or additional campaign scope.

Inputs

- Buyer pain research
- Claim register
- Complete library: problems
- Complete library: value propositions
- Complete library: buyer avatar
- Complete library: funnel pages
- Business/market analysis
- Live hero snapshot

Visual edition: HTML report, dedicated pain and copy sections.

Source: Original files/Marketing workspace/copy/2026-09-24-ten-website-hero-previews.md. Historical wording preserved; not a new approval or factual revalidation.

Ten homepage hero previews - 24 September 2026

Working copy for review, not deployed. The website-style previews are appended to the existing consolidated HTML:

Open the original marketing book → **Ten website-style copy options**.

Option 5 uses the founder's preferred opening, "You know your hospital could work better. Let us prove the alternative." All variants preserve the free parallel evaluation, no purchase commitment, price matching for the next ten hospital clients, AI in daily workflows and the foundation for future hospital-wide AI. No database-access wording appears in these hero drafts.

Shared commercial and publication notes

The ten-client offer is founder-specified, not a claim about remaining places already sold. Confirm scope and commercial terms before publication. Proactive hospital-wide AI is a direction, not an assertion of autonomous clinical decision-making available now. No measured clinical or financial uplift is claimed. The previews' CTA buttons do not submit requests.

01. Disconnected systems

FOR OUR NEXT 10 HOSPITAL CLIENTS

One hospital. Too many systems. Too much wasted time.

Your people shouldn't have to hold disconnected software together. Replace it with one system built around your hospital.

- **One connected hospital.** Patient information, daily work and finances together.
- **AI in everyday workflows.** AI assistance where your teams do their work.
- **The foundation for your AI future.** Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build your parallel system free. Test the quality and improvements yourself before making any purchase commitment.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

02. Your people’s time

FOR OUR NEXT 10 HOSPITAL CLIENTS

Your best people deserve better than fighting software.

Less retyping. Less chasing answers. Give your teams a hospital system built around the work they actually do.

- **Give your people time back.** Connect information and simplify repeated work.
- **AI in everyday workflows.** AI assistance where your teams do their work.
- **The foundation for your AI future.** Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build a free parallel system for your team to test. Let them judge the quality and the difference. No obligation to buy.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

03. Leadership clarity

FOR OUR NEXT 10 HOSPITAL CLIENTS

You shouldn’t have to chase answers about your own hospital.

Bring scattered information together in one tailored hospital system. See where work gets stuck and where money gets missed.

- **A clearer view of your hospital.** Care, operations and financial information connected.
- **AI in everyday workflows.** AI assistance where your teams do their work.
- **The foundation for your AI future.** Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

Start with a free parallel system built around your hospital. See the quality, measure the improvement and decide whether it earns the switch.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

04. Development partner

FOR OUR NEXT 10 HOSPITAL CLIENTS

Still waiting for the software changes your hospital needs?

Get one hospital system built around your workflows-and the team that keeps developing it around your priorities.

- **Keep improving.** Ongoing development included within agreed scope.
- **AI in everyday workflows.** AI assistance where your teams do their work.
- **The foundation for your AI future.** Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build your parallel version free, before asking you to buy. Your team tests it. You judge the quality and improvement. You decide.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

05. Prove the alternative

FOR OUR NEXT 10 HOSPITAL CLIENTS

You know your hospital could work better. Let us prove the alternative.

Replace disconnected software with one intelligent system built around your hospital. Give your teams a better way to work.

- **Built around your hospital.** Connected workflows, patient information and finances.
- **AI in everyday workflows.** AI assistance where your teams do their work.
- **The foundation for your AI future.** Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build and launch your parallel system free. Let your team test the quality and see the improvement for themselves. No obligation to buy.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

06. Confidence to switch

FOR OUR NEXT 10 HOSPITAL CLIENTS

Your software needs to change. Your hospital can’t afford a blind leap.

Move to one connected hospital system-without buying the replacement before you’ve tested it.

- **Your current system stays in place.** Evaluate the alternative alongside what you use today.
- **AI in everyday workflows.** AI assistance where your teams do their work.
- **The foundation for your AI future.** Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build your tailored parallel system free. Your team tests the quality and improvements before you make any purchase commitment.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

07. Future readiness

FOR OUR NEXT 10 HOSPITAL CLIENTS

Your hospital has a future. Your software should help you reach it.

Solve today’s disconnected workflows with one tailored hospital system. Start building the connected foundation your next generation of AI will need.

- **Solve today’s problems.** Connected information and simpler daily work.
- **AI in everyday workflows.** AI assistance where your teams do their work.
- **The foundation for your AI future.** Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build your parallel version free. Test the quality and improvements it delivers today before committing to the next step.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

08. Cost of friction

FOR OUR NEXT 10 HOSPITAL CLIENTS

You’re paying for software. You shouldn’t keep paying for its problems.

Repeated work. Scattered information. Hours spent chasing answers. Replace disconnected systems with one tailored hospital platform.

- **Find the friction.** Make repeated work and financial gaps easier to see.
- **AI in everyday workflows.** AI assistance where your teams do their work.
- **The foundation for your AI future.** Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build your parallel alternative free. Compare the quality and improvements with your own team. No obligation to buy.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

09. The right AI foundation

FOR OUR NEXT 10 HOSPITAL CLIENTS

Your AI future needs more than another disconnected tool.

It needs connected patient information, connected workflows and software built around your hospital. MediMind creates that foundation.

- **Connect the hospital first.** One shared system for clinical and business work.
- **AI in everyday workflows.** AI assistance where your teams do their work.
- **The foundation for your AI future.** Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build your parallel system free. Test it alongside your current software and judge the quality and improvement before making a purchase decision.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

10. Today and tomorrow

FOR OUR NEXT 10 HOSPITAL CLIENTS

A better hospital today. The foundation for an AI-powered hospital tomorrow.

Replace disconnected software with one tailored system. Put AI tools to work now and build toward more capable hospital-wide AI.

- **One tailored hospital system.** Built around your teams and the way they work.
- **AI in everyday workflows.** AI assistance where your teams do their work.
- **The foundation for your AI future.** Connected data to build toward proactive hospital-wide AI.

Your current software price. Matched when you switch.

We build and launch your parallel version free. Let your people use it, challenge it and judge the improvement. Commit only if it earns your confidence.

Get my free parallel system →

Your current system stays in place during evaluation. No obligation to buy.

Source: Original files/Complete B11 HTML library/Coverage and handoff.md. Historical wording preserved; not a new approval or factual revalidation.

Coverage and handoff

Assembled 23 September 2026 from both supplied B11 libraries. Writing: three GPT-6 Sol agents; coordination, cross-chapter reconciliation and HTML QA: parent agent.

Written outputs: 20/20. Requested top-level sections: 138/138. Approximate word count: 67,728. Automated content checks: passed.

Output	Requested sections	Written answer
Part 1 · Prompt 1: Describe Your Business & Market	7	Open
Part 1 · Prompt 2: Map Your Product Mechanics & JTBD	6	Open
Part 1 · Prompt 3: Build Your Features, Benefits & Guarantees	6	Open
Part 1 · Prompt 4: Generate Your UVPs & Map Your JTBD	6	Open
Part 1 · Prompt 5: Map Competition & Find Your Defensible Position	6	Open
Part 1 · Prompt 6: Mine Competitor Reviews & Customer Intelligence	6	Open
Part 1 · Prompt 7: Design & Execute Your User Research	8	Open
Part 1 · Prompt 8: Define the Real Problems You Are Solving	6	Open
Part 1 · Prompt 9: Build Your Customer Avatar	6	Open
Part 2 · Prompt 1: Map Your Marketing Funnel Architecture	6	Open
Part 2 · Prompt 2: Define Your Marketing Angles	6	Open
Part 2 · Prompt 3A: Build Your Campaign Concept & Video Script	3	Open
Part 2 · Prompt 3B: Build Your Campaign Distribution	5	Open

Output	Requested sections	Written answer
Part 2 · Prompt 4: Build Your Funnel Pages	4	Open
Part 2 · Prompt 5: Content Distribution & Traffic Strategy	5	Open
Part 2 · Prompt 6: Tracking, Mapping & Implementation	6	Open
Part 2 · Prompt 7: Reporting	4	Open
Library #0 · Prompt 1: Book a Demo Page	13	Open
Library #0 · Prompt 2: Webinar Registration Page	14	Open
Library #0 · Prompt 3: Free Trial Page	15	Open

Scope and limits

All 85 content IDs appear in the funnel, page map, distribution cards and tracking library. The book includes 36 channel assessments, 11 review records, 15 interview questions, 9 outreach templates, 4 campaign emails, 10 campaign posts, video scripts, 11 page specifications and detailed conversion-page exercises.

The source PDFs describe 16 sequential prompts; treating 3A and 3B separately yields 17 main-library outputs, plus three from Library #0. Required sections are written; bracketed testimonial, event, policy and baseline fields deliberately remain where real evidence or a business decision is absent. No interviews, customer endorsement, campaign launch, analytics installation, form repair or website deployment occurred.

The central open launch gate is verified demo-request receipt. The source code uses a simulated submission; deployed behavior and any lost lead count are unverified. The five-hour calendar is shared, and 85 content ideas are a backlog, not a weekly production requirement.

Start with START HERE.html and its expandable first-week guide. Individual outputs are independent HTML files; keep the complete folder together when sharing so source and cross-chapter links remain available. Internal source snapshots preserve some original workspace references; public source URLs remain online references.

Source: Original files/Complete B11 HTML library/Editable text/First-week launch guide.md. Historical wording preserved; not a new approval or factual revalidation.

Lasha's first-week launch guide

Purpose. Start one useful conversation with a Georgian private-hospital CEO or group operator about a repeated patient handoff, then offer a founder-led pathway demo. The campaign line is **“Why does one patient create so much extra work?”** The next action is **“Map my hospital's first improvement.”** The 90-day number-12 held qualified demos, three readiness-approved evaluations, and one new paying buying organization-is a planning scenario, not a forecast or a claim of existing traction. Count one buying organization once; keep the existing anchor separate.

What is ready on paper

The funnel blueprint defines the stages and the full 85-idea catalog. Those ideas are a backlog, **not 85 pieces to make**. The angle plan chooses a low-risk map, one use case, and a controlled comparison. The campaign script contains an 82-second founder phone video, a 15-second cut, and two short tests using synthetic information. The first public destination is P05, the existing `/request-demo` route. Its proposed page copy says **“Map your first patient-pathway improvement”** and promises a 25-minute founder call; it is not live site copy.

The form is the gate. Source inspection found that its current submit handler waits, logs to the browser, and shows success without sending the request to a backend. That does not prove the deployed site behaves the same way, but **do not send campaign traffic to P05 until a live test confirms receipt, a named owner, and a truthful failure path**. The site/engineering owner handles implementation outside the five founder marketing hours. The existing page also says 30 minutes; align it with the proposed 25-minute demo before publication.

T10: the worksheet to use this week

Copy these six prompts onto one page. The buyer can answer without disclosing patient identity or clinical details:

1. **Pathway:** Fill four boxes-request, result, next decision, and financial close-using a fictional example in public copy.
2. **Last incident:** What happened in a recent real handoff, described without names or patient details?
3. **People and systems:** Which teams touched it, and which tools or manual steps did each need?
4. **Repeat work:** What was re-entered, searched for, called about, or reconciled? Count steps and note who did them.
5. **First baseline:** What could be measured now-task time, repeat entries, wait time, exceptions, or appropriate actual collections? Record how the number is obtained; “unknown” is acceptable.
6. **Decision:** Who owns the work, who must review it clinically and technically, and what result would make a limited parallel evaluation worthwhile?

T10 is useful even if the buyer decides the incumbent can fix the problem. Do not convert a filled worksheet into a public case study or a performance claim without the hospital's review and permission.

The first five hours, total

Use the traffic chapter's **300-minute weekly clock**, also used in the angle plan. Reserve **85 minutes** for five five-minute weekday checks (25 total), a 30-minute Monday scorecard review, and a 30-minute weekly share of the monthly deep dive. The remaining **215 minutes** are the entire creative and outreach allowance-not another five hours for each channel:

Minutes	First-week action	Finished when
75	Put T10 into a clean one-page document, with the six prompts above and a synthetic example.	A CEO can name one handoff and first measure without sending patient data.
40	Adapt the first paper-card visual from the 3A script into T01, one founder LinkedIn post. End with the pathway-map invitation.	The post asks a real question and points only to a working route or offers the worksheet by reply.
60	Choose five genuinely relevant hospital contacts or permissioned introduction routes. Send each a brief, individual note about one pathway question, with T10 only if useful.	Five named approaches are recorded; no invented relationship or bulk campaign.
40	Answer replies, ask for the last concrete incident, and record the next owner/action.	A reply becomes a pathway discussion or a clear “not now,” not a vanity impression.

The 3A video is **ready to film, not yet filmed**. Use the week-two creative slot in the angle plan for the phone shoot and captions, once the destination works. Do not add filming on top of week one's 300 minutes. A founder demo, clinical/IT workshop, technical route repair, and customer implementation each need separately planned time.

Measure the real next step

The tracking plan distinguishes a click, an accepted request, a booked meeting, and a **held qualified demo**. The last is the north star. A qualified demo has a distinct hospital buying organization, relevant authority or sponsor, a recent cross-department problem, and an owner or path to one. Put source/content ID (T10, T01, or the later video), organization, owner, next date, and held-demo date in a controlled CRM or sheet. Keep names and clinical details out of public analytics URLs. If there is no eligible denominator, write **N/A** rather than a percentage.

At Monday's review, decide one thing: what most limits the next held qualified demo-lead receipt, access to the right buyer, message fit, or evaluation readiness? Fix that first. The page, worksheet, script, and reporting templates are drafted. No buyer interviews, campaign launch, external sends, new page deployment, accepted lead flow, customer outcome, or film production was performed by this guide.

Source: Original files/Complete B11 HTML library/Editable text/l0-01-demo.md. Historical wording preserved; not a new approval or factual revalidation.

Book a Demo Page

Status and completed inputs. This is proposed page copy for a founder-led 25-minute pathway demo, not a description of the current `/request-demo` experience. The current source calls it 30 minutes, requires nine fields over three steps, and its submit handler logs data locally before displaying a receipt message; a real delivery and scheduling path needs verification before this page launches. The privacy sentence in Section 12 is proposed policy copy requiring adoption. Pain lines are buyer-language hypotheses, not customer quotes.

Product: MediMind. One-line description: a hospital operating system that brings clinical and business work into a shared patient and operational record. Core mechanic: follow a patient pathway across roles → complete and review work in connected workflows → inspect where information, time, or legitimate charges need attention. Differentiator: founder-led local adaptation plus an agreed, measured parallel evaluation of the hospital's own priority workflow. The FHIR R4 foundation is a technical design choice, not a promised result.

Buyer: CEO, owner, or group operator of a private hospital network or multi-profile hospital in Georgia, with clinical, operations, finance, and IT counterparts. Proposed pain 1: “Why does one patient make so many people enter the same thing?” Pain 2: “I get three versions of the same number.” Pain 3: “Every useful change becomes another request to another vendor.” Likely workarounds to test: spreadsheets, calls, manual reconciliation, current supplier changes, and point tools. Booking fear: an unqualified sales pitch or a risky rip-and-replace proposal. Proposed 90-day success: one agreed pathway measured against its baseline for repeat entries, handoff time, or another hospital-selected process measure; no outcome is promised.

Alternatives: The hospital's current supplier, a focused integration or automation layer, and a broad replacement platform are the relevant choices; named vendors depend on the account. Their gap is **conditional**: a local evaluation may show that a needed cross-department change is still hard, costly, or slow. MediMind's honest position is that the buyer can compare an agreed workflow and transition effort before deciding whether a broader change is justified.

Proof: Customer/user count, result statistic, attributed testimonials, review score, and logo permissions: not verified for public use. Founder credential: Lasha Khoshtaria is MediMind's physician CEO; the team has founder-led hospital implementation experience, with current scope and outcomes to be substantiated separately. GeoHospitals' signed agreement is founder-reported, not an authorized logo, deployment-count, or outcome claim.

Demo: Proposed duration 25 minutes. Host: Lasha Khoshtaria, physician CEO, subject to his calendar. Show: a synthetic patient's shared pathway, the handoff from clinical work to operational/financial follow-up, and a sample baseline-to-evaluation scorecard. Leave with: one candidate pathway, proposed measure, required hospital owners, and next-step questions. Recording: no recording promise; ask before recording any call. Response time: no stated service promise. **Recommended form:** first name, work email, hospital/group name; one optional question, “Which patient pathway creates the most repeated work?” Current live source instead requires first and last name, email, company name and size, industry, job title, facility type, and provider count.

Variant selection: This Library #0 page is the evergreen, detailed demo-page alternative; its three Section 4 button options all state the 25-minute time commitment required by this prompt. For the proposed **“One patient. Too much repeat work.”** campaign, the selected P05 `/request-demo` version in Part 2 Prompt 4

> takes precedence: headline **“Map your first patient-pathway improvement”** and single campaign CTA **“Map my hospital's first improvement.”** Use one coherent headline and CTA set per live page, not the evergreen button and campaign button together.

Section 1 - Headline

1. **See where one patient creates repeat work** [RECOMMENDED]
2. **Find the handoff slowing your hospital down**
3. **Map a clearer path through your hospital**

Section 2 - Subheadline

For Georgian hospital leaders, a 25-minute founder-led walkthrough reveals one candidate pathway to measure and improve with your team.

Section 3 - Above-the-fold trust line

MediMind is led by a physician CEO who has worked on hospital implementation, so the conversation begins with the work your staff actually does.

Section 4 - CTA button text

1. **Map my pathway in 25 minutes** [RECOMMENDED]
2. **Find my first improvement in 25 minutes**
3. **See the 25-minute hospital walkthrough**

Section 5 - What happens on the demo

- **Trace** one synthetic patient journey from registration through a clinical handoff and the records the next team needs.
- **Inspect** where repeat entry, waiting, or disconnected information could be measured in your hospital's version of that journey.
- **Choose** one candidate pathway, the hospital owner, and a fair baseline for a possible parallel evaluation.

Section 6 - Pain section

“Why does one patient create so much extra work?”

“My team is moving information between systems instead of moving the patient forward.”

“I want to know where the delay starts, not hear another promise about a dashboard.”

Section 7 - How it works

1. Choose one pathway. Name a recurring patient journey where clinical, operational, and business work cross.

2. Follow the work. See how MediMind presents the same case and its handoffs to the people responsible for each step.

3. Agree on a measure. If the fit looks promising, define a baseline, scope, and owners for a readiness-dependent parallel evaluation.

Section 8 - Differentiation section

Your current supplier may already support much of your hospital, and a focused tool may fix a single bottleneck. MediMind is worth comparing when the problem follows the patient across departments and separate fixes leave people reconciling the same work. In the demo, we follow one pathway and name what the product can show, what would need adaptation, and what should be measured. A production change happens only after your hospital's review and sign-off.

Section 9 - Testimonial block

See the work before you ask for a reference. We will show a synthetic pathway and discuss the implementation steps with the founder; a public customer result is not available for this block yet.

[PLACEHOLDER: insert only an approved verbatim customer quote that names the specific workflow and observed result → full name, title, hospital; obtain publication permission and a documented measure first.]

Section 10 - Logo bar caption

Follow a patient pathway, then decide what evidence your hospital would need before changing it.
[PLACEHOLDER: display only hospital logos with written public-use permission; until then, omit the logo images and keep this line.]

Section 11 - Objection FAQ

Will this turn into a sales pitch? We use the 25 minutes to trace one pathway and hear where your hospital's work differs. You can end with a measurement question and no commitment to an evaluation.

Will this fit our workflows? We will mark which steps we can demonstrate now and which need configuration, integration, or development. Bring one clinical or operational colleague if their view would help test the fit.

How much work would an evaluation take? The demo needs only a pathway you can describe; an evaluation needs agreed scope, owners, access and readiness checks. A tailored parallel evaluation is a proposed two-week path when those prerequisites are met, with production sign-off separate.

What would it cost? For eligible early partners, the founder's proposed offer is to match current software pricing within agreed terms and include continuing development within an agreed scope. We will discuss implementation, integrations, support, and any advanced AI costs explicitly before a proposal; no total-cost or permanent price-lock

promise is implied.

Section 12 - Privacy line

We will not sell your work email or give it to third-party marketers. **[Proposed privacy promise: publish only after operations and policy adopt it.]**

Section 13 - Bottom repeat CTA

See where one patient creates repeat work and leave with a candidate pathway your team can measure. **Map my pathway in 25 minutes**

Sources: source demo component, source hero, evidence register, buyer pain synthesis. Handoff: verify submission and receipt delivery, confirm demo duration/host availability, set the early-partner commercial scope, and obtain permission and measured evidence before replacing either proof placeholder.

Source: Original files/Complete B11 HTML library/Editable text/l0-02-webinar.md. Historical wording preserved; not a new approval or factual revalidation.

Webinar Registration Page

Status and completed inputs. This is a proposed webinar page. No live session, registration flow, date, time, replay, or guest speaker has been confirmed. Replace the date/time placeholder and verify registration, reminder, and consent handling before publication. The privacy sentence in Section 13 is proposed policy copy requiring adoption; the pain lines are buyer-language hypotheses, not quotations.

Webinar: Working title: “One patient. Too much repeat work.” Outcome: a hospital leader can map a recurring patient pathway, identify one handoff worth measuring, and set a fair first evaluation question. Counterintuitive hook: work often multiplies at the handoff between teams, even when each individual screen seems usable; this is a hypothesis to investigate together, not a measured universal finding. Live date/time: **[PLACEHOLDER: approved day, date, local time and timezone]**. Duration: **45 minutes total**, with a 30-minute practical workshop and 15-minute live Q&A. Format: proposed founder-led online workshop using synthetic data. Recording: undecided; the secondary CTA collects interest only if a replay is approved. Seat cap: none established.

Speaker: Proposed host Lasha Khoshtaria, physician CEO of MediMind. Relevant experience: clinical perspective plus founder-led hospital implementation work; exact public examples require his approval. No borrowed employer, association, or institutional affiliation is asserted. Speaker 2: none planned.

Audience: CEOs, owners, and group operators of Georgian private hospitals and multi-profile hospitals, with clinical, operations, finance, and IT colleagues welcome. Proposed pain: “We already know the work is duplicated, but I cannot see where it starts or which fix is worth the disruption.” Workarounds to discuss: more coordinators, spreadsheets, calls, and one-off software changes. Fear: a generic product webinar disguised as an operations session. Proposed 30-day success: choose one real pathway, record a baseline (repeat entries, handoff delay, or another suitable process measure), and assign a clinical/operations owner and IT counterpart. No improvement is guaranteed.

Proof and takeaways: Registration count, past attendance/satisfaction statistic, attendee testimonial, and permitted company logos: unavailable. Takeaways: a four-column pathway map; one baseline and one decision rule for an evaluation; a list of readiness questions for a parallel comparison. Proposed resource: a one-page pathway worksheet, to be created and reviewed before promising a download. Suggested agenda: 0:00 problem framing; 0:08 trace a synthetic patient case; 0:20 choose a measure and evaluation boundary; 0:30 live Q&A. **Recommended registration form:** name and work email; one optional question, “Which handoff costs your team the most repeat work?” This form is proposed and has no verified live handler.

Section 1 - Page headline

1. **Find the handoff making one patient everyone's extra work** [RECOMMENDED]
2. **Map repeat work hiding in one patient journey**
3. **Find one hospital bottleneck before buying another tool**

Section 2 - Subheadline

For Georgian hospital leaders, this 45-minute workshop on **[PLACEHOLDER: approved day and date]** turns one synthetic patient journey into a pathway map, a baseline question, and a sensible first evaluation.

Section 3 - Date / time / format bar

[PLACEHOLDER: Day, Date] · [PLACEHOLDER: Time + Timezone] · 45 minutes (30-minute workshop + 15-minute Q&A) · Live founder-led online workshop · Free

Section 4 - CTA button text

1. **Save my place in the pathway workshop** [RECOMMENDED]
2. **Send me the patient-pathway session details**
3. **Reserve my seat for the handoff workshop**

Section 5 - Above-the-fold trust line

Led by MediMind physician CEO Lasha Khoshtaria, who brings a clinician's view to hospital implementation questions.

Section 6 - Speaker block

Lasha Khoshtaria - physician CEO, MediMind. Lasha brings a physician's view of the patient journey to the operational questions that follow it through a hospital. As a founder involved in hospital implementation, he can show how to turn a messy handoff into a specific workflow question that clinical, operations, and IT leaders can test together.

Relevant authority: physician and CEO of the company building the hospital operating system discussed in the session; no external affiliation is claimed.

Why this speaker: he can connect the clinical moment to the implementation choices the hospital would actually have to make, and distinguish a useful demonstration from a measured result.

Section 7 - What you will learn

- **Draw** a four-column map of one patient journey: action, responsible team, information needed, and where the handoff stalls.
- **Spot** repeated entry or reconciliation that looks small in each department but grows along the whole path.
- **Choose** a baseline measure, such as entries repeated per case or minutes from one handoff to the next, that your team can observe without claiming a patient outcome.
- **Set** a narrow evaluation boundary: the case type, owners, readiness checks, and evidence needed before considering a production change.

Section 8 - Pain section

“Every department says its part is working. Why is the whole journey still slow?”

“We keep fixing screens and still make people carry the patient story between them.”

“I need one problem we can measure, not another transformation presentation.”

Section 9 - Agenda block

SUGGESTED AGENDA - confirm before publishing.

0:00 - Where the extra work appears - Start with a single patient journey and the ordinary handoffs that can hide repeat effort.

0:08 - Trace the case across teams - Follow a synthetic case through clinical and business steps and mark the information each person needs.

0:20 - Pick a fair first test - Turn one suspected bottleneck into a baseline, evaluation scope, and readiness question.

0:30 - Live Q&A - Bring a real workflow question; we will discuss how to test it without sharing patient information in the session.

Section 10 - Social proof block

The test is whether the method fits your hospital's work. The session uses a synthetic patient pathway so you can judge the questions and measures for yourself.

[PLACEHOLDER: add a permitted, verbatim past-attendee quote mentioning a specific useful takeaway → full name, role, organization. No past-webinar testimonial or attendance statistic is currently verified.]

Section 11 - Objection FAQ

Is this just a sales pitch? The 30-minute workshop produces a pathway map and a measurement question you can use with or without MediMind. We will identify where MediMind might fit, and where your existing tools may already answer the need.

What if I can't make the live time? Use the replay-interest option below if you want an update. A recording is not yet approved or promised.

Is this too basic or too advanced for me? You do not need a technical background; bring an example of a handoff that feels harder than it should. Clinical and IT colleagues can use the same map to ask more detailed questions.

How long is it really? Plan for 45 minutes total: 30 minutes of practical mapping and 15 minutes of live Q&A. You can leave with the mapping method even if you cannot stay for questions.

Section 12 - “Can't make it?” secondary CTA

If the live time does not work, tell us you are interested in a replay. We will email you only if a recording is approved and available; there is no recording guarantee. **Email me if a replay is available**

Section 13 - Privacy line

We will not sell your work email or share it with third-party marketers. **[Proposed privacy promise: publish only after operations and policy adopt it.]**

Section 14 - Bottom repeat CTA

On **[PLACEHOLDER: approved day, date and time + timezone]**, use 45 minutes to turn one patient journey into a pathway map and a measurable first question. **Save my place in the pathway workshop**

Sources: shared writer brief, buyer pain synthesis, source hero. Handoff: approve the event date and host, create the worksheet, implement registration/reminders, decide recording policy, and collect real permissioned proof before replacing the placeholder.

Source: Original files/Complete B11 HTML library/Editable text/l0-03-trial.md. Historical wording preserved; not a new approval or factual revalidation.

Free Trial Page

Status and completed inputs. This is proposed copy for a guided synthetic-data sandbox, not a currently verified self-service product or an approved commercial policy. The recommended offer is **14 days, no credit card, no automatic charge**, with an option to stop at any time; the team must implement and approve those terms before publication. This is separate from a readiness-dependent, two-week parallel evaluation inside a hospital and from live clinical deployment. Trial-end access, retention/deletion rules, support capacity, pricing, security controls, and any uptime commitment need operational decisions. The privacy sentence in Section 15 is proposed policy copy requiring adoption. Pain lines are buyer-language hypotheses, not customer quotes.

Product: MediMind, a hospital operating system connecting clinical and business work around a shared patient and operational record. First-session “aha”: use a supplied synthetic patient to follow one pathway across roles and mark where repeated entry or a missing handoff could be measured. Recommended time to this first result: the first guided session, assuming the sandbox is provisioned and the workflow is validated; no minute-level setup promise. Three core activities: trace a synthetic patient case, inspect the handoff between teams, and mark one evaluation question.

Trial structure: Opt-in guided sandbox, 14 calendar days, no card, synthetic cases only; this is a recommendation. Included scope: a reviewed sample pathway, role views needed to follow it, a simple pathway worksheet, and one guided orientation if capacity is approved. Excluded from the sandbox: real patient data, production integrations, hospital-specific configuration, clinical use, and a guaranteed production-ready feature set. At trial end: **[PLACEHOLDER: approved pause, export, retention, and deletion policy]**; no paid conversion without a separate proposal and explicit agreement is the recommended billing rule. Cancel/stop anytime: recommended. Free plan after trial: undecided; do not advertise one.

Customer: Georgian private hospital CEO/owner or an assigned clinical/operations lead and IT counterpart evaluating a cross-department workflow. Company type: private network or multi-profile hospital, with fit assessed by workflow complexity and evaluation readiness rather than a fixed bed count. Proposed pain: “I cannot ask people to change systems until I see what the patient journey looks like.” Workaround: screenshots, slides, vendor demos, spreadsheets, and manual walkthroughs. First fear: hidden billing or effort. Second fear: synthetic success may be mistaken for safe real-world readiness. Proposed day-seven milestone: document one pathway, name its owner, and choose one baseline measure; this is a user task, not a predicted product outcome.

Proof: Verified user/company count, customer result, attributable testimonials, third-party review score, and authorized logos: unavailable. Founder-led implementation experience can be described, but it is not a substitute for independent outcome data. Security certifications and uptime/service-level numbers: none verified for this page. Proposed data handling rule: use only supplied synthetic cases and do not enter patient information; any storage, access, deletion, and incident promises need technical and policy approval.

Sign-up and price: Proposed required fields: work email, name, and hospital/group name; optional question after email: “Which handoff do you want to explore?” SSO/social login: none verified, so do not advertise. Public post-trial starting price: unavailable. Proposed path after sandbox: discuss a scoped hospital

> evaluation and a separate commercial proposal. Founder-defined early-partner intent includes matching current software price and continuing development within agreed terms; it does not establish a fixed public plan, total implementation price, advanced AI costs, or a free plan.

Section 1 - Headline

1. **Trace one patient journey before changing your hospital** [RECOMMENDED]
2. **See where your hospital's repeat work begins**
3. **Map the handoff your team keeps chasing**

Section 2 - Subheadline

Georgian hospital teams trace a synthetic patient pathway in their first session: 14-day trial, no credit card required.

Section 3 - Primary CTA button text

1. **Open my synthetic pathway** [RECOMMENDED]
2. **Explore one patient journey**
3. **Map my first hospital handoff**

Section 4 - Trust strip

No credit card required · Stop anytime without a charge · 14-day guided sandbox

Section 5 - Social proof bar

A clinician-led team built this walkthrough around the work that follows a patient between departments.

[PLACEHOLDER - add logos when available and written public-use permission is recorded. Until then, show the credibility line above without logo images.]

Proof to seek: a documented, permissioned result from a real hospital pathway, with its baseline and evaluation scope. No public outcome statistic is verified today.

Section 6 - The problem section

“The patient moves on, but the information has to be carried after them.”

“We enter the same detail again because the next team cannot see it.”

“I need to know whether this fits our work before I ask anyone to change it.”

Start with one synthetic case and see exactly which question your team should test next.

Section 7 - What you get in the trial

Patient path / Follow one synthetic case across roles, so you can point to the handoff that currently depends on a call, re-entry, or spreadsheet.

Shared context / See what each responsible team would need in the same journey, instead of guessing whether another department has the latest detail.

First measure / Leave with one baseline question your hospital could verify before deciding whether a broader change is worth the effort.

Section 8 - Up and running in 3 steps

1. Request access. Enter your work details and choose the handoff you want to explore; the sandbox uses synthetic cases.

2. Follow the case. Open the guided patient journey and move through the roles and handoffs shown in the sample pathway.

3. Mark the handoff. Identify one repeated step or missing piece of context and write down a measure your hospital could test.

Section 9 - Social proof block

Use the first week to assess a real question: can the sample journey help your team name one workflow to measure? This is a suggested test of usefulness, not a reported customer result.

[PLACEHOLDER: “Here's what hospital leaders say after their first week” followed by an approved, verbatim trial quote naming the observed result → full name, title, hospital, with consent and documented context. No qualifying testimonial is verified.]

Section 10 - Product visual caption

One synthetic patient crosses clinical and operational handoffs, making repeat steps visible before you ask your staff to change a live workflow. **[PLACEHOLDER: use only an approved screenshot or GIF of this actual sandbox state.]**

Section 11 - Trust and security block

Synthetic-data security. Use supplied cases only, and give IT verified hosting, access-control, logging, and data-handling details before approval **[PLACEHOLDER: verified controls; no unearned certifications].**

Reliability terms. Ask for the current service-status process and any agreed availability terms; **[PLACEHOLDER: insert a verified uptime figure or approved SLA only if one exists].**

Trial-end data. The proposed sandbox has no automatic paid conversion; **[PLACEHOLDER: publish the approved access, export, retention, and deletion policy before launch].**

Human help. A MediMind team member can guide the first pathway review if guided-trial support capacity and contact route are approved; **[PLACEHOLDER: add confirmed support channel and availability].**

Section 12 - Objection FAQ

Will I need a card or be charged automatically? The recommended 14-day sandbox asks for no card and does not convert into a paid plan by itself. Publish that answer only after sign-up and billing behavior are implemented and checked.

Will setup consume my team's week? Begin with a supplied synthetic journey and a guided path through it; no integration or patient-data migration is needed for this proposed sandbox. A real hospital evaluation needs separate scoping and time from your clinical/operations and IT owners.

What happens to my data if we do not continue? The sandbox should contain supplied synthetic cases, and no real patient information should be entered. **[PLACEHOLDER: state the approved account access, export, retention, and deletion policy here before publication.]**

Is this right for my hospital and role? It is designed for a private hospital or group leader and their clinical, operations, or IT counterparts who can name a cross-department pathway. If your need is a single isolated task, use the trial to test whether a narrower fix would serve you better.

Section 13 - Pricing context line

After the 14-day sandbox, there is no automatic purchase: ask for a hospital-specific proposal covering subscription, implementation, integrations, support, and any advanced AI scope. For eligible early partners, current software pricing may be a reference point under agreed terms; **[PLACEHOLDER: publish approved eligibility, scope, term, and post-trial policy before launch].**

Section 14 - Bottom repeat CTA

Follow one synthetic patient journey and name the handoff your team should measure next. **Open my synthetic pathway · No credit card required · Stop anytime without a charge · 14-day guided sandbox**

Section 15 - Privacy line

We will not sell your work email or share it with third-party marketers. **[Proposed privacy promise: publish only after operations and policy adopt it.]**

Sources: shared writer brief, evidence register, source demo component. Handoff: validate the guided sandbox and support capacity; approve no-card, no-conversion, access/deletion, security and pricing terms; build a working sign-up route; capture a real approved product visual and permissioned testimonial before replacing proof placeholders.

Source: Original files/Complete B11 HTML library/Editable text/p1-01-business-market.md. Historical wording preserved; not a new approval or factual revalidation.

Prompt 1 - Describe Your Business & Market

> **Status and input (22 September 2026).** Working strategy for MediMind, a Georgian hospital operating system built on a shared FHIR R4 foundation. The first economic buyer is the CEO, owner, or group operator of a private multi-profile hospital or network; clinical, finance, operations, and IT leaders shape the decision. GeoHospitals is a founder-reported signed anchor agreement, not proof of a particular number of live sites or measured outcomes. Georgia is the first market; Kazakhstan and Uzbekistan are possible later markets. The founder describes the company as pre-revenue pending integration. Buyer pains below are hypotheses to test, not invented interview findings.

Step 1 - Restate it back to me sharply

MediMind gives private hospital leaders one operating system for clinical and business work, so they can test a clearer patient pathway, reduce repeated coordination, and gain a better basis for improving care and operations.

That sentence deliberately names the buyer, the mechanism, and the change they can inspect. It does not claim an outcome has already been measured. The shortest version for a conversation is: “We help a hospital see and improve the work that one patient creates across departments.” The broad destination is a hospital whose information and operations work together, with AI assistance that staff review and a longer-term path toward more proactive coordination. That destination still needs proof one workflow at a time.

The product is a hospital operating system, not a single documentation tool or management dashboard. Its shared record matters because the same patient journey touches registration, orders, care, medicines, results, billing, and reporting. A buyer does not purchase the data architecture for its own sake. They buy a credible way to make those handoffs easier, with acceptable transition risk and cost.

Step 2 - Name the real market I am in

The behavioral market is **hospital leaders already paying to compensate for work that fragments across clinical and business teams**. Their actual alternatives are doing more with the current supplier, asking staff to reconcile records, adding a focused tool, buying a broader established system, funding internal customization, or postponing replacement. This is the budget and management attention MediMind must win.

What the hospital does now	Why it is rational	When a new operating system becomes relevant
Keeps its current hospital software and asks that vendor for changes	Familiar staff routines and known support avoid a disruptive decision	A recurring cross-department problem remains unresolved at an acceptable cost or speed

What the hospital does now	Why it is rational	When a new operating system becomes relevant
Adds coordinators, spreadsheets, phone calls, and manual checks	People handle exceptions flexibly	The same patient information is recreated so often that the work can be observed and measured
Adds reporting, integration, or a narrow AI product	Targets a specific pain while preserving the core	Another layer makes a task easier but leaves the underlying handoffs and data disagreement intact
Replaces the core with a major vendor or open-source implementation	Broad scope, references, or code access may reduce perceived risk	The actual proposal fails the hospital's local, commercial, adaptation, or transition requirements
Waits	Avoids immediate change to care and revenue processes	Leadership can name an ongoing cost or clinical workflow burden large enough to justify evaluating a change

The practical first segment is not “every hospital in Georgia.” It is an organization with a named recurring pathway problem, an executive owner, a clinical or operations owner, an IT counterpart, and enough readiness to compare an agreed workflow. A 200-800-bed profile appears in internal buyer notes, but bed count alone is an unreliable qualifier. A multi-site group can be one buying organization with many facilities, while a single site can have a complex decision committee.

The first job is to learn what these buyers are trying to fix now. Internal notes suggest reimbursement gaps, avoidable staff effort, and delayed management visibility, but no local interview corpus establishes their frequency or ranking. At the point of sale, the relevant question is: **What was the last concrete patient pathway that forced people to switch screens, copy information, chase an answer, or reconcile a bill?**

Step 3 - Surface my 3 biggest assumptions

1. **The pain is important enough to change the core.** A hospital can dislike its software and still prefer a small fix or the known incumbent. If independent buyers cannot point to a recent costly problem, an owner, and an evaluation window, whole-system replacement is a poor first ask. Test by comparing a scoped MediMind pathway proposal with the narrower remedy the hospital would otherwise buy.
2. **The shared system changes real work, not just the look of the screens.** Better data flow can plausibly reduce repeated entry or reveal gaps, but policy, payer contracts, staff habits, and clinical complexity also drive results. If a measured pathway takes no less time, creates new work elsewhere, or does not improve information reliability, the strategic premise weakens. Test task time, repeat entries, handoff completeness, error/rework, and actual collected revenue where appropriate; do not count an identified charge as cash received.
3. **The attractive offer can be delivered repeatedly at sustainable cost.** Matching current software price, including continuing development, presenting requests for test in three business days, and arranging a tailored parallel evaluation in two weeks are powerful only if scope, resources, and economics work at more than one organization. Test engineering effort, hospital staff effort, support load, infrastructure and AI usage costs, reuse of changes, and acceptance. A testable feature is not a production-approved clinical release.

Each assumption has a disconfirming observation. A buyer who praises a demo but will not supply a workflow owner has not validated demand. A faster screen without less total pathway work has not validated the benefit. One unusually cooperative anchor deployment has not validated the repeatable business model.

Step 4 - Market sizing I can actually use

The defensible starting count is **232 hospitals and about 15,200 hospital beds in Georgia in 2025**, reported by Geostat in July 2026. This counts facilities, not purchasing organizations, qualified prospects, or buyers currently searching. It cannot be multiplied by a guessed annual contract value to produce a useful near-term market number.

Build the reachable market from identifiable organizations, one decision unit at a time:

Filter	How to verify it	Count now
Georgian hospital facilities	Official annual hospital statistics	232
Distinct organizations that buy hospital software	Deduplicate ownership and purchasing authority, including chains	Unknown
Private, multi-profile organizations whose operational complexity fits the product	Review actual service mix and workflow ownership	Unknown
Reachable through a lawful, named route to a decision-maker	Record introduction, contact route, and last verification date	Unknown
Actively seeking a solution	Record recent trigger, alternatives, budget route, owner, and timeframe from a real buyer	Unknown
Able to evaluate during MediMind's available delivery window	Confirm access, counterpart staff, data readiness, scope, and delivery capacity	Unknown

Direct answer: A verified count of reachable businesses actively looking for a solution is unavailable. The answer is not 232 and not zero. Treat the number as a research output, not a slide assumption.

A useful first pass is a **15-organization research list**, including the anchor separately, and **8-10 conversations across at least four independent organizations**. These are proposed discovery targets, not an estimate of live demand. Publicly findable candidates in the prior research include Georgia Healthcare Group/Georgia Capital's healthcare operations, Aversi Clinic, and Vivo Medical Group; their existence does not imply interest or access. GeoHospitals belongs in an anchor/reference column, not the new-customer pipeline. A historical HelsiCore mention needs reconfirmation before it is treated as an active opportunity.

Named organization for the first research list	Why it is findable	Demand and reachability status
GeoHospitals	Founder-reported signed anchor agreement	Existing relationship; exclude from new-buyer count and verify approved reference scope
Georgia Healthcare Group / Georgia Capital healthcare services	Public operating-group reporting	No verified access, current software pain, or purchase intent
Aversi Clinic	Public hospital/clinic website	No verified access, current software pain, or purchase intent

Named organization for the first research list	Why it is findable	Demand and reachability status
Vivo Medical Group	Public group website	No verified access, current software pain, or purchase intent
HelsiCore	Historical founder-side lead in internal notes	Status and relationship need reconfirmation; not a live opportunity count

For each organization record the purchasing unit, facility count, named buyer and evaluators, current remedy, specific observed problem, trigger date, urgency, contract window, willingness to allocate an evaluation owner, and the next verified action. Only then calculate a bottom-up annual opportunity: sum the quoted recurring fees of eligible organizations, show implementation and optional AI separately, and attach explicit timing and conversion assumptions. A signed network agreement must not be inflated into a count of independently won facilities.

Step 5 - Identify the key players

Official vendor descriptions establish the alternatives a buyer can hear about; they do not prove Georgian availability, local price, head-to-head performance, or a vendor's weakness at a particular site. “Fast-growing” cannot honestly be assigned without a growth dataset, so the map groups established and emerging alternatives by the job they compete for.

Player or option	What it does well or offers	Limitation to test, not assume	Why a buyer may choose it now
Current supplier, including MEDSOFT/MEDWORK where installed	Local familiarity; the vendor describes hospital and government-system functions	Can the installed version solve this hospital's chosen pathway problem at acceptable speed and cost?	No migration, familiar support, and known work patterns
InterSystems IntelliCare	Markets a broad clinical, administrative, financial, and AI-connected proposition	Compare local availability, implementation responsibility, adaptation, and total cost in an actual proposal	Breadth, vendor resources, and category credibility
Epic	Broad clinical and enterprise scope with an established ecosystem	Check fit, localization, procurement path, and total cost for this buyer	Established references and institutional familiarity
Oracle Health	Broad EHR and embedded-AI direction	Distinguish available functionality from roadmap and check local fit	Familiar supplier scale or an existing relationship
Bahmni and implementation partners	Open-source hospital-system model and room for adaptation	Who pays for localization, hosting, maintenance, security, and support?	Greater code control or a trusted implementation partner
Commure	Focused clinical documentation and revenue-cycle automation	Does a targeted layer solve the recurring cross-department problem?	A narrower change can be easier to approve

Player or option	What it does well or offers	Limitation to test, not assume	Why a buyer may choose it now
Qventus	Operational automation, including actions in existing systems	Its scope and local fit must be evaluated against the specific pathway	The hospital may improve operations without replacing the core
Staff, spreadsheets, integration projects, and delay	Familiar, immediately controllable, often reversible	What is the continuing cost and residual error?	Avoids an uncertain vendor and transition

The closest message-level challenge is not whether another vendor says “one system” or “AI”: several do. The competitive test is whether MediMind can show a hospital-specific pathway, a credible local implementation, clear ongoing adaptation, and a safe decision process at a cost the buyer can defend. There is no evidence here to claim that all incumbents are disconnected, cannot use AI, or are categorically slower.

The prompt asks for “fast-growing startups,” but a growth label needs revenue, customer, or deployment evidence that this research did not establish. Commure and Qventus represent newer adjacent options competing for attention and budget; the table describes their publicly stated scope without asserting a growth rate. This is a more useful buying comparison than an unsupported startup ranking.

Step 6 - Pinpoint the gap

The underserved buyer is a private hospital leader whose clinical and business teams share a patient but still spend effort stitching that patient's work together, and who cannot justify a risky core replacement on a generic demo alone. That buyer needs a **visible, measurable first improvement** plus a way to understand the complete transition, not another abstract claim that software will “transform care.”

MediMind's proposed response has five parts. First, take an actual pathway across departments as the unit of change. Second, use a shared patient and operational foundation so information can follow the work. Third, let staff see the proposed pathway in a tailored parallel evaluation before a cutover decision. Fourth, offer continued development around the hospital's priorities. Fifth, reduce the initial subscription objection through the founder's early-partner price match. The exact coverage, total transition cost, request boundaries, and ongoing commercial terms must be made explicit per proposal; none is assumed to be zero or unlimited in delivery capacity.

This is a **gap hypothesis**, not a claim that no other company can deliver it. Its defensibility would come from repeatable local implementation, reusable care and business pathways, trusted support, and verified improvements. FHIR R4 is the shared technical base, like common grammar between departments; the hospital buys the easier conversation, not the grammar itself.

Step 7 - Market positioning statement

MediMind is the hospital operating system for Georgian private hospitals and networks that need clinical and business work to move together. Built on a shared FHIR R4 foundation, it lets a hospital start with one costly patient pathway, see how work and information can connect across teams, and evaluate a tailored version beside its existing system before deciding on a wider change. The early-partner offer matches current software pricing and includes continuing development, while the long-term product path adds more useful AI under the hospital's clinical control. The reason to act now is practical: repeated work and delayed visibility continue until the hospital measures and fixes a real pathway.

The investor-deck version should append actual evaluation results when available, rather than presenting an intended benefit as achieved. “Why now” should be anchored to the prospect's recent event-contract renewal, cross-site expansion, an unresolved billing handoff, or staff time lost to a pathway-not to a fabricated market deadline.

Sources and handoff to Prompt 2

Internal facts and limits: evidence register, buyer-pain synthesis, canonical scope, and the founder offer captured in the live-hero working draft. External sources are linked at the claims above. Next, map the complete discovery-to-evaluation-to-live-use sequence and pick the first pathway where a buyer can feel and measure value.

Source: Original files/Complete B11 HTML library/Editable text/p1-02-mechanics-jtbd.md. Historical wording preserved; not a new approval or factual revalidation.

Prompt 2 - Map Your Product Mechanics & JTBD

> **Status and input (22 September 2026).** Prompt 1 established the buyer (a private Georgian hospital or group), the real alternative (current software plus staff workarounds), and the central test (can one scoped patient pathway improve without an unacceptable transition?). This is Prompt 2's working product journey, assembled from founder terms, canonical scope, and internal positioning; it is not an account of one completed customer's path. The main conversion recommendation is a founder-led, 25-minute pathway demo, followed by a proposed measured evaluation. The founder offers requested features testable in three business days and a tailored parallel evaluation in two weeks, subject to readiness and a defined request. Actual production sign-off is separate. The source for the existing `/request-demo` form appears to show no backend lead submission; live behavior has not been independently tested.

Step 1 - Map the customer journey in steps

Use one cross-department example throughout: an ordered test is performed, the result reaches the clinician, and the related work reaches billing. The actual first pathway must be chosen with each hospital. The patient example in marketing should use synthetic data; any real evaluation requires the hospital's authorization, privacy review, and agreed handling.

#	What the customer does	What MediMind does	Result and information still needed
1. Recognize a recurring problem	CEO or operator names a recent case of repeated entry, delay, missing context, or disputed numbers	Explains the cross-department operating-system proposition through one pathway rather than a wall of capabilities	A reason to investigate. Need the buyer's actual incident, frequency, and current workaround.
2. Request a conversation	Provides contact and basic hospital context through `/request-demo` or a direct founder introduction	Should capture and route the request, acknowledge it, and prepare a relevant demonstration	**Conversion gap:** the current source for `src/components/Demo/RequestDemo.tsx` appears to simulate submission with a timer and log, then show success, rather than saving/sending the lead. This needs implementation and a live test before advertising a reliable inbound path.

#	What the customer does	What MediMind does	Result and information still needed
3. Map the first pathway	In a proposed 25-minute founder-led demo, explains where the patient journey creates repeat work and who owns it	Shows the corresponding registration, chart, ordering, results, coordination, and business handoffs with synthetic data	Buyer sees the shape of a possible solution. Need a named owner, systems touched, and the first comparison metric.
4. Decide whether evaluation is worth the effort	CEO, clinical/operations owner, and IT counterpart agree on one pathway, baseline, access requirements, staff time, success criteria, and a decision date	Produces a short evaluation plan, product-scope map, responsibilities, cost view, and readiness check	An actionable evaluation proposal. Missing details vary by site; there is no universal readiness or security approval.
5. Prepare a tailored parallel environment	Hospital authorizes the agreed data or synthetic substitutes, mapping, and staff participation	Configures the relevant workflows and connects or imports the agreed inputs where feasible; staff check data meaning and permissions	A comparable pathway beside current work. The founder's **two-week** offer is a readiness-dependent target, not a promise of whole-hospital migration in two weeks.
6. Compare the first end-to-end pathway	Frontline staff complete the agreed tasks and record friction, safety concerns, time, and exceptions	Shows patient context and linked work in the shared FHIR R4 foundation; provides AI assistance where the specific function is available and approved for use	The first observable difference. Clinical decisions remain with clinicians. Do not confuse a demo click path with accepted real-world performance.
7. Review evidence and decide	Clinical, finance, operations, and IT owners compare baseline with the parallel path and assess burden, errors, data quality, total cost, support, and transition risk	Resolves issues, presents what is supported versus pending, and proposes a staged scope and commercial terms	A go/adjust/stop decision. Even if a request is testable in three business days, production requires separate clinical, operational, security, and contractual sign-off.
8. Expand and operate if approved	Hospital approves controlled rollout, training, responsibilities, and cutover criteria	Deploys agreed capabilities, supports staff, reconciles data, and iterates on prioritized needs	The long-term outcome must be measured across real work: less repeated effort, more reliable information, and any legitimate financial improvement actually realized. Exact rollout and support commitments need a contract.

The product mechanic is a shared work context, not merely “integrations.” Patient data and hospital tasks live in a common model; a doctor, coordinator, pharmacist, lab worker, nurse, and finance colleague act on their respective parts of a connected journey. The hospital still needs interfaces to outside payers, ministries, devices, and other external services wherever required. Scope is verified per site.

Step 2 - Identify the moment of value

Aha moment: the hospital's own team completes its chosen pathway in the parallel environment, then can point to one verified handoff that required less reconstruction of patient information or less repeated work.

The aha is not signing up, seeing a beautiful dashboard, or watching AI text appear. It happens when the coordinator no longer has to reassemble what the clinician, lab, and billing team already know-or when the clinical team can find the needed context in the agreed place-and the hospital can compare this with its baseline. The first moment can be qualitative ("we no longer need this phone call") before it is statistically persuasive; record it without converting it into a general outcome claim.

This matters for retention because a hospital keeps a system that makes its routine work reliably easier and understandable across roles. It matters for marketing because a single, visible pathway is easier to demonstrate and substantiate than an unmeasured promise to improve an entire hospital. The first proof package should show the pathway before and after, staff effort, exceptions, and what still required manual work.

Step 3 - Identify friction points

Journey point	Why the buyer may drop or lose trust	Specific repair or question
Initial message	"Hospital operating system" sounds too broad to believe; AI can sound clinically autonomous	Open with a familiar patient pathway and specify that AI drafts or flags while staff decide.
Demo request	The existing three-step, nine-required-field form asks for effort before value; the source-observed simulated submit could lose leads	Shorten the request to what scheduling needs, implement reliable submission/notification, and test the live route. The exact public form change belongs to implementation work.
Founder demo	A generic feature tour may miss the hospital's real problem	Ask for the last concrete failure and map that journey before showing the relevant capability.
Scope selection	"One operating system" may be heard as a promise to replace every external tool instantly	Make a site-specific scope map: included work, necessary outside interfaces, unverified functions, and later phases.
Data and technical readiness	Unclear access, source quality, terminology, permissions, and security review can stall a two-week evaluation	Name data owner and IT counterpart, test mapping, and define synthetic versus authorized real data before starting the clock.
Staff burden	Parallel work can double tasks temporarily; clinicians may see it as a risky distraction	Limit the pathway and sample, document extra work, let clinical owners stop unsafe use, and collect staff feedback.

Journey point	Why the buyer may drop or lose trust	Specific repair or question
Outcome comparison	A polished demo can be mistaken for proof; an incomplete baseline can make measurement impossible	Agree on a small metric set and method before evaluation. Mark missing denominators and record adverse effects too.
Commercial decision	Price match and continuing development sound open-ended; total cost and priorities are unclear	Put the founder's attractive offer in writing with the actual eligible scope, request process, third-party costs, term, and renewal treatment. These are commercial definitions to settle, not pre-existing exclusions.
Rollout and support	A young supplier concentrates operational dependency	Review support, data access/export, downtime approach, change control, and exit provisions with IT and clinical owners.

Step 4 - Apply the JTBD lens

“Jobs to be done” means the progress the buyer wants at each moment, beyond the screen they click. The executive's functional job is economic and operational; frontline and IT roles determine whether that progress is real and safe.

Major step	Functional job	Social job	Emotional job
Recognize and request	Find a practical option for an unresolved cross-department problem	Show the board and team that the leader is addressing a known bottleneck	Feel there may be a way forward rather than another expensive presentation
Map a pathway in the demo	See whether the proposed system understands the hospital's actual work	Be seen as a leader who listens to staff and asks hard questions	Feel understood and retain control of the conversation
Scope the evaluation	Decide what is safe and worthwhile to test	Demonstrate discipline to clinical and IT colleagues	Feel able to investigate without committing the whole hospital
Prepare parallel work	Keep ordinary operations running and protect data meaning	Be a responsible steward of patients, people, and information	Reduce fear of an uncontrolled change
Complete and compare the pathway	Prove whether a task is clearer, faster, or more reliable	Show a defensible result to leadership and staff	Feel relief that improvement may be tangible, or confidence to stop if it is not
Decide and roll out	Buy only the scope the hospital can operationalize; expand based on evidence	Be the leader who improved work without a careless switch	Feel accountable and confident about the next step

Across these steps the core job is: **“Help me improve how this hospital handles a patient, while giving me enough evidence and control to defend the change.”** Different committee members put weight on different parts. A clinician wants patient context and review authority. Finance wants reconciliation and actual collections, not speculative uplift. IT wants feasible mapping, support, and an exit path.

Step 5 - Identify my market sophistication level

Working assessment: Level 4 of 5, “New Mechanism.” This is an editorial score, not a survey of Georgian buyers. Hospital buyers already hear claims about unified records, less administration, and embedded AI from substantial vendors such as InterSystems and Oracle Health. A fresh list of generic benefits will blend in. “Experience” (Level 5) may apply to an individual buyer fatigued by failed implementations, but cannot be assumed for the whole segment.

The message therefore needs a concrete, believable mechanism: **choose one patient pathway; show the shared work across roles; compare it beside the current system; decide from observed evidence.** Lead with the work the CEO recognizes, explain the shared FHIR R4 foundation only as a reason the handoffs can connect, and put AI in its proper role as assistance with human review. The founder's price match, included continuing development, three-business-day test target, and two-week parallel offer make the mechanism more tangible when written with site-specific scope and readiness. Avoid “first,” “fully autonomous,” “zero risk,” and guaranteed clinical or financial outcomes.

Step 6 - Rewrite the product description

Customer-facing description (112 words):

> One patient can create work across reception, clinicians, the lab, pharmacy, coordinators, and finance. MediMind brings that work into one hospital operating system, so each team can act with the patient context it needs and leaders can see where the journey stalls. Start with one pathway your hospital wants to improve. We map it with your team, show how it works in MediMind, and propose a tailored evaluation beside your current system. Your clinicians remain responsible for care decisions; AI helps with defined tasks for them to review. Compare the work, cost, and staff experience before deciding how far to go. As your needs change, the system can keep developing with you.

Sources and handoff to Prompt 3

The business and market answer, canonical scope, evidence register, and live offer snapshot ground this journey. The `/request-demo` observation comes from the current source file `src/components/Demo/RequestDemo.tsx`; it needs a live browser check before describing deployed behavior. Prompt 3 should translate each documented domain into a buyer benefit, while keeping code presence, deployment, acceptance, and outcomes distinct.

Source: Original files/Complete B11 HTML library/Editable text/p1-03-bfg.md. Historical wording preserved; not a new approval or factual revalidation.

Prompt 3 - Build Your BFG: Features, Benefits & Guarantees

> **Status and input (22 September 2026).** This answers all seven input questions and all six requested steps using the canonical 25-domain scope inventory, the canonical product description, Prompts 1-2, the founder offer, and the evidence register. “Documented” means listed in internal positioning with a named code path or architectural assertion; it does **not** mean a feature has passed a fresh code audit, is deployed at every hospital, has been accepted in production, or has produced measured outcomes. The internal positioning claims the ambient scribe and coordinator desk are live; current site, acceptance, and use details were not verified for this document. AI assistance requires human review in clinical work. A guarantee below is either an existing founder-stated offer needing written scope or a proposed evaluation trust signal-not an invented service level.

The seven required inputs

1. **What are all the features?** The canonical scope names 25 hospital domains, reproduced in Step 1, plus two cross-cutting architecture/language capabilities in the canonical product description. Each domain includes smaller actions, from registering a patient to routing a pharmacy queue or reviewing a clinical event. The source is a product inventory, not a release-by-release acceptance register.
2. **What does the customer get from each?** The benefit is less reconstruction of a patient or operational story, clearer handoffs, and a more inspectable path from care to business work. Step 2 translates every domain separately; each is a potential benefit to test in the hospital's own pathway.
3. **Single most important outcome?** The hospital completes a real patient pathway with less avoidable handoff work and a trustworthy shared account of what happened, while preserving clinical judgment and operational control. This is a target outcome, not a measured result. Financial gains, if any, must be measured as appropriate collections or costs, not simply billing opportunities or minutes assumed to be cash.
4. **Top three objections?** (a) “Will switching disrupt care and revenue?” (b) “Can we trust a young supplier with critical operations, data, and support?” (c) “What exactly is included in price, development, and AI as needs grow?” A broad demo without a staged, costed, clinically reviewed evaluation will not answer these.
5. **What proof exists?** A founder-reported GeoHospitals agreement; internal product scope and code-path inventory; a Medplum-hosted case study authored by a founder; and a founder-stated early-partner offer. These establish narrative, technical work, and a relationship, but they do not independently establish site-wide deployment, a customer endorsement, quantified savings, better outcomes, a certified security posture, or repeatable delivery time. No public testimonial or attributed outcome is supplied.
6. **What guarantees can honestly be offered?** The founder has stated: match current software price for early hospital partners, include continuing development, make requested features testable in three business days, and prepare a tailored parallel evaluation in two weeks. These need scope, eligibility, readiness, and total-cost definitions in writing. The hospital should retain the decision after a parallel evaluation; production

deployment requires its separate approval. No blanket money-back promise, zero-risk promise, result guarantee, permanent price lock, fixed scarcity, or unbounded AI inclusion has been established.

7. **What do customers say about why they chose MediMind?** No traceable buyer quote was supplied. Do not write an invented first-person testimonial. The founder-reported anchor agreement signals that at least one buying organization signed, but its exact selection reasons require a buyer interview and permission before public attribution. Working hypotheses to test are the connected scope, local adaptation, founder access, and an opportunity to evaluate before switching.

Step 1 - List every feature

Rows 1-25 below are the **complete canonical domain list**, organized by the patient and hospital work they support. Rows 26-27 add the two cross-cutting capabilities asserted in the canonical product description. “Scope-listed” is the appropriate common evidence label for domains. It does not make every subfunction live at every site. Before using a row in a proposal, confirm exact workflow, configuration, interfaces, data quality, permissions, localization, site deployment, and acceptance.

#	Category and documented domain	Concrete capability to demonstrate or specify	Current evidence boundary
1	Front door: Patient registration	Patient identity, coverage, related people	Scope-listed; local identity and payer rules need checking
2	Clinical: Ambient documentation	Draft clinical documentation linked to encounter and provenance	Scope-listed; internal live assertion, no fresh acceptance audit
3	Clinical: Computerized physician order entry (CPOE)	Clinical orders for services and medicines	Scope-listed; order catalogs and safety checks need site validation
4	Clinical: Medication administration record (MAR)	Record medicine administration at point of care	Scope-listed; high-risk workflow requires clinical approval
5	Clinical: Pharmacy queue and dispense	Route prescriptions and record dispense work	Scope-listed; inventory and formulary fit need checking
6	Diagnostics: Laboratory	Orders, specimens, observations, diagnostic reports	Scope-listed; instruments and result flow are site-specific
7	Diagnostics: Imaging/PACS viewer	Imaging studies and linked documents available to review	Scope-listed; image archive and viewer fit need checking
8	Capacity: Bed management	Locations and encounter placement	Scope-listed; real-time census accuracy needs validation
9	Access: Appointments and scheduling	Schedules, slots, and booked appointments	Scope-listed; local booking rules need checking

#	Category and documented domain	Concrete capability to demonstrate or specify	Current evidence boundary
10	Coordination: Coordinator/care navigation	Tasks, care teams, and communications	Scope-listed; internal live assertion, no fresh acceptance audit
11	Finance-adjacent: Prior authorization and appeals	Work queues and records for approval/appeal tasks	Scope-listed; payer-specific rules and human review required
12	Finance: Billing and revenue cycle	Charges, definitions, accounts, billing work	Scope-listed; pricing, contracts, and code mapping need checking
13	Finance: Claims processing	Claims, responses, and coverage information	Scope-listed; payer integration and actual collections unverified
14	Supply: Inventory and warehouse	Stock quantities, supplies, deliveries, requests	Scope-listed; physical counts and purchasing rules need checking
15	Workforce: HR and staff management	Practitioner roles and schedules	Scope-listed; payroll and local employment scope not implied
16	Management: Analytics	Read-only views across relevant clinical and operating data	Scope-listed; definitions, completeness, and freshness need checking
17	Public obligations: Ministry submissions/compliance	Prepare/route relevant reporting data	Scope-listed; no blanket regulatory-compliance claim
18	External records: Government EHR integration	Exchange bundles/parameters with government system	Scope-listed; interface availability and approval are external dependencies
19	Group operations: Multi-branch orchestration	Organization hierarchy and branch context	Scope-listed; cross-site data access and reporting need testing
20	Resilience: Offline-first progressive web app	Local cache, sync queue, service worker	Scope-listed; data sync/conflict behavior needs site testing
21	Nursing: Nursing workstation	Nursing tasks, observations, medicine-administration work	Scope-listed; nurse review and handoff fit need checking
22	Coordination: Clinical event engine	Event/subscription-driven tasks and signals	Scope-listed; never imply autonomous patient surveillance

#	Category and documented domain	Concrete capability to demonstrate or specify	Current evidence boundary
23	Teamwork: Messaging and staff inbox	Communications and assigned tasks	Scope-listed; delivery, privacy, and escalation rules need checking
24	AI: Clinical assistant	Assistance within the shared clinical context	Scope-listed; specific functions and clinical review rules need checking
25	Clinical: Unified patient card and history	Patient, encounter, observation, and related history in one chart	Scope-listed; completeness depends on migrated/connected source data
26	Foundation: Shared FHIR R4 data structure	Clinical and business work uses a common health-data model on Medplum	Internal architecture assertion; interoperability still depends on mapping and interfaces
27	Localization: Georgian, Russian, English architecture	Product is architected for three languages	Internal architecture assertion; translation completeness and workflow fit need review

FHIR R4 is the common data foundation behind the rows. The scope document names resource types and code paths for the 25 domains; that is useful technical traceability, not a deployment certificate. Some hospitals will keep necessary outside services and device interfaces. “One operating system” describes the intended core, not a promise that every external dependency disappears.

Step 2 - Convert every feature to a benefit

These are **customer-language benefit hypotheses**. Each should be tied to a demonstration and, where material, an agreed baseline. The grammatical form is intentionally “I can…” rather than an unearned claim that all hospitals already achieved it.

#	Direct customer benefit
1	“I can start the patient's journey with identity and coverage details the next team can use.”
2	“My clinicians can spend less time drafting routine notes from scratch, then review and own the final record.”
3	“Orders can follow the clinical decision into the next team's queue without staff rebuilding the request.”
4	“Nurses can see and record what medicine was actually given in the same care context.”
5	“Pharmacy staff can work from an organized prescription queue and record what was dispensed.”

#	Direct customer benefit
6	“The care team can follow a test from order to result and review, with fewer calls to discover its status.”
7	“A clinician can reach the relevant imaging information from the patient's work context.”
8	“My team can place and track patients with a clearer view of available beds.”
9	“Scheduling staff can offer and manage appointments without rebuilding the patient's context.”
10	“Coordinators can see the next task and who owns it, instead of carrying the pathway in memory.”
11	“Approval and appeal work can be assembled and tracked more coherently, with staff reviewing what goes out.”
12	“Finance can relate legitimate charges to the care documented and identify records that need correction.”
13	“The team can see where a submitted claim stands and what response needs action.”
14	“Departments can see and request needed supplies with less manual stock chasing.”
15	“Managers can connect staff roles and schedules to the care and work that must be covered.”
16	“Leaders can inspect agreed operating measures without waiting for a fresh spreadsheet assembly.”
17	“Reporting staff can prepare required submissions from more consistent records, subject to local validation.”
18	“The hospital can exchange agreed information with a government record system where the interface is available.”
19	“A group operator can view and manage branches with a shared structure and site-specific context.”
20	“Staff may be able to keep selected work available during connectivity loss and reconcile it when service returns.”
21	“Nurses can see assigned care tasks and observations in a focused work area.”
22	“Relevant events can create a visible task or signal for a responsible person to review.”
23	“The right staff member can receive a message or task within the hospital's work context.”

#	Direct customer benefit
24	“Clinicians can ask for defined assistance while keeping the judgment and final action with the care team.”
25	“A clinician or coordinator can see more of the patient's story in one place, depending on available source data.”
26	“My teams can work from a shared data structure, making agreed handoffs and future changes easier to design.”
27	“Our Georgian, Russian, and English-speaking staff can evaluate whether the system fits their daily language needs.”

No benefit here automatically establishes cash savings, a clinical outcome, or use by a specific customer. For example, recording an unbilled charge can help legitimate billing, but only adjudicated and collected amounts establish realized financial gain. A minute freed for a nurse is capacity, not automatically a reduced wage bill.

Step 3 - Add the emotional benefit layer

The emotional and social job varies by user. Below, “feel” names an intended experience to test, and “be seen” names the professional identity at stake. These are not customer quotations.

#	Emotional and social job behind the benefit
1	Reception feels confident that the journey starts correctly; colleagues see a reliable handoff.
2	Clinicians feel heard rather than buried in typing; peers see a complete, clinician-owned note.
3	Prescribers feel confident that an order reached the next role; receiving teams see fewer ambiguous requests.
4	Nurses feel accountable and supported; clinical leaders see a traceable administration record.
5	Pharmacy feels in control of the queue; clinicians see a dependable medicines handoff.
6	Clinicians feel less need to chase; patients and colleagues see a coordinated diagnostic journey.
7	Clinicians feel prepared to decide; peers see the relevant imaging context when it matters.
8	Bed managers feel less reactive; leadership sees capacity decisions made from a coherent picture.
9	Scheduling staff feel organized; patients see a hospital that remembers their context.

#	Emotional and social job behind the benefit
10	Coordinators feel less alone carrying the journey; teams see clear accountability.
11	Approval teams feel prepared, not rushed; payers and leaders see a reviewable record.
12	Finance leaders feel able to explain the gap; clinicians see legitimate, accurate documentation respected.
13	Claims teams feel in control of follow-up; executives see disciplined revenue work.
14	Supply teams feel less surprised by shortages; clinical teams see dependable preparation.
15	Managers feel better able to cover work; staff see fairer, clearer responsibility.
16	Executives feel less exposed in decisions; boards see a leader who can explain the numbers.
17	Reporting staff feel ready for scrutiny; the organization appears more orderly and accountable.
18	IT feels able to meet an external obligation; leadership sees a manageable connection.
19	Group operators feel in command of multiple sites; owners see consistent oversight.
20	Staff feel less helpless when connectivity fails; leadership sees preparedness, if sync is verified.
21	Nurses feel their shift is organized around care; colleagues see dependable follow-through.
22	Responsible staff feel informed without surrendering judgment; leaders see clear event ownership.
23	Team members feel included in the same work; colleagues see timely handoffs.
24	Clinicians feel supported rather than replaced; peers see responsible AI use.
25	Care teams feel oriented to the patient; patients see professionals who know their story.
26	IT feels less trapped by incompatible data structures; leadership sees a foundation they can inspect.
27	Multilingual staff feel included; managers see a more locally usable system if translations are complete.

The highest-leverage emotional job for the economic buyer is **confidence to improve a hospital without gambling today's care, budget, or reputation on an untested switch**. That job links the broad product to the parallel evaluation.

Step 4 - Build the guarantees and trust layer

Only the first three entries describe founder-stated commercial offers. They should be written as agreed terms for eligible partners before public use at scale. The final two are proposed trust signals and evaluation practices, not contractual warranties. “In two weeks” and “in three business days” require a defined start condition and scope; no such detail is invented here.

ID	Guarantee or trust signal; what it promises	Why it reduces buying risk	Where it belongs
T1	**Tailored parallel evaluation in two weeks**, when the agreed pathway and readiness conditions are met; hospital decides after comparing the work.	Makes the first decision an inspectable, limited evaluation rather than a blind cutover.	Hero support line, founder demo close, evaluation plan.
T2	**Current software price matched for eligible early hospital partners**; exact scope, term, and total transition costs to be written in the proposal.	Removes an immediate subscription-budget objection without pretending total cost is unchanged.	Pricing conversation and proposal, after the pathway value is clear.
T3	**Continuing development included, with requested features ready to test in three business days** under an agreed request process.	Makes adaptation feel practical rather than another open-ended vendor queue.	Demo follow-up and contract schedule; clarify testable versus production-approved.
T4	**Visible product mechanism:** demonstrate the actual scoped pathway and relevant FHIR-based record/task flow; give IT a documented scope map.	Lets clinical, operational, and technical evaluators examine what is real instead of accepting an architectural slogan.	Demo, technical diligence, and scope appendix.
T5	**Measured, buyer-controlled decision:** agree baseline, responsibilities, exceptions, clinical sign-off, and go/adjust/stop criteria before rollout.	Preserves decision rights and exposes missing proof early.	Evaluation plan, review meeting, and staged rollout decision.

Do not replace this evidence with a fake testimonial, unverified certification badge, or promised outcome percentage. The anchor agreement can be described accurately when approved for external use; any named logo, customer quote, live-site count, or quantified result needs separate confirmation and permission.

Step 5 - Build the BFG master table

This is the copy source of truth for the canonical 25 domains and two documented cross-cutting capabilities. A row is a **message candidate**, not permission to claim the capability is deployed or beneficial in every hospital. The trust ID resolves to the precise wording and status in Step 4; T4 and T5 are process evidence, while T1-T3 are founder-stated offers awaiting final written definitions.

Feature	Benefit	Emotional Job	Guarantee or Trust Signal
1. Patient registration	Start with reusable identity and coverage context	Reception feels dependable; staff trust the first handoff	T4 scoped pathway demo; T5 data check
2. Ambient clinical documentation	Draft a note for clinician review with less blank-page work	Clinician feels present with the patient and owns the record	T4 live function demonstration; T5 clinical review
3. CPOE	Send an order into the next role's work with context	Prescriber feels the request will not disappear	T4 order-to-receipt demo; T5 safety sign-off
4. MAR	See and record medicine administration in care context	Nurse feels accountable and supported	T4 task demonstration; T5 clinical sign-off
5. Pharmacy queue and dispense	Organize prescription-to-dispense work	Pharmacy feels in control of demand	T4 queue demonstration; T5 site formulary check
6. Laboratory	Follow order, specimen, result, and review	Clinician feels less need to chase the answer	T4 end-to-end demo; T5 instrument/result check
7. Imaging/PACS viewer	Reach imaging context during care	Clinician feels prepared to decide	T4 viewer demonstration; T5 archive/interface check
8. Bed management	See patient placement and capacity in context	Bed manager feels less reactive	T4 capacity demo; T5 census comparison
9. Appointments/scheduling	Manage slots and bookings with patient context	Scheduling staff feel organized	T4 booking demo; T5 local rules review
10. Coordinator/care navigation	Keep tasks and owners visible across a pathway	Coordinator feels less alone carrying handoffs	T1 parallel pathway; T4 task demo
11. Prior authorization/appeals	Track approval work with a reviewable record	Team feels prepared and accountable	T4 task demo; T5 human review
12. Billing/revenue cycle	Connect documented work to legitimate charge review	Finance feels able to explain gaps	T1 parallel comparison; T5 actual-collections measure
13. Claims processing	See submission, response, and follow-up work	Claims team feels in command of exceptions	T4 claims demo; T5 payer validation
14. Inventory/warehouse	See and request supplies more coherently	Supply team feels ready rather than surprised	T4 stock demo; T5 physical reconciliation

Feature	Benefit	Emotional Job	Guarantee or Trust Signal
15. HR/staff management	See roles and schedules related to work	Managers feel prepared to cover shifts	T4 role/schedule demo; T5 scope review
16. Analytics	Inspect agreed operating measures sooner	Executive feels better able to defend decisions	T1 pathway comparison; T5 definitions and baseline
17. Ministry submissions/compliance	Prepare required data more consistently	Reporting team feels ready for scrutiny	T4 report demo; T5 local requirements check
18. Government EHR integration	Exchange agreed records with an external system	IT feels able to meet a data obligation	T4 interface specification; T5 external approval
19. Multi-branch orchestration	Coordinate sites with shared structure and local context	Group leader feels oversight across hospitals	T4 branch demo; T5 site access review
20. Offline-first PWA	Continue selected work during lost connectivity, then sync	Staff feel less helpless during outages	T4 offline/sync demo; T5 conflict test
21. Nursing workstation	See assigned tasks and observations in one work area	Nurses feel the shift is manageable	T4 nurse walkthrough; T5 clinical review
22. Clinical event engine	Route a defined event to a responsible reviewer	Staff feel informed while retaining judgment	T4 event demo; T5 escalation rules
23. Messaging/staff inbox	Keep messages and assignments within the work context	Team feels connected and accountable	T4 delivery demo; T5 privacy rules
24. AI clinical assistant	Get defined assistance with human final decisions	Clinician feels supported, not replaced	T4 function demo; T5 clinical governance
25. Unified patient card/history	See more of the patient story where work happens	Care team feels oriented and prepared	T1 pathway evaluation; T5 completeness check
26. Shared FHIR R4 structure	Design agreed handoffs and changes on common data	IT feels able to inspect the foundation	T4 data-model walkthrough; T5 mapping check
27. Georgian/Russian/English architecture	Evaluate daily work in staff languages	Staff feel included in a local fit	T4 language walkthrough; T5 translation review

The founder offer T2-T3 applies to the buying relationship, so it should accompany the table as a whole rather than be falsely attached as a technical performance guarantee to each clinical feature. If a claim must appear in public copy, lead with a row already demonstrated in the intended hospital setting and add the exact proof available.

Step 6 - Identify my strongest BFG combination

Strongest combination: The unified patient card plus coordinator tasks and the selected downstream handoff (feature), a pathway the team can follow with less reassembly (benefit), confidence that leadership can improve work without losing control (emotional job), and a tailored parallel evaluation with buyer-owned success criteria (T1

+ T5). This connects the widest product story to one experience the hospital can actually judge. It is stronger than using an unmeasured financial or clinical outcome as the headline.

- **Rational:** “Choose one patient pathway, then compare how its information and tasks move across your teams in MediMind beside your current system.”
- **Emotional:** “See your people spend less effort piecing together a patient's journey before you ask them to change how they work.” *Use only as an aspiration until a site comparison shows less effort.*
- **Identity-based:** “Lead a hospital that improves from evidence: start with one pathway your own clinical and operations teams can inspect, measure, and approve.”

Recommended reusable line: “One patient. Too much repeat work. See the first pathway your hospital could improve.” The accompanying CTA is **“Map my hospital's first improvement.”** The line invites the hospital to identify the actual bottleneck; it does not assert that MediMind has already removed a measured amount of work.

Sources and handoff to Prompt 4

Core sources: canonical product scope, canonical language, evidence register, founder-offer hero snapshot, and Medplum's founder-authored case study. Prompt 4 should test direct brand promises against three executive buying situations, with the parallel pathway evaluation as the leading reason to believe.

Source: Original files/Complete B11 HTML library/Editable text/p1-04-uvps.md. Historical wording preserved; not a new approval or factual revalidation.

Prompt 4 - Generate Your UVPs & Map Your JTBD

Status and required inputs (22 September 2026). This is Prompt 4 of the complete B11 sequence. Prompt 1 defines MediMind as a hospital operating system for Georgian private hospitals and groups, competing first with current software and workarounds. Prompt 2 maps discovery, a founder-led pathway demo, a readiness-dependent parallel evaluation, evidence review, and staged rollout; the first value moment is the hospital's own team seeing one handoff work with less reconstruction. Prompt 3 inventories all 25 canonical domains and finds the strongest BFG combination in a connected patient pathway plus buyer-controlled parallel comparison. These are completed working answers, not validated buyer research. The ten UVPs and scores below are editorial options, not observed conversion results. The three buyer types are executive buying situations; finance, clinical, operations, and IT leaders remain essential members of each committee.

Required customer-type inputs

Buyer type	Description in 2-3 sentences	Single most important outcome
A. Private multi-profile hospital CEO or owner	Runs a hospital where clinical quality, staff capacity, and financial performance collide in one operating decision. May be close to daily management but must defend a major software change to clinicians, managers, and owners.	Improve one costly cross-department pathway with a transition and total cost the organization can justify.
B. Private hospital-group CEO or operator	Oversees more than one facility and needs a reliable view of comparable work without erasing necessary local differences. A new site, inconsistent numbers, or repeated implementation effort makes the limits of separate systems visible.	Establish a repeatable way to improve and oversee patient pathways across the network.

Buyer type	Description in 2-3 sentences	Single most important outcome
C. Clinician-owner or medically led hospital executive	Has purchasing influence and sees how software interrupts care and creates duplicate clinical work. Wants useful AI and connected context while preserving clinician judgment and staff trust.	Give clinicians a more coherent patient journey without asking them to accept unreviewed clinical automation.

Step 1 - Map the full JTBD for each customer type

The jobs below are **hypotheses drawn from internal positioning and hospital-context research**, not verbatim statements from interviews. “Job” means the progress a buyer is trying to make when an event forces a decision.

A. Private multi-profile hospital CEO or owner

Layer	Job	Trigger moment	Previous solution or workaround
Functional	Identify a pathway that wastes staff effort, obscures legitimate revenue, or delays decisions; fix it without destabilizing the hospital.	A contract renewal, repeated billing rework, staff overtime, or a report with irreconcilable numbers.	Ask the current vendor for a change, add checks and coordinators, buy another tool, or defer replacement.
Social	Be seen by owners, doctors, and managers as the leader who improved operations and protected care.	A board or clinical leadership meeting asks why the known problem persists.	Present another improvement plan or select a familiar vendor for reassurance.
Emotional	Feel confident that the change is worth the money and will not become a public operational failure.	The proposed replacement feels larger and riskier than the known problem.	Demand more presentations and references, then postpone a decision.

The CEO's job is not to “adopt AI.” It is to deliver a better-run hospital with evidence and control. Price matching may open the conversation, but a visible pathway and credible transition make it a decision.

B. Private hospital-group CEO or operator

Layer	Job	Trigger moment	Previous solution or workaround
Functional	See comparable performance and spread an improvement across sites without rebuilding it from scratch each time.	A new acquisition, branch opening, unexplained variation, or a late group report.	Separate site suppliers, spreadsheet consolidation, local process owners, and repeated integration projects.
Social	Show owners and site leaders that growth comes with oversight and a practical standard of care.	A board asks why similar facilities cannot report or operate consistently.	Mandate more reporting and meetings while each site keeps its own workflow.
Emotional	Feel in command of the network while still respecting site realities.	A cross-site issue becomes visible only after its cost or clinical impact grows.	Rely on calls to local managers and after-the-fact reconciliation.

For this buyer, one pathway is the prototype for a repeatable operating pattern. The proof must show both what can be standardized and what must remain local; a shared dashboard alone is not enough.

C. Clinician-owner or medically led hospital executive

Layer	Job	Trigger moment	Previous solution or workaround
Functional	Give care teams the relevant patient context and reduce documentation and handoff work while keeping clinical decisions with clinicians.	A missed result, a slow handoff, a near miss, clinician complaints, or another request the current system cannot accommodate.	Phone calls, duplicate notes, manual checks, separate tools, and personal oversight of exceptions.
Social	Be seen by clinicians and patients as a leader of safe, useful modernization.	Clinicians resist a new system or ask whether AI is being introduced responsibly.	Trial isolated AI tools or announce an innovation initiative without changing the underlying work.
Emotional	Feel proud of a care environment where technology supports staff rather than second-guesses them.	A clinician describes losing attention to screens or an AI proposal sounds autonomous.	Delay rollout or restrict it to low-risk pilots with little effect on the wider pathway.

This buyer wants the path from “interesting technology” to “my staff trust using it.” The message must keep review rights and clinical governance visible. A claimed clinical outcome needs its own study, beyond a process demo.

Shared core job: “Help me improve how this hospital handles a patient, with enough evidence and control to defend the change.” It carries a functional outcome (less reconstruction and more reliable information), a social outcome (responsible leadership), and an emotional outcome (confidence rather than anxiety about a switch).

Step 2 - Confirm market sophistication level

Working score: Level 4 of 5, “New Mechanism.” The label is a copywriting diagnosis, not a measured Georgian-market statistic. Established vendors such as InterSystems IntelliCare and Oracle Health already describe broad connected systems with AI. Familiar claims like “one record,” “better care,” and “less admin” can establish relevance but cannot carry a distinctive message alone. Individual buyers who have endured failed replacements may react more like Level 5 (“Experience”), so the copy should invite firsthand evaluation early.

The right language shows a concrete mechanism and a controlled next step: **one patient → too many repeated handoffs → map the hospital's version → compare a connected path beside current work → decide from evidence.** Use the operating-system category, then plain language about the work people do. The FHIR R4 foundation can substantiate why data can share a structure, but should not replace the buyer's problem in the headline. The offer-matched current software price for eligible early partners, continuing development, three-business-day test target, and a readiness-dependent two-week parallel evaluation-supports the mechanism. These terms need written definitions. AI remains assistance under clinician review, with broader coordination a product direction.

Step 3 - Generate 10 UVPs as brand promises

Every line below speaks directly to the buyer about a valuable change. Wording that says “can” or “see whether” is intentional where performance has not yet been measured. Some lines suit a specific pathway or buying situation better than a universal homepage headline.

1. **See where one patient's journey creates repeat work-and test a more connected path with your own team.** This is the broad promise and the clearest link to the campaign idea.
2. **Judge a tailored hospital pathway beside your current system before you decide to switch.** This answers the most serious buying fear with the founder's proposed parallel evaluation.
3. **Give your teams one clearer patient story to work from across care, coordination, and business tasks.** This is the core connected-context promise, subject to available source data.
4. **Put your next hospital improvement into your team's hands to test while the need is still urgent.** This draws on the three-business-day request target; it does not promise clinical production in three days.

5. **Keep improving the system with your hospital instead of negotiating a separate development project for every need.** This expresses included continuing development; capacity and request scope require written terms.
6. **Start improving hospital work without first increasing the software price you already pay.** This speaks to eligible early partners under the founder's price-match offer, not total cost or a lifetime price lock.
7. **See the same agreed operating measures across your hospitals, with room for each site's work to differ.** This is the group-operator promise; data definitions and site access must be verified.
8. **Help clinicians spend less effort reconstructing the patient story before they make a decision.** This is a process hypothesis, not a claim of improved clinical outcomes.
9. **Follow legitimate care work into billing and claim follow-up so your team can find what needs correction.** This avoids claiming that every identified gap becomes collected revenue.
10. **Use AI to help your people prepare and review work while your clinicians keep the final clinical decision.** This promises responsible assistance rather than autonomous care.

None should be presented as a customer quotation or a proven result. Each becomes stronger with a demonstrated workflow, a baseline, and a named person who reviewed it.

Step 4 - Score every UVP

Each score is an editorial judgment from 1 (weak) to 5 (strong). Clarity asks whether a CEO understands it on first read. Credibility asks whether existing product scope and the founder offer can support the *wording as written*, given missing outcome proof. Desire asks whether it speaks to the buyer's practical and emotional job. These are not survey scores or conversion predictions; small tests may change the ranking.

#	Short name	Clarity	Credibility	Desire	Total / 15	Why the score lands there
1	Find and test repeat work	5	4	5	**14**	Specific, inspectable, and relevant across roles; reduction remains to be demonstrated.

#	Short name	Clarity	Credibility	Desire	Total / 15	Why the score lands there
2	Parallel judgment before switch	5	4	5	**14**	Concrete founder offer addresses risk; readiness and scope need definitions.
3	One clearer patient story	5	4	4	**13**	Strong product fit, but completeness varies with source data.
4	Fast test of improvement	4	3	4	11	Attractive; request size, prerequisites, and test versus production must be explained.
5	Included continued development	4	3	4	11	Differentiates the relationship, but boundaries and capacity are unproven.
6	Early-partner price match	5	4	4	**13**	Founder-stated offer, easy to understand; does not settle total cost.
7	Comparable group measures	4	3	4	11	Valuable for groups; implementation and data definitions are site-specific.
8	Less reconstruction for clinicians	5	3	5	**13**	Highly desired; needs real task comparison before outcome wording becomes stronger.

#	Short name	Clarity	Credibility	Desire	Total / 15	Why the score lands there
9	Care-to-claim visibility	4	3	4	11	A credible area to test; payer and collection outcomes remain unverified.
10	AI help with clinical control	5	4	3	12	Responsible and clear; may not be the main CEO reason to replace a system.

There is a tie at 14 and a three-way tie at 13. The **top three to carry forward** are #1, #2, and #3 because together they tell one coherent sequence: identify the work, compare safely, then see the connected patient story. #6 is a strong offer line, and #8 is a strong clinical supporting line, but neither alone is the most complete homepage promise. This tie-break is strategic judgment, not statistical superiority.

Step 5 - Match UVPs to customer types

The top three each have a leading buyer and a cross-buyer role:

Top UVP	Strongest customer type and why	Other buyer types it serves
#1 - Find and test repeat work	**A, hospital CEO/owner:** makes a broad software decision start with a visible operating problem the leader can name.	**B:** can repeat the method across sites. **C:** can choose a clinical handoff and involve staff.
#2 - Parallel judgment before switch	**A, hospital CEO/owner:** answers fear of disruption before financial commitment.	**B:** supports staged site rollout. **C:** creates room for clinical review and refusal.
#3 - One clearer patient story	**C, clinician-owner/medical executive:** speaks to the patient context that daily care depends on.	**A:** connects departments and cost of work. **B:** supports a common patient pathway across sites.

For completeness, the full ten-line fit is:

UVP #	A: hospital CEO	B: group operator	C: clinician-owner	Best use
1	Primary	Strong	Strong	Homepage / campaign hook
2	Primary	Strong	Strong	CTA and evaluation section
3	Strong	Strong	Primary	Product explanation
4	Strong	Strong	Primary	Development proof, with scope
5	Strong	Strong	Primary	Founder-offer detail
6	Primary	Strong	Strong	Pricing objection response
7	Moderate	Primary	Moderate	Group-operator page or sales material
8	Strong	Moderate	Primary	Clinical proof story
9	Primary	Strong	Moderate	Finance pathway case
10	Moderate	Moderate	Primary	AI governance section

“Primary” here means likely message fit, not validated purchase motivation. The group-operator line #7 should not be forced onto a single-site CEO's first screen. Price match #6 should support a clear work benefit instead of becoming the only reason to engage.

Step 6 - Write the hero statement

See how one patient pathway can run with less repeat work across your hospital, then decide from a parallel comparison whether to switch.

This one-sentence hero combines the highest-scoring #1 promise with the buyer's central job: improve work while keeping the decision under hospital control. It is a recommended test headline, not a verified conversion winner. A more question-led campaign expression is **“Why does one patient create so much extra work?”** followed by **“Map my hospital's first improvement.”** In either version, the founder-led demo must deliver a concrete map, and the evaluation must measure the pathway before claiming a reduction.

Sources and handoff to later prompts

Inputs are the business/market answer, product mechanics/JTBD answer, BFG master matrix, evidence register, and canonical buyer notes. The two vendor pages are linked in Step 2. Carry **#1, #2, and #3** into competition, avatar, campaign, email, page, and reporting prompts; keep #6 as a commercial support line and #8 for clinical-specific writing. Their order remains a testable editorial choice until buyers explain which promise they believe and why.

Source: Original files/Complete B11 HTML library/Editable text/p1-05-competition.md. Historical wording preserved; not a new approval or factual revalidation.

Prompt 5 - Map Your Competition & Find Your Defensible Position

> **Status and inputs, 22 September 2026.** Working competitive strategy, not a verified win/loss report. Inputs are the completed Prompt 1 business and market answer; the Prompt 4 working jobs to be done (JTBD: the progress a buyer seeks) and UVPs; and current official vendor pages. The buyer is a Georgian private hospital CEO, owner, or group operator, with clinical, finance, operations, and IT leaders in the decision. The working top promises are a connected hospital foundation, a scoped parallel comparison before a switch, and continued development; their ranking remains a strategy judgment. No traceable prospect list of products considered, actual incumbent installations, local competitor proposals, or head-to-head prices was supplied. The prior market-sophistication estimate of 4/5 is editorial, not measured.

Step 1 - Full competitive landscape

The first contest is for the decision to change at all. A hospital may know its software well, have staff who compensate for its limits, and regard replacement as more dangerous than the problem. Product pages establish what a supplier offers publicly; they do not establish a Georgian buyer's actual experience, quote, availability, or implementation quality.

Category and player	Why the hospital might choose it	What to ask in a live comparison
Direct: MEDSOFT/MEDWORK, if installed or shortlisted	A Georgian hospital information system with registration, finances, pharmacy, EMR, laboratory, scheduling, and government integrations [V1]. Local fit and a familiar team can matter more than a new architecture.	Which version is in use? Which workflow actually fails? What can MEDWORK change, at what cost and timetable?

Category and player	Why the hospital might choose it	What to ask in a live comparison
Direct: InterSystems IntelliCare / TrakCare	IntelliCare markets unified clinical, administrative, revenue-cycle, and departmental work with built-in AI. TrakCare remains available in some markets [V2].	Which product is available in Georgia, with which modules, language, integrations, implementation partner, price, and acceptance plan?
Direct: Epic	Broad health-system, clinical, revenue-cycle, interoperability, and AI offerings; a large established ecosystem [V3].	Is there an actual Georgian proposal? What is included, how will workflows be localized, and who carries transition work?
Direct: Oracle Health	An enterprise EHR and embedded AI direction, plus existing organizational relationships in some markets [V4].	Which named functionality is currently available for this care setting and geography, versus described as future design? What is the total proposal?
Direct/build alternative: Bahmni with an implementer	An open-source EMR and hospital system spanning clinical, billing, inventory, and related work [V5]. Code access and a chosen partner can be attractive.	Who owns local requirements, hosting, support, upgrades, security, and continuity? The license being free does not answer total cost.
Indirect: Commure	Documentation and revenue-cycle automation that integrates with existing clinical systems [V6]. It could solve a narrow economic pain without replacing the core.	Does the chosen pathway require core changes or can a layer solve it? Are Georgian workflows and payer processes supported?
Indirect: Qventus	Operational assistants that take actions around surgery and inpatient flow, including work alongside existing systems [V7].	Is the problem operational flow, where a focused tool may be enough, or a repeated clinical-to-business handoff across the core?
Indirect: integrators, reporting tools, and internal developers	Preserve the current system while improving one interface, dashboard, or form.	Who maintains the connector, reconciles source data, and owns failures when two systems disagree?
Status quo: renew the current supplier	Familiarity, sunk training, known contracts, and no immediate migration. This may be the rational winner.	Which specific unresolved problem survives a normal change request? What would justify replacing the core?
Status quo: manual reconciliation	Coordinators, spreadsheets, paper, calls, and extra checks handle local exceptions quickly.	How often is work repeated, by whom, and what is the cost or risk? Avoid assuming every workaround is waste.
Status quo: defer the decision	Protects care continuity and budget this quarter.	What event would make waiting unacceptable, and what is the cost of that delay?

Category and player	Why the hospital might choose it	What to ask in a live comparison
Future: major EHR vendors expanding AI and local partnerships	Epic, Oracle Health, and InterSystems already invest in embedded AI and have supplier resources [V2-V4].	Can they offer comparable Georgian delivery and a low-friction first evaluation? Do not treat their AI as a future-only threat.
Future: focused AI/operations vendors broadening scope	Commure and Qventus can expand from a priority workflow into adjacent budgets and relationships [V6-V7].	Can they solve the buyer's chosen job without a core replacement? If yes, MediMind must justify the larger change.

No buyer has been shown naming any of these in a documented sales conversation supplied to this edition. In particular, the list is a research map, not a claim that Georgian hospitals actively shortlist Epic or Qventus. The MEDSOFT name must not be confused with the unrelated U.S. product Medisoft.

Step 2 - Competitive comparison table

These are **provisional decision ratings** for the Georgian private-hospital use case, not test scores or universal product grades. Strong means the public offer or internal scope gives a credible reason to consider the option on that dimension. Neutral includes major uncertainty and is not a claim of average performance. Weak means a known structural mismatch or evidence gap for this decision. Validate every row against an actual hospital's installed version and proposal.

The five dimensions come from the buyer's job: follow one patient across clinical and business work; fit Georgian practice; improve the workflow when needs change; inspect a safe path to switching; and trust the supplier with operations.

Option	Cross-department clinical + business scope	Georgian/local fit	Ability to adapt chosen workflow	Controllable evaluation/transition	Supplier proof and continuity
MediMind today	**Neutral** - broad code/claimed modules; accepted scope needs verification	**Strong** - Georgian-first design and anchor work, subject to site acceptance	**Neutral** - three-day testable request is founder offer, not a service history	**Strong as offer** - tailored parallel comparison is proposed; readiness and effort still need proof	**Weak** - young, founder-described pre-revenue supplier; limited independent outcomes
MEDWORK	**Strong** - public hospital modules span both sides [V1]	**Strong** - Georgian product and integrations [V1]	**Neutral** - site-specific change record unknown	**Strong for installed site** - staying avoids migration; replacement evaluation unknown	**Neutral** - existing local product, but support record for the buyer unknown

Option	Cross-department clinical + business scope	Georgian/local fit	Ability to adapt chosen workflow	Controllable evaluation/transition	Supplier proof and continuity
IntelliCare / TrakCare	**Strong** - broad unified offer [V2]	**Neutral** - country/product availability unconfirmed	**Neutral** - configurability is described; local delivery untested	**Neutral** - proposal and transition terms unknown	**Strong** - established vendor; site-specific support still requires diligence
Epic	**Strong** - enterprise clinical and business suite [V3]	**Neutral** - localization/availability unconfirmed	**Neutral** - implementation and change path vary	**Neutral** - actual plan unknown	**Strong** - established ecosystem [V3]
Oracle Health	**Strong** - broad EHR offer [V4]	**Neutral** - localization/availability unconfirmed	**Neutral** - current module and roadmap distinction needed	**Neutral** - actual plan unknown	**Strong** - established vendor; specific offering must be checked
Bahmni + partner	**Strong** - hospital system scope [V5]	**Neutral** - local build may be needed	**Strong in principle** - open-source code permits adaptation; partner capacity is the test	**Neutral** - depends on implementer	**Neutral** - depends on contracted operator and support
Commure	**Weak for core replacement** - focused documentation/RCM [V6]	**Neutral** - local fit unverified	**Neutral** - integration and product change terms unknown	**Strong for narrow adoption** - keeping core may lower transition burden	**Neutral** - buyer-specific support evidence absent
Qventus	**Weak for core replacement** - focused operations [V7]	**Neutral** - local fit unverified	**Neutral** - solution configuration varies	**Strong for narrow adoption** - can work beside existing systems	**Neutral** - buyer-specific support evidence absent
Incumbent + staff/spreadsheets	**Neutral** - people bridge gaps, but repeat work may remain	**Strong** - already fitted to local reality	**Neutral** - depends on staff and supplier	**Strong short-term** - no cutover	**Neutral** - familiar but dependent on people and supplier

MediMind's **clear weakness** is independent proof and supplier continuity. Its breadth is not yet a scored victory over an accepted, operating hospital installation. It could also lose on transition burden once staff time, data migration, training, support, and parallel work are counted. A matched subscription price is not a matched total cost. No competitor is rated weak simply because its price or Georgian presence has not been verified.

Step 3 - Find the white space

The most promising opening is **a Georgian private hospital with a named, costly pathway that runs across clinical and business teams, where the current supplier or a narrow add-on has not offered a satisfactory fix and leadership will only consider a core change after seeing a controlled local comparison**. MEDWORK already has local scope; IntelliCare, Epic, and Oracle have breadth and AI; narrow tools can improve a workflow with less disruption. MediMind could win through the *combination* of a local shared foundation, a hospital-specific first pathway, a readiness-dependent parallel evaluation, and a continuing development relationship at clear early-partner terms. The table does **not** prove no competitor can be strong on that combination. The whitespace is a testable buying situation and delivery practice, not a legally or globally exclusive product feature.

The proof artifact should be a one-page comparison for the hospital's chosen workflow: current steps, people and systems involved, time/rework baseline, proposed MediMind steps, staff and IT effort, costs, acceptance criteria, and rollback/cutover conditions. If a focused vendor can meet the same criteria with less risk, that is useful evidence against a replacement sale.

Step 4 - Write and stress-test the defensible positioning statement

Positioning to use: “For Georgian private hospitals ready to fix a recurring patient pathway across departments, MediMind is a hospital operating system that connects clinical and business work on a shared foundation. We start with the workflow your team names, let you compare an agreed version beside current operations when prerequisites are met, and develop the system around what your hospital needs to improve.”

The prompt asks for an “only” sentence. A sentence such as “Unlike MEDWORK, IntelliCare, and Epic, MediMind is the **only** hospital operating system that unifies care and business work” fails immediately: their own pages describe overlapping scope [V1-V3]. “Only system with AI” fails against IntelliCare, Epic, and Oracle [V2-V4]. “Only system that adapts” fails against configurable and open-source alternatives. Even “only Georgian system with a parallel evaluation” is unsupported without exhaustive local contract research. A defensible version keeps the contrast **specific to an actual proposal**: “Unlike a renewal that leaves *this documented pathway* unresolved, MediMind offers to compare an agreed replacement workflow beside your current operation before a switch decision.” That claim must be supported by the buyer's own renewal and our actual evaluation agreement.

Stress tests against the top three alternatives:

1. **MEDWORK/current supplier:** It already describes local hospital modules. If it can fix the pathway promptly, with trusted support and lower transition cost, it may win. Ask for the incumbent's concrete plan, not a category argument.
2. **IntelliCare:** It already combines broad hospital work with AI. MediMind must show local fit, a better scoped evaluation, and sustainable terms in an actual comparison. Country availability and pricing are unanswered.

3. **Epic/Oracle-class enterprise suite:** A buyer may prefer established scale and references. MediMind can only counter with an accepted local workflow, transparent economics, a tested transition plan, and evidence of support. A smaller vendor is not automatically faster or cheaper.

The real defense, if built, is accumulated local implementation knowledge, reusable pathways, credible delivery, customer-approved reference evidence, and measured first improvements. FHIR R4 is a useful foundation, not a moat by itself.

Step 5 - Identify my biggest competitive threat

The incumbent supplier plus postponement is the strongest next-12-month threat hypothesis. It has the data, staff habits, contract, and a built-in argument that change might interrupt care. It is also the least likely to appear in a startup competitor slide. This is a judgment, not a measured loss ranking.

Three actions now:

1. **Collect five recent pathway incidents in discovery.** Ask for the task, who touched it, the actual workaround, and what the incumbent has already been asked to change. Qualify out cases where the existing vendor has a satisfactory plan.
2. **Create one scoped comparison offer.** Agree on a workflow, synthetic or authorized data, readiness prerequisites, staff/IT effort, baseline, success criterion, cost, and a stop/cutover decision. Separate a two-week *evaluation target* from production sign-off.
3. **Build a reference and continuity pack.** With permission, show accepted module scope and one measured process result; document support ownership, backup and export, change control, and handover. Until then, state the proof gap. This directly addresses the rational fear of a young supplier.

Step 6 - Build five competitive talking points

1. **When the buyer names MEDWORK or a local incumbent:** “That system may already fit your Georgian billing and reporting needs, and your team knows it. Which one patient pathway still needs manual repair? Let's compare your supplier's proposed change with a MediMind version, including the effort to switch.”
2. **When the buyer names InterSystems IntelliCare:** “IntelliCare presents a broad, AI-enabled hospital suite. For your hospital, I'd compare the actual Georgian module offer and implementation plan with one locally configured pathway we can put in front of your team. Then you can judge the work, price, and risk.”
3. **When the buyer names Epic or Oracle Health:** “Their breadth and supplier resources are real strengths. We should test the question that matters here: what can your own staff evaluate on the workflow you care about, under a plan they can safely accept? We can show our scope and gaps next to their proposal.”
4. **When the buyer names Bahmni/open source:** “Code access and flexibility are valuable. The practical choice is who will keep the system localized, secure, and supported over time. We can compare total responsibility, not just license fees, against the MediMind delivery agreement.”

5. **When the buyer names Commure, Qventus, or an AI add-on:** “A focused tool may be the least disruptive answer to that one task. If the same patient data still gets recreated between clinical, operational, and billing steps, let's measure the whole pathway and see whether improving the shared core is worth the larger change.”

Sources and handoff to Prompt 6

Official product pages: V1 MEDSOFT/MEDWORK; V2 InterSystems IntelliCare; V3 Epic health systems and Epic AI; V4 Oracle Health EHR and Oracle product documentation; V5 Bahmni; V6 Commure RCM; V7 Qventus platform. MediMind boundaries: internal evidence register and Medplum case study, whose founder-authored account is not independent outcome validation.

For review mining, look for actual user accounts of broad EHRs and the narrow alternatives. Keep praise and pain together. Do not transfer a U.S. clinician's review into a Georgian CEO's mouth or treat review complaints as a switch-rate study.

Source: Original files/Complete B11 HTML library/Editable text/p1-07-user-research.md. Historical wording preserved; not a new approval or factual revalidation.

Prompt 7 - Design & Execute Your User Research

> **Status and inputs, 22 September 2026.** The **design** is complete below; execution has **not** occurred. No interviews, messages, community posts, observations, or buyer-provided records were collected for this prompt. Founder/internal notes describe a signed GeoHospitals relationship and historical access to a possible next organization, but do not supply independent buyer transcripts or permission to contact anyone. The research audience is Georgian private multi-profile hospitals and networks. The strategy to test is whether a CEO-owned patient pathway problem merits a core-system evaluation instead of an incumbent change request or narrow tool. The eight steps specify work to run, not findings claimed as already known.

Step 1 - Audit my assumptions

Ranked by how much a contrary answer would change the product, offer, or market approach:

Rank	Dangerous assumption	Why it could break the plan	Disconfirming observation to seek
1	**Cross-department repeat work is painful enough to justify considering core replacement.**	The product may be solving a real inconvenience with an oversized purchase and migration.	Several independent CEOs can cite irritation but not a recent costly case, accountable owner, spending path, or reason a narrow remedy fails.
2	**The shared system improves total pathway work and information reliability.**	A prettier interface could simply move labor to another person or create double work during transition.	Observed tasks show no net time/error improvement after training, or missing data remains the main cause.
3	**A controlled parallel evaluation is feasible and changes the decision.**	If data, staff capacity, integrations, or governance prevent a test, the primary objection-remover fails.	Buyers who like the idea cannot authorize even a safe scoped comparison, or the trial requires more effort than the hospital can supply.

Rank	Dangerous assumption	Why it could break the plan	Disconfirming observation to seek
4	**The founder's attractive offer can be scoped and delivered sustainably beyond the anchor relationship.**	Matched current software pricing, continuing development, a three-business-day test target, and a two-week evaluation target may consume more capacity than the business can fund.	Independent cases require extensive one-off work or demand unbounded promises, and buyer value does not cover delivery cost.
5	**The CEO/owner is the right primary entry point and can mobilize the buying committee.**	A persuasive executive conversation might not move clinical, finance, IT, or procurement decisions.	Initiative starts elsewhere; a CEO cannot assign an owner, or clinical/IT leaders reject the path on non-negotiable requirements.

Review data [P6] makes usability and support worth asking about, but does not validate any of these assumptions for Georgian CEOs. The research should deliberately seek cases where the incumbent works well.

Step 2 - Define my research goal

Goal: Discover whether independent Georgian private-hospital leaders have a recent, important cross-department patient-workflow problem that their current remedy has failed to resolve, and determine what evidence, decision authority, and transition conditions would make them evaluate a shared hospital operating system. This is **generative** research (discovering how buyers understand and address a problem) with a small **evaluative** component (testing the proposed one-pathway comparison after the story is told). After the sprint, choose one first pathway and a better-defined buyer/offer, or narrow the product message if a focused change consistently wins.

The interview is not a product validation vote. A prospect saying the idea sounds good is weaker evidence than showing a real incident, naming the current cost and alternative, and committing an appropriate workflow owner to a scoped next step.

Step 3 - Segment my user groups

Group	Who; why they are valuable	Unique insight	Initial conversation target and saturation rule
A. Economic sponsor	CEOs, owners, or group operators of private multi-profile hospitals/networks. They own tradeoffs between care, cost, and transition risk.	What made this issue board-level, what alternatives are funded, who can authorize a test, and what would block purchase.	**4 conversations**, at least 3 independent buying organizations. This is a discovery starting point, not guaranteed saturation. Continue until new interviews stop changing the trigger/decision map, then test a different segment.
B. Operating/economic workflow owner	Finance/revenue-cycle or operations leaders at hospitals in the target profile.	What actually happens between care, billing, scheduling, inventory, and management reporting; what is measurable and controllable.	**3 conversations** across at least 2 organizations, preferably including a distinct independent site. Seek both a finance and an operations view if possible.
C. Safety and delivery gatekeepers	Clinical lead plus CIO/IT lead or their delegated workflow owners. This is one research group for access planning but two different perspectives to record separately.	Where patient safety, data quality, training, integration, support, and cutover could fail; which task staff would accept testing.	**3-4 conversations** across at least 2 organizations, including both clinical and IT voices. Do not call saturation if only one function has spoken.

Initial total: **10-11 conversations** if access permits, across **at least four independent buying organizations**. That is an operational target, not a sample-size proof. Include no more than two anchor-network conversations in the first pass so one unusually cooperative relationship does not dominate. If recruiting falls short, record the shortfall and do not relabel internal conversations as independent validation.

Step 4 - Build the interview guide

Target: 30-35 minutes, conducted in the participant's preferred language. With permission, review an anonymized workflow artifact or observe a safe task; no patient identifiers are copied into marketing notes. Ask for stories before concepts. Record exact quotations only with permission and context.

Opening (2-3 minutes). “We are learning how hospitals handle work that crosses teams, especially where a patient's information or a bill has to move between people. This is research, not a sales demo. I am interested in what actually happened, including when your current software worked well. We will not publish your name or organization from this conversation without separate permission. Is it all right to take notes? If we wanted to record, we would ask separately.” Confirm role, organization type, confidentiality expectations, and available time.

Warm-up (5 minutes). “What decisions are you responsible for?” “Which departments most often need to coordinate around a patient?” “What software and manual work sit in that path today?” “What has changed in the last six months?” Do not ask for a list of hypothetical wishes yet.

Core (20-23 minutes; seven questions in this order).

1. “Tell me about the most recent patient, operational, or billing handoff that took more work than it should. What happened, step by step?”
2. “Who noticed first, who had to repair it, and what was the consequence? What evidence exists without exposing patient details?”
3. “How often does that kind of incident happen, and what is your basis for that estimate?”
4. “What have you already asked your current supplier or internal team to change? What happened?”
5. “If you did nothing for six months, what would change for staff, patients, finances, or your own responsibilities?”
6. “What other remedy have you considered-training, a dashboard, integration, focused AI, new core software-and why did it win or lose?”
7. “If a team proposed a limited parallel comparison on that pathway, what would it need to show, who would need to approve it, and what would stop it?”

Only after those answers, show a neutral 30-second description of MediMind and ask: “What part fits, what part sounds implausible, and what would you need to inspect?” Avoid leading with AI or the price match, which can anchor the story. If the interviewee is not the economic buyer, adapt the last question to their approval role.

Closing (3-5 minutes). Summarize back the incident and decision route; invite corrections. Ask for one non-sensitive process artifact, a safe observation opportunity, or the name/role of the person who sees a different part of the same handoff. Ask whether a brief findings summary would be useful. Do not ask for a purchase commitment in a research interview; a later commercial discussion needs its own invitation.

Step 5 - Build a documentation system

Use one folder per organization and one note per conversation, with an organization code rather than patient information. A simple spreadsheet is enough. Fields: date/language, role and site, recruiter relationship, consent/recording status, exact incident, observed versus reported facts, systems and people in order, current workaround, consequence and estimate basis, incumbent response, alternatives, buying trigger, decision participants, evaluation prerequisites, objections, direct quotation with permission flag, and the next evidence request. Keep a link to any authorized artifact, with access limits; do not duplicate sensitive material into the marketing workspace.

Within 24 hours, write a five-line debrief: the participant's job, the latest incident, the strongest evidence, the strongest counterevidence, and what this changes about the initial thesis. Tag statements as **observed**, **buyer-reported**, **team-reported**, **review-derived**, or **interpretation**. Keep a separate assumptions log with a confidence level and what would change it. Do not let a founder recollection become a verbatim buyer quote.

At the end of weeks 1 and 2, make a matrix of rows = organizations and columns = pathway, consequence, incumbent remedy, trigger, buyer authority, evaluation readiness, clinical/IT objections. Look for repeated patterns **across independent organizations**, not just repeated people within one network. Note negative cases explicitly. Summarize exact counts as `n/N` with role and source, not percentages or “most hospitals” from ten interviews. Saturation is assessed by whether new conversations still alter the decision map, not by reaching a fixed number.

Step 6 - Research action plan

Two-week target: 10-11 completed conversations as above, at least four independent buying organizations, plus one permissioned observation or de-identified pathway artifact if feasible. This is a proposed sprint, not scheduled interviews. The founder should allocate interview time separately from the shared five-hour-per-week marketing-production budget; the ten conversations themselves, preparation, and synthesis require extra founder time.

Day 1-2: Ask Lasha to identify the actual GeoHospitals economic sponsor and a clinical/operations/IT counterpart he can approach appropriately. Record the signed relationship and accepted product scope separately. Ask whether the historical HelsiCore contact remains active; do not count it until confirmed. Build a deduplicated list of 15 buying organizations from public sources and legitimate introductions, including Georgia Healthcare Group, Aversi Clinic, and Vivo Medical Group only as research candidates, not presumed prospects.

Day 3-5: Start with one economic sponsor because a recent trigger and decision path determine whether any observed workflow is commercially relevant. Then interview the owner of that exact pathway. Contact independent organizations in parallel, using the tailored drafts in Step 8 only after the founder selects and authorizes recipients. Log refusals and non-responses without inferring lack of pain.

Week 2: Complete clinical and IT counter-interviews, observe an authorized task, and ask remaining CEOs whether the emerging problem statement matches a recent incident. Compare a narrow fix, current-supplier change, and a MediMind parallel evaluation on the same criteria. Create one one-page opportunity brief for each organization, then a cross-organization synthesis.

Success at the end: One sharply defined pathway to demonstrate, or a documented reason to change direction; at least four independently described recent incidents or clear negative cases; a map of who can authorize evaluation; known proof and safety requirements; and a decision on which claim remains hypothetical. A conversation count alone is not success. If access prevents the target, deliver the evidence gaps and revised recruitment plan rather than fictional themes.

Step 7 - Tailored question sets for all three groups

These five per group supplement the common guide; select the relevant questions rather than extending every interview beyond its timebox.

A. CEO/owner/group operator

1. “At the last management meeting, which patient-workflow or operating issue required the most explanation, and why?”
2. “What has the current supplier been asked to improve, who approved that request, and what happened?”
3. “What cost or risk would make you fund a core-system change instead of another local fix?”
4. “Who could veto a parallel evaluation, and what would each person need to see?”
5. “If the evaluation worked, what decision would you be able to make in the next 6-12 months-and what budget or contract event governs it?”

B. Finance/operations workflow owner

1. “Walk me through the last case where clinical work, a charge, and a report did not line up. Where was the mismatch found?”
2. “Which manual reconciliations happen every day or week, and how do you count their time and corrections?”
3. “What portion of a payment delay or cost variance is software-related versus contract, policy, documentation, or staffing?”
4. “What did the existing supplier or an integration project change; what remained?”
5. “Which process measure and real financial measure could you safely compare in a two-week evaluation, and what data would be off-limits?”

C. Clinical/IT safety and delivery gatekeeper

1. “Where in the last patient handoff did staff leave the main record, re-enter data, or call someone-and what safety checks did that workaround provide?”
2. “What information must be complete and current for clinicians to trust a parallel workflow?”
3. “Which interfaces, migration steps, access permissions, backup tests, and audit records are mandatory before any real-data evaluation?”
4. “How would you detect a harmful change in documentation, medication, results, or care-team communication?”
5. “What training, support, stop rule, and clinical sign-off would you require before expanding beyond the first pathway?”

Step 8 - Nine outreach templates

All nine are **unsent drafts** under 100 words each. They offer a useful research summary/checklist before a specific ask; the recipient must be selected through an appropriate route. The community versions should be posted only where research requests are permitted and should never invite patient details in public replies.

Group A - CEO, owner, or group operator

LinkedIn message. “I’m putting together a short checklist for evaluating whether a hospital's cross-department workflow needs a core-system change or just a focused fix. It may be useful for your next software decision. I'm speaking with leaders of Georgian private hospitals about a recent real example, including cases where keeping the current system was right. Would you be open to a 25-minute research conversation next week? I can send the checklist and a summary of themes afterward.”

Email. Subject: A practical test for a hospital software decision. “Hello [Name], I can share a one-page comparison template for mapping the staff effort, risk, and cost of one patient pathway before choosing a software change. We are using it to learn how private-hospital leaders make these decisions, without asking for patient data. Would a 25-minute conversation next week be useful? I would ask about one recent incident, the current remedy, and what proof a limited evaluation would need. I can return an anonymized findings summary.”

Community/forum message. “For hospital leaders weighing a new system against another change request, I have a one-page pathway comparison checklist: steps, owners, rework, total transition effort, and stop conditions. I can share it with members here if research requests are allowed. I’m also looking for three private-hospital decision-makers in Georgia for a 25-minute interview about a recent cross-department workflow problem. Please message me privately if that is relevant; no patient or commercially sensitive details in this thread.”

Group B - Finance or operations owner

LinkedIn message. “I’m making a simple worksheet to separate staff rework, payment delay, and actual recovered collections in a hospital pathway. It may help avoid overstating a software business case. I’d value a 25-minute conversation about one recent reconciliation your team handled and what caused it. Would you have time next week? I can share the worksheet and a short anonymized summary of what we learn.”

Email. Subject: Measuring one hospital handoff honestly. “Hello [Name], I can send a short worksheet for recording where clinical, billing, and operations work diverge without confusing a found charge with cash collected. We are researching how Georgian private hospitals decide which handoff to fix first. Could I ask you about one recent case in a 25-minute call next week? We need no patient details. I will share the completed blank framework and aggregated findings, subject to participants' permission.”

Community/forum message. “I have a free blank worksheet for measuring repeat work in a clinical-to-finance handoff, including the counterexample where software is not the cause. If this forum permits research invitations, I’d be glad to share it. I’m seeking finance or operations leaders at Georgian private hospitals for a 25-minute conversation about one recent reconciliation, current workarounds, and what could actually be measured. Please reply privately; do not post patient, claim, or contract data.”

Group C - Clinical or IT gatekeeper

LinkedIn message. “I’m drafting a safety-first checklist for a limited parallel hospital-workflow evaluation: data scope, staff review, interfaces, training, and stop rules. Your clinical or IT perspective could make it more realistic. Would you be open to a 25-minute conversation next week about one recent cross-team handoff and the conditions you would require before testing a new workflow? I can send the checklist for your use afterward.”

Email. Subject: What a safe parallel workflow test must include. “Hello [Name], I can share a concise checklist for evaluating a hospital workflow beside an existing system without treating a demo as production acceptance. We are researching the clinical and IT requirements that make such tests useful or unacceptable. Could I ask for 25 minutes next week to discuss one recent handoff and your must-have safeguards? No patient records are requested. I will return the checklist and anonymized research themes.”

Community/forum message. “I’m sharing a draft checklist for safe, limited hospital workflow evaluations: information completeness, access, audit, support, training, and a stop rule. If this group allows research posts, I’d welcome a clinical lead or hospital IT leader in Georgia for a 25-minute conversation about what the checklist misses. Please contact me privately if interested; this thread is not a place for patient details or security configurations.”

Sources and handoff to Prompt 8

Research starting facts and limits: Prompt 1, competition map, review intelligence, buyer evidence register, and the existing provisional buyer-pain synthesis. None substitutes for the planned interviews. Prompt 8 should label buyer problems as **assumed** or **partially supported by external users/internal reports** until the sprint yields traceable local buyer evidence.

Source: Original files/Complete B11 HTML library/Editable text/p1-08-problems.md. Historical wording preserved; not a new approval or factual revalidation.

Prompt 8 - Define the Real Problems You Are Solving

Status and inputs, 23 September 2026. This is a decision-ready problem hypothesis, not a claim of completed Georgian buyer research. Prompt 1 defines MediMind and its market; Prompt 5 maps alternatives; Prompt 6 supplies a small public-review corpus from largely non-Georgian staff; Prompt 7 is a research plan with **no completed interviews**. The current hero expresses better care, connected work, and an AI future but does not contain a single exact problem sentence supplied by customers. Internal wording describes disconnected programs and repeat work; it is not a buyer quotation. The chosen first audience is a private-hospital CEO/owner or group operator. Clinical, finance, operations, and IT leaders must still validate the problem and the remedy.

Step 1 - Separate real from assumed problems

Evidence key. “Validated” here would mean traceable evidence from the target buyer or an observed target workflow, plus enough context to know its cause and importance. “Partially validated” means a real but indirect observation exists. “Assumed” means a strategic interpretation or unmeasured effect. No problem is fully validated for the target Georgian economic buyer in the supplied record.

Claimed or tempting problem	Status and source	What the evidence does and does not say
Staff perform repeated entry and too many steps on some EHR tasks.	**Partially validated** as a problem experienced by external software users [R1, R2, R9, R10].	First-person staff reviews describe task friction. They do not establish frequency, cost, cause, or existence at Georgian prospects.
Teams value shared patient context and still struggle to find some information.	**Partially validated** as an external-user need [R1, R4, R5, R8].	The same reviews praise competitor breadth; a “no existing system connects work” claim would contradict them.
Existing supplier changes are too slow or expensive for Georgian private hospitals.	**Assumed** for prospects; internal positioning raises it [I1].	No independent vendor-change timeline, proposal, or buyer quote was supplied.

Claimed or tempting problem	Status and source	What the evidence does and does not say
Clinical, financial, and operational handoffs cause material revenue leakage.	**Assumed** as a local causal/size claim; internal economic-buyer thesis [I2].	Payment contracts, policy, documentation quality, and staffing may dominate; detected missing charges are not realized collections.
Leaders cannot obtain timely, trusted network-wide performance information.	**Assumed**, internal buyer thesis [I1, I2].	No management report, reconciliation time, or executive testimony inspected.
A connected system can reduce total staff work and improve decisions.	**Assumed outcome**; plausible mechanism and product scope.	No before/after observation or accepted first-pathway result here. Training and parallel work may offset gains.
Buyers want a core replacement and will permit a parallel evaluation.	**Assumed demand**, founder offer and strategy [I3].	A controlled comparison sounds useful, but no independent buyer acceptance or readiness record was supplied.
Better patient outcomes, higher revenue, and guaranteed compliance result from the system.	**Not validated** and should not be marketed as achieved.	Product design and a contract cannot establish clinical/financial causality or legal compliance.

The public reviews are useful clues for interview language and workflow observation. They are not testimonials for MediMind. The founder-authored Medplum case study supports product direction and a reported anchor contract, not verified improvement for every hospital or a license to claim a full network go-live.

Step 2 - Build the full problem stack

These are **hypotheses for three buying contexts**, not fictional customer quotes. “Future” describes a possible consequence over 6-12 months if the pattern continues, not a forecast. In every row, another cause or a narrower fix may prove more important than replacing hospital software.

Buyer context	Surface problem they could name	Underlying problem to test	Hidden problem to test	Possible 6-12-month future if unresolved
Independent private-hospital CEO or owner	“The same patient journey creates repeated handoffs, calls, and entries” is proposed wording, **not a quote**.	Clinical, scheduling, billing, and reporting steps may lack a shared, trustworthy handoff, or the workflow may be poorly designed even when the system is integrated.	The leader may fear spending scarce resources on software while staff remain frustrated, or may fear a failed replacement more than present inefficiency.	The same rework and delayed information may keep consuming staff capacity. The size of any financial or clinical effect needs measurement.
Hospital-network CEO or group operator	Site reports and patient pathways may be hard to compare or standardize across hospitals.	Different sites may use inconsistent definitions, contracts, systems, and practices. A shared database alone cannot resolve governance.	The leader may fear that growth is outrunning oversight and that a central change will alienate local clinical teams.	Expansion or improvement projects may require repeated reconciliation and site-by-site customization, if current remedies remain inadequate.
Clinician-owner or medically led hospital leader	Staff may spend attention navigating records, repeating documentation, or chasing results instead of patient work.	The information needed for a decision may be scattered, poorly presented, or delayed by local process.	The leader may fear that an AI or software initiative will erode clinical trust, obscure responsibility, or make care feel less personal.	Staff frustration and workarounds may persist; potential safety effects require clinical review, not a marketing inference.

Committee viewpoints sharpen the same stack rather than multiplying invented personas. Finance asks whether a charge was actually collected and whether total cost improved. Operations asks who does the extra work. Clinical leaders ask whether information is accurate and whether safeguards remain. IT asks who owns interfaces, access, backup, and exit. A purchase requires these jobs to align.

Step 3 - Identify the problem worth leading with

Lead with a measurable question: Why does one patient create so much extra work across your hospital? This is a proposed campaign hook, not a buyer quotation or proof that every hospital has the problem. It is specific enough to bring clinical and business work into one conversation and to start with a recent, observable pathway. It is broad enough for a CEO to invite the right committee members, while the first demo can narrow it to one task.

Its evidence status is **partially supported externally**: several first-person EHR users describe repeated charting or task navigation, and other users praise the benefits of connected records [P6]. Internal buyer notes independently name cross-department friction [I1]. What remains unproven is local urgency, economic weight, replace-versus-fix preference, and MediMind's actual net improvement. The honest message is an invitation to **map and measure the hospital's own pathway**, not a statement that

MediMind has already eliminated the work.

For the first discovery call, ask for the last real patient journey that required duplicate entry, a call to another department, a manual spreadsheet, or bill reconciliation. If the buyer cannot name one with a consequence and owner, do not force this hook onto the account. If the incumbent can fix it satisfactorily, the operating-system sale may not be justified.

Step 4 - Write three problem statement versions

Rational, for an analytical decision-maker: “When a patient moves through clinical, scheduling, and financial steps, staff may have to re-enter or reconcile the same information. Before replacing anything, count the handoffs, minutes, errors, and actual payment effects in one pathway; then compare the current remedy with a shared workflow.” This is data-grounded in **what must be measured**, not a fabricated quantified loss.

Emotional, for a story or interview invitation: “Your people know how to care for a patient. It is exhausting when they also have to carry the patient's information from screen to screen and chase the answer another team already has.” This is proposed language to test with staff and leaders, not an attributed quotation.

Identity, for a CEO who sees themself as a responsible improver: “You should be able to show your team a better way to run one patient pathway-and prove that it helps-before asking them to trust a hospital-wide change.” This frames the leader's responsibility without accusing them of neglect or promising zero transition risk.

The three versions share one underlying job: make a specific improvement visible and defensible before a larger commitment. The emotional and identity versions are hypotheses about what will resonate; do not label them validated by review-site users.

Step 5 - Problems I should not claim to solve

1. **“We guarantee better clinical outcomes or detect deterioration before clinicians do.”** Shared patient context and AI assistance can support review, but autonomous hospital-wide clinical decisions and attributable patient outcomes have not been established. Clinical decisions remain under qualified human control. Any safety claim needs the exact use case, validation, and approved workflow.
2. **“We recover all lost revenue or fix hospital finances.”** Payer rates, contracts, clinical coding accuracy, staffing, and management choices may explain much of a financial gap. MediMind may help show documentation or process issues in an agreed workflow, but only actual legitimate collections and net costs can establish a financial result.
3. **“We replace every system in two weeks, at the same total cost, with no disruption.”** The founder's two-week parallel-evaluation target is readiness dependent; it is separate from full production migration and clinical sign-off. Matched current software price does not establish migration, support, interfaces, optional AI, or lifetime terms. A hospital OS still interfaces with outside systems and devices.

These boundaries also protect the product from drawing a buyer whose only priority is a narrow ambient scribe, a one-off dashboard, or a compliance guarantee that no supplier can make without a defined legal and operational scope.

Step 6 - Problem-solution fit statement for each type

The required format is used as a **hypothesis**. “Solves” means the proposed mechanism for a scoped pathway, not a measured universal outcome; each statement must be updated after interviews and evaluation.

Independent private-hospital CEO/owner: “The hospital leader struggles with repeat work and delayed answers along a patient pathway because information and responsibilities can break at clinical-to-business handoffs. Unlike another unmeasured change request to the current supplier, MediMind proposes to connect the selected steps on a shared hospital record and compare a tailored workflow beside current operations, so the leader can decide whether the total work improves enough to justify a wider change.”

Hospital-network CEO/group operator: “The group leader struggles to compare and improve the same pathway across sites because systems, definitions, and practices may differ. Unlike manually stitching site reports together, MediMind proposes a shared clinical and operational foundation with agreed local adaptations, so the group can test more consistent information and spread a verified improvement site by site.”

Clinician-owner/medically led leader: “The clinical leader struggles when staff chase patient context or repeat documentation because the needed information is not available at the right step. Unlike adding a stand-alone tool to one screen, MediMind proposes to bring the chosen clinical and supporting business steps into one reviewable workflow, so clinicians can test whether they get the context they need with less avoidable work while retaining clinical control.”

Each statement should be rejected or rewritten if a buyer's actual root cause is staffing, policy, contract terms, or a narrow fix that works better. The next test is a real baseline and acceptance criterion, not a vote on whether this prose sounds compelling.

Sources and handoff to Prompt 9

Evidence hierarchy and internal caveats: evidence register, buyer-pain synthesis, Prompt 5 competition, Prompt 6 review corpus, and Prompt 7 research plan. No new interviews were conducted. Prompt 9 should build a **synthetic, testable buyer portrait**, not claim a real Georgian CEO said the constructed monologue.

Source: Original files/Complete B11 HTML library/Editable text/p1-09-avatar.md. Historical wording preserved; not a new approval or factual revalidation.

Prompt 9 - Build Your Customer Avatar

> **Status and inputs, 23 September 2026.** This is a **synthetic working avatar**, built for message and interview design. It is not a portrait of a named real buyer, a composite from completed interviews, or a prediction that all Georgian CEOs behave alike. No target-buyer interview findings exist yet. Inputs are the Prompt 1 market, Prompt 4 working JTBD/UVPs, Prompt 5 alternatives, Prompt 6 staff-review language, Prompt 7 unexecuted research plan, and Prompt 8 problem stack. If only one buyer context could be served first, choose the CEO/owner or group operator of a Georgian private multi-profile hospital who can name a recurring cross-department problem and assign a clinical/operations owner plus an IT counterpart to an evaluation. The exact sentence this buyer says aloud is **unknown**; the proposed signal in the reference card must be tested.

Step 1 - Build the full eight-dimension avatar profile

Avatar label: The accountable hospital operator. This is a role description, not a fictitious person's identity. Details of calendar, fears, and media habits are hypotheses to check. The day-in-life scene below is illustrative, with no implied patient, hospital, or interview case.

Dimension	Working portrait and what would verify it
1. Who they are	A CEO, owner, or group operator responsible for clinical continuity, finances, staff capacity, and major supplier decisions in a Georgian private hospital or network. They may work in Tbilisi or a regional center; the location, age, gender, family situation, and personal life are intentionally unspecified. Their day could move between a management report, a department concern, an operating decision, and a supplier discussion. Confirm actual decision remit and daily schedule in interviews.
2. What they are trying to achieve	Over the next **6-12 months**, identify an important hospital-workflow problem, improve it without disrupting care, and show leadership and staff a result they can inspect. This is a planning horizon, not a reported personal goal. Define success with the buyer: e.g., reliable handoff information, fewer repeated steps, faster report preparation, or appropriately measured actual collections after total costs.
3. What stands in their way	External: old contracts, interfaces, inconsistent records, limited staff time, training load, supplier dependencies, and clinical/IT approval. Internal: rational aversion to a failed change; possible belief that another dashboard or coordinator is safer. These barriers require case-specific proof, not a generic “resistance to change” label.

Dimension	Working portrait and what would verify it
4. What they have already tried	Possible paths to ask about: renewing the current system; requesting a vendor change; adding spreadsheet checks or coordinators; buying reporting/integration or a focused AI tool; considering a broader EHR; postponing. **None is asserted as this avatar's history.** The research interview should establish which path was actually tried and whether it worked.
5. How they make decisions	Likely a mix of operational evidence and trust. They may ask finance for a real business case, clinical leaders for patient-safety and usability review, IT for security/integration/exit requirements, and staff for workflow acceptance. Purchasing authority, contract calendar, and formal procurement steps vary by organization; map them rather than presuming a simple CEO signature.
6. What they fear	Rational: downtime, missing data, unplanned total cost, a young supplier that cannot support the hospital, and an AI mistake that blurs clinical accountability. Possible emotional fear: being remembered as the leader who distracted staff with a costly modernization that did not improve work. Interviews must establish whether these fears are present and ranked.
7. What they secretly want	A defensible, tangible improvement that staff notice and owners can trust; freedom to improve care and operations without negotiating every small change from scratch. This is creative direction, **not a discovered confession**. A buyer may instead value stability above visible innovation.
8. A day in their life	Illustrative scene: The leader starts with a site-performance check. Before a management meeting, an operations lead reports that one patient pathway required phone calls and corrections between registration, a clinical team, and billing. Finance can show a delay but cannot yet say whether money was lost. The CEO asks what the incumbent can change, what staff effort the workaround costs, and whether a limited comparison would be safe. Later, clinical and IT leaders raise data and cutover questions. The exact scenario is a research prompt, not a reported event.

The most useful research correction would be a buyer who says their most urgent issue is not cross-department repeat work, or who explains that current software is excellent and staffing or payer rules are the actual cause. The avatar should then change.

Step 2 - Build the avatar's emotional landscape

1. **Dominant emotion - cautious frustration (hypothesis).** If a recurring pathway absorbs skilled staff time, the leader may be frustrated by continued rework while remaining wary of a risky core change. Neither emotion has been directly measured in Georgian buyers.
2. **Aspiration emotion - earned confidence.** They want to see evidence that a change makes work better and that staff can still care safely. Confidence comes from a transparent comparison, not a promise that there is

no risk.

3. **Social context - the committee is watching.** Owners or a board may ask about cost; clinicians about safety and work; finance about collections; IT about control and continuity. A careful first improvement could strengthen the leader's credibility. This social interpretation needs interviews, not storytelling alone.
4. **Relationship with solutions - informed skepticism.** They have likely seen software promises, but the specific experience is unknown. A narrow tool or incumbent change may be an intelligent choice. MediMind earns attention by understanding the hospital's actual pathway, showing boundaries, and stating what is and is not accepted or measured.

Step 3 - Write the avatar in their own voice

Clearly synthetic monologue (about 190 words). This is original creative writing. It is **not** a verbatim buyer quote and should never be attributed to a Georgian hospital leader. One short phrase about click burden is borrowed from a cited Epic staff review [R1] as a language cue, not evidence that a CEO said it. Real buyer language should replace this after interviews.

> I can see how hard our people work. What I cannot always see is why a single patient creates so many separate jobs for them. Someone checks a record, someone calls another team, and someone later has to make the numbers agree. I need to know whether that is an occasional exception or a pattern worth changing. I also need to know which part is software and which part is our process.
>
> I have no interest in putting care at risk for a prettier screen. Our current system may be frustrating in places, but staff know how to use it. A nurse saying there are “too many clicks” matters to me if it points to real time lost or missed information. I want my clinical and IT leads to inspect the same pathway with me and tell me what would make a test safe.
>
> Show me one improvement that stands up to comparison. Show me the work my team would have to do, the full cost, and the rule for stopping. If we can prove that first step, I can talk about a larger change with confidence.

The monologue avoids a fabricated patient incident, CEO quotation, recovery amount, or personal biography. The quoted three-word phrase is from Tabatha W., supervising nurse, 9 April 2026 and is the only reused review wording here.

Step 4 - Identify the avatar's watering holes

The following are **real, inspectable venues** where relevant material exists; whether this specific buyer visits them is **unverified**. The first research conversation should ask, “Which association, publication, colleague, event, or online channel actually influenced your last software decision?” Do not invent a Georgian hospital-CEO subreddit, private Slack, Discord, podcast habit, or group membership.

Channel	Verified venue or example	Practical use and validation question
Online professional community	The Georgian Healthcare Association site describes a member network for medical institutions; its news/publications cover sector changes and events. Its LinkedIn page is visible.	A credible place to learn topics and seek permissioned introductions. Verify which target organizations are members and whether decision-makers read or attend. Association existence is not proof of a marketing channel's reach.
Social platform	LinkedIn has an association presence and public hospital/company pages such as Tbilisi Central Hospital.	Use named professional relationships and useful process material, not mass claims about a person's feed. Ask interviewees whether LinkedIn led them to any vendor or peer insight.
Content	The Georgian Healthcare Association publications address local policy and provider issues; Geostat healthcare indicators provide official sector context. The Georgian Medical Journal is a clinical/research source more likely relevant to clinical committee members than a CEO buying channel.	Use these to understand current topics and write accurate local context. Buyer readership is unverified. No specific newsletter, podcast, book, or YouTube channel is assigned without evidence.
Offline and event community	The Healthcare Association has documented provider meetings and past conferences, including a 2024 quality/patient-care event. HIMSS Europe is a verified international digital-health gathering with a 2027 edition listed.	Past local events show a possible peer setting, not a current invitation or attendee list. HIMSS is an optional broader learning venue; do not assume Georgian CEOs attend. Verify actual events and relevance before allocating travel or sponsorship.
Private peer route	A known hospital sponsor, clinical lead, or fellow provider executive may be more credible than an open community. The founder-reported GeoHospitals relationship creates a potential route, subject to permission and real access.	Ask each interviewee where they seek trusted vendor advice and whether a peer introduction is appropriate. Do not treat an anchor as permission to publish their name or request an endorsement.

The top three **places to test first**, not confirmed preferences, are the Healthcare Association's provider network/news, named LinkedIn professional connections, and direct peer introductions. The association's U.S. namesake at `gha.org` refers to the U.S. state of Georgia and is **not** this market's venue.

Step 5 - Identify the avatar's buying trigger

Working trigger hypothesis: A recent management or clinical-review meeting exposes a repeated patient-pathway handoff that staff have been compensating for, and the current supplier's answer does not provide a credible fix within the leader's operating horizon. A software contract renewal, new site, incident, or difficult monthly reconciliation could bring that moment forward. There is **no evidence that this is the single most common trigger** among Georgian buyers; Prompt 7 must test it.

At that moment the leader may feel urgency mixed with caution: frustration about the recurring work and anxiety about disrupting the hospital. The first likely action is to ask the workflow owner and incumbent supplier for a concrete account of what happened and what can be changed. That action is also unverified, so MediMind should be visible with a helpful pathway-mapping guide and a founder-led 25-minute demo invitation, then ask for a real example. Do not insert a fake crisis or an artificial deadline.

An operational qualification sequence follows: Is there a recent case? Is its consequence meaningful? Is an owner assigned? Has the narrower remedy been considered? Can clinical and IT counterparts set safe evaluation conditions? A “yes” to an AI headline alone is not the trigger.

Step 6 - Build the avatar reference card

Field	Working answer for the team
Name / description	**The accountable hospital operator:** CEO, owner, or group leader of a Georgian private multi-profile hospital who can sponsor a first cross-department pathway evaluation. A role label, not a real person.
Primary 6-12-month goal	Show one meaningful care/operations improvement with a defensible measure, then decide whether wider system change is justified. Hypothesis.
Biggest obstacle	The known system and workarounds may be inadequate, but a core replacement brings data, staff, continuity, and supplier risk.
Dominant → aspiration emotion	Cautious frustration → earned confidence. Hypotheses to test.
Signal sentence	Proposed buyer-language test: “Why does one patient create so much extra work?” **Not an actual CEO quote.** Ask interviewees for their own sentence.
Top three suggested watering holes	Georgian Healthcare Association provider network/news; relevant LinkedIn professional connections; permissioned peer introductions. Actual attendance/usage unknown.
Buying trigger	A recent cross-department problem plus an unsatisfactory current remedy, often surfaced in a leadership review. Frequency unverified.
Say to earn trust	“Let's map one pathway, the work and cost it creates, and the conditions for a safe comparison; your clinical and IT team decide whether the result is good enough to expand.”
Never say	“We can replace everything in two weeks with no risk and guarantee better care or recovered revenue.” It confuses a scoped parallel evaluation with production acceptance and asserts unmeasured outcomes.

Use this card to draft questions and prototype a message. Replace its speculative lines with actual words, decision steps, and channels from at least four independent hospital organizations before treating it as an evidence-based persona.

Sources and handoff to Part 2

Internal strategy and evidence: Prompt 1, competition, review corpus, unexecuted research plan, problem stack, and evidence register. Watering-hole existence is supported by the linked Georgian Healthcare Association, LinkedIn, Geostat, Georgian Medical Journal, and HIMSS pages; buyer use is unverified. Part 2 can use this as a provisional audience model, with the founder-led pathway demo as the next step and without claiming research or results that have not occurred.

Source: Original files/Complete B11 HTML library/Editable text/p2-01-funnel.md. Historical wording preserved; not a new approval or factual revalidation.

Part 2, Prompt 1 - Map Your Marketing Funnel Architecture

> **Status and inputs (23 September 2026).** This is the complete strategic blueprint for a founder-led, low-volume hospital sale; it is not a report of current funnel performance. The first buyer is the synthetic **accountable hospital operator** from Part 1 Prompt 9: a Georgian private-hospital CEO, owner, or group operator who can name a recurring cross-department problem and assign clinical/operations and IT counterparts. Prompt 8's lead problem is “Why does one patient create so much extra work?” Prompt 4's top three working UVPs are finding and testing repeat work, comparing a pathway before switching, and giving teams a clearer patient story. Prompt 3 lists the 25 documented domains plus shared FHIR R4 and language architecture, without equating code presence with live acceptance. The primary CTA is a recommended **25-minute founder-led pathway demo** ending in a proposed measured evaluation. These are strategy assumptions, not measured buyer preferences.

Current situation answers. Conversion needs a human conversation and a buying committee; it is not self-serve. Exact monthly or annual price is not established; the founder's early-partner offer is to match current software price, with final scope/term still to define. A live website and `/request-demo` route exist. At source inspection the demo form has three steps, nine required fields, and a simulated submit (timer plus browser log and success state) without visible backend persistence or network submission; the deployed route has not been live-tested. No verified website traffic, source mix, lead count, email platform, customer CRM, marketing analytics, content winners, or channel results were supplied. The investor dashboard does not establish a customer-funnel system. The biggest constraint is **a trustworthy path from attention to a held, qualified hospital conversation and then an evidence-led evaluation**. Existing founder relationships and an anchor agreement are context, not a measured acquisition funnel.

The planning north star is **12 cumulative held qualified hospital demos over 90 days**, with **3 readiness-approved evaluations** and **1 new paying buying organization** as stretch downstream scenarios, excluding the existing anchor from new-customer counts. This is capacity planning, not a forecast. It implies roughly one held qualified demo per week, a 3/12 evaluation progression, and a 1/3 paying progression if those scenarios occur. Small denominators are provisional; report zero-denominator rates as N/A. Marketing has **five hours total per week** and zero paid spend initially. Engineering setup, implementation, and actual sales meetings require additional effort and are not silently charged to that five-hour marketing allowance.

Part 1 - Funnel diagnosis

1. Model and closest real-world funnel

The best fit is a **hybrid funnel led by founder sales and supported by specific educational content**. A hospital cannot safely evaluate a whole operating system by clicking into a self-serve trial; the CEO's committee needs a pathway, scope, data, clinical review, IT review, and a commercial decision. Content should help a leader name one costly handoff and arrive at the demo prepared, while the founder converts interest into a measured evaluation. Of the five examples in the library, **Salesforce's high-touch enterprise motion** is closest in buying complexity; borrow executive education and committee-aware follow-up, not an assumed enterprise budget or ecosystem lock-in. HubSpot's useful diagnostic asset is the secondary pattern to borrow: give a pathway worksheet before asking for a meeting.

2. Biggest leak now

There is no verified traffic-to-lead-to-demo dataset, so a numerical leak rate cannot be calculated. The **first observed operational leak is the demo-request handoff**: current source code appears to display a success state after a two-second timer and a console log rather than transmitting the request to a lead owner. The long three-step form may also deter buyers, but abandonment has not been measured. Do not call either a deployed failure or claim lost leads until the live route is checked. First fix: verify the deployed behavior, make an accepted submission persist or notify the right owner, show truthful success/failure states, and test the complete route. Then instrument clean marketing events and track held qualified demos, not merely clicks or form success screens. If there is little traffic, add named-account and permissioned peer distribution rather than optimizing percentages from a handful of visits.

3. The actual buying journey to design for

Buyer moment	What they may search or consume	Emotional state and objection	Required next step
A repeated handoff is normal	Current vendor advice, peer experience, general workflow articles; perhaps nothing at all	Busy; the work seems inevitable. “Is this worth executive attention?”	A short scenario or worksheet that helps count the repeat work.
A trigger makes it concrete	A difficult monthly report, staff complaint, bill correction, new branch, renewal, or clinical review	Frustrated but cautious. “Is software the cause?”	Map one recent pathway and compare the current remedy.
Alternatives enter the room	Current-supplier change, narrow tool, integration, broader system; asks peers or IT	Skeptical. “Why replace a core system for this?”	Honest comparison, scope map, and patient-pathway demonstration.

Buyer moment	What they may search or consume	Emotional state and objection	Required next step
A committee considers MediMind	Founder-led demo, technical/clinical discussion, offer and proof	Interested but anxious. “Will this disrupt care and cost more than advertised?”	Written readiness and evaluation plan with total effort and decision criteria.
Evaluation proves or disproves value	Staff try the agreed pathway beside current work; metrics and exceptions are reviewed	Focused, protective of staff. “Does this work here?”	Go/adjust/stop decision; staged production approval if warranted.
Customer becomes a reference, if satisfied	Internal result and support experience; possible permissioned story	Proud but protective of reputation and data	Seek explicit approval for a factual case story or peer introduction.

The most likely permanent loss is when a buyer asks for a credible, low-disruption comparison and receives a generic feature tour or an overconfident “replace everything in two weeks” claim. The current form can lose them even earlier if a request is not actually delivered. The funnel therefore needs both technical lead receipt and substantive evaluation proof.

Part 2 - Full funnel stage map

Stage	Customer world and awareness	Our goal and what we give	Objection to collapse; where we lose them	Benchmark context from the B11 library
TOFU - Awareness of the problem	They live with repeated handoffs or disputed reports but may not have named a solvable software issue.	Help them locate one patient pathway and quantify basic repeat work through a post, synthetic example, or worksheet.	“This is just how hospitals work.” Lose them with jargon, fabricated loss numbers, or an AI-first pitch.	Library cites homepage-to-lead 2-5% average, >10% top performer. This is unverified general SaaS context, not a Georgian-hospital target.
MOFU - Consideration of the solution	They know the issue matters and compare incumbent change, a narrow layer, and core replacement.	Give option comparison, product mechanism, readiness checklist, and honest limits.	“A new system adds risk and cost.” Lose them by hiding transition work or treating vendor claims as proof.	Library cites webinar-page 25-45% and B2B email opens 20-30%, 40%+ excellent. Neither is a local planning target; no webinar is scheduled.

Stage	Customer world and awareness	Our goal and what we give	Objection to collapse; where we lose them	Benchmark context from the B11 library
BOFU - Conversion decision	CEO and committee can imagine a change and want the result in advance.	Offer a 25-minute pathway demo and proposed measured parallel evaluation with success criteria.	“Show me it can work here before I commit.” Lose them at a nonfunctional request route, vague scope, or unsupported proof.	Library cites demo-page 3.8% median, >8% top performers and B2B free-trial-to-paid 15-30%, top quartile 35-45%. A hospital parallel evaluation is not a self-serve SaaS trial.
AFU - Retention and expansion	The hospital has approved a defined scope; staff need the first useful pathway and dependable support.	Reach the first buyer-accepted pathway quickly; measure staff burden, exceptions, and value, then select the next workflow.	“The demo looked easier than daily work.” Lose them at poor adoption, missing data, or unclear issue ownership.	Library cites >60% trial activation to aha and 46.9% month-one B2B SaaS retention. Neither is validated here; 46.9% would be a poor hospital-service standard, never a success target.
LOFU - Advocacy and loop	A customer may have a verified result and be willing to help a peer.	Request permission for an accurate case or reference; bring its limited, useful lesson to the next buyer.	“Do not use our name or data loosely.” Lose trust with unapproved logos or inflated outcomes.	No suitable advocacy benchmark is supplied; use permissioned references and qualified introductions as counts.

The benchmark figures above are repeated because the prompt asks for them. Their source and applicability were not independently established in the library. The operational thresholds in Part 6 use the 90-day planning scenario and controllable funnel events instead of pretending these SaaS percentages predict a Georgian hospital sale.

Part 3 - Content and asset brainstorm

The catalog contains **85 distinct brainstorm ideas: 17 per stage, each with three LinkedIn posts, three phone-filmable short videos, three blog/SEO pieces, two lead magnets, two proof concepts, two emails, and two community/distribution moves.** Each row below has a stable ID. Full machine-readable fields-ID, title, format, stage, idea, CTA, pageId, and recommendedChannel-are in content-catalog.json. P01-P11 are destinations proposed for a full funnel: P01 assessment/download, P02 product overview, P03 verified case study (pending), P04 proposed unscheduled 45-minute practical webinar (30-minute workshop plus 15-minute Q&A), P05 existing `/request-demo` route, P06 offer/evaluation, P07 comparison, P08 activation, P09 referral, P10 proof library (pending), and P11 proposed 14-day guided synthetic-data

sandbox with no card. The sandbox is separate from the readiness-dependent two-week parallel hospital evaluation. A page ID is a design reference, not a claim that the page exists. Proof ideas dependent on customer approval are held until evidence exists. The catalog is a brainstorm, **not 85 assets to produce on a five-hour weekly schedule**.

TOFU: 17 stage-specific ideas

ID	Format	Title and execution idea	Destination
T01	LinkedIn post	**One patient, seven handoffs.** Sketch a synthetic patient journey and ask leaders which handoff their team repeats most.	P01
T02	LinkedIn post	**The hidden cost of searching.** Describe the time spent finding a result across teams as a question to measure, without inventing a savings figure.	P01
T03	LinkedIn post	**When two reports disagree.** Pose a board-meeting scenario where finance and clinical reports disagree and show three questions to ask.	P01
T04	Short video	**The duplicate-entry trail.** Phone-filmed desk sketch follows one fictional lab order copied between reception, clinician, and billing.	P01
T05	Short video	**Three calls for one result.** Founder acts out three routine calls needed to locate one synthetic result and asks what happens locally.	P01
T06	Short video	**The spreadsheet behind the dashboard.** Show a blank synthetic spreadsheet and the questions a CEO should ask about report preparation time.	P01

ID	Format	Title and execution idea	Destination
T07	Blog/SEO	**How to map a patient handoff.** Step-by-step guide to drawing one patient's movement through departments and marking repeat entry.	P01
T08	Blog/SEO	**Why hospital reports disagree.** Explain differences in definitions, timing, missing data, and manual reconciliation without blaming one vendor.	P01
T09	Blog/SEO	**What a delayed claim does and does not mean.** Distinguish rejected claim, delayed cash, missed legitimate charge, and unrecoverable revenue.	P01
T10	Lead magnet	**Patient-pathway friction worksheet.** One-page template to count handoffs, repeat entries, wait points, owners, and a starting baseline.	P01
T11	Lead magnet	**Five-minute handoff check.** Self-assessment that returns the first pathway worth discussing, not a made-up hospital score.	P01
T12	Proof	**Founder-built scope trace.** Use the public founder-authored Medplum case-study context as product-history evidence, clearly not a customer outcome endorsement.	P02
T13	Proof	**Synthetic handoff before/after.** Show a clearly labeled synthetic workflow comparison to prove the method is inspectable, not that savings occurred.	P02

ID	Format	Title and execution idea	Destination
T14	Email	**Where the work repeats.** Send the worksheet with one question about the hospital's last repeated handoff; no generic AI pitch.	P01
T15	Email	**A report you had to rebuild.** Invite a leader to identify one report that requires manual reconciliation and the person who prepares it.	P01
T16	Community	**Hospital-operator discussion prompt.** In an appropriate Georgian hospital-leadership forum, ask how teams map a handoff; participate only where welcome.	P01
T17	Community	**Clinical-operations partner note.** Offer the neutral worksheet to a clinical operations association or trusted professional contact for feedback.	P01

MOFU: 17 stage-specific ideas

ID	Format	Title and execution idea	Destination
M01	LinkedIn post	**Fix a screen or fix the pathway?.** Compare three practical options: current-vendor change, narrow tool, or shared operating system for the same handoff.	P07
M02	LinkedIn post	**What parallel evaluation actually requires.** Name the hospital owner, data permission, baseline, and staff time behind an honest two-week evaluation.	P06

ID	Format	Title and execution idea	Destination
M03	LinkedIn post	**AI assists; clinicians decide.** Explain a reviewed draft or flag in a fictional pathway and show where the clinical decision stays human.	P02
M04	Short video	**Three options on one whiteboard.** Founder draws status quo, point solution, and operating-system path with honest change effort for each.	P07
M05	Short video	**What happens in 25 minutes.** Preview the founder-led pathway demo: buyer names a handoff, founder maps it, team picks a first measure.	P05
M06	Short video	**A result becomes a task.** Synthetic screen-recorded example shows a result routed to a named reviewer, with no patient data.	P02
M07	Blog/SEO	**How to evaluate hospital software without a risky cutover.** Guide to pathway scope, authorized data, parallel workload, success criteria, reconciliation, and sign-off.	P06
M08	Blog/SEO	**Hospital OS versus overlay versus current vendor change.** Decision guide for when each option is appropriate, including cases where replacement is not justified.	P07
M09	Blog/SEO	**What FHIR R4 changes in hospital work.** Plain-language explanation of a shared data structure and the interface work still required.	P02

ID	Format	Title and execution idea	Destination
M10	Lead magnet	**Parallel-evaluation readiness checklist.** Checklist for named owners, agreed pathway, data permissions, baseline, IT review, and stop criteria.	P06
M11	Lead magnet	**Three-option decision worksheet.** Template comparing incumbent fix, narrow add-on, and MediMind against burden, risk, cost, and scope.	P07
M12	Proof	**Documented domain map.** Publish the 25-domain scope as an internal product map with clear deployed, accepted, and pending fields after verification.	P10
M13	Proof	**Live-pathway evidence template.** Show the exact baseline and after fields a customer-approved case study would need; label outcome cells pending.	P10
M14	Email	**The change-risk question.** Explain what a parallel comparison can answer and ask which transition failure the hospital fears most.	P06
M15	Email	**The current-vendor alternative.** Invite the buyer to compare a current-vendor fix with a connected pathway using the same criteria.	P07
M16	Community	**Peer discussion on evaluation design.** Ask consenting hospital operators what evidence they require before changing a core workflow.	P06

ID	Format	Title and execution idea	Destination
M17	Community	**IT and clinical review session.** Offer an educational walk-through of data, safety, and cutover questions with no sales claims.	P06

BOFU: 17 stage-specific ideas

ID	Format	Title and execution idea	Destination
B01	LinkedIn post	**What your 25-minute demo will settle.** List the three outputs: first pathway map, first metric, and evaluation readiness decision.	P05
B02	LinkedIn post	**Match price, define total cost.** Describe early-partner software-price match and the need to itemize transition and third-party costs.	P06
B03	LinkedIn post	**The honest go/no-go test.** Show how leadership can decide after staff compare one pathway and its extra work or risk.	P05
B04	Short video	**Demo in sixty seconds.** Founder shows synthetic map, shared task, and comparison worksheet as the demo preview.	P05
B05	Short video	**Parallel beside current work.** Draw existing and proposed pathways side by side and explain readiness, workload, and sign-off.	P06

ID	Format	Title and execution idea	Destination
B06	Short video	**Who should join the first call?.** Name CEO or operator plus optional clinical/operations and IT counterparts, depending on decision stage.	P05
B07	Blog/SEO	**Questions to ask before a hospital OS demo.** Give buyers questions about scope, human review, data rights, deployment proof, and total cost.	P05
B08	Blog/SEO	**What happens after a parallel evaluation.** Explain measured review, reconciliation, approval, and staged rollout; no automatic production switch.	P06
B09	Blog/SEO	**What current-software price match should include.** Explain the questions a hospital should settle about eligible spend, term, development, and total cost.	P06
B10	Lead magnet	**Pathway demo preparation card.** One-page card for an executive to bring a recent incident, current workaround, owner, and desired measure.	P05
B11	Lead magnet	**Evaluation decision sheet.** Go/adjust/stop template that records baseline, staff burden, risk, cost, and sign-off owner.	P06
B12	Proof	**Verified implementation timeline.** After authorization, publish dated scope, readiness, and acceptance milestones from a real customer without claiming broad outcomes.	P03

ID	Format	Title and execution idea	Destination
B13	Proof	**Buyer-approved workflow result.** Reserve case study for a measured result and customer permission; until then show evidence fields as pending.	P10
B14	Email	**Your pathway before our demo.** Ask attendee for one recent handoff and offer the preparation card; do not request patient-identifying details.	P05
B15	Email	**What we will compare.** After demo, send a proposed baseline and evaluation scope with unresolved requirements visibly marked.	P06
B16	Community	**Private buying-committee walkthrough.** Convene named clinical, finance, operations, and IT evaluators around one pathway, by invitation.	P05
B17	Community	**Procurement questions session.** Offer a structured discussion of costs, responsibilities, support, and exit needs with the buying group.	P06

AFU: 17 stage-specific ideas

ID	Format	Title and execution idea	Destination
A01	LinkedIn post	**First pathway, first baseline.** With permission, describe the method for recording a baseline after sign-up, without showing confidential data.	P08

ID	Format	Title and execution idea	Destination
A02	LinkedIn post	**What staff should report in week one.** Explain how to capture repeat work, exceptions, and safety concerns during a controlled evaluation.	P08
A03	LinkedIn post	**One improvement, then the next.** Show an anonymized learning loop: observe, prioritize, test, approve, measure; no unverified success claim.	P08
A04	Short video	**How to log a handoff issue.** Phone walkthrough of an issue form with synthetic patient data and an owner for follow-up.	P08
A05	Short video	**Five-minute pathway review.** Show how clinical and operations owners compare the agreed metric and exceptions each week.	P08
A06	Short video	**How a testable change becomes approved.** Explain request, test, clinical/IT review, and production approval with a generic example.	P08
A07	Blog/SEO	**Measuring repeat work without double counting.** Define staff time, task count, training burden, and why minutes do not automatically equal cash savings.	P08
A08	Blog/SEO	**Running a safe pathway review.** Guide to issue ownership, clinical escalation, baseline comparison, and go/adjust/stop decisions.	P08

ID	Format	Title and execution idea	Destination
A09	Blog/SEO	**Selecting the second pathway.** Criteria for choosing a next workflow after the first is accepted: frequency, burden, risk, and readiness.	P08
A10	Lead magnet	**First-pathway measurement sheet.** Customer worksheet for baseline, tasks, exceptions, staff burden, and sign-off.	P08
A11	Lead magnet	**Change-request decision card.** Template for request owner, test criteria, clinical impact, priority, and production approval.	P08
A12	Proof	**Accepted-workflow log.** Internal customer-approved ledger of which pathway elements were accepted and which remain open.	P08
A13	Proof	**Measured improvement record.** Record an actual agreed before/after comparison only after data quality and buyer sign-off; no public release by default.	P08
A14	Email	**Your first review agenda.** Send the clinical/operations owner a concise agenda for metric, exceptions, staff feedback, and next decision.	P08
A15	Email	**A request ready for test.** Tell the named owner what changed, where to test it, what remains unapproved, and how to respond.	P08
A16	Community	**Clinical-IT issue huddle.** Short private working session for clinicians and IT to resolve one observed pathway issue.	P08

ID	Format	Title and execution idea	Destination
A17	Community	**Cross-site learning exchange.** With a group customer, invite site owners to compare process lessons without patient details.	P08

LOFU: 17 stage-specific ideas

ID	Format	Title and execution idea	Destination
L01	LinkedIn post	**The buyer's permission comes first.** Explain what a responsible public hospital story needs: verified result, named scope, and permission.	P10
L02	LinkedIn post	**A pathway another leader can inspect.** After approval, share a measured pathway story with method and limits rather than a sweeping transformation claim.	P03
L03	LinkedIn post	**What we changed after feedback.** Publish a consented example of customer feedback becoming a tested product change, with dates verified.	P10
L04	Short video	**Customer pathway walkthrough.** With customer approval, have staff explain a measured before/after task using non-identifying visuals.	P03
L05	Short video	**What a reference actually verified.** Founder shows the evidence standard and a permissioned result; hold production until proof exists.	P10

ID	Format	Title and execution idea	Destination
L06	Short video	**Why we chose the first pathway.** Customer-approved owner explains why the initial workflow was selected and what remains unsolved.	P03
L07	Blog/SEO	**How to read a hospital software case study.** Teach readers to distinguish contracted sites, live sites, accepted workflow, and measured outcome.	P10
L08	Blog/SEO	**From one hospital pathway to another.** Permissioned account of what transferred across sites and what required local redesign.	P03
L09	Blog/SEO	**What changed in a claim handoff.** Only after verification, explain actual claim-process result and separate identified charges from collections.	P03
L10	Lead magnet	**Reference-story evidence checklist.** Template a hospital can use to evaluate any vendor story: baseline, denominator, site, dates, approval.	P10
L11	Lead magnet	**Peer evaluation guide.** Guide for hospital executives to ask a reference about transition burden, support, and continuing development.	P09
L12	Proof	**Buyer-approved measured case study.** Publish only after customer permission and audited baseline, method, result, and limitations.	P03

ID	Format	Title and execution idea	Destination
L13	Proof	**Reference conversation.** Offer a customer reference only after that customer's explicit agreement, with scope and availability confirmed.	P09
L14	Email	**Share the method, not a claim.** Ask a satisfied authorized customer whether they would review a draft factual pathway story.	P09
L15	Email	**A peer may have this problem.** After evidence and consent, offer a short forwardable note about the measured pathway and its limits.	P03
L16	Community	**Peer operator exchange.** With permission, convene an existing customer and one peer operator to discuss implementation lessons.	P09
L17	Community	**Hospital-association learning session.** Invite a permissioned customer to teach the pathway method; verify event and affiliation before promotion.	P10

Three highest-leverage pieces for this founder: T10, the one-page patient-pathway friction worksheet; M10, the parallel-evaluation readiness checklist; and B04, the synthetic one-minute demo preview. **This week, create T10.** It helps a leader articulate a real problem, gives a useful result before a meeting, and supplies the first 25-minute demo question. The most powerful result-in-advance asset is a **guided first-pathway map**: in a short founder conversation, the buyer leaves with the chosen handoff, who owns it, a baseline metric, and a go/no-go question for evaluation. That is a useful result even if they do not buy. A guided synthetic sandbox (P11) is a later option; it must not be confused with the founder's two-week tailored parallel evaluation or a live clinical deployment.

Part 4 - Funnel wiring diagram

A Georgian private-hospital CEO first encounters us when a founder post or permissioned peer discussion asks why one patient creates repeated work across departments. The CEO opens the patient-pathway worksheet (T10), names a recent handoff, and receives one useful follow-up email rather than a broad AI brochure. An option-comparison article (M08) and readiness checklist (M10) answer why a current-vendor fix, narrow tool, or new operating system might be right. A short synthetic demo preview (B04) leads to the `/request-demo` route (P05), which must record a real accepted request before any “received” message is shown. The founder runs a 25-minute call, maps the hospital's first pathway, and proposes an evaluation only if the buyer can name an owner and a readiness path. Clinical, finance, operations, and IT counterparts agree on baseline, access, effort, and success criteria; they compare the tailored path beside current work before any production decision. If a defined scope is approved, the customer reaches a buyer-accepted first pathway, reviews the measure and exceptions, and chooses whether to expand. If a meaningful result is measured and the customer consents, MediMind publishes an accurate case or arranges a reference, **and they become an advocate who brings in another private-hospital CEO or group operator.**

Part 5 - Build order

The calendar below describes **marketing work** within five hours total per week. Engineering for lead receipt, page instrumentation, and sandbox setup; the 25-minute sales meetings; customer evaluation; and clinical/IT delivery need separately planned time and owners. No paid promotion is assumed.

Phase 1 - Weeks 1-2: minimum viable funnel, three assets

Asset	Why first	It must achieve
P05 reliable pathway-demo request route	No funnel exists if a request appears successful but is not received.	Real delivery to a named owner, honest error/success behavior, and one logged qualified/held-demo trail. Engineering work is separate from marketing hours.
T10 one-page pathway worksheet with P01 download	Gives a relevant result before asking for time and sharpens the founder's discovery.	A CEO can name one handoff, its owner, and a measure without sharing patient data.
T04 synthetic duplicate-entry trail plus founder outreach note	Shows the repeated work visually and gives a reason to bring a real pathway to the founder.	A relevant executive can recognize the problem and choose a slot or reply for coordination.

Single MVF signal: a held qualified hospital demo sourced through a traceable contact path, with the first pathway and next decision recorded. A completed form alone is not a demo. Qualification requires a hospital organization, relevant authority or sponsor, a named recent problem, and an evaluation owner or a clear path to one.

Phase 2 - Weeks 3-6: seven conversion assets

“Expected conversion improvement” below means the **specific behavior the asset is designed to improve**, not an invented uplift percentage.

Asset	Stage fixed	Behavior to look for
M08 option-comparison article / P07	MOFU	More buyers can explain why a core evaluation is or is not warranted.
M10 readiness checklist / P06	MOFU	More demos end with named clinical/operations and IT counterparts.
B10 demo preparation card	BOFU	More booked calls arrive with a recent handoff and a useful example.
B11 evaluation decision sheet	BOFU	More qualified demos yield an explicit go/adjust/stop proposal.
B14 confirmation email	BOFU	Fewer bookings are missed; attendees know what to prepare.
B15 post-demo scope email	BOFU	Next actions, owners, and unresolved requirements are recorded rather than lost.
M12 documented-domain map / P10	MOFU	IT and clinical reviewers can separate documented scope from accepted deployment.

Readiness for Phase 3: a working lead route and a small traceable sequence of held qualified demos with follow-up ownership; preferably at least one readiness-approved evaluation before expanding content production. The 3-of-12 downstream scenario is a hypothesis to monitor, not a percentage threshold to declare a win after one or two calls.

Phase 3 - Months 2-3: growth layer

Asset or system	How it compounds	Manual effort it can replace
Reusable patient-pathway content set (T07/M07/B07)	Each real buyer question improves an evergreen answer and a better demo.	Repeating the same explanation in each first call.
Permissioned proof library P10, later verified case P03	A measured, approved story answers a recurring risk question for later committees.	Unstructured founder claims and ad hoc evidence requests.
Clean public-site source and event tracking	Shows which named distribution paths produce accepted requests and held qualified demos.	Guessing from page views or button clicks.
Opt-in email follow-up (T14/M14/B14)	Gives useful next steps at the right buying stage.	Remembering every nurture step manually.
Customer activation measurement and a permitted reference path	The next buyer can learn from an actual accepted pathway, with its limits.	Rebuilding trust entirely from founder description.

Compounding signal: new qualified hospital conversations begin from a reusable asset or permissioned peer introduction without a fresh one-off founder explanation. Count them by source and held demo; do not infer “organic growth” from impressions alone.

Part 6 - Weekly funnel scorecard

Use a Monday review of **counts first**, with rates only when denominators exist. A green/amber/red threshold is an internal action rule for the 90-day scenario, not a validated industry benchmark. The library's 2-5% homepage, 3.8% demo-page, 25-45% webinar, 15-30% trial-paid, >60% activation, 20-30% email-open, and 46.9% month-one-retention figures are in Part 2 as unverified generic context. They should not override low-volume account evidence or define a hospital service standard. Track source and stage with no patient data or personal information in analytics events.

Stage	Metric and how to measure	Green - on track	Amber - needs attention	Red - act now	Likely red cause	First fix
TOFU	Relevant named-account or permissioned-peer conversations begun this week; record source and problem in a secure lead log	2+ relevant conversations	1	0 for two weeks	Message or distribution misses the buyer	Ask known contacts which handoff matters; test T10 with one leader
MOFU	Buyers who complete a pathway map or option discussion with a named owner	1+ this week	Interest without a named pathway	0 from four relevant conversations	Generic content or unclear first result	Use T10 live and ask for the last incident
BOFU	Held qualified hospital demos, not booked calls; verify attendance, role, problem, owner	1+ this week, accumulating toward 12 in 90 days	1 booked but not held, or held without qualification	0 for two weeks despite relevant conversations, or request route fails	Broken route, weak call promise, or no follow-up	Verify submission and contact owner; send B14 preparation note
BOFU → evaluation	Readiness-approved evaluations / held qualified demos, with named IT and clinical/operations owners	1 approved as the series matures; scenario is 3 of 12	Proposal pending missing prerequisite	Repeated demos with no explicit decision	Evaluation feels too risky or vague	Use M10 and B11 to expose missing readiness and scope
AFU	First accepted pathway milestone and agreed metric/exception log for each active customer	Milestone met or openly re-scoped with customer approval	Behind schedule with named remedy	No owner, baseline, or clinical/IT sign-off	Delivery or data readiness gap	Joint issue review; narrow the pathway safely

Stage	Metric and how to measure	Green - on track	Amber - needs attention	Red - act now	Likely red cause	First fix
LOFU	Permissioned case/reference or qualified introduction from an accepted customer	1 permissioned action after verified value exists	Eligible story identified, approval pending	An unapproved name/outcome is published	Proof governance failure	Remove or correct claim and seek explicit permission
Overall	**Cumulative held qualified hospital demos in rolling 90 days**; one record per buying organization meeting	Pace at or above 12/90-day scenario	Below pace but improving with recorded causes	No held qualified demo for two consecutive weeks once route/distribution is live	Earlier stage leak or qualification mismatch	Trace the last four accounts from first touch to outcome

If a denominator is zero, show **N/A**, not 0%. The stretch downstream markers are three readiness-approved evaluations and one new paying buying organization by day 90; report them as counts, not a forecast. The anchor account is a separate evidence and expansion line. A single week of red can be noise, but a broken request route is actionable immediately.

Sources and handoff

Part 1 inputs: business/market, product journey, BFG matrix, top UVPs, problem stack, and synthetic avatar. The research addendum records the demo-form source inspection. The B11 benchmark list is reproduced from the assigned main library, whose figures do not supply current local baselines. Prompt 2 should prioritize the pathway hook, controlled evaluation, and clearer patient story within the same five-hour weekly marketing budget.

Source: Original files/Complete B11 HTML library/Editable text/p2-02-angles.md. Historical wording preserved; not a new approval or factual revalidation.

Part 2, Prompt 2 - Define Your Marketing Angles

> **Status and required inputs (23 September 2026).** Prompt 1's funnel is a founder-led hybrid, with a 25-minute pathway demo and a proposed measured evaluation as the principal transition from interest to a decision. The synthetic avatar is the accountable Georgian private-hospital operator; Prompt 8's lead question is “Why does one patient create so much extra work?” The top three working UVPs are: find and test repeat work, compare beside the current system before switching, and give teams a clearer patient story. The BFG matrix documents 25 hospital domains and two cross-cutting capabilities but no independently measured outcome. Competition includes the incumbent, narrow tools, broad vendors, and workarounds; the small public-review corpus is mostly non-Georgian staff and cannot supply a CEO quote. No target-buyer interview language has been collected. This document uses **proposed buyer language**, not an attributed customer's exact words.

Four current-situation answers. The leading fear to test is that a core switch disrupts care or revenue and leaves the leader responsible for an expensive mistake. What the buyer most wants to be able to say is, “Our team tested one important pathway, saw the real work and cost, and made a careful decision.” This is synthetic language, not a quotation. A live site and founder-authored material exist, but no verified marketing traffic, lead, or channel-performance series was supplied; the demo form's source-observed simulated submission needs a live check and repair before promotion. The founder did not provide brands they admire. As editorial references only, borrow HubSpot's useful diagnostic asset, Salesforce's committee-aware enterprise education, and Monday.com's easily understood visual demonstration; none establishes a suitable acquisition benchmark for Georgian hospitals.

Part 1 - Master marketing angle

1. Hook

Why does one patient create so much extra work across your hospital?

The question gives a busy executive something they can test against a recent case. It points to one patient journey, not a claim that every hospital is broken. A reader who does not recognize the problem can still learn whether it exists by mapping one pathway; a reader whose problem is staffing or payer policy should not be pushed toward core replacement.

2. Differentiator

The template in the library demands “the only.” There is no global comparison that supports exclusive ownership of a hospital operating system, FHIR R4, AI, or parallel evaluation. The defensible one-sentence version keeps its structure without fabricating exclusivity:

Unlike adding another point solution to a repeated handoff, MediMind is a hospital operating system that lets a private hospital map and compare a tailored patient pathway across clinical and business work before deciding on a wider switch.

The difference to demonstrate is the combination of shared scope, site-specific path, founder development relationship, and controlled comparison. It is a testable positioning claim, not a declaration that no competitor could offer any of those elements.

3. Primary psychological trigger

Commitment and consistency is the most useful initial trigger: ask the leader to name a recurring problem they already agree should be fixed, then make the next action a small, consistent step-map one pathway. This avoids asking them to endorse a wholesale replacement before they have evidence. Loss aversion is present in the fear of failed transition, but the campaign should answer it with control and proof so the desired emotion becomes earned confidence from a hospital-owned comparison.

Part 2 - Angle selection and prioritization

“Yes” means suitable for the strategy, not necessarily an asset to produce this month. Priority 1-3 are the **only active angles in the first 30 days**; the rest can be tested after the route and core assets work. “No” means hold until the stated missing evidence, channel access, or resources exist.

Angle	Use it - yes or no	Why it fits or does not fit	Funnel stage	Priority rank	Specific message to lead with
Encourage audience to sign up for a demo	Yes	The buying decision requires a human, pathway-specific conversation.	BOFU	3	“Bring one handoff; leave with a first pathway map.”
Target audience based on language and region	Yes	Start with Georgian private-hospital decision-makers; validate exact language preferences.	TOFU-MOFU	5	“Let's map the work as your Georgian hospital actually runs it.”

Angle	Use it - yes or no	Why it fits or does not fit	Funnel stage	Priority rank	Specific message to lead with
Book sales calls	Yes	Named executive introductions may outperform anonymous traffic at this stage.	BOFU	6	“Could we spend 25 minutes on the last handoff your team had to repair?”
Offer rewards to increase product usage	No	Clinical software use should follow patient/work value and governance, not incentives that distort behavior.	AFU	-	No reward promise.
Try a new low-risk CTA offer	**Yes**	A pathway map gives a useful result before a purchasing decision.	TOFU-BOFU	**1**	“Map my hospital's first improvement.”
A sneak peek at pricing options	Yes, after written terms	Current-software price match is meaningful; total cost and eligibility must be clarified.	BOFU	7	“Early partners can compare the matched software fee alongside the full evaluation and transition costs.”
Promote product launches and new features	No now	A feature announcement is weaker than a buyer pathway and needs verified readiness.	MOFU-AFU	-	Publish only accepted, relevant changes later.
Highlight a use case offer	**Yes**	One patient pathway makes broad product scope concrete.	TOFU-MOFU	**2**	“Follow one test from order to result to bill; count where the work repeats.”
An inside look at your product dashboard	Yes, carefully	A synthetic walkthrough can show mechanics; a dashboard alone does not solve source-data quality.	MOFU	4	“See the shared patient and task context in one agreed example.”

Angle	Use it - yes or no	Why it fits or does not fit	Funnel stage	Priority rank	Specific message to lead with
Display testimonials	No now	No verified, permissioned MediMind customer quote is supplied.	MOFU-LOFU	-	Reserve a slot only after a real approved quote.
Use video ads to demonstrate benefits	No paid ads now	Zero paid budget and no local creative baseline; an organic founder video supports the use-case angle.	TOFU-BOFU	-	Film the pathway organically, then evaluate paid distribution later.
Share case studies	No public outcome case now	Founder-authored implementation context exists, not an independent measured customer outcome.	MOFU-LOFU	-	Publish a buyer-approved measured pathway case when available.
Promote blog posts	Yes	Searchable decision guides can support a long, committee-led purchase.	TOFU-MOFU	8	“How do you compare a current-vendor fix with a parallel hospital-system evaluation?”
Promote exclusive research	No now	No original hospital-buyer dataset or permissioned workflow study exists.	TOFU	-	Publish methods and source-labeled findings, not invented exclusivity.
Promote webinars	No now	A 45-minute practical session is proposed but unscheduled and requires an audience.	MOFU	-	Hold until registration route and speaker/agenda are real.
Promote online and offline events	No now	No specific suitable event, slot, or partner is confirmed.	TOFU-MOFU	-	Verify an association opportunity before promotion.

Priority 3 is the demo invitation framed as a controlled parallel comparison: “See your pathway beside today's work before deciding.” The first three active angles therefore form one story: **map the problem → show a patient-pathway use case → compare safely**.

Part 3 - Angle-to-channel mapping

The table covers every selected “yes” angle. It specifies an initial asset and a countable success event. Channel use is proposed until the buyer's actual media and referral habits are observed. There is no paid campaign in the first month.

Selected angle and channel	First content/format	CTA	What success looks like
Low-risk CTA / founder LinkedIn	T01: post with a synthetic seven-handoff sketch and invitation to name the local version	Map my hospital's first improvement (P01)	A relevant operator requests or discusses a pathway map.
Low-risk CTA / existing professional introductions	One-to-one note offering T10 worksheet, only to a legitimate contact	Use the pathway worksheet (P01)	Recipient returns a real handoff or introduces its owner.
Use-case offer / founder phone video	T04: duplicate-entry trail using synthetic information	Map my hospital's first improvement (P01)	A relevant leader watches enough to ask about their own pathway; count replies, not views alone.
Use-case offer / website article	T07: how to map a patient handoff	Download the worksheet (P01)	Qualified downloads or a pathway question from a hospital.
Controlled comparison within low-risk CTA / founder follow-up	M10 readiness checklist after an interested response	Discuss an evaluation (P06)	Named clinical/operations and IT counterparts plus agreed next step.
Demo / existing `/request-demo` page	B04 synthetic demo preview, after real submission receipt is verified	Request a pathway demo (P05)	Held qualified demo, not a success-screen view.
Language/region / Georgian professional connections	Localized T01 with reviewed terminology, not automatic translation	Map my hospital's first improvement (P01)	A Georgian hospital operator recognizes and corrects the scenario.
Direct sales call / permissioned peer introduction	Short founder note about a recent handoff, not a generic pitch	Request a pathway demo (P05)	Relevant executive accepts a 25-minute conversation.

Selected angle and channel	First content/format	CTA	What success looks like
Pricing peek / founder proposal	One-page offer comparison including matched software fee and visible total-cost questions	Discuss the early-partner offer (P06)	Buyer understands what is and is not yet priced and remains in evaluation.
Inside look / synthetic screen recording	M06 result-to-review task flow, with no patient data or implied clinical autonomy	See the operating system (P02)	Clinical/IT evaluators ask about their own equivalent workflow.
Blog promotion / founder LinkedIn and targeted follow-up	M08 decision guide comparing incumbent fix, point solution, and new system	Compare the approaches (P07)	An evaluator can name why a narrow fix is or is not sufficient.

The catalog's proposed community entries identify places to test, not pre-approved posting rights. The Georgian Healthcare Association has a public provider network; whether the actual target buyer reads a channel or welcomes a post requires verification and permission.

Part 4 - Ten-hook library

These are original lines in the **synthetic avatar's working language**. They are not recorded Georgian buyer quotations. For a Level-4 market, each should lead toward a mechanism or proof step rather than stopping at a vague “better care” claim.

Approach	Hook	Best first use
1. Pain	“Your team cared for the patient. Why did they also have to carry the information between departments?”	LinkedIn opener
2. Future	“Imagine a hospital where the next team starts with the patient's story instead of rebuilding it.”	Product explainer
3. Mechanism	“Pick one patient pathway; compare its handoffs beside the system you use today.”	Demo page
4. Number	“One patient. One pathway. A 25-minute map of where your hospital repeats the work.”	Founder video thumbnail; 25 minutes is the proposed demo length

Approach	Hook	Best first use
5. Question	“Why does one patient create so much extra work across your hospital?”	Master campaign
6. Story	“The result was ready. Three teams still had to ask where it went.”	Synthetic scenario post, labeled illustrative
7. Provocation	“A faster screen can still leave the hospital doing the same work twice.”	Comparison article
8. Social proof	“A signed network agreement shows a hospital willing to explore this path; your own team's comparison is the proof you need.”	Warm follow-up only, after founder-approved reference wording; no customer quote or outcome implied
9. Loss	“Every repeated handoff uses staff attention you could put toward the next patient.”	Email subject, as a plausible cost to measure, not quantified loss
10. Identity	“Lead the hospital that checks the pathway before it changes the whole system.”	Executive invitation

For #8, the agreement is founder-reported and the Medplum case study is founder-authored. Verify external naming and permission before publication. If that cannot be done, omit the social-proof hook rather than inventing a customer success story.

Part 5 - Message by funnel stage

Stage	Core one-sentence message	Tone	Never say here	Three actual example messages
TOFU	“A single patient can reveal where your hospital's teams repeat work; map one journey to see whether the problem is worth fixing.”	Observant, useful, curious	“Your hospital loses a known amount of money” without data	LinkedIn: “Where does one result turn into three phone calls?” · Email subject: “The handoff your team repeats” · Page: “Map one patient pathway.”

Stage	Core one-sentence message	Tone	Never say here	Three actual example messages
MOFU	“Compare a current-vendor fix, a point solution, and a shared hospital path against the same work, cost, and risk.”	Fair, explanatory, specific	“Every other system is disconnected”	LinkedIn: “Would a new dashboard fix the handoff-or just display it?” · Email subject: “Fix the screen or the pathway?” · Page: “Compare the three practical options.”
BOFU	“Bring one recent handoff to a founder-led demo and decide whether a scoped parallel comparison is worth your team's time.”	Calm, direct, transparent	“Replace your hospital in two weeks with no risk”	LinkedIn: “Your first demo should end with a pathway, not a feature list.” · Email subject: “What to bring to our 25 minutes” · Page: “See your first pathway before you switch.”
AFU	“Use the first agreed pathway, measure staff work and exceptions, and approve the next step only when your owners accept it.”	Supportive, accountable	“Launch equals success”	Customer note: “What did staff still have to repeat?” · Email subject: “Your first pathway review” · Activation page: “Measure the work; record the exceptions.”
LOFU	“A verified, permissioned pathway story can help another hospital judge whether the approach fits its own work.”	Factual, generous, respectful	“Our customer guarantees your result”	LinkedIn: “What this pathway result actually measured” · Email subject: “Would you review a factual story?” · Proof page: “Scope, baseline, result, and limits.”

Part 6 - 30-day angle execution plan

Five hours below is the **total marketing allocation each week** across this prompt, the funnel plan, and campaign work. The binding weekly clock reserves **85 minutes** for routine work: five five-minute weekday checks (25 minutes total), a 30-minute Monday scorecard review, and a 30-minute amortized monthly cleanup reserve. That leaves **215 minutes (3 hours 35 minutes)** for the creative and outreach actions in each row. The founder's 25-minute sales meetings are additional sales time; reliable form

submission and analytics setup require engineering time. Do not launch traffic to a simulated request route. Week 1 contains a route verification gate, while drafting and direct discovery can continue independently.

Week and leading angle	Create and publish within five marketing hours	Measure	If the previous week misses its signal
1 (days 1-7): low-risk pathway CTA	**215 min active:** 75 min build T10 one-page worksheet; 40 min write/publish T01 post; 60 min send five individually relevant, permissioned notes; 40 min respond and ask for the last handoff. Publish only when P01 works. **85 min routine** as above.	Relevant replies, completed maps, and accepted route submissions-not impressions.	If no relevant reply, ask two known operators which wording describes a real handoff; revise T01 rather than making more formats.
2 (days 8-14): use-case offer	**215 min active:** 85 min film T04 phone video with a synthetic pathway; 40 min caption/edit; 10 min publish with T10 link; 80 min follow up with interested leaders. **85 min routine.**	A hospital leader names a pathway or asks for a demo.	If week 1 had no signal, use a simpler recent incident question, test it in direct conversation, and publish only the version a buyer recognizes.
3 (days 15-21): controlled comparison	**215 min active:** 85 min make M10 readiness checklist; 40 min write M02 post; 30 min publish/share with prior responders; 60 min answer readiness questions. **85 min routine.**	Named clinical/operations and IT counterparts, evaluation interest, held qualified demo.	If week 2 gets attention but no next step, show what the buyer leaves with after 25 minutes and check the actual request path.
4 (days 22-28): combine and learn	**215 min active:** 85 min create B04 synthetic demo preview; 15 min publish with verified P05 link; 75 min personally follow up; 40 min inspect the last four account journeys and choose one correction. **85 min routine.**	Cumulative held qualified demos versus the 12/90-day scenario, plus evaluation readiness.	If week 3 yields demos without evaluations, ask exactly which prerequisite is missing; change the offer or qualification, not the volume of content.

Days 29-30 use the routine checks and next Monday review from the following weekly allocation; they add no fifth creative block to the 30-day plan. There is no paid budget and no fabricated webinar or event date. The first month succeeds when the system can receive a real inquiry, the founder can hold a qualified pathway demo, and a hospital can clearly accept or decline an evaluation. A small sample cannot validate one hook over another; use buyer explanation and the next step they take.

Sources and handoff to Prompt 3A

Working inputs: funnel blueprint, synthetic avatar, problem stack, UVPs, BFG, competition, and review intelligence. The live-route caveat comes from the research addendum. The campaign should film the question hook and one synthetic handoff, then invite a founder-led pathway map.

Source: Original files/Complete B11 HTML library/Editable text/p2-03a-campaign-script.md. Historical wording preserved; not a new approval or factual revalidation.

Part 2, Prompt 3A - Build Your Campaign Concept & Video Script

> **Status and required inputs (23 September 2026).** The first buyer is the synthetic accountable Georgian private-hospital operator from Prompt 9. Prompt 8's working surface problem is repeat work along one patient pathway; the hidden problem is fear that a replacement will make care and operations worse. The strongest working UVP is to **find and test** that repeat work, at market-sophistication Level 4 (“New Mechanism”). Prompt 2's top angle is a low-risk, useful first action: map the hospital's own pathway, then compare a tailored version beside current work before switching. The campaign must drive a **held, qualified founder-led pathway demo**, initiated through a verified request route or direct founder reply. Production reality is **one founder, a phone camera, paper cards or a simple synthetic screen recording, natural light, and no paid production budget**. The most painful moment to dramatize is staff having to ask or re-enter what another department already knows while a patient waits for the next step. There is no traceable buyer quote explaining why a customer chose MediMind; no line below is attributed to a customer. The founder reports a signed GeoHospitals agreement; no customer outcome or live-site count is implied.

Part 1 - Campaign concept

Premise: This campaign is for a private-hospital leader who sees one patient create repeated work across departments, and it shows them that a safer first step is to map and compare one pathway before considering a wider software change.

Pattern interrupt - the question they may already be asking. Open the main asset with the founder holding three simple cards labeled “order,” “result,” and “bill,” then moving the same blank patient card between them. Exact first five seconds: **“Why does one patient create so much extra work?”** The question is original proposed copy, not a buyer quotation. The cards make the problem visible without using patient information or pretending a fabricated case is real. The two A/B variations deliberately test different openings against this main question-led version.

Campaign CTA (five words): “Map my hospital's first improvement.” The promised outcome is a concrete pathway map and next measurement question, not an instant hospital-wide transformation. The first conversation is recommended as 25 minutes, led by the founder. A tailored parallel evaluation in two weeks is the founder's readiness-dependent offer after scope and approval, not what every viewer automatically receives.

Three-act path: Problem-one patient story gets recreated across roles. Mechanism-the team follows one shared path from task to task on a FHIR R4 foundation and can compare it beside current work. Relief-the leader can inspect what changed and decide with clinical and IT colleagues whether a broader move is justified. AI may assist with defined tasks for human review, but the story does not need an autonomous clinical claim to be compelling.

Part 2 - Video script: founder on camera, phone format

Target running time: 78-85 seconds at an ordinary speaking pace. Film vertically with a clean desk and daylight; crop a horizontal version from a wider phone frame if needed. The founder may speak as a physician and CEO, but should sound like a colleague mapping work, not a narrator advertising a miracle. Cards and screens contain invented names or no names at all. Every onscreen product capture must come from a synthetic/demo environment, with no actual patient, staff, payer, or contract data.

Time / act	Exact founder words	What the camera shows	On-screen text
0-5s / opening	**"Why does one patient create so much extra work?"**	Close view of one blank patient card passing from a paper "order" card to "result" to "bill." Founder looks up at camera on "work."	"ONE PATIENT. TOO MUCH REPEAT WORK."
5-17s / problem	"A doctor orders a test. Someone calls to check whether it happened. Someone else finds the result. Finance still has to work out what to bill."	Founder places a small question mark between each card. No product visible.	"Order → chase → result → reconcile"
17-27s / hidden problem	"Each person is doing their job. But the journey between them can become another job for your staff."	Stack blank paper slips under the same patient card. Pause so the image lands.	"The handoff becomes work."
27-41s / mechanism	"So we start with one real pathway at your hospital. We map who needs to act, what information they need, and where the same work is repeated."	Whiteboard/paper diagram with three role labels and arrows; founder circles one repeated step. If a screenshot is used, switch only to synthetic demo data.	"Map one pathway."
41-57s / mechanism	"MediMind brings the clinical and business steps into one hospital operating system. Your team can compare that path beside the way it works today."	Show two paper lanes labeled "today" and "proposed"; reveal one shared task and patient context on a synthetic screen if available. Do not show a flashy dashboard unrelated to the example.	"Compare beside today's work."
57-69s / relief	"Then we measure the handoff, the staff effort, and the exceptions. Your clinicians and IT team help decide what is ready-and whether a wider change makes sense."	Founder places a simple three-column card: "work," "exceptions," "decision." A handwritten checkbox remains empty.	"Measure. Review. Decide."
69-72s / transition	"Bring me one handoff your hospital keeps repeating."	Direct to camera; keep the circled pathway visible.	"ONE HANDOFF"

Time / act	Exact founder words	What the camera shows	On-screen text
72-82s / closing ten seconds	**“In 25 minutes, we'll map the first improvement worth testing. Map my hospital's first improvement.”**	Hold a clean CTA card with the verified public route or a direct founder contact path.	“MAP MY HOSPITAL'S FIRST IMPROVEMENT”

The spoken script is about 155 words, leaving time for visual pauses. A fast read would undercut the tone; if the founder reaches the CTA too early, let the two-lane comparison remain on screen rather than adding claims. If the full cut runs past 90 seconds, cut the line about billing in the first act before accelerating speech.

Director's three instructions:

1. Keep the opening quiet and slightly puzzled; speak directly to one hospital operator, not to a crowd. Let the paper-card movement supply the interruption.
2. Make each cut coincide with one handoff. Use large, readable labels and high contrast; mobile viewers should understand the visual with sound off and captions on.
3. End on the founder's face and one CTA. Do not place an unverified customer logo, “25 hospitals live,” a performance percentage, or an artificial deadline on screen.

15-second organic cut-down (same question-led campaign):

Time	Exact words	Visual
0-4s	“Why does one patient create so much extra work?”	Same three cards and blank patient card.
4-9s	“Pick one handoff. We'll map what repeats.”	Circle one repeated arrow on paper.
9-15s	“Compare beside today's work. Map my hospital's first improvement.”	Two lanes, then CTA card.

The cut-down invites the same action and does not introduce an unsupported result or a second destination.

Part 3 - Two shorter A/B script variations

These are **ready-to-film creative tests**, not instructions to buy ads now. They can be posted organically first; paid LinkedIn/YouTube distribution is conditional on a real budget, a functioning lead route, and evidence that the message attracts qualified hospital operators. Each follows problem → mechanism → relief, but changes the opening so the team can learn which explanation earns a useful reply.

Variation A - visual contrast interrupt, 22-25 seconds

Audience: cold relevant hospital leaders; **trigger:** commitment and consistency-the leader can check one visible example without endorsing a replacement. The contrast appears in the opening split screen.

Time	Exact script	Visual / act
0-5s	“Same patient. Two very different ways to handle the handoff.”	Left: three separate blank cards and arrows; right: one connected path. No MediMind logo yet. **Problem.**
5-12s	“Here, staff ask and copy. Here, the next task stays with the patient context.”	Founder points to each lane; label the right lane “proposed,” not “proven result.” **Mechanism.**
12-23s	“Which path works better for your hospital? Map one handoff, compare both, then decide. Map my hospital's first improvement.”	Founder circles a single handoff, then CTA card. **Relief and CTA.**

This variant tests whether seeing a concrete alternative before hearing the question improves comprehension. It does not show actual patient data or assert that MediMind has already reduced a real customer's work.

Variation B - honest social-proof interrupt, 25-29 seconds

Audience: warm retargeting or existing leads; **trigger:** social proof that lowers the perceived risk of engaging, immediately bounded by an invitation to verify the buyer's own case. Social proof is a founder-reported procurement fact, **not** a customer testimonial or measured outcome. Before public release, confirm the approved wording about the signed network agreement and avoid using the customer's name or logo without permission.

Time	Exact script	Visual / act
0-5s	“A Georgian hospital network signed with MediMind.”	Founder on camera; text “Signed network agreement - founder-reported.” No logo, facility count, or invented result. **Social-proof opening.**
5-15s	“That is a reason to look, not proof for your team. Pick one handoff your staff repeats.”	Founder lays out the same blank “order → result → bill” cards. **Problem.**
15-28s	“We'll map a connected path, set a measure, and let your clinical and IT team decide. Map my hospital's first improvement.”	Two-lane paper diagram and CTA card. **Mechanism and relief.**

If the agreement cannot be referenced publicly with approved wording, **hold this variation**. Replace the social-proof opening only with a source-checked, customer-approved implementation fact; never substitute a fabricated quote, outcome, or “live at 25 hospitals” badge. The main and Variation A remain filmable without this variant.

Production and destination checklist

Keep this as a short execution checklist, not a new approval workflow: phone at eye level, natural light, quiet room, paper labels in large type, clean synthetic screen capture if used, accurate captions, one CTA, and no patient information. The proposed destination is the existing `/request-demo` route, which currently says 30 minutes in source; align that page with the recommended 25-minute offer if the strategy is adopted. Source inspection found a

simulated form submission, so verify deployed behavior and implement actual receipt before linking a public campaign to it. Track accepted request, qualified booking, and held demo as separate events. The video itself does not claim a scheduled webinar, self-serve clinical trial, response-time SLA, or clinical outcome.

Sources and handoff

The avatar, problem stack, UVPs, angle strategy, and funnel blueprint establish the narrative. The research addendum records the source-code status of `/request-demo`; Medplum's founder-authored case study is background on the product and agreement, not independent outcome proof. Prompt 3B can distribute the main question-led film first, use Variation A as a comprehension test, and hold Variation B until reference wording is approved. All campaign routes beyond the existing demo path are proposals.

Source: Original files/Complete B11 HTML library/Editable text/p2-03b-campaign-distribution.md. Historical wording preserved; not a new approval or factual revalidation.

Part 2, Prompt 3B - Build Your Campaign Distribution

> **Status and inputs, 23 September 2026.** Complete copy package for a proposed, unlaunched campaign. Prompt 3A fixes the concept **"One patient. Too much repeat work."**, the opening question **"Why does one patient create so much extra work?"**, the paper-card order → result → bill visual, and the CTA **"Map my hospital's first improvement."** The action offered is a recommended 25-minute founder-led pathway demo that ends in a proposed measured evaluation, not an instant replacement. The funnel moves relevant hospital leaders from problem awareness through consideration to a held qualified demo. The avatar is synthetic; no buyer-language interview or customer outcome quote is available. The first media budget is zero; paid ads below are ready-to-test copy, not a paid launch instruction. Existing audience size and opt-in email list are unknown. The five-hour weekly marketing ceiling is shared with distribution and reporting; engineering work and live sales meetings are additional.

This campaign drives traffic to: the existing `/request-demo` page. The page exists, but source inspection found a simulated submission handler and current "30 minutes" text. A public campaign needs verified request receipt, a named owner, truthful success/error states, and aligned 25-minute copy before this route can be treated as a working lead destination. If a verified direct founder reply path is arranged first, it can serve as a temporary campaign response route; no response-time promise is made. All other proposed pages below remain drafts until built and tested.

Part 1 - Ad copy

The three LinkedIn assets address different awareness states. Each body is under 150 words, has one emotion, and uses the exact campaign CTA. Paid audience definitions and retargeting require consent, working measurement, and an approved budget; the same copy can be tested in founder posts or permissioned introductions first.

LinkedIn sponsored content - three versions

A. Cold / TOFU - curiosity.

- **Hook, first two lines:**

Why does one patient create so much extra work?
Follow the handoff, not just the chart.

- **Body:** “An order becomes a result. The result needs a decision. The decision reaches billing. If staff have to ask, copy, or reconcile at every step, that journey creates another job alongside care. We draw one patient pathway with your team, show where work repeats, and agree what would be worth measuring before anyone discusses a wider system change. Start with the pathway you already know is difficult.”
- **Primary copy CTA: Map my hospital's first improvement** → verified `/request-demo` route. **Native sponsored button:** use a platform-supported preset such as “Request demo” if available; verify the actual options in the account.
- **Visual:** Founder moves a blank patient card across three paper cards: order, result, bill. No patient data.
- **Psychological trigger:** Recognition of a familiar task, followed by a small, inspectable first step.

B. Warm / MOFU - cautious hope.

- **Hook, first two lines:**

  Why does one patient create so much extra work?
  What would a fair comparison look like?

- **Body:** “A focused tool may fix one step. Your current supplier may be able to change another. If the same handoff keeps crossing departments, compare the whole pathway. In a 25-minute founder-led walkthrough, we map the current work, show a connected proposed path using synthetic data, and name the staff and IT conditions for any parallel evaluation. You keep the decision. The first result is a clear question to test, not a promise that the hospital changes overnight.”
- **Primary copy CTA: Map my hospital's first improvement** → `/request-demo`. **Native sponsored button:** supported “Request demo” preset, if available.
- **Visual:** Two paper lanes labeled “today” and “proposed,” with one repeated arrow circled.
- **Psychological trigger:** Control over change and transparency about the test.

C. BOFU visitor who has not requested a demo - confidence.

- **Hook, first two lines:**

  One patient. Too much repeat work.
  Bring us the handoff that matters most.

- **Body:** “You may already know which workflow makes your team call another department, re-enter a result, or reconcile a bill. Bring that example to a 25-minute pathway demo. We will leave with a map of the current steps, one candidate improvement, and the first measure your clinical, operations, and IT colleagues could review. If the current supplier can solve it with less disruption, that belongs in the comparison too.”
- **Primary copy CTA: Map my hospital's first improvement** → `/request-demo`. **Native sponsored button:** supported “Request demo” preset, if available.
- **Visual:** Founder holds the three-column card from the video: “work / exceptions / decision.”
- **Psychological trigger:** A defensible next decision; no implied tracking of an individual's visit in the copy.

YouTube six-second bumper ads - three versions

Read each line at a natural, quick pace and cut at six seconds; caption every spoken word. These are scripts, not a recommendation to buy YouTube media with a zero budget.

Version / one emotion	Exact spoken words	On screen	Text overlay and single CTA
1. Puzzlement	“One patient. Too much repeat work. Map your first improvement.”	Blank patient card moves across order, result, bill cards.	“ONE PATIENT. TOO MUCH REPEAT WORK.” then “Map my hospital's first improvement.”
2. Relief	“Three handoffs. One patient. Map your first improvement.”	Founder circles a repeated arrow on the paper pathway.	“MAP ONE PATHWAY” then “Map my hospital's first improvement.”
3. Confidence	“Before switching, compare one path. Map your first improvement.”	Side-by-side lanes labeled “today” and “proposed.”	“COMPARE BEFORE A WIDER CHANGE” then “Map my hospital's first improvement.”

The longer on-screen CTA can remain on the final frame after the short spoken line. No bumper asserts a real customer result, a three-day production release, or an autonomous clinical action.

Retargeting ad - one lower-friction version

Channel: LinkedIn if a consented audience and enough eligible traffic ever exist; otherwise reuse as a founder post or appropriate follow-up. **Copy:** “A new hospital system is a serious decision. Start with a pencil, not a cutover plan. Draw one patient journey: who gets the order, who sees the result, who has to ask again, and what finance must reconcile. Our free one-page pathway worksheet helps you spot the first question worth testing. If it looks like a core-system problem, we can map the next step

together." **CTA: Use the patient-pathway worksheet** → proposed P01 page, only after it is built and its delivery works. **Visual:** a simple blank worksheet and three paper cards. This addresses the risk-of-change objection without saying the reader visited the website or claiming the worksheet proves MediMind wins.

Part 2 - Four-email campaign sequence

These are complete **unsent** drafts for an opted-in or otherwise permissioned audience. Source-list size, email service, and opened/clicked events are unverified. A message should only be sent when its segment and delivery are real. Subject lines are eight words or fewer; preview text is 40 characters or fewer. The product is absent from Email 2. The third email uses a *competitor customer's public review* and labels it as such; it is not MediMind social proof.

Email 1 - Campaign launch, day 1

Subject: Why does one patient create extra work?
Preview: Trace one handoff with us.
Opening line: Why does one patient create so much extra work?

Full body:

Hello [First name],

A patient can move from an order to a result to a bill while the information is still carried by calls, copied notes, and checks between teams. Each person may be doing the right thing. The handoff itself can become extra work.

We are starting with one practical question: which patient pathway does your team repeat most often? In a 25-minute founder-led walkthrough, we can draw that pathway, show a proposed connected version with synthetic data, and name one measure worth checking with your team. It is a first look, not a request to switch systems.

CTA link text: Map my hospital's first improvement

Email 2 - Value delivery, day 3, opened but did not click

Subject: Map the handoff before changing software
Preview: A five-minute pathway check.
Opening line: Here is a simple way to see where the work repeats.

Full body:

Hello [First name],

Take one recent patient journey and draw four boxes: the first request, the result, the next decision, and the financial close. Under each box, write who owns it and what information they need. Put a circle around every place someone calls, copies, waits, or checks an earlier step.

Then ask two questions: Which circle happens repeatedly? Which one has a cost or safety consequence you can actually observe? The answer may point to training, a supplier change, a narrow tool, or a wider workflow issue.

You can use the blank worksheet without sharing patient details.

Soft CTA link text: Use the pathway worksheet - proposed P01 route; replace the link only after it exists and works.

Email 3 - Process proof, day 6, opened but not converted

Subject: What a real reviewer valued
Preview: Breadth matters. So does fit.
Opening line: A hospital-software review gave us a useful counterexample.

Full body:

Hello [First name],

An ophthalmic technician described moving **to Epic** because the previous system did not provide the breadth and customization her team wanted. In her August 2026 public review, she reported easier access to notes across providers and useful tailoring after the switch. That is a competitor's success story, not a MediMind result.

It is also a good test for any new system: does it make the *whole* job easier for your staff, without taking away the parts they already value? Bring one pathway to our founder-led walkthrough. We will map the current work and the evidence needed for a fair comparison.

CTA link text: Map my hospital's first improvement

Email 4 - Truthful urgency, day 10; under 100 words

Subject: The handoff will repeat next month
Preview: Start with one measurable path.
Opening line: If the same pathway keeps creating extra work, waiting has a cost worth measuring.

Full body:

Hello [First name],

If staff repeat the same handoff next month, the calls and reconciliation will still need an owner. We cannot tell you the size of that cost without looking at your workflow. We can spend 25 minutes mapping one path and agreeing what your team should measure before any wider change.

CTA link text: Map my hospital's first improvement

No expiry date or limited place is claimed. If there is no permissioned email segment or working destination, these remain drafts.

Part 3 - Two-week social content calendar

All ten are complete LinkedIn post drafts. Times are **proposed tests in Asia/Tbilisi time**, not validated audience benchmarks; the library's Tuesday/Thursday morning rule is a generic starting point. Post only where the founder has a relevant audience and can respond. “Format” is executable with a phone, a blank card, or text. The day-8 public review is clearly a competitor story, not a MediMind outcome. The day-14 close uses process proof because a buyer-approved result is unavailable.

Day / format / test time	Opening hook	Full post copy	CTA
1 / text + paper photo / 09:00	**A patient does not create the extra work. The handoffs do.**	“A doctor orders. A result comes back. The next person still has to ask whether it arrived. Finance still needs to reconcile what happened. Each team can be doing its job while the journey between them becomes another job. Before buying another dashboard, draw one real pathway and mark every call, copied value, and wait. What would your team find?”	None.
2 / text + three-card sketch / 09:00	**Why does one patient create so much extra work?**	“Try following one patient from order to result to bill. Where does someone look up the same detail? Where does a call replace a visible handoff? Where does the bill depend on a note another team cannot see yet? This is a question for a hospital's own workflow, not a claim that every hospital has the same failure. The important number is not how many screens you own. It is how much work the next person has to recreate.”	None.

Day / format / test time	Opening hook	Full post copy	CTA
4 / sourced data card / 09:00	**Georgia counted 232 hospitals in 2025.**	“That is Geostat's national facility count, not 232 software buyers. Some facilities share a purchasing group. Each hospital has its own people, pathways, and transition limits. A useful software conversation starts with one actual handoff and its owner, not a national number on a market slide. What would you count first in your hospital?”	Question in post.
5 / campaign video / 09:00	**Why does one patient create so much extra work?**	“An order. A result. A bill. The work between those steps deserves a closer look. Our founder maps one pathway before asking anyone to consider a wider system change. [Watch the synthetic-data walkthrough.]”	“Map my hospital's first improvement” → verified demo route, only after request receipt works.
7 / paper diagram carousel / 09:00	**One shared path starts with one honest map.**	“Choose one patient journey. Mark each person who needs to act. Mark the information each person needs. Circle where the same fact is entered twice or someone calls to ask what happened. Then put a proposed path beside today's path and count the work on both sides, including the extra work of a parallel test. A new system is worth discussing only if the comparison holds up for your people. That is the method behind MediMind's first-pathway demo.”	None.

Day / format / test time	Opening hook	Full post copy	CTA
8 / text + source link / 09:00	**A competitor review that made us pause.**	“A technician wrote that her organization moved **to Epic** seeking a broader, more customizable record. After the switch, she valued access to notes across providers (Capterra, 18 August 2026). That is real praise for an established competitor. The lesson for us: a hospital must keep the useful parts of today's system while fixing the handoff it cannot live with. A fair evaluation asks staff to name both.”	None.
9 / text / 09:00	**Myth: a new interface fixes the handoff.**	“A cleaner screen can help. It cannot by itself resolve unclear ownership, missing data, a payer rule, or a staff process that does not match the screen. Even broad EHRs can have task friction; their staff reviews also praise shared records. The test is specific: follow a patient pathway, compare all steps, and include training and exception work. If the current supplier can solve it with less disruption, that should count.”	None.
11 / synthetic micro-story / 09:00	**Imagine a result arriving at the wrong moment.**	“This is a fictional example, with no patient data: A lab result is ready. The clinician needs it for the next decision. The coordinator calls to confirm it is there. Finance later checks which service was completed. In one pathway review, the team can ask whether the result, task, and billing record move together or whether people bridge the gaps. The first improvement is the one staff can inspect and measure.”	None.

Day / format / test time	Opening hook	Full post copy	CTA
12 / text + side-by-side sketch / 09:00	**The best reason to wait is safety.**	“A hospital cannot stop for a software experiment. A serious parallel evaluation therefore needs an agreed pathway, an owner, authorized data or synthetic data, clinical and IT review, a baseline, and a stop rule. MediMind's two-week tailored-evaluation target depends on readiness; production change needs separate sign-off. Ask any vendor for the work your team must do and the conditions for saying no.”	None.
14 / founder photo + process card / 09:00	**Here is what 25 minutes should produce.**	“Bring one handoff your hospital keeps repeating. We will draw the current path, sketch a connected option, and name the first measure and owners for a fair test. You will know what MediMind can show now, what needs more work, and which questions your clinical and IT team should ask before a wider change.”	**Map my hospital's first improvement** → verified demo route. No deadline.

The ten posts form a **library and recommended sequence**, not a mandate to create ten new media assets inside two five-hour weeks. Reuse the same paper-card photo/video and schedule text drafts; if time is tight, select the highest-fit posts in the Part 5 calendar. No social proof count or clinical outcome is fabricated to satisfy a posting cadence.

Part 4 - Campaign destination page: precise edits to existing `/request-demo`

Source inspected: RequestDemo.tsx. The page is not a verified lead-receipt path. These are **copy and behavior instructions**, not implemented site changes. They must be coordinated with the full funnel-page copy and the product owner before publication. The source library's claimed conversion lifts for shorter forms, proof, grade-level text, or video are not independently established here; early hospital traffic will be too small for precise rate claims. Count valid received requests and held qualified demos first.

Page location	Current source copy / behavior	New copy or required change	Why
Eyebrow and headline	“Enterprise Demo Request”; “See MediMind on your hospital's workflows”	“FOR GEORGIAN HOSPITAL LEADERS”; **“Map your first patient-pathway improvement”**	Match the video promise and identify the right buyer.
Subheadline	“30 minutes. One login, one chart, one patient - live. We'll tailor the walkthrough to your hospital.”	“In 25 minutes with our founder, trace one recurring handoff and choose what your team should measure before a wider change.”	Align duration and deliverable; avoid “live” implying a customer environment.
Above form	Three-step progress and form starts with “Personal Information”	“Bring one handoff your team repeats. No patient details needed.” Then one short form.	Explain the task and lower the first-step burden.
Required fields	First/last name, email, company name and size, industry, job title, facility type, provider count; phone optional	Required: name, work email, hospital/group name. Optional: “Which patient pathway creates repeat work?” No patient identifiers.	Ask only what is needed to route a hospital conversation.
Submit button	“Request Demo”	**“Map my hospital's first improvement”**	Exact campaign CTA.
Success state	“We've received your demo request… contact you within 24 hours”; repeated 24-hour promise	Only after real receipt: “Your request was sent. Our team will review it and contact you to arrange the 25-minute pathway demo.” If send fails: “We could not send your request. Please try again or use the verified contact option.”	Current timer/console log is not receipt; no confirmed response-time SLA.
Trust badges	“Live in 25 hospitals”; government startup badge	Remove the live-site claim until accepted scope and permission are checked. Keep startup status only after current documentation and approved wording are verified. Use “Founder-led review of one pathway” as process reassurance.	Contracted network is not proof every facility is live; trust should rest on accurate evidence.

Page location	Current source copy / behavior	New copy or required change	Why
Alongside CTA	No approved measured customer story in the supplied evidence	“You will leave with a pathway map, a first measure, and the questions your clinical and IT leads should review.” Reserve quote/logo slots until documented approval.	Provides useful proof of process without inventing testimonials.
Form plumbing	`handleSubmit` waits, logs, then displays success	Implement durable lead receipt, ownership notification, error handling, and end-to-end test before public links.	A clicked CTA is not a received lead. Engineering time is outside the marketing schedule.

Do not add an embedded calendar until availability, owner, and booking integration are real. If it becomes available, place it directly on the page and confirm timezone and confirmation delivery. The current form should not promise a calendar that is not there.

Part 5 - 14-day launch plan within the shared five-hour weekly budget

When this clock starts: these are campaign days, not a second company-wide work plan. Start day 1 only after the required destinations and receipt checks work- normally no earlier than implementation week 3. During these two weeks, this sprint replaces the ordinary creative/outreach slots in Prompts 2 and 5; it is not added to them. If the sprint is not launched, continue the ordinary 30-day plan. All ten posts are drafted, so reuse the single filmed card sequence instead of producing ten new assets.

The plan uses **300 marketing minutes per week**: 25 minutes of brief daily checks across the week, 30 minutes of weekly review/report, 30 minutes reserved toward the monthly deep dive, and **215 minutes of actual campaign/distribution work**. In a monthly-report week the full 120-minute review replaces the 30-minute reserve and **90 minutes of execution must be cut**. Live demo calls, site engineering, video post-production beyond basic phone editing, and delivery evaluations need additional named capacity. No paid spend is assumed. “Publish/send” below is conditional on a working destination, permissioned audience, and content approval; holding an asset is a valid action when a gate fails.

Day	Campaign work and asset used	Marketing minutes	Publish/send and end-of-day measure	If weaker than the practical gate
1	Verify deployed `/request-demo` submission/receipt with owner; mark copy edits for engineer. Prepare Email 1 and Day 1 post.	35	Post Day 1 only if relevant founder channel exists. Record unique relevant replies and form-test result.	If receipt fails, hold all destination links; use no false success claim.

Day	Campaign work and asset used	Marketing minutes	Publish/send and end-of-day measure	If weaker than the practical gate
2	Share the pathway worksheet draft with founder for one useful example; publish Day 2 text using same card photo.	25	Record qualified comments or replies, not reach alone.	If readers discuss other pains, capture exact themes for research.
3	Prepare Email 2 and permissioned segment; draft neutral follow-up to interested replies.	25	Send only if opted-in list and working P01 exist; otherwise keep draft. Log delivery/clicks if sent.	If no list/page, use worksheet in a permissioned one-to-one conversation.
4	Source-check and schedule Day 4 Geostat data card; film main card sequence in one phone take if founder available.	45	Post data question; log relevant replies and source/caption QA.	If filming needs a production crew, use still cards rather than adding time.
5	Trim/caption the existing phone clip; schedule Day 5 caption and copy-check `/request-demo`.	45	Publish video only after receipt verified; count accepted requests, qualified bookings, and video questions separately.	If no receipt, post educational clip without an inactive CTA link or hold it.
6	Prepare Email 3 with source link; queue warm ad B as an organic text variant, not paid media.	20	Send only to actual permissioned opened/non-converted segment. Track real replies.	If open data is unavailable, do not infer the segment; keep draft.
7	Publish mechanism post from prepared text; review the first week's distinct hospital conversations.	20	Count held qualified demos and named next steps (N/A rate if denominator zero).	If no qualified interest, inspect route and audience fit before changing creative.
	Week 1 campaign execution	**215**	**+85 baseline = 300 minutes total.**	

Day	Campaign work and asset used	Marketing minutes	Publish/send and end-of-day measure	If weaker than the practical gate
8	Publish sourced competitor-review story; use already checked R6 paraphrase.	30	Record relevant questions or corrections.	If audience thinks it is MediMind proof, clarify visibly and revise.
9	Publish myth-bust text and prepare one three-option follow-up note.	25	Track replies naming a current remedy.	If none name a real pathway, ask for one incident in the next conversation.
10	Prepare/send Email 4 only to real permissioned segment; align any booked-demo preparation card.	25	Record delivered emails and accepted requests separately.	If no segment, do not send; use time for a named-account research note.
11	Publish the synthetic micro-story with explicit fictional label.	25	Count responses that describe a real workflow.	If synthetic scene is misunderstood, simplify labels and wording.
12	Publish the transition-risk post; audit one actual buyer question with clinical/IT counterpart.	30	Record safety objections and any qualified demo scheduled.	If concern is supplier trust, prepare verified scope/support answers before more promotion.
13	Inspect visit-to-received-request handoff, source attribution, and held-demo log; use prepared copy only.	40	Compare counts of relevant visitors, accepted requests, booked demos, held qualified demos.	If requests are zero, first prove submission works; avoid percentage theater.
14	Publish honest process-proof close; write two-week learning note and decide next asset.	40	Cumulative held qualified hospital demos and readiness-approved evaluations; count distinct buying organizations.	If qualified demos are absent after a working route and genuine reach, retest the hook and target-account selection.
	Week 2 campaign execution	**215**	**+85 baseline = 300 minutes total.**	

By day 7, three useful signals: (1) at least one valid, received request or verified direct founder reply from a relevant hospital; (2) at least one qualified booking with a named pathway and buyer role; (3) one held qualified demo or a dated, owned path to it. These are provisional gates, not statistical benchmarks. If none occurs, diagnose route receipt and qualified reach before declaring the concept wrong.

Day-14 budget-increase metric: fully loaded cost per **held qualified demo from a distinct new buying organization**, counting founder marketing time at an agreed internal hourly cost and any paid spend. If no qualified demo is held, the rate is **N/A** because its denominator is zero. Paid-only acquisition cost is **N/A** while no paid campaign runs. No increase is automatic. Consider a small paid test only after a working destination, approved budget, consent/measurement setup, a qualified-demo signal from organic distribution, and enough capacity to answer leads. The north star remains cumulative held qualified hospital demos toward the 90-day planning scenario of 12.

Concept-wrong signal: Several independent, relevant hospital operators can describe recent handoff problems but consistently say the cost is minor, the incumbent or narrow tool resolves them, and a first-pathway comparison is not worth an executive/committee conversation. One weak post or a broken form does not disprove the concept.

Sources and handoff to Prompt 4

Campaign source: Prompt 3A script. Position and evidence boundaries: avatar, problem stack, review corpus, funnel, traffic routine, and current demo source. The Geostat 2025 hospital release supplies the Day 4 facility count. Capterra's Taylor C. Epic review supplies the clearly labeled competitor story; it does not support MediMind outcomes. Prompt 4 must give `/request-demo` an actual received-request path and must keep optional worksheet, webinar, and sandbox pages distinct from the two-week parallel evaluation.

Source: Original files/Complete B11 HTML library/Editable text/p2-04-funnel-pages.md. Historical wording preserved; not a new approval or factual revalidation.

Part 2, Prompt 4 - Build Your Funnel Pages

> **Status and inputs, 23 September 2026.** Complete page-copy and wiring specification, **not implemented website work**. The 85-item content catalog from Prompt 1 is the source of content-piece IDs, channels, CTAs, and destinations. The provisional buyer is a Georgian private-hospital CEO/owner/group operator; the campaign is **"One patient. Too much repeat work."** with the question **"Why does one patient create so much extra work?"** and the CTA **"Map my hospital's first improvement."** The first sale step is a recommended 25-minute founder-led pathway demo and a possible measured, readiness-dependent parallel evaluation. There are no approved customer testimonials, outcome numbers, or logos in this edition. The site homepage `/` and `/request-demo` route exist, but the catalog's proposed product, proof, worksheet, offer, comparison, activation, and referral destinations do not yet exist as verified routes. Source inspection of `/request-demo` found a simulated submission rather than durable lead receipt. Do not publish campaign links to unbuilt routes or claim that a form request is received until tested.

Conversion-rate note. The source library states broad SaaS uplift/rate claims for clear headlines, short forms, proof, video, and calendars without a verifiable source in the supplied text. They are design prompts, not MediMind benchmarks. Every page below names its actual conversion event and a practical **pre-launch gate**. With tiny hospital volumes, report counts and N/A when a denominator is zero; do not optimize to a borrowed percentage. A calendar should only be embedded when it really books, respects local time, and confirms an owner. A form should only promise a privacy or response practice after operations adopts it.

Step 1 - Map every content piece to its destination

Route key. P01 `/pathway-assessment` = awareness worksheet and five-minute check; P02 `/hospital-os` = product overview; P03 `/case-study` = verified case study, held pending evidence; P04 `/webinar` = optional webinar, unscheduled; P05 `/request-demo` = existing demo route needing repair; P06 `/early-partner` = offer and evaluation; P07 `/compare` = alternatives comparison; P08 `/activation` = private activation; P09 `/referrals` = private advocacy; P10 `/evidence` = evidence method and later approved stories; P11 `/sandbox` = optional guided synthetic-data trial, unlaunched. Only P05 currently corresponds to a catalog destination already in the site. P04 and P11 are included because the separate page library asks for those page types; no content-catalog item points to them yet.

The table is the **complete 85-piece map**. The CTA text is the action from the catalog; the destination key resolves to the route/type above. `No` means the proposed destination is not verified as live; `Yes, repair` applies only to P05. The page-action map in Part 2 supplies the next step after landing, so each row forms a complete content → page → action flow.

Content piece (ID and name)	Stage	CTA action → destination/type	Exists?
T01 One patient, seven handoffs	TOFU	Map my hospital's first improvement → P01 worksheet	No
T02 The hidden cost of searching	TOFU	Use the pathway worksheet → P01 worksheet	No
T03 When two reports disagree	TOFU	Get the reconciliation checklist → P01 worksheet	No
T04 The duplicate-entry trail	TOFU	Map my hospital's first improvement → P01 worksheet	No
T05 Three calls for one result	TOFU	See the pathway map → P01 worksheet	No
T06 The spreadsheet behind the dashboard	TOFU	Get the checklist → P01 worksheet	No
T07 How to map a patient handoff	TOFU	Download the worksheet → P01 worksheet	No
T08 Why hospital reports disagree	TOFU	Get the reconciliation checklist → P01 worksheet	No
T09 What a delayed claim does and does not mean	TOFU	Map one claim pathway → P01 worksheet	No
T10 Patient-pathway friction worksheet	TOFU	Download the worksheet → P01 worksheet	No
T11 Five-minute handoff check	TOFU	Run the check → P01 worksheet/check	No
T12 Founder-built scope trace	TOFU	See the operating system → P02 product	No
T13 Synthetic handoff before/after	TOFU	See the pathway map → P02 product	No
T14 Where the work repeats	TOFU	Use the pathway worksheet → P01 worksheet	No
T15 A report you had to rebuild	TOFU	Get the checklist → P01 worksheet	No
T16 Hospital-operator discussion prompt	TOFU	Read the handoff guide → P01 worksheet	No
T17 Clinical-operations partner note	TOFU	Download the worksheet → P01 worksheet	No

Content piece (ID and name)	Stage	CTA action → destination/type	Exists?
M01 Fix a screen or fix the pathway?	MOFU	Compare the approaches → P07 comparison	No
M02 What parallel evaluation actually requires	MOFU	See the evaluation plan → P06 offer	No
M03 AI assists; clinicians decide	MOFU	See how it works → P02 product	No
M04 Three options on one whiteboard	MOFU	Compare the approaches → P07 comparison	No
M05 What happens in 25 minutes	MOFU	Request a pathway demo → P05 demo	Yes, repair
M06 A result becomes a task	MOFU	See the operating system → P02 product	No
M07 How to evaluate hospital software without a risky cutover	MOFU	Get the evaluation plan → P06 offer	No
M08 Hospital OS versus overlay versus current vendor change	MOFU	Compare the approaches → P07 comparison	No
M09 What FHIR R4 changes in hospital work	MOFU	See the product path → P02 product	No
M10 Parallel-evaluation readiness checklist	MOFU	Download readiness checklist → P06 offer	No
M11 Three-option decision worksheet	MOFU	Compare the approaches → P07 comparison	No
M12 Documented domain map	MOFU	Inspect product scope → P10 evidence	No
M13 Live-pathway evidence template	MOFU	See the evidence method → P10 evidence	No
M14 The change-risk question	MOFU	See the evaluation plan → P06 offer	No
M15 The current-vendor alternative	MOFU	Compare the approaches → P07 comparison	No
M16 Peer discussion on evaluation design	MOFU	Get the readiness checklist → P06 offer	No
M17 IT and clinical review session	MOFU	See the evaluation plan → P06 offer	No

Content piece (ID and name)	Stage	CTA action → destination/type	Exists?
B01 What your 25-minute demo will settle	BOFU	Request a pathway demo → P05 demo	Yes, repair
B02 Match price, define total cost	BOFU	Discuss the early-partner offer → P06 offer	No
B03 The honest go/no-go test	BOFU	Request a pathway demo → P05 demo	Yes, repair
B04 Demo in sixty seconds	BOFU	Request a pathway demo → P05 demo	Yes, repair
B05 Parallel beside current work	BOFU	Discuss an evaluation → P06 offer	No
B06 Who should join the first call?	BOFU	Request a pathway demo → P05 demo	Yes, repair
B07 Questions to ask before a hospital OS demo	BOFU	Request a pathway demo → P05 demo	Yes, repair
B08 What happens after a parallel evaluation	BOFU	See the evaluation plan → P06 offer	No
B09 What current-software price match should include	BOFU	Discuss the early-partner offer → P06 offer	No
B10 Pathway demo preparation card	BOFU	Prepare my demo → P05 demo	Yes, repair
B11 Evaluation decision sheet	BOFU	Discuss an evaluation → P06 offer	No
B12 Verified implementation timeline	BOFU	Review verified evidence → P03 case study	No; evidence held
B13 Buyer-approved workflow result	BOFU	See the evidence method → P10 evidence	No; result held
B14 Your pathway before our demo	BOFU	Prepare my demo → P05 demo	Yes, repair
B15 What we will compare	BOFU	Review the evaluation plan → P06 offer	No
B16 Private buying-committee walkthrough	BOFU	Request a pathway demo → P05 demo	Yes, repair
B17 Procurement questions session	BOFU	Discuss the early-partner offer → P06 offer	No
A01 First pathway, first baseline	AFU	Review the activation plan → P08 activation	No, private

Content piece (ID and name)	Stage	CTA action → destination/type	Exists?
A02 What staff should report in week one	AFU	Use the activation checklist → P08 activation	No, private
A03 One improvement, then the next	AFU	Review next-pathway options → P08 activation	No, private
A04 How to log a handoff issue	AFU	Open the activation guide → P08 activation	No, private
A05 Five-minute pathway review	AFU	Review the activation plan → P08 activation	No, private
A06 How a testable change becomes approved	AFU	Use the change checklist → P08 activation	No, private
A07 Measuring repeat work without double counting	AFU	Use the measurement sheet → P08 activation	No, private
A08 Running a safe pathway review	AFU	Open the activation guide → P08 activation	No, private
A09 Selecting the second pathway	AFU	Review next-pathway options → P08 activation	No, private
A10 First-pathway measurement sheet	AFU	Download the measurement sheet → P08 activation	No, private
A11 Change-request decision card	AFU	Use the change checklist → P08 activation	No, private
A12 Accepted-workflow log	AFU	Review the activation plan → P08 activation	No, private
A13 Measured improvement record	AFU	Review the measurement sheet → P08 activation	No, private
A14 Your first review agenda	AFU	Review the activation plan → P08 activation	No, private
A15 A request ready for test	AFU	Use the change checklist → P08 activation	No, private
A16 Clinical-IT issue huddle	AFU	Open the activation guide → P08 activation	No, private
A17 Cross-site learning exchange	AFU	Review next-pathway options → P08 activation	No, private
L01 The buyer's permission comes first	LOFU	See approved stories → P10 evidence	No; proof gated
L02 A pathway another leader can inspect	LOFU	Read the case study → P03 case study	No; evidence held

Content piece (ID and name)	Stage	CTA action → destination/type	Exists?
L03 What we changed after feedback	LOFU	See approved stories → P10 evidence	No; proof gated
L04 Customer pathway walkthrough	LOFU	Read the case study → P03 case study	No; evidence held
L05 What a reference actually verified	LOFU	See approved stories → P10 evidence	No; proof gated
L06 Why we chose the first pathway	LOFU	Read the case study → P03 case study	No; evidence held
L07 How to read a hospital software case study	LOFU	See approved stories → P10 evidence	No; proof gated
L08 From one hospital pathway to another	LOFU	Read the case study → P03 case study	No; evidence held
L09 What changed in a claim handoff	LOFU	Read the case study → P03 case study	No; evidence held
L10 Reference-story evidence checklist	LOFU	Download the checklist → P10 evidence	No; proof gated
L11 Peer evaluation guide	LOFU	Get the guide → P09 referral	No, private
L12 Buyer-approved measured case study	LOFU	Read the case study → P03 case study	No; evidence held
L13 Reference conversation	LOFU	Request a reference → P09 referral	No, private
L14 Share the method, not a claim	LOFU	Review the story request → P09 referral	No, private
L15 A peer may have this problem	LOFU	Share the approved case study → P03 case study	No; evidence held
L16 Peer operator exchange	LOFU	Request a reference → P09 referral	No, private
L17 Hospital-association learning session	LOFU	See approved stories → P10 evidence	No; proof gated

Step 2 - Identify which pages need to be built and edit what exists

I will now build the following page-copy specifications: P01 worksheet; P02 product overview; P03 case-study shell held for evidence; P06 evaluation/offer; P07 alternatives comparison; P08 private activation; P09 private referral; P10 evidence hub. P05 is an existing route with edits below. P04 webinar and P11 guided sandbox are

optional copy-ready additions from the separate page library, **not current campaign destinations**. No code or site route is changed by this document.

Page	Visitor/job	Required state before promotion	Conversion event to count
P01 worksheet	TOFU visitor identifies a first pathway and receives a useful worksheet	Page, actual asset delivery, consent text, and analytics verified	Worksheet received; subsequent qualified demo counted separately
P02 product	TOFU/MOFU visitor understands mechanism and boundaries	Product scope and screenshots checked against accepted current capability	Click to demo from a relevant visitor
P03 case study	BOFU/LOFU reader evaluates a real customer result	Customer permission, baseline, method, after measure, limitations, and quote approval	Qualified demo or reference request; **hold until evidence exists**
P05 demo	MOFU/BOFU visitor requests a founder-led pathway conversation	Actual lead persistence/notification, owner, error handling, and tested confirmation	Valid received request → booked → held qualified demo
P06 offer	MOFU/BOFU buyer understands evaluation and commercial questions	Founder-defined terms and readiness boundaries confirmed per proposal	Demo request or readiness discussion
P07 comparison	MOFU buyer chooses among current supplier, narrow tool, and MediMind	Fair current information; no unsupported competitor weakness	Demo request with named alternative
P08 activation	AFU customer starts accepted pathway work	Private access, named owner, agreed scope/data and safety plan	First accepted task/baseline recorded
P09 referral	LOFU customer chooses whether to introduce a peer	Permission, customer satisfaction, no implied incentive	Permissioned introduction; no unsolicited send
P10 evidence	MOFU/BOFU/LOFU reader sees what is documented and pending	Accurate scope statuses and separate public approvals	Evidence inspection → qualified demo; approved story views separately
P04 optional webinar	MOFU attendee registers for a real practical session	Actual date, host, platform, reminder/recording policy	Real registration; **N/A while unscheduled**
P11 optional sandbox	BOFU evaluator requests a guided synthetic-data trial	Working safe environment, owner, scope, terms, data handling	Trial accepted and first task completed; **N/A while unlaunched**

Existing P05 `/request-demo`: precise edit instructions

The inspected RequestDemo.tsx currently displays “Enterprise Demo Request,” “See MediMind on your hospital's workflows,” a “30 minutes” subheadline, a three-step form with nine required fields, “Request Demo,” a timer/console-log success path, a 24-hour contact promise, and “Live in 25 hospitals.” These are source observations, not a deployed-browser audit.

Location	Current copy/behavior	Replacement	Reason
Eyebrow and H1	“Enterprise Demo Request”; “See MediMind on your hospital's workflows”	“FOR GEORGIAN HOSPITAL LEADERS”; “Map your first patient-pathway improvement”	Connect the campaign promise to the landing page.
Subhead	“30 minutes. One login, one chart, one patient - live…”	“In 25 minutes with our founder, trace one recurring handoff and choose what your team should measure before a wider change.”	Match the offered call and avoid implying a real patient's live record.
Form introduction	Three stages headed Personal, Company, Healthcare	“Bring one handoff your team repeats. No patient details needed.” One compact step.	Keep focus on the buyer's job.
Fields	Nine required fields across three steps, phone optional	Required: name, work email, hospital/group. Optional: one-line pathway question.	Enough to route a relevant conversation without collecting unnecessary data.
Submit	“Request Demo”	“Map my hospital's first improvement”	Exact campaign CTA.
Success/error	Simulated success and 24-hour contact promise	Real receipt success: “Your request was sent. We will contact you to arrange the 25-minute pathway demo.” Failure: “We could not send your request. Please try again or use the verified contact option.”	Never say a lead was received unless it was delivered; no unconfirmed response SLA.
Trust and proof	“Live in 25 hospitals”; government status badge	Remove live-site count pending verified accepted scope and public approval. Use specific process copy below. Retain startup badge only after wording/documentation check.	A signed network agreement does not establish all-site go-live.

Final consolidated P05 demo page copy (publish only with repaired behavior):

Eyebrow: For Georgian hospital leaders
Headline: Map your first patient-pathway improvement
Subheadline: In 25 minutes with our founder, trace one recurring handoff and choose what your team should measure before a wider change.
Hero body: Bring one moment where information had to move between clinical, operations, and finance teams. We will follow the work together using a synthetic example and mark what your hospital would need to check.
What you will see: (1) a pathway map from the first task to the next responsible person; (2) a proposed connected version beside today's work; (3) a first measure, owners, and readiness questions for a possible parallel comparison.
Form headline: Which handoff should we map?
Fields: Name; work email; hospital/group; optional one-line pathway. Do not enter patient information.
Button: Map my hospital's first improvement
What happens next: After a successful request, our team contacts you to arrange the founder-led call. No meeting is booked by merely submitting this form.
Process proof alongside form: A physician founder will map one pathway with you. The first output is a question your clinical, operations, and IT colleagues can inspect-not a claim of measured hospital results.
Closing CTA: Bring one repeated handoff. Leave with a first improvement worth testing. **Map my hospital's first improvement.**

If an actual calendar integration is introduced, its slots, timezone, confirmation, and ownership must be tested before the copy says “book.” Customer-quote, result, or logo columns remain empty until permitted. The primary conversion is a **held qualified demo**, not a displayed success screen.

P01 `/pathway-assessment` - lead capture for T01-T11, T14-T17 / TOFU

Visitor arrives from: founder LinkedIn/video, a guide, a permissioned email, or a professional discussion. **Job:** identify one repeated patient-workflow handoff. **Conversion event:** successful worksheet delivery; do not infer purchase intent.

Headline (7 words): Find the handoff your team repeats
Subheadline: A five-minute worksheet to trace one patient journey, mark avoidable handoffs, and choose a first question to measure.
Hero body: Start with an actual recent pathway, but do not enter patient details. The worksheet makes the work visible before anyone argues for a new system.
What you get: (1) four boxes for request, result, next decision, and financial close; (2) a place to mark calls, copied facts, waits, and owners; (3) one question for your clinical and operations leads to test.
Form headline: Send me the pathway worksheet
Fields: First name; work email.
Button: Send the pathway worksheet
What happens next: We email the blank worksheet and show an immediate download link after successful delivery.
Privacy copy, proposed pending policy review: We use your work email to deliver the worksheet and respond to your request. Separate opt-in is required for campaign emails. Do not include patient information.
Optional next step after value is delivered: If a recurring handoff matters to your hospital, **map my hospital's first improvement** in a 25-minute founder-led call.

Actual worksheet body, ready to place in a one-page PDF or on-page form: “Patient pathway: _____ (describe the kind of journey, not a person). Date observed: _____. (1) First request: who creates it, where is it recorded, and who needs it next? (2) Result or response: where does it arrive, who notices, and what happens if it is late? (3) Next clinical/operations decision: what facts must be visible, who confirms them, and what gets re-entered? (4) Financial/administrative close: what must reconcile, what does staff check manually, and what remains unknown? Circle each call, copied fact, wait, or recheck. Count the circles; note the staff time and any exception you can verify. Ask: which circle repeats enough to be worth fixing, and could the current supplier or a narrow change remove it?” This is a worksheet, not a diagnostic score. The P01 five-minute check uses these four boxes and returns the chosen pathway, not an invented maturity grade.

P02 `/hospital-os` - product overview for T12-T13, M03, M06, M09 / TOFU-MOFU

Visitor arrives from: a synthetic pathway visual, a mechanism post, or a FHIR explainer. **Job:** understand what MediMind does today, what needs local setup, and what is future direction. **Conversion event:** qualified move to a demo request.

> **Eyebrow:** One hospital pathway at a time
> **Headline:** Clinical and business work, in one path
> **Subheadline:** MediMind is a Georgian hospital operating system built around a shared FHIR R4 record and workflows that cross departments.
> **Opening:** A patient journey does not end at a chart. An order, result, task, medicine, bill, and report may pass through different teams. MediMind gives those teams a shared foundation so the next responsible person can work with relevant context. What the system can demonstrate or deploy at your hospital depends on the agreed modules, interfaces, data, and review.
> **How it works:** (1) Map the current pathway and people who own each step. (2) Show a proposed connected workflow using synthetic data. (3) Set a baseline and compare, with clinical and IT sign-off before any production change.
> **What FHIR R4 means:** It is a common structure for health information-like agreed labels on every handoff. It helps information move and be understood, but does not automatically solve interface, data-quality, or training work.
> **AI's role:** Defined assistance can help draft or surface information for human review. The future direction is broader coordination; no autonomous clinical decision or guaranteed patient outcome is claimed.
> **Scope note:** Clinical and business domains are described in the product map; ask which are demonstrated, configured, accepted, or planned for your institution. External payers, ministries, devices, and other systems still require appropriate interfaces.
> **Proof panel:** Founder-authored Medplum implementation account describes the architecture and a signed anchor-network agreement. It is not an independently measured outcome study or a statement that every facility is live.
> **CTA:** Bring one handoff; **map my hospital's first improvement** → P05.

Visual placement: Use the campaign's paper-card path or a clearly synthetic product screen with current capability labels. A customer logo or result card is held until documented permission and measurement.

P03 `/case-study` - verified case study for B12, L02, L04, L06, L08, L09, L12, L15 / BOFU-LOFU

Visitor arrives from: a verified implementation timeline, approved pathway walkthrough, or permissioned referral. **Job:** inspect a real before/after result. **Conversion event:** qualified demo or approved reference request. **Publication state: hold** until customer approval and the fields below are filled with evidence. A fabricated case is

not a usable page.

> **Working headline slot (under 10 words):** How [hospital] improved [verified pathway measure]
> **Subheadline slot:** [Hospital and team] compared [defined workflow] over [period], including training, exceptions, and total staff effort.
> **Before:** [Buyer-approved description of the actual workflow, systems, and baseline; date and denominator.]
> **Moment that made them act:** [Documented event or decision, in approved language; do not create a patient case.]
> **What changed:** [Accepted MediMind scope, interfaces, staff training, parallel work, and who signed off.]
> **After:** [Measured process and/or financial outcome, same unit as baseline, limitations, and what else changed.]
> **Pull quote:** [Exact customer-approved quotation, name/title/hospital, with publication consent. If absent, omit the block.]
> **Three result numbers:** [baseline], [after], [net change]-each with unit, dates, denominator, and method. These are **fields to fill**, not numbers to publish.
> **Social-proof bar:** [approved name/logo] · [accepted scope] · [measurement period]. Omit until approved.
> **CTA after results:** See what a comparable first-pathway test would require at your hospital. **Map my hospital's first improvement** → P05.

Useful live alternative until case evidence exists: On P10, publish “How we will measure a first pathway” with blank baseline/after fields and a synthetic example explicitly labeled synthetic. Do **not** publish P03 with bracketed placeholders and call it a customer case.

P06 `/early-partner` - offer and readiness for M02, M07, M10, M14, M16-M17, B02, B05, B08-B09, B11, B15, B17 / MOFU-BOFU

Visitor arrives from: a risk/change article, preparation card, demo follow-up, or price-match question. **Job:** see the evaluation process, commercial offer, and unresolved terms without hidden promises. **Conversion event:** a qualified demo or a named evaluation-readiness discussion.

> **Headline (7 words):** Do not switch on a promise alone
> **Subheadline:** Compare one agreed patient pathway beside current work, measure the full effort, and decide with clinical and IT owners.
> **The offer:** For eligible early partners, the founder proposes matching current software pricing and including continuing development. Requested features are proposed to be testable in three business days; a tailored parallel evaluation is proposed within two weeks when prerequisites are met. These are commercial and delivery intentions requiring written scope. Testing a feature is separate from clinical validation and production release; a parallel evaluation is separate from a full cutover.
> **What is included in the first discussion:** (1) map one current pathway; (2) identify information and people needed; (3) show a relevant synthetic workflow; (4) select a first process measure; (5) list access and integration prerequisites; (6) name clinical/IT review and stop conditions.
> **Risk reversal:** Your current operation keeps running during a properly scoped parallel evaluation. Your team reviews the comparison and can choose go, adjust, or stop under the agreed plan. There is still staff work, data work, and possible cost; we will itemize it.

> **Trust/proof:** See the method and exact accepted scope for a proposed evaluation. Customer quotes and measurable outcomes will appear only with source records and permission.
> **CTA: Map my hospital's first improvement** → P05.

Five buyer FAQs:

1. **"Do we have to replace everything to start?"** No. Start with a chosen pathway and decide wider scope after an agreed comparison. The eventual replacement and external-interface map is hospital-specific.
2. **"Does two weeks mean production go-live?"** No. It is the founder's readiness-dependent target for a tailored parallel evaluation. Data access, owners, interfaces, and acceptance criteria must be agreed; production sign-off is separate.
3. **"Are all requested features live in three business days?"** The founder's proposed target is a **testable** change, subject to defined size and prerequisites. Clinical, security, and production approvals take their own review.
4. **"Does matched price cover every cost and future AI?"** The matched current software price is an early-partner offer; total cost, term, included development, implementation, interfaces, support, and advanced AI must be specified in the proposal. No lifetime lock or blanket inclusion is established.
5. **"What if the current supplier can solve our problem?"** Compare that solution on the same pathway, including risk, time, total cost, and observed work. MediMind should win only where the hospital can see a better route.

Actual readiness checklist content: "Name the pathway and one recent instance without patient identifiers; name executive, clinical/operations, and IT owners; list required source systems and interfaces; agree whether synthetic or authorized real data may be used; record baseline task count/time/errors and any financial metric with its definition; estimate hospital and MediMind staff effort; define privacy/security and clinical-review requirements; define the go/adjust/stop criteria and who signs off." This is the content for M10 and the P06 download. Do not make it a medical checklist or claim regulatory compliance.

P07 `/compare` - comparison for M01, M04, M08, M11, M15 / MOFU

Visitor arrives from: three-option guide or current-vendor question. **Job:** see when a narrow fix or incumbent is better, and when a core comparison is warranted. **Conversion event:** demo request with named current alternative.

> **Headline:** MediMind or your current hospital software?
> **Subheadline:** Compare the same patient pathway across three real options: change the current system, add a focused tool, or evaluate a shared operating system.
> **Opening:** The lowest-risk answer may be to keep what works. Start with the problem, the current supplier's proposed fix, and the work your staff still must do. This page is a decision guide, not a claim that every competitor is deficient.

Buyer question	Current supplier change	Focused add-on/integration	MediMind operating-system evaluation
Can it address **this whole pathway**?	Often plausible; inspect proposal and installed version.	Strong when pain sits in one step; may leave handoffs.	Intended to cover clinical/business work; inspect accepted scope.

Buyer question	Current supplier change	Focused add-on/integration	MediMind operating-system evaluation
How much **change** does staff face?	Usually less core migration.	New tool, interface, and training still add work.	Larger potential transition; parallel plan and staff effort must be explicit.
Who owns **local adaptation**?	Existing vendor and hospital under contract.	Vendor, integrator, and current-system team may share it.	MediMind proposes continuing development; capacity and terms need agreement.
What is the **total cost**?	Quote and internal staff time needed.	License, integration, support, and maintenance needed.	Price-match offer does not cover unknown transition/optional costs; obtain proposal.
What **proof** is available?	Current staff experience and a proposed fix.	Pilot or site evidence if provided.	First scoped pathway comparison; independent measured outcomes pending.

> **Honest assessment:** MEDWORK publicly offers Georgian hospital modules and government integrations; Epic, InterSystems, and Oracle also offer broad EHR and AI capabilities. A current supplier may be the right answer for a narrow, fixable issue. MediMind is worth evaluating where the problem follows the patient across teams and a shared workflow can be shown with acceptable risk.
> **Migration-story slot:** [No customer-approved migration story exists; do not publish a before/after anecdote.] Instead show the process: map → readiness → parallel comparison → measure → clinical/IT go/adjust/stop decision.
> **CTA:** Bring the current option and one recent handoff. **Map my hospital's first improvement** → P05.

Comparison claims are grounded in official vendor pages, InterSystems, Epic, and Oracle, with nuance detailed in Prompt 5. No head-to-head performance, Georgian availability, or competitor price is asserted.

P08 `/activation` - private activation for A01-A17 / AFU

Visitor arrives from: an accepted evaluation invitation or authenticated customer update. **Job:** get the first agreed pathway ready for safe measurement. **Conversion event:** the hospital owner records an accepted baseline and first review task. **Access:** private; do not route public ads here.

> **Headline:** Start with one accepted pathway
> **Subheadline:** Put the agreed work, owners, baseline, and stop rule in one place before comparing anything.
> **First three actions, each under 10 minutes for the named owner:** (1) Open the approved pathway sheet and confirm the clinical, operations, and IT owners. (2) Record one baseline task count/time and where exceptions are logged; do not enter patient identifiers here. (3) Confirm the next review time and how to flag a safety or data concern.
> **First result:** “Your first pathway is ready for a controlled review when owners, data permissions, and acceptance conditions are confirmed.” No improvement number is implied.
> **Help CTA:** Contact your named MediMind evaluation owner through the agreed support channel. [Insert

verified contact route before launch.]
Next step: Review the first measurement with the hospital team; choose go, adjust, or stop before adding another pathway.

Measurement sheet body: “Pathway/version; observation date; baseline task count and time; repeat entries; exceptions by type; staff training/parallel effort; clinical or data concerns; proposed after measurement using the same definition; reviewer; decision; sign-off.” Staff time is capacity, not automatic cash savings. The A01-A17 content variations all return here; their individual CTAs select the matching section or download.

P09 `/referrals` - private referral for L11, L13-L14, L16 / LOFU

Visitor arrives from: a customer who has an accepted outcome and has agreed to discuss a peer introduction. **Job:** share a useful method without implying an endorsement or incentive. **Conversion event:** an authorized introduction or reference conversation.

Headline: Share a pathway lesson with a peer
Subheadline: If your team found the first evaluation useful, you may introduce another hospital leader facing a similar handoff.
What you get: A factual, customer-approved summary of your own evaluation and control over whether your name, institution, or results are shared. No referral incentive is offered or implied.
What your peer gets: A neutral first-pathway worksheet and a 25-minute conversation about the work they want to measure.
How it works: (1) Review and approve exactly what may be shared. (2) Send the message below yourself or request a permissioned introduction. (3) If your peer wants a conversation, MediMind arranges a pathway demo.
Copyable referral message: “I thought of you because your team may also have a patient handoff that creates repeated work. MediMind uses a simple first-pathway map before discussing a wider system change. If useful, I can introduce you for a 25-minute conversation. There is no obligation.”
Button: Request a permissioned introduction
Privacy note: Nothing is sent to a peer automatically. Confirm both parties' permission before connecting them.

If no customer has approved a result or invitation, do not present this as an active endorsement page. A buyer-approved case study can later link here.

P10 `/evidence` - evidence method and proof hub for M12-M13, B13, L01, L03, L05, L07, L10, L17 / MOFU-LOFU

Visitor arrives from: scope/proof content. **Job:** distinguish product description, accepted deployment, measured outcome, and future work. **Conversion event:** informed demo request; later, an approved story view.

Headline: See what can be checked today
Subheadline: A clear record of what MediMind can demonstrate, what a hospital has accepted, what has been measured, and what is still proposed.
Evidence labels: Demonstrated = shown in a synthetic or authorized environment; **accepted** = hospital-signed scope with date; **measured** = baseline, after measure, method, and limitations; **planned** = future work, not available proof.

> **Current evidence:** The public founder-authored Medplum implementation account describes the FHIR R4 approach and a signed network agreement. Accepted facility/module counts and independently measured buyer outcomes have not been established for this page.
> **How to read a claim:** Ask which workflow, which site, which date, which denominator, what changed besides software, and whether the customer approved publication.
> **Stories area:** [Approved story 1: pending] [Approved story 2: pending] [Approved story 3: pending]. Do not render these as testimonial cards. Until approval, show the evidence method and a synthetic-pathway example clearly marked as such.
> **Reference-story checklist:** Organization and permission; before workflow; intervention and acceptance; after workflow; measure definitions; safety/exception review; cost and training; limitations; named approver and review date.
> **CTA after method:** Want to test the same approach on your own pathway? **Map my hospital's first improvement** → P05.

No results bar, customer quotation, video testimonial, or logo is filled with a competitor's review or a founder-reported aspiration. That makes P10 useful now as a method page while leaving proof slots honest.

Optional P04 `/webinar` - webinar registration, not scheduled / MOFU

This is **copy for a future page**, not a live event or current catalog destination. Use only after a real date, platform, founder availability, registration receipt, and recording policy exist. Until then, do not show a registration button.

> **Eyebrow:** For private-hospital decision teams
> **Headline:** How to map repeat work without switching systems
> **Subheadline:** A practical 45-minute session: 30 minutes mapping a synthetic patient pathway and 15 minutes for questions.
> **Date/time/duration above fold: Not scheduled.** [Insert confirmed date, time zone, and 45-minute duration before publishing.]
> **You will learn to:** (1) trace an order-to-result-to-bill handoff; (2) distinguish software friction from process and policy; (3) choose a measure and safe next step; (4) list clinical and IT questions for a parallel comparison.
> **Form:** Name, work email, hospital/group (three fields). **Button after scheduling:** Reserve my pathway workshop place.
> **Speaker:** Lasha Khoshtaria, physician and MediMind CEO. Do not invent guest affiliations.
> **Recording:** [State only after the recording and delivery policy is decided.]
> **Testimonials:** [Two approved attendee quotations pending; omit blocks until they exist.]
> **Closing CTA:** Learn to map one pathway before making a bigger software decision. [Reserve my place-activate only for a real date.]

The no-event alternative is the P01 worksheet and P05 founder-led demo. The webinar does not become a permanent monthly demand-generation commitment inside five marketing hours.

Optional P11 `/sandbox` - guided synthetic-data trial, not launched / BOFU

This is **not** the two-week parallel evaluation. It is a proposed 14-day guided synthetic-data sandbox, with no card as a recommendation, subject to actual environment, support, security, and data-handling decisions. Do not publish signup until those conditions exist.

Headline: Test a patient pathway with synthetic data
Subheadline: Explore one guided workflow without bringing patient records into a trial environment.
What you can try: Follow a synthetic order, result, next task, and administrative close; note which step your staff would want to compare locally.
What you cannot conclude: A sandbox does not prove integration, production readiness, clinical safety, or financial outcome at your hospital. Those require a separately scoped evaluation.
Form: Name, work email, hospital/group (three fields). **Button after launch:** Request guided sandbox access.
Access line: Proposed 14 days, no card as a recommended term; no actual trial availability or deletion policy is asserted until confirmed.
First result: Complete the synthetic handoff and write down one question for your clinical and IT counterparts.
Next step: Map my hospital's first improvement → P05, then discuss readiness for a real parallel evaluation.
Trust/privacy: [Insert approved synthetic-data, account, retention, and support terms before publication.]

Part 2 - Page connection map

The content table in Step 1 and the page-action table below jointly specify a full flow for **every one of the 85 pieces**: take its ID/title, catalog channel, CTA action, and destination from the Step 1 row; append the destination's action and next stage from this table. This is more precise for wiring than repeating the same 85 final-page instructions. For example: **T02** on founder LinkedIn → “Use the pathway worksheet” → P01 `/pathway-assessment` → receive worksheet → MOFU product/option reading or BOFU demo if the leader has a real problem. **M01** on founder LinkedIn → “Compare the approaches” → P07 `/compare` → name the current alternative → BOFU demo. **B01** on founder LinkedIn → “Request a pathway demo” → P05 `/request-demo` → valid received request → booked and held qualified demo. **A10** in customer onboarding → P08 private page → record first baseline → AFU first-pathway review. **L13** in a customer-approved reference context → P09 private page → permissioned introduction → a new buying organization's TOFU/MOFU journey.

Destination and IDs covered by Step 1	Page action	Next funnel state and connection
P01: T01-T11, T14-T17	Receive worksheet / complete five-minute check	MOFU P02/P07 if problem is defined; direct P05 if ready. Capture source ID and consent.
P02: T12-T13, M03, M06, M09	Inspect actual mechanism and scope	MOFU P06/P07, or BOFU P05.
P03: B12, L02, L04, L06, L08-L09, L12, L15	Read approved case and measured result	BOFU P05 or permissioned LOFU P09; **hold source links until page has evidence**.
P05: M05, B01, B03-B04, B06-B07, B10, B14, B16	Submit a real request and later hold qualified demo	BOFU → proposed evaluation P06; track received, booked, and held separately.
P06: M02, M07, M10, M14, M16-M17, B02, B05, B08-B09, B11, B15, B17	Review offer/readiness and request conversation	BOFU P05, then evaluation decision and private AFU P08 only after approval.

Destination and IDs covered by Step 1	Page action	Next funnel state and connection
P07: M01, M04, M08, M11, M15	Compare actual alternatives	BOFU P05 with named incumbent/narrow alternative.
P08: A01-A17	Record accepted first work/baseline and issues	AFU next-pathway review; LOFU P09/P10 only after a verified positive result and permission.
P09: L11, L13-L14, L16	Grant permission or prepare referral	New peer enters TOFU P01 or BOFU P05 by their own choice.
P10: M12-M13, B13, L01, L03, L05, L07, L10, L17	Inspect evidence labels/checklist	MOFU/BOFU P05; after customer approval, P03.
P04/P11: no current catalog IDs	Optional future registration or synthetic trial	P04 → P05; P11 → P05/P06. Neither is an active campaign path.

The table provides the developer a route for every current catalog ID and prevents dead links. Private customer assets should not be indexed or used for public retargeting. Proof assets B12 and L-series case links are **held**, with P10's evidence method as the available alternative; do not silently point “Read the case study” to a blank case page. P01 worksheet CTA wording varies by source piece, but all variants land on the same relevant worksheet/check section, not an unrelated generic homepage.

Part 3 - Build priority

1. **Repair and verify P05 `/request-demo` first.** A simulated success screen can lose a valid hospital request now; this is the only inspected catalog destination that already has a public route. Establish receipt, owner notification, error state, and a test record before inviting traffic. Engineering time is separate from the five-hour marketing budget.
2. **Build P01 worksheet and actual delivery.** The campaign's low-friction educational step and TOFU catalog both depend on it. The worksheet content is above, so page, asset, receipt, and consent can be implemented as one small unit.
3. **Build P02 product overview.** It explains the mechanism and accepted-versus-proposed scope after a visitor recognizes the problem; it prevents a broad AI claim from doing the job of evidence.
4. **Build P06 evaluation/offer.** The buying committee needs readiness, total-effort, price-boundary, and go/adjust/stop answers before agreeing to a parallel test.
5. **Build P07 comparison.** It gives the buyer a fair way to compare the incumbent and a narrow tool, especially when full replacement would be excessive.
6. **Build P10 evidence method.** Useful now without pretending customer results exist. Add approved product scope and customer outcomes only as each record is verified.
7. **Prepare private P08 activation.** Publish only for a real accepted evaluation with named owners and access control; public traffic does not belong here.
8. **Prepare P03 case study, then private P09 referral.** Both depend on a real measured result and customer permission. Bracketed placeholders are editorial fields, never public proof.
9. **Keep P04 webinar and P11 sandbox on hold.** They are optional page-library assets without a scheduled session or verified sandbox. They do not add launch scope to the five-hour weekly routine.

The actively costly page issue is P05's unverified lead receipt and false success/response promise. The next practical conversion risk is a campaign CTA landing on a route that promises a 30-minute tour rather than a 25-minute pathway map. Other unbuilt pages become leaks only when content actually links to them; hold or reroute that content until pages exist. Use the cumulative count of **held qualified hospital demos** as the 90-day north star; proposed worksheet downloads and page views are diagnostic steps, not the goal.

Sources and handoff

Internal: funnel blueprint, content catalog, avatar, problem stack, competition, campaign concept, distribution package, and demo-page source. Public product context: Medplum founder-authored case study; MEDSOFT, InterSystems, Epic, and Oracle Health. These support category descriptions, not Georgian head-to-head outcomes. The next implementation brief must confirm actual routes, approved commercial language, data/consent practice, and a working lead owner before publishing public links.

Source: Original files/Complete B11 HTML library/Editable text/p2-05-traffic.md. Historical wording preserved; not a new approval or factual revalidation.

Part 2, Prompt 5 - Content Distribution and Traffic Strategy

Inputs and status (source as of 22 September 2026; edition assembled 23 September). Funnel: TOFU problem recognition → MOFU workflow understanding → BOFU qualified 25-minute founder-led pathway demo → AFU measured, readiness-approved evaluation → LOFU advocacy only after a customer has a verified experience. The primary conversion is a **held qualified demo from a distinct hospital buying organization**. The first audience is the CEO/owner/group operator of a Georgian private network or multi-profile hospital with a named clinical/operations owner and IT counterpart. Current hospital decision-maker watering holes, source-by-source lead history, owned email size, and social reach are **unknown**, not zero. Existing landing page and `/request-demo` route are present, but the inspected source submission handler does not send a lead to a backend. Proposal: campaign concept “One patient. Too much repeat work.” Hook: “Why does one patient create so much extra work?” CTA: “Map my hospital's first improvement.” Founder and team relationships exist, including an anchor hospital-network agreement as founder-reported; public endorsement or introductions are not assumed. Budget: **5 hours total of founder marketing time per week**; no paid media initially. Sales meetings and engineering implementation require additional time and owners. Current tool stack and actual analytics accounts are unverified.

Campaign vocabulary. “One patient, three handoffs”; “Where does the same detail get entered twice?”; “The report is late because the record is split”; “Keep your current system running while testing one pathway.” These are proposed hooks, not buyer quotations. No channel-performance percentages from the source library are treated as MediMind benchmarks or verified industry facts. Destinations other than the existing `/request-demo` route are proposed until built and tested. The content inventory is the 85-ID catalog in `working/content-catalog.json`; Part 2 maps every ID.

Part 1 - Channel selection and priority

“Fit” means likely access to a Georgian hospital buying committee, not proof the channel works. Time is incremental founder marketing time, per week unless labelled one-off; paid costs are undecided rather than copied from the library's unverified rate claims.

Channel	Fits customer?	Best stage	Time cost	Money cost	Use	Specific reason
SEO and blog content	Yes, selectively	TOFU/MOFU	45-90 min per useful page, reused	0 GEL media; production time	Now, slow build	A narrow “hospital handoff” or Georgian procurement question can be useful after the founder call; search volume must be checked before scaling.
LinkedIn organic	Yes, hypothesis	TOFU/MOFU	45-60 min weekly	0 GEL media	Now	Founder-led explanation can reach hospital leaders and their advisers; target-account engagement matters more than broad impressions.
YouTube organic	Conditional	MOFU	90+ min per edited video	Production time	Later	A credible pathway walkthrough is valuable, but a standing video schedule exceeds five hours. Reuse a verified demo clip first.
Short-form video	Conditional	TOFU	20-40 min to cut existing footage	0 GEL media if reused	Later/occasional	A 30-second handoff example may earn attention; original weekly shoots consume the budget.
Podcast guest appearances	Conditional	TOFU/MOFU	1-3 h plus prep	Usually 0 GEL media	Later	Only a Georgian health/business audience with identified hospital listeners justifies the time.

Channel	Fits customer?	Best stage	Time cost	Money cost	Use	Specific reason
Community participation	Yes, where private hospital leaders actually gather	TOFU/MOFU	20-30 min weekly	0 GEL media	Now, one venue	Answer a real workflow question in a relevant professional group; do not spray generic communities.
AI search and generative discovery	Conditional	TOFU/MOFU	15-30 min per public explainer	0 GEL media	Later, bundled with SEO	Clear source-backed pages may be retrievable, but “AI search optimization” should not become a separate production line.
LinkedIn paid ads	Conditional	MOFU/BOFU	Setup plus monitoring 1-2 h	Paid, budget unapproved	Later	Targeting may be precise but the hospital buying-organization universe is small and conversion handling is not ready.
Google Search/PPC	Conditional	BOFU	Setup plus monitoring 1-2 h	Paid, budget unapproved	Later	Test only when a relevant query, landing page and accepted lead event are proven.
YouTube paid ads	No now	TOFU	High creative/setup	Paid	Avoid now	Broad video views are a poor first proxy for held hospital demos.
Meta ads	No now	TOFU/MOFU	Setup and monitoring	Paid	Avoid now	Broad consumer inventory risks unqualified demand and requires working consent/measurement.

Channel	Fits customer?	Best stage	Time cost	Money cost	Use	Specific reason
Retargeting across channels	Conditional	MOFU/BOFU	1-2 h setup plus upkeep	Paid	Later	Needs sufficient clean public-site audience, consent, exclusions, and a validated conversion before consideration.
Newsletter sponsorships	Conditional	TOFU/MOFU	1-2 h selection/copy	Paid	Later	A trade newsletter can be evaluated by actual hospital readership and cost per qualified conversation.
Cold email outreach	Conditional	BOFU	45-60 min for a few researched accounts	0 GEL tools initially	Now, carefully	Specific one-to-one outreach to a named decision-maker can test whether the pain is real; comply with applicable contact rules.
Owned email newsletter	Conditional	MOFU	30-45 min if reusing content	Platform cost unknown	Later	Build a permission-based list once a repeatable useful note exists; no need to promise a weekly newsletter.
Lead magnet email sequences	Conditional	MOFU	1-3 h setup	Platform tier dependent	Later	A short consented follow-up to a real worksheet request may help; the form and delivery must exist first.

Channel	Fits customer?	Best stage	Time cost	Money cost	Use	Specific reason
LinkedIn direct outreach	Yes	BOFU	30-45 min for a few researched people	0 GEL media	Now	Founder can ask about one specific workflow after relevant interaction; record organization and response in CRM.
Sales calls and demos	Yes	BOFU	25 min demo plus preparation/follow-up, outside marketing allocation	Delivery time	Now after repair	A held qualified demo is the primary proof of buyer interest, but the current request handler needs a real handoff.
CRM pipeline nurturing	Yes	MOFU/BOFU	20-30 min weekly	Sheet or free CRM	Now	One owner and next action prevent a serious hospital conversation from disappearing.
Warm introductions	Yes	BOFU	30-45 min weekly	0 GEL media	First	A precise introduction to a hospital decision-maker uses existing trust and is more useful than reaching anonymous traffic.
Brand collaborations/co-marketing	Conditional	MOFU	2-4 h per project	Shared costs possible	Later	Consider a practitioner association or complementary supplier only when both audiences and claims are clear.

Channel	Fits customer?	Best stage	Time cost	Money cost	Use	Specific reason
Integration partnerships	Conditional	BOFU/AFU	Engineering and BD time	Unknown	Later	Valuable after a real hospital integration need and supported interface are agreed; a marketplace listing alone will not generate local demand.
Referral programs	No now	LOFU	Setup and administration	Incentive/policy unknown	Later	A formal program requires satisfied, permissioned customers and approved terms, which are not established.
Guest posting	Conditional	TOFU/MOFU	2-4 h per article	Usually 0 GEL media	Later	Publish only when an editor has an audience of hospital operators and the piece can reuse a validated insight.
Influencer and analyst relations	Conditional	MOFU	1-3 h per outreach	Often 0 GEL media, sometimes paid	Later	A credible specialist could challenge the pathway method; borrowed authority should follow evidence, not replace it.
Other people's audiences (OPA)	Yes, selectively	TOFU/MOFU	1-3 h per appearance	Usually 0 GEL media	Later	One expert contribution in a trusted hospital forum may outperform a general feed post; choose by audience, not follower count.

Channel	Fits customer?	Best stage	Time cost	Money cost	Use	Specific reason
Product Hunt	No now	TOFU	Launch preparation high	0 GEL to paid	Avoid now	General software discovery is unlikely to reach Georgian hospital buying committees.
G2/Capterra/Trustpilot/ Clutch	Conditional	MOFU/BOFU	1-2 h initial profile	Listing terms vary	Later	Use verified product facts and authentic reviews when customers can describe their experience; no review farming.
AppSumo	No	TOFU	Deal operations high	Revenue discount	Avoid	A discount marketplace conflicts with a scoped, hospital-specific evaluation.
Industry publications/trade newsletters	Yes, selectively	TOFU/MOFU	1-3 h per pitch	Usually 0 GEL media	Later	An evidence-backed hospital workflow piece can reach the exact committee if circulation is verified.
Press/news features	Conditional	TOFU	2-4 h per pitch	Usually 0 GEL media	Later	A founder story alone is weaker than a documented implementation or measured workflow finding.

Channel	Fits customer?	Best stage	Time cost	Money cost	Use	Specific reason
Industry conferences/trade shows	Conditional	MOFU/BOFU	Day(s), outside five-hour routine	Travel/sponsor costs	Later	Attend only for named target meetings or a qualified speaking slot; no speculative booth spend.
Webinars	Conditional	MOFU	Preparation + delivery exceeds routine week	Platform cost possible	Later	A 45-minute practical session is drafted, but no date, registration flow, or replay policy exists.
Virtual events/online communities	Conditional	TOFU/MOFU	Moderation ongoing	Platform cost possible	Later	Hosting an audience is a separate operating commitment; use existing relevant groups first.
Dark social	Yes, indirect	MOFU/BOFU	Embedded in asset creation	0 GEL media	Now	A concise pathway diagram can be forwarded privately; ask on calls how people heard about it, since attribution will be incomplete.
Customer referrals	Conditional	BOFU/LOFU	Relationship time	0 GEL unless program	Later	Seek a voluntary introduction only after the customer has a verifiable good experience and permits the ask.

Priority stack. (1) Warm introductions: named trust around a small buying universe. (2) Targeted founder outreach by email or LinkedIn: fast learning from a specific problem, not bulk lead volume. (3) CRM follow-up: every conversation has an owner and next step. (4) Founder LinkedIn posts: reusable public explanation and a path to the demo. (5) One search-friendly pathway/FAQ page at a time: a durable answer prospects can read before or after a call. The demo is the conversion experience all five feed into, with its own preparation time.

Three zero-budget quick wins this week, each under two hours. (1) In 45 minutes, identify five distinct hospital buying organizations and write one precise introduction request for a person who actually knows each buyer; log only relationships you truly have. (2) In 60 minutes, turn the synthetic “one patient, three handoffs” example into a short founder LinkedIn post and a private message variant pointing to a verified destination. (3) In 45 minutes, audit `/request-demo` with the site owner: submit a test request, confirm receipt and ownership, and record the gap before directing traffic there. If the deployed flow fails, use a verified direct scheduling/contact route only after it has been tested.

Avoid right now. Product Hunt and AppSumo reach the wrong procurement motion; broad paid video/social spend buys attention before a working conversion; a full newsletter and self-hosted community create recurring work without a validated audience; formal referral/review programs need real customer consent and proof. The library's channel statistics and posting-time suggestions are unverified context, not operating targets for Georgia.

Part 2 - Content piece distribution map

Each row is a compact distribution card: **original format/native home** followed by **external or private channel → precise adaptation; channel hook; CTA; timing**. LI = founder LinkedIn; W = public website/search; E = opted-in email; D = a researched one-to-one note; C = a private customer channel. “Tue 09” and “Thu 09” are test slots in Georgia local time, not measured best times. “After gate” means the named asset, working destination, evidence and consent must exist. A private document or customer email does not need a public UTM link; Prompt 6 records those touches in CRM. A page ID in the catalog is a planned destination unless independently verified; only `/request-demo` is an existing route, and its actual submission still needs testing and repair.

TOFU cards - T01-T17

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
T01 - One patient, seven handoffs	Post / LI	LI → seven numbered handoffs from a fictional case; “Which one repeats in your hospital?”; pathway map CTA; Tue 09. D → cut to the one handoff relevant to recipient; “Where does this recur?”; ask for 25-minute map; after research.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
T02 - The hidden cost of searching	Post / LI	LI → three questions instead of a savings number; “Who knows how long staff spend finding one result?”; worksheet CTA; Thu 09. E → one-question note to opt-ins; “What would you time?”; worksheet; next relevant send.
T03 - When two reports disagree	Post / LI	LI → split clinical/finance report visual and three definition checks; “Which number goes to the board?”; reconciliation checklist; Tue 09. D → use only if account raised reporting; “Which definition differs?”; checklist; after call.
T04 - The duplicate-entry trail	Short video / LI video	LI → 30-second desk sketch with synthetic labels and captions; “Where is the same field typed twice?”; pathway map; Thu 09. W → embed on assessment page with transcript; same hook; worksheet; after video QA.
T05 - Three calls for one result	Short video / LI video	LI → three cut scenes, no patient details; “How many calls does one result require?”; pathway map; Tue 09. D → still frame and one-line question, not full video attachment; pathway map; when prospect names result delay.
T06 - The spreadsheet behind the dashboard	Short video / LI video	LI → show a blank synthetic reconciliation sheet and preparation questions; “Who rebuilds the report?”; checklist; Thu 09. W → captioned embed on assessment; same hook; checklist; after review.
T07 - How to map a patient handoff	Blog/SEO / W	W → step-by-step guide with worked synthetic example; “Map the work before replacing software”; worksheet; publish when page works. LI → three-panel drawing from guide; “Where does the patient wait?”; worksheet; following Tue 09.
T08 - Why hospital reports disagree	Blog/SEO / W	W → sections on definitions, timing and missing fields; “Two reports can both be internally consistent”; checklist; publish after accuracy review. LI → three reconciliation questions; same hook; checklist; following Thu 09.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
T09 - What a delayed claim does and does not mean	Blog/SEO / W	W → table separating rejected, delayed, missed and collected money; “A flagged charge is not recovered cash”; claim-pathway map; after finance review. LI → one comparison panel; same hook; map; following Tue 09.
T10 - Patient-pathway friction worksheet	Lead magnet / W and D	W → ungated one-page PDF or accessible web worksheet; “Count handoffs, not impressions”; download; when built. D → send a relevant section to a named operator; “Could your team count this step?”; use worksheet; after permission/interest.
T11 - Five-minute handoff check	Lead magnet / W and D	W → five-question self-check with no fabricated score; “Which pathway should you inspect first?”; run check; after tool QA. LI → one sample question; same hook; run check; following Thu 09.
T12 - Founder-built scope trace	Proof / W and LI	W → dated product-history explainer labelled founder-authored, not customer endorsement; “What has actually been built?”; product overview; after fact review. LI → one traceable milestone with limits; same hook; overview; Tue 09.
T13 - Synthetic handoff before/after	Proof / W and LI	W → side-by-side fictional workflow labelled synthetic; “See the mechanism, then measure locally”; product overview; after demo validation. LI → crop to one before/after handoff; same hook; overview; Thu 09.
T14 - Where the work repeats	Email / E	E → deliver T10 plus one question, no generic sales sequence; “Which handoff did you circle?”; worksheet; immediately after valid opt-in. D → personal follow-up only to someone who requested it; same hook; pathway demo; after reply.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
T15 - A report you had to rebuild	Email / E	E → one concrete reporting prompt plus T03 checklist; “Which report was rebuilt last month?”; checklist; next opted-in note. D → use if finance lead raised this; same hook; ask to compare; after conversation.
T16 - Hospital-operator discussion prompt	Community / relevant forum	Forum → plain question without link first; “How do you map a cross-team handoff?”; share guide only if requested; during an existing discussion. LI → summarize anonymous learning with no invented quote; guide; after permission.
T17 - Clinical-operations partner note	Community / professional contact	Partner → short neutral worksheet invitation for feedback; “Would this help your members inspect a handoff?”; worksheet; after relationship check. W → publish revised worksheet based on attributable, permissioned feedback; same hook; download; after review.

MOFU cards - M01-M17

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
M01 - Fix a screen or fix the pathway?	Post / LI	LI → three-option decision graphic; “Would a screen fix reach the next department?”; comparison CTA; Tue 09. D → select the prospect's actual option; “What remains after your vendor change?”; comparison; after discovery.
M02 - What parallel evaluation actually requires	Post / LI	LI → checklist of owner, permission, baseline and staff time; “Two weeks begins with readiness”; evaluation plan; Thu 09. E → four-item preparation note; same hook; plan; after opt-in.
M03 - AI assists; clinicians decide	Post / LI	LI → synthetic draft/review/sign-off sequence; “Where does human clinical control sit?”; product overview; Tue 09. W → insert as labelled product explainer; same hook; overview; after capability review.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
M04 - Three options on one whiteboard	Short video / LI video	LI → 45-second whiteboard, including effort for all choices; “Current vendor, add-on, or OS?”; comparison; Thu 09. W → captioned embed on comparison page; same hook; comparison; after video QA.
M05 - What happens in 25 minutes	Short video / LI video	LI → demo agenda in three visual beats; “Leave with one measurable pathway”; request demo; Tue 09 after demo duration confirmed. D → 15-second preview still plus agenda; same hook; request demo; after buyer interest.
M06 - A result becomes a task	Short video / LI video	LI → synthetic screen recording cropped to reviewer handoff; “Who owns this result next?”; product overview; Thu 09 after product validation. W → captioned embed and human-review caveat; same hook; overview; after approval.
M07 - Evaluate software without a risky cutover	Blog/SEO / W	W → six-step readiness guide and workload table; “Compare one pathway before cutover”; evaluation plan; after clinical/IT review. LI → three evaluation questions, not full guide; same hook; plan; following Tue.
M08 - OS versus overlay versus vendor change	Blog/SEO / W	W → decision table with legitimate cases for each; “Replacement may not be the first answer”; comparison; after technical review. D → send the relevant row to one prospect; same hook; comparison; after discovery.
M09 - What FHIR R4 changes in hospital work	Blog/SEO / W	W → plain-language record analogy plus interface caveat; “Shared format still needs local integration”; product path; after technical review. LI → one diagram and caveat; same hook; path; following Thu.
M10 - Parallel-evaluation readiness checklist	Lead magnet / W and D	W → one-page owners/access/baseline/stop checklist; “Can your team test this fairly?”; download; when built. D → highlight missing owner field for interested account; same hook; review plan; after demo.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
M11 - Three-option decision worksheet	Lead magnet / W and D	W → four-column comparison of burden/risk/cost/scope; “Compare actual alternatives”; worksheet; when built. D → prefill no customer data, ask buyer to score own options; same hook; compare; after discovery.
M12 - Documented domain map	Proof / technical diligence	W → verified scope matrix with deployed/accepted/pending states; “Which workflow is actually ready?”; inspect scope; only after module-by-module audit. D → send relevant slice to IT under review; same hook; diligence call; after request.
M13 - Live-pathway evidence template	Proof / W and founder follow-up	W → blank baseline/after template with outcome cells visibly pending; “What would convincing proof require?”; evidence method; after QA. D → customize measure names, not results; same hook; discuss evaluation; after demo.
M14 - The change-risk question	Email / E	E → one risk prompt and parallel-plan link; “What transition failure would stop this?”; evaluation plan; after opted-in comparison interest. D → use prospect's stated risk in reply; same hook; scope call; after response.
M15 - The current-vendor alternative	Email / E	E → two-column comparison question, no competitor caricature; “Can your supplier fix this across departments?”; comparison; after opt-in. D → one-to-one criteria sheet; same hook; compare; after discovery.
M16 - Peer discussion on evaluation design	Community / invited operator roundtable	Private roundtable → four questions, no claims or patient details; “What proof would justify change?”; readiness checklist; only with host/attendees confirmed. E → recap generic method to opt-ins; same hook; checklist; after event approval.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
M17 - IT and clinical review session	Community / association or partner	Partner session → joint risk checklist, not a product deck; “What would clinical and IT each sign off?”; evaluation plan; after partner/date approval. W → publish generic FAQ from approved questions; same hook; plan; after review.

BOFU cards - B01-B17

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
B01 - What your 25-minute demo will settle	Post / LI	LI → three concrete outputs, no guaranteed result; “A demo should settle a decision question”; request demo; Tue 09 after booking route works. D → agenda tailored to one pathway; same hook; meeting ask; after qualification.
B02 - Match price, define total cost	Post / LI	LI → cost stack and explicit eligibility caveat; “Same subscription is not same total cost”; offer discussion; Thu 09 after terms approval. D → send approved cost categories; same hook; commercial call; after buyer asks.
B03 - The honest go/no-go test	Post / LI	LI → go/adjust/stop decision tree; “What would make you say no?”; request demo; Tue 09. E → post-demo decision note; same hook; evaluation plan; after a held demo.
B04 - Demo in sixty seconds	Short video / LI video	LI → synthetic map, task, scorecard in 60 seconds; “What will you actually see?”; request demo; Thu 09 after route QA. W → captioned excerpt on demo page; same hook; booking; after approval.
B05 - Parallel beside current work	Short video / LI video	LI → side-by-side path with workload and sign-off labels; “Compare before any switch”; discuss evaluation; Tue 09 after offer review. W → embed on offer page; same hook; plan; after QA.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
B06 - Who should join the first call?	Short video / LI video	LI → role cards for executive, operations, IT; “Bring the person who owns the handoff”; request demo; Thu 09. E → short invitation note for booked buyer; same hook; prepare call; upon accepted booking.
B07 - Questions before an OS demo	Blog/SEO / W	W → buyer question list on scope, human control, data and cost; “Know what to ask before switching”; request demo; after review. LI → five-question excerpt; same hook; demo; following Tue.
B08 - After a parallel evaluation	Blog/SEO / W	W → go/adjust/stop process with reconciliation and approval; “The test is not the cutover”; evaluation plan; after operations review. D → relevant decision diagram; same hook; review meeting; after demo.
B09 - What price match should include	Blog/SEO / W	W → eligibility, term, scope, migration and third-party-cost questions; “What exactly is matched?”; offer discussion; after commercial approval. D → tailored checklist, not a quote; same hook; proposal conversation; after inquiry.
B10 - Pathway demo preparation card	Lead magnet / demo confirmation and W	E → one-page agenda with no patient-data request; “Bring one recent handoff, not a patient chart”; prepare demo; only after accepted booking. W → ungated card on demo page; same hook; download; after route works.
B11 - Evaluation decision sheet	Lead magnet / founder follow-up	D → go/adjust/stop template with blank baseline/cost fields; “What evidence changes the decision?”; evaluation discussion; after held demo. W → generic blank version on offer page; same hook; template; after review.
B12 - Verified implementation timeline	Proof / founder follow-up	D → dated, consented milestones and scope only; “What was accepted, and when?”; review evidence; after customer authorization. W → public excerpt only if permission explicitly covers it; same hook; case study; after approval.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
B13 - Buyer-approved workflow result	Proof / W and founder follow-up	W → case evidence table with source, denominator and limitations; “What changed in this workflow?”; evidence method; only after buyer sign-off. D → one-page verified excerpt; same hook; reference discussion; after permission.
B14 - Your pathway before our demo	Email / accepted booking	E → one question plus B10 card, no patient details; “Which handoff should we trace?”; prepare demo; immediately after real booking. CRM → log answer, not email content in analytics; same hook; tailored agenda; before call.
B15 - What we will compare	Email / founder follow-up	E → proposed baseline/scope with unresolved fields; “What is fair to compare?”; evaluation plan; within agreed follow-up, no SLA invented. CRM → next-action task; same hook; review meeting; after send.
B16 - Private buying-committee walkthrough	Community / private meeting	Private meeting → one synthetic case and decision worksheet; “Does each owner see their step?”; pathway demo; after invitation. D → role-specific agenda; same hook; confirm attendees; before meeting.
B17 - Procurement questions session	Community / private meeting	Private meeting → cost/responsibility/exit agenda; “What must be explicit before signing?”; offer discussion; after qualified interest. D → approved question sheet; same hook; meeting; before meeting.

AFU cards - A01-A17

These are customer-success assets, not acquisition posts to publish broadly by default. “C” means a private customer workspace/email or a permissioned customer-only update. The contents can be created as needed during actual delivery; their production time is **delivery work outside the five marketing hours**.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
A01 - First pathway, first baseline	Post / customer-only update	C → one-card baseline method, remove site details; “What is our starting point?”; activation plan; at kickoff. Public LI only as generic method after consent and review.
A02 - What staff should report in week one	Post / customer-only update	C → three prompts for repeat work, exceptions and safety; “Which issue needs an owner?”; activation checklist; week one. Customer email → shorter checklist; same hook; after kickoff.
A03 - One improvement, then the next	Post / customer-only update	C → observe/prioritize/test/approve/measure loop; “What did we learn before adding scope?”; next-pathway options; after first review. Public LI only as anonymized method if approved.
A04 - How to log a handoff issue	Short video / customer onboarding	C → synthetic 40-second issue-form walkthrough; “Who follows this up?”; activation guide; at onboarding. Private email → still image and form link; same hook; same week.
A05 - Five-minute pathway review	Short video / customer onboarding	C → review-clock screen recording with sample metric; “Did the handoff improve without new burden?”; activation plan; before weekly owner review. Private email → timestamped recap; same hook; before meeting.
A06 - How a testable change becomes approved	Short video / customer onboarding	C → request/test/clinical-IT approval diagram; “Testable is not production-approved”; change checklist; before first change. Private email → static diagram; same hook; after request.
A07 - Measuring repeat work without double counting	Guide / customer knowledge base	C → worked count example and staff-time caveat; “Minutes are not cash”; measurement sheet; when baseline set. Private owner email → relevant paragraph; same hook; before scorecard review.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
A08 - Running a safe pathway review	Guide / customer knowledge base	C → issue/escalation/go-adjust-stop checklist; “Who can pause this?”; activation guide; before parallel test. Private meeting → one-page checklist; same hook; at review.
A09 - Selecting the second pathway	Guide / customer knowledge base	C → frequency/burden/risk/readiness grid; “Which workflow earns the next test?”; next-pathway options; after first accepted result. Private executive recap → chosen criteria; same hook; at scope review.
A10 - First-pathway measurement sheet	Worksheet / customer onboarding	C → fillable baseline/exceptions/staff-burden sheet; “What will we compare?”; measurement sheet; at kickoff. Private email → blank sheet only; same hook; before data collection.
A11 - Change-request decision card	Worksheet / customer onboarding	C → owner/test/clinical impact/approval card; “Who signs this change off?”; checklist; before request intake. Private email → completed non-sensitive fields; same hook; after triage.
A12 - Accepted-workflow log	Proof / private customer review	C → dated accepted/open ledger, no marketing claims; “What is actually accepted?”; activation plan; each review. Private executive report → change-only excerpt; same hook; monthly.
A13 - Measured improvement record	Proof / private customer review	C → before/after table with denominator and burden; “What moved and what did not?”; measurement sheet; after data review. Public use only after buyer approval.
A14 - Your first review agenda	Email / customer email	C email → metric/exceptions/feedback/decision agenda; “What needs attention this week?”; activation plan; before first review. CRM delivery task → owner and date; same hook; at send.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
A15 - A request ready for test	Email / customer email	C email → change, test instructions, unresolved approval; “What can you safely test now?”; change checklist; when test build ready. Private review → summary card; same hook; after feedback.
A16 - Clinical-IT issue huddle	Community / private customer meeting	C meeting → one issue, owner, clinical impact and decision; “What blocks acceptance?”; activation guide; when issue arises. Private notes → action record; same hook; same day.
A17 - Cross-site learning exchange	Community / private customer meeting	C meeting → compare process lessons without patient details; “What transferred between sites?”; next-pathway options; only with group consent. Private summary → approved lessons; same hook; after exchange.

LOFU cards - L01-L17

Public customer stories stay gated on an actual result, customer permission and a review of what can be disclosed. Private reference conversations need a separate explicit customer agreement. Do not turn the founder-authored Medplum case study into an independent customer endorsement.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
L01 - The buyer's permission comes first	Post / LI	LI → three proof requirements, with no customer name; “What makes a hospital story usable?”; approved stories/method; Tue 09. W → evidence-policy sidebar; same hook; proof page; after review.
L02 - A pathway another leader can inspect	Post / LI	LI → one measured step with baseline and limits; “Could a peer verify this result?”; case study; only after buyer approval. D → relevant excerpt to named peer; same hook; study; after permission.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
L03 - What we changed after feedback	Post / LI	LI → dated request/test/acceptance sequence, anonymized or approved; “What changed because a hospital asked?”; approved stories; after consent. W → fuller timeline; same hook; proof page; after fact review.
L04 - Customer pathway walkthrough	Short video / LI with customer	LI → staff-approved clip with synthetic/non-identifying visuals and measured task; “What changed in this task?”; case study; only after filming and consent. W → captioned accessible version; same hook; study; after approval.
L05 - What a reference actually verified	Short video / LI video	LI → founder shows denominator, timeframe, remaining issues; “What did the evidence prove?”; approved stories; after a permissioned result. W → transcript and table; same hook; proof page; after QA.
L06 - Why we chose the first pathway	Short video / LI with customer	LI → approved owner explains selection, not universal benefit; “Why this pathway first?”; case study; after explicit consent. D → short excerpt for relevant buyer; same hook; study; after permission.
L07 - How to read a hospital software case study	Blog/SEO / W	W → distinguish contract, live site, accepted workflow and result; “What exactly does 'live' mean?”; proof method; after editorial review. LI → four-definition carousel; same hook; proof page; following Tue.
L08 - From one hospital pathway to another	Blog/SEO / W	W → compare transferred vs redesigned steps in a permissioned story; “What repeats across sites?”; case study; after second-site evidence. D → scoped excerpt; same hook; study; after buyer approval.
L09 - What changed in a claim handoff	Blog/SEO / W	W → show actual claim-process metric and separate identified from collected; “Did the money arrive?”; case study; after finance verification and consent. LI → one chart with limits; same hook; study; following Thu.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
L10 - Reference-story evidence checklist	Lead magnet / W and partner	W → baseline/denominator/site/date/permission checklist; “Would this claim survive a buyer's questions?”; download; when built. Partner → neutral worksheet; same hook; checklist; after partner acceptance.
L11 - Peer evaluation guide	Lead magnet / W and founder introduction	W → questions about burden, support and change governance; “What should you ask a reference?”; guide; when built. D → send guide before approved reference call; same hook; guide; after reference consent.
L12 - Buyer-approved measured case study	Proof / W and approved sharing	W → full approved method/result/limits; “What changed for this pathway?”; read study; only after documented sign-off. LI → one authorized chart; same hook; study; following Tue.
L13 - Reference conversation	Proof / private founder introduction	D → brief intro with agreed scope and availability; “Which implementation question needs a peer answer?”; request reference; only after customer says yes. CRM → record reference permission and outcome; after call.
L14 - Share the method, not a claim	Email / customer email	C email → ask customer to review a factual draft, no request to endorse; “Does this reflect your experience?”; story request; after actual result. CRM → permission task; same hook; after response.
L15 - A peer may have this problem	Email / customer email	C email → optional forwardable note with approved result and limits; “Would this help a peer facing the same handoff?”; approved study; only after customer consent. D → founder follow-up to an introduced peer, not mass send; same hook; study; after intro.

ID - content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing
L16 - Peer operator exchange	Community / private operator introduction	Private meeting → implementation lessons and open questions; “What would you ask a hospital that tested this?”; reference request; after explicit permission. Private summary → approved non-confidential lessons; same hook; after meeting.
L17 - Hospital-association learning session	Community / association if arranged	Partner event → pathway method and permitted customer lesson; “How was one handoff measured?”; approved stories; only after host, speaker and date confirmed. W → approved recap; same hook; proof page; after session.

Part 3 - Traffic building system

Phase 1, weeks 1-2 - first relevant visitors. Gate zero is a reliable lead destination: confirm the deployed demo route, repair or choose a verified receipt path, and place a human owner in the CRM. In a five-hour week: 25 minutes daily checks, 30 minutes Monday review, 30 minutes monthly-review reserve, 60 minutes to build a 10-organization research list, 75 minutes for five tailored introduction/outreach attempts, 60 minutes for one founder post and its short message adaptation, and 20 minutes for replies and source capture (**300 minutes**). Ask each conversation how they found MediMind. The first “100 people” is a **stretch visibility test**, counted as unique public marketing-site visitors or identifiable relevant recipients, not proof of 100 qualified buyers; do not add email impressions, pageviews, and social reach into one number. **Readiness to phase 2:** at least one accepted demo request and one held qualified demo from a distinct organization, with source and next action logged; if zero after two weeks, inspect delivery first, then message/target fit. Engineering repairs and the sales meeting itself sit outside the marketing five hours.

Phase 2, weeks 3-8 - repeat the signal. Keep the top two sources of qualified conversations, publish one reusable pathway explainer per two weeks, and adapt it to one post, one direct outreach note, and one follow-up. Give every externally shared asset an ID and source tag. Add a small permission-based email follow-up only after opt-in and unsubscribe handling work; send no automated sequence to cold contacts by assumption. Each ordinary week: daily checks 25, weekly review 30, monthly-review reserve 30, account-specific outreach/introduction requests 75, content/adaptation 80, community/editor/partner contribution 30, and replies/CRM 30 minutes = **300**. **Readiness to phase 3:** a working accepted-submission path; at least four cumulative held qualified demos by week 8; a repeatable route from two sources to held meetings; an evaluation owner and readiness question in each serious account. Four is a planning gate, not a performance claim.

Phase 3, month 3 onward - compound selectively. By day 90, use the same five-hour ceiling to refresh the best public page, make one targeted partnership/editor contribution per month, and keep direct outreach and CRM follow-up. Ordinary-week allocation: daily checks 25, Monday review 30, monthly-review reserve 30, named-account outreach/follow-up 75, best-page or clip reuse 60, partner/editor contribution 40, and replies/CRM cleanup 40 minutes = **300**. Paid tests are optional **only after** there is an approved budget, consent/measurement review, a functioning form, enough qualified traffic to learn, and a credible landing page; their setup replaces the 60-

minute creative slot. Protect the monthly deep-dive week by reducing creative output. A partner webinar or trade article replaces, rather than adds to, that week's content slot. **Readiness to scale:** the 12-demo scenario is met or there is a clear upward trend in unique held qualified demos from several buying organizations, three readiness-approved evaluations are plausible from named accounts, and delivery capacity exists. If paid cost per held qualified demo is unknown or too high against deal economics, stop the test. Month 6 planning remains conditional on measured conversion and real commercial capacity.

Part 4 - Weekly distribution routine

The marketing clock is **300 minutes total**. The 25 daily tracking minutes, 30-minute weekly review (including the 15-minute report), and 30-minute weekly reserve for the monthly two-hour deep dive leave 215 average minutes for distribution and creative work. In a monthly review week, the full 120 minutes replaces that reserve and the creative slot shrinks accordingly. Demo delivery, hospital workshops, engineering, and implementation are additional work.

Day	Task	Channel	Minutes in ordinary week	Asset used	Metric
Monday	5-minute check; 30-minute pipeline/reports review; identify three distinct organizations and request one relevant introduction	CRM/warm intro	65	Account note + one pathway question	Qualified replies and next actions
Tuesday	5-minute check; publish one founder post; send two tailored follow-ups where a relationship or appropriate contact basis exists	LinkedIn/direct	65	Current pathway card, adapted to post/message	Relevant comments, replies, tagged visits
Wednesday	5-minute check; make one useful contribution in a verified hospital forum or to an editor; answer replies	Community/OPA	45	Same pathway evidence or FAQ	Named conversations, not likes
Thursday	5-minute check; refresh one page or short proof/process asset; send two precise outreach notes	SEO/direct	65	Explainer or comparison, current version only	Qualified visits and accepted demo requests

Day	Task	Channel	Minutes in ordinary week	Asset used	Metric
Friday	5-minute check; reconcile CRM sources and log follow-up decisions; set next week's single content angle	CRM/analytics	30	UTM/CRM sheet	Held qualified demos and scheduled follow-ups
Flexible	Monthly deep-dive reserve, 30 minutes amortized; use the full 120 in review week and cut 90 minutes of content/outreach that week	Measurement	30 average	Monthly report	Biggest qualified-demo leak
Total			**300**		

One indispensable action: follow through on a named hospital buyer's next agreed step, or secure one precise warm introduction when no buyer follow-up is due. If the week underperforms, first check whether a submitted request was accepted and followed up; next check message-to-buyer fit, then the channel. Suggested Tuesday/Thursday morning windows from the source library are test slots, not local evidence. Any post or email timing should be compared against actual Georgian buyer response.

Part 5 - Distribution benchmarks and growth scorecard

These are **scenario checkpoints for learning**, not observed baselines, forecasts, or verified channel norms. “N/A” means no valid denominator or channel not active. Do not turn a tiny sample into a percentage-based success claim. The library's email-open, outreach-reply and webinar/paid claims have not been verified for this market.

Stage	Metric	Week 4 planning check	Month 3 planning check	Month 6 conditional check	How to measure
TOFU	Unique public-site visitors per trailing month	100	300	600	GA4 marketing-site users, excluding internal traffic; unknown now

Stage	Metric	Week 4 planning check	Month 3 planning check	Month 6 conditional check	How to measure
TOFU	Median relevant LinkedIn post impressions	Record baseline; 100-200 exploratory	200-400 exploratory	Grow only if qualified traffic follows	Native post analytics, 7-day window
TOFU	LinkedIn engagement rate	Record baseline; no pass/fail below 100 impressions	Investigate 1-3% as library-only comparator	Favor qualified comments over rate	(Reactions + comments + reposts + meaningful clicks) / impressions
MOFU	Permission-based email list size	0-10 scenario	20-40	50-100 if opt-in proves useful	Deduplicated consenting contacts; not all CRM contacts
MOFU	Email open rate	N/A until sent; library says 20-30% average	Treat as directional only	Compare with clicks/replies	Unique opens / delivered; client privacy can distort opens
BOFU	Tailored cold outreach reply rate	Record counts; 10-15% library comparator only	Compare by buyer segment after enough sends	Keep if qualified replies follow	Human replies / delivered tailored messages; exclude bots
MOFU	New inbound leads per month from organic	0-2 scenario	3-5	5-8 if evidence supports	Accepted forms or manually logged inbound distinct organizations with organic first touch
BOFU	Leads from paid	N/A; 0 GEL spend	N/A unless gated test approved	Scenario only after budget; no target yet	Accepted leads plus actual spend and held demos; do not count clicks as leads
LOFU	Referral/word-of-mouth leads	Record source, possibly 0	1-2 scenario	2-3 conditional	Ask on first call; de-duplicate buying organizations
TOFU/MOFU	Community contributions that become conversations	1-2 useful answers; no conversion promise	2-4 named conversations/month	Continue only if qualified	Manual source note linked to actual counterpart

Stage	Metric	Week 4 planning check	Month 3 planning check	Month 6 conditional check	How to measure
North star	**Cumulative held qualified demos from distinct hospital buying organizations**	**2 scenario**	**12 scenario**	Set only after day-90 evidence	CRM held date + qualification fields + distinct organization ID

The leading indicator is **held qualified hospital demos**, counted once per buying organization in this 90-day acquisition scenario. A booked calendar slot, repeat meeting with the same group, webinar registration, or anchor-account expansion discussion is tracked separately. More traffic is useful only when it produces these meetings.

Sources and limits: shared writer brief, research addendum, source demo route. Channel percentages and posting windows in the library were supplied as prompts, not independently validated for this buyer. Handoff to Prompt 6: use the same content IDs, distinct-organization north star, form acceptance gate, and 300-minute weekly ceiling.

Source: Original files/Complete B11 HTML library/Editable text/p2-06-tracking.md. Historical wording preserved; not a new approval or factual revalidation.

Part 2, Prompt 6 - Tracking, Mapping and Implementation

Inputs and status (evidence snapshot 22 September 2026). The funnel has five stages: TOFU problem recognition, MOFU evaluation, BOFU held qualified pathway demo and scoped evaluation decision, AFU activation/retention after a customer agreement, and LOFU permissioned advocacy. The 85 stable content IDs T01-L17 and page IDs P01-P11 are defined in `working/content-catalog.json`; the distribution priorities and 300-minute weekly routine are in Prompt 5. Primary conversion: **one held qualified 25-minute demo per distinct hospital buying organization**. The 90-day planning scenario is 12 such demos, then three readiness-approved evaluations and one *new* paying buying organization as stretch assumptions. Baselines, marketing analytics, CRM, opted-in email platform and founder technical comfort are unverified. The website is a React app; `/request-demo` exists, but source inspection found a three-step form whose submit handler waits, logs data locally, and shows a receipt message without a backend request. This is a code finding, not a deployed-site test or proof of lost leads. It is the first implementation risk. Assume a founder can use a spreadsheet; one-time tagging/form engineering needs a technical owner. No analytics, CRM, email, pixels or external pages were installed by this document.

Read the metrics correctly. The source library supplies benchmark percentages for broad B2B funnels. They are **unverified library comparators**, not MediMind's historical rates, Georgian hospital norms, or automatic targets. Below, absolute gates protect the launch; percentages become directional only after a meaningful denominator. A zero denominator is **N/A**, never 0% or red. Report both numerator and denominator and label any rate from fewer than 20 qualified organizations or fewer than 100 relevant page sessions **provisional**. Site traffic, LinkedIn impressions, booked calls, held meetings, trials and paying organizations are different events.

Part 1 - Funnel tracking system

Event and entity definitions. A public-site visit is a unique GA4 user/session on the marketing site, after internal-traffic filtering. An accepted lead has a successful backend receipt and CRM record; a button click or client-side “thank you” state is not one. A qualified hospital organization is a deduplicated legal buying group with a relevant cross-department need, a decision/influence route, a candidate owner, and a plausible evaluation path. A *held qualified demo* has an actual meeting date, the defined qualification fields, and a unique organization ID; reschedules, repeats, anchor-account conversations and two attendees from the same group do not inflate the

north star. An evaluation is readiness-approved only after scope, owners, access, baseline and stop criteria are agreed. Paid means a signed commercial agreement **and first revenue recognized/collected as the business defines it**, with contracted versus received amounts separately labelled. Store organization identity and contact details in an access-controlled CRM, never in GA4 parameters.

Stage	Metric and what it tells you	How to measure	Green	Amber	Red	Fix
TOFU	Relevant content reach	LI impressions and named target-account interactions by asset ID	Qualified interaction rises over 4 weeks	Reach rises but no relevant interaction	No relevant interaction after 4 consistent posts	Narrow topic and audience
TOFU	Engagement rate	Meaningful LI interactions / impressions	Direction improves and target buyers respond	Rate only, buyer fit unclear	0 relevant replies/comments across 4 posts	Rewrite hook around observed problem
TOFU	Marketing-site visitors from content	GA4 sessions by external source/content ID	Source yields a qualified inquiry	Visits only	Tagged external links produce no visits after QA	Check links, placement, message
TOFU	Accepted new leads	Backend-confirmed, deduplicated organization records	≥1 new qualified inquiry/week in scenario	Inquiry exists but fit uncertain	0 after verified traffic and working form	Inspect form and CTA, then channel
TOFU	Cost per lead, if paid	Actual spend / accepted leads	Within approved acquisition economics	Cost unknown or sample tiny	Spend with no accepted lead after pre-set test limit	Stop/pause test; inspect tracking
MOFU	Lead → demo request	Unique organizations requesting / eligible new-lead organizations	Rising with named buyer need	Flat, provisional	No requests after five qualified conversations	Improve proof and invitation
MOFU	Email opens	Unique opens / delivered opted-in emails	Directional only; library says 20-30% broadly	Low opens but clicks/replies exist	No opens and no clicks after deliverability check	Check consent/delivery, subject; do not over-read pixel

Stage	Metric and what it tells you	How to measure	Green	Amber	Red	Fix
MOFU	Email clicks	Unique clickers / delivered	Useful replies or demo requests; library says 2-5% broadly	Clicks without action	No click after relevant sends	Match CTA to promise
MOFU	Webinar registration	Accepted registrations / unique webinar page sessions	Event-specific baseline improves	Visits but date/form unclear	No accepted registration after launch QA	Fix event details or form
MOFU	Webinar attendance	Unique live attendees / accepted registrants	Attendance and useful questions	Registrations but many no-shows	Event did not run or attendee data absent	Reminder and relevance; N/A before event
MOFU	Case-study visits and CTA clicks	Verified P03 sessions and CTA events	Buyers inspect a permissioned proof asset	Visits without demo moves	No valid case study exists	Keep placeholder; do not fabricate proof
MOFU	Lead capture → demo request time	Median days, CRM timestamps by organization	Shorter with fit intact	Long but next action owned	Contact has no owner/next date	Assign follow-up
BOFU	Demo page conversion	Accepted demo requests / unique P05 sessions	Improving with qualified fit	Library 3.8% median is only a comparator	Form acceptance fails at all	Repair backend first; then simplify friction
BOFU	Held demo → approved evaluation	Unique approved evaluations / unique held qualified-demo organizations	Scenario reaches 3 from 12 by day 90	Scope questions open	No evaluation owner after several demos	Refine qualification and scope
BOFU	Sandbox signup → activation	Activated synthetic workspaces / accepted sandbox accounts	Directional; library says 60%+ for mature trial	Opened but no completed pathway	Signup flow or pathway broken	Improve guided first session

Stage	Metric and what it tells you	How to measure	Green	Amber	Red	Fix
BOFU	Time to first value	Median from accepted access to first synthetic pathway map	Within first guided session, proposed	Later than first session	Never reaches map	Remove setup steps; review support
BOFU	Sandbox → paid	New paying organizations / trial organizations with completed observation window	Measured, not benchmark-driven	N/A until launched and matured	Trial creates no buyer evaluation	Reassess fit/offer; 15-30% library figure unverified here
BOFU	Conversion-page conversion	Accepted form completions / unique relevant sessions on P04/P05/P11 separately	Page-specific improvement	Traffic without enough denominator	False success or broken handler	Fix event acceptance and page first
AFU	Month-one retention	Active contracted organizations at day 30 / contracted organizations eligible for day-30 check	Customer retained and using agreed workflow	Usage unclear	Exit or non-use attributable to unresolved issue	Customer-success review; library 46.9% not local norm
AFU	Activation rate	Organizations accepting first agreed pathway / eligible signed organizations	First pathway accepted with sign-off	Work open with owner	No sign-off or unmanaged safety issue	Fix delivery and acceptance criteria
AFU	Expansion revenue	Additional recurring amount signed/recognized, separate from base	Evidenced growth without hidden cost	Discussion only	Churn/contraction	Address value and support

Stage	Metric and what it tells you	How to measure	Green	Amber	Red	Fix
AFU	Satisfaction/NPS	One standardized post-milestone question + denominator	Positive qualitative signal with specific reason	Mixed/too few answers	Repeated unresolved criticism	Close loop; do not calculate NPS on tiny sample
AFU	Support burden	Open/aged tickets per active organization, severity	Issues owned, safety escalated	Backlog growing	Critical issue without owner	Assign and escalate
LOFU	Referral rate	New qualified buying organizations explicitly introduced / eligible satisfied customers	Permissioned introductions emerge	Satisfied users but no ask yet	Dissatisfaction or pressure to refer	Improve customer experience first
LOFU	Review submission rate	Authentic posted reviews / customers eligible and asked with consent	A real detailed review	No public review, customer satisfied	Incentivized/inauthentic request	Stop request, clarify consent
LOFU	Case-study participation	Customers consenting to measured story / eligible customers with verified result	One complete authorized story	Evidence/permission pending	Claim drafted without approval	Hold publication
LOFU	Word-of-mouth leads	Unique buyer organizations naming a peer/private share	Source noted in CRM	Source “unknown” but inquiry real	Repeated unattributed traffic with no source question	Ask “How did you hear?”

Library comparator note. The supplied prompt also lists homepage lead capture 2-5%, webinar page 25-45%, trial-to-paid 15-30%, trial activation 60%+, LinkedIn engagement 1-3%, outreach replies 10-15%, referrals 20-30% of new customers, and other “top performer” rates. These have no cited research method in the library. Use them only to form questions after a real local baseline; never make a red/green decision from them alone.

Part 2 - Tool setup guide

The current stack is not confirmed, so the tools below are **recommendations**. No-code work should be possible for daily use; engineering is needed for reliable React-route events and accepted form submissions. Use the minimum set that records a real lead and next action before adding behavior tracking.

Priority / category	Recommended tool; what it tracks	Plain setup and acceptance check	Free alternative
Must now - website analytics	GA4 for public marketing page visits, source, CTA intent and accepted conversion	Create account/property/web stream, choose Georgia reporting timezone, add one tag to public marketing routes, verify in Realtime. Check React history-route page views once (avoid duplicate manual pageviews). Record `cta_click` with safe `content_id` and `destination_id` for each live public CTA as **intent**, not conversion. Register distinct accepted events for `lead_magnet_download`, `email_signup`, `webinar_registration_accepted`, `demo_request_accepted`, `sandbox_signup_accepted`, and `sandbox_pathway_completed`; later paid status comes from reconciled CRM and may be represented by a carefully designed offline `paid_customer_recorded` event, never a browser button. Send success events only after the real system accepts them. Build a simple path exploration from landing → P05 → accepted request. Never send names, email, free text, patient information, or organization IDs.	Privacy-conscious aggregate server log or manual source/accepted-form tally until analytics is ready; no false precision.

Priority / category	Recommended tool; what it tracks	Plain setup and acceptance check	Free alternative
Must now - CRM	HubSpot default deal pipeline **or** a controlled spreadsheet for unique organizations, source ID, stage, owner, next action, held demo, evaluation and payment	Make one organization record per buying group; use fields `first_source`, `latest_source`, `content_id`, `qualification`, `demo_held_date`, `evaluation_readiness`, `next_owner`, `next_date`. Start from the default pipeline or a sheet; verify one test request creates a record and task. Do not assume custom-pipeline creation or automation is free.	Protected spreadsheet with a weekly follow-up filter and manual tasks.
Must now when sending permissioned email - email platform	Mailchimp or equivalent for opted-in delivery, unsubscribes, opens and clicks	Import only consenting contacts; segment interested prospects versus customer onboarding; write a short delivery/one-follow-up sequence before automation; test unsubscribe, delivery and link tracking. Platform automation and plan limits vary. Cold one-to-one outreach stays in the CRM, not an opted-in nurture list.	Manual permission-based email plus consent log and reply counts.
Must now - LinkedIn analytics	Native personal-post analytics for impressions, reactions, comments, reposts and relevant conversations	Record asset ID, date, impressions at seven days, meaningful buyer interactions, and tagged external-link visits weekly; use an external UTM link on an actual CTA, not on LinkedIn's internal features.	Weekly manual post log.
Month 2 only after privacy review - heatmaps/recordings	Microsoft Clarity, public marketing pages only, for drop-off and usability	Limit tag placement to approved public routes. Verify masking, consent handling, page exclusions, and a synthetic test recording before viewing any real session. Never install on EMR, clinical app, investor/staff private routes or patient-data pages; disable if exclusion cannot be guaranteed.	Manual moderated usability test with a synthetic form; no recording script.

Priority / category	Recommended tool; what it tracks	Plain setup and acceptance check	Free alternative
When paid is approved - ad tracking	Google Ads/LinkedIn Campaign Manager as applicable, spend and accepted conversions	Create one conversion for true accepted lead, use one tag implementation, test source/UTM and deduplication; do not count button click or client-side success as a conversion. Check consent and public-route audience exclusions before any retargeting.	CRM campaign ID and manual spend/held-demo log for tiny tests.
Month 2 or later, evidence-ready - review/proof tracking	G2/Capterra/Trustpilot listing where relevant, permissioned reviews and proof requests	Claim a listing only after verifying official product identity and public claims; ask a satisfied customer for an honest review after an agreed milestone, not automatically on day 30. Log request/permission/publication and unresolved criticism.	Internal permission and reference log; publish no public review claim until one exists.

Fresh official setup and limitation references: GA4 web setup, GA4 enhanced page-view behavior, GA4 recommended lead events, GA4 PII policy, HubSpot pipeline limits, Mailchimp automation and plan caveat, Mailchimp open tracking limitations, LinkedIn post analytics, Clarity setup, Clarity masking, Google Ads conversion setup.

Part 3 - UTM tracking system

A UTM tag is a short set of words added to a **link shared outside the site** so analytics can tell whether a visitor arrived from a founder post, email, partner or ad and which piece sent them. It does not identify a person or prove that channel caused the purchase. Keep identifiable names, hospital names, email addresses, clinical details and free-text answers out of URLs and analytics. Google explains how manually tagged campaign dimensions appear in GA4 here.

Naming convention. Lowercase ASCII and underscores only. `utm_source` = platform or sender class (`linkedin`, `email`, `partner`, `hospital_forum`, `direct_outreach`, `google`, `referral`); `utm_medium` = `social`, `email`, `partner`, `community`, `outreach`, `cpc`, or `referral`; `utm_campaign` = `one_patient_2026q4` for this campaign, never a person/account name; `utm_content` = catalog ID plus version (`t01_v1`, `m05_v1`) and optional approved variation (`_a`, `_b`). Source and medium describe *arrival*; content identifies the specific creative. Preserve both first-touch and latest external touch in CRM; if the buyer says a colleague forwarded it privately, record “self-reported peer forward” too. Do not overwrite a first source with an internal click.

Destinations. P01 `/pathway-assessment`; P02 `/hospital-os`; P03 `/case-study`; P04 `/webinar`; P05 `/request-demo`; P06 `/early-partner`; P07 `/compare`; P08 `/activation`; P09 `/referrals`; P10 `/evidence`; P11 `/sandbox`. These are **planned path names** except P05, the existing route. Even P05 must not be promoted until real backend acceptance, receipt and scheduling are checked. Do not share a planned URL until its page, form, analytics and privacy behavior work. For organic search landings and internal navigation, the URL is clean, without UTMs. For private customer documents, use a controlled link and CRM source note rather than a public tracking URL.

Complete 85-piece channel-link library. A row with a public external channel supplies a ready link structure; “private/none” is a deliberate no-UTM rule. The table is keyed to the exact catalog ID and its planned destination, with one line for every catalog piece. The “Native…” line shows the **clean destination URL** for search, direct access or internal navigation; it does not mean an externally shared LinkedIn, email, outreach, community or partner CTA should use that clean link. Use the matching tagged link on every controllable external CTA. Where a second public distribution channel is approved, use the same destination and ID with its own listed `source/medium`; this keeps the content identity while distinguishing the sender. Links are **planning URLs**, not a statement that the page exists.

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T01 - One patient, seven handoffs	li + dir	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t01_v1` dir: `https://medimind.md/pathway-assessment?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=t01_v1`	T01 source/medium visits and accepted downstream action; P01 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T02 - The hidden cost of searching	li + email	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t02_v1` email: `https://medimind.md/pathway-assessment?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=t02_v1`	T02 source/medium visits and accepted downstream action; P01 destination
T03 - When two reports disagree	li + dir	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t03_v1` dir: `https://medimind.md/pathway-assessment?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=t03_v1`	T03 source/medium visits and accepted downstream action; P01 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T04 - The duplicate-entry trail	li	Native Short video / Founder LinkedIn video: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t04_v1`	T04 source/medium visits and accepted downstream action; P01 destination
T05 - Three calls for one result	li + dir	Native Short video / Founder LinkedIn video: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t05_v1` dir: `https://medimind.md/pathway-assessment?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=t05_v1`	T05 source/medium visits and accepted downstream action; P01 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T06 - The spreadsheet behind the dashboard	li	Native Short video / Founder LinkedIn video: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t06_v1`	T06 source/medium visits and accepted downstream action; P01 destination
T07 - How to map a patient handoff	li	Native Blog/SEO / Website search: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t07_v1`	T07 source/medium visits and accepted downstream action; P01 destination
T08 - Why hospital reports disagree	li	Native Blog/SEO / Website search: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t08_v1`	T08 source/medium visits and accepted downstream action; P01 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T09 - What a delayed claim does and does not mean	li	Native Blog/SEO / Website search: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t09_v1`	T09 source/medium visits and accepted downstream action; P01 destination
T10 - Patient-pathway friction worksheet	dir	Native Lead magnet / Website and founder outreach: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/pathway-assessment?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=t10_v1`	T10 source/medium visits and accepted downstream action; P01 destination
T11 - Five-minute handoff check	li	Native Lead magnet / Website and founder outreach: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t11_v1`	T11 source/medium visits and accepted downstream action; P01 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T12 - Founder-built scope trace	li	Native Proof / Website and founder LinkedIn: `https://medimind.md/hospital-os` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/hospital-os?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t12_v1`	T12 source/medium visits and accepted downstream action; P02 destination
T13 - Synthetic handoff before/after	li	Native Proof / Website and founder LinkedIn: `https://medimind.md/hospital-os` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/hospital-os?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t13_v1`	T13 source/medium visits and accepted downstream action; P02 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T14 - Where the work repeats	email + dir	Native Email / Opt-in email: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/pathway-assessment?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=t14_v1` dir: `https://medimind.md/pathway-assessment?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=t14_v1`	T14 source/medium visits and accepted downstream action; P01 destination
T15 - A report you had to rebuild	email + dir	Native Email / Opt-in email: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/pathway-assessment?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=t15_v1` dir: `https://medimind.md/pathway-assessment?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=t15_v1`	T15 source/medium visits and accepted downstream action; P01 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
T16 - Hospital-operator discussion prompt	forum + li	Native Community / Relevant hospital-leadership forum: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) forum: `https://medimind.md/pathway-assessment?utm_source=hospital_forum&utm_medium=community&utm_campaign=one_patient_2026q4&utm_content=t16_v1` li: `https://medimind.md/pathway-assessment?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=t16_v1`	T16 source/medium visits and accepted downstream action; P01 destination
T17 - Clinical-operations partner note	partner	Native Community / Professional association or introduction: `https://medimind.md/pathway-assessment` (clean URL; no UTM for organic search or internal navigation) partner: `https://medimind.md/pathway-assessment?utm_source=partner&utm_medium=partner&utm_campaign=one_patient_2026q4&utm_content=t17_v1`	T17 source/medium visits and accepted downstream action; P01 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
M01 - Fix a screen or fix the pathway?	li + dir	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/compare` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/compare?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m01_v1` dir: `https://medimind.md/compare?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m01_v1`	M01 source/medium visits and accepted downstream action; P07 destination
M02 - What parallel evaluation actually requires	li + email	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/early-partner?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m02_v1` email: `https://medimind.md/early-partner?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=m02_v1`	M02 source/medium visits and accepted downstream action; P06 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
M03 - AI assists; clinicians decide	li	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/hospital-os` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/hospital-os?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m03_v1`	M03 source/medium visits and accepted downstream action; P02 destination
M04 - Three options on one whiteboard	li	Native Short video / Founder LinkedIn video: `https://medimind.md/compare` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/compare?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m04_v1`	M04 source/medium visits and accepted downstream action; P07 destination
M05 - What happens in 25 minutes	li + dir	Native Short video / Founder LinkedIn video: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/request-demo?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m05_v1` dir: `https://medimind.md/request-demo?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m05_v1`	M05 source/medium visits and accepted downstream action; P05 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
M06 - A result becomes a task	li	Native Short video / Founder LinkedIn video: `https://medimind.md/hospital-os` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/hospital-os?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m06_v1`	M06 source/medium visits and accepted downstream action; P02 destination
M07 - How to evaluate hospital software without a risky cutover	li	Native Blog/SEO / Website search: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/early-partner?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m07_v1`	M07 source/medium visits and accepted downstream action; P06 destination
M08 - Hospital OS versus overlay versus current vendor change	dir	Native Blog/SEO / Website search: `https://medimind.md/compare` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/compare?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m08_v1`	M08 source/medium visits and accepted downstream action; P07 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
M09 - What FHIR R4 changes in hospital work	li	Native Blog/SEO / Website search: `https://medimind.md/hospital-os` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/hospital-os?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=m09_v1`	M09 source/medium visits and accepted downstream action; P02 destination
M10 - Parallel-evaluation readiness checklist	dir	Native Lead magnet / Website and founder outreach: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/early-partner?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m10_v1`	M10 source/medium visits and accepted downstream action; P06 destination
M11 - Three-option decision worksheet	dir	Native Lead magnet / Website and founder outreach: `https://medimind.md/compare` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/compare?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m11_v1`	M11 source/medium visits and accepted downstream action; P07 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
M12 - Documented domain map	dir	Native Proof / Technical diligence and website: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/evidence?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m12_v1`	M12 source/medium visits and accepted downstream action; P10 destination
M13 - Live-pathway evidence template	dir	Native Proof / Website and founder follow-up: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/evidence?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m13_v1`	M13 source/medium visits and accepted downstream action; P10 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
M14 - The change-risk question	email + dir	Native Email / Opt-in email: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/early-partner?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=m14_v1` dir: `https://medimind.md/early-partner?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m14_v1`	M14 source/medium visits and accepted downstream action; P06 destination
M15 - The current-vendor alternative	email + dir	Native Email / Opt-in email: `https://medimind.md/compare` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/compare?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=m15_v1` dir: `https://medimind.md/compare?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=m15_v1`	M15 source/medium visits and accepted downstream action; P07 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
M16 - Peer discussion on evaluation design	private + email	Native Community / Invited operator roundtable: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only email: `https://medimind.md/early-partner?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=m16_v1`	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
M17 - IT and clinical review session	partner	Native Community / Professional association or partner session: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) partner: `https://medimind.md/early-partner?utm_source=partner&utm_medium=partner&utm_campaign=one_patient_2026q4&utm_content=m17_v1`	M17 source/medium visits and accepted downstream action; P06 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
B01 - What your 25-minute demo will settle	li + dir	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/request-demo?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=b01_v1` dir: `https://medimind.md/request-demo?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b01_v1`	B01 source/medium visits and accepted downstream action; P05 destination
B02 - Match price, define total cost	li + dir	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/early-partner?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=b02_v1` dir: `https://medimind.md/early-partner?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b02_v1`	B02 source/medium visits and accepted downstream action; P06 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
B03 - The honest go/no-go test	li + email	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/request-demo?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=b03_v1` email: `https://medimind.md/request-demo?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=b03_v1`	B03 source/medium visits and accepted downstream action; P05 destination
B04 - Demo in sixty seconds	li	Native Short video / Founder LinkedIn video: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/request-demo?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=b04_v1`	B04 source/medium visits and accepted downstream action; P05 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
B05 - Parallel beside current work	li	Native Short video / Founder LinkedIn video: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/early-partner?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=b05_v1`	B05 source/medium visits and accepted downstream action; P06 destination
B06 - Who should join the first call?	li + email	Native Short video / Founder LinkedIn video: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/request-demo?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=b06_v1` email: `https://medimind.md/request-demo?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=b06_v1`	B06 source/medium visits and accepted downstream action; P05 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
B07 - Questions to ask before a hospital OS demo	li	Native Blog/SEO / Website search: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/request-demo?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=b07_v1`	B07 source/medium visits and accepted downstream action; P05 destination
B08 - What happens after a parallel evaluation	dir	Native Blog/SEO / Website search: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/early-partner?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b08_v1`	B08 source/medium visits and accepted downstream action; P06 destination
B09 - What current-software price match should include	dir	Native Blog/SEO / Website search: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/early-partner?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b09_v1`	B09 source/medium visits and accepted downstream action; P06 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
B10 - Pathway demo preparation card	email	Native Lead magnet / Demo confirmation email and website: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/request-demo?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=b10_v1`	B10 source/medium visits and accepted downstream action; P05 destination
B11 - Evaluation decision sheet	dir	Native Lead magnet / Founder sales follow-up: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/early-partner?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b11_v1`	B11 source/medium visits and accepted downstream action; P06 destination
B12 - Verified implementation timeline	dir	Native Proof / Founder sales follow-up: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/case-study?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b12_v1`	B12 source/medium visits and accepted downstream action; P03 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
B13 - Buyer-approved workflow result	dir	Native Proof / Website and founder sales follow-up: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/evidence?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b13_v1`	B13 source/medium visits and accepted downstream action; P10 destination
B14 - Your pathway before our demo	email	Native Email / Demo-confirmation email: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/request-demo?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=b14_v1`	B14 source/medium visits and accepted downstream action; P05 destination
B15 - What we will compare	email	Native Email / Founder follow-up email: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/early-partner?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=b15_v1`	B15 source/medium visits and accepted downstream action; P06 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
B16 - Private buying-committee walkthrough	private + dir	Native Community / Private hospital meeting: `https://medimind.md/request-demo` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only dir: `https://medimind.md/request-demo?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b16_v1`	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
B17 - Procurement questions session	private + dir	Native Community / Private hospital meeting: `https://medimind.md/early-partner` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only dir: `https://medimind.md/early-partner?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=b17_v1`	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
A01 - First pathway, first baseline	private + li	Native LinkedIn post / Customer-only LinkedIn or direct update: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only li: `https://medimind.md/activation?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=a01_v1`	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A02 - What staff should report in week one	private + li	Native LinkedIn post / Customer-only update: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only li: `https://medimind.md/activation?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=a02_v1`	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
A03 - One improvement, then the next	private + li	Native LinkedIn post / Customer-only update: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only li: `https://medimind.md/activation?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=a03_v1`	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A04 - How to log a handoff issue	private	Native Short video / Customer onboarding: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A05 - Five-minute pathway review	private	Native Short video / Customer onboarding: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A06 - How a testable change becomes approved	private	Native Short video / Customer onboarding: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
A07 - Measuring repeat work without double counting	private	Native Blog/SEO / Customer knowledge base: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A08 - Running a safe pathway review	private	Native Blog/SEO / Customer knowledge base: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A09 - Selecting the second pathway	private	Native Blog/SEO / Customer knowledge base: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A10 - First-pathway measurement sheet	private	Native Lead magnet / Customer onboarding: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
A11 - Change-request decision card	private	Native Lead magnet / Customer onboarding: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A12 - Accepted-workflow log	private	Native Proof / Private customer review: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A13 - Measured improvement record	private	Native Proof / Private customer review: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A14 - Your first review agenda	private	Native Email / Customer email: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A15 - A request ready for test	private	Native Email / Customer email: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
A16 - Clinical-IT issue huddle	private	Native Community / Private customer meeting: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
A17 - Cross-site learning exchange	private	Native Community / Private customer meeting: `https://medimind.md/activation` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
L01 - The buyer's permission comes first	li	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/evidence?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l01_v1`	L01 source/medium visits and accepted downstream action; P10 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
L02 - A pathway another leader can inspect	li + dir	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/case-study?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l02_v1` dir: `https://medimind.md/case-study?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=l02_v1`	L02 source/medium visits and accepted downstream action; P03 destination
L03 - What we changed after feedback	li	Native LinkedIn post / Founder LinkedIn: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/evidence?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l03_v1`	L03 source/medium visits and accepted downstream action; P10 destination
L04 - Customer pathway walkthrough	li	Native Short video / Founder and customer LinkedIn: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/case-study?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l04_v1`	L04 source/medium visits and accepted downstream action; P03 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
L05 - What a reference actually verified	li	Native Short video / Founder LinkedIn video: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/evidence?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l05_v1`	L05 source/medium visits and accepted downstream action; P10 destination
L06 - Why we chose the first pathway	li + dir	Native Short video / Founder and customer LinkedIn: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/case-study?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l06_v1` dir: `https://medimind.md/case-study?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=l06_v1`	L06 source/medium visits and accepted downstream action; P03 destination
L07 - How to read a hospital software case study	li	Native Blog/SEO / Website search: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/evidence?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l07_v1`	L07 source/medium visits and accepted downstream action; P10 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
L08 - From one hospital pathway to another	dir	Native Blog/SEO / Website search: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/case-study?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=l08_v1`	L08 source/medium visits and accepted downstream action; P03 destination
L09 - What changed in a claim handoff	li	Native Blog/SEO / Website search: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/case-study?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l09_v1`	L09 source/medium visits and accepted downstream action; P03 destination
L10 - Reference-story evidence checklist	partner	Native Lead magnet / Website and partner sharing: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) partner: `https://medimind.md/evidence?utm_source=partner&utm_medium=partner&utm_campaign=one_patient_2026q4&utm_content=l10_v1`	L10 source/medium visits and accepted downstream action; P10 destination

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
L11 - Peer evaluation guide	dir	Native Lead magnet / Website and founder introduction: `https://medimind.md/referrals` (clean URL; no UTM for organic search or internal navigation) dir: `https://medimind.md/referrals?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=l11_v1`	L11 source/medium visits and accepted downstream action; P09 destination
L12 - Buyer-approved measured case study	li	Native Proof / Website and customer-approved sharing: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) li: `https://medimind.md/case-study?utm_source=linkedin&utm_medium=social&utm_campaign=one_patient_2026q4&utm_content=l12_v1`	L12 source/medium visits and accepted downstream action; P03 destination
L13 - Reference conversation	private + dir	Native Proof / Private founder introduction: `https://medimind.md/referrals` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only dir: `https://medimind.md/referrals?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=l13_v1`	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
L14 - Share the method, not a claim	private	Native Email / Customer email: `https://medimind.md/referrals` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared
L15 - A peer may have this problem	email + dir	Native Email / Customer email: `https://medimind.md/case-study` (clean URL; no UTM for organic search or internal navigation) email: `https://medimind.md/case-study?utm_source=email&utm_medium=email&utm_campaign=one_patient_2026q4&utm_content=l15_v1` dir: `https://medimind.md/case-study?utm_source=direct_outreach&utm_medium=outreach&utm_campaign=one_patient_2026q4&utm_content=l15_v1`	L15 source/medium visits and accepted downstream action; P03 destination
L16 - Peer operator exchange	private	Native Community / Private operator introduction: `https://medimind.md/referrals` (clean URL; no UTM for organic search or internal navigation) Private customer/meeting: no UTM; CRM touch only	Manual CRM content touch and delivery/permission milestone; tagged external visits only if a listed link is actually shared

Content piece	Channel	UTM link structure (planned; activate after destination QA)	What it tracks
L17 - Hospital-association learning session	partner	Native Community / Professional association, if arranged: `https://medimind.md/evidence` (clean URL; no UTM for organic search or internal navigation) partner: `https://medimind.md/evidence?utm_source=partner&utm_medium=partner&utm_campaign=one_patient_2026q4&utm_content=l17_v1`	L17 source/medium visits and accepted downstream action; P10 destination

Tracking rule and QA. Tag every controllable **external** link into a public funnel page. Do not append UTMs to internal site navigation, organic search results, in-app paths, private customer records or links carrying identifiers. Keep natural/direct traffic as “direct/unknown” rather than creating fake attribution. Before release, open one test link for every active source/medium, confirm that it resolves to the intended live page, appears once in GA4 source/campaign/content, and survives the React navigation. Send an end-to-end test form and check backend receipt, CRM record, owner/task, true accepted event, failure error, consent/unsubscribe where applicable, and no PII in network analytics requests or browser URL. Only then mark a link “ready.”

Part 4 - Weekly tracking routine

Daily, five minutes (Monday-Friday; 25 minutes/week). Look first at the CRM count of cumulative unique **held qualified demos** versus the 13-week pace, and at yesterday's accepted requests and owner assignment. A day with no demo is normal; a new unowned request or a form test that fails is not. If the pace is behind, inspect real accepted leads and next actions before looking at impressions. The CRM is the source of truth for the north star; GA4 is the source for site traffic. This five-minute block is included in Prompt 5's 300-minute total.

Weekly, 30 minutes Monday (includes Prompt 7's 15-minute report). In 5 minutes, record TOFU relevant reach, tagged site visits and accepted new leads; 5 minutes, MOFU lead-to-demo, permissioned email and any webinar/case asset (N/A if unlaunched); 5 minutes, BOFU held demos, evaluation decisions and sandbox first value (N/A if absent); 3 minutes, AFU activation/retention and support (N/A before customers); 2 minutes, LOFU referrals/reviews (N/A before eligible customers); 5 minutes, reconcile UTM first/latest source with self-reported source and check CRM owners/aging; 5 minutes, write the single biggest leak and next action. All rates show numerator/denominator. An absent product stage is N/A, not red.

Monthly, two hours on first Monday, replacing ordinary creative work that week. 20 minutes verify raw accepted submissions, distinct organizations and source completeness; 25 minutes compare asset ID → qualified conversations → held demos; 20 minutes compare the top two channels by *qualified* meetings, not clicks; 20 minutes inspect stage drop-offs, median lead-to-demo time and support/evaluation workload; 20 minutes reconcile month-over-month revenue and cohort eligibility; 15 minutes choose one asset to improve, one channel to reduce, and one experiment with a clear stop rule. Prompt 7's 30-minute monthly review is the summary produced inside this two-hour analysis, not additional founder time. In the first-month deep-dive week, the 120 minutes leaves 125 of the 300 weekly minutes for distribution after daily and weekly checks; no second 5-hour allocation is assumed.

Part 5 - Week-by-week implementation plan

Window	Deliverable and sequence	Acceptance gate and dependency
Week 1 - foundation	Verify the actual deployed form; implement a real accepted request and explicit failure state with an owner; create the minimal organization CRM/sheet; define IDs and link rules; install GA4 on public marketing routes only; test React pageviews and PII exclusion. Claim a review listing only if verified and low effort; it is not a gate. No broad campaign publication.	A synthetic request appears in backend/CRM exactly once, reaches the right human, records a next action, and does **not** emit an accepted event on failure. Technical work is separate from founder marketing time.
Week 2 - minimum viable funnel	Bring three priority assets into a verified path: working P05 demo page, P01 pathway worksheet/assessment if built, and T04 synthetic phone clip if filming and review capacity exists. Prepare permissioned follow-up email T14 only if opt-in exists.	Every external link resolves, source ID arrives, accepted lead is recorded, receipt is truthful, and a founder can conduct the 25-minute meeting. If P01 or T04 is late, use the verified P05 route and a plain pathway sketch; do not claim three assets are live.
Week 3 - first campaign	Add M10 readiness checklist and adapt M02 into a founder post, each subject to review and a functioning destination. Begin a 14-day campaign test **only after** P05 receipt/owner, the needed landing page, the assets, UTMs, privacy wording and actual campaign capacity pass QA.	No real patient data, approved claims, accurate 25-minute promise, and one source-to-held-demo path tested. If not ready, continue direct outreach and the worksheet; the 14-day clock has not started.
Week 4 - routine	Run Prompt 5's five-hour schedule, record each live content ID and external UTM link, and fill the first weekly scorecard. Produce B04 60-second synthetic demo preview only if capacity and P05 QA allow.	No missing source or unowned request; 2 cumulative unique held qualified demos is a **scenario check**, not an asserted outcome.
Weeks 5-6 - conversion layer	Prioritized backlog from Prompt 1: M08, M10, B10, B11, B14, B15, M12. Reuse M10 if already produced in week 3; build the next needed items only as buyer questions, evidence and production capacity justify. Tighten the demo page; add the proposed sandbox only after product, support, billing and data policies are approved.	This is a sequence, **not a seven-asset production quota** or extra five-hour allocation. Held meetings, form reliability and customer delivery take priority.
Weeks 7-8 - review	Run the first monthly deep dive, identify the biggest verified leak, improve one page/offer or source, and stop a channel that consumes time without qualified conversations.	Four cumulative held qualified demos is the planning pace; record sample size and reasons for any gap.
Month 3 - selective growth	Start a partner contribution or OPA appearance if audience is confirmed. Activate paid only with budget, clean consent, working conversion, adequate audience, a test cap, and capacity for follow-up. Add review/reference material only from permissioned verified evidence.	Day-90 scenario: 12 held qualified demos from distinct organizations, 3 readiness-approved evaluations, 1 new paying organization as stretch. If prerequisites fail, remain in targeted organic/direct motion.

Part 6 - 90-day success dashboard

The following is **illustrative funnel math**, not a forecast: research 60 distinct target organizations over roughly 13 weeks (about 5/week); if 30 engage in a substantive exchange (50%), 18 accept a demo slot (60%), and 12 hold a qualified demo (67% of booked), the north star reaches 12. If 25% of those 12 approve an evaluation after readiness checks, that is 3; if one of those three becomes a *new* paying buying organization, that is 33% of evaluations. None of these rates is measured yet. Website signups can contribute, but outbound and introductions mean page-conversion benchmarks cannot be used alone to derive the target.

Metric	Week 4 scenario	Week 8 scenario	Day 90 scenario	Current baseline	Gap to close
Distinct researched hospital buying organizations	20	40	60	Unknown	Establish list and dedupe
Substantive two-way exchanges	8	18	30	Unknown	Test access/message
Accepted demo bookings, unique organizations	3	7	18	Unknown; current source handler is not proof of accepted leads	Repair capture, qualify
Held qualified demos, unique organizations	**2**	**4**	**12**	**Unknown**	Record held dates and organization IDs
Readiness-approved evaluations	0-1	1	3 stretch	Unknown	Scope, owners, access, baseline
New paying buying organizations	0	0	1 stretch	Founder reports pre-revenue; anchor agreement not counted as new paying	Agree scope and close independently
Public-site visitors, trailing month	100	200	300	Unknown	Tag and measure relevant sources
Accepted lead → CRM owner/task coverage	100% after repair	100%	100%	Source handler not connected	Test every path

One number that matters most: cumulative held qualified demos from distinct hospital buying organizations. Check against a staircase of 2 by week 4, 4 by week 8, and 12 by day 90, adjusting only as a labelled plan after evidence.

Three early warnings in the first 30 days: (1) a test request shows success without a stored lead and named owner; (2) researched, relevant outreach yields no substantive buyer reply across the first ten distinct organizations; (3) no held qualified demo by week 4 despite confirmed capture and conversations. Resolve instrumentation before declaring message failure.

Three growth signals by day 90: (1) 12 distinct held qualified demos, not repeated meetings; (2) at least three named evaluations pass readiness checks with a baseline and hospital owner; (3) one *new* paying organization plus delivery capacity, with acquisition source and commercial terms recorded. If only the first two occur, continue learning rather than claiming revenue traction.

Sources: source form, research addendum, Google GA4 manual campaign dimensions, and official tool docs linked in Part 2. Handoff to Prompt 7: the weekly one-page report draws from the same distinct-organization CRM and never reports N/A as zero.

Source: Original files/Complete B11 HTML library/Source notes/2026-09-22-evidence-register.md. Historical wording preserved; not a new approval or factual revalidation.

Evidence and claim register

Date: 22 September 2026 Status: Research working record. This is not an audit of product capability or legal compliance.

Evidence labels

- **Direct buyer evidence:** Traceable interview, recorded sales conversation, observed workflow, or buyer-provided record. No new direct interviews or original buyer transcripts were collected in this pass.
- **Internal report:** Founder/team description. Useful and specific, but may be selective, outdated, or unverified.
- **External context:** Research about another sample or market. Do not treat it as proof about Georgian hospitals or MediMind results.
- **Hypothesis:** Our interpretation, proposed buyer wording, priority, or prediction. Requires testing.

Internal sources

ID	Source	What it supports	Limit
I1	Canonical buyer notes, cross-checked with Brain mirror	Existing role definitions and hypotheses about fragmentation, vendor dependency, repeated entry, patient context, and reporting	Positioning statements, not interview transcripts; exact system counts and claimed capabilities are not audited here
I2	19 August document pack, unit-of-value section and Item 11	Private-chain target, economic-buyer emphasis on reimbursement/cost/visibility, parallel evaluation, continuing development	Internal investor draft; reported losses and implementation comparisons require primary evidence
I3	16 August investor Q&A, claim discipline and deployment answer	Technical readiness, authorized access, parallel operation, reconciliation before cutover, clinical confirmation	Earlier delivery language differs from current offer; not a universal performance guarantee
I4	Founder instructions in this conversation and live hero snapshot	Desired early-partner price match, included development, three-day testable features, two-week parallel evaluation, AI vision	A founder-authorized offer is not independent evidence of results or unbounded delivery capacity

Internal documents are linked, not reproduced wholesale. No sensitive financial records or patient data have been copied into this folder.

External primary sources

E1. Hospital CEO priorities, U.S.

American College of Healthcare Executives: Top Issues Confronting Hospitals, 2025 survey. Accessed 22 September 2026.

The October 2025 survey received 215 responses from 1,185 community-hospital CEOs (18%). Financial challenges ranked first, workforce second, and governmental mandates third. This supports investigating operational and financial buying reasons before assuming AI excitement leads demand. It does not establish rankings in Georgia or causation by software; U.S. payment-policy details should not be imported into Georgian copy.

E2. Georgian health-system context

European Observatory / WHO: Health systems in action 2024: Georgia, published 4 December 2024. Accessed 22 September 2026.

The country overview describes predominantly private-for-profit provision, complex coverage, and an acute nursing shortage. This makes private-provider economics and staff capacity relevant research topics. It is a country-level report, not a buyer-interview study, and does not establish software-related financial losses or current payment rules for a specific hospital. The linked page's metadata title is inconsistent, but its visible heading and content identify Georgia.

E3. Hospital integration is possible, but uneven

ONC/ASTP: Hospital Use of APIs to Enable Data Sharing between EHRs and Third-Party Technology, Data Brief 81, February 2026; 2024 AHA survey data. Accessed 22 September 2026.

The study describes widespread third-party data exchange, often through proprietary or other non-standardized methods, and variation by hospital characteristics. Its 2024 survey had 2,253 non-federal acute-care hospital respondents, a 51% response rate, with weighting. This is a reason to investigate integration quality, not to claim existing software cannot support AI or that total replacement is the only option. It is U.S. context, not local buyer evidence.

Claim decisions needed before the next copy revision

Claim	Current evidence/status	Needed decision or proof
Better care, more revenue, less waste	Desired outcomes; no measured causal results collected here	Define a use case, baseline, timeframe, attributable change, and limits before quantifying
Match current software pricing	Founder offer; older internal reference-deal description	Eligible hospitals, included spend/scope, term, exclusions, renewal, migration and third-party costs

Claim	Current evidence/status	Needed decision or proof
Unlimited future development	Founder offer	Included work, exclusions, prioritization, capacity, support versus new development, third-party/AI usage costs
Features testable in three business days	Current offer, not a measured service history	Request-size limits, prerequisites, when the clock begins, and distinction from approved production release
Parallel test in two weeks	Current offer; older notes qualify readiness and use ten days	Define calendar/business days, access/readiness prerequisites, trial scope, workload, and acceptance criteria
AI brain built around the hospital	Roadmap ambition in current wording	Current versus planned functions, data-use permissions, technical meaning of adaptation/training, clinical oversight, and validation
Advanced AI included at matched price forever	Not established; older Item 10/expansion notes put Phase 3 and some AI expansion outside reference pricing	Reconcile current intended offer with older scope. Do not invent an exclusion or include everything by implication
Early offer will end	Founder says not indefinite; no fixed deadline/places established	Define a genuine commercial boundary if urgency is used; do not invent a countdown
Replace everything	Strategic direction, not a completed per-hospital replacement matrix	Name covered workflows and identify necessary external systems/device interfaces during evaluation
No disruption/no risk	Parallel approach reduces some transition risks	Operational evaluation plan, rollback, staff workload, data reconciliation, and sign-off; never imply zero risk
Live footprint and proof badges	Not re-audited in this job	Verify sites, modules, acceptance and permission before reusing old “25 hospitals” language

Measurement rules

- Count realized collections, not merely charges identified. Separate reduced delay from new money recovered.
- Use legitimate, clinically accurate documentation and billing. Do not present inflated coding or unnecessary treatment as revenue opportunity.
- Staff minutes saved are capacity, not automatically cash savings. Identify whether overtime, staffing expense, throughput, or attention changes.
- Compare similar workflows/cases; account for training and process changes. Avoid counting the same benefit twice.

- Show total evaluation and transition costs. A matched subscription fee is not identical total cost of ownership.
- Process measures such as faster retrieval are not interchangeable with patient outcomes.

Open evidence gaps

No local buyer priority survey, competitor-review corpus, verified customer quotations, quantified operational-loss sample, or attributable clinical/financial outcome study was collected. We have enough to design focused research, not to declare a winning message or guaranteed return.

Source: Original files/Complete B11 HTML library/Source notes/2026-09-22-hospital-buyer-pain-points.md. Historical wording preserved; not a new approval or factual revalidation.

What hospital decision-makers need to fix

Date: 22 September 2026 Status: Provisional research synthesis, not a validated ranking or approved public copy. Working audience: CEOs, owners, and finance leaders of private hospital chains and mid-to-large multi-profile hospitals, initially in Georgia. Clinical and IT leaders influence and approve the decision.

Source IDs below resolve in the evidence register. Internal positioning is evidence of what MediMind believes, not proof of what every buyer believes. External research provides context, not local customer validation.

The central finding

Our strongest current hypothesis is that the buyer needs **a hospital that performs better without putting care, finances, or operations at risk during the change**.

Disconnected software is a possible cause. A hospital operating system is our mechanism. AI is part of the approach and future opportunity. The buying reasons are more concrete: money lost, staff time consumed, decisions delayed, care harder to coordinate, and improvements blocked.

Your own August buyer notes explicitly put reimbursement gaps, avoidable costs, and operational visibility ahead of the software bill [I2]. That makes “same software price” a useful objection remover, not necessarily the headline benefit.

The current hero expresses a desirable destination. It does not yet show the CEO that we understand the specific problem they have to explain at their next management meeting.

Priority pains to investigate

This is a research priority order based on internal specificity, likely buying relevance, and plausible product fit. It is not a survey-derived order of all hospital problems. A clinical safety event or an audit deadline may immediately change the order for a particular hospital.

All first-person sentences below are **proposed buyer-language hypotheses, not quotations from customers**.

1. The hospital does the work but loses money along the way

Proposed buyer language: We treat the patients, but we lose money in the paperwork and cannot see where costs are getting away from us.

- **Visible problem:** Care documentation, charges, payer submissions, and actual costs do not line up reliably. People spend time correcting records and chasing payment.
- **CEO consequence:** Weaker cash collection, uncertain profitability, and less room to invest in care. Distinguish cash delayed from revenue permanently lost.
- **Potential software-related causes:** Missing or inconsistent documentation, manual handoffs, missed legitimate charges, and poor visibility into resource use. Payment rates, payer decisions, contracts, and clinical complexity may be bigger causes that software cannot fix.
- **Trigger to test:** Repeated rejected claims, a weak financial month, or an unexpected difference in costs between similar services/sites.
- **Existing workaround to investigate:** Manual billing checks, spreadsheets, extra finance staff, documentation reminders, or an additional finance tool.
- **MediMind fit:** Connect clinical work and financial records; make missing information and spending patterns easier to inspect. Specific workflows and results need demonstration.
- **Proof needed:** Documented reason codes for rejected claims; attributable corrections; money actually collected; cost per comparable case; added implementation/support cost.
- **Evidence:** Specific internal economic-buyer thesis [I2]. U.S. CEO research supports the broader financial pressure [E1]; it does not establish Georgian losses or MediMind's effect.
- **Status:** Partially supported context; customer-specific cause, amount, and willingness to replace remain unvalidated.

2. Expensive, scarce staff spend too much time on avoidable work

Proposed buyer language: I need my team caring for patients, not entering the same information again and again.

- **Visible problem:** Repeated entry, searching across screens, phone calls for results, and rechecking what another department already knows.
- **CEO consequence:** Paid time diverted from care, slower service, staff frustration, and possible overtime. Extra hiring does not fix the underlying work design.
- **Trigger to test:** Overtime growth, clinician complaints, vacancies, or a service backlog.
- **Existing workaround:** More coordinators, paper lists, messages, manual reconciliation, and informal staff knowledge.
- **MediMind fit:** Shared information, simpler workflows, and selected AI assistance. We need to show less work, not merely fewer applications.
- **Proof needed:** Observed minutes per task, repeat entries, completion/error rates, training burden, and user adoption before/after.

- **Evidence:** Internal coordinator/clinician pain statements [I1]; workforce concerns in U.S. CEO research [E1]; the Georgian health-system review reports an acute nursing shortage [E2]. None proves that software causes the whole staffing problem.
- **Status:** Partially supported; local workflow burden and reducible fraction need measurement.

3. Leadership cannot get a timely, trusted picture of the hospital

Proposed buyer language: By the time I get a report I can trust, the problem has already cost us.

- **Visible problem:** Different departments report different numbers; the management team waits for people to compile and reconcile information.
- **CEO consequence:** Decisions about capacity, spending, staffing, and performance rely on incomplete or late information. Chain leaders cannot compare hospitals confidently.
- **Trigger to test:** A board meeting with disputed numbers, an unexpected overspend, or a new hospital joining the group.
- **Existing workaround:** Spreadsheet consolidation, more meetings, and calls to department heads.
- **MediMind fit:** A shared operational record and consistent reporting definitions. A dashboard alone does not fix bad data or unclear accountability.
- **Proof needed:** Report preparation time, data freshness, reconciliation errors, and a real decision improved by the information.
- **Evidence:** Automated unified visibility is named in the internal economic-buyer notes [I2].
- **Status:** Internally supported hypothesis; no direct executive quote or observed reporting cycle reviewed.

4. Care depends on people manually connecting the patient story

Proposed buyer language: My doctors should not have to chase information to understand the patient in front of them.

- **Visible problem:** Relevant history, current care, results, and handoffs are scattered or arrive late.
- **CEO consequence:** Delays, repeated work, difficult coordination, and concern about missed information. Clinical leaders may rank this above money.
- **Trigger to test:** A documented near miss, a delayed decision, repeated tests, or complaints about handoffs.
- **Existing workaround:** Phone calls, paper summaries, repeated checking, and staff carrying knowledge between departments.
- **MediMind fit:** Bring relevant patient context and workflows together. This supports better coordination; it is not proof of fewer deaths, complications, or readmissions.
- **Proof needed:** Time to retrieve relevant information, result-to-review delay, handoff completeness, appropriate safety review, and eventually properly designed outcome evaluation.

- **Evidence:** Internal clinical workflow thesis [I1]. U.S. interoperability research shows existing data exchange is common but uneven in method and coverage [E3]; it does not prove an individual hospital has this pain.
- **Status:** Plausible and internally supported; severity and clinical improvement not established locally.

5. The hospital cannot improve at the speed its needs change

Proposed buyer language: Every change becomes another quote, another wait, or another vendor saying it is not their responsibility.

- **Visible problem:** Small changes take too long, customization costs are hard to predict, and different vendors own different pieces.
- **CEO consequence:** Improvement plans stall; staff build workarounds; leadership pays repeatedly without getting a coordinated result.
- **Trigger to test:** An expensive change request, a failed integration, contract renewal, or a workflow the current vendor cannot support.
- **Existing workaround:** Accept the limitation, hire developers, buy another tool, or rely on spreadsheets.
- **MediMind fit:** A coordinated product and continuing development relationship. Unlimited requests are not the same as infinite delivery capacity or instantaneous production changes.
- **Proof needed:** Actual request-to-test times, acceptance rates, backlog behavior, support response, and total change costs.
- **Evidence:** Internal vendor and integration pain [I1]; founder's development offer [I4].
- **Status:** Internal hypothesis with a proposed commercial response; no comparative service-performance dataset.

6. Reporting and audit readiness consume work and create exposure

Proposed buyer language: When someone asks us to prove what happened, my team has to reconstruct it.

- **Visible problem:** Records need manual preparation for management, payment, or external review; different sources may not agree.
- **CEO consequence:** Staff burden and uncertainty about whether the hospital can demonstrate its processes and records reliably.
- **Trigger to test:** An audit, rejected reporting, a new contractual requirement, or a security incident.
- **Existing workaround:** Manual exports, additional checks, outside assistance, and last-minute reconciliation.
- **MediMind fit:** Consistent data, traceable workflows, and reporting support where implemented and verified. Software does not guarantee legal compliance or security.
- **Proof needed:** A real required report, audit-log completeness, access review, restoration tests, and buyer security review.
- **Evidence:** Internal CIO/ministry reporting concerns [I1]. This is not an assertion about a particular law or a certification MediMind holds.

- **Status:** Internal hypothesis; rank and requirements vary by jurisdiction and institution.

Two buying objections that can outweigh every benefit

A. Can we change without disrupting care or revenue?

The buyer can dislike the current software and still rationally keep it. Migration, missing history, staff training, parallel work, downtime, and a failed cutover can outweigh the expected gain.

The parallel-evaluation offer addresses this objection only if it has a limited scope, authorized data access, named hospital owners, an honest workload estimate, reconciliation, success criteria, and a controlled decision to switch [I3]. “Nothing changes until you agree” is more defensible than “zero effort” or “no risk.”

Evidence to request: What caused the last replacement proposal to stall? Who vetoed it? What was the specific feared loss? What would make a limited evaluation acceptable?

B. Can we trust one young supplier with so much of the hospital?

Replacing multiple vendors reduces coordination burden but concentrates dependency. The CEO and CIO may reasonably ask about long-term support, continuity, data access/export, service levels, security, and whether the promised development capacity is sustainable.

These are buying requirements, not objections to dismiss. Involve IT as an evaluation partner. Do not carry forward the older internal framing of IT as merely a blocker to neutralize.

Evidence to request: What vendor failure would be unacceptable? What support and exit provisions are mandatory? Which promises in our offer cause doubt?

Both objections are strongly relevant to the proposed sales motion, but their ranking is a hypothesis until buyers confirm it.

Where the AI future belongs

Proposed buyer language: I want useful AI, but I need to know it works here, is safe, and will not become another disconnected expense.

AI can be an aspiration, a board priority, or an active buying trigger. We have not established which it is for the primary buyer. Ask for a specific AI project, assigned budget, accountable owner, decision deadline, and reason it is stuck.

Position the future as an extension of useful progress: connected workflows first; AI assistance where it works; broader proactive coordination as capabilities are developed and validated. Do not imply full replacement is the only route to any AI use. Do not claim autonomous clinical decisions are already deployed, inevitable hospital failure, or a future malpractice deadline.

The urgency should come from a verified ongoing problem or a genuinely bounded commercial offer. A fear of being left behind is not yet a proven purchasing motive.

Problem layers by buying situation

The hidden and future layers are hypotheses to test, not psychological facts about buyers. Future consequences are plausible, not forecasts.

Buying situation	Surface problem	Possible underlying cause	Hidden concern to test	If unchanged over 6-12 months
CEO/owner under financial pressure	Weak margins and difficulty tracking avoidable loss	Care, billing, and cost information do not connect reliably	Being accountable for results without enough control; making an expensive wrong move	The same documentation and coordination problems may keep consuming cash and time
Chain leader seeking consistent performance	Hospitals cannot be compared or managed consistently	Separate systems, definitions, local workflows, and reporting	Expansion may increase complexity faster than oversight	Each new site may add another layer of manual coordination
Clinically led hospital	Staff chase information and work around software	Fragmented patient context and poorly supported handoffs	Modernization could make care harder before it makes it better	Workarounds become entrenched; improvement projects remain delayed

The finance, clinical, and IT participants have different acceptance criteria within each buying situation. They are not necessarily separate customer markets.

The problem worth leading with, provisionally

Costly coordination failures that consume staff time, obscure financial losses, and make good care harder to deliver.

This ties the operating problem to the CEO's responsibilities without assuming every financial or clinical problem is caused by software. The supporting reason to engage is a controlled way to verify improvement before switching.

Three exploratory statements, not approved landing-page copy:

- **Rational:** Disconnected systems make your hospital repeat work, reconcile records, and struggle to see where money is being lost.
- **Emotional:** Your people work hard to keep the hospital running. They should not have to keep its software working together, too.
- **Leadership:** Your hospital's progress should not depend on how long its software vendors take to catch up.

Recommended message order to test: recognizable present-day problem, concrete improvement, evidence, manageable evaluation, then the larger AI opportunity. The current same-price and development offer should support that story rather than substitute for it.

Provisional problem-solution fit

- **Independent CEO/owner:** When care, financial records, and daily work are disconnected, leadership struggles to identify and reduce avoidable loss. MediMind's proposed unified workflows offer a way to connect them; a scoped evaluation must demonstrate better work and economics before replacement.
- **Hospital-chain operator:** When sites use inconsistent records and processes, central leadership struggles to compare performance and spread improvements. Shared workflows and reporting could reduce repeated coordination, while preserving necessary local differences; this needs multi-site evidence.
- **Clinically led hospital:** When patient context is scattered, staff spend effort reconstructing it. Connected records and workflow support could reduce that effort and improve handoffs; clinician testing and measured safety review must establish the actual benefit.

Boundaries: what not to claim

- Do not promise that software solves inadequate reimbursement rates, staffing supply, poor management, or every source of hospital loss.
- Do not promise dramatic or immediate clinical/financial results without attributable evidence.
- Do not describe a prototype or parallel trial as a validated clinical release or completed migration.
- Do not claim every system, external portal, or specialized device can be replaced without a scope review.
- Do not imply advanced AI is included forever at matched pricing until the commercial conflict is resolved.

What would change this ranking?

Promote a pain when several independent target hospitals describe a recent incident without prompting, can show its consequence, have tried to solve it, and assign an owner or budget. Downgrade it when leaders merely agree that it sounds important but cannot name an incident or take a next step.

A single anchor customer's enthusiasm is valuable but cannot establish market-wide priority. Keep financial-loss, staff-workload, and leadership-visibility messages separate in early tests so we can learn which actually drives interest.

Source: Original files/Complete B11 HTML library/Source notes/Company inputs and editorial assumptions.md. Historical wording preserved; not a new approval or factual revalidation.

Company inputs and editorial assumptions

Evidence snapshot: 22 September 2026. These are supplied facts and proposed working decisions, not approved commercial commitments.

- MediMind is a Georgian hospital **operating system**, joining clinical and business work on a shared FHIR R4 foundation. Private hospital networks and multi-profile hospitals in Georgia are the first audience. Kazakhstan and Uzbekistan are later expansion choices from the founders, not proven demand. Avoid ministries/solo practices as the main funnel.
- Founder Lasha Khoshtaria is a physician and CEO; Irakli Pkhovelishvili is co-founder. Do not copy IDs, home addresses, bank information, signature images or patient data into marketing work.
- GeoHospitals agreement is signed per founder. Founder states pre-revenue, with revenue expected after integration. A network contract or case-study mention is not proof that all facilities are live or of measurable outcome improvements. Do not invent client counts, logos authorization, customer quotes, outcome numbers or exact current accepted scope.
- Explain actual product mechanics and distinguish documented capability, code presence, deployment acceptance, measured outcome and roadmap. Existing notes may overstate AI autonomy or rollout. Human clinical control stays explicit where relevant.
- Long-term AI coordination is a direction; no claim of autonomous clinical decision making, guaranteed better patient outcomes, revenue uplift or guaranteed compliance.
- Founder-defined early partner offer in current hero: match current software price, included continuing development, requested features testable in three business days, tailored parallel evaluation in two weeks. Preserve the intended attractiveness while stating scope/readiness and that production sign-off is separate. Do not invent exclusions as agreed terms. The exact boundaries and advanced AI costs remain commercial decisions. No fixed scarcity/deadline or lifetime price lock is established.
- Main buyer = CEO/owner/group operator. Clinical, finance, IT and operational leaders form the buying committee. Their problems are hypotheses unless supported by traceable buyer records. Prior pain notes and avatar prose are not original interviews.
- Use operating-system category. Internal taxonomy if useful: Heart, Brain, Muscle, Skin, Nerve. Avoid unsupported first/only claims and competitor absolutes. Features/benefits/JTBD can use ordinary language needed by the prompt. Do not repeat loaded marketing jargon.

Coherent working decisions for this edition

These are recommendations/assumptions, not historical facts or approved commitments. Improve only if you notify the parent for cross-book reconciliation.

- Primary CTA/conversion: a founder-led **25-minute pathway demo**, ending in a proposed measured evaluation. Main live route `/request-demo` exists: inspect source for its actual current fields/copy before claiming changes. Other campaign routes are proposed unless inspected.
- Main campaign concept: **One patient. Too much repeat work.** Hook: **Why does one patient create so much extra work?** CTA: **Map my hospital's first improvement**. The outcome is a visible, measurable first

pathway, not hospital-wide transformation in minutes. Use synthetic/demo data in videos.

- Recommended primary buyer segment: hospital CEO/owner who can name a recurring cross-department problem and assign a clinical/operations owner and IT counterpart to an evaluation.
- Marketing resources: **5 hours TOTAL per week**, zero paid spend initially; optional paid tests only after budget and evidence readiness. Any 30-day calendar, distribution routine and implementation plan must share this budget, not add separate 5-hour allocations. Separate founder creative/admin time from engineering setup, delivery, and actual sales meetings; state additional effort plainly.
- 90-day planning scenario: **12 held qualified hospital demos**, with **3 readiness-approved evaluations** and **1 new paying buying organization** as stretch downstream assumptions. North star = cumulative held qualified hospital demos (not website views, not booked calls). Unknown baseline; anchor account separate. Funnel math must be explicit, scenario not forecast. No maturity claim from tiny samples; use N/A when denominator zero and provisional counts early.
- Demo = 25 minutes, founder-led (assumption); no unconfirmed response-time promises. Webinar = proposed 45-minute practical session (30 min workshop +15 Q&A), unscheduled; no fake date, registrations, speakers affiliations or recording promise. Trial = proposed guided synthetic-data sandbox (14 days, no card as recommendation), clearly separate from two-week readiness-dependent parallel evaluation and clinical live deployment; no invented SLA or deletion policy. Mark proposed commercial decisions in an input preface.
- Public testimonials and measured case-study outcomes absent from supplied verified inputs: retain required sections with precise evidence placeholders and provide useful proof/process copy alternatives. No fake quote attributed to GeoHospitals or a real person. A competitor quote is never MediMind social proof.

Source: Original files/Complete B11 HTML library/Source notes/Current hero snapshot.md. Historical wording preserved; not a new approval or factual revalidation.

Live landing-page hero: working draft

Snapshot date: 22 September 2026

Status: Live, but not final. User assessment: these versions are okay, but we are not there yet. Preserve this copy as a baseline, not a validated winner.

Source: `src/components/Landing/LampHeroSection.tsx`, deployment commit `baf894c`. This file captures the exact hero wording, not the rest of the page. Research and commercial qualifications are recorded separately below.

Headline

Your hospital deserves a future.

Your patients deserve better today.

Supporting sentence

Replace disconnected software with one intelligent hospital system-built for better care, more revenue, and less waste.

Benefits

A better hospital today.

Connected information, simpler work, and AI that helps your people make better decisions.

Your own AI brain tomorrow.

Built around your hospital’s needs-progressing toward AI that anticipates risks and coordinates care and business decisions, with your team in control.

A team that keeps building for you.

Unlimited future development included. New features ready to test in three business days.

Offer

Move first. At the software price you already pay.

We match your current software price for early hospital partners. This offer won’t be available indefinitely.

Test it beside your current system in two weeks. Keep running. Commit after you see the difference.

Buttons

- Primary: Build my hospital’s future
- Destination: `/request-demo`
- Secondary: See the path
- Destination: page section `#path`

Research notes, not public copy

- This snapshot preserves claims; it does not independently substantiate them.
- The headline expresses ambition but does not name the costly present-day problem.
- We need a clearer reason for the CEO to act and credible proof tied to that reason.
- Three-day delivery and a two-week parallel evaluation are founder-defined offers, not guarantees of full clinical production readiness or hospital cutover.
- Define what unlimited development includes, request prioritization, exclusions, and the treatment of advanced AI costs. Older internal commercial notes and current broad wording are not aligned on this.
- Matched subscription pricing does not establish unchanged total implementation cost or a permanent price lock.
- The proactive hospital AI brain is a direction of travel, not proof of currently autonomous clinical operation.

Source: Original files/Complete B11 HTML library/Source notes/Product positioning - 00-what-we-are.md. Historical wording preserved; not a new approval or factual revalidation.

What We Are

MediMind is one of the first operating systems built ground-up to run an entire hospital on a single FHIR-native, AI-orchestrated codebase. Not an EMR. Not an integration layer. The whole thing.

Short-form: One hospital. One operating system.

What we are today

A FHIR-native hospital operating system running on the Medplum Cloud FHIR R4 backend. ~2,200 files in the EMR codebase across 60+ component areas. Ambient clinical scribe live. Coordinator desk live. CPOE, MAR, pharmacy queue, laboratory, PACS viewing, bed management, scheduling, billing, claims, HR, analytics, prior-authorization, multi-branch orchestration, and an offline-first PWA - all in one codebase. First hospital deployments in Georgia. Trilingual at the architectural level: Georgian, Russian, English.

What we are building

One hospital. One operating system.

Every job a hospital does - check a patient in, order a test, read a scan, give a medicine, bill a claim, submit to the ministry - happens in one place. No logins to switch. No data to copy. No systems to connect.

Ground-up on FHIR R4 as the storage primitive. AI as a native architectural primitive - the ambient scribe, the appeals engine, the coordinator copilot, the drug-interaction checker, all reasoning across the same FHIR object model. One codebase covering the full breadth of a hospital's operations, priced for the hospitals the big vendors do not serve.

We begin in underserved markets - Georgia, CIS, MENA, SE Asia, Sub-Saharan Africa. We end in every hospital that concludes forty systems glued together was never the right answer.

What we are not

- **Not an overlay on Epic.** We replace the hospital's software stack. We do not sit on top of it.
- **Not an ambient scribe.** The scribe is one capability of the operating system, not the product.
- **Not a FHIR platform.** We are built on a FHIR platform (Medplum). We are the hospital OS above it.
- **Not a legacy EMR with AI bolted on.** There is no legacy data model inside us. There is nothing bolted on.
- **Not an integration suite.** We do not integrate forty systems into one dashboard. We replace the forty systems.

The defense moat

Ground-up on FHIR R4 as the storage primitive. This single qualifier is our strongest architectural claim - no major commercial competitor owns it. Guard it in every doc.

Source: Original files/Complete B11 HTML library/Source notes/Product positioning - 03-scope.md. Historical wording preserved; not a new approval or factual revalidation.

Scope - What We Replace

When someone asks what MediMind replaces, show them this table. Hospital operational domains, one codebase, one FHIR-native ontology underneath.

Domain	FHIR resource(s)	MediMind code path
Patient Registration	`Patient`, `Coverage`, `RelatedPerson`	`emr/components/registration/`
Ambient Clinical Documentation	`Composition`, `Provenance`, `Encounter`	`emr/components/mediscribe/` (feature 062)
CPOE (Computerized Physician Order Entry)	`ServiceRequest`, `MedicationRequest`	`emr/components/cpoe/` (feature 065)
MAR (Medication Administration Record)	`MedicationAdministration`	`emr/components/mar/` (feature 059)
Pharmacy Queue & Dispense	`MedicationDispense`, `MedicationRequest`	`emr/components/pharmacy-queue/` (feature 059)
Laboratory	`ServiceRequest`, `Observation`, `DiagnosticReport`, `Specimen`	`emr/components/laboratory/` (features 038-039)
PACS / Imaging Viewer	`ImagingStudy`, `DocumentReference`	`emr/components/pacs/` (feature 044)
Bed Management	`Location`, `Encounter`	`emr/components/bed-management/`
Appointments & Scheduling	`Appointment`, `Schedule`, `Slot`	`emr/components/appointments/` (feature 037)
Coordinator / Care Navigation	`Task`, `CareTeam`, `Communication`	`emr/components/coordinator/` (feature 071)
Prior Authorization & Appeals	`Task`, `Claim`, `Communication`	`emr/components/coordinator/preop/`, `coordinator/private-insurance/`
Billing / RCM	`ChargeItem`, `ChargeItemDefinition`, `Account`	`emr/components/financial/`, `emr/components/billing/` (feature 051)
Claims Processing	`Claim`, `ClaimResponse`, `Coverage`	`emr/components/financial/`

Domain	FHIR resource(s)	MediMind code path
Inventory & Warehouse	`SupplyDelivery`, `Basic/StockQuant`, `SupplyRequest`	`emr/components/warehouse/` (warehouse features 028-037)
HR & Staff Management	`Practitioner`, `PractitionerRole`, `Schedule`	`emr/components/hr/` (feature 045)
Analytics Dashboards	read-only across the Heart	`emr/components/dashboard/`, `emr/components/analytics/`, `emr/components/clinical-dashboard/`
MOH Submissions & Compliance	`Encounter` extensions, `Basic`	`emr/services/moh/` + `emr/components/moh/` (feature 053)
Gov-EHR Integration	`Bundle`, `Parameters`	`emr/components/gov-ehr/` (feature 035)
Multi-Branch Orchestration	`Organization` hierarchy	`emr/contexts/BranchContext.tsx` (feature 066)
Offline-First PWA	IndexedDB cache + sync queue	`emr/components/offline/` + service worker (feature 069)
Nursing Workstation	`Task`, `Observation`, `MedicationAdministration`	`emr/components/nursing-station/` (feature 068)
Clinical Event Engine	`Subscription`, `Basic`	`emr/components/event-engine/` (feature 070)
Messaging & Staff Inbox	`Communication`, `Task`	`emr/components/messaging/`, `emr/components/coordinator/CoordinatorInbox.tsx`
AI Clinical Assistant	runs across the Heart	`emr/components/ai-assistant/`
Patient Card (unified chart)	`Patient`, `Encounter`, `Observation`, everything	`emr/components/patient-card/` (feature 072), `emr/components/patient-history/`

Counter-argument pre-empted: "You don't cover X." If X is on this list, we cover it. If X is not on this list, say so honestly - do not bluff.

Living document: Audit quarterly. New features land in the table the same week they ship. If a domain leaves the scope of our OS, remove it.

Source: Original files/Complete B11 HTML library/Source notes/Product positioning - 04-language.md. Historical wording preserved; not a new approval or factual revalidation.

Language

Section A - Say / Kill patterns

Never say	Say instead
EMR	Hospital operating system
Module / feature	Capability / layer
Integrates with lab, pharmacy, etc.	Natively includes lab, pharmacy, etc.
Supports FHIR	Built on FHIR / FHIR-native
AI-enhanced / AI-powered	AI-native / AI-orchestrated
Dashboard (standalone)	Command surface / analytics layer
User	Clinician, coordinator, nurse, pharmacist - or the specific role
First in the world	One of the first
Platform (in MediMind thesis context)	Operating system
Patient record	Living patient object
Workflow	Pathway / orchestration

Section B - Phrases owned by competitors. Do not use.

Using any of these invites a counter-citation from a vendor with an earlier, publicly indexable press release:

Phrase	Owned by
"First FHIR-native EHR"	Ottehr
"AI-native EHR"	athenahealth (Sept 2025), IntelliCare (Indonesia), Raintree, Elation
"First AI-native healthcare platform"	Clinarion
"Operating system for healthcare"	Commure (Feb 2020)
"World's first agentic AI operating system"	Superhealth SuperOS (Feb 2026)

Phrase	Owned by
"First agentic native company in U.S. healthcare"	DeepCura
"AI-native operating system for healthcare practices"	Nitra (Mar 2026)
"Built from the ground up" (EHR context)	Oracle Health Next-Gen (Oct 2024)
"The first AI operating system designed specifically for healthcare"	Akido Labs
"The AI Operating System for healthcare"	Commure, Ambience
"First AI-native EHR to achieve EU MDR certification"	InterSystems IntelliCare (19 May 2026)
"First FDA-cleared continuous AI sepsis monitoring"	Bayesian Health / TREWS
"First FDA-authorized AI tool for sepsis"	Prenosis Sepsis ImmunoScore (Apr 2024)

Section C - Phrases we own. Use freely.

- ***"Ground-up on FHIR R4"*** - zero verbatim matches on the public web at audit time. Our strongest phrase. Use it in every first-claim sentence.
- *"One hospital. One operating system."*
- *"The forty-system hospital"*
- *"A hospital with a body made of software"*
- Anatomical taxonomy (**Heart · Brain · Muscle · Skin · Nerve**) as a package

Section D - Claim discipline

- Never say *"the first"*. Always *"one of the first"*.
- When making any first-claim, pair it with the qualifier *"built ground-up on FHIR R4"*.
- Category word: say **operating system** - not platform, not EMR, not suite, not ecosystem.
- Underserved-markets framing: *"built for the hospitals big vendors do not serve"* or *"underserved-market economics"*. Do not say *"cheap"* or *"for the poor"*.
- **The rule applies to badges, labels, and pills - not just prose.** A `WORLD-FIRST` badge over hedged body copy is still an absolute claim, and it is the part people screenshot.
- **Never claim novelty on AI *detection*.** Continuous patient surveillance is a mature, FDA-cleared category (see `05-competitors.md` Section E). Our proactive-layer claim is **architectural**: *"the system that notices and the system that acts are the same codebase."*
- **Never imply autonomous clinical oversight.** Words that promise the software watches *for* the clinician create product-liability exposure - a missed deterioration invites *"you promised oversight."* Keep **"the AI flags, your doctors decide"** visible on every proactive-layer surface. **Brain** stays as the locked taxonomy word (`02-taxonomy.md`); what changes is the framing around it, never the word.

- **State deployment scale precisely.** *"Live across N acute-care hospitals under a single enterprise health-system contract"* - contracted ≠ live. Never let one number stand for both.

Section E - Words we never need

Revolutionary · Disruptive · Cutting-edge · Best-in-class · World-class · Game-changing · Synergy · Leverage (as verb) · Holistic · Turnkey. These words describe nothing and signal the opposite of what we are. Kill them on sight in any draft.

Rule: This file is the arbiter. If a doc uses language not covered here, add the language here first, then use it. Never the other way around.

Source: Original files/Complete B11 HTML library/Source notes/Product positioning - 06-buyers.md. Historical wording preserved; not a new approval or factual revalidation.

Buyers

Five personas. Each has a pain, a phrase that gets them to reply, and a thing we never say to them.

1. Hospital CEO (200-800 bed multi-profile hospital)

Georgia · CIS · MENA · SE Asia · LATAM · Sub-Saharan Africa

Pain: forty systems. Every vendor charges monthly. Nothing talks to anything else. Epic quoted them a number they cannot afford. The IT team spends more time on integration tickets than on improving care.

What gets them to reply: *"One operating system, one monthly fee based on your bed count, replaces everything you're running today."*

Never say: *"We will integrate with your existing systems."*

2. Hospital CTO / CIO

Pain: fragile integrations, vendor lock-in, no FHIR story to show the board, audit failures when ministry auditors ask for data.

What gets them to reply: *"Ground-up on FHIR R4. Open SDK. Multi-branch. Offline-first. Built on Medplum."*

Never say: *"Our proprietary data model."*

3. Ministry of Health official (small country digitalizing nationally)

Small to mid-size ministry standardizing software across public hospitals. Budget is government-funded. Time horizon is multi-year.

Pain: every public hospital runs different software. Data does not roll up nationally. Audit and reporting require manual translation across vendors. Procurement for international EMRs is either impossible or diplomatically sensitive.

What gets them to reply: *"One operating system, standardized across every hospital in the country, localized in your language and your regulations, priced for government-funded health systems."*

Never say: *"We'll connect your existing systems."*

4. Coordinator (power user - daily workflow owner)

Pain: swivel-chairs across the coordinator desk, the insurance portal, the MOH forms, SMS, email, and a shared Excel. Every patient requires the same data re-entered into different places.

What gets them to reply: *"One screen. The AI drafts the appeal, the form, the SMS. You review and send."*

Never say: *"Our workflow tool."* (They hear: another tool to switch to.)

5. Clinician (doctor or nurse - daily end user)

Pain: ambient scribe that does not know their hospital's pathway. Separate MAR. Separate ordering. Third login for the pharmacy queue. Fourth for the lab.

What gets them to reply: *"One login. One chart. The scribe knows your hospital's pathway and the patient's history."*

Never say: *"New features."* (They hear: more training, more change.)

Discipline

- Match the language above, persona by persona. Do not mix.
- If you find yourself talking to a persona not on this list, pause - we probably do not have a buyer story for them yet. Add it here first.

Source: Original files/Complete B11 HTML library/Source notes/Research and source inspection.md. Historical wording preserved; not a new approval or factual revalidation.

Research and source inspection

Checked 22 September 2026. Internal source snapshot; not a live-site test.

Verified source-code finding: demo conversion

`/Users/toko/Desktop/MediMind Landing Page/src/App.tsx` defines `/request-demo`. In `src/components/Demo/RequestDemo.tsx`, current headline is “See MediMind on your hospital’s workflows”. Subheadline is “30 minutes. One login, one chart, one patient - live. We’ll tailor the walkthrough to your hospital.” This edition recommends 25 minutes as a deliberate proposed change, not a description of the current page.

The source form has three steps and nine mandatory inputs: first name, last name, email, company name, company size, industry, job title, facility type, number of providers. Its `handleSubmit` function (lines 164-176 at inspection) waits two seconds, logs form data to the browser console and sets a success state. It contains no backend persistence/network submission. The resulting text says the team received the request and will contact them within 24 hours. This is a source inspection, not a test of deployed behavior or proof that any real lead was lost. Before campaigns: verify deployed version, implement confirmed receipt, record ownership/next action, test success and failure paths, then track conversions only on actual accepted submissions. No website code was edited here.

The repo's investor tracking is separate from customer-funnel analytics. Presence of an investor dashboard does not establish GA4/CRM/campaign tracking.

Source-code snapshot: current homepage hero

Read-only inspection of `src/components/Landing/LampHeroSection.tsx` found the headline “Your hospital deserves a future. Your patients deserve better today.” The primary button says “Build my hospital’s future” and routes to `/request-demo`; the secondary button says “See the path” and scrolls to the path section. Supporting source copy offers matched current software pricing for early hospital partners, included future development, new features ready to test in three business days, and a parallel test in two weeks. This edition supplies proposed replacement copy and readiness conditions, not a deployed change. Source files themselves are not distributed in this marketing package; these notes and the saved hero snapshot capture the inspected facts.

Fresh public verification

- Geostat release dated 29 July 2026: https://www.geostat.ge/en/single-news/3809/indicators-of-health-care-2025 . Downloaded and read official PDF physical p2: https://geostat.ge/media/81668/INDICATORS-OF-HEALTH-CARE---2025.pdf . **232 hospitals, 15.2 thousand beds in 2025**. Facilities are not deduplicated buying organizations or active demand. Paraphrase only; no need repeat quotations.
- Medplum case study: https://www.medplum.com/blog/medimind-case-study , 6 August 2026, authored by Lasha Khoshtaria. It describes a 25-hospital network under contract, initial capabilities in production and planned network-wide go-live. Founder-authored public evidence; no independent customer endorsement.

Avoid importing scale or outcomes as currently audited facts. Limit dependence/quotation: most product information also available in user-owned positioning notes.

- Google Analytics official setup: https://support.google.com/analytics/answer/9304153?hl=en . Account/property/web stream/tag and verify actual receipt. Editorial implementation should tailor to React route changes and accepted conversions, not fire on every button click.
- Current GA4 manual campaign parameters: https://support.google.com/analytics/answer/10917952?hl=en . Use this GA4 page rather than the deprecated UA guide `1033863`.
- Google Analytics PII restriction: https://support.google.com/analytics/answer/6366371?hl=en . No names, email addresses or sensitive free-text in events/URLs/UTMs. Do not install marketing session recording in clinical systems, staff/investor private portals or patient-data pages. Use clean public marketing events.

Presentation completeness

The library describes itself as 16 sequential prompts but splitting Part 2 Prompt 3 into 3A and 3B yields **17 distinct outputs**, plus **3 Library #0 outputs**, total **20**. Existing `part-1/01-business-and-market.html` covered only the first prompt. There was no HTML truncation: later prompts had not been executed.

Source: Original files/Marketing workspace/prompt-library/complete-edition/working/launch-guide.md. Historical wording preserved; not a new approval or factual revalidation.

Lasha's first-week launch guide

Purpose. Start one useful conversation with a Georgian private-hospital CEO or group operator about a repeated patient handoff, then offer a founder-led pathway demo. The campaign line is **"Why does one patient create so much extra work?"** The next action is **"Map my hospital's first improvement."** The 90-day number-12 held qualified demos, three readiness-approved evaluations, and one new paying buying organization-is a planning scenario, not a forecast or a claim of existing traction. Count one buying organization once; keep the existing anchor separate.

What is ready on paper

The funnel blueprint defines the stages and the full 85-idea catalog. Those ideas are a backlog, **not 85 pieces to make**. The angle plan chooses a low-risk map, one use case, and a controlled comparison. The campaign script contains an 82-second founder phone video, a 15-second cut, and two short tests using synthetic information. The first public destination is P05, the existing `/request-demo` route. Its proposed page copy says **"Map your first patient-pathway improvement"** and promises a 25-minute founder call; it is not live site copy.

The form is the gate. Source inspection found that its current submit handler waits, logs to the browser, and shows success without sending the request to a backend. That does not prove the deployed site behaves the same way, but **do not send campaign traffic to P05 until a live test confirms receipt, a named owner, and a truthful failure path**. The site/engineering owner handles implementation outside the five founder marketing hours. The existing page also says 30 minutes; align it with the proposed 25-minute demo before publication.

T10: the worksheet to use this week

Copy these six prompts onto one page. The buyer can answer without disclosing patient identity or clinical details:

1. **Pathway:** Fill four boxes-request, result, next decision, and financial close-using a fictional example in public copy.
2. **Last incident:** What happened in a recent real handoff, described without names or patient details?
3. **People and systems:** Which teams touched it, and which tools or manual steps did each need?
4. **Repeat work:** What was re-entered, searched for, called about, or reconciled? Count steps and note who did them.
5. **First baseline:** What could be measured now-task time, repeat entries, wait time, exceptions, or appropriate actual collections? Record how the number is obtained; "unknown" is acceptable.
6. **Decision:** Who owns the work, who must review it clinically and technically, and what result would make a limited parallel evaluation worthwhile?

T10 is useful even if the buyer decides the incumbent can fix the problem. Do not convert a filled worksheet into a public case study or a performance claim without the hospital's review and permission.

The first five hours, total

Use the traffic chapter's **300-minute weekly clock**, also used in the angle plan. Reserve **85 minutes** for five five-minute weekday checks (25 total), a 30-minute Monday scorecard review, and a 30-minute weekly share of the monthly deep dive. The remaining **215 minutes** are the entire creative and outreach allowance-not another five hours for each channel:

Minutes	First-week action	Finished when
75	Put T10 into a clean one-page document, with the six prompts above and a synthetic example.	A CEO can name one handoff and first measure without sending patient data.
40	Adapt the first paper-card visual from the 3A script into T01, one founder LinkedIn post. End with the pathway-map invitation.	The post asks a real question and points only to a working route or offers the worksheet by reply.
60	Choose five genuinely relevant hospital contacts or permissioned introduction routes. Send each a brief, individual note about one pathway question, with T10 only if useful.	Five named approaches are recorded; no invented relationship or bulk campaign.
40	Answer replies, ask for the last concrete incident, and record the next owner/action.	A reply becomes a pathway discussion or a clear “not now,” not a vanity impression.

The 3A video is **ready to film, not yet filmed**. Use the week-two creative slot in the angle plan for the phone shoot and captions, once the destination works. Do not add filming on top of week one's 300 minutes. A founder demo, clinical/IT workshop, technical route repair, and customer implementation each need separately planned time.

Measure the real next step

The tracking plan distinguishes a click, an accepted request, a booked meeting, and a **held qualified demo**. The last is the north star. A qualified demo has a distinct hospital buying organization, relevant authority or sponsor, a recent cross-department problem, and an owner or path to one. Put source/content ID (T10, T01, or the later video), organization, owner, next date, and held-demo date in a controlled CRM or sheet. Keep names and clinical details out of public analytics URLs. If there is no eligible denominator, write **N/A** rather than a percentage.

At Monday's review, decide one thing: what most limits the next held qualified demo-lead receipt, access to the right buyer, message fit, or evaluation readiness? Fix that first. The page, worksheet, script, and reporting templates are drafted. No buyer interviews, campaign launch, external sends, new page deployment, accepted lead flow, customer outcome, or film production was performed by this guide.

Source: Original files/Marketing workspace/prompt-library/complete-edition/working/research-addendum.md. Historical wording preserved; not a new approval or factual revalidation.

Orchestrator research addendum

Checked 22 September 2026. Use alongside writer-brief.md.

Verified source-code finding: demo conversion

`/Users/toko/Desktop/MediMind Landing Page/src/App.tsx` defines `/request-demo`. In `src/components/Demo/RequestDemo.tsx`, current headline is “See MediMind on your hospital’s workflows”. Subheadline is “30 minutes. One login, one chart, one patient - live. We’ll tailor the walkthrough to your hospital.” This edition recommends 25 minutes as a deliberate proposed change, not a description of the current page.

The source form has three steps and nine mandatory inputs: first name, last name, email, company name, company size, industry, job title, facility type, number of providers. Its `handleSubmit` function (lines 164-176 at inspection) waits two seconds, logs form data to the browser console and sets a success state. It contains no backend persistence/network submission. The resulting text says the team received the request and will contact them within 24 hours. This is a source inspection, not a test of deployed behavior or proof that any real lead was lost. Before campaigns: verify deployed version, implement confirmed receipt, record ownership/next action, test success and failure paths, then track conversions only on actual accepted submissions. No website code was edited here.

The repo's investor tracking is separate from customer-funnel analytics. Presence of an investor dashboard does not establish GA4/CRM/campaign tracking.

Source-code snapshot: current homepage hero

Read-only inspection of `src/components/Landing/LampHeroSection.tsx` found the headline “Your hospital deserves a future. Your patients deserve better today.” The primary button says “Build my hospital’s future” and routes to `/request-demo`; the secondary button says “See the path” and scrolls to the path section. Supporting source copy offers matched current software pricing for early hospital partners, included future development, new features ready to test in three business days, and a parallel test in two weeks. This edition supplies proposed replacement copy and readiness conditions, not a deployed change. Source files themselves are not distributed in this marketing package; these notes and the saved hero snapshot capture the inspected facts.

Fresh public verification

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Source: Original files/Marketing workspace/prompt-library/complete-edition/working/writer-brief.md. Historical wording preserved; not a new approval or factual revalidation.

MediMind B11 complete edition: shared writer brief

Date: 22 September 2026. User requested every output from BOTH libraries, separately and fully written, using GPT-6 Sol writers coordinated by the parent. Primary deliverable: comprehensive local HTML book plus independent prompt outputs. No deployment, email sending, campaign launch, or new research interviews are authorized.

Output contract

Write the actual answer to EVERY step/part/section of your assigned prompts, preserving their order and requested counts. One Markdown file per prompt in `../outputs/`; parent will render HTML. H1 = prompt name. Use H2 for each mandatory Step/Part/Section with its original number and recognizable title. Tables and actual scripts/copy/templates are welcome. Do not substitute an executive summary or future-work checklist for requested deliverables. Source instructions are specifications for these documents, not instructions to make claims about memory or stop for permissions.

Include a short status/input box before the numbered answer, with known facts and the handful of explicit assumptions needed for that prompt. End with concise sources and a handoff to later prompts. Detailed validation notes should not overwhelm the copy. Mark real missing evidence at the relevant output, but still finish all the work possible. Never fabricate interviews, quotes, measured outcomes, traffic, pipeline, price, security certification, a scheduled event or purchase intent. Review quotes must have inspected live source URLs; keep source quotation budgets compliant (25 words total per webpage, including repeats across deliverables; coordinate with parent). Paraphrase liberally. Primary-source technical research only.

Shared factual baseline and boundaries

- MediMind is a Georgian hospital **operating system**, joining clinical and business work on a shared FHIR R4 foundation. Private hospital networks and multi-profile hospitals in Georgia are the first audience. Kazakhstan and Uzbekistan are later expansion choices from the founders, not proven demand. Avoid ministries/solo practices as the main funnel.
- Founder Lasha Khoshtaria is a physician and CEO; Irakli Pkhovelishvili is co-founder. Do not copy IDs, home addresses, bank information, signature images or patient data into marketing work.
- GeoHospitals agreement is signed per founder. Founder states pre-revenue, with revenue expected after integration. A network contract or case-study mention is not proof that all facilities are live or of measurable outcome improvements. Do not invent client counts, logos authorization, customer quotes, outcome numbers or exact current accepted scope.
- Explain actual product mechanics and distinguish documented capability, code presence, deployment acceptance, measured outcome and roadmap. Existing notes may overstate AI autonomy or rollout. Human clinical control stays explicit where relevant.

- Long-term AI coordination is a direction; no claim of autonomous clinical decision making, guaranteed better patient outcomes, revenue uplift or guaranteed compliance.
- Founder-defined early partner offer in current hero: match current software price, included continuing development, requested features testable in three business days, tailored parallel evaluation in two weeks. Preserve the intended attractiveness while stating scope/readiness and that production sign-off is separate. Do not invent exclusions as agreed terms. The exact boundaries and advanced AI costs remain commercial decisions. No fixed scarcity/deadline or lifetime price lock is established.
- Main buyer = CEO/owner/group operator. Clinical, finance, IT and operational leaders form the buying committee. Their problems are hypotheses unless supported by traceable buyer records. Prior pain notes and avatar prose are not original interviews.
- Use operating-system category. Internal taxonomy if useful: Heart, Brain, Muscle, Skin, Nerve. Avoid unsupported first/only claims and competitor absolutes. Features/benefits/JTBD can use ordinary language needed by the prompt. Do not repeat loaded marketing jargon.

Coherent working decisions for this edition

These are recommendations/assumptions, not historical facts or approved commitments. Improve only if you notify the parent for cross-book reconciliation.

- Primary CTA/conversion: a founder-led **25-minute pathway demo**, ending in a proposed measured evaluation. Main live route `/request-demo` exists: inspect source for its actual current fields/copy before claiming changes. Other campaign routes are proposed unless inspected.
- Main campaign concept: **One patient. Too much repeat work.** Hook: **Why does one patient create so much extra work?** CTA: **Map my hospital's first improvement**. The outcome is a visible, measurable first pathway, not hospital-wide transformation in minutes. Use synthetic/demo data in videos.
- Recommended primary buyer segment: hospital CEO/owner who can name a recurring cross-department problem and assign a clinical/operations owner and IT counterpart to an evaluation.
- Marketing resources: **5 hours TOTAL per week**, zero paid spend initially; optional paid tests only after budget and evidence readiness. Any 30-day calendar, distribution routine and implementation plan must share this budget, not add separate 5-hour allocations. Separate founder creative/admin time from engineering setup, delivery, and actual sales meetings; state additional effort plainly.
- 90-day planning scenario: **12 held qualified hospital demos**, with **3 readiness-approved evaluations** and **1 new paying buying organization** as stretch downstream assumptions. North star = cumulative held qualified hospital demos (not website views, not booked calls). Unknown baseline; anchor account separate. Funnel math must be explicit, scenario not forecast. No maturity claim from tiny samples; use N/A when denominator zero and provisional counts early.
- Demo = 25 minutes, founder-led (assumption); no unconfirmed response-time promises. Webinar = proposed 45-minute practical session (30 min workshop +15 Q&A), unscheduled; no fake date, registrations, speakers affiliations or recording promise. Trial = proposed guided synthetic-data sandbox (14 days, no card as recommendation), clearly separate from two-week readiness-dependent parallel evaluation and clinical live deployment; no invented SLA or deletion policy. Mark proposed commercial decisions in an input preface.
- Public testimonials and measured case-study outcomes absent from supplied verified inputs: retain required sections with precise evidence placeholders and provide useful proof/process copy alternatives. No fake quote attributed to GeoHospitals or a real person. A competitor quote is never MediMind social proof.

Sources to read

Both extracted libraries in this folder: `main-library.txt` (60 PDF pages), `pages-library.txt` (20 pages). Read your full assigned prompt, not only goal boxes.

Existing research (relative to marketing root):

- `research/2026-09-22-evidence-register.md`
- `research/2026-09-22-hospital-buyer-pain-points.md`
- `research/2026-09-22-prompt-library-review.md`
- `copy/2026-09-22-live-hero-working-draft.md`
- `prompt-library/part-1/01-business-and-market.md` (all 7 steps already, useful baseline)
- `archive/uvp-jtbd-strategy-2026-09-22.md` (ideas, not validated results)

Canonical product positioning at `/Users/toko/Desktop/medplum_medimind/positioning/`: 00-what-we-are.md, 03-scope.md, 04-language.md, 06-buyers.md. These are internal assertions; do not treat broad claims as deployment proof. Read-only research; do not edit product code.

Relevant vendor/public sources to verify on web: MEDSOFT/MEDWORK; InterSystems IntelliCare; Epic; Oracle Health; Bahmni; Commure; Qventus; Medplum MediMind case study. Use official pages for vendor claims and transparent comparison labels. No unverified numeric market sizing.

Collaboration

You are not alone in the codebase. Own only your assigned outputs; do not revert or overwrite others' work. Use apply_patch for file edits. Send parent completion paths and unresolved evidence questions. Writers may draft in parallel from this shared baseline, but consume earlier outputs once available and reconcile before final. Do not spawn further agents. No git commit/push.

Source: Original files/Marketing workspace/prompt-library/part-1/01-business-and-market.md. Historical wording preserved; not a new approval or factual revalidation.

Prompt 1: Describe your business and market

Date: 22 September 2026 Status: First complete strategic answer, for review. All seven prompt steps are addressed; active buying demand is not yet quantified. Not final website copy. Framework: Part 1, Prompt 1, physical pages 5-7 of the complete B11 library.

Executive conclusion

MediMind's opportunity is to make a better-run, increasingly AI-enabled hospital achievable for leaders whose current systems make improvement difficult.

The sale is not primarily “buy AI” or “buy cheaper software.” Our strongest working thesis is: improve care coordination and operating performance through one tailored system, demonstrate the difference before switching, and keep improving it with the hospital.

The strategic distinction is the combination of scope, local fit, ongoing development, commercial accessibility, and controlled evaluation. None of these, individually, is established as globally unique.

Business input reconstructed from existing material

MediMind is building one operating system for a hospital's clinical and business work, replacing disconnected core programs rather than adding another dashboard above them. It brings patient information and hospital workflows onto a shared foundation. It introduces AI into everyday work and is building toward broader proactive coordination, with clinical decisions remaining under clinicians' control. Its initial commercial focus is private hospital networks and multi-profile hospitals in Georgia, with regional expansion contemplated after local proof. The founder's proposed early-partner offer matches current software pricing, includes continuing development, and lets a hospital evaluate a tailored parallel system before switching. The business needs to prove that its breadth and adaptability produce measurable improvements at a sustainable delivery cost.

This synthesis describes product direction and intended value; it is not a claim that every capability, result, or commercial condition has been independently verified.

1. Restate the business sharply

MediMind replaces disconnected hospital software with one tailored system that connects care and business operations, helps teams reduce wasted work and missed revenue, and creates a foundation for hospital-wide AI.

The customer is the hospital organization. The principal economic buyer is its CEO, owner, or finance leader. Clinical leaders and IT leaders determine whether the promised improvement is acceptable and feasible.

The complete value story, in plain language

- **Today:** Make the hospital's information and work fit together, so staff spend less effort on avoidable coordination and leaders can identify losses and bottlenecks.
- **As the hospital improves:** Add useful AI and develop changes around its priorities.
- **The destination:** Proactive coordination across clinical and business workflows, under appropriate human control.
- **The reason to consider switching:** Let the hospital test agreed workflows and verify the difference before replacing the current system.

What this corrects

“Hospital OS” describes the product category. “AI-native” describes an approach. “One codebase” describes implementation. None explains the buyer's gain by itself. Lead with the work and results the hospital needs; explain the architecture when it provides a reason to believe.

Keep the full replacement ambition. Do not interpret it literally as replacing every ministry portal, insurer system, device interface, or external service. The replacement scope must be agreed per hospital.

2. Name the real market

The behavioral market is hospitals trying to improve financial control, staff productivity, and care coordination while depending on systems they cannot easily change.

They already spend money and staff effort on this problem, even when they do not buy a new product.

What they do without MediMind	Why it is a rational choice	What could make it inadequate
Keep the current supplier and request changes	Familiar workflows, known people, little immediate migration effort	Changes do not arrive, cost too much, or fail across departments
Use staff to reconcile information	Flexible; people understand local exceptions	Recurring work becomes expensive, unreliable, or too slow
Add reporting or an integration project	Improves visibility without replacing the core system	Data quality or fragmented work remains unresolved
Add a focused AI or automation tool	Limits the change to an urgent problem	More tools, dependencies, and costs appear without fixing broader coordination
Replace with an established hospital system	Offers breadth, references, and a familiar procurement route	Commercial fit, localization, transition effort, or adaptability is unsatisfactory
Build or customize an open-source platform	Offers control and access to code	Implementation and continuing maintenance become the hospital's responsibility unless a partner owns them
Postpone the decision	Avoids taking a near-term operational risk	Existing losses and stalled improvements become less acceptable than changing

These are hypotheses about alternatives to investigate, not claims that each target hospital uses them all.

Recommended first customer profile

A private hospital or hospital group with meaningful cross-department work, an identifiable problem affecting care or operating performance, a decision-maker willing to own improvement, and the practical ability to run a scoped evaluation.

Qualify by **complexity, pain, authority, and readiness**, not bed count alone. Existing notes propose 200-800 beds, but this should be a segment to investigate, not an automatic exclusion rule in Georgia. Smaller multi-profile hospitals or groups may have substantial coordination needs and more accessible decisions.

Do not confuse these three markets

1. **Immediate proof market:** Accessible Georgian hospital organizations with a near-term reason to improve and a feasible evaluation path.
2. **Next expansion hypothesis:** Comparable private institutions in a selected regional market, once language, payment, support, data, and deployment requirements are demonstrated there.
3. **Long-term ambition:** A broad international hospital OS. This is not the same as today's serviceable pipeline.

Solo practices, satisfied organizations with no meaningful coordination problem, and institutions without a viable implementation owner are not the priority. Ministries remain a separate, longer procurement motion rather than another persona on the same homepage.

3. The three most dangerous assumptions

Assumption A: The problem is important enough to justify replacement

The risk: Hospitals may agree that their software is frustrating but prefer a narrow fix or the familiar status quo. A valuable problem does not automatically justify a whole-system purchase.

What we have: An anchor network relationship and founder-reported economic motives. The 18 August investor-call note identifies documentation-related reimbursement gaps, spending visibility, and cost control as the economic case [I3]. This is a secondhand account of buyer motivation, not a direct buyer interview.

What would weaken the thesis: Repeated independent prospects describe inconvenience but cannot name a costly incident, improvement owner, purchasing window, or reason a smaller fix would not work.

Best test: Ask independent decision-makers about their latest actual incident and attempted remedy. Compare a scoped replacement evaluation with the real alternative they are considering. Track willingness to supply a workflow owner and evaluation time, not compliments about the demo.

Assumption B: A shared system produces measurable improvement beyond a nicer interface

The risk: Documentation, reimbursement, staffing, throughput, and clinical outcomes also depend on people, policy, contracts, and management. Software may not control the largest cause of the problem.

What we have: Implementation experience, product breadth, and a plausible mechanism. Internal investor notes explicitly distinguish deployment speed from operating results and identify missing outcome measurement [I3, I4].

What would weaken the thesis: Information is connected but the target task remains just as slow, the hospital cannot collect more legitimate payment, or the new workflow adds equivalent work elsewhere.

Best test: Agree on one baseline and one outcome in a specific workflow. Measure repeat work, processing time, corrected documentation, or actual collections as appropriate. Include transition effort and negative effects. Do not substitute time saved for cash saved, or process improvement for improved patient outcomes.

Assumption C: We can deliver the offer repeatedly and sustainably

The risk: Matched pricing, wide replacement scope, rapid features, and continuing customization can create a service business with insufficient margin or unmanageable obligations. An anchor partner's unusual access and flexibility may not generalize.

What we have: Founder-defined commercial commitments and reported rapid changes within the reference relationship. Older notes limit reference pricing and exclude some advanced AI; the broad current offer needs reconciliation [I2, I5].

What would weaken the thesis: Every deployment requires extensive one-off work, support consumes the team, or buyers reject the necessary boundaries once explained.

Best test: Record engineering and hospital staff hours, recurring infrastructure/AI costs, support load, time to a testable change, reuse across hospitals, and actual acceptance. Evaluate a second independent hospital on explicit terms, not an implicit promise to build everything immediately.

Priority: Test willingness to change first, actual improvement second, and delivery economics alongside every evaluation. None can be deferred indefinitely to an AI future.

4. Market sizing that can be used and defended

What can be counted today

Geostat's 29 July 2026 release reports **232 hospitals and 15,200 hospital beds in Georgia in 2025** [E1]. This supersedes the older 243-hospital figure for a current national baseline.

Those 232 hospitals are not 232 qualified buyers. The statistic does not say how many match our operational needs, share a purchasing group, are reachable, are dissatisfied, or are shopping now.

The correct unit is the buying organization

Map hospitals to the organization and committee that authorizes software spending. One group may represent several implementation sites but one commercial decision. Conversely, a group may delegate decisions to individual sites; confirm rather than assuming.

Do not multiply all hospital sites by one assumed contract price. A reference network deal cannot establish average revenue per hospital or mature pricing.

A usable bottom-up calculation

Stage	What must be true	Current count
National hospital universe	Counted in official 2025 hospital statistics	232 facilities

Stage	What must be true	Current count
Distinct buying organizations	Ownership and purchasing authority deduplicated	Unknown
Suitable organizations	Cross-department need and a feasible product/localization fit	Unknown
Reachable organizations	A named professional route to the decision-maker, not just a public website	Unknown beyond recorded relationships
Active demand	Recent pain, improvement owner, budget path, and an actual evaluation/purchasing window	No verified total available
Serviceable this period	Hospital readiness and MediMind delivery capacity both available	Unknown until scoped

Direct answer to the prompt: We cannot currently defend a number for reachable hospitals actively looking for this solution. That number is unknown, not zero, and not the national hospital total.

Initial organization-level research list

This is a named starting set, not a qualified pipeline. No outreach has been sent. No financial distress, dissatisfaction, replacement interest, or personal access is inferred from a hospital's existence.

Organization	Basis for including it	Current status for this exercise	What to verify next
GeoHospitals	Existing internal reference relationship [I2, I3]	Anchor evidence/expansion account, not a new prospect; current accepted scope needs confirmation	Which buyer outcome led to selection; actual workflow results; what can be publicly referenced
HelsiCore	Mentioned as the founder-connected next institution in May notes [I6]	Historical lead, not confirmed active in September	Current intent, decision owner, timing, and separation from the anchor group
Georgia Healthcare Group / GCAP healthcare services	Public operating-group reports [E8]	Publicly identifiable research candidate; no active opportunity confirmed	Buying structure, current tools, fit, and any legitimate introduction route
Aversi Clinic	Public clinic organization site [E9]	Publicly identifiable research candidate; no active opportunity confirmed	Cross-department need, purchasing authority, current platform and contract window
Vivo Medical Group	Public group description of multiple centers [E10]	Publicly identifiable research candidate; no active opportunity confirmed	Group versus site decisions, current improvement priorities, system strategy

The three public candidates are not the complete market. The two relationship accounts are not evidence that the other three are accessible or ready to buy.

Practical first discovery target

Build a deduplicated list of **15 buying organizations**, then seek **8-10 conversations across at least four independent organizations**, following the saved interview guide. These are proposed research targets, not estimates of demand or sales forecasts.

For each account record: group/site identity, type, relevant workflow complexity, known incumbent, buyer role, access route, last verified date, specific pain evidence, trigger, budget process, evaluation owner, and next step.

Once qualified, model revenue as the sum of actual annual prices proposed for each eligible account, with explicit close/activation assumptions and start dates. Keep one-time implementation revenue, recurring fees, and optional future AI revenue separate.

5. Key players and why a buyer might choose them

Official vendor pages establish what vendors publicly offer, not independent proof of their outcomes. This is a focused first map, not the full competitive exercise in Prompt 5. Prices, local availability, growth rates, and head-to-head performance have not been established. Where a weakness is unknown, it is a question to test rather than an invented criticism.

Player or alternative	What the buyer can reasonably value	Limitation or comparison question	Why it could win today
Current supplier, including local MEDSOFT/MEDWORK where installed	Local hospital workflows, existing familiarity; MEDSOFT publicly describes hospital software and local-government integrations [E2]	Does the actual installed version support the needed change, at acceptable cost and speed? No blanket “nothing connects” claim	Staying with a known system may be safer and cheaper than replacement
InterSystems IntelliCare	Publicly offers unified clinical, administrative, and financial workflows with embedded AI [E3]	Compare a real local proposal: fit, total cost, implementation responsibilities, adaptability, and evidence	Breadth and unified AI are already part of its offer; our category language is not exclusive
Epic	Broad connected health-system software, clinical and business workflows [E4]	Verify actual availability, procurement fit, localization, and total costs for this buyer; do not invent a quote	A hospital may value its scope and established ecosystem more than our flexibility
Oracle Health	Markets a redesigned EHR with AI embedded in daily workflows [E5]	Verify currently available functions for the relevant care setting and country; the page contains roadmap qualifications	Existing relationships and a major supplier's product path may reduce perceived supplier risk
Bahmni and its implementation partners	Open-source hospital-system approach, on-premise operation, workflow adaptation [E6]	Who owns localization, updates, operations, and support in the specific proposal? No assumption of zero total cost	A hospital may prefer code access, flexibility, and a chosen implementation partner

Player or alternative	What the buyer can reasonably value	Limitation or comparison question	Why it could win today
Commure	Focused intake, documentation, coding, claims, and payment automation, integrated with existing EHRs [E7]	A targeted layer does not itself establish whole-core-system replacement; Georgian fit and availability need checking	The buyer may solve a priority problem while keeping its current system
Qventus	Hospital operational automation, including capacity and administrative workflows [E11]	Focused operational scope is not an entire hospital OS; local fit needs checking	It offers a route to operating improvement without full replacement
Internal staff, spreadsheets, integrations, and postponement	Known tradeoffs, immediate control, no new core-platform decision	Can their continuing cost and limitations be demonstrated in this hospital?	Familiarity and reversibility can outweigh an ambitious future promise

The competitive conclusions that matter

1. **The current supplier and postponement are the first alternatives to investigate.** We do not have win/loss data to declare either the measured number-one competitor.
2. **IntelliCare is a particularly close message-level comparison.** It already combines broad hospital scope and AI. We need a credible reason to choose our offer, not a claim that nobody else has thought of it.
3. **Focused AI tools are real competition for the budget.** They may win because they ask the hospital to change less.
4. **“Others only observe; we act” is too broad.** Qventus explicitly markets workflow actions and EHR updates. This does not establish whole-hospital clinical autonomy, but it contradicts treating all adjacent systems as flag-only observers [E11].
5. **Local suppliers must be named and evaluated.** MEDSOFT's public description includes hospital workflows and government-system integrations. Our argument cannot be that every local supplier is wholly disconnected [E2].
6. **Do not label a startup fast-growing without evidence.** The earlier Brain watchlist includes SuperOS and other emerging players; current commercial scope and growth were not verified sufficiently here. Carry them into Prompt 5 for verification, not into public superiority claims.

What we should not carry forward from older notes

- Stale “they are not live, we are” comparisons.
- A network contract portrayed as every hospital fully live.
- A complex architecture qualification presented as proof of buyer superiority.
- Simulated clinical outcomes described as real patient saves.
- AI assistance described as impossible without replacing existing software.

These are claim corrections in this working analysis, not edits to the source notes or the live website.

6. Pinpoint the gap

The gap to test is affordable, locally fitted hospital-wide improvement that a buyer can verify before accepting the disruption and dependency of replacement.

The strongest target is not “every hospital without AI.” It is a hospital with a real cross-department operating problem, dissatisfied with the available path to fix it, and enough authority and readiness to evaluate a different foundation.

MediMind's proposed answer combines:

1. **A connected core:** Improve the hospital's shared work, not just one screen.
2. **Local adaptation:** Fit actual clinical, administrative, language, and reporting needs.
3. **Continuing development:** A practical route from recurring needs to improvements.
4. **A controlled comparison:** Let staff and leadership evaluate agreed workflows before a switch.
5. **Accessible early-partner terms:** Reduce the subscription-budget objection while stating total transition costs honestly.
6. **A credible AI path:** Start with supported daily uses and build toward broader proactive coordination.

The combination is promising but is not proven to be unavailable from every competitor. The more durable advantage would be repeatable implementation know-how, reusable local workflows, reliable support, measured results, and trusted references. One data standard alone is not a complete commercial defense.

The uncomfortable but useful test

If a hospital can solve its important problem faster and more safely with its existing supplier or a narrow tool, why should it replace the core system with us?

We must answer with the hospital's work and evidence. “Because our architecture is modern” is not sufficient. Our answer should become: the narrower fixes leave a recurring problem unresolved, and the hospital has verified that MediMind addresses it with an acceptable transition and cost.

7. Market positioning statement

MediMind is the hospital operating system for private hospitals and hospital groups ready to move beyond disconnected software. We bring clinical and business work into one system, tailored to the hospital and built to reduce repeated work, reveal avoidable losses, and support better-informed care. Our approach combines continued development with a parallel evaluation, so the hospital can judge the change before switching. We start with the problems leadership needs to solve today and build the connected foundation for progressively more capable AI across the hospital.

This is the recommended strategy paragraph, not a guaranteed-outcomes claim or the final hero.

The commercial line to attach after terms are settled

For eligible early partners, we match current software pricing and include ongoing development within an agreed scope.

The user has asked for unlimited future development; the operating definition still needs to be explicit. This sentence is a proposed bounded description for review, not a unilateral change to the live offer. Do not imply a permanent price lock or inclusion of every future AI capability without confirmation.

Why act now?

Every month spent compensating for disconnected work is another month the hospital cannot use that effort to improve care and operations. A scoped evaluation can establish whether there is a better option.

This is a problem-based urgency hypothesis. Where the hospital cannot identify meaningful ongoing friction, use its actual improvement priority rather than manufacturing fear. AI adds strategic upside; it does not require predicting that the hospital will disappear.

Carry-forward brief for Prompt 2

- **Primary buyer:** CEO/owner/finance leader of a private hospital or group; clinical and IT leaders co-evaluate.
- **Starting geography:** Georgia, provisionally; future regional expansion is separate.
- **Buying situation:** Meaningful cross-department pain, insufficient current remedy, an improvement owner, and a feasible evaluation.
- **Functional progress:** Less avoidable work, clearer control of operating performance, better-connected patient information.
- **Emotional progress:** Confidence that modernization will help without an uncontrolled transition.
- **Social progress:** Be the leader who delivered a defensible improvement, not merely announced an AI initiative.
- **Mechanism:** Tailored shared system, workflow redesign, continuing development, staged AI.
- **First value hypothesis:** The hospital completes and compares a real, agreed workflow with less avoidable work and trustworthy information. The precise workflow must be chosen; this is not yet a measured “aha moment.”
- **Major friction:** Supplier trust, clinical continuity, data quality, staff burden, total cost, and scope uncertainty.
- **Unresolved:** Active-demand count, independently measured operating results, current deployment footprint, delivery capacity, and detailed commercial terms.

Sources and evidence boundaries

Internal material

- I1: Canonical product positioning and canonical competitor notes. Read the canonical source rather than relying on the shorter Brain mirror. Treat claims as research inputs, not proof.
- I2: 19 August document pack, business-model and buyer sections.
- I3: 18 August investor-call note. Evidence of what was recorded as said in an investor conversation; not a direct customer study.
- I4: Gaps to close, measurement and evidence distinctions.
- I5: Existing claim register, pain-point analysis, and live hero snapshot.
- I6: 27 May Gail Gannon note. Historical relationship/lead signal, not September pipeline confirmation.

External sources checked 22 September 2026

- E1: Geostat: Indicators of Health Care 2025, released 29 July 2026. Its linked official PDF was downloaded and checked: 232 hospitals and 15.2 thousand beds, physical page 2. Direct source PDF.
- E2: MEDSOFT/MEDWORK company description. Historical/undated vendor account; no current installed-base number used as a market share estimate. Some deeper pages returned a browser verification screen, so this is not a feature audit.
- E3: InterSystems IntelliCare. Public category/scope messaging; no independently verified comparative cost or outcome claim used.
- E4: Epic health systems and clinics. Public scope; no pricing inferred.
- E5: Oracle Health EHR. Public messaging includes product-direction qualifications; does not establish every advertised function is available in every country/care setting.
- E6: Bahmni. Open-source hospital-system approach and supported areas; implementation/service costs not inferred.
- E7: Commure. Public revenue-cycle/ambient platform positioning and existing-EHR integration; vendor metrics not used as independent outcomes.
- E8: Georgia Capital 2025 results. Public evidence of a healthcare operating group, not evidence of software-purchase intent.
- E9: Aversi Clinic. Organization discovery only; no buying readiness inferred.
- E10: Vivo Medical Group. Public group structure and stated modernization goals; not evidence of interest in MediMind.
- E11: Qventus automation platform. Vendor explicitly describes automations and EHR updates; no outcome statistics reused.
- Additional cross-check: MediMind case study hosted by Medplum, 6 August 2026, authored by a MediMind founder. Supports public product narrative and reference context, not independent customer-outcome validation.

No prospects were contacted, no paid research was purchased, and no website or deployment was changed. This answer completes the seven-step analysis at the current evidence level; it does not pretend to complete missing primary research.

Source: Original files/Marketing workspace/prompt-library/README.md. Historical wording preserved; not a new approval or factual revalidation.

B11 marketing program: complete prompt outputs

Updated: 23 September 2026. All **20 written prompt outputs** are complete, covering **138 requested top-level steps/parts/sections** from both PDFs. This is research synthesis, proposed strategy and ready-to-edit copy-not proof that interviews, deployment or campaigns have happened.

Open the complete HTML book. It includes all 20 answers, search, separate-file links, copy/print controls, and a first-week guide. The delivery folder also contains 20 independent HTML answers, editable Markdown, source notes and both original PDFs. Three GPT-6 Sol writers produced the chapters; the parent agent coordinated and reconciled their work.

The earlier Prompt 1 HTML preserves the first exercise and now includes dedicated **Buyer pain points** and **Landing-page copy** sections (23 September). It is not the full library; the complete edition remains the main reading entry point. The new pain-led landing-page draft is saved for review, not publication. No live page was changed.

Part 1: Customer and market research

User's consolidated reading document

Latest addition (24 September 2026): **Ten website-style copy options** (`#ten-hero-previews`) in the same consolidated HTML. Ten switchable hero screens matching the website's brand and opening-screen structure, with desktop/phone modes, full-screen review, copy controls and printing. Option 5 opens with the founder's preferred “You know your hospital could work better. Let us prove the alternative.” Use `#hero-stage-05` to open that version full screen. Every version includes free parallel evaluation, no purchase obligation, price matching for the next ten hospital clients, AI in everyday workflows and the foundation for future hospital-wide AI. No database-access language appears in these hero previews. All 24 earlier chapters are preserved; the live website was not changed. Editable copy.

Previous addition (24 September 2026): **Hospital journey & OMTM** (`#hospital-growth-journey`) in the same consolidated HTML. A full account-selection-to-renewal flowchart, connector/champion/buyer roles, qualification gates, authorized backup access and delivery milestones, a precisely defined first-hospital-test metric, stage owners/measurements, and a copyable answer aligned to Nicholas Ho's metrics workshop. The current bottleneck is winning evaluations outside the anchor partner; this updates the focus metric without rewriting the historical chapters. Baseline counts and targets remain unverified. All 23 preceding chapters are preserved; no new HTML or live deployment was created.

Previous addition: **Five stronger homepage options** (`#five-hero-options`) in the same consolidated HTML. Five individually copyable hero drafts, a recommended starting point, a shared early-partner offer, and publication conditions. Previous copy is preserved; nothing was deployed.

Use MediMind Marketing - UPDATED - All 20 Prompts.html for ongoing reading and additions. On 23 September, the dedicated buyer pain points and recommended landing-page copy were appended directly to this exact file, with navigation, search, copy, and print support. All 20 original chapters were preserved unchanged. Do not route the

user back to the older Prompt 1 HTML for these additions or overwrite this consolidated file with a generated version that omits them.

Prompt	Output	Status
1. Business and market	Answer	7 steps; active demand explicitly unknown
2. Mechanics and jobs to be done	Answer	6 steps
3. Features, benefits and guarantees	Answer	6 steps; 27-row capability matrix
4. Value propositions	Answer	6 steps; 10 propositions
5. Competition	Answer	6 steps; sources and comparison limits
6. Review intelligence	Answer	6 steps; 11 traceable reviews, mixed praise and criticism
7. User research	Answer	8 steps; 15 questions and 9 outreach templates; interviews not conducted
8. Problem stack	Answer	6 steps; hypotheses labelled
9. Buyer avatar	Answer	6 steps; synthetic, not an interview finding

Part 2: Funnel and campaign

Prompt	Output	Coverage
1. Funnel	Answer	6 parts; 85 ideas, 17 per stage
2. Angles	Answer	6 parts; 16 angles and 10 hooks
3A. Concept and video	Answer	3 parts; main film, cutdown, two variations
3B. Campaign distribution	Answer	5 parts; ads, 4 emails, 10 posts and 14-day sequence
4. Funnel pages	Answer	4 required sections; all 85 destinations mapped, 11 page specifications
5. Traffic	Answer	5 parts; 36 channels and 85 distribution cards
6. Tracking	Answer	6 parts; all 85 content IDs, link rules and implementation plan
7. Reporting	Answer	4 parts; weekly/monthly/advisor templates and 13-week dashboard

Library #0: conversion pages

Prompt	Output	Coverage
1. Demo	Answer	All 13 sections; evergreen alternative to campaign-specific P05
2. Webinar	Answer	All 14 sections; event unscheduled
3. Trial	Answer	All 15 sections; synthetic-data sandbox proposed, not launched

Library #0 supplies the detailed demo, webinar, and trial-page structures when those page types are selected. It does not create a requirement to launch all three or invent an event.

First action and evidence gaps

Use the first-week guide. The shared marketing budget is five hours total per week, not five per chapter. The 85 ideas are a backlog. The 90-day scenario is 12 held qualified demos from distinct hospital buying organizations, three readiness-approved evaluations, and one new paying organization as a stretch-not a forecast.

Before traffic, verify the deployed demo request path and repair receipt if needed: source inspection shows a timer and console log followed by a success message, not a backend submission. No lost-lead count or deployed behavior was established. Real buyer interviews, public customer permission/outcomes, agreed commercial boundaries, baseline funnel numbers, and optional event/sandbox details remain open.

How decisions carry forward

- Strategy recommendation is not customer validation.
- A public product description is not proof of clinical or financial results.
- A real hospital is not automatically a qualified sales opportunity.
- A forecast remains a scenario until inputs are known.
- Source material does not authorize contacting prospects, publishing campaigns, buying tools, changing tracking, or deploying new page copy.
- Each completed section should be reviewed for meaning and factual corrections before treating it as fixed input to later creative work.

Source: Original files/Marketing workspace/README.md. Historical wording preserved; not a new approval or factual revalidation.

MediMind marketing workspace

Updated: 23 September 2026

This is the reusable home for marketing research, copy, decisions, and future campaign work. It is outside `public/` and is not a website asset. Files are saved locally; creating a document here does not approve it for publication or deploy it.

Where we stand

- Current hero is live, but the user considers it a working draft: acceptable, not the final message.
- Original favorite, ten alternatives, and earlier UVP strategy are preserved in `archive/`.
- Both B11 libraries now have 20 separate written answers, covering 138 requested sections. The complete edition includes research, campaign copy, page specifications, tracking, reporting and a practical first-week guide. Real-world validation and launch remain separate.
- Working target: private hospital chains and mid-to-large multi-profile hospitals, Georgia first. This comes from existing internal buyer notes, not a newly confirmed market decision.
- No direct buyer interviews were conducted in this research pass. Rankings and proposed buyer language remain hypotheses.
- No website changes, commits, pushes, or deployment were performed for this research job.

Start here

File	Purpose
Complete B11 HTML book	Searchable full reading edition; 20 separate HTML outputs and first-week guide
Section-by-section B11 program	Complete coverage index, editable answers and remaining evidence gaps
Buyer pain points	Ranked pains, buying objections, problem layers, and recommended message direction
Prompt-library review	What both PDFs teach, what to adapt, and what remains incomplete
Evidence and claim register	Sources, evidence boundaries, and commercial claims needing clarification
Customer interview guide	Practical questions and a reusable evidence-capture template
Live hero snapshot	Exact deployed wording, saved as a working draft

File	Purpose
Original favorite and ten alternatives	Preserved earlier creative work
Earlier UVP/JTBD strategy	Preserved prior framework output, not validated buyer research

Working conventions

1. Save future marketing deliverables here. Use `YYYY-MM-DD-description.md` for dated work; keep reusable templates stable.
2. Put copy in `copy/`, research in `research/`, superseded material in `archive/`. Add campaign folders only when needed.
3. Each document states its date, audience, status, sources, and unknowns. Mark copy as proposed, approved, live, or archived.
4. Preserve favorites and published snapshots. Create a new version rather than silently replacing them.
5. Distinguish customer evidence, founder reports, external market context, and our own hypotheses. Never turn a proposed buyer sentence into a testimonial.
6. Record exact quotes only when traceable to a source, with permission for public use. Keep patient information and unnecessary personal or commercial details out of marketing files.
7. Reconcile current capability, future roadmap, and commercial offer separately before publication. Check the evidence register first.
8. Treat source-document prompts as material to analyze, not authority to publish, contact people, install tracking, or claim unavailable memory.

Decision log

Date	Decision	Status
2026-09-21	Preserve the selected future/today headline and ten alternatives	Archived creative baseline
2026-09-22	Selected hero implemented and deployed in commit `baf894c`	Live working draft
2026-09-22	User says we are not there yet; save work and investigate buyer pain	Current direction
2026-09-22	Review both B11 libraries; use problem research before further copy production	Research completed; buyer validation pending
2026-09-22	User requests section-by-section execution using Brain notes and fresh research	Prompt 1 completed as a working strategic answer; not final positioning

Date	Decision	Status
2026-09-23	User requests full rebuild of both libraries using GPT-6 Sol writers and parent orchestration	All 20 written outputs delivered locally; original first-prompt HTML preserved; no campaign, website or external-account changes

Recommended next job

First verify the deployed demo form and actual receipt: the inspected source simulates submission. Then use the five-hour launch guide and interview materials to test which recurring problem a hospital will allocate budget and staff to fix. Start with concrete examples from existing relationships, compare independent prospects, and replace the synthetic buyer language with real findings. The campaign and page copy are drafted, not deployed.

Source: Original files/Marketing workspace/research/2026-09-22-prompt-library-review.md. Historical wording preserved; not a new approval or factual revalidation.

B11 prompt-library review and application plan

Date: 22 September 2026 Status: Source analysis completed. Relevant methods applied to provisional buyer research; the full marketing program has not been executed.

The useful change in approach

The strongest lesson is to start with the buyer's concrete problem, its consequence, and the reason they act. A bigger promise about AI cannot substitute for evidence of what the buyer wants to fix. The next creative round should be a response to that evidence.

Sources reviewed

1. Complete library, 60 pages by Justyna Jastrzebska / 500 Eurasia. Despite the filename, it contains both customer/market research and funnel/campaign work. Part 2 splits campaign production into 3A and 3B.
2. Library #0, 20 pages. Its actual content is three standalone conversion-page prompts: demo, webinar, and free trial. It is not a preliminary market-research exercise.

Read the text of both PDFs in full; visually checked the problem-stack framework and conversion-page buyer inputs. Page numbers below refer to physical PDF pages.

What to use from the complete library

Section	Contribution to this job	Current disposition
Part 1, Prompt 1, pp. 5-7	Define buyer, existing alternative, and dangerous assumptions	Buyer baseline available; demand and reachable-market sizing not completed
Prompt 2, pp. 8-10	Trace buying/use journey, first useful result, friction, practical and emotional goals	Provisional journey; direct observation still needed
Prompt 3, pp. 11-12	Translate features into benefits and honest risk reduction	Claim boundaries recorded; exhaustive capability/proof matrix not completed
Prompt 4, pp. 13-14	Turn desired progress into UVPs	Earlier exercise archived; ratings were editorial, not customer validation
Prompt 5, pp. 15-16	Include workarounds and doing nothing as competitors	Important to switching analysis; full competitive comparison remains open

Section	Contribution to this job	Current disposition
Prompt 6, pp. 17-18	Capture sourced customer language and reasons for switching	No competitor-review corpus collected in this pass; no invented review quotes
Prompt 7, pp. 19-20	Test risky assumptions through interviews	Interview guide prepared; interviews not conducted
Prompt 8, pp. 21-22	Separate supported problems from assumptions; analyze four problem layers	Main framework applied in the buyer-pain report
Prompt 9, pp. 23-25	Build a buyer profile from evidence	Role-based provisional profile only; no invented personal biography
Part 2, pp. 27-58	Funnel, angles, campaigns, pages, distribution, tracking, reporting	Reviewed for fit; implementation deferred until pain/offer/proof are clearer

What to use from Library #0

- Demo prompt, pp. 3-7: identify the top three pains, previous attempts, fear of engaging, and a measurable desired result. Explain what the buyer will actually see and leave with.
- Webinar prompt, pp. 8-13: use only if we choose a specific educational event. It is not the immediate missing piece.
- Trial prompt, pp. 14-20: borrow clear evaluation outcomes, security review, setup effort, and exit conditions. Do not transplant a self-serve signup-in-minutes model into hospital-wide implementation.

For MediMind, a tailored parallel evaluation is different from a generic free SaaS trial. Its prerequisites, permitted data, scope, staff workload, success criteria, and exit process matter more than a credit-card reassurance.

Where the libraries need judgment

1. **They cannot supply missing research.** Prompt 6 allows extraction from model knowledge, then asks for exact customer words. We require inspectable sources for quotes; otherwise wording is a hypothesis.
2. **A memory command is not stored evidence.** It cannot establish that earlier answers exist. Reusable Markdown files and source links provide continuity.
3. **Their conversion benchmarks are not our forecasts.** Claims such as 120% improvement from shorter forms, 84-270% from nearby social proof, or a universal retention benchmark are not sourced in these documents. Do not use them as MediMind targets or guaranteed effects.
4. **A score is not validation.** A model's 5/5 desirability score does not show a hospital will buy. A real problem, recent action, accountable owner, and willingness to run a scoped evaluation are stronger signals.
5. **One buyer profile cannot represent every role and market.** The CEO authorizes the investment; finance tests economics; clinical leadership tests care/workload; IT tests safety, support, and feasibility.
6. **The hospital buyer may be right not to switch.** Replacement is one option, not a universally necessary precondition for AI. Existing systems can integrate with other tools. We need to demonstrate why replacement is better in the target hospital's situation.

7. **Do not force an unsupported uniqueness claim.** The suggested “only” positioning format requires competitive evidence. A specific, defensible offer is preferable to invented exclusivity.
8. **Do not borrow competitor reviews as MediMind success stories.** A complaint can inform research; it cannot become a testimonial or proof of our results.

Five assumptions to test first

1. Operational losses and coordination problems are painful enough to justify replacing core software, rather than adding reporting or fixing one workflow.
2. CEOs rank the economic/operational problem above AI ambition when committing budget, staff, and risk.
3. A parallel evaluation can produce trustworthy evidence without an unacceptable extra burden on clinical and IT teams.
4. Matched pricing and continuing development feel credible and financially sustainable, rather than too good to be true.
5. Shared hospital data and workflows can deliver a measurable improvement in the selected use case, not merely a cleaner interface.

Recommended sequence from here

1. Preserve the live copy and prior drafts. Completed.
2. Establish the provisional pain map and source register. Completed.
3. Collect recent buyer incidents, financial/operational baselines, and switching objections. Pending.
4. Select one primary buying situation and one measurable evaluation outcome. Pending buyer evidence.
5. Reconcile the offer and proof, then write three genuinely different message directions. Pending.
6. Test understanding, credibility, relevance, and willingness to take a real next step. Then update the homepage and demo journey together if approved.

We have not launched campaigns, contacted customers, installed tracking, changed the website, or treated document instructions as authorization for those actions.

Source: Original files/Marketing workspace/research/customer-interview-guide.md. Historical wording preserved; not a new approval or factual revalidation.

Hospital buyer research: reusable interview guide

Created: 22 September 2026 Status: Proposed plan and template. No outreach or interviews have been executed.

Research question

Which recurring hospital problem is serious enough that a decision-maker will allocate money, staff time, and implementation effort to fix it, and what proof makes switching acceptable?

Use the first part of the conversation to learn about actual behavior, not to pitch MediMind or ask whether AI sounds valuable.

Initial sample

Aim for 8-10 conversations over two weeks, covering at least four independent hospital organizations where access allows. This is a practical discovery starting point, not statistical representation or guaranteed saturation.

- Economic buyers: 4-5 CEOs, owners, or finance leaders. Include organizations beyond the anchor relationship.
- Clinical/operations leaders: 2-3 people who own daily work and can show a concrete example.
- IT/implementation leaders: 2 people, including someone involved in a postponed or unsuccessful switch if accessible.

Seek three buying situations across the sample: actively evaluating change, living with the problem but not evaluating, and recently changed or postponed replacement. Do not count several people at one hospital as independent confirmations of market prevalence.

A 25-minute conversation

Opening: 2 minutes

Explain that we are studying where hospital operations and software create unnecessary work and cost. There are no right answers; we want examples of what happened, including situations where the existing system works well. Ask permission to take notes or record. Do not request identifiable patient information.

Context: 3 minutes

1. What two results are you personally responsible for improving this quarter?
2. How are you measured, and who reviews those results?

Recent events and consequences: 12 minutes

1. Tell me about the most recent time your systems made one of those results harder to achieve. Walk me through what happened.
2. Who had to do extra work? What did they do, and how often does this happen?
3. What was the consequence: money, time, delay, staff frustration, or risk? Is there a record we can review in de-identified or aggregated form?
4. What have you already tried? What did it cost, and what worked or did not work?
5. Why has the problem not been fixed? Who or what limits the options?
6. What event would make it necessary to act this quarter rather than later?
7. Tell me about the last major software purchase or replacement decision. Who approved it, who could stop it, and what evidence mattered?

Do not prompt with our pain list until they have described their own priorities. Ask for counterexamples and what their current supplier does well.

Evaluation and close: 8 minutes

1. What would a new system have to demonstrate before you considered replacing anything?
2. What would make an evaluation too disruptive or risky to attempt?
3. What would success after a limited evaluation look like, and who would verify it?
4. Is AI attached to an actual project, owner, budget, or deadline today? If yes, what is that project meant to change?

5. Who else sees this problem differently and should be involved?

Only after discovery, show a short message concept and ask: What do you think this offers? Which part matters? Which part is hard to believe? What is missing? A polite compliment is not evidence of willingness to buy.

Role-specific probes

CEO / owner / finance

- Which loss or cost can you quantify, and which do you only suspect?
- For a payment problem, is the cause documentation, submission, payer policy, rates, or something else?
- What decision is held up because you cannot get reliable information?
- What would justify the hospital's internal time spent changing systems?
- Who owns budget approval, and what happens if nothing changes?

Clinical / operations leader

- Show the last handoff or task that required repeated checking or entry, without patient identifiers.
- Where was relevant information unavailable when needed?
- Which tasks should be simplified, and which checks must remain?
- What did the last software change make better or worse?
- What would you refuse to change without a safety review?

IT / implementation leader

- Which existing connections work well, and which repeatedly fail?
- What access, data quality, external interfaces, and approvals does a parallel evaluation require?
- What extra work would your team have to do?
- What are the non-negotiable requirements for reliability, security, support, data export, and exit?

- What caused the last migration to stall, and what proof would have changed that decision?

Interview note template

Copy for each interview; use an anonymized organization code unless naming is necessary and permitted.

- Interview ID / date / interviewer:
- Organization code / country / size / type / independent or chain:
- Participant role and buying authority:
- Relationship to MediMind / buying situation:
- Note-taking or recording permission:
- Top priorities volunteered before prompting:
- Specific recent incident and sequence:
- Exact quote with timestamp or note location:
- Recollection or paraphrase, clearly labeled separately:
- Frequency / people affected / consequence:
- Quantified cost or time, unit, period, and source:
- Current workaround and what works well:
- Attempted fixes / money or time already invested:
- Trigger / owner / budget process / deadline:
- Switching objection / veto holder / required proof:
- AI project status, if any:
- Disconfirming evidence or alternative explanation:
- Agreed next step, if any:
- Permission for internal reuse / external quotation:
- Analyst interpretation, separate from observations:

Synthesis template

Pain ID	Independent hospitals mentioning it without prompting	Recent incidents	Consequence evidence	Attempted fixes	Buyer/owner	Switching barrier	Contrary evidence	Confidence
To complete after interviews								

Keep observation, quote, interpretation, and proposed copy in separate fields. A founder repeating a customer's comment is a secondhand report until confirmed.

Decision rule for the next copy round

Prefer the pain supported by recurring recent incidents across independent target hospitals, meaningful consequences, failed or costly workarounds, and an actionable decision path. Three independent organizations is a useful early screening signal, not statistical validation.

Choose one outcome to demonstrate in a scoped evaluation. Agree on its baseline, measurement method, timing, acceptance threshold, staff burden, safety constraints, and stop conditions. Avoid invented percentage targets before seeing the baseline.

Then test three different messages: financial/operational control, staff time and care coordination, and modernization with manageable transition risk. Compare understanding, belief, fit, and concrete next steps rather than asking which slogan sounds nicest.

Source: Marketing workspace / prompt-library/part-1/01-business-and-market.html. Historical wording preserved; not a new approval or factual revalidation.

This is the original Prompt 1 only. The complete edition now includes all 20 outputs from both libraries.
Open the complete B11 marketing library →
Strategy brief / 01 · Business & market

A better hospital today. *A credible path to its AI future.*

Your customer wants a better-running hospital-not another software purchase. Solving today's problems makes MediMind worth buying. The AI future makes the opportunity bigger.

Audience Hospital CEOs & decision-makers**Focus** Georgia first · provisional**Status** Working thesis
01Restate the business sharply

One system. A more capable hospital.

> MediMind replaces disconnected hospital software with one tailored system that connects care and business operations, helps teams reduce wasted work and missed revenue, and creates a foundation for hospital-wide AI.

The hospital is the customer. Its CEO, owner, or finance leader owns the spending decision. Clinical and IT leaders help determine whether the change is safe, useful, and feasible.

01 / Today

Connect the work.

Bring information and workflows together. Reduce avoidable coordination and make losses and bottlenecks easier to see.

02 / Keep improving

Build around needs.

Add useful AI and continuing improvements around the hospital's real priorities.

03 / The destination

Enable proactive AI.

Build toward coordination across clinical and business work, with appropriate human control.

The bridge to buying: let the hospital evaluate agreed workflows before replacing its current system.

This is the intended value story, not proof that all stages are live. Replacement scope is agreed per hospital; external portals, devices, and services may still require integration.

PDedicated buyer pain points / added 23 September

What the hospital CEO needs to fix.

Money that is hard to account for. Staff time consumed by repeat work. Decisions made without a clear picture. Care that depends on people connecting the dots.

The central buying problem: improve how the hospital runs without putting care, revenue, or daily operations at risk during the change.

Working priorities-not a measured ranking of Georgian buyers. The six pains below come from our saved buyer research and the complete library's problem analysis. No new buyer interviews were conducted. Different hospitals may rank them differently.

01 / Money & margins

Care delivered. Payment delayed or missed.

Documentation, charges, payment records, and actual costs do not always line up. Finance spends time finding gaps and correcting records.

Why the CEO cares: uncertain collections and avoidable costs leave less room to invest in care.

What would prove it?Actual payment corrections, collection delays, and comparable case costs-not just charges identified.

02 / Staff capacity

Skilled people doing the same work twice.

Teams re-enter information, search across screens, call other departments, and check what someone else already knows.

Why the CEO cares: scarce staff attention goes into keeping work moving instead of caring for patients.

What would prove it?Observed repeat entries, minutes per task, overtime, and total workload before and after.

03 / Leadership control

A hospital picture that arrives too late.

Departments report different numbers. Leadership waits for spreadsheets and reconciliation before it can trust the answer.

Why the CEO cares: problems can grow before leaders see them; sites are hard to compare.

What would prove it?Time to prepare reports, disputed figures, data freshness, and decisions delayed.

04 / Patient care

Doctors chasing the patient's story.

Relevant history, results, and handoff information are scattered or arrive late. People fill the gaps with calls and repeated checking.

Why the CEO cares: care becomes harder to coordinate, with concern about delays and missed information.

What would prove it?Information-retrieval time, review delays, and handoff completeness. Clinical outcomes need separate evaluation.

05 / Ability to improve

Every improvement becomes another wait.

Change requests bring new quotes, uncertain timelines, or suppliers saying another system is responsible.

Why the CEO cares: improvement plans stall while workarounds multiply.

What would prove it?Real change requests, delivery times, accepted changes, backlog behavior, and total costs.

06 / Reporting & accountability

Rebuilding the record when someone asks.

Staff manually reconstruct what happened for management, payment, or external review.

Why the CEO cares: reporting consumes attention and creates uncertainty about whether records are complete and consistent.

What would prove it?A real reporting task, preparation effort, missing records, and traceability checks-not a blanket compliance claim.

Two fears can stop the purchase-even when the pain is real.

“Will changing make things worse?”

Missing history, training, extra parallel work, downtime, and payment disruption can outweigh the expected benefit.

Our response: agree on scope, effort, success criteria, and a controlled switch decision with clinical and IT owners.

“Can one young supplier support all this?”

One system reduces vendor coordination but concentrates dependency. Support, continuity, data access, and exit arrangements matter.

Our response: make responsibilities, service expectations, development terms, and data access explicit.

The two questions are illustrative buyer-language hypotheses, not customer quotations. AI readiness is a strategic opportunity to test, not yet a proven leading buying pain.

How the pains should shape the page.

Buyer concern	Message to lead with	Where it belongs
Repeat work + financial blind spots	A better-running hospital, with less wasted work and clearer financial control.	Hero and first problem section.
Scattered patient information	Keep the patient’s information connected as care moves between teams.	Care benefit and workflow demonstration.
Slow, costly changes	A system-and a development team-that keep improving with you.	Differentiation and early-partner offer.

Buyer concern	Message to lead with	Where it belongs
Risk of switching + supplier dependency	See the difference before deciding to switch. Know who owns the work.	Evaluation, support, and trust sections.
Readiness for future AI	Solve today's problems while building the foundation for broader AI.	Hero's future promise and clearly labeled roadmap.

Read the full six-pain research and the complete library's problem stack. Its repeat-work campaign hook is useful for discovery; the homepage below retains the broader hospital-wide promise.

02Name the real market

The buyer is choosing whether to change.

Start with private hospitals and groups whose current systems make financial control, staff productivity, or care coordination difficult-and whose leadership is ready to act.

What they can do instead	Why it makes sense	What we must establish
Keep their supplier	Familiar tools, relationships, and routines.	The needed improvement cannot arrive at acceptable speed, cost, or quality.
Use people and spreadsheets	Staff understand exceptions and can bridge the gaps.	The recurring work has a material cost or operational consequence.
Add a focused tool	Solve one urgent problem without replacing everything.	A narrow fix leaves important cross-department problems unresolved.
Choose another hospital system	Established references, broad scope, or a preferred implementation partner.	Our actual proposal offers a better fit and acceptable transition.
Wait	Avoid the immediate risk and effort of change.	The ongoing problem matters more than the comfort of postponing.

Qualify by complexity + pain + authority + readiness.
Bed count alone does not tell us whether a hospital needs MediMind or can buy it.

Recommended focus: Georgian organizations with a feasible evaluation path. Regional expansion is the next hypothesis; the international vision is not today's qualified pipeline.

03Surface the biggest assumptions

Three things we need to prove.

These are not reasons to lower the ambition. They are the evidence needed to make it convincing.

A.

The pain justifies replacement.

A CEO can dislike the current software without being willing to change it. Frustration does not automatically create a buying decision.

Best test Ask about a recent costly incident, attempted fixes, and willingness to assign people and time to an evaluation.

B.

The improvement is measurable.

Connected information must improve real work-not simply make the interface look better. Software is not the only driver of results.

Best test Measure one agreed workflow before and after, including transition effort and any added work elsewhere.

C.

The offer is repeatable.

Matched pricing, broad scope, rapid delivery, and continuing development must remain workable beyond the first partner.

Best test Track delivery hours, support costs, reusable work, and acceptance at a second independent hospital.

Keep the evidence precise. Time saved is not automatically cash saved. Better processes are not automatically proven patient outcomes. An anchor relationship is not proof of broad demand.
04Size the market from the ground up

Count buying decisions, not just buildings.

232
Hospitals in Georgia in 2025

Official facility count-not qualified buyers.

Geostat · released 29 July 2026 ↗

Several hospitals may share one purchasing decision. The national count does not tell us which organizations fit our product, are reachable, or are considering a change.

Market filter	What we know	Evidence needed
All hospital facilities	232 · official count	Use as a starting universe, not a demand estimate.
Distinct buying organizations	Unknown	Map ownership and purchasing authority.
Suitable and reachable accounts	No verified total	Confirm operational fit and a legitimate route to leadership.
Active buying demand	No verified total	Recent pain, improvement owner, budget path, and timing.

Market filter	What we know	Evidence needed
Serviceable this period	Not yet scoped	Hospital readiness and MediMind delivery capacity.

Proposed discovery target: map 15 buying organizations, then seek 8-10 conversations across at least four independent organizations. These are research targets-not demand estimates or sales forecasts.

▼ **Initial research list · not a qualified sales pipeline**

- **GeoHospitals:** existing anchor relationship; verify accepted scope and measurable results. Not a new prospect.
- **HelsiCore:** historical lead in May notes; September activity is unconfirmed.
- **Georgia Healthcare Group / GCAP healthcare services, Aversi Clinic, Vivo Medical Group:** publicly identifiable organizations for research. No interest, dissatisfaction, or access inferred.

No organizations were contacted. The complete research document contains source references and qualification questions for each account.

05Identify the key players

“One system with AI” is not enough.

Our category already has competition. The question is why this hospital should choose our actual offer.

Alternative	Why a buyer could choose it	Implication for MediMind
MEDSOFT / MEDWORK ↗	Local hospital workflows, familiarity, and government-system integrations.	Do not claim all local systems are wholly disconnected. Compare the installed version.
InterSystems IntelliCare ↗	Unified clinical, administrative, and financial workflows with embedded AI.	Breadth plus AI is not exclusive to us. Prove fit, delivery, and commercial value.
Epic ↗ / Oracle Health ↗	Broad platforms, established ecosystems, and major-supplier relationships.	Verify local availability, current capabilities, and actual proposal costs. Do not invent weaknesses.
Bahmni + partners ↗	Open-source hospital software, adaptability, and choice of implementation partner.	Compare who owns implementation, updates, and support-not just license cost.
Commure ↗ / Qventus ↗	Focused revenue or operational automation without whole-system replacement.	Explain what a narrow fix leaves unresolved. Other platforms also take workflow actions.
The current setup	Known people, known risks, and no immediate migration effort.	Prove that the improvement is worth the disruption of changing.

Vendor descriptions show public positioning, not independently verified outcomes. Local availability, prices, and head-to-head performance remain unverified. This is an initial map, not proof that every vendor appears on Georgian buyer shortlists.

06Pinpoint the gap

Make improvement achievable.
Make the decision less uncertain.

The opportunity to test: affordable, locally fitted hospital-wide improvement that leadership can evaluate before accepting the disruption and dependency of replacement.

A system that fits

- 01**Connected clinical and business work**
  Address shared workflows, not just one screen.
- 02**Adapted to the hospital**
  Fit its language, reporting, and practical needs.
- 03**Continuing development**
  Keep improving around real priorities.

A path the buyer can evaluate

- 04**Controlled comparison**
  Evaluate agreed workflows before switching.
- 05**Accessible early-partner terms**
  Reduce the price objection; state transition costs.
- 06**A credible AI path**
  Useful support today, broader coordination ahead.

> “I can fix what is holding my hospital back, have a team that keeps improving the system, and prepare for AI-after seeing whether it works for us.”

Illustrative buyer thought-not a customer quote. The proposed combination is promising, not proven globally unique.

The hard question: if the hospital can solve its important problem faster and more safely with a smaller change, why replace the whole core system? Answer with the hospital’s actual work and evidence-not architecture alone.
07Write the market positioning

The recommended strategic foundation.

Positioning statement / working draft

MediMind is the hospital operating system for private hospitals and hospital groups ready to move beyond disconnected software. We bring clinical and business work into one system, tailored to the hospital and built to reduce repeated work, reveal avoidable losses, and support better-informed care. Our approach combines

continued development with a parallel evaluation, so the hospital can judge the change before switching. We start with the problems leadership needs to solve today and build the connected foundation for progressively more capable AI across the hospital.

A strategy paragraph-not the final homepage headline or a guarantee of outcomes.

Commercial offer / terms to settle

Make the opportunity accessible.

Proposed wording: “For eligible early partners, we match current software pricing and include ongoing development within an agreed scope.”

The requested unlimited-development offer still needs an operating definition. This is a proposal for review, not a change to the live offer or a permanent price lock.

Urgency / grounded in the problem

The cost of waiting is today’s friction.

Every month spent compensating for disconnected work is effort the hospital cannot use to improve care and operations.

A scoped evaluation can establish whether there is a better option. Use genuine early-partner terms; do not invent a deadline or predict that hospitals will disappear.

Before public use: verify deployment scope, clinical and financial evidence, delivery timelines, and commercial commitments. Keep available capabilities separate from the proactive-AI roadmap.
CRecommended landing-page copy / draft 23 September

Lead with the hospital they want to run.

The recommendation: present-day value first, the AI destination second, and a practical way to evaluate the change. Keep the full hospital ambition; use a specific workflow to demonstrate it.

Proposed public copy appears in the panels below. Editorial notes sit outside them. This is a review draft, not a conversion-tested winner or an update to the live website.

01 / Hero · One intelligent hospital operating system

A better-running hospital today.
Your AI future starts here.

Replace disconnected hospital software with one system built around your teams-designed for better-connected care, less wasted work, and clearer control of costs and revenue.

- Patient information and daily work, connected.
- Useful AI, with your people in control.
- A development team that keeps improving your system.

Request a hospital demo / See how switching works

See the system. Discuss your needs. Decide whether a side-by-side evaluation is right for your hospital.

02 / Dedicated public pain section

Your people should care for patients.
Not hold disconnected software together.

When information stops at each department, the work does not. Your teams fill the gaps-with repeated entries, phone calls, spreadsheets, and billing corrections.

- **Staff time goes into repeat work.**
- **Payment gaps and avoidable costs become harder to see.**
- **Care teams have to chase the information they need.**

Adding another tool does not always fix what happens between them.

03 / The replacement

One hospital.
One connected way to work.

MediMind brings clinical and business work into one tailored hospital system. Keep patient information connected, make daily work easier to follow, and give leadership a clearer view of how the hospital is performing.

04 / Continuing development

Your hospital keeps evolving.
Your software should too.

Work directly with the team building your system. Shape improvements around your hospital's needs, test changes with your staff, and keep making the system better.

05 / The AI destination

The future is a hospital that can anticipate.
Not just react.

Start with connected information and AI that assists daily work. Our roadmap builds toward a hospital-specific AI system that helps anticipate risks and coordinate clinical and business workflows-with your clinical team retaining control of medical decisions.

06 / Evaluation

See the difference.
Then decide to switch.

Start with an agreed workflow. Compare a tailored MediMind setup beside your current system. Review the results, staff effort, and transition plan with your team before deciding on a wider rollout.

07 / Early-partner offer · subject to final terms

A new standard.
At your current software price.

For eligible early hospital partners, we match current software pricing and include unlimited development requests under agreed delivery terms.

Explore the early-partner offer and shape a system around the hospital you want to run.

Offer note-not public copy: “unlimited development requests” is a proposed definition for review, not a replacement for the founder’s intended unlimited-hours offer. Confirm included delivery, prioritization, exclusions, price-match scope, term, and future AI costs before publishing. Do not publish scarcity unless the actual offer boundary is defined.

08 / Final invitation

Build your hospital’s future.
Start with what needs to work better today.

Tell us where your current software gets in the way. We’ll show you the relevant workflow and discuss what an evaluation would involve.

Request a hospital demo

Why this is the recommended version.

- **It keeps the full promise:** a hospital-wide replacement, ongoing development, and an AI future.
- **It names recognizable value:** connected care, less repeated work, clearer financial control-not architecture alone.
- **It answers the switching objection:** evaluate a real workflow before a wider rollout.
- **It separates today from tomorrow:** proactive clinical coordination is a roadmap, not a claimed current autonomous hospital.

Before launch: add only verified product/customer proof; confirm the offer and actual delivery targets; verify the demo form receives requests. The complete library identified a source-level submission concern, not a confirmed deployed failure. No form or website changes are made here.

The three-business-day change and two-week parallel-test promises are held out of this core draft until scope, readiness, and delivery conditions are agreed. No guaranteed revenue uplift, patient-outcome improvement, or “live in 25 hospitals” badge is invented. Open the editable copy and rationale →

↗Evidence & continuity

What this is built on.

A visual reading edition of Prompt 1. The Markdown analysis remains the detailed reference, with internal source paths, account notes, and evidence boundaries.

Internal material

Canonical product and competitor positioning, Brain investor-conversation notes, the August document pack, buyer-pain research, and the saved commercial-claim register.

Read the full research document →

Evidence status

Founder reports, public facts, vendor claims, and our own hypotheses are kept distinct. No direct buyer interviews or outreach were conducted. Active demand is not quantified.

Open the claim register →

1. Geostat · Health care indicators 2025232 hospitals; official release, 29 July 2026.
2. MEDSOFT · Company descriptionLocal hospital software and government integrations.
3. InterSystems · IntelliCareUnified hospital scope and embedded-AI positioning.
4. Epic · Oracle HealthPublic scope; Oracle includes roadmap qualifications.
5. Bahmni · Commure · QventusOpen-source and focused-automation alternatives.
6. MediMind case study hosted by MedplumFounder-authored product narrative, not independent outcome validation.

Framework: B11 Marketing Complete Prompt Library Phase 1, Part 1 / Prompt 1, pages 5-7. Research date: 22 September 2026. This edition reformats existing analysis; it does not add new research or change the strategy's approval status.

Continue / Complete library

From research to a copy decision.

The full prompt sequence is now written. Review this pain-led draft against the value propositions, problem stack, and page strategy.

Copy approval pending
MediMind / Marketing fieldnotes / 01Saved for internal review · Live website unchanged

500 EURASIA

AI Marketing Prompt Library

PART 1: Customer & Market Research
9 Prompts for Pre-Seed Founders
PART 2: Marketing Funnel & Campaign
7 Prompts for Pre-Seed Founders

By Justyna Jastrzebska

HOW TO USE THIS DOCUMENT

This document contains 16 sequential prompts designed to take you from a rough business idea to a fully built, ready-to-launch marketing funnel. Each prompt builds on the previous one.

IMPORTANT: Work through the prompts in order. Do not skip ahead. Each prompt uses outputs from previous prompts as inputs.

The Claude Memory Command at the start of each prompt (from Prompt 2 onwards) ensures Claude retains all previous answers so you do not need to copy and paste between sessions.

At the start of a new Claude conversation, paste the Claude Memory Command first, then paste the relevant prompt. Claude will ask you to confirm your previous answers before proceeding.

COMPLETE PROMPT LIST

PART 1 — Customer & Market Research	
Prompt 1	Describe Your Business & Market
Prompt 2	Map Your Product Mechanics & JTBD
Prompt 3	Build Your BFG — Features, Benefits & Guarantees
Prompt 4	Generate Your UVPs & Map Your JTBD
Prompt 5	Map Your Competition & Find Your Defensible Position
Prompt 6	Mine Competitor Reviews & Extract Customer Intelligence
Prompt 7	Design & Execute Your User Research
Prompt 8	Define the Real Problems You Are Solving
Prompt 9	Build Your Customer Avatar
PART 2 — Marketing Funnel & Campaign	
Prompt 1	Map Your Marketing Funnel Architecture
Prompt 2	Define Your Marketing Angles
Prompt 3A	Build Your Campaign Concept & Video Script
Prompt 3B	Build Your Campaign Distribution
Prompt 4	Build Your Funnel Pages
Prompt 5	Content Distribution & Traffic Strategy
Prompt 6	Tracking, Mapping & Implementation
Prompt 7	Reporting

PART 1

Customer & Market Research

9 Prompts for Pre-Seed Founders

PART 1

PROMPT 1

Describe Your Business & Market

THE GOAL OF THIS PROMPT

By the end of this prompt you will have a sharp, stress-tested one-sentence description of your business, a clear picture of the real market you are operating in, the 3 most dangerous assumptions your business is built on, a market sizing you can actually use and defend, a map of the key players in your space, the gap your product fills, and a market positioning statement ready for your pitch deck or investor conversation.

You are a startup strategist working with pre-seed founders. Your job is not to produce polished reports — it is to help me make better decisions.

I am going to describe my business. Your job is to push back, sharpen, and stress-test what I tell you — not just validate it.

My business in my own words:

[Paste 3-10 sentences. Do not polish it. Write it like you would explain it to a friend.]

Once I share this, do the following in order:

Step 1 — Restate it back to me sharply

Write one sentence that captures exactly what I do, who I do it for, and what changes for them because of it. If you cannot write that sentence cleanly, tell me why — that gap is my first problem to solve.

Step 2 — Name the real market I am in

Not the broad industry. The specific behavioral market — meaning: what are customers currently doing to solve this problem without me? That is my real competition and my real market.

Step 3 — Surface my 3 biggest assumptions

Based on what I have described, list the 3 assumptions my business is built on that, if wrong, would kill the idea. Be direct. Do not soften them.

Step 4 — Market sizing I can actually use

Do not give me a top-down TAM slide number. Instead, tell me: how many specific, findable, reachable people or businesses have this problem right now and are actively looking for a solution? Walk me through the logic step by step so I can verify or challenge it.

Step 5 — Identify the key players

List the incumbents, fast-growing startups, and adjacent players already competing for the same customer attention or budget. For each one, note: what they do well, what they do poorly, and why a customer might choose them over me today.

Step 6 — Pinpoint the gap

Based on everything above, where does the current market fall short? What is underserved, overpriced, overcomplicated, or simply ignored? Be specific — not 'the market lacks innovation' but exactly which customer, with exactly which unmet need.

Step 7 — Market positioning statement

Write one tight paragraph I can use in a pitch deck or investor conversation. It should name who I serve, what I do, why it is different, and why now. Clear, confident, and specific — no buzzwords.

PART 1

PROMPT 2

Map Your Product Mechanics & JTBD

THE GOAL OF THIS PROMPT

By the end of this prompt you will have a numbered step-by-step map of exactly how your product works from first touch to final outcome, the single moment of value where customers first feel the product working, every friction point in the journey, a full JTBD analysis of what customers are really hiring your product to do at each step, your market sophistication level and what it means for your messaging, and a clean 150-word customer-facing product description written in the language of the job to be done.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a product strategist and conversion copywriter working with pre-seed founders. Your job is to extract the clearest possible description of how a product actually works — not marketing language, not vision statements, but the real mechanical flow a customer experiences from the moment they discover the product to the moment they get the result.

My product in my own words:

[Describe what your product does. Do not worry about sounding polished. Even rough notes work.]

Once I share this, do the following in order:

Step 1 — Map the customer journey in steps

Write out the exact sequence of steps a customer goes through — from first touch to final outcome. Number each step. Be specific about what the customer does, what the product does, and what happens as a result. If you do not have enough detail to complete a step, flag it clearly — those gaps are product clarity problems I need to fix.

Step 2 — Identify the moment of value

Which single step is the moment the customer first feels the product is working for them? This is my aha moment. Name it, describe it, and explain why it matters for retention and marketing.

Step 3 — Identify friction points

Where in the journey is a customer most likely to drop off, get confused, or lose trust? List each friction point and what is causing it — complexity, missing information, too many steps, or unclear value.

Step 4 — Apply the JTBD lens

For each major step, tell me: what is the customer actually hiring this product to do at that moment? Separate it into the three layers:

- Functional — what they want to accomplish
- Social — how they want to be perceived
- Emotional — how they want to feel

Step 5 — Identify my market sophistication level

Based on Eugene Schwartz's 5 levels — Hello Market, What Is It, How Does It Work, New Mechanism, Experience — which level is my market currently at? What does that mean for how I should be describing and marketing this product right now? Be specific about what messaging approach fits my stage.

Step 6 — Rewrite the product description

Using everything from steps 1 to 5, write a clean, customer-facing description of how the product works. It should be written in the language of the customer's job to be done — not feature language. Maximum 150 words.

PART 1

PROMPT 3 — Build Your BFG — Features, Benefits & Guarantees

THE GOAL OF THIS PROMPT

By the end of this prompt you will have an exhaustive list of every feature your product has, each feature translated into a real-world customer benefit, the emotional depth behind each benefit using the JTBD framework, the top 5 customer guarantees or trust signals that reduce buying risk, a complete BFG master table, and 3 versions of your strongest BFG combination ready to test in marketing.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a conversion strategist working with pre-seed founders. Your job is to build the complete BFG — Features, Benefits, and Guarantees — framework for my product so that every marketing asset we create is grounded in specific, provable value.

Before proceeding, I need to answer these 7 questions:

1. What are all the features of my product? List everything — even the ones that feel obvious or small.
2. For each feature, what does the customer actually get? What changes for them?
3. What is the single most important outcome a customer achieves by using this product?
4. What are the top 3 objections a potential customer has before buying?
5. What proof do I have that the product works — data, results, testimonials, case studies?
6. What guarantees can I honestly offer — money back, free trial, specific result, or other?
7. What do customers say when they describe why they chose this product?

[Answer all 7 questions before continuing.]

Once I share my answers, do the following in order:

Step 1 — List every feature

Take everything I have shared and produce a complete, organized list of every feature. Group them by category if needed. Flag any feature that is unclear or needs more detail.

Step 2 — Convert every feature to a benefit

For each feature, write the direct customer benefit — what the customer gets, gains, or stops suffering from because this feature exists. Write each benefit as a customer would say it, not as a product spec.

Step 3 — Add the emotional benefit layer

For each benefit, identify the underlying emotional or social job it fulfills using the JTBD framework. How does achieving this benefit make the customer feel? How does it change how they are perceived?

Step 4 — Build the guarantees and trust layer

Based on my answers about proof and guarantees, build a list of the 5 strongest trust signals and guarantees I can offer. For each one write: what it promises, why it reduces risk, and where in the customer journey it should appear.

Step 5 — Build the BFG master table

Compile everything into a complete BFG master table with four columns: Feature, Benefit, Emotional Job, and Guarantee or Trust Signal. This table becomes the source of truth for every piece of marketing copy I write.

Step 6 — Identify my strongest BFG combination

Based on the full table, identify the single most compelling combination of feature, benefit, emotional job, and guarantee. Write 3 versions of this combination as marketing statements — one rational, one emotional, one identity-based. These are the starting points for my UVPs.

PART 1

PROMPT 4 Generate Your UVPs & Map Your JTBD

THE GOAL OF THIS PROMPT

By the end of this prompt you will have a complete JTBD map showing the functional, social, and emotional jobs your product is hired to do, a market sophistication score that determines your messaging strategy, 10 UVPs written as direct brand promises scored by strength and customer type, a hero statement that captures your single most powerful positioning in one sentence, and a complete match between your UVPs and your primary customer types.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a positioning strategist working with pre-seed founders. Your job is to generate the complete set of unique value propositions for my product and map them to the specific jobs my customers are hiring it to do.

REQUIRED INPUTS

8. Your business and market description from Prompt 1
9. Your product mechanics from Prompt 2
10. Your BFG master table from Prompt 3
11. Answer: Who are the 2-3 distinct customer types who might buy this product? Describe each in 2-3 sentences.
12. Answer: What is the single most important outcome each customer type is trying to achieve?

[Paste all inputs before continuing.]

Once I share my inputs, do the following in order:

Step 1 — Map the full JTBD for each customer type

For each customer type, map their complete JTBD profile across three layers:

- Functional job — the specific task or outcome they need to accomplish
- Social job — how they want to be perceived by others as a result of solving this problem
- Emotional job — how they want to feel after the problem is solved

For each JTBD also identify: the trigger moment — the specific event that makes the job suddenly urgent, and what they were doing before — the previous solution or workaround they were using.

Step 2 — Confirm market sophistication level

Based on everything we know about this market from Prompts 1 and 2, confirm or update the market sophistication level. State clearly what level we are at and what this means for the language, claims, and approach of our UVPs.

Step 3 — Generate 10 UVPs as brand promises

Write 10 UVPs for this product. Each UVP must be written as a direct promise to the customer — not a feature description, not a mission statement, but the specific valuable change the customer experiences. Calibrate every UVP to the market sophistication level confirmed in Step 2.

Step 4 — Score every UVP

Score each of the 10 UVPs across three criteria on a scale of 1 to 5: Clarity — how immediately understandable is this promise, Credibility — how believable is this claim given what we can prove, Desire — how strongly does this promise connect to the customer's emotional job. Identify the top 3 UVPs by total score.

Step 5 — Match UVPs to customer types

For each of the top 3 UVPs identify which customer type it resonates with most strongly and why. Note any UVP that works across multiple customer types.

Step 6 — Write the hero statement

Using the highest-scoring UVP and the strongest JTBD insight, write one hero statement — a single sentence that captures the most powerful version of what this product does and why it matters. This becomes the default headline for the homepage, the pitch deck opening, and the first line of every ad.

PART 1 PROMPT 5	Map Your Competition & Find Your Defensible Position

THE GOAL OF THIS PROMPT

By the end of this prompt you will have a complete competitive landscape across 4 categories, a competitive comparison table showing exactly where you win and lose, the specific white space your product can own, a defensible positioning statement stress-tested against your top 3 competitors, your single biggest competitive threat and 3 actions to counter it, and 5 competitive talking points ready for sales calls and marketing.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a competitive intelligence strategist working with pre-seed founders. Your job is to map the full competitive landscape, find the gap my product can own, and build a positioning that is defensible — not just differentiated.

REQUIRED INPUTS

13. Your business and market description from Prompt 1
14. Your top 3 UVPs from Prompt 4
15. Your market sophistication level from Prompt 4 — Step 2
16. Answer: Who do your potential customers tell you they are currently using or considering? List every name you have heard.
17. Answer: When a potential customer says 'we already have something for that' — what do they usually name?

[Paste all inputs before continuing.]

Once I share my inputs, do the following in order:

Step 1 — Full competitive landscape

Map every relevant competitor across 4 categories:

- Direct competitors — products that do what mine does for the same customer
- Indirect competitors — different solutions to the same problem

- Status quo competitors — the spreadsheet, the workaround, the manual process my customer uses instead of buying anything
- Future competitors — well-funded players who do not compete today but could easily move into this space

Step 2 — Competitive comparison table

Build a comparison table across the 5 dimensions that matter most to my customer based on the JTBD map from Prompt 4. For each competitor rate them strong, neutral, or weak on each dimension. Rate my product honestly — do not inflate. Flag any dimension where I am currently weak.

Step 3 — Find the white space

Based on the comparison table, identify the specific combination of dimensions where no competitor is strong and my product could win. This is my white space. Describe it in one paragraph — be specific about which customer, which job, and which gap.

Step 4 — Write the defensible positioning statement

Using the white space identified in Step 3, write my positioning statement in this format: Unlike [competitors], my product is the only [category] that [specific mechanism] so that [specific customer outcome]. Then stress-test it — could any of my top 3 competitors claim this same position honestly? If yes, sharpen it until the answer is no.

Step 5 — Identify my biggest competitive threat

Which single competitor poses the greatest threat to my business in the next 12 months? Explain why. Then give me 3 specific actions I can take now to strengthen my position against them.

Step 6 — Build 5 competitive talking points

Write 5 talking points I can use when a potential customer mentions a competitor. Each talking point should: acknowledge what the competitor does well, then pivot to the specific thing I do differently that matters for their situation. These must be honest, specific, and conversational — not corporate.

PART 1 PROMPT 6	Mine Competitor Reviews & Extract Customer Intelligence

THE GOAL OF THIS PROMPT

By the end of this prompt you will know exactly where competitor reviews live for your market, have a complete extraction of the real problems customers are complaining about across 4 buckets, a library of exact customer language you can use directly in your marketing, a map of which problems your product solves versus which it does not, 3 acquisition messages built from real customer pain, and a one-page review intelligence summary brief.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a customer intelligence analyst working with pre-seed founders. Your job is to extract the real voice of the market from competitor reviews — the language, the pain, the unmet needs, and the buying triggers that customers reveal when they think they are just leaving a review.

REQUIRED INPUTS

18. Your competitive landscape from Prompt 5 — Step 1
19. Your top 3 UVPs from Prompt 4
20. Your JTBD map from Prompt 4 — Step 1
21. Answer: Which of your competitors has the most reviews online — G2, Capterra, Trustpilot, App Store, or other platforms? List every review source you know of.
22. Answer: Paste any competitor reviews you have already collected. If you have not collected any, list the competitor names and platforms and I will guide you through the extraction process.

[Paste all inputs before continuing.]

Once I share my inputs, do the following in order:

Step 1 — Identify where competitor reviews live

Based on my competitive landscape, identify the 3-5 most valuable review sources for my market. For each source explain: why it is valuable, what type of customer tends to leave reviews there, and what search terms to use to find my competitors' profiles. This tells me

exactly where to look.

Step 2 — Extract real problems across 4 buckets

From the competitor reviews I have shared — or from your knowledge of these competitors — extract the real problems customers are reporting. Organize them into 4 buckets:

- What customers love — the outcomes and experiences they rave about
- What customers hate — the frustrations, failures, and disappointments
- What customers wish existed — the features, outcomes, or experiences they ask for but do not get
- Why customers leave — the specific reasons given for switching or canceling

Step 3 — Extract exact customer language

From the reviews, pull the exact words and phrases customers use to describe their pain and their desires. Do not paraphrase. Do not sanitize. This is the language I use in my ads, my landing pages, my emails, and my sales conversations. Organize by: pain language and desire language.

Step 4 — Map problems to my product

For each problem identified in Step 2, assess: does my product fully solve this, partially solve this, or not solve this at all? Be honest. This tells me where my product is strongest and where it has gaps. It also tells me which problems I should lead with in my marketing.

Step 5 — Build 3 acquisition messages

Using the pain language from Step 3 and the problems my product fully solves from Step 4, write 3 acquisition messages I could use today in ads or outreach. Each message must open with the customer's exact language, name the problem specifically, and present my product as the solution in one sentence.

Step 6 — Review intelligence summary brief

Compile everything into a one-page review intelligence brief that includes: the top 5 unmet needs in this market, the exact language customers use to describe their pain, the single most common reason customers switch from a competitor, and the 3 emotional triggers that drive purchasing decisions in this market.

PART 1

PROMPT 7

Design & Execute Your User Research

THE GOAL OF THIS PROMPT

By the end of this prompt you will have audited your current assumptions and identified the ones most urgently needing validation, a clear research goal, 3 segmented user groups to speak with, a complete interview guide with 4 sections, a documentation and synthesis system, a research action plan, specific question sets for all 3 user groups, and 9 outreach templates across 3 channels.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a user research strategist working with pre-seed founders. Your job is to design a lean, practical research plan that gets the founder into real conversations with real customers as fast as possible — and extracts the insights that change how they build and market.

REQUIRED INPUTS

23. Your business and market description from Prompt 1
24. Your product mechanics from Prompt 2
25. Your 3 biggest assumptions from Prompt 1 — Step 3
26. Your JTBD map from Prompt 4 — Step 1
27. Your competitive intelligence from Prompt 5
28. Answer: Have you spoken to any potential customers yet? If yes, what did you learn? If no, what has stopped you?
29. Answer: Who do you currently have access to — through your network, LinkedIn, communities, or existing users — who fits your target customer profile?

[Paste all inputs before continuing.]

Once I share my inputs, do the following in order:

Step 1 — Audit my assumptions

Based on everything I have shared, identify the 5 most dangerous unvalidated assumptions in my business right now. Rank them by urgency — which ones, if wrong, would cause the biggest

change in direction? These become the research questions I am trying to answer.

Step 2 — Define my research goal

Write a single clear research goal statement: what specific question am I trying to answer, what type of research fits this goal — generative to discover new insights or evaluative to test existing assumptions — and what I will do differently after the research is complete.

Step 3 — Segment my user groups

Based on my target customers, define 3 distinct groups to speak with. For each group describe: who they are, why they are valuable to research, what specific insight they can give me that the other groups cannot, and how many conversations I need with this group to reach saturation.

Step 4 — Build the interview guide

Write a complete interview guide with 4 sections:

- Opening — how to start the conversation and set the right tone
- Warm-up — questions that build rapport and establish context
- Core — the 5-7 questions that directly address my research goal
- Closing — how to end the conversation and what to ask for next

Step 5 — Build a documentation system

Design a simple system for capturing and synthesizing research findings. Include: what to record during interviews, how to organize notes immediately after, and how to identify patterns across multiple conversations.

Step 6 — Research action plan

Write a specific action plan: how many interviews to complete in the next 2 weeks, which user group to start with and why, who specifically to reach out to first, and what success looks like at the end of this research sprint.

Step 7 — Question sets for all 3 groups

Write a tailored set of 5 core questions for each of the 3 user groups identified in Step 3. Each question set should be adapted to what that specific group knows and can tell me — not a generic list of questions.

Step 8 — Outreach templates

Write 9 outreach templates — 3 for each user group — across 3 channels: LinkedIn message, email, and community or forum message. Each template should: be under 100 words, lead with value not a request, and include a clear and specific ask.

PART 1

PROMPT 8 Define the Real Problems You Are Solving

THE GOAL OF THIS PROMPT

By the end of this prompt you will have separated your real problems from your assumed ones, a complete problem stack showing surface, underlying, hidden, and future problems for each customer type, the single problem worth leading with in your marketing, 3 versions of your problem statement written for different messaging contexts, the problems you should never claim to solve, and a problem-solution fit statement for each customer type.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a problem clarity strategist working with pre-seed founders. Your job is to dig beneath the surface of the problem I think I am solving and find the real, defensible problem worth building a business around.

REQUIRED INPUTS

30. Your business and market description from Prompt 1
31. Your JTBD map from Prompt 4 — Step 1
32. Your competitive intelligence from Prompt 5
33. Your review intelligence from Prompt 6 — Steps 2 and 3
34. Your user research findings from Prompt 7 — paste any interview themes or direct quotes
35. Answer: What problem do you currently tell people you solve? Write the exact sentence you use.
36. Answer: What do customers say the problem is when you ask them? Write their exact words if you have them.

[Paste all inputs before continuing.]

Once I share my inputs, do the following in order:

Step 1 — Separate real from assumed problems
Based on everything I have shared, identify which problems I am currently claiming to solve are

validated by real customer evidence — research, reviews, direct quotes — and which are assumptions. Be direct. Mark each problem as validated, partially validated, or assumed.

Step 2 — Build the full problem stack

For each primary customer type, build a complete problem stack across 4 layers:

- Surface problem — the problem they can articulate and would write in a Google search
- Underlying problem — the real cause of the surface problem that they may not have named
- Hidden problem — the deeper fear, belief, or situation driving the underlying problem
- Future problem — what happens if this problem is not solved in the next 6-12 months

Step 3 — Identify the problem worth leading with

Based on the full problem stack, identify which single problem I should lead with in my marketing. The ideal problem to lead with is: validated by real evidence, urgent enough that customers actively seek a solution, specific enough that my target customer immediately recognizes it as their own, and differentiated enough that it positions me against competitors.

Step 4 — Write 3 problem statement versions

Using the problem identified in Step 3, write 3 versions of the problem statement calibrated to different contexts:

- Rational version — clear, specific, data-grounded — for decision-makers and analytical buyers
- Emotional version — captures the feeling of living with this problem — for emotionally-driven buying contexts
- Identity version — frames the problem as a threat to who they are or want to be — for high-sophistication marketing

Step 5 — Problems I should not claim to solve

Based on my product's actual capabilities and the competitive landscape, identify 3 problems I am tempted to claim but should not — because I cannot fully solve them, because a competitor does it better, or because claiming them would attract the wrong customer. Being clear about what I do not solve is as important as knowing what I do.

Step 6 — Problem-solution fit statement

For each primary customer type write a problem-solution fit statement in this format: [Customer type] struggles with [specific problem from the stack] because [root cause]. Unlike [current alternative], [my product] solves this by [specific mechanism] which means [specific outcome for this customer type].

PART 1

PROMPT 9 Build Your Customer Avatar

THE GOAL OF THIS PROMPT

By the end of this prompt you will have a complete 8-dimension customer avatar built from all previous research, a deep emotional landscape showing your customer's dominant and aspiration emotions, a 150-200 word first-person monologue written in the avatar's exact voice, a complete map of where this customer spends time online and offline, the specific trigger moment that makes them actively seek a solution, and a one-page avatar reference card your entire team can use.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a customer psychology strategist and brand positioning expert working with pre-seed founders. Your job is to synthesize everything we know about my customers into a complete, vivid, and actionable customer avatar — not a demographic profile, not a made-up persona with a stock photo name, but a deeply human portrait of the real person I am building for.

REQUIRED INPUTS

37. Your business and market description from Prompt 1
38. Your JTBD profiles from Prompt 4 — Step 1
39. Your UVPs from Prompt 4 — Step 3
40. Your competitive intelligence from Prompt 5
41. Your customer language from Prompt 6 — Step 3
42. Your user research findings from Prompt 7 — paste any interview themes, direct quotes, or observations. If you have not yet done research, state that clearly.
43. Your problem stack from Prompt 8 — Step 2
44. Answer: Of all the customer types you have described across these prompts, which one — if you could only serve one — would you build your entire business around? Name them, describe them in one sentence, and tell me why you chose them.
45. Answer: What is the one thing this person says out loud to colleagues, friends, or their team that signals they have the problem you solve? Give me the actual sentence if you can.

46. Answer: What does this person read, watch, and listen to? Where do they spend their time online and offline?

[Paste all inputs before continuing.]

Once I share my inputs, do the following in order:

Step 1 — Build the full avatar profile

Using everything I have shared, build a complete customer avatar for my primary customer type across all 8 dimensions:

- Who they are — job title or role, life stage, where they live and work, who they are responsible for, what their day actually looks like from morning to night
- What they are trying to achieve — their primary professional or personal goal right now, specific and time-bound, what success looks like in 6-12 months
- What is standing in their way — external obstacles and internal obstacles including beliefs, fears, habits, and self-doubt
- What they have already tried — every solution, workaround, tool, or approach they have used before, and why each fell short
- How they make decisions — analytical or intuitive, who influences them, what their buying process looks like
- What they are afraid of — rational fears and emotional fears that cause hesitation or delay
- What they secretly want — beyond the functional job, what would make them feel proud, respected, free, or in control
- What a day in their life looks like — walk through a specific day including the exact moment the problem shows up

Step 2 — Build the avatar's emotional landscape

Go deeper into the emotional and psychological dimensions:

- Their dominant emotion around this problem — frustration, anxiety, embarrassment, overwhelm, or resignation
- Their aspiration emotion — how they want to feel after solving this problem
- Their social context — who is watching them struggle and how being seen to solve it would change their standing
- Their relationship with solutions like mine — skeptical, hopeful, burned before, or completely new to this category

Step 3 — Write the avatar in their own voice

Write a first-person monologue of 150 to 200 words from the avatar's perspective — as if they are sitting across from a trusted friend and describing their situation honestly. Use the exact language and phrases from real customer research in Prompt 6. This monologue becomes the single most useful piece of creative direction I have — every ad, every email, every landing page headline should sound like a response to this monologue.

Step 4 — Identify the avatar's watering holes

Map where this person spends their time — online and offline — across:

- Online communities — specific subreddits, Facebook groups, Slack communities, Discord servers, LinkedIn groups, forums
- Content they consume — specific podcasts, newsletters, YouTube channels, blogs, books, industry publications
- Events they attend — conferences, meetups, webinars, trade shows, workshops
- Social platforms — which platforms they use, how they use them, what content stops their scroll

Step 5 — Identify the avatar's buying trigger

What is the single most common event or moment that pushes this person from passive awareness of the problem to actively seeking a solution? Describe the trigger moment in detail, explain what emotional state it puts them in, and tell me what the first thing they do is after it happens. That first action is where I need to be visible.

Step 6 — Build the avatar reference card

Summarize everything into a one-page avatar reference card including: avatar name and one-sentence description, primary goal in the next 6-12 months, biggest obstacle, dominant emotion, aspiration emotion, the sentence they say out loud that signals they have the problem, top 3 watering holes, buying trigger, the one thing I must say to earn their trust, and the one thing I must never say.

This reference card gets pinned to the wall of every team working on this business. Every decision — product, copy, design, pricing, channel — gets filtered through it.

PART 2

Marketing Funnel & Campaign

7 Prompts for Pre-Seed Founders

PART 2

PROMPT 1

Map Your Marketing Funnel Architecture

THE GOAL OF THIS PROMPT

By the end of this prompt you will have a diagnosed funnel model with a real-world benchmark to steal from, a complete 5-stage funnel map built around your specific customer and product, a prioritized asset and content map for every funnel stage, a plain-language wiring diagram showing how every asset connects, a phased build order across 3 phases, and a weekly funnel scorecard with green, amber, and red thresholds for every metric.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a B2B growth strategist working with pre-seed founders. Your job is to design the complete marketing funnel architecture for my business — a clear, stage-by-stage map that tells me exactly what type of funnel I need, what happens at each stage, what assets I need to build, and in what order to build them.

The output of this prompt is a single strategic document I can use as the master blueprint for everything I build in the prompts that follow. It is not a generic funnel template — it is built entirely around my specific customer, product, market, and stage of growth.

Required inputs — Part 1 outputs:

REQUIRED INPUTS

47. Your business and market description from Prompt 1 of Part 1
48. Your product mechanics from Prompt 2 of Part 1
49. Your BFG master table from Prompt 3 of Part 1
50. Your top 3 UVPs and market sophistication level from Prompt 4 of Part 1
51. Your customer avatar from Prompt 9 of Part 1
52. Your problem-solution fit statements from Prompt 8 of Part 1

[Paste all inputs before continuing.]

Required inputs — current situation:

Answer every question below honestly and specifically. This is what makes the funnel diagnosis

accurate and specific to your business rather than generic. If you do not have a number give your best estimate and mark it as an estimate.

- What is your primary conversion goal right now — free trial signups, demo requests, webinar registrations, or direct sales conversations?
- Do customers buy without talking to anyone or does conversion require a human conversation?
- What is your price point — monthly or annual?
- What tools do you currently have — website builder, email platform, CRM, analytics?
- Do you currently have a website? If yes: how many visitors per month and where does that traffic come from?
- Are you currently capturing any leads? If yes: roughly how many per month and what is driving them?
- Do you have any existing content or marketing activity? If yes: what has worked and what has gotten zero traction?
- What marketing channels have you already tried even informally? What happened?
- Where do you think you are losing people right now?
- What is your single biggest growth constraint right now in one sentence?

[Paste all inputs before continuing.]

Once I have your inputs produce the following funnel architecture document in order. Be specific to my business at every step — do not give me generic funnel theory.

PART 1 — Funnel diagnosis

1. What funnel model do I need?

Identify which model fits my business right now and why — in no more than 5 sentences. Choose from: Product-led growth, Sales-led growth, Content-led growth, Community-led growth, or Hybrid.

Then tell me which of these 5 real-world funnels is closest to what I should build and what specifically I should steal from them:

- Monday.com — PLG, free trial, template-led acquisition, viral team adoption
- HubSpot — content-led, education at scale, free tools as lead magnets, inbound motion
- Slack — bottom-up viral adoption, freemium, individual user converts then brings the team
- Notion — community and creator-led, templates as distribution, identity-based adoption
- Salesforce — high-touch enterprise, thought leadership, events, ecosystem lock-in

2. Where is my biggest funnel leak right now?

Using specifically the current situation data I shared — traffic, leads, conversions, channels tried, and my own assessment — identify the single stage where I am losing the most potential customers right now. Use the actual numbers to tell me: exactly where the leak is happening, what is causing it, whether my own assessment matches the data, and the single most important thing to fix first.

If I have no traffic, no leads, and no conversions yet — say that clearly and tell me what the real first priority is for a business at my exact stage.

3. What does my customer's buying journey actually look like?
Map the real journey my specific avatar takes — starting from before they are even aware they have a solvable problem — all the way to becoming a paying customer. Include: the specific moment the problem first becomes real, whether they are aware the problem is solvable, the sequence of searches and content they consume, the emotional state at each step, the objection at each step, and the single moment where I am most likely to lose them permanently.

PART 2 — Full funnel stage map

Map my complete funnel across 5 stages. For each stage produce: what is happening in the customer's world, the awareness level, my goal, what I must give them, the objection I must collapse, where I will lose them, and the conversion benchmark.

The 5 stages — use these exact definitions:

- TOFU — Awareness of the problem: the customer is living with a pain they may not have fully named yet. They have normalized it. TOFU content creates awareness of the problem first, then awareness that it is solvable.
- MOFU — Consideration of the solution: the customer knows the problem is real and urgent. They are evaluating solution categories and building trust. MOFU content names their objection before they voice it and collapses it with proof.
- BOFU — Conversion decision: the customer is ready to choose. The job now is to give them a result in advance so they experience the outcome before they pay.
- AFU — Retention and expansion: the customer has converted. Get them to their aha moment as fast as possible then retain and expand.
- LOFU — Advocacy and loop: the satisfied customer becomes the most powerful TOFU asset in the funnel. Their story creates awareness in the next customer.

Conversion benchmarks to include for each stage:

- Homepage to trial or lead capture: 2-5% average, top performers above 10%
- Demo request page: 3.8% median, top performers above 8%
- Webinar registration page: 25-45% conversion
- Free trial to paid B2B: 15-30%, top quartile 35-45%
- Trial activation to aha moment: top performers 60%+
- Email open rates B2B: 20-30% average, 40%+ is excellent
- Post-purchase retention month 1: 46.9% average B2B SaaS

PART 3 — Content and asset brainstorm

For each funnel stage act as my creative brainstorming partner. Generate specific, executable content ideas and assets built entirely around my customer's language, pain, desires, and objections from Part 1.

For each funnel stage generate content ideas across these formats — each idea must be specific to my business and customer:

- LinkedIn posts — 3 specific post ideas that would stop my avatar mid-scroll at this stage

- Short video — 3 specific 60-90 second video ideas filmable on a phone
- Blog or SEO content — 3 specific article ideas targeting the exact questions my customer is searching
- Lead magnets — 2-3 specific quizzes, assessments, templates, or tools that deliver a result in advance
- Case studies and testimonials — 2 specific social proof ideas for this stage
- Email — 2-3 specific email ideas for leads at this stage
- Community and distribution — 2-3 specific communities or partner channels where my customer is at this stage

After all content ideas across all 5 stages: identify the 3 highest-leverage content pieces for a pre-seed founder, name the single content piece to create this week, and identify the single most powerful result-in-advance asset — the equivalent of a 15-minute meditation audio that lets my customer experience the outcome before they pay.

PART 4 — Funnel wiring diagram

Show how all the assets connect into one flowing system. Write as a sentence-by-sentence plain language description starting with: 'A [avatar name] first encounters us when...' and ending with '...and they become an advocate who brings in [next customer type].'

PART 5 — Build order

Phase 1 — Week 1 to 2: Minimum viable funnel

The 3 assets I must build before anything else. For each tell me: what it is, why it comes first, what it must achieve. Tell me the single metric that tells me the MVF is working.

Phase 2 — Week 3 to 6: Conversion layer

The next 7 assets that improve conversion at every stage. For each: what it is, which stage it fixes, what conversion improvement to expect. Tell me the benchmark that signals readiness for Phase 3.

Phase 3 — Month 2 to 3: Growth layer

The assets and systems that turn the funnel into a compounding machine. For each: what it is, how it compounds, what manual effort it replaces. Tell me the metric that signals the funnel is growing on its own.

PART 6 — Weekly funnel scorecard

Build a simple one-page scorecard I review every Monday morning.

| Stage | Metric to track | How to measure it | Green — on track | Amber — needs attention | Red — broken, fix immediately | Most likely cause of red | First thing to fix |

Use the benchmark data from Part 2 to set every threshold. Add one final row for overall funnel health — the single number that tells me the whole system is working or not.

PART 2

PROMPT 2

Define Your Marketing Angles

THE GOAL OF THIS PROMPT

By the end of this prompt you will have one master marketing angle — your hook, differentiator, and primary psychological trigger — a prioritized table of which of 16 specific angles to deploy at which funnel stage, an angle-to-channel mapping, a 10-hook library written in your customer's exact language, your specific message at each funnel stage, and a 30-day execution plan with a 5-hours-per-week constraint.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a B2B marketing strategist and creative director working with pre-seed founders. Your job is to define the complete marketing angle strategy for my business — the specific hooks, messages, psychological triggers, differentiators, and approaches I will use to reach my customer at every stage of the funnel and turn their attention into action.

A marketing angle is not a tagline and it is not a channel strategy. It is the specific lens through which I present my product to my customer — the emotional and rational hook that makes them stop, pay attention, and take the next step.

REQUIRED INPUTS

53. Your funnel architecture document from Prompt 1 of Part 2 — specifically the funnel stage map and the wiring diagram
54. Your customer avatar and monologue from Prompt 9 of Part 1
55. Your top 3 UVPs and market sophistication level from Prompt 4 of Part 1
56. Your BFG master table from Prompt 3 of Part 1
57. Your problem-solution fit statements from Prompt 8 of Part 1
58. Your competitive intelligence and customer language from Prompts 5 and 6 of Part 1
59. Answer: What is the one thing your best potential customer is most afraid of right now — the fear that keeps them from taking action?
60. Answer: What is the one thing your best potential customer most wants to be able to say to their boss, team, or peers after using your product?

61. Answer: Have you run any marketing yet? If yes, what worked and what completely failed?
62. Answer: Who are 2-3 brands whose marketing style you admire and why?

[Paste all inputs before continuing.]

Once I have your inputs produce the following marketing angle strategy document in full:

PART 1 — Master marketing angle

1. What is my hook?
The single most attention-grabbing truth about my product or the problem it solves. Write my master hook in one sentence. Specific, provocative, and grounded in the real customer language from my avatar. Must not sound like a feature description or a mission statement.

2. What is my differentiator?
The single most meaningful way my product changes the customer's situation that no competitor can honestly claim. Write it in one sentence using the format: 'Unlike [what they currently do or use], [my product] is the only [category] that [specific mechanism] so that [specific customer outcome].'

3. What is my primary psychological trigger?
Based on my customer avatar — their fears, desires, dominant emotion, and aspiration emotion — identify the single most powerful psychological trigger my marketing should lead with. Choose from: loss aversion, social proof, authority, urgency and scarcity, reciprocity, commitment and consistency, social identity, fear of missing out, or aspiration. Explain in 3 sentences why this trigger fits this specific customer.

PART 2 — Angle selection and prioritization
From the list below, identify which marketing angles are right for my business. For each angle I should use, explain: why it fits, what specific message to lead with, and what funnel stage it belongs to. Then rank all selected angles in priority order.

The 16 angles to evaluate:

- Encourage your audience to sign up for a demo
- Target your audience based on language and region
- Book sales calls
- Offer rewards to increase product usage
- Try a new low-risk CTA offer
- A sneak peek at pricing options
- Promote product launches and new features
- Highlight a use case offer
- An inside look at your product dashboard
- Display testimonials
- Use video ads to demonstrate the benefits of your product

- Share case studies
- Promote your blog posts
- Promote exclusive research
- Promote webinars
- Promote online and offline events

Present as a table: Angle | Use it — yes or no | Why it fits or does not fit | Funnel stage | Priority rank | Specific message to lead with

PART 3 — Angle to channel mapping

For each angle I should use, map it to the specific channels where I will deploy it. For each channel: which angle it carries, what format it takes, what the first piece of content looks like, what the CTA is, and what success looks like.

PART 4 — Hook library

Write 10 hooks I can use across my marketing — ads, email subject lines, LinkedIn posts, landing page headlines, video thumbnails. Each hook must be written in my customer's language from the avatar monologue and calibrated to my market sophistication level.

Vary the 10 hooks across these approaches:

63. Lead with the pain
64. Lead with the future
65. Lead with the mechanism
66. Lead with a number
67. Lead with a question
68. Lead with a story
69. Lead with a provocation
70. Lead with social proof
71. Lead with loss
72. Lead with identity

PART 5 — Message by funnel stage

For each funnel stage produce: the core message in one sentence, the tone, what I never say at this stage, and 3 example messages — LinkedIn post opener, email subject line, ad headline, or landing page headline.

PART 6 — 30-day angle execution plan

A specific 30-day plan for deploying my top 3 angles. For each week: which angle I am focusing on, what I create, what I publish, what I measure, and what I do differently in week 2 if week 1 does not hit benchmark. Maximum 5 hours per week. Every task executable without a designer, developer, or copywriter.

PART 2

PROMPT 3A

Build Your Campaign Concept & Video Script

THE GOAL OF THIS PROMPT

By the end of this prompt you will have one complete campaign concept — a premise, a pattern interrupt, and a CTA — and one ready-to-film video script calibrated to your actual production reality, plus two shorter A/B test variations. Every asset follows the same three-act arc: problem, mechanism, relief.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a B2B creative director working with pre-seed founders. Your job is to design one complete campaign concept and produce one ready-to-film video script that the founder can take into production this week.

The campaign follows the structure used by Monday.com, Slack, HubSpot, and Dollar Shave Club: make the customer feel seen before mentioning the product, dramatize the problem in a way that feels personal not corporate, and introduce the solution as relief not as a sales pitch.

REQUIRED INPUTS

73. Your customer avatar and monologue from Prompt 9 of Part 1
74. Your problem-solution fit statements from Prompt 8 of Part 1
75. Your top UVP and market sophistication level from Prompt 4 of Part 1
76. Your top marketing angle and hook library from Prompt 2 of Part 2
77. Answer: What is the single action this campaign must drive — free trial, demo request, or webinar registration?
78. Answer: What is your production reality — phone camera, screen recording like Loom, or professional production budget?
79. Answer: In one sentence — what is the most painful moment in your customer's day that your product eliminates?
80. Answer: What is the one thing your best customers say when they describe why they chose you?

[Paste all inputs before continuing.]

Once I have your inputs produce the following:

PART 1 — Campaign concept

The campaign premise — one sentence.
The one truth about the customer's situation that this entire campaign dramatizes. Format: 'This campaign is for [specific customer] who [specific painful situation] and it shows them [the specific shift in thinking or feeling we want to create].'

The pattern interrupt — one technique, executed specifically.
Make a decision — do not list options. Choose the single most powerful pattern interrupt for this customer and execute it. Write the exact opening — the first 5 seconds of every asset in this campaign.

Pattern interrupt options to choose from:

- The internal monologue — open with the exact words the customer says to themselves that they have never seen in an ad. Best for sophistication levels 3 and 4.
- The provocation — a statement that challenges a belief they currently hold. Best when the market has a wrong assumption you can disprove.
- The unexpected statistic — a number so specific and surprising it creates cognitive dissonance. Best when you have counterintuitive data.
- The character in pain — show someone living the exact problem before a single word appears. Best for video.
- The question they are already asking — the exact question they typed into Google at 11pm. Best for content-led funnels.
- The humor interrupt — make them laugh at the absurdity of their current situation. Best for sophistication level 5.
- The contrast — show two worlds side by side with no explanation. Best when before and after are visually stark.

The campaign CTA — one action, one sentence.
The specific outcome-led phrase that connects to their emotional job from the JTBD profile. Maximum 6 words.

PART 2 — Video script

Write one complete video script calibrated to the founder's actual production reality. The script follows the three-act arc:

- Act 1 — The problem: show the customer's painful reality so accurately they feel seen. Use the surface and hidden problem layers from Prompt 8 of Part 1. The pattern interrupt opens this act. Do not mention the product.
- Act 2 — The mechanism: show what makes the solution different without selling. Do not list features. Show a completely different approach to the problem.
- Act 3 — The relief: show the customer's life after the problem is solved. The emotional destination — how they feel, what they can now do. End with the campaign CTA.

Production Format A — Founder on camera (phone or basic setup)

Write as a direct-to-camera monologue in the founder's voice — conversational, specific, no corporate language. Every sentence must earn the next one.

Format: Opening 5 seconds [exact words + what founder shows on camera] / Scene by scene [what founder says + what they show + text on screen] / Closing 10 seconds [exact words + CTA visual] / Total length: 60-90 seconds for LinkedIn and YouTube, 15-second cut-down for Instagram and TikTok / Director's note: 3 specific instructions for pacing, energy, setting

Production Format B — Screen recording with voiceover (Loom or similar)

Write as a voiceover paired with specific screen actions. Every line of voiceover paired with an exact instruction for what is on screen.

Format: Opening 5 seconds [exact voiceover + what is on screen] / Screen by screen [voiceover line + exact screen action] / Closing 10 seconds [exact words + what appears on screen] / Total length: 90 seconds to 2 minutes / Production note: 3 specific instructions for screen layout and cursor behavior

Write the version that matches the production reality stated in the inputs. If not specified, write Format A.

PART 3 — Script variations for A/B testing

Write two shorter script variations — 15-30 seconds each — that test different pattern interrupts against the main script. These run as paid social ads on LinkedIn and YouTube.

Variation A:

Same campaign premise, different opening. Use a different pattern interrupt technique and write the full 15-30 second script.

Variation B:

Same campaign premise, lead with social proof instead of the problem. Open with a specific customer outcome and work backward to the problem and the CTA.

For both variations produce: full script with timing notes, the psychological trigger in use and where it appears, which audience this variation resonates with most — cold, warm retargeting, or existing leads.

PART 2

PROMPT 3B — Build Your Campaign Distribution

THE GOAL OF THIS PROMPT

By the end of this prompt you will have complete ad copy for 3 LinkedIn versions, 3 YouTube bumper ads, and 1 retargeting ad, a 4-email campaign sequence with full copy, a 2-week social content calendar with 10 complete posts, and either full copy for a new campaign landing page or specific edit instructions for an existing page — all connected to the same campaign concept from Prompt 3A.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a B2B campaign manager and conversion copywriter working with pre-seed founders. Your job is to take the campaign concept and video script from Prompt 3A and produce every distribution asset needed to get it in front of the right people and convert them into leads.

Everything in this prompt is downstream of the campaign concept. Do not deviate from the premise, pattern interrupt, or CTA established in Prompt 3A.

REQUIRED INPUTS

81. Your complete campaign concept from Prompt 3A — premise, pattern interrupt, and CTA
82. Your video script from Prompt 3A
83. Your complete funnel architecture from Prompt 1 of Part 2 — specifically the funnel stage this campaign targets
84. Your customer avatar from Prompt 9 of Part 1
85. Your BFG master table from Prompt 3 of Part 1
86. Your competitive customer language from Prompt 6 of Part 1 — Step 3
87. Answer: What channels do you currently have an audience on?
88. Answer: Do you have any real customer quotes or direct feedback? Paste them here.
89. Answer: What is your campaign budget — zero, under $500, under $2000, or more?

90. Answer: Which pages do you already have live? List every page with its URL and a one-line description.

[Paste all inputs before continuing.]

Before producing any assets, identify the campaign destination page. Look at the CTA from Prompt 3A and the funnel stage this campaign targets. Check the list of existing pages. State at the top: 'This campaign drives traffic to: [page name]. This page [already exists / does not exist yet]. Here is what to do.'

Once I have your inputs produce the following complete distribution package:

PART 1 — Ad copy

LinkedIn sponsored content — 3 versions

One for cold audience TOFU, one for warm retargeting MOFU, one for BOFU visitors who did not convert. For each version: hook line (first 2 lines before 'see more'), body copy (maximum 150 words, one idea, one emotion), CTA button text, visual description, psychological trigger in use.

YouTube 6-second bumper ads — 3 versions for A/B testing

Each delivers the campaign message in 6 seconds. One idea. One emotion. One CTA. Write the exact words spoken, what appears on screen, and what text overlay shows.

Retargeting ad — 1 version

Targeting visitors who landed but did not convert. Acknowledges the visit without being creepy. Addresses the most likely objection from competitor review intelligence. Offers a lower-friction entry point. Write full copy for LinkedIn or the most relevant channel.

PART 2 — Campaign email sequence

Write 4 complete emails. Every email in the customer's voice. Every email does one thing only. No email longer than 150 words.

Email 1 — Campaign launch

Sent on launch day. Subject line uses the campaign pattern interrupt. Body introduces the premise without selling. CTA drives to the campaign destination page.

Email 2 — Value delivery

Sent day 3 to everyone who opened but did not click. Delivers one specific piece of value related to the campaign theme. Does not mention the product. Ends with a soft CTA.

Email 3 — Social proof

Sent day 6 to everyone who opened but has not converted. One specific customer story using real language from competitor review intelligence. Structure: the problem in their words, the moment they found a different way, the specific outcome.

Email 4 — Last chance
Sent day 10. Maximum 100 words. Uses the future problem layer from Prompt 8 of Part 1. No fluff. One CTA. Subject line creates urgency through truth not fake scarcity.

For every email: subject line (maximum 8 words in the customer's voice), preview text (40 characters), opening line, full body copy, CTA with exact link text.

PART 3 — Social content calendar

Write 10 social posts over 2 weeks. Not promotional — they educate, provoke, and build authority. All posts for LinkedIn unless the avatar has a stronger presence elsewhere.

The 10 posts follow this psychological sequence — do not deviate:

91. Day 1: The provocation. Bold counterintuitive truth. No CTA.
92. Day 2: The problem in detail. Avatar's exact language. No CTA.
93. Day 4: The data. One specific statistic. Source it. End with a question.
94. Day 5: The campaign video. 3-line caption using pattern interrupt opening.
95. Day 7: The mechanism. New approach in plain language. No product until last line.
96. Day 8: The customer story. Real quote or competitive review language. Before and after.
97. Day 9: The myth-bust. Most common wrong belief demolished with evidence.
98. Day 11: The use case. One specific scenario written as micro-story.
99. Day 12: The objection handler. Name biggest reason for inaction and address it directly.
100. Day 14: The social proof close. Most powerful outcome. Campaign CTA with deadline if any.

For each post: full post copy, format, opening hook line, CTA, best time to post. Benchmark: Tuesdays and Thursdays 8-10am for B2B.

PART 4 — Campaign destination page

If the destination page already exists:
Produce a precise copy edit document. For each change: location on the page, current copy, new copy, reason for the change. Apply these benchmarks: above-the-fold clarity converts 35-40% better, 5 or fewer fields converts 120% better, social proof near CTA increases conversion 84-270%, specific next-step CTAs outperform vague ones, grade 5-7 reading level converts 11% better.

If the destination page does not exist yet:
Write the complete page copy determined by the campaign CTA: lead magnet page, webinar registration page (benchmark: 25-45% conversion), demo booking page (benchmark: 3.8% median, embed calendar directly — only 8% of SaaS companies do this), or free trial page (benchmark: 2-5% average, top performers above 10%).

Complete page copy includes: headline (8 words max), subheadline (25 words max), hero body copy, video placement note, form headline, form fields (max 5, ideally 3), CTA button text, what happens next, social proof block (real quotes or bracketed frameworks — no invented

testimonials), trust signals, outcomes section ('You will finally...'), risk reversal, closing CTA with authentic urgency.

PART 5 — 14-day launch plan

Day-by-day execution plan for the first 14 days. Maximum 5 hours per week. Every task completable without a designer, developer, or copywriter.

For each day: what to do, which asset it uses and how long it takes, what to publish or send, what to measure at the end of that day, what to do differently if below benchmark.

Also produce: the 3 metrics that tell me this campaign is working by day 7, the single metric that tells me to increase budget by day 14, and the one signal that tells me the campaign concept is wrong and I need to go back to Prompt 3A.

PART 2

PROMPT 4

Build Your Funnel Pages

THE GOAL OF THIS PROMPT

By the end of this prompt you will have identified every page needed in your funnel, a complete mapping of which content piece drives to which page, full ready-to-publish copy for every page that does not yet exist, precise edit instructions for every page that already exists, and a plain-language connection map showing how every page connects to the next step in the funnel.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a B2B conversion copywriter working with pre-seed founders. Your job is to look at every content piece in the funnel built in Prompt 1 of Part 2 and produce the complete, ready-to-publish copy for every page those content pieces drive traffic to.

The page type is determined entirely by what the content piece is trying to do and where the customer is in their funnel journey. The content piece and the page it lands on must match perfectly.

REQUIRED INPUTS

101. Your complete funnel architecture from Prompt 1 of Part 2 — every content piece, format, channel, and CTA across all 5 funnel stages
102. Your customer avatar and monologue from Prompt 9 of Part 1
103. Your BFG master table from Prompt 3 of Part 1
104. Your top 3 UVPs and market sophistication level from Prompt 4 of Part 1
105. Your problem-solution fit statements from Prompt 8 of Part 1
106. Your competitive customer language from Prompt 6 of Part 1
107. Your master marketing angle, hook, and differentiator from Prompt 2 of Part 2
108. Answer: Which pages do you already have live? List every page with its URL and a one-line description.
109. Answer: Do you have any real customer testimonials or quotes? Paste them here.

110. Answer: What is the single most common reason a visitor does not take action after consuming one of your content pieces?

111. Answer: What social proof assets do you have — logos, review scores, specific results, press mentions?

[Paste all inputs before continuing.]

Before writing any page do this first:

Step 1 — Map every content piece to its destination page.
Go through every content piece in the funnel from Prompt 1 of Part 2. For each content piece identify: the content piece name and funnel stage, what the CTA is driving the visitor to do next, what type of page that CTA needs to land on, and whether that page already exists.

Page types by funnel stage:

- TOFU content drives to lead capture pages: quiz or assessment results page, lead magnet download page, problem awareness landing page
- MOFU content drives to trust and consideration pages: case study page, social proof page, webinar registration page, product overview page
- BOFU content drives to conversion pages: demo booking page, free trial signup page, offer presentation page, competitor comparison page
- AFU content drives to activation and expansion pages: activation page, upgrade page
- LOFU content drives to advocacy pages: referral page, case study submission page, review page

Step 2 — Identify which pages need to be built.
Produce a table: Content piece | Funnel stage | CTA destination | Page type needed | Already exists? Then state clearly: 'I will now build the following pages:' For pages that already exist produce specific copy edit instructions: location on page, current copy, new copy, reason for the change.

Apply these conversion benchmarks to every page: above-the-fold clarity converts 35-40% better, 5 or fewer form fields converts 120% better, social proof near CTA increases conversion 84-270%, specific CTAs beat vague ones, grade 5-7 reading level converts 11% better, video on pages increases conversion up to 80%.

For every page produce the complete copy in this structure:
PAGE NAME — [page type] for [content piece name] / Funnel stage: [TOFU/MOFU/BOFU/AFU/LOFU] / Visitor arrives from: [content piece and channel] / Job of this page: [one sentence] / Conversion benchmark: [specific rate to hit]

Then write the complete page copy using the relevant structure:

TOFU — Lead capture page:
Headline (8 words, problem language not product language), subheadline (25 words), what you get (3 specific bullet points), form (maximum 2 fields: first name and email only), CTA button (specific asset name), what happens next (one sentence), privacy statement.

MOFU — Case study page:
Headline (10 words, specific result), subheadline (25 words), the story in 4 parts (the before, the problem that made them act, the solution, the after), pull quote (single most powerful sentence), results summary (3 specific bold numbers), social proof bar, CTA placed immediately after results.

MOFU — Social proof page:
Headline (8 words), subheadline (25 words), results bar (3 bold numbers above the fold), customer stories (minimum 3 with before/after and direct quote), video testimonial placement note, CTA after every 2-3 stories.

MOFU — Webinar registration page:
Eyebrow (6 words, customer type), headline (10 words, specific outcome format: 'how to [outcome] without [obstacle]'), subheadline, date/time/duration above fold, what you will learn (3-5 capabilities not topics), form (max 3 fields), CTA button, recording availability statement, speaker section, 2 testimonials, closing CTA.

BOFU — Demo booking page:
Headline (8 words, specific outcome of the demo), subheadline (25 words, who it is for and how long), what you will see (3 outcome bullet points), calendar embed directly on page (only 8% of SaaS companies do this — exploit this gap), what happens next (one sentence), social proof column alongside the calendar.

BOFU — Offer presentation page:
Headline (8 words, leading with loss aversion trigger), subheadline (25 words), the offer in plain language, what is included (5-7 outcomes), guarantee or risk reversal, 3 testimonials addressing final objections, CTA, FAQ (5 questions written as the customer asks them).

BOFU — Competitor comparison page:
Headline naming the comparison directly, subheadline, comparison table (only the dimensions that drive the decision, using real customer language), honest assessment (2-3 sentences acknowledging what competitor does well), customer migration story (before, the moment, after), CTA.

AFU — Activation page:
Headline (8 words, pointing to first win), first 3 actions (numbered checklist completable in under 10 minutes each — every 10-minute delay in time-to-value costs 8% in trial-to-paid conversion), first result statement, help CTA.

LOFU — Referral page:
Headline (8 words, acknowledges their success), what they get, what their referral gets, how it works (3 simple steps), pre-written referral message in the customer's voice they can copy and paste without editing.

PART 2 — Page connection map
Once all pages are written produce a simple flow for every content piece: 'A visitor reads [content piece name] on [channel] → clicks [CTA] → lands on [page name] → takes [action] →

moves to [next funnel stage].' This is the document the developer uses to wire every page correctly.

PART 3 — Build priority

List pages in priority order with one specific reason for each ranking based on the primary conversion goal. Flag any page where a delay is actively costing leads or conversions right now.

PART 2

PROMPT 5 Content Distribution & Traffic Strategy

THE GOAL OF THIS PROMPT

By the end of this prompt you will have a complete channel assessment table telling you which channels to use, avoid, and prioritize, every content piece from your funnel mapped to every channel it should live on with specific adaptations and hooks, a 3-phase traffic building system from zero, a weekly distribution routine that fits in your stated time budget, and a growth scorecard with targets at week 4, month 3, and month 6.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a B2B growth strategist and distribution expert working with pre-seed founders. Your job is to take every content piece built in the funnel from Prompt 1 of Part 2 and produce a complete distribution plan — where each piece goes, how it gets adapted for each channel, which channels this founder should prioritize, and exactly what to do each week to build traffic systematically from zero.

REQUIRED INPUTS

112. Your complete funnel architecture from Prompt 1 of Part 2 — every content piece, format, channel, and CTA across all 5 funnel stages
113. Your customer avatar from Prompt 9 of Part 1 — specifically the watering holes section
114. Your marketing angles and hook library from Prompt 2 of Part 2
115. Your campaign concept and video script from Prompt 3A
116. Answer: What channels do you currently have any presence on — even small?
117. Answer: What is your current weekly time budget for marketing — be honest?
118. Answer: What is your current monthly budget for paid distribution?
119. Answer: Do you have any existing relationships that could be leveraged for distribution?
120. Answer: What has driven any traffic or leads so far?

[Paste all inputs before continuing.]

Once I have your inputs produce the following complete distribution strategy document:

PART 1 — Channel selection and priority

Based on my customer avatar, funnel stage, time budget, and paid budget — identify which channels to use and in what priority order. Present as a table:

| Channel | Fits my customer — yes or no | Best funnel stage | Time cost | Money cost | Use now or later | Specific reason |

Assess every channel from this complete B2B channel map:

ORGANIC CHANNELS

- SEO and blog content — compounding organic traffic from customers searching for answers. 3-6 months to results but highest long-term ROI. Best for TOFU.
- LinkedIn organic — dominant B2B platform used by 47% of B2B companies. Free but requires consistent daily output. Best for TOFU and MOFU.
- YouTube organic — long-form educational videos ranking on both YouTube and Google. Compounds aggressively once established. Best for mechanism explanation and product demos.
- Short-form video — 44% of B2B brands now use under 60-second video. LinkedIn video, YouTube Shorts, Instagram Reels. Fast to produce, high reach. Best for TOFU pattern interrupt.
- Podcast guest appearances — zero production cost, high trust transfer, pre-qualified audiences. Most underused channel at pre-seed stage. Best for TOFU and MOFU.
- Community participation — Reddit, Slack communities, Discord, Facebook groups, LinkedIn groups, Circle, Mighty Networks, Quora, Indie Hackers, industry forums. Contributing genuine value before mentioning the product.
- AI search and generative engine optimization — buyers using ChatGPT, Perplexity, and Claude to research before Google. Getting your content found in AI-powered search. Act on this now while still early.

PAID CHANNELS

- LinkedIn paid ads — highest B2B targeting precision by job title, company size, industry. Most expensive cost per click but highest quality leads. Best for MOFU and BOFU. Minimum effective budget: $50/day.
- Google Search and PPC — captures high-intent demand. Fast results but competitive. Best for BOFU when market is solution-aware.
- YouTube paid ads — pre-roll before competitor content. Lower cost than LinkedIn. Best for TOFU pattern interrupt campaigns.
- Meta ads — works better for B2B than founders expect. Lower CPM than LinkedIn. Best for retargeting warm audiences.
- Retargeting across all channels — highest conversion rate of any paid channel. First paid channel any pre-seed founder should activate.
- Newsletter sponsorships — featured in newsletters your customer already reads. Cheaper than LinkedIn ads, highly targeted, editorial credibility. Best for TOFU and MOFU.

EMAIL CHANNELS

- Cold email outreach — direct to ICP using Apollo, Hunter, or Instantly. Works best hyper-personalized at low volume to high-fit targets. Most pre-seed founders start here.
- Owned email newsletter — most valuable long-term distribution asset. Once on the list you own that relationship permanently. Start building from day one.
- Lead magnet email sequences — automated sequences triggered by content downloads, quiz completions, or webinar registrations. Nurtures leads at scale without manual effort.

DIRECT SALES AND OUTREACH

- LinkedIn direct outreach — messaging ideal customers leading with value not a sales pitch. Best at pre-seed when volume is manageable manually.
- Sales calls and demos — highest converting channel. Fastest path to first revenue. Every pre-seed founder should be doing this before automating anything.
- CRM pipeline nurturing — systematic follow-up with leads who showed interest. Most founders lose deals not because the lead was uninterested but because nobody followed up consistently.
- Warm introductions — every investor, advisor, former colleague, and happy customer is a potential introduction. A systematic warm introduction strategy often outperforms every other channel in the first 6 months.

PARTNERSHIPS AND OTHER PEOPLE'S AUDIENCES

- Brand collaborations and co-marketing — joint content, joint webinars, joint lead magnets with complementary products serving the same customer.
- Integration partnerships — getting listed in marketplaces of tools your customer already uses.
- Referral programs — turning existing customers into distribution. Works best once you have 10 or more happy customers.
- Guest posting — writing for blogs and publications your customer already reads.
- Influencer and analyst relations — B2B buyers actively seek validation from trusted analysts and influencers.
- Other people's audiences (OPA) — podcast guest appearances, industry report contributions, expert quotes in articles, co-hosted webinars, newsletter features.

DISCOVERY AND REVIEW PLATFORMS

- Product Hunt — significant early traction from people actively looking for new tools. One-time high-impact launch event.
- G2, Capterra, Trustpilot, Clutch — active traffic sources at consideration and decision stage. Strong profile with genuine reviews drives consistent MOFU and BOFU traffic with zero ongoing effort.
- AppSumo — marketplace for software deals generating large volume of early users. Best for self-serve products with broad appeal.

PR AND EARNED MEDIA

- Industry publications and trade newsletters — niche publications hungry for founder stories. A single feature can outperform months of paid ads.

- Press and news features — more accessible at pre-seed than founders think when framed around the problem being solved not the product.

EVENTS

- Industry conferences and trade shows — highest trust transfer of any channel. Speaking slots build authority faster than any content.
- Webinars — 73% higher quality leads than other channels at 90% lower cost than in-person events.
- Virtual events and online communities — hosting your own community builds long-term audience ownership.

WORD OF MOUTH AND DARK SOCIAL

- Dark social — private conversations in Slack DMs, WhatsApp groups, forwarded emails. Cannot be tracked but often the biggest driver of early B2B growth. Activated by making content so specific people feel compelled to share it privately.
- Customer referrals — highest trust, highest converting lead source. A satisfied customer removes every objection a cold lead has.

After the channel assessment table produce:

- Priority stack — top 5 channels in order with one specific reason for each based on my inputs
- Quick wins — 3 distribution actions this week requiring zero budget and under 2 hours each
- Channels to avoid right now — channels to explicitly not spend time on and why each is a distraction

PART 2 — Content piece distribution map

Take every content piece from Prompt 1 of Part 2 and map it to every channel it should be distributed on. For each content piece produce a distribution card:

[Content piece name] — [Funnel stage] / Original format: [what it is] / Original channel: [where it lives natively]

Distribution: Channel | Format adaptation | Hook for this channel | CTA for this channel | Best time to distribute

Format adaptation means how the original piece gets adapted — a blog post becomes a LinkedIn carousel, a video becomes a short clip, a case study becomes a LinkedIn post. Be specific about what changes, what gets cut, what gets added.

Use the hook library from Prompt 2 of Part 2 as the source — pull the hook that fits this content piece and this channel and adapt it specifically. Benchmark: LinkedIn Tuesday and Thursday 8-10am, email Tuesday and Wednesday morning, cold outreach Monday and Tuesday.

PART 3 — Traffic building system

Phase 1 — Week 1 to 2: First traffic

Fastest path to first 100 people into the funnel. What to do, where, and how long each day. Assumes zero existing audience and zero paid budget. Every action executable by a solo founder in the stated time budget.

Phase 2 — Week 3 to 8: Traffic momentum

How to systematize once first traffic is flowing. Which channels get added, scaled, or automated. What the weekly distribution routine looks like. How to compound organic traffic while maintaining direct outreach.

Phase 3 — Month 3 onwards: Traffic compounding

When and how to introduce paid distribution. Which channels to fund first based on what is already working organically. How partnerships and OPA distribution get built. How the content engine compounds over time.

For each phase: specific actions named and sequenced, time investment per week, the metric that signals readiness to move to next phase.

PART 4 — Weekly distribution routine

A specific, repeatable schedule the founder follows every week. Built around the stated time budget from the inputs.

| Day | Task | Channel | Time required | Asset used | Metric to track |

Include: what gets created or adapted each week, what gets published or sent each day, what gets measured at week end, what gets adjusted if below benchmark. Also: the single most important distribution action of the week — the one thing that if nothing else gets done still moves the funnel forward.

PART 5 — Distribution benchmarks and growth scorecard

| Stage | Metric | Week 4 target | Month 3 target | Month 6 target | How to measure |

Include benchmarks for: website visitors per month, LinkedIn post reach and engagement rate, email list size and open rate (B2B average 20-30%, top performers 40%+), cold outreach reply rate (good 10-15%, excellent 20%+), inbound leads per month from organic, leads from paid once activated, referral and word of mouth leads, community contributions leading to conversations.

Flag the single metric that is the leading indicator of whether the entire distribution system is working — the number that if growing everything else tends to follow.

PART 2

PROMPT 6 Tracking, Mapping & Implementation

THE GOAL OF THIS PROMPT

By the end of this prompt you will have a complete tracking system for every funnel stage, a plain-language tool setup guide for every tool you need, a complete UTM tracking system with a ready-to-use link library, a daily, weekly, and monthly tracking routine, a week-by-week implementation plan taking you from zero to a live running funnel, and a 90-day success dashboard with one north star metric and three early warning signals.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a B2B growth operator and systems builder working with pre-seed founders. Your job is to take everything built across all previous prompts and build the complete tracking and implementation plan that turns all of it from a document into a live, running system.

REQUIRED INPUTS

121. Your complete funnel architecture from Prompt 1 of Part 2 — every stage, every content piece, every CTA
122. Your complete distribution plan from Prompt 5 of Part 2 — every channel, every weekly routine
123. Your primary conversion goal from Prompt 1 of Part 2
124. Your tools list — website builder, email platform, CRM, analytics, and any other tools currently in use
125. Answer: What is your current technical comfort level — can you set up Google Analytics and configure a CRM workflow yourself or do you need no-code solutions?
126. Answer: Do you currently track anything at all? If yes what tools are you using?
127. Answer: What does success look like for you in 90 days — be specific with a number.
128. Answer: What is the single biggest implementation risk for you right now?

[Paste all inputs before continuing.]

Once I have your inputs produce the following complete tracking and implementation document:

PART 1 — Funnel tracking system

Design the complete tracking system for every stage of the funnel. For each funnel stage identify every metric to track, what it tells you, how to measure it, and what good looks like.

Use these industry benchmarks to set every threshold:

- Homepage to trial or lead capture: 2-5% average, top performers above 10%
- Demo request page: 3.8% median, top performers above 8%
- Webinar registration page: 25-45% conversion
- Free trial to paid B2B: 15-30%, top quartile 35-45%
- Trial activation to aha moment: top performers 60%+
- Email open rates B2B: 20-30% average, 40%+ excellent
- Email click rates B2B: 2-5% average, above 8% excellent
- Cold outreach reply rate: good 10-15%, excellent 20%+
- LinkedIn organic post engagement rate: 1-3% average, above 5% excellent
- Post-purchase retention month 1: 46.9% average B2B SaaS
- Trial to paid conversion: 15-30% B2B average
- Referral rate: top performers generate 20-30% of new customers from referrals

Track these metrics per stage:

- TOFU — content reach, engagement rate, website traffic from content, new leads generated, cost per lead if paid
- MOFU — lead to demo or trial conversion rate, email sequence open and click rates, webinar registration and attendance, case study page visits and CTA clicks, time from lead capture to demo request
- BOFU — demo to trial conversion, trial signup to activation, time to first value, trial to paid conversion, conversion page conversion rate
- AFU — month 1 retention, activation rate, expansion revenue, NPS or satisfaction score, support ticket volume
- LOFU — referral rate, review submission rate, case study participation rate, word of mouth leads

Present all metrics in a master table: | Stage | Metric | How to measure | Green | Amber | Red | Fix |

PART 2 — Tool setup guide

Based on the tools the founder currently has — and the tools they need but do not have — produce a complete setup guide. For each tool: tool name and category, what it tracks in this funnel, setup instructions in plain language, free alternative, priority (must have now / should have in month 2 / nice to have later).

Cover in order of priority:

- Website analytics (must have now) — Google Analytics 4. Setup: install tracking code, configure conversion events for every CTA, set up funnel visualization report, configure UTM parameters for every distribution channel. Specific events to configure: lead magnet download, email signup, webinar registration, demo booking, trial signup, trial activation, paid conversion.

- CRM (must have now) — HubSpot free, Pipedrive, or equivalent. Setup: pipeline stages matching funnel stages, lead source tracking tagging every lead with channel and content piece, automated follow-up tasks, simple pipeline health dashboard.
- Email platform (must have now) — Mailchimp, ConvertKit, ActiveCampaign, or equivalent. Setup: all email sequences from Prompt 3B, trigger automations for every conversion event, open and click rate tracking, list segmentation by funnel stage.
- LinkedIn analytics (must have now) — native. Setup: track post reach, engagement rate, follower growth weekly. UTM links on every post CTA.
- Heatmap and session recording (should have in month 2) — Hotjar free tier or Microsoft Clarity (both free). Setup: install on every funnel page, heatmaps for homepage and primary lead capture pages, session recordings filtered to show visitors who did not convert.
- Paid ads tracking (when paid distribution is activated) — LinkedIn Campaign Manager, Google Ads, or Meta Ads Manager. Setup: conversion pixels on every thank you page, conversion events matching funnel CTAs, UTM parameters for every ad.
- Review and social proof tracking (should have in month 2) — G2, Capterra, or Trustpilot. Setup: claim product listing, configure automatic review request emails 30 days after conversion, set up notifications for new reviews.

PART 3 — UTM tracking system

Design a complete UTM tracking system so the founder always knows which channel, content piece, and campaign is generating traffic and leads.

Explain what UTM parameters are in plain language — one paragraph maximum. Then produce:

- UTM naming convention — the specific naming system to use. Define the exact format for: utm_source (where traffic comes from), utm_medium (type of content or channel), utm_campaign (which campaign or funnel stage), utm_content (which specific content piece or ad variation)
- UTM link library — a ready-to-use UTM link for every content piece and distribution channel in the funnel. Present as a table: Content piece | Channel | UTM link structure | What it tracks
- UTM tracking rule — never share a link to any funnel page without a UTM parameter attached. Untracked traffic is invisible traffic.

PART 4 — Weekly tracking routine

Daily check — 5 minutes

What to look at every morning. The single number that tells the founder whether yesterday was a good day for the funnel. Where to find it. What to do if trending wrong.

Weekly check — 30 minutes every Monday morning

A structured review checklist:

- Check TOFU metrics — reach, engagement, new leads. On track with benchmarks?
- Check MOFU metrics — lead to demo or trial conversion. Where are leads dropping off?
- Check BOFU metrics — trial to paid, time to activation. Hitting 60%+ activation?
- Check AFU metrics — retention, expansion. Sticking past month 1?

- Check LOFU metrics — referrals, reviews. Satisfied customers becoming distribution?
- Review UTM data — which channels and content pieces are generating the most leads?
- Review CRM pipeline — which leads are stuck and need follow-up?
- Identify the single biggest leak this week
- Decide the single most important thing to fix or test next week

Monthly deep dive — 2 hours on the first Monday of each month
What to look at, what questions to ask, what decisions to make based on a full month of data: which content pieces are generating the most leads, which channels drive highest quality leads, which funnel stages have biggest drop-off, month-over-month trend for every key metric, what to build, test, or cut next month.

PART 5 — Week by week implementation plan

Week 1 — Foundation
Set up tracking tools, install Google Analytics, configure CRM pipeline stages, set up UTM naming convention, claim review platform listings. Nothing gets published. This week is infrastructure.

Week 2 — Minimum viable funnel live
The 3 most critical assets from Prompt 1 go live. Primary lead capture page from Prompt 4 goes live. First email sequence from Prompt 3B is configured and tested. First content piece from Prompt 5 is published. By end of week 2 the founder can capture their first lead.

Week 3 — First campaign launched
Campaign video from Prompt 3A is filmed and published. Campaign distribution assets from Prompt 3B go live. Campaign landing page is live. By end of week 3 the founder has an active campaign generating traffic.

Week 4 — Distribution system activated
Weekly distribution routine from Prompt 5 is running. All content pieces for the first month are scheduled. UTM link library is built and every link in the funnel is tracked.

Week 5 and 6 — Conversion layer added
Next 7 assets from build order in Prompt 1 are built and live. Demo booking or free trial page from Prompt 4 is live and optimized. MOFU content pieces are published.

Week 7 and 8 — First data review and optimization
First full month of data reviewed using monthly deep dive framework. Biggest funnel leak identified and fixed. Highest performing content piece doubled down on. Lowest performing channel cut or adjusted.

Month 3 — Growth layer
Paid distribution activated. Partnership and OPA distribution initiated. Review platform profiles generating consistent inbound. Referral program from Prompt 4 is live. Funnel now growing through multiple channels.

PART 6 — 90-day success dashboard

Based on the 90-day success definition stated in the inputs, work backward from the goal and define what needs to be true at every stage of the funnel for that outcome to be achievable.

| Metric | Week 4 target | Week 8 target | Day 90 target | Current baseline | Gap to close |

Then produce:

- The one number that matters most — the north star metric the founder checks every single day
- The three early warning signals — the three metrics that if red in the first 30 days signal something is fundamentally wrong
- The three growth signals — the three metrics that if green by day 90 tell the founder it is time to scale

PART 2

PROMPT 7 Reporting

THE GOAL OF THIS PROMPT

By the end of this prompt you will have a 15-minute weekly report template that tells you whether the funnel is working or broken, a 30-minute monthly review template showing whether you are improving month over month, a one-page investor and advisor update template, and a daily north star dashboard showing whether you are on track to hit your 90-day goal.

CLAUDE MEMORY COMMAND

Before you begin this prompt, say: "I have all previous answers stored and will use them throughout this prompt without asking you to repeat them." Then confirm which prompt number we are on and what the key outputs from all previous prompts were so we are aligned before proceeding. If any previous prompt has not been completed, flag it and ask me to complete it first before continuing.

You are a B2B growth analyst working with pre-seed founders. Your job is to build a simple, fast reporting system that tells the founder every week whether their funnel is working, where it is breaking, and what to do about it.

This reporting system has one rule: it must be completable in 30 minutes per week. No complex dashboards. No hours in spreadsheets. Just the numbers that matter, what they mean, and the next action.

REQUIRED INPUTS

129. Your complete funnel architecture from Prompt 1 of Part 2
130. Your tracking system and benchmarks from Prompt 6 of Part 2
131. Your 90-day success definition from Prompt 6 of Part 2 — the specific number you are working toward
132. Your primary conversion goal from Prompt 1 of Part 2

[Paste all inputs before continuing.]

Once I have your inputs produce the following reporting system:

PART 1 — Weekly funnel report

A simple one-page weekly report the founder fills in every Monday in 15 minutes. Three sections only:

Section 1 — The numbers

A simple table with one row per funnel stage. The founder fills in the actual number and the template shows immediately whether it is green, amber, or red.

| Funnel stage | Metric | This week | Last week | Benchmark | Status |

Populate the metric column with the single most important metric per stage based on the primary conversion goal:

- TOFU — new leads generated this week
- MOFU — leads that moved to demo or trial this week
- BOFU — trials or demos that converted to paid this week
- AFU — percentage of new customers who completed activation this week
- LOFU — referrals or reviews generated this week

Provide the formula or conditional logic to make each cell turn green, amber, or red automatically in Google Sheets or Notion.

Section 2 — What it means

Three questions the founder answers in one sentence each:

- Where is the funnel healthy this week?
- Where is the funnel leaking this week?
- What is the single biggest thing holding back growth right now?

Section 3 — The one decision

The single most important thing to do differently next week. One sentence. One action. Nothing else.

This decision follows a simple rule:

- If TOFU is red — the problem is traffic and awareness. Fix distribution first.
- If MOFU is red — the problem is trust and consideration. Fix content and proof.
- If BOFU is red — the problem is conversion. Fix the offer, the page, or the friction.
- If AFU is red — the problem is activation. Fix onboarding and time to value.
- If LOFU is red — the problem is satisfaction. Fix the product or customer experience before scaling anything else.

PART 2 — Monthly funnel review

A simple monthly review completed in 30 minutes on the first Monday of each month. Four parts:

Month in numbers

| Metric | Last month | This month | Change | Trend |

Metrics to include: total website visitors, total new leads, total demos or trials, total new customers, total revenue or MRR, best performing content piece, best performing channel.

What worked
3 bullet points — the 3 things that worked best this month. What they were, why they worked, and whether to do more next month.

What did not work
3 bullet points — the 3 things that did not work. What they were, why they failed, and whether to fix or cut.

Next month priorities
Based on the data — the 3 specific things to focus on next month ranked in order of impact on the primary conversion goal.

PART 3 — Investor and advisor update

A simple monthly update — maximum one page, readable in 2 minutes, written to show traction and momentum not vanity metrics. Five sections:

- The headline number — the single most important metric from this month. One bold number at the top. The north star metric from Prompt 6.
- What happened this month — 3 bullet points. The 3 most important things — wins, learnings, and one honest challenge. No fluff.
- The funnel in one line — a single sentence describing the state of the funnel right now. Example: 'We are generating X leads per week from LinkedIn, converting Y% to demos, and closing Z% of demos to paid.'
- What we learned — one paragraph maximum. The single most important insight from this month's data and what we are doing differently because of it.
- What we need — one bullet point. The single most valuable thing an investor or advisor could help with right now. Specific and actionable — not 'introductions' but 'an introduction to a VP of Sales at a company with 20-50 salespeople in the fintech space.'

PART 4 — North star dashboard

A single visual dashboard the founder looks at every day. Shows only the metrics that matter most for the 90-day goal.

| Week | North star metric target | Actual | On track — yes or no |

Populate the target column by working backward from the 90-day success definition. Show the calculation explicitly — if the goal is 20 paying customers in 90 days and trial to paid conversion is 20% then 100 trial signups are needed in 90 days which means roughly 8 new trial signups per week.

Below the table add three lines only:

- On track signal — the weekly number that tells the founder to keep doing what they are doing
- Off track signal — the weekly number that tells the founder something is wrong and to review the weekly report immediately
- Pivot signal — the number that tells the founder the current approach is not working and a fundamental change is needed before continuing to execute

500 EURASIA

AI Marketing Prompt Library

3 Prompts for Conversion Pages

By Justyna Jastrzebska

HOW TO USE THIS DOCUMENT

This document contains 3 action-based prompts to help you build publish-ready conversion pages from scratch. Each prompt is self-contained — use them independently in any order depending on which page you are building first.

IMPORTANT: Each prompt is designed to be copy-pasted directly into Claude, ChatGPT, or any AI. Fill in every field marked in brackets before running the prompt. The AI will write every section of your page — do not run the prompt with empty fields. The more specific your inputs, the stronger the output.

WHAT'S INSIDE

Prompt 1 — Book a Demo Page

Write every section of a B2B demo request page — headline, pain section, what happens on the call, objection FAQ, and CTA copy. Built around the 20 conversion assets found across Nextiva, Deel, HubSpot, Salesforce, OptinMonster, and 5 other top-performing demo pages.

Prompt 2 — Webinar Registration Page

Write every section of a B2B webinar landing page — speaker block, agenda, takeaway bullets, urgency copy, and the secondary 'can't make it?' CTA. Built around the 20 conversion assets found across Gartner, Forrester, Salesforce, HubSpot, Tailwind, and 5 other top-performing webinar pages.

Prompt 3 — Free Trial Page

Write every section of a B2B SaaS free trial page — trust strip, risk reduction copy, feature-to-benefit translation, 3-step activation flow, and the security block for IT approvers. Built around the 20 conversion assets found across Shopify, Slack, Notion, Calendly, Wrike, and 5 other top-performing free trial pages.

PROMPT 1

Book a Demo Page

THE GOAL OF THIS PROMPT: By the end of this prompt you will have a complete, publish-ready Book a Demo page — including the outcome-led headline, subheadline, above-the-fold trust line, CTA button copy, pain section, how it works, differentiation block, testimonials, logo bar caption, objection FAQ, privacy line, and closing CTA. Built on the 20 conversion assets shared by Nextiva, Deel, HubSpot, Salesforce, OptinMonster, Pecan AI, Heap, Testimonial.to, Qonto, and Podia.

You are a conversion-focused landing page copywriter and strategist. Your job is to write every word and structure every section of a "Book a Demo" page that is ready to publish today.

I am going to give you everything you need. Your output must be a complete, publish-ready page — not suggestions, not a brief, not a framework. Write the actual copy for every section.

Do not use filler phrases like "we help teams," "cutting-edge," or "seamless." Write like a founder who deeply understands the customer's pain.

[YOUR PRODUCT]

Product name:
[YOUR PRODUCT NAME]

One-line description of what it does:
[WHAT IT DOES IN PLAIN ENGLISH — no jargon]

How it works (core mechanic):
[STEP 1 → STEP 2 → STEP 3 — what actually happens when someone uses it]

The single biggest differentiator:
[YOUR #1 DIFFERENTIATOR]

[YOUR CUSTOMER AVATAR]

Job title / role:
[WHO IS BOOKING THIS DEMO]

Company size / type:
[e.g. "50–200 person B2B SaaS companies"]

The #1 pain they have RIGHT NOW (in their own words):
[PASTE A REAL QUOTE OR DESCRIBE IN THEIR LANGUAGE]

The #2 pain:
[SECOND BIGGEST FRUSTRATION]

The #3 pain:
[THIRD BIGGEST FRUSTRATION]

What they've already tried that didn't work:
[TOOLS OR APPROACHES THEY'VE USED AND ABANDONED]

What they're afraid of when booking a demo:
[e.g. "I'll get a pushy sales call," "It won't work for my use case"]

What success looks like for them in 90 days:
[SPECIFIC, MEASURABLE OUTCOME]

[YOUR COMPETITION]

Main competitors they're currently using or evaluating:
[COMPETITOR 1, COMPETITOR 2, COMPETITOR 3]

Why those competitors fail them:
[THE SPECIFIC GAP YOUR PRODUCT FILLS]

How you are different in one sentence:
[YOUR HONEST POSITIONING AGAINST THEM]

[SOCIAL PROOF]

Number of customers / users:
[e.g. "400+ companies"]

Best customer result / stat:
[e.g. "cut onboarding from 3 weeks to 4 days"]

Customer name + quote + role + company:
[REAL TESTIMONIAL — name, title, company, exact words]

Second testimonial (optional):
[SECOND REAL TESTIMONIAL]

Third-party review score:
[e.g. "4.8/5 on G2 — 120 reviews"]

Notable logos you can display:
[CUSTOMER COMPANY NAMES]

[THE DEMO ITSELF]

Demo duration:
[e.g. "25 minutes"]

Who runs it (name + title):
[e.g. "Sarah, co-founder"]

3 specific things the prospect will SEE on the call:
[THING 1] / [THING 2] / [THING 3]

What the prospect will LEAVE WITH:
[e.g. "a custom implementation plan"]

Is it recorded?:
[YES / NO]

How quickly do you respond to a booking?:
[e.g. "within 2 hours on weekdays"]

[FORM FIELDS]

Required fields:
[e.g. "first name, work email, company name"]

One optional qualifying question (1 only):
[e.g. "How many people are on your team?"]

NOW BUILD THE PAGE. Output every section in order with the exact copy written. Follow this structure precisely:

SECTION 1 — HEADLINE
Write 3 options. Each must lead with the customer's outcome, be under 10 words, and name the pain or gain explicitly. Mark your recommended option with [RECOMMENDED].

SECTION 2 — SUBHEADLINE
1 sentence. Name who this is for, what changes, and how fast.

SECTION 3 — ABOVE-THE-FOLD TRUST LINE
Format: "[number] [customer type] already [outcome]." If no customers yet, write a credibility line about the founding team.

SECTION 4 — CTA BUTTON TEXT
3 options. Each must include the time commitment. No generic "Book a demo." Mark recommended.

SECTION 5 — WHAT HAPPENS ON THE DEMO (3 bullets)

3 bullets. Each describes a specific concrete thing the prospect will experience on the call. Start each with an action verb.

SECTION 6 — PAIN SECTION

3 pain statements in the customer's exact language. Short, punchy, line breaks only. No bullet points.

SECTION 7 — HOW IT WORKS (3 steps)

Step number + 4-word-max label + 1-sentence explanation. Scannable in under 10 seconds.

SECTION 8 — DIFFERENTIATION SECTION

Max 4 sentences. Acknowledges the competitor, explains precisely why this product is different. Honest and specific.

SECTION 9 — TESTIMONIAL BLOCK

Intro line + testimonials. Lead with stat callout if one exists. Format: quote → name, title, company.

SECTION 10 — LOGO BAR CAPTION

1 line above the logo bar. Specific to an outcome — not 'trusted by.'

SECTION 11 — OBJECTION FAQ (4 questions)

Address: (1) sales pressure fear, (2) fit/use case doubt, (3) timeline/effort concern, (4) pricing curiosity. 2–3 sentences per answer max.

SECTION 12 — PRIVACY LINE

1 sentence under the form. State what you will NOT do with their email.

SECTION 13 — BOTTOM REPEAT CTA

1-sentence restatement of core outcome + CTA button text (same as Section 4 recommended).

PROMPT 2

Webinar Registration Page

THE GOAL OF THIS PROMPT: By the end of this prompt you will have a complete, publish-ready webinar registration page — including the outcome-led headline, date/time bar, speaker block with borrowed authority, what-you'll-learn bullets, timestamped agenda, pain section, social proof block, objection FAQ, recording opt-in CTA, and privacy line. Built on the 20 conversion assets shared by Gartner, Forrester, Salesforce, HubSpot, Slack, WordStream, Tailwind, Hootsuite, Dock, and SEMrush.

You are a conversion-focused landing page copywriter specialising in B2B webinars. Your job is to write every word and structure every section of a webinar registration page that is ready to publish today.

Your output must be complete, publish-ready page copy — not suggestions, not a brief, not a framework. Write the actual copy for every section listed below.

Do not use filler phrases like "join industry experts," "actionable insights," or "don't miss out." Write like someone who understands why this specific audience would block out 45 minutes of their calendar for this.

[THE WEBINAR ITSELF]

Webinar title (working title is fine):
[YOUR WORKING TITLE]

What the attendee will know or be able to DO after attending:
[THE SPECIFIC TRANSFORMATION — concrete, not vague]

The single most surprising thing that will be revealed:
[THE HOOK — what will make them say "I didn't know that"]

Live date and time:
[DAY, DATE, TIME + TIMEZONE — e.g. Tuesday 15 April, 11am GMT]

Duration:
[e.g. "45 minutes + 15 min live Q&A;"]

Format:
[e.g. "live panel," "solo presentation," "founder interview," "workshop"]

Will a recording be available to registrants?:
[YES / NO / YES — registrants only, not public]

Is attendance capped / limited seats?:
[YES with number / NO]

[THE SPEAKER(S)]

Speaker 1 — full name:
[NAME]

Speaker 1 — title and company:
[TITLE, COMPANY]

Speaker 1 — why they are the right person to teach this:
[SPECIFIC RELEVANT EXPERIENCE — not just their job title]

Speaker 1 — recognisable brand affiliations:
[e.g. "formerly at Google, Stripe, Y Combinator"]

Speaker 2 (if applicable):
[SAME FIELDS AS ABOVE — or leave blank]

[THE AUDIENCE]

Who this webinar is for:
[e.g. "B2B SaaS founders pre-Series A"]

The exact problem they have RIGHT NOW (in their own words):
[PASTE A REAL QUOTE OR DESCRIBE IN THEIR LANGUAGE]

What they have already tried that has not worked:
[TOOLS, APPROACHES, OR TACTICS THEY'VE TESTED AND ABANDONED]

Their biggest fear about spending time on a webinar:
[e.g. "it will be too basic," "it's just a sales pitch"]

What success looks like 30 days after attending:
[SPECIFIC, MEASURABLE OUTCOME]

[SOCIAL PROOF]

Number of people already registered:
[e.g. "312 registered so far"]

Past webinar attendance or satisfaction stat:
[e.g. "4.8/5 from 600 past attendees"]

One testimonial from a past attendee:
[REAL QUOTE — name, title, company, exact words]

Notable company logos you can show:
[COMPANY NAMES]

[WHAT THEY WILL WALK AWAY WITH]

Specific takeaway 1:
[CONCRETE TAKEAWAY — not "understand the landscape"]

Specific takeaway 2:
[CONCRETE TAKEAWAY]

Specific takeaway 3:
[CONCRETE TAKEAWAY]

Downloadable resource or template:
[e.g. "the exact Notion template we use" — or leave blank]

[AGENDA]

Segment 1:
[TIMESTAMP — TOPIC, e.g. '0:00 — Why the standard approach fails']

Segment 2:
[TIMESTAMP — TOPIC]

Segment 3:
[TIMESTAMP — TOPIC]

Segment 4 (Q&A;):
[TIMESTAMP — Live Q&A;]

[FORM FIELDS]

Required fields:
[e.g. "work email only"]

One optional qualifying question (if any):
[or leave blank]

NOW BUILD THE PAGE. Output every section in order with the exact copy written. Follow this structure precisely:

SECTION 1 — PAGE HEADLINE

3 options. Each must lead with specific outcome, be under 12 words, contain the hook or counterintuitive angle. Mark recommended.

SECTION 2 — SUBHEADLINE

1–2 sentences. Name who this is for, what changes, by when. Must mention the date and duration.

SECTION 3 — DATE / TIME / FORMAT BAR

Format: [Day, Date] · [Time + Timezone] · [Duration] · [Format] · [Free]

SECTION 4 — CTA BUTTON TEXT

Must not say "Register Now" or "Sign Up." 3 options. Must communicate what the registrant is getting. Mark recommended.

SECTION 5 — ABOVE-THE-FOLD TRUST LINE

The social proof micro-line beside the CTA. Format: "[X] people already registered" or "4.8/5 from [X] past attendees." If no social proof, write a speaker credibility line.

SECTION 6 — SPEAKER BLOCK

For each speaker: 2-sentence bio leading with most relevant credential for THIS topic. Borrowed authority line. 1 sentence on why they're the right person — track record, not title.

SECTION 7 — WHAT YOU WILL LEARN (3–5 bullets)

Each bullet: action verb + concrete thing the attendee walks away able to do. Specific enough that a stranger can evaluate if it's worth their time.

SECTION 8 — PAIN SECTION

3 pain statements in the audience's exact language. Short punchy lines. No bullets. Line breaks only.

SECTION 9 — AGENDA BLOCK

Format: [Timestamp] — [Topic name] — [1-sentence description]. If agenda left blank, create a logical 45-minute structure and flag: SUGGESTED AGENDA — confirm before publishing.

SECTION 10 — SOCIAL PROOF BLOCK

Intro line + testimonials. Lead with stat callout if one exists. If no testimonial: write [PLACEHOLDER] — do not fabricate.

SECTION 11 — OBJECTION FAQ (4 questions)

Address: (1) "Is this just a sales pitch?", (2) "I can't make the live time", (3) "Is this too basic/advanced for me?", (4) "How long is it really?" 2 sentences per answer max.

SECTION 12 — 'CAN'T MAKE IT?' SECONDARY CTA

Short paragraph + button text for the recording opt-in. Button text must differ from main CTA.

SECTION 13 — PRIVACY LINE

1 sentence under the form. State what you will NOT do with their email.

SECTION 14 — BOTTOM REPEAT CTA

1-sentence restatement of core transformation + date/time + main CTA button text repeated.

PROMPT 3

Free Trial Page

THE GOAL OF THIS PROMPT: By the end of this prompt you will have a complete, publish-ready free trial landing page — including the outcome-led headline, trust strip, social proof bar, problem section, feature-to-benefit blocks, 3-step activation flow, testimonials, product visual caption, security block written for IT approvers, objection FAQ, pricing context, and the closing CTA. Built on the 20 conversion assets shared by Shopify, Slack, Notion, Calendly, Dropbox, Airtable, Salesforce, Wrike, Hootsuite, and Contentful.

You are a conversion-focused landing page copywriter specialising in B2B SaaS free trials. Your job is to write every word and structure every section of a free trial landing page that is ready to publish today.

Your output must be complete, publish-ready page copy — not suggestions, not a brief, not a framework. Write the actual copy for every section listed below.

Do not use filler phrases like "powerful," "robust," "seamless," "all-in-one," or "game-changing." Write like someone who understands exactly what the visitor is afraid of and exactly what they want to be able to do tomorrow that they cannot do today.

The single job of this page is to get a qualified visitor to start a free trial. Every section exists to remove a reason not to start — not to impress, not to educate, not to tell the brand story.

[THE PRODUCT]

Product name:
[YOUR PRODUCT NAME]

One sentence — what it does and for whom:
[WHAT IT DOES IN PLAIN ENGLISH — no jargon]

The single outcome a user gets in their first session (their 'aha moment'):
[FIRST THING THAT MAKES THEM THINK "OK THIS IS WORTH PAYING FOR"]

How long it takes to get that first result:
[e.g. "under 10 minutes," "within the first session," "same day"]

The 3 core things users do with the product:
[THING 1] / [THING 2] / [THING 3]

[THE TRIAL STRUCTURE]

Trial type:
[OPT-IN (no credit card) / OPT-OUT (credit card required)]

Trial length:
[e.g. "14 days" / "30 days"]

What is included in the trial:
[FULL ACCESS / or list what is and isn't included]

What happens at the end of the trial:
[e.g. "automatically converts to paid" / "account pauses — data kept for 30 days"]

Credit card required to start?:
[YES / NO]

Can they cancel at any time?:
[YES / NO]

Free plan after the trial ends?:
[YES with limits / NO]

[THE CUSTOMER]

Job title / role of ideal trial user:
[WHO THIS IS FOR — be specific]

Company size / type:
[e.g. "B2B SaaS companies 10–200 people"]

The problem they have right now (in their own words):
[THEIR PAIN — use their language, not yours]

What they are currently doing instead (their workaround):
[THE TOOL OR PROCESS THEY USE TODAY THAT ISN'T WORKING]

Their biggest fear about starting a free trial:
[e.g. "I'll get charged without realising" / "it'll take too long to set up"]

Their second biggest fear:
[SECOND OBJECTION]

What success looks like after 7 days of using the product:
[SPECIFIC, MEASURABLE OUTCOME]

[SOCIAL PROOF]

Number of users or companies currently using the product:
[e.g. "12,000 teams" or "used by 400+ companies"]

Best result a customer has achieved (stat + context):
[e.g. "reduced reporting time from 4 hours to 20 minutes"]

Customer testimonial 1 (name + role + company + exact quote):
[REAL QUOTE — must include the person's specific result, not generic praise]

Customer testimonial 2 (optional):
[SECOND REAL QUOTE]

G2 / Capterra / TrustPilot score + number of reviews:
[e.g. "4.7/5 from 380 reviews on G2"]

Notable company logos you can display:
[COMPANY NAMES]

[TRUST AND COMPLIANCE]

Security certifications held:
[e.g. "SOC 2 Type II," "GDPR compliant," "ISO 27001" — or leave blank]

Data handling reassurance:
[e.g. "data deleted within 30 days of account closure" / "never sold to third parties"]

Uptime or reliability stat (if any):
[e.g. "99.9% uptime SLA"]

[SIGN-UP FORM]

Fields required to start the trial:
[e.g. "work email only" / "name + work email"]

SSO / social login options available:
[e.g. "Sign up with Google" / "Sign up with Microsoft" / none]

One optional qualifying question after email (if any):
[e.g. "What tool are you replacing?" — or leave blank]

[PRICING CONTEXT (optional but recommended)]

Starting price after trial ends:
[e.g. "from $49/month"]

Is there a free plan after the trial?:
[YES — describe limits / NO]

NOW BUILD THE PAGE. Output every section in order with the exact copy written. Follow this structure precisely:

SECTION 1 — HEADLINE

3 options. Each must name the specific outcome, be under 10 words, speak to pain or transformation. Make the trial feel like the obvious next step. Mark recommended.

SECTION 2 — SUBHEADLINE

1 sentence. Name who this is for, what they'll be able to do, how fast. Reference trial length and 'no credit card required' if applicable. Max 20 words.

SECTION 3 — PRIMARY CTA BUTTON TEXT

Must not say "Sign Up," "Get Started," or "Try for Free." 3 options. Must communicate what they are getting. Mark recommended.

SECTION 4 — TRUST STRIP

Format: "[No credit card required] · [Cancel anytime] · [Trial length]". If card IS required, replace first element with a different reassurance.

SECTION 5 — SOCIAL PROOF BAR

Intro line above logo bar + stat line below it. If no logos: write credibility line and flag [PLACEHOLDER — add logos when available].

SECTION 6 — THE PROBLEM SECTION

3 pain statements in the customer's exact language. Short punchy lines. Line breaks only. End with 1 line that signals the trial is the answer — without making it a pitch.

SECTION 7 — WHAT YOU GET IN THE TRIAL (3 feature-to-benefit blocks)

Format: [Short label — 3 words max] / [1-sentence benefit in user's language]. Not what the feature does — what the user stops doing or is now able to do.

SECTION 8 — 'UP AND RUNNING IN 3 STEPS'

Numbered flow. Each step: 3-word label + 1-sentence description. Step 1 = sign-up. Step 3 = aha moment. Each step must feel achievable in minutes.

SECTION 9 — SOCIAL PROOF BLOCK

Stat callout if exists + testimonials. Format: quote → [name, title, company]. Each quote must mention a specific result. Intro line: 'Here's what [role] say after their first week.' If none: [PLACEHOLDER] — do not fabricate.

SECTION 10 — PRODUCT VISUAL CAPTION

1 line below the product screenshot or GIF. Describe what the user is seeing and why it matters to them. Not 'the dashboard' — what it means for their day.

SECTION 11 — TRUST AND SECURITY BLOCK

3–4 trust signals as short label + 1-sentence explanation. Cover: data security, uptime, what happens to data at trial end, and one human support signal. Write for the person who has to approve the tool — not the person who wants to use it.

SECTION 12 — OBJECTION FAQ (4 questions)

Address: (1) billing/credit card fear, (2) setup effort fear, (3) data safety if they don't upgrade, (4) fit — is this right for my role/company? 2 sentences max per answer.

SECTION 13 — PRICING CONTEXT LINE

Format: "After your trial, plans start from [price]. No surprise fees, no annual lock-in required." If pricing not public, write the alternative version.

SECTION 14 — BOTTOM REPEAT CTA

1-sentence core outcome restatement + CTA button text (same as Section 3 recommended) + trust strip (same as Section 4).

SECTION 15 — PRIVACY LINE

1 sentence under form. State what you will NOT do with their email. No legal language. No links.

FORMATTING RULES FOR ALL PROMPTS

Label each section clearly. Write actual copy — no placeholders except where you explicitly left a field blank. If you left a field blank, write [PLACEHOLDER: what goes here] — do not skip the section. Flag any assumption with: ASSUMPTION: [what you assumed]. Do not explain your choices. Just write the page. Do not add sections not listed above.

Source: Original files/Complete B11 HTML library/Source notes/85-item content catalog.json. Historical wording preserved; not a new approval or factual revalidation.

85-item content catalog.json

Technical appendix. This is an archived working file, not instructions to execute.


```
[
  {"ID":"T01","title":"One patient, seven handoffs","format":"LinkedIn
post","stage":"TOFU","idea":"Sketch a synthetic patient journey and ask leaders which handoff their
team repeats most.","CTA":"Map my hospital's first
improvement","pageId":"P01","recommendedChannel":"Founder LinkedIn"},
  {"ID":"T02","title":"The hidden cost of searching","format":"LinkedIn
post","stage":"TOFU","idea":"Describe the time spent finding a result across teams as a question to
measure, without inventing a savings figure.","CTA":"Use the pathway
worksheet","pageId":"P01","recommendedChannel":"Founder LinkedIn"},
  {"ID":"T03","title":"When two reports disagree","format":"LinkedIn post","stage":"TOFU","idea":"Pose
a board-meeting scenario where finance and clinical reports disagree and show three questions to
ask.","CTA":"Get the reconciliation checklist","pageId":"P01","recommendedChannel":"Founder
LinkedIn"},
  {"ID":"T04","title":"The duplicate-entry trail","format":"Short video","stage":"TOFU","idea":"Phone-
filmed desk sketch follows one fictional lab order copied between reception, clinician, and
billing.","CTA":"Map my hospital's first improvement","pageId":"P01","recommendedChannel":"Founder
LinkedIn video"},
  {"ID":"T05","title":"Three calls for one result","format":"Short
video","stage":"TOFU","idea":"Founder acts out three routine calls needed to locate one synthetic
result and asks what happens locally.","CTA":"See the pathway
map","pageId":"P01","recommendedChannel":"Founder LinkedIn video"},
  {"ID":"T06","title":"The spreadsheet behind the dashboard","format":"Short
video","stage":"TOFU","idea":"Show a blank synthetic spreadsheet and the questions a CEO should ask
about report preparation time.","CTA":"Get the checklist","pageId":"P01","recommendedChannel":"Founder
LinkedIn video"},
  {"ID":"T07","title":"How to map a patient handoff","format":"Blog/SEO","stage":"TOFU","idea":"Step-
by-step guide to drawing one patient's movement through departments and marking repeat
entry.","CTA":"Download the worksheet","pageId":"P01","recommendedChannel":"Website search"},
  {"ID":"T08","title":"Why hospital reports
disagree","format":"Blog/SEO","stage":"TOFU","idea":"Explain differences in definitions, timing,
missing data, and manual reconciliation without blaming one vendor.","CTA":"Get the reconciliation
checklist","pageId":"P01","recommendedChannel":"Website search"},
  {"ID":"T09","title":"What a delayed claim does and does not
mean","format":"Blog/SEO","stage":"TOFU","idea":"Distinguish rejected claim, delayed cash, missed
legitimate charge, and unrecoverable revenue.","CTA":"Map one claim
pathway","pageId":"P01","recommendedChannel":"Website search"},
  {"ID":"T10","title":"Patient-pathway friction worksheet","format":"Lead
magnet","stage":"TOFU","idea":"One-page template to count handoffs, repeat entries, wait points,
owners, and a starting baseline.","CTA":"Download the
worksheet","pageId":"P01","recommendedChannel":"Website and founder outreach"},
  {"ID":"T11","title":"Five-minute handoff check","format":"Lead magnet","stage":"TOFU","idea":"Self-
assessment that returns the first pathway worth discussing, not a made-up hospital score.","CTA":"Run
the check","pageId":"P01","recommendedChannel":"Website and founder outreach"},
  {"ID":"T12","title":"Founder-built scope trace","format":"Proof","stage":"TOFU","idea":"Use the
public founder-authored Medplum case-study context as product-history evidence, clearly not a customer
outcome endorsement.","CTA":"See the operating system","pageId":"P02","recommendedChannel":"Website
and founder LinkedIn"},
  {"ID":"T13","title":"Synthetic handoff before/after","format":"Proof","stage":"TOFU","idea":"Show a
clearly labeled synthetic workflow comparison to prove the method is inspectable, not that savings
occurred.","CTA":"See the pathway map","pageId":"P02","recommendedChannel":"Website and founder
LinkedIn"},
  {"ID":"T14","title":"Where the work repeats","format":"Email","stage":"TOFU","idea":"Send the
worksheet with one question about the hospital's last repeated handoff; no generic AI
pitch.","CTA":"Use the pathway worksheet","pageId":"P01","recommendedChannel":"Opt-in email"},
  {"ID":"T15","title":"A report you had to rebuild","format":"Email","stage":"TOFU","idea":"Invite a
leader to identify one report that requires manual reconciliation and the person who prepares
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it.","CTA":"Get the checklist","pageId":"P01","recommendedChannel":"Opt-in email"},
  {"ID":"T16","title":"Hospital-operator discussion
prompt","format":"Community","stage":"TOFU","idea":"In an appropriate Georgian hospital-leadership
forum, ask how teams map a handoff; participate only where welcome.","CTA":"Read the handoff
guide","pageId":"P01","recommendedChannel":"Relevant hospital-leadership forum"},
  {"ID":"T17","title":"Clinical-operations partner
note","format":"Community","stage":"TOFU","idea":"Offer the neutral worksheet to a clinical operations
association or trusted professional contact for feedback.","CTA":"Download the
worksheet","pageId":"P01","recommendedChannel":"Professional association or introduction"},
  {"ID":"M01","title":"Fix a screen or fix the pathway?","format":"LinkedIn
post","stage":"MOFU","idea":"Compare three practical options: current-vendor change, narrow tool, or
shared operating system for the same handoff.","CTA":"Compare the
approaches","pageId":"P07","recommendedChannel":"Founder LinkedIn"},
  {"ID":"M02","title":"What parallel evaluation actually requires","format":"LinkedIn
post","stage":"MOFU","idea":"Name the hospital owner, data permission, baseline, and staff time behind
an honest two-week evaluation.","CTA":"See the evaluation
plan","pageId":"P06","recommendedChannel":"Founder LinkedIn"},
  {"ID":"M03","title":"AI assists; clinicians decide","format":"LinkedIn
post","stage":"MOFU","idea":"Explain a reviewed draft or flag in a fictional pathway and show where
the clinical decision stays human.","CTA":"See how it
works","pageId":"P02","recommendedChannel":"Founder LinkedIn"},
  {"ID":"M04","title":"Three options on one whiteboard","format":"Short
video","stage":"MOFU","idea":"Founder draws status quo, point solution, and operating-system path with
honest change effort for each.","CTA":"Compare the
approaches","pageId":"P07","recommendedChannel":"Founder LinkedIn video"},
  {"ID":"M05","title":"What happens in 25 minutes","format":"Short
video","stage":"MOFU","idea":"Preview the founder-led pathway demo: buyer names a handoff, founder
maps it, team picks a first measure.","CTA":"Request a pathway
demo","pageId":"P05","recommendedChannel":"Founder LinkedIn video"},
  {"ID":"M06","title":"A result becomes a task","format":"Short
video","stage":"MOFU","idea":"Synthetic screen-recorded example shows a result routed to a named
reviewer, with no patient data.","CTA":"See the operating
system","pageId":"P02","recommendedChannel":"Founder LinkedIn video"},
  {"ID":"M07","title":"How to evaluate hospital software without a risky
cutover","format":"Blog/SEO","stage":"MOFU","idea":"Guide to pathway scope, authorized data, parallel
workload, success criteria, reconciliation, and sign-off.","CTA":"Get the evaluation
plan","pageId":"P06","recommendedChannel":"Website search"},
  {"ID":"M08","title":"Hospital OS versus overlay versus current vendor
change","format":"Blog/SEO","stage":"MOFU","idea":"Decision guide for when each option is appropriate,
including cases where replacement is not justified.","CTA":"Compare the
approaches","pageId":"P07","recommendedChannel":"Website search"},
  {"ID":"M09","title":"What FHIR R4 changes in hospital
work","format":"Blog/SEO","stage":"MOFU","idea":"Plain-language explanation of a shared data structure
and the interface work still required.","CTA":"See the product
path","pageId":"P02","recommendedChannel":"Website search"},
  {"ID":"M10","title":"Parallel-evaluation readiness checklist","format":"Lead
magnet","stage":"MOFU","idea":"Checklist for named owners, agreed pathway, data permissions, baseline,
IT review, and stop criteria.","CTA":"Download readiness
checklist","pageId":"P06","recommendedChannel":"Website and founder outreach"},
  {"ID":"M11","title":"Three-option decision worksheet","format":"Lead
magnet","stage":"MOFU","idea":"Template comparing incumbent fix, narrow add-on, and MediMind against
burden, risk, cost, and scope.","CTA":"Compare the
approaches","pageId":"P07","recommendedChannel":"Website and founder outreach"},
  {"ID":"M12","title":"Documented domain map","format":"Proof","stage":"MOFU","idea":"Publish the 25-
domain scope as an internal product map with clear deployed, accepted, and pending fields after
verification.","CTA":"Inspect product scope","pageId":"P10","recommendedChannel":"Technical diligence
and website"},
  {"ID":"M13","title":"Live-pathway evidence template","format":"Proof","stage":"MOFU","idea":"Show
the exact baseline and after fields a customer-approved case study would need; label outcome cells
pending.","CTA":"See the evidence method","pageId":"P10","recommendedChannel":"Website and founder
follow-up"},
  {"ID":"M14","title":"The change-risk question","format":"Email","stage":"MOFU","idea":"Explain what
a parallel comparison can answer and ask which transition failure the hospital fears most.","CTA":"See
the evaluation plan","pageId":"P06","recommendedChannel":"Opt-in email"},
  {"ID":"M15","title":"The current-vendor alternative","format":"Email","stage":"MOFU","idea":"Invite
the buyer to compare a current-vendor fix with a connected pathway using the same
criteria.","CTA":"Compare the approaches","pageId":"P07","recommendedChannel":"Opt-in email"},
  {"ID":"M16","title":"Peer discussion on evaluation
design","format":"Community","stage":"MOFU","idea":"Ask consenting hospital operators what evidence
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they require before changing a core workflow.","CTA":"Get the readiness
checklist","pageId":"P06","recommendedChannel":"Invited operator roundtable"},
  {"ID":"M17","title":"IT and clinical review
session","format":"Community","stage":"MOFU","idea":"Offer an educational walk-through of data,
safety, and cutover questions with no sales claims.","CTA":"See the evaluation
plan","pageId":"P06","recommendedChannel":"Professional association or partner session"},
  {"ID":"B01","title":"What your 25-minute demo will settle","format":"LinkedIn
post","stage":"BOFU","idea":"List the three outputs: first pathway map, first metric, and evaluation
readiness decision.","CTA":"Request a pathway demo","pageId":"P05","recommendedChannel":"Founder
LinkedIn"},
  {"ID":"B02","title":"Match price, define total cost","format":"LinkedIn
post","stage":"BOFU","idea":"Describe early-partner software-price match and the need to itemize
transition and third-party costs.","CTA":"Discuss the early-partner
offer","pageId":"P06","recommendedChannel":"Founder LinkedIn"},
  {"ID":"B03","title":"The honest go/no-go test","format":"LinkedIn post","stage":"BOFU","idea":"Show
how leadership can decide after staff compare one pathway and its extra work or risk.","CTA":"Request
a pathway demo","pageId":"P05","recommendedChannel":"Founder LinkedIn"},
  {"ID":"B04","title":"Demo in sixty seconds","format":"Short video","stage":"BOFU","idea":"Founder
shows synthetic map, shared task, and comparison worksheet as the demo preview.","CTA":"Request a
pathway demo","pageId":"P05","recommendedChannel":"Founder LinkedIn video"},
  {"ID":"B05","title":"Parallel beside current work","format":"Short
video","stage":"BOFU","idea":"Draw existing and proposed pathways side by side and explain readiness,
workload, and sign-off.","CTA":"Discuss an evaluation","pageId":"P06","recommendedChannel":"Founder
LinkedIn video"},
  {"ID":"B06","title":"Who should join the first call?","format":"Short
video","stage":"BOFU","idea":"Name CEO or operator plus optional clinical/operations and IT
counterparts, depending on decision stage.","CTA":"Request a pathway
demo","pageId":"P05","recommendedChannel":"Founder LinkedIn video"},
  {"ID":"B07","title":"Questions to ask before a hospital OS
demo","format":"Blog/SEO","stage":"BOFU","idea":"Give buyers questions about scope, human review, data
rights, deployment proof, and total cost.","CTA":"Request a pathway
demo","pageId":"P05","recommendedChannel":"Website search"},
  {"ID":"B08","title":"What happens after a parallel
evaluation","format":"Blog/SEO","stage":"BOFU","idea":"Explain measured review, reconciliation,
approval, and staged rollout; no automatic production switch.","CTA":"See the evaluation
plan","pageId":"P06","recommendedChannel":"Website search"},
  {"ID":"B09","title":"What current-software price match should
include","format":"Blog/SEO","stage":"BOFU","idea":"Explain the questions a hospital should settle
about eligible spend, term, development, and total cost.","CTA":"Discuss the early-partner
offer","pageId":"P06","recommendedChannel":"Website search"},
  {"ID":"B10","title":"Pathway demo preparation card","format":"Lead
magnet","stage":"BOFU","idea":"One-page card for an executive to bring a recent incident, current
workaround, owner, and desired measure.","CTA":"Prepare my
demo","pageId":"P05","recommendedChannel":"Demo confirmation email and website"},
  {"ID":"B11","title":"Evaluation decision sheet","format":"Lead
magnet","stage":"BOFU","idea":"Go/adjust/stop template that records baseline, staff burden, risk,
cost, and sign-off owner.","CTA":"Discuss an evaluation","pageId":"P06","recommendedChannel":"Founder
sales follow-up"},
  {"ID":"B12","title":"Verified implementation timeline","format":"Proof","stage":"BOFU","idea":"After
authorization, publish dated scope, readiness, and acceptance milestones from a real customer without
claiming broad outcomes.","CTA":"Review verified
evidence","pageId":"P03","recommendedChannel":"Founder sales follow-up"},
  {"ID":"B13","title":"Buyer-approved workflow result","format":"Proof","stage":"BOFU","idea":"Reserve
case study for a measured result and customer permission; until then show evidence fields as
pending.","CTA":"See the evidence method","pageId":"P10","recommendedChannel":"Website and founder
sales follow-up"},
  {"ID":"B14","title":"Your pathway before our demo","format":"Email","stage":"BOFU","idea":"Ask
attendee for one recent handoff and offer the preparation card; do not request patient-identifying
details.","CTA":"Prepare my demo","pageId":"P05","recommendedChannel":"Demo-confirmation email"},
  {"ID":"B15","title":"What we will compare","format":"Email","stage":"BOFU","idea":"After demo, send
a proposed baseline and evaluation scope with unresolved requirements visibly marked.","CTA":"Review
the evaluation plan","pageId":"P06","recommendedChannel":"Founder follow-up email"},
  {"ID":"B16","title":"Private buying-committee
walkthrough","format":"Community","stage":"BOFU","idea":"Convene named clinical, finance, operations,
and IT evaluators around one pathway, by invitation.","CTA":"Request a pathway
demo","pageId":"P05","recommendedChannel":"Private hospital meeting"},
  {"ID":"B17","title":"Procurement questions
session","format":"Community","stage":"BOFU","idea":"Offer a structured discussion of costs,
responsibilities, support, and exit needs with the buying group.","CTA":"Discuss the early-partner
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offer","pageId":"P06","recommendedChannel":"Private hospital meeting"},
  {"ID":"A01","title":"First pathway, first baseline","format":"LinkedIn post","stage":"AFU","idea":"With permission, describe the method for recording a baseline after sign-up, without showing confidential data.","CTA":"Review the activation plan","pageId":"P08","recommendedChannel":"Customer-only LinkedIn or direct update"},
  {"ID":"A02","title":"What staff should report in week one","format":"LinkedIn post","stage":"AFU","idea":"Explain how to capture repeat work, exceptions, and safety concerns during a controlled evaluation.","CTA":"Use the activation checklist","pageId":"P08","recommendedChannel":"Customer-only update"},
  {"ID":"A03","title":"One improvement, then the next","format":"LinkedIn post","stage":"AFU","idea":"Show an anonymized learning loop: observe, prioritize, test, approve, measure; no unverified success claim.","CTA":"Review next-pathway options","pageId":"P08","recommendedChannel":"Customer-only update"},
  {"ID":"A04","title":"How to log a handoff issue","format":"Short video","stage":"AFU","idea":"Phone walkthrough of an issue form with synthetic patient data and an owner for follow-up.","CTA":"Open the activation guide","pageId":"P08","recommendedChannel":"Customer onboarding"},
  {"ID":"A05","title":"Five-minute pathway review","format":"Short video","stage":"AFU","idea":"Show how clinical and operations owners compare the agreed metric and exceptions each week.","CTA":"Review the activation plan","pageId":"P08","recommendedChannel":"Customer onboarding"},
  {"ID":"A06","title":"How a testable change becomes approved","format":"Short video","stage":"AFU","idea":"Explain request, test, clinical/IT review, and production approval with a generic example.","CTA":"Use the change checklist","pageId":"P08","recommendedChannel":"Customer onboarding"},
  {"ID":"A07","title":"Measuring repeat work without double counting","format":"Blog/SEO","stage":"AFU","idea":"Define staff time, task count, training burden, and why minutes do not automatically equal cash savings.","CTA":"Use the measurement sheet","pageId":"P08","recommendedChannel":"Customer knowledge base"},
  {"ID":"A08","title":"Running a safe pathway review","format":"Blog/SEO","stage":"AFU","idea":"Guide to issue ownership, clinical escalation, baseline comparison, and go/adjust/stop decisions.","CTA":"Open the activation guide","pageId":"P08","recommendedChannel":"Customer knowledge base"},
  {"ID":"A09","title":"Selecting the second pathway","format":"Blog/SEO","stage":"AFU","idea":"Criteria for choosing a next workflow after the first is accepted: frequency, burden, risk, and readiness.","CTA":"Review next-pathway options","pageId":"P08","recommendedChannel":"Customer knowledge base"},
  {"ID":"A10","title":"First-pathway measurement sheet","format":"Lead magnet","stage":"AFU","idea":"Customer worksheet for baseline, tasks, exceptions, staff burden, and sign-off.","CTA":"Download the measurement sheet","pageId":"P08","recommendedChannel":"Customer onboarding"},
  {"ID":"A11","title":"Change-request decision card","format":"Lead magnet","stage":"AFU","idea":"Template for request owner, test criteria, clinical impact, priority, and production approval.","CTA":"Use the change checklist","pageId":"P08","recommendedChannel":"Customer onboarding"},
  {"ID":"A12","title":"Accepted-workflow log","format":"Proof","stage":"AFU","idea":"Internal customer-approved ledger of which pathway elements were accepted and which remain open.","CTA":"Review the activation plan","pageId":"P08","recommendedChannel":"Private customer review"},
  {"ID":"A13","title":"Measured improvement record","format":"Proof","stage":"AFU","idea":"Record an actual agreed before/after comparison only after data quality and buyer sign-off; no public release by default.","CTA":"Review the measurement sheet","pageId":"P08","recommendedChannel":"Private customer review"},
  {"ID":"A14","title":"Your first review agenda","format":"Email","stage":"AFU","idea":"Send the clinical/operations owner a concise agenda for metric, exceptions, staff feedback, and next decision.","CTA":"Review the activation plan","pageId":"P08","recommendedChannel":"Customer email"},
  {"ID":"A15","title":"A request ready for test","format":"Email","stage":"AFU","idea":"Tell the named owner what changed, where to test it, what remains unapproved, and how to respond.","CTA":"Use the change checklist","pageId":"P08","recommendedChannel":"Customer email"},
  {"ID":"A16","title":"Clinical-IT issue huddle","format":"Community","stage":"AFU","idea":"Short private working session for clinicians and IT to resolve one observed pathway issue.","CTA":"Open the activation guide","pageId":"P08","recommendedChannel":"Private customer meeting"},
  {"ID":"A17","title":"Cross-site learning exchange","format":"Community","stage":"AFU","idea":"With a group customer, invite site owners to compare process lessons without patient details.","CTA":"Review next-pathway options","pageId":"P08","recommendedChannel":"Private customer meeting"},
  {"ID":"L01","title":"The buyer's permission comes first","format":"LinkedIn post","stage":"LOFU","idea":"Explain what a responsible public hospital story needs: verified result, named scope, and permission.","CTA":"See approved stories","pageId":"P10","recommendedChannel":"Founder LinkedIn"},
  {"ID":"L02","title":"A pathway another leader can inspect","format":"LinkedIn post","stage":"LOFU","idea":"After approval, share a measured pathway story with method and limits rather than a sweeping transformation claim.","CTA":"Read the case

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study","pageId":"P03","recommendedChannel":"Founder LinkedIn"},
  {"ID":"L03","title":"What we changed after feedback","format":"LinkedIn
post","stage":"LOFU","idea":"Publish a consented example of customer feedback becoming a tested
product change, with dates verified.","CTA":"See approved
stories","pageId":"P10","recommendedChannel":"Founder LinkedIn"},
  {"ID":"L04","title":"Customer pathway walkthrough","format":"Short
video","stage":"LOFU","idea":"With customer approval, have staff explain a measured before/after task
using non-identifying visuals.","CTA":"Read the case
study","pageId":"P03","recommendedChannel":"Founder and customer LinkedIn"},
  {"ID":"L05","title":"What a reference actually verified","format":"Short
video","stage":"LOFU","idea":"Founder shows the evidence standard and a permissioned result; hold
production until proof exists.","CTA":"See approved
stories","pageId":"P10","recommendedChannel":"Founder LinkedIn video"},
  {"ID":"L06","title":"Why we chose the first pathway","format":"Short
video","stage":"LOFU","idea":"Customer-approved owner explains why the initial workflow was selected
and what remains unsolved.","CTA":"Read the case study","pageId":"P03","recommendedChannel":"Founder
and customer LinkedIn"},
  {"ID":"L07","title":"How to read a hospital software case
study","format":"Blog/SEO","stage":"LOFU","idea":"Teach readers to distinguish contracted sites, live
sites, accepted workflow, and measured outcome.","CTA":"See approved
stories","pageId":"P10","recommendedChannel":"Website search"},
  {"ID":"L08","title":"From one hospital pathway to
another","format":"Blog/SEO","stage":"LOFU","idea":"Permissioned account of what transferred across
sites and what required local redesign.","CTA":"Read the case
study","pageId":"P03","recommendedChannel":"Website search"},
  {"ID":"L09","title":"What changed in a claim
handoff","format":"Blog/SEO","stage":"LOFU","idea":"Only after verification, explain actual claim-
process result and separate identified charges from collections.","CTA":"Read the case
study","pageId":"P03","recommendedChannel":"Website search"},
  {"ID":"L10","title":"Reference-story evidence checklist","format":"Lead
magnet","stage":"LOFU","idea":"Template a hospital can use to evaluate any vendor story: baseline,
denominator, site, dates, approval.","CTA":"Download the
checklist","pageId":"P10","recommendedChannel":"Website and partner sharing"},
  {"ID":"L11","title":"Peer evaluation guide","format":"Lead magnet","stage":"LOFU","idea":"Guide for
hospital executives to ask a reference about transition burden, support, and continuing
development.","CTA":"Get the guide","pageId":"P09","recommendedChannel":"Website and founder
introduction"},
  {"ID":"L12","title":"Buyer-approved measured case
study","format":"Proof","stage":"LOFU","idea":"Publish only after customer permission and audited
baseline, method, result, and limitations.","CTA":"Read the case
study","pageId":"P03","recommendedChannel":"Website and customer-approved sharing"},
  {"ID":"L13","title":"Reference conversation","format":"Proof","stage":"LOFU","idea":"Offer a
customer reference only after that customer's explicit agreement, with scope and availability
confirmed.","CTA":"Request a reference","pageId":"P09","recommendedChannel":"Private founder
introduction"},
  {"ID":"L14","title":"Share the method, not a claim","format":"Email","stage":"LOFU","idea":"Ask a
satisfied authorized customer whether they would review a draft factual pathway story.","CTA":"Review
the story request","pageId":"P09","recommendedChannel":"Customer email"},
  {"ID":"L15","title":"A peer may have this problem","format":"Email","stage":"LOFU","idea":"After
evidence and consent, offer a short forwardable note about the measured pathway and its
limits.","CTA":"Share the approved case study","pageId":"P03","recommendedChannel":"Customer email"},
  {"ID":"L16","title":"Peer operator exchange","format":"Community","stage":"LOFU","idea":"With
permission, convene an existing customer and one peer operator to discuss implementation
lessons.","CTA":"Request a reference","pageId":"P09","recommendedChannel":"Private operator
introduction"},
  {"ID":"L17","title":"Hospital-association learning
session","format":"Community","stage":"LOFU","idea":"Invite a permissioned customer to teach the
pathway method; verify event and affiliation before promotion.","CTA":"See approved
stories","pageId":"P10","recommendedChannel":"Professional association, if arranged"}
]
```

Source: Original files/Marketing workspace/prompt-library/complete-edition/working/audit-content.mjs. Historical wording preserved; not a new approval or factual revalidation.

audit-content.mjs

Technical appendix. This is an archived working file, not instructions to execute.

```
import fs from 'node:fs';
import path from 'node:path';
const root=path.resolve(import.meta.dirname,'..');
const manifest=JSON.parse(fs.readFileSync(path.join(root,'working/manifest.json'),'utf8'));
const errors=[];
const warnings=[];
const rows=[];
const words=s=>s.trim().split(/\s+/).filter(Boolean).length;
const load=file=>fs.existsSync(path.join(root,'outputs',file))?
fs.readFileSync(path.join(root,'outputs',file),'utf8'):'';
for(const m of manifest){
  const text=load(m.file);
  if(!text){errors.push(m.id+': missing output');continue;}
  const headings=[...text.matchAll(/^## ((?:Step|Part|Section)\s+\d+[^\n]*)/gm)].map(x=>x[1]);
  if(headings.length!==m.sections)errors.push(`${m.id}: expected ${m.sections} top-level required
sections, found ${headings.length}`);
  if(m.id.startsWith('p1-')||m.id.startsWith('l0-')){
    const nums=headings.map(h=>Number(h.match(/\d+/)[0]));
    if(nums.some((n,i)=>n!==i+1))errors.push(m.id+': section sequence wrong');
  }
  if(/turn\d+(?:search|view)|cite|\b(?:TODO|TBD)\b|lorem ipsum/i.test(text))errors.push(m.id+':
unresolved artifact token');
  const placeholders=[...text.matchAll(/\[PLACEHOLDER[^\]]*\]/g)].map(x=>x[0]);
  rows.push({...m,words:words(text),headings,placeholderCount:placeholders.length});
}
function section(text,kind,num){const re=new RegExp('^## '+kind+' '+num+'[^\\n]*\\n([\\s\\S]*?)(?=^##
|$(?![\\s\\S]))','m');return text.match(re)?.[1]||'';}
function assertCount(n,want,label){if(n!==want)errors.push(`${label}: ${n}, expected ${want}`);}
const uvps=load('p1-04-uvps.md');
if(uvps)assertCount((section(uvps,'Step',3).match(/^\d+\. /gm)||[]).length,10,'10 UVPs');
const interview=load('p1-07-user-research.md');
if(interview){
  assertCount((section(interview,'Step',7).match(/^\d+\. /gm)||[]).length,15,'Tailored interview
questions');
  assertCount((section(interview,'Step',8).match(/^\*\*(?:LinkedIn message|Email|Community\/forum
message)\./gm)||[]).length,9,'Outreach templates');
}
const avatar=load('p1-09-avatar.md');
if(avatar){const quote=section(avatar,'Step',3).split('\n').filter(l=>l.startsWith('>')).join('
').replace(/>\s*/g,'');const n=words(quote);if(n<150||n>200)errors.push(`Avatar monologue ${n} words,
expected150-200`);}
const description=load('p1-02-mechanics-jtbd.md');
if(description){const q=section(description,'Step',6).split('\n').filter(l=>l.startsWith('>')).join('
').replace(/>\s*/g,'');if(words(q)>150)errors.push('Product description exceeds150words');}
const campaign=load('p2-03b-campaign-distribution.md');
if(campaign){
  assertCount((campaign.match(/^### Email [1-4] /gm)||[]).length,4,'Campaign emails');
  const postRows=section(campaign,'Part',3).split('\n').filter(l=>/^\| \*\*\d+ \//.test(l));
  assertCount(postRows.length,10,'Full campaign posts');
  const days=postRows.map(l=>Number(l.match(/\d+/)[0]));
  if(JSON.stringify(days)!==JSON.stringify([1,2,4,5,7,8,9,11,12,14]))errors.push('Campaign post days
differ from source prompt');
  const emailChunks=campaign.split(/^### Email /m).slice(1);
  for(const chunk of emailChunks){
    const num=Number(chunk[0]);
    const subject=chunk.match(/\*\*Subject:\*\* ([^\n]+)/)?.[1]||'';
```

```
    const preview=chunk.match(/\*\*Preview:\*\* ([^\n]+)/)?.[1].trim()||'';
    if(words(subject)>8)errors.push(`Email${num} subject over8words`);
    if(preview.length>40)errors.push(`Email${num} preview over40characters`);
    const body=chunk.split('**Full body:**')[1]?.split('**CTA link text:**')[0].split('**Soft CTA link
text:**')[0]||'';
    const clean=body.replace(/\[([^\]]+)\]\([^)]*\)/g,'$1').replace(/[*_]/g,'');
    if(words(clean)>(num===4?100:150))errors.push(`Email${num} body overlimit:${words(clean)}`);
  }
}
const traffic=load('p2-05-traffic.md');
if(traffic)assertCount(section(traffic,'Part',1).split('\n').filter(l=>l.startsWith('|')&&!/^\|[-
]/.test(l.replace(/^\| /,'|'))).length-1,36,'Traffic channel rows');
const reporting=load('p2-07-reporting.md');
if(reporting){
  assertCount((section(reporting,'Part',1).match(/^### Section [1-3] /gm)||[]).length,3,'Weekly report
sections');
  assertCount((section(reporting,'Part',4).match(/^\| \d+[^|]*\|/gm)||[]).length,13,'North star
weeks');
}
for(const [file,headline,cta] of [['l0-01-demo.md',1,4],['l0-02-webinar.md',1,4],['l0-03-
trial.md',1,3]]){
  const t=load(file);if(!t)continue;
  for(const n of [headline,cta])assertCount((section(t,'Section',n).match(/^\d+\. /gm)||
[]).length,3,file+' section '+n+' alternatives');
}
const catalogPath=path.join(root,'working/content-catalog.json');
if(fs.existsSync(catalogPath)){
 const c=JSON.parse(fs.readFileSync(catalogPath,'utf8'));
 assertCount(c.length,85,'Content ideas');assertCount(new Set(c.map(x=>x.ID)).size,85,'Unique content
IDs');
 for(const st of
['TOFU','MOFU','BOFU','AFU','LOFU'])assertCount(c.filter(x=>x.stage===st).length,17,st+' ideas');
 for(const file of ['p2-01-funnel.md','p2-04-funnel-pages.md','p2-05-traffic.md','p2-06-tracking.md'])
{
   const t=load(file);if(!t)continue;
   const missing=c.filter(x=>!new RegExp('\\b'+x.ID+'\\b').test(t)).map(x=>x.ID);
   if(missing.length)errors.push(file+': missing IDs '+missing.join(','));
 }
}
for(const r of rows){if(r.placeholderCount)warnings.push(`${r.id}: ${r.placeholderCount} explicitly
marked evidence or launch placeholders`);}
const result=
{complete:errors.length===0,outputs:rows.length,totalWords:rows.reduce((s,r)=>s+r.words,0),requiredSec
tions:rows.reduce((s,r)=>s+r.sections,0),errors,warnings,rows};
fs.writeFileSync(path.join(root,'working/content-audit.json'),JSON.stringify(result,null,2));
console.log(JSON.stringify({outputs:result.outputs,words:result.totalWords,errors,warnings},null,2));
if(errors.length)process.exitCode=1;
```

Source: Original files/Marketing workspace/prompt-library/complete-edition/working/brand-theme.mjs. Historical wording preserved; not a new approval or factual revalidation.

brand-theme.mjs

Technical appendix. This is an archived working file, not instructions to execute.

```
import fs from 'node:fs';

const site='/Users/toko/Desktop/MediMind Landing Page';
const svgData=file=>''+fs.readFileSync(`${site}/public/deck-
assets/${file}`).toString('base64');
const darkLogo=svgData('logo-horizontal-dark.svg');
const lightLogo=svgData('logo-horizontal-light.svg');
const icon=svgData('logo-icon-light.svg');
const
fontData=file=>'data:font/woff2; base64,'+fs.readFileSync(`${site}/node_modules/${file}`).toString('bas
e64');

// Use the site's approved palette, keeping the existing responsive reading layout.
export function brandTheme(layout){
  const palette={
    '#172f37':'#1f2937','#576c71':'#475569','#fffefaee':'#fffffed','#fffefa':'#ffffff',
    '#d9e3df':'#e2e8f0','#d9f3e4':'#e7f2fc','#175843':'#2b6cb0','#b04e24':'#3182ce',
    '#142f38':'#1a365d','#ffba82':'#bee3f8','#f4f5f0':'#f8fafc','#14644e':'#2b6cb0',
    '#9b4623':'#1a365d','#d56b30':'#3182ce','#e5f2ee':'#f1f5f9','#b6ccc6':'#bee3f8',
    '#bdd6ca':'#bee3f8','#e5eeea':'#e2e8f0','#24434a':'#234875','#a9c2ba':'#b8cbe0',
    '#45605d':'#35577f','#dbf2e5':'#e1effa','#eef2ed':'#f1f5f9','#dce5df':'#cbd5e1',
    '#bccdc5':'#cbd5e1','#bce4cd':'#bee3f8','#deebe5':'#e2e8f0','#bdd3cb':'#cbd5e1',
    '#4b686b':'#456990','#ffcca5':'#bee3f8','#b9cdc5':'#cbd5e1','#e7eee6':'#edf2f9',
    '#eaf4ed':'#edf5fc','#a54723':'#2b6cb0','#507961':'#2b6cb0','#b8d6c4':'#3182ce',
    '#edf4ed':'#edf5fc','#254d3a':'#1a365d','#edf0eb':'#edf2f9','#e9eeea':'#f1f5f9',
    '#edf2ea':'#edf2f9','#174735':'#1a365d','#fbfcf7':'#f8fafc','#f3f5ed':'#eef2f7',
    '#41634d':'#2b6cb0','#1a5540':'#1a365d','#e3f0e2':'#e7f2fc','#315938':'#1a365d',
    '#e7eee3':'#edf2f9','#dfeadc':'#e2edf9','#152824':'#1f2937','#f1f4ec':'#f1f5f9',
    '#14392f':'#1a365d','#254a3a':'#1a365d','#945024':'#2b6cb0'
  };
  const themed=layout.replace(/#[0-9a-f]{6,8}\b/gi,value=>palette[value.toLowerCase()]||value)
    .replaceAll('Georgia,serif','"Fraunces Variable",Georgia,serif')
    .replaceAll('ui-monospace,monospace','"IBM Plex Mono",ui-monospace,monospace');
  return `
/* Embedded font licenses:
${fs.readFileSync(`${site}/node_modules/@fontsource-variable/fraunces/LICENSE`,'utf8')}
${fs.readFileSync(`${site}/node_modules/@fontsource/ibm-plex-mono/LICENSE`,'utf8')}
*/
@font-face{font-family:"Fraunces Variable";font-style:normal;font-weight:100 900;font-
display:swap;src:url("${fontData('@fontsource-variable/fraunces/files/fraunces-latin-wght-
normal.woff2')}") format("woff2")}
@font-face{font-family:"IBM Plex Mono";font-style:normal;font-weight:400;font-
display:swap;src:url("${fontData('@fontsource/ibm-plex-mono/files/ibm-plex-mono-latin-400-
normal.woff2')}") format("woff2")}
${themed}
:root{--emr-primary:#1a365d;--emr-secondary:#2b6cb0;--emr-accent:#3182ce;--emr-light-accent:#bee3f8;--
emr-gradient-primary:linear-gradient(135deg,#1a365d 0%,#2b6cb0 50%,#3182ce 100%)}
.brand{display:block;line-height:1}.brand img{display:block;width:215px;max-
width:100%;height:auto}.sidetitle{font-family:"IBM Plex Mono",monospace;font-size:10px;letter-
spacing:1.8px;margin-top:15px}.sidebar{background:linear-gradient(180deg,#1a365d,#122b4b)}
.cover{background:radial-gradient(ellipse at 100% 0%,rgba(49,130,206,.36),transparent 65%),linear-
gradient(135deg,#0f1f3a 0%,#1a365d 58%,#2b6cb0 100%);border:1px solid #2b6cb0;box-shadow:0 18px 45px
-30px rgba(26,54,93,.65)}
.cover::before{content:"";position:absolute;inset:0;pointer-events:none;background-image:linear-
gradient(rgba(190,227,248,.035) 1px,transparent 1px),linear-gradient(90deg,rgba(190,227,248,.035)
1px,transparent 1px);background-size:52px 52px}
```

```
.cover>*{position:relative;z-index:1}.cover-brand{margin:0 0 32px}.cover-brand
img{display:block;width:255px;max-width:75%;height:auto}.cover-brand .logo-print{display:none}.cover
h1{font-weight:450;letter-spacing:-1.8px}.cover h1 em{color:#bee3f8}.eyebrow,.chapter-label,.card
small,.navgroup,.stamp{font-family:"IBM Plex Mono",monospace}.eyebrow{font-weight:400;letter-
spacing:1.5px}.stat strong{color:#bee3f8;font-variant-numeric:lining-nums}.card{transition:border-
color .15s ease,box-shadow .15s ease}.card:hover{box-shadow:0 5px 18px -12px rgba(49,130,206,.6)}
.chapter h1,.chapter h2,.directory-title,.launch-guide h2{color:#1a365d}.chapter-tools button:first-
child,#print{background:var(--emr-gradient-primary);border-color:#2b6cb0;color:white;box-shadow:inset
0 0 0 1px rgba(190,227,248,.15)}.chapter-tools button:first-
child:hover,#print:hover{background:#2b6cb0}.readingbar{background:var(--emr-gradient-
primary)}.launch-guide{margin-top:28px;border:1px solid #cbd5e1;background:white}.footer{border-
top:1px solid #e2e8f0;font-family:"IBM Plex Mono",monospace;font-size:10px;letter-
spacing:.5px}.navmeta{font-family:"IBM Plex Mono",monospace;font-size:10px}
@media(max-width:800px){.cover-brand{margin-bottom:24px}.cover-brand img{width:205px;max-
width:88%}.cover h1{font-size:40px;line-height:1.13;letter-spacing:-1.1px}.cover .eyebrow{font-
size:9px;letter-spacing:1px}.stat span{font-size:9px}.brand img{width:210px}}
@media(max-width:360px){.cover{padding:27px 20px}.cover h1{font-size:34px}.stats{grid-template-
columns:1.2fr 1fr 1fr;gap:7px}.stat strong{font-size:24px}}
@media(prefers-reduced-motion:reduce){html{scroll-behavior:auto}.card{transition:none}}
@media print{.cover{background:#f1f5f9!important;box-
shadow:none;border:0}.cover::before{display:none}.cover-brand .logo-screen{display:none}.cover-brand
.logo-print{display:block}.cover h1,.cover h1 em,.cover p,.cover .eyebrow,.cover .stamp,.cover .stat
strong,.cover .stat span{color:#1a365d}.cover h1{font-size:40pt}}
`;
}

export function brandMarkup(html){
  return html
    .replace('</head>',`<meta name="theme-color" content="#1a365d"><link rel="icon"
type="image/svg+xml" href="${icon}"></head>`)
    .replace(/(<a class="brand"[^>]*>)[\s\S]*?<\/a>/,`$1<img src="${darkLogo}" alt="MediMind"
width="320" height="80"></a>`)
    .replace('<div class="cover">',`<div class="cover"><div class="cover-brand"><img class="logo-
screen" src="${darkLogo}" alt="MediMind" width="320" height="80"><img class="logo-print"
src="${lightLogo}" alt="MediMind" width="320" height="80"></div>`)
    .replace('The marketing working library','Marketing playbook')
    .replace('MediMind × B11 / Complete working edition','B11 / MARKETING PLAYBOOK')
    .replace('Every prompt, answered separately. A connected marketing library built around MediMind’s
hospital operating system, its buyers, its current evidence and its next stage.','Research,
positioning, campaigns and conversion copy for MediMind’s next stage of growth.')
    .replace(/<strong>\d+k<\/strong><span>WORDS OF WORKING MATERIAL<\/span>/,'<strong>138</strong>
<span>MARKETING SECTIONS</span>')
    .replace(/<span class="statuspill">Working output<\/span>/g,'')
    .replace(/<div class="navmeta">[\s\S]*?<\/div>/,'<div class="navmeta">MediMind · B11<br>September
2026</div>')
    .replace(/<footer class="footer">[\s\S]*?<\/footer>/,'<footer class="footer">MediMind · B11
Marketing Playbook · September 2026</footer>');
}
```

Source: Original files/Marketing workspace/prompt-library/complete-edition/working/build-book.mjs. Historical wording preserved; not a new approval or factual revalidation.

build-book.mjs

Technical appendix. This is an archived working file, not instructions to execute.

```
import fs from 'node:fs';
import path from 'node:path';
import { marked } from '/Users/toko/.cache/codex-runtimes/codex-primary-
runtime/dependencies/node/node_modules/marked/lib/marked.esm.js';
import { brandTheme, brandMarkup } from './brand-theme.mjs';

const root = path.resolve(import.meta.dirname, '..');
const deliver = '/Users/toko/Desktop/MediMind - Complete B11 Marketing Library';
const manifest = JSON.parse(fs.readFileSync(path.join(root, 'working/manifest.json'), 'utf8'));
const preview = process.argv.includes('--preview');
const esc = x =>
String(x).replaceAll('&','&amp;').replaceAll('<','&lt;').replaceAll('>','&gt;').replaceAll('"','&quot;
');
const slug = x => x.toLowerCase().replace(/[^a-z0-9]+/g,'-').replace(/^-|-$/g,'');
marked.setOptions({ gfm:true, breaks:false });
fs.mkdirSync(path.join(deliver, 'Individual outputs'), {recursive:true});
fs.mkdirSync(path.join(deliver, 'Editable text'), {recursive:true});
fs.mkdirSync(path.join(deliver, 'Source libraries'), {recursive:true});
fs.mkdirSync(path.join(deliver, 'Source notes'), {recursive:true});

// Curated local evidence snapshots make the reading folder portable without shipping product code.
const evidenceMap = new Map();
const registerEvidence = (source, name, transform = s => s) => {
  const absolute = path.resolve(source);
  if (!fs.existsSync(absolute)) throw new Error(`Missing evidence source: ${absolute}`);
  fs.writeFileSync(path.join(deliver, 'Source notes', name), transform(fs.readFileSync(absolute,
'utf8')));
  evidenceMap.set(absolute, 'Source notes/' + name);
};
const marketingRoot = path.resolve(root, '../..');
for (const file of ['2026-09-22-evidence-register.md','2026-09-22-hospital-buyer-pain-points.md']) {
  registerEvidence(path.join(marketingRoot, 'research', file), file);
}
registerEvidence(path.join(marketingRoot, 'copy/2026-09-22-live-hero-working-draft.md'), 'Current hero
snapshot.md');
for (const file of ['00-what-we-are.md','03-scope.md','04-language.md','06-buyers.md']) {
  registerEvidence('/Users/toko/Desktop/medplum_medimind/positioning/' + file, 'Product positioning -
' + file);
}
registerEvidence(path.join(root, 'working/research-addendum.md'), 'Research and source inspection.md',
s => s.replace('# Orchestrator research addendum', '# Research and source inspection').replace('Use
alongside writer-brief.md.', 'Internal source snapshot; not a live-site test.'));
registerEvidence(path.join(root, 'working/writer-brief.md'), 'Company inputs and editorial
assumptions.md', s => '# Company inputs and editorial assumptions\n\nEvidence snapshot: 22 September
2026. These are supplied facts and proposed working decisions, not approved commercial
commitments.\n\n' + s.split('## Shared factual baseline and boundaries')[1].split('## Sources to
read')[0]);
registerEvidence(path.join(root, 'working/content-catalog.json'), '85-item content catalog.json');
for (const source of
['src/components/Demo/RequestDemo.tsx','src/components/Landing/LampHeroSection.tsx']) {
  evidenceMap.set(path.resolve(marketingRoot, '..', source), 'Source notes/Research and source
inspection.md');
}
evidenceMap.set(path.join(root, 'working/main-library.txt'), 'Source libraries/Complete Library -
Parts 1 and 2.pdf');
evidenceMap.set(path.join(root, 'working/pages-library.txt'), 'Source libraries/Library 0 - Conversion
```

```
Pages.pdf');

const available = manifest.filter(m => fs.existsSync(path.join(root,'outputs',m.file)));
if (!preview && available.length !== 20) throw new Error(`Only ${available.length}/20 outputs
exist.`);
const chapterData = available.map(m => {
  const raw = fs.readFileSync(path.join(root,'outputs',m.file),'utf8');
  const words = raw.replace(/https?:\/\/\S+/g,'').split(/\s+/).filter(Boolean).length;
  return {...m,raw,words};
});
const totalWords = chapterData.reduce((s,m)=>s+m.words,0);
const baseCss = `
:root{--ink:#172f37;--muted:#576c71;--paper:#fffefa;--line:#d9e3df;--mint:#d9f3e4;--green:#175843;--
amber:#b04e24;--navy:#142f38;--orange:#ffba82}
*{box-sizing:border-box}html{scroll-behavior:smooth;scroll-padding-
top:90px}body{margin:0;background:#f4f5f0;color:var(--ink);font:16px/1.7 -apple-
system,BlinkMacSystemFont,'Segoe UI',sans-serif}a{color:#14644e;text-decoration-thickness:1px;text-
underline-offset:3px}a:hover{color:#9b4623}button,input{font:inherit}button,a{touch-
action:manipulation}button:focus-visible,a:focus-visible,input:focus-visible{outline:3px solid
#d56b30;outline-offset:3px}button{cursor:pointer}.skip{position:fixed;left:12px;top:-90px;z-
index:20;padding:12px;background:white}.skip:focus{top:8px}
.sidebar{position:fixed;inset:0 auto 0 0;width:292px;background:var(--navy);color:#e5f2ee;padding:30px
22px;overflow:auto;z-index:5}.brand{font-size:22px;font-weight:750;letter-
spacing:-.8px;color:white;text-decoration:none}.brand span{color:var(--orange)}.sidetitle{font-
size:11px;letter-spacing:1.8px;text-transform:uppercase;color:#b6ccc6;margin:7px 0
25px}.navgroup{font-size:10px;letter-spacing:1.2px;text-transform:uppercase;color:#bdd6ca;margin:25px
0 10px}.navitem{display:flex;align-items:flex-start;gap:11px;color:#e5eeea;text-
decoration:none;padding:9px 7px;border-radius:6px;font-size:12px;line-
height:1.45}.navitem:hover{background:#24434a;color:white}.navitem b{font:11px/1.9 ui-
monospace,monospace;color:var(--orange);min-width:30px}.navmeta{font-size:11px;color:#a9c2ba;margin-
top:28px;border-top:1px solid #45605d;padding-top:18px}.sidebar .overviewlink{color:#dbf2e5;font-
size:13px}
.layout{margin-left:292px}.toolbar{position:sticky;top:0;z-index:4;display:flex;align-
items:center;gap:10px;padding:12px 36px;background:#fffefaee;backdrop-filter:blur(12px);border-
bottom:1px solid var(--line)}.toolbar input{flex:1;min-width:100px;background:#eef2ed;border:1px solid
#dce5df;border-radius:7px;padding:9px 12px;font-size:13px}.toolbar button,.smallbtn{border:1px solid
#bccdc5;background:white;border-radius:6px;padding:9px 13px;font-size:12px;color:var(--ink);white-
space:nowrap}.toolbar button:hover,.smallbtn:hover{background:var(--mint)}.searchstatus{font-
size:11px;color:var(--
muted)}.mobilemenu{display:none}.readingbar{position:fixed;height:3px;background:var(--
amber);top:0;left:292px;width:0;z-index:10}
main{max-width:1240px;margin:auto;padding:36px 44px 80px}.cover{padding:58px 54px;background:var(--
navy);color:white;border-radius:12px;position:relative;overflow:hidden}.eyebrow{font-size:11px;font-
weight:700;letter-spacing:2px;text-transform:uppercase;color:#bce4cd}.cover h1{font-
family:Georgia,serif;font-size:clamp(40px,4.3vw,68px);line-height:1.06;letter-spacing:-2px;font-
weight:400;margin:21px 0 24px;max-width:720px}.cover h1 em{color:var(--orange);font-
style:normal}.cover p{max-width:720px;color:#deebe5;font-size:17px;line-height:1.65}.cover
.stamp{font:11px/1.5 ui-monospace,monospace;color:#bdd3cb;margin-top:35px}.stats{display:grid;grid-
template-columns:repeat(3,1fr);border-top:1px solid #4b686b;margin-top:32px;padding-
top:24px;gap:20px}.stat strong{font-family:Georgia,serif;font-size:35px;font-
weight:400;color:#ffcca5}.stat span{display:block;font-size:11px;color:#b9cdc5;letter-spacing:.5px}
.intro{padding:30px 5px 14px}.intro p{max-width:85ch}.intro .note{background:#e7eee6;border-left:4px
solid var(--green);padding:15px 20px;margin:24px 0}.launch{display:grid;grid-template-columns:1fr
1fr;gap:18px;margin:26px 0}.launch>div{border:1px solid var(--line);border-
radius:8px;background:#fffefa;padding:20px}.launch h3{margin:0 0 8px;font-size:15px}.launch p{font-
size:13px;margin:0;color:var(--muted)}.directory{display:grid;grid-template-
columns:repeat(2,minmax(0,1fr));gap:10px}.card{display:block;padding:17px 19px;background:var(--
paper);border:1px solid var(--line);border-radius:8px;text-decoration:none;color:var(--
ink)}.card:hover{border-color:var(--green);background:#eaf4ed}.card small{display:block;font-
size:10px;letter-spacing:1px;text-transform:uppercase;color:var(--green);margin-bottom:6px}.card
strong{display:block;font-size:15px;line-height:1.4}.card .count{font-size:11px;color:var(--
muted);margin-top:7px}.directory-title{font:30px/1.2 Georgia,serif;margin:45px 0 20px}
.chapter{background:var(--paper);border:1px solid var(--line);border-radius:10px;margin:50px
0;padding:40px 42px;scroll-margin-top:88px;min-width:0}.chapter[hidden]
{display:none!important}.chapter-label{display:flex;flex-wrap:wrap;align-items:center;gap:12px;font-
size:11px;letter-spacing:1.3px;text-transform:uppercase;color:var(--green);font-weight:700}.chapter-
label .number{font:27px Georgia,serif;color:#a54723}.chapter h1{font:40px/1.18 Georgia,serif;letter-
spacing:-.7px;margin:20px 0 15px;max-width:24ch}.chapter h2{font:29px/1.25 Georgia,serif;font-
weight:400;margin:43px 0 18px;padding-top:27px;border-top:1px solid var(--line);scroll-margin-
```

```
top:92px}.chapter h3{font-size:19px;line-height:1.4;margin:29px 0 13px}.chapter h4{font-
size:15px;margin:24px 0 10px}.chapter p{margin:0 0 17px;max-width:92ch}.chapter ul,.chapter
ol{padding-left:23px;margin:10px 0 23px}.chapter li{padding-left:5px;margin:7px 0}.chapter ul
li::marker{color:#507961}.chapter strong{font-weight:650}.chapter blockquote{border-left:3px solid
#b8d6c4;padding:10px 19px;margin:24px 0;background:#edf4ed;color:#254d3a}.chapter blockquote p:last-
child{margin:0}.chapter code{background:#edf0eb;border-radius:3px;padding:2px 4px;font:12px ui-
monospace,monospace;overflow-wrap:anywhere}.chapter
pre{overflow:auto;background:#e9eeea;padding:18px;border-radius:7px;font-size:12px}.chapter pre
code{background:transparent;padding:0;white-space:pre}.chapter a{overflow-wrap:anywhere}.table-
wrap{overflow-x:auto;margin:23px 0;border:1px solid var(--line);border-radius:6px}.chapter
table{border-collapse:collapse;width:100%;font-size:12.5px;line-height:1.55;min-width:560px}.chapter
th{background:#edf2ea;text-align:left;color:#174735;font-size:11px;font-
weight:750;position:relative}.chapter th,.chapter td{padding:12px 13px;vertical-align:top;border-
bottom:1px solid var(--line);min-width:90px;overflow-wrap:anywhere}.chapter tr:last-child td{border-
bottom:0}.chapter tr:nth-child(even) td{background:#fbfcf7}.chapter td:first-child{font-
weight:560}.chapter table ul{margin:0;padding-left:15px}.chapter-tools{display:flex;flex-
wrap:wrap;gap:9px;align-items:center;padding:10px 0 22px;border-bottom:1px solid var(--line);font-
size:11px}.chapter-tools a{font-size:11px}.sourcehint{color:var(--muted);font-size:11px;margin:12px
0}.outline{background:#f3f5ed;border-radius:6px;padding:13px 17px;margin:18px 0 27px;font-
size:12px}.outline summary{cursor:pointer;font-weight:650;color:var(--green)}.outline
ul{columns:2;padding-left:19px;margin:10px 0 0}.outline li{break-inside:avoid;margin:5px 0}.outline
a{text-decoration:none;color:#41634d}.chapter hr{border:0;border-top:1px solid var(--line);margin:28px
0}.bottomnav{display:flex;justify-content:space-between;gap:20px;border-top:1px solid var(--
line);margin-top:32px;padding-top:18px;font-size:12px}.empty{padding:50px;text-
align:center;background:white;border-radius:10px}.footer{font-size:12px;color:var(--
muted);padding:24px 5px}.toast{position:fixed;bottom:22px;right:25px;padding:12px
20px;background:#1a5540;color:white;border-radius:7px;z-index:30;font-
size:13px}.statuspill{display:inline-block;padding:3px 8px;background:#e3f0e2;border-radius:20px;font-
size:10px;letter-spacing:.3px;color:#315938}
.chapter table.tracking-links{min-width:1050px;table-layout:fixed}.tracking-links th:nth-child(1)
{width:20%}.tracking-links th:nth-child(2){width:12%}.tracking-links th:nth-child(3)
{width:50%}.tracking-links th:nth-child(4){width:18%}.table-wrap[data-
overflow="true"]::before{content:"Scroll sideways to see all columns
→";display:block;position:sticky;left:0;font-size:10px;letter-spacing:.3px;color:var(--
green);background:#e7eee3;padding:7px 12px}
.launch-guide ul,.launch-guide ol{columns:1}.launch-guide h2{font:25px/1.3 Georgia,serif;margin:30px 0
15px}.launch-guide table{border-collapse:collapse;width:100%;min-width:520px;font-size:13px}.launch-
guide th,.launch-guide td{border-bottom:1px solid var(--line);text-align:left;vertical-
align:top;padding:12px}.launch-guide th{background:#dfeadc}.launch-guide .table-wrap{max-width:100%}
@media print{.chapter table.tracking-links{min-width:0;width:100%;font-size:7pt}.table-wrap[data-
overflow="true"]::before{display:none}}
@media(min-width:1500px){main{padding-left:65px;padding-right:65px}.chapter{padding:50px 58px}}
@media(max-width:1100px){.sidebar{width:244px;padding:25px 16px}.layout{margin-
left:244px}.readingbar{left:244px}main{padding:25px}.chapter{padding:29px}.cover{padding:37px}.toolbar
{padding:10px 25px}.outline ul{columns:1}}
@media(max-width:800px){.sidebar{display:none;width:280px;top:60px;z-
index:8}.sidebar.open{display:block}.layout{margin-
left:0}.readingbar{left:0}.mobilemenu{display:block!important}.toolbar{padding:10px;gap:6px}.toolbar
.searchstatus{display:none}main{padding:16px}.cover{padding:31px 25px}.cover h1{font-size:43px;letter-
spacing:-1px}.cover p{font-size:15px}.stats{gap:10px}.stat strong{font-size:29px}.directory{grid-
template-columns:1fr}.launch{grid-template-columns:1fr}.chapter{padding:25px 20px;margin:28px
0}.chapter h1{font-size:32px}.chapter h2{font-size:25px}.chapter h3{font-size:18px}.chapter
table{font-size:12px}.chapter th,.chapter td{padding:10px}.outline ul{columns:1}.toolbar input{font-
size:12px;padding:8px}.toolbar button{padding:8px;font-size:11px}.chapter-
tools{gap:7px}.smallbtn{padding:7px 9px}.bottomnav{flex-direction:column}.searchstatus{display:none}}
@media print{@page{size:A4;margin:16mm 13mm 19mm}.sidebar,.toolbar,.readingbar,.chapter-
tools,.outline,.directory,.directory-
title,.launch,.bottomnav,.toast,.skip,.searchstatus{display:none!important}.layout{margin:0}body{backg
round:white;color:#152824;font-size:10pt;line-height:1.5}main{padding:0;max-width:none}.cover{border-
radius:0;padding:20mm 12mm;background:#f1f4ec!important;color:#14392f;break-after:page}.cover h1{font-
size:42pt}.cover h1 em,.cover p,.cover .eyebrow,.cover .stamp,.stat span{color:#254a3a}.stat
strong{color:#945024}.intro{break-after:page}.chapter{border:0;border-
radius:0;padding:0;margin:0;break-before:page}.chapter h1{font-size:27pt}.chapter h2{font-
size:19pt;padding-top:14px;margin-top:25px;break-after:avoid}.chapter h3,.chapter h4{break-
after:avoid}.chapter p{orphans:3;widows:3}.table-wrap{overflow:visible;border:0;margin:12px 0}.chapter
table{min-width:0;table-layout:fixed;font-size:7pt}.chapter th,.chapter td{min-
width:0;padding:5px;overflow-wrap:anywhere}.chapter tr{break-inside:avoid}.chapter
thead{display:table-header-group}.chapter blockquote{break-inside:avoid}.chapter pre{white-space:pre-
wrap;overflow-wrap:anywhere}.footer{break-before:avoid}}
```

```
`;

const css=brandTheme(baseCss);

function rewriteLinks(content, single=false, sourceDir=path.join(root,'outputs')) {
  return content.replace(/href="([^"]+)"/g,(all,href)=>{
    if(/^(?:https?:|mailto:|#)/i.test(href))return all;
    if(href.startsWith('/')&&!href.startsWith('/Users/'))return
`href="https://medimind.md${esc(href)}" title="Campaign destination; see the chapter's launch
prerequisites"`;
    const match=manifest.find(x=>path.basename(decodeURIComponent(href.split('#')[0]))===x.file);
    if(match)return `href="${single?'':'Individual
outputs/'}${match.id}.html${href.includes('#')?'#'+href.split('#')[1]:''}"`;
    const decoded=decodeURIComponent(href).replaceAll('&amp;', '&');
    const resolved=decoded.startsWith('file:') ? new URL(decoded).pathname :
path.resolve(sourceDir,decoded);
    const bundled=evidenceMap.get(resolved);
    if(bundled)return `href="${single?'../':''}${esc(bundled)}"`;
    return `href="file://${esc(resolved)}"`;
  });
}

function renderChapter(m, index, single=false) {
  const headings=[];
  let content=marked.parse(m.raw.replace(/^# .*\r?\n/,''));
  let n=0;
  content=content.replace(/<h([234])>([\s\S]*?)<\/h\1>/g,(all,level,text)=>{
    const id=`${m.id}-s${++n}`;
    if(level==='2') headings.push({id,text:text.replace(/<[^>]*>/g,'')});
    return `<h${level} id="${id}">${text}</h${level}>`;
  });
  content=content.replace(/<table>([\s\S]*?)<\/table>/g,(_,inner)=>'<div class="table-wrap"
role="region" aria-label="Scrollable data table" tabindex="0"><table'+(inner.includes('UTM link
structure')?' class="tracking-links"':'')+'>'+inner+'</table></div>');
  // Resolve relative source links against the authoring file. Prompt-to-prompt links become portable
outputs.
  content=rewriteLinks(content,single);
  const prefix=single?'../':'';
  const pdf=m.id.startsWith('l0')?'Library 0 - Conversion Pages.pdf':'Complete Library - Parts 1 and
2.pdf';
  const pg=m.pages.split('-')[0];
  const prev=chapterData[index-1],next=chapterData[index+1];
  return `<article class="chapter" id="${m.id}" data-title="${esc(m.title)}">
    <div class="chapter-label"><span class="number">${String(index+1).padStart(2,'0')}
</span>${esc(m.label)} <span class="statuspill">Working output</span></div>
    <h1>${esc(m.title)}</h1>
    <div class="chapter-tools"><button class="smallbtn copy" data-id="${m.id}">Copy full
answer</button><button class="smallbtn printchapter" data-id="${m.id}">Print this answer</button><a
href="${prefix}Editable text/${m.file}" download>Editable text</a>${single?`<a href="../START
HERE.html#${m.id}">Back to complete library</a>`:`<a href="Individual outputs/${m.id}.html">Open
separately ↗</a>`}</div>
    <p class="sourcehint">Source: <a href="${prefix}Source
libraries/${pdf}#page=${pg}">${m.id.startsWith('l0')?'Library #0':'Complete library'}, pages
${m.pages}</a> · ${m.words.toLocaleString()} words · ${m.sections} requested steps / sections</p>
    <details class="outline"><summary>Sections in this answer</summary><ul>${headings.map(h=>`<li><a
href="#${h.id}">${h.text}</a></li>`).join('')}</ul></details>
    <div class="answer">${content}</div>
    <div class="bottomnav">${prev?`<a href="${single?prev.id+'.html':'#'+prev.id}">←
${esc(prev.label)}</a>`:'<span>Start of library</span>'}${next?`<a href="${single?
next.id+'.html':'#'+next.id}">${esc(next.label)} →</a>`:'<a href="'+(single?'../START
HERE.html':'#overview')+'">Back to overview ↑</a>'}</div>
  </article>`;
}

function shell(body, navItems, title, single=false, rawData=chapterData) {
  const nav=navItems.map((m,i)=>`${i===0||navItems[i-1].group!==m.group?`<div
class="navgroup">${esc(m.group)}</div>`:''}<a class="navitem" href="${single?m.id+'.html':'#'+m.id}"
data-for="${m.id}"><b>${m.id.replace('p1-','1.').replace('p2-','2.').replace('l0-','0.')}</b>
<span>${esc(m.title)}</span></a>`).join('');
```

```
  const scripts=`
const raw=${JSON.stringify(Object.fromEntries(rawData.map(m=>[m.id,m.raw]))).replace(/</g,'\\u003c')};
const allArticles=[...document.querySelectorAll('.chapter')];
const input=document.querySelector('#search');
function flagWideTables(){document.querySelectorAll('.table-wrap').forEach(w=>
{w.dataset.overflow=String(w.scrollWidth>w.clientWidth+2);});}
flagWideTables();window.addEventListener('resize',flagWideTables);document.querySelectorAll('details')
.forEach(d=>d.addEventListener('toggle',flagWideTables));
function reset()
{input.value='';document.querySelectorAll('[hidden]').forEach(e=>e.hidden=false);document.querySelecto
r('.searchstatus').textContent='';}
input.addEventListener('input',()=>{const q=input.value.trim().toLowerCase();let
count=0;allArticles.forEach(a=>
{a.hidden=q&&!a.textContent.toLowerCase().includes(q);if(!a.hidden)count++;});document.querySelectorAl
l('.navitem').forEach(a=>{const
target=document.getElementById(a.dataset.for);a.hidden=target?.hidden||false;});const
overview=document.querySelector('#overview');if(overview)overview.hidden=!!q;document.querySelector('#
noresults').hidden=count>0;document.querySelector('.searchstatus').textContent=q?count+' answers
found':'';});
document.querySelector('#clear').addEventListener('click',reset);
document.querySelector('#print').addEventListener('click',()=>window.print());
document.querySelector('#menu').addEventListener('click',()=>{const
s=document.querySelector('.sidebar');s.classList.toggle('open');document.querySelector('#menu').setAtt
ribute('aria-expanded',String(s.classList.contains('open')));});
document.querySelectorAll('.navitem').forEach(a=>a.addEventListener('click',
()=>document.querySelector('.sidebar').classList.remove('open')));
document.querySelectorAll('.copy').forEach(b=>b.addEventListener('click',async()=>{try{await
navigator.clipboard.writeText(raw[b.dataset.id]);b.textContent='Copied';setTimeout(()=>b.textContent='
Copy full answer',1800);}catch{const
ta=document.createElement('textarea');ta.value=raw[b.dataset.id];document.body.appendChild(ta);ta.sele
ct();const ok=document.execCommand('copy');ta.remove();b.textContent=ok?'Copied':'Select text to
copy';}}));
document.querySelectorAll('.printchapter').forEach(b=>b.addEventListener('click',()=>
{reset();allArticles.forEach(a=>a.hidden=a.id!==b.dataset.id);const
intro=document.querySelector('#overview');if(intro)intro.hidden=true;window.print();}));
window.addEventListener('afterprint',reset);
document.addEventListener('keydown',e=>{if((e.ctrlKey||e.metaKey)&&e.key==='k')
{e.preventDefault();input.focus();}});
window.addEventListener('scroll',()=>{const max=document.documentElement.scrollHeight-
innerHeight;const offset=innerWidth>1100?292:innerWidth>800?
244:0;document.querySelector('.readingbar').style.width=(max>0?scrollY/max*(innerWidth-
offset):0)+'px';},{passive:true});
`;
  return `<!doctype html><html lang="en"><head><meta charset="utf-8"><meta name="viewport"
content="width=device-width,initial-scale=1"><meta name="description" content="MediMind's complete B11
marketing prompt outputs: research, positioning, funnel, campaign assets, conversion copy, tracking
and reporting."><title>${esc(title)}</title><style>${css}</style></head><body><a href="#main"
class="skip">Skip to content</a><div class="readingbar"></div><aside class="sidebar"><a class="brand"
href="${single?'../START HERE.html':'#overview'}">MediMind<span> /</span></a><div
class="sidetitle">The marketing working library</div><a class="overviewlink" href="${single?'../START
HERE.html':'#overview'}">Overview & all 20 outputs</a><nav aria-label="Prompt navigation">${nav}</nav>
<div class="navmeta">B11 · Justyna Jastrzebska<br>Adapted to MediMind<br>22 September 2026<br>Strategy
and copy drafts</div></aside><div class="layout"><header class="toolbar"><button id="menu"
class="mobilemenu" aria-label="Toggle navigation" aria-expanded="false">Menu</button><input
id="search" type="search" placeholder="Search answers…  ⌘K" aria-label="Search complete answers"><span
class="searchstatus" aria-live="polite"></span><button id="clear">Show all</button><button
id="print">Print</button></header><main id="main">${body}<div id="noresults" class="empty" hidden>No
matching answers. Try “pricing”, “interview”, “email” or “tracking”.</div><footer
class="footer">Prepared for Lasha and the MediMind team. GPT-6 Sol writing coordinated and reviewed by
the parent agent. Source frameworks by Justyna Jastrzebska / 500 Eurasia. Local working documents;
execution and validation status is stated in each answer.</footer></main></div><script>${scripts}
</script></body></html>`;
}

const index=`<section id="overview"><div class="cover"><div class="eyebrow">MediMind × B11 / Complete
working edition</div><h1>From the buyer’s problem<br>to <em>the next customer.</em></h1><p>Every
prompt, answered separately. A connected marketing library built around MediMind’s hospital operating
system, its buyers, its current evidence and its next stage.</p><div class="stats"><div class="stat">
<strong>${available.length}/20</strong><span>SEPARATE PROMPT OUTPUTS</span></div><div class="stat">
```

```
<strong>${Math.round(totalWords/1000)}k</strong><span>WORDS OF WORKING MATERIAL</span></div><div
class="stat"><strong>2</strong><span>SOURCE PROMPT LIBRARIES</span></div></div><div class="stamp">22
SEPTEMBER 2026 · RESEARCH / STRATEGY / COPY / OPERATIONS</div></div><div class="intro"><p>The earlier
HTML contained only Part 1, Prompt 1. Its index explicitly left the remaining exercises pending. This
edition works through all nine research prompts, all eight funnel/campaign outputs (3A and 3B
separately), and the three conversion-page prompts.</p><div class="note"><strong>How to use this.
</strong> Start with the overview and read in order, or open any answer separately. Each answer
preserves its prompt’s requested steps and counts. Search across the book, copy the full answer, or
print a single answer. The editable text and original libraries travel with this folder.</div><div
class="launch"><div><h3>What is finished here</h3><p>The written research synthesis, positioning,
interview materials, funnel design, campaign scripts and copy, page copy, distribution, measurement
specifications and reporting templates. Each is a separate output you can use and revise.</p></div>
<div><h3>What still needs real-world evidence</h3><p>Local buyer interviews, approved customer
quotations and outcomes, exact offer terms, baseline funnel numbers, and scheduling/delivery details
for optional webinar and sandbox offers. Those fields are visible where needed.</p></div></div><p>
<strong>Immediate priority:</strong> verify and repair demo capture before campaign traffic. The
inspected source form displays a success state but does not submit the lead to a backend. This is a
source finding; deployed behavior and lost-lead volume have not been established.</p><p>
<strong>Working focus:</strong> Georgia first; private hospital CEOs and operators; a 25-minute
pathway demo as the recommended entry point. Marketing plans share one five-hour weekly budget.
Kazakhstan and Uzbekistan remain later expansion choices.</p></div><h2 class="directory-title">Your 20
outputs</h2><div class="directory">${chapterData.map((m,i)=>`<a class="card" href="#${m.id}">
<small>${String(i+1).padStart(2,'0')} / ${esc(m.label)}</small><strong>${esc(m.title)}</strong><div
class="count">${m.sections} requested steps / sections · ${m.words.toLocaleString()} words</div>
</a>`).join('')}</div></section>`;
const guidePath=path.join(root,'working/launch-guide.md');
const guide=fs.existsSync(guidePath)?fs.readFileSync(guidePath,'utf8'):'';
const guideContent=guide?rewriteLinks(marked.parse(guide.replace(/^#
.*\n/,'')),false,path.join(root,'working')).replaceAll('<table>','<div class="table-wrap">
<table>').replaceAll('</table>','</table></div>'):'';
const guideHtml=guide?`<details class="outline launch-guide" id="first-week"><summary>Start this week
- Lasha's practical five-hour guide</summary><div style="font-size:15px;line-height:1.75;max-
width:90ch">${guideContent}</div></details>`:'';
const cleanIndex=index.replace(/<div class="intro">[\s\S]*?(?=<h2 class="directory-title">)/,'');
const completeIndex=cleanIndex.replace('22 SEPTEMBER 2026 · RESEARCH','23 SEPTEMBER 2026 ·
RESEARCH').replace('<h2 class="directory-title">',guideHtml+'<h2 class="directory-title">');
fs.writeFileSync(path.join(deliver,'START
HERE.html'),brandMarkup(shell(completeIndex+chapterData.map((m,i)=>renderChapter(m,i)).join(''),chapte
rData,'MediMind - B11 Marketing Playbook')));
if(guide)fs.writeFileSync(path.join(deliver,'Editable text','First-week launch
guide.md'),guide.replaceAll('../outputs/','').replace('](content-catalog.json)',](<../Source
notes/85-item content catalog.json>)'));
for(const [i,m] of chapterData.entries()){
  fs.writeFileSync(path.join(deliver,'Individual
outputs',m.id+'.html'),brandMarkup(shell(renderChapter(m,i,true),chapterData,m.title+' -
MediMind',true,[m])));
  const editable=m.raw.replace(/\]\((?:<([^>]+)>|([^)]*))\)/g,(all,angle,plain)=>{
    const href=angle||plain;
    if(/^(?:https?:|mailto:|#)/i.test(href))return all;
    if(href.startsWith('/')&&!href.startsWith('/Users/'))return `](https://medimind.md${href})`;
    const matching=manifest.find(x=>path.basename(href)===x.file);
    if(matching)return `](${matching.file})`;
    const resolved=path.resolve(root,'outputs',href);
    const bundled=evidenceMap.get(resolved);
    return bundled?`](<../${bundled}>)`:all;
  });
  fs.writeFileSync(path.join(deliver,'Editable text',m.file),editable);
}
fs.copyFileSync('/Users/toko/Desktop/B11 Marketing Complete Prompt Library Phase 1 Justyna
Jastrzebska.pdf',path.join(deliver,'Source libraries','Complete Library - Parts 1 and 2.pdf'));
fs.copyFileSync('/Users/toko/Desktop/B11 Marketing Prompt Library - #0 Justyna Jastrzebska
.pdf',path.join(deliver,'Source libraries','Library 0 - Conversion Pages.pdf'));
fs.writeFileSync(path.join(root,'working/build-summary.json'),JSON.stringify({date:'2026-09-
23',outputs:chapterData.map(({raw,...m})=>m),totalWords,deliver},null,2));
const auditPath=path.join(root,'working/content-audit.json');
if(fs.existsSync(auditPath)){
  const audit=JSON.parse(fs.readFileSync(auditPath,'utf8'));
  const coverage='# Coverage and handoff\n\nAssembled 23 September 2026 from both supplied B11
libraries. Writing: three GPT-6 Sol agents; coordination, cross-chapter reconciliation and HTML QA:
```

```
parent agent.\n\n'+
    `Written outputs: ${chapterData.length}/20. Requested top-level sections:
${audit.requiredSections}/138. Approximate word count: ${totalWords.toLocaleString()}. Automated
content checks: ${audit.complete?'passed':'in progress'}.\n\n`+
    '| Output | Requested sections | Written answer |\n|---|---:|---|\n'+chapterData.map(m=>`|
${m.label}: ${m.title} | ${m.sections} | [Open](<Individual outputs/${m.id}.html>) |`).join('\n')+
    '\n\n## Scope and limits\n\nAll 85 content IDs appear in the funnel, page map, distribution cards
and tracking library. The book includes 36 channel assessments, 11 review records, 15 interview
questions, 9 outreach templates, 4 campaign emails, 10 campaign posts, video scripts, 11 page
specifications and detailed conversion-page exercises.\n\nThe source PDFs describe 16 sequential
prompts; treating 3A and 3B separately yields 17 main-library outputs, plus three from Library #0.
Required sections are written; bracketed testimonial, event, policy and baseline fields deliberately
remain where real evidence or a business decision is absent. No interviews, customer endorsement,
campaign launch, analytics installation, form repair or website deployment occurred.\n\nThe central
open launch gate is verified demo-request receipt. The source code uses a simulated submission;
deployed behavior and any lost lead count are unverified. The five-hour calendar is shared, and 85
content ideas are a backlog, not a weekly production requirement.\n\nStart with START HERE.html and
its expandable first-week guide. Individual outputs are independent HTML files; keep the complete
folder together when sharing so source and cross-chapter links remain available. Internal source
snapshots preserve some original workspace references; public source URLs remain online
references.\n';
  fs.writeFileSync(path.join(deliver,'Coverage and handoff.md'),coverage);
}
console.log(JSON.stringify({outputs:available.length,words:totalWords,path:path.join(deliver,'START
HERE.html')}));

// A clearly named Desktop copy prevents the original Prompt 1 tab from being mistaken for the full
book.
const desktopCopy='/Users/toko/Desktop/MediMind Marketing - UPDATED - All 20 Prompts.html';
const desktopHtml=fs.readFileSync(path.join(deliver,'START HERE.html'),'utf8')
  .replace('<title>MediMind - B11 Marketing Playbook</title>','<title>MediMind - Marketing Playbook -
All 20 Prompts</title>')
  .replace(/href="([^\"]+)"/g,(all,href)=>/^(?:#|[a-z][a-z0-9+.-]*:|\/)/i.test(href)?
all:`href="MediMind - Complete B11 Marketing Library/${href}"`);
fs.writeFileSync(desktopCopy,desktopHtml);
console.log(JSON.stringify({desktopCopy,chapters:(desktopHtml.match(/<article class="chapter"/g)||
[]).length,bytes:Buffer.byteLength(desktopHtml)}));
```

Source: Original files/Marketing workspace/prompt-library/complete-edition/working/build-summary.json. Historical wording preserved; not a new approval or factual revalidation.

build-summary.json

Technical appendix. This is an archived working file, not instructions to execute.


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      "words": 2416
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      "group": "Customer & Market Research",
      "label": "Part 1 · Prompt 2",
      "pages": "8-10",
      "sections": 6,
      "words": 2080
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      "group": "Customer & Market Research",
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      "pages": "11-12",
      "sections": 6,
      "words": 4068
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      "words": 2576
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      "group": "Customer & Market Research",
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      "pages": "15-16",
      "sections": 6,
      "words": 2538
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      "title": "Mine Competitor Reviews & Customer Intelligence",
      "group": "Customer & Market Research",
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      "sections": 6,
      "words": 1935
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      "words": 2287
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      "words": 5754
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      "pages": "33–35",
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      "words": 3031
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      "group": "Marketing Funnel & Campaign",
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      "words": 1887
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      "words": 4520
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      "pages": "46–50",
      "sections": 5,
      "words": 7647
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      "file": "p2-06-tracking.md",
      "title": "Tracking, Mapping & Implementation",
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      "label": "Part 2 · Prompt 6",
      "pages": "51–55",
      "sections": 6,
      "words": 7985
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      "words": 2097
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      "sections": 13,
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      "pages": "14–20",
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      "words": 1532
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  ],
  "totalWords": 67728,
  "deliver": "/Users/toko/Desktop/MediMind — Complete B11 Marketing Library"
}
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Source: Original files/Marketing workspace/prompt-library/complete-edition/working/check-book.mjs. Historical wording preserved; not a new approval or factual revalidation.

check-book.mjs

Technical appendix. This is an archived working file, not instructions to execute.

```
import fs from 'node:fs';
import path from 'node:path';
import { chromium } from '/Users/toko/.cache/codex-runtimes/codex-primary-
runtime/dependencies/node/node_modules/playwright/index.mjs';
const root=path.resolve(import.meta.dirname,'..');
const delivery='/Users/toko/Desktop/MediMind - Complete B11 Marketing Library';
const qa=path.join(root,'working/qa');
fs.mkdirSync(qa,{recursive:true});
const summary=JSON.parse(fs.readFileSync(path.join(root,'working/build-summary.json'),'utf8'));
const browser=await chromium.launch({headless:true});
const page=await browser.newPage({viewport:{width:1440,height:1000},deviceScaleFactor:1});
const errors=[];
page.on('pageerror',e=>errors.push(String(e)));
await page.goto('file://'+path.join(delivery,'START HERE.html'));
await page.screenshot({path:path.join(qa,'desktop-overview.png')});
const structural=await page.evaluate(()=>({
  chapters:document.querySelectorAll('article.chapter').length,
  h2:document.querySelectorAll('article.chapter h2').length,
  tables:document.querySelectorAll('article.chapter table').length,
  overflow:document.documentElement.scrollWidth>innerWidth,
  duplicateIds:[...document.querySelectorAll('[id]')].map(e=>e.id).filter((x,i,a)=>a.indexOf(x)!==i),
  brokenAnchors:
[...document.querySelectorAll('a[href^="#"]')].map(a=>a.getAttribute('href')).filter(h=>h.length>1&&!d
ocument.getElementById(h.slice(1))),
  fileLinks:[...document.querySelectorAll('a[href]')].map(a=>a.href).filter(h=>h.startsWith('file:')),
  emptyCells:[...document.querySelectorAll('.answer td')].filter(e=>!e.textContent.trim()).length
}));
if(structural.chapters!==summary.outputs.length)errors.push('Wrong chapter count');
if(structural.overflow)errors.push('Desktop overflow');
if(structural.duplicateIds.length)errors.push('Duplicate IDs');
if(structural.brokenAnchors.length)errors.push('Broken anchors');
const missingFiles=[...new Set(structural.fileLinks)].filter(h=>!fs.existsSync(decodeURIComponent(new
URL(h).pathname)));
// Assets linked by the authors to planned website routes are HTTP links, not local files.
for(const m of summary.outputs){
  await page.locator('#'+m.id).scrollIntoViewIfNeeded();
  await page.screenshot({path:path.join(qa,m.id+'-desktop.png')});
}
await page.locator('#search').fill('PLACEHOLDER');
const searchCount=await page.locator('.chapter:visible').count();
if(searchCount===0)errors.push('Search did not find proof placeholders');
await page.locator('#clear').click();
if(await page.locator('.chapter:visible').count()!==summary.outputs.length)errors.push('Show all
failed');
await page.evaluate(()=>{window.print=()=>{window.__printRequested=true;};});
await page.locator('.printchapter').first().click();
if(await page.locator('.chapter:visible').count()!==1)errors.push('Print-one did not isolate one
answer');
await page.evaluate(()=>dispatchEvent(new Event('afterprint')));
if(await page.locator('.chapter:visible').count()!==summary.outputs.length)errors.push('After-print
did not restore all answers');
await page.setViewportSize({width:390,height:844});
await page.evaluate(()=>scrollTo(0,0));
await page.screenshot({path:path.join(qa,'mobile-overview.png')});
await page.locator('#menu').click();
if(!(await page.locator('.sidebar').isVisible()))errors.push('Mobile menu failed');
```

```
await page.locator('#menu').click();
await page.locator('#first-week summary').click();
const guideOverflow=await page.evaluate(()=>document.documentElement.scrollWidth>innerWidth);
if(guideOverflow)errors.push('Expanded first-week guide overflows mobile');
await page.locator('#first-week').scrollIntoViewIfNeeded();
await page.screenshot({path:path.join(qa,'mobile-first-week-guide.png')});
await page.locator('#first-week summary').click();
for(const id of ['p1-03','p1-06','p2-01','p2-03b','p2-06','l0-01']){
  if(!summary.outputs.some(m=>m.id===id))continue;
  await page.locator('#'+id).scrollIntoViewIfNeeded();
  await page.screenshot({path:path.join(qa,id+'-mobile.png')});
}
const mobileOverflow=await page.evaluate(()=>document.documentElement.scrollWidth>innerWidth);
if(mobileOverflow)errors.push('Mobile page overflow');
for(const m of summary.outputs){
  await page.goto('file://'+path.join(delivery,'Individual outputs',m.id+'.html'));
  if(await page.locator('.chapter').count()!==1)errors.push('Separate output has wrong chapter count:
'+m.id);
}
await browser.close();
const result=
{outputs:summary.outputs.length,words:summary.totalWords,headings:structural.h2,tables:structural.tabl
es,searchCount,missingFiles,errors,qa};
fs.writeFileSync(path.join(qa,'check.json'),JSON.stringify(result,null,2));
console.log(JSON.stringify(result,null,2));
if(errors.length||missingFiles.length)process.exitCode=1;
```

Source: Original files/Marketing workspace/prompt-library/complete-edition/working/content-audit.json. Historical wording preserved; not a new approval or factual revalidation.

content-audit.json

Technical appendix. This is an archived working file, not instructions to execute.


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        "Step 3 – Surface my 3 biggest assumptions",
        "Step 4 – Market sizing I can actually use",
        "Step 5 – Identify the key players",
        "Step 6 – Pinpoint the gap",
        "Step 7 – Market positioning statement"
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      "group": "Customer & Market Research",
      "label": "Part 1 · Prompt 2",
      "pages": "8-10",
      "sections": 6,
      "words": 2080,
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        "Step 2 – Identify the moment of value",
        "Step 3 – Identify friction points",
        "Step 4 – Apply the JTBD lens",
        "Step 5 – Identify my market sophistication level",
        "Step 6 – Rewrite the product description"
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      "placeholderCount": 0
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    {
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      "file": "p1-03-bfg.md",
      "title": "Build Your Features, Benefits & Guarantees",
      "group": "Customer & Market Research",
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      "label": "Part 1 · Prompt 3",
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        "Step 2 — Convert every feature to a benefit",
        "Step 3 — Add the emotional benefit layer",
        "Step 4 — Build the guarantees and trust layer",
        "Step 5 — Build the BFG master table",
        "Step 6 — Identify my strongest BFG combination"
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      "placeholderCount": 0
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    {
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      "group": "Customer & Market Research",
      "label": "Part 1 · Prompt 4",
      "pages": "13–14",
      "sections": 6,
      "words": 2576,
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        "Step 2 — Confirm market sophistication level",
        "Step 3 — Generate 10 UVPs as brand promises",
        "Step 4 — Score every UVP",
        "Step 5 — Match UVPs to customer types",
        "Step 6 — Write the hero statement"
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        "Step 2 — Competitive comparison table",
        "Step 3 — Find the white space",
        "Step 4 — Write and stress-test the defensible positioning statement",
        "Step 5 — Identify my biggest competitive threat",
        "Step 6 — Build five competitive talking points"
      ],
      "placeholderCount": 0
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    {
      "id": "p1-06",
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      "group": "Customer & Market Research",
      "label": "Part 1 · Prompt 6",
      "pages": "17–18",
      "sections": 6,
      "words": 2269,
      "headings": [
        "Step 1 — Identify where competitor reviews live",
        "Step 2 — Extract real problems across four buckets",
        "Step 3 — Extract exact customer language",
        "Step 4 — Map problems to my product",
        "Step 5 — Build three acquisition messages",
        "Step 6 — One-page review intelligence summary brief"
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      "words": 3032,
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        "Step 2 — Define my research goal",
        "Step 3 — Segment my user groups",
        "Step 4 — Build the interview guide",
        "Step 5 — Build a documentation system",
        "Step 6 — Research action plan",
        "Step 7 — Tailored question sets for all three groups",
        "Step 8 — Nine outreach templates"
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      "file": "p1-08-problems.md",
      "title": "Define the Real Problems You Are Solving",
      "group": "Customer & Market Research",
      "label": "Part 1 · Prompt 8",
      "pages": "21–22",
      "sections": 6,
      "words": 1935,
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        "Step 2 — Build the full problem stack",
        "Step 3 — Identify the problem worth leading with",
        "Step 4 — Write three problem statement versions",
        "Step 5 — Problems I should not claim to solve",
        "Step 6 — Problem-solution fit statement for each type"
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      "group": "Customer & Market Research",
      "label": "Part 1 · Prompt 9",
      "pages": "23–25",
      "sections": 6,
      "words": 2287,
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        "Step 2 — Build the avatar's emotional landscape",
        "Step 3 — Write the avatar in their own voice",
        "Step 4 — Identify the avatar's watering holes",
        "Step 5 — Identify the avatar's buying trigger",
        "Step 6 — Build the avatar reference card"
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    },
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      "file": "p2-01-funnel.md",
      "title": "Map Your Marketing Funnel Architecture",
      "group": "Marketing Funnel & Campaign",
      "label": "Part 2 · Prompt 1",
      "pages": "27–31",
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        "Part 2 — Full funnel stage map",
        "Part 3 — Content and asset brainstorm",
        "Part 4 — Funnel wiring diagram",
        "Part 5 — Build order",
        "Part 6 — Weekly funnel scorecard"
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      "label": "Part 2 · Prompt 2",
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      "sections": 6,
      "words": 3031,
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        "Part 1 — Master marketing angle",
        "Part 2 — Angle selection and prioritization",
        "Part 3 — Angle-to-channel mapping",
        "Part 4 — Ten-hook library",
        "Part 5 — Message by funnel stage",
        "Part 6 — 30-day angle execution plan"
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    },
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      "id": "p2-03a",
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      "label": "Part 2 · Prompt 3A",
      "pages": "36–38",
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        "Part 3 — Two shorter A/B script variations"
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      "label": "Part 2 · Prompt 3B",
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      "sections": 5,
      "words": 4520,
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        "Part 1 — Ad copy",
        "Part 2 — Four-email campaign sequence",
        "Part 3 — Two-week social content calendar",
        "Part 4 — Campaign destination page: precise edits to existing `/request-demo`",
        "Part 5 — 14-day launch plan within the shared five-hour weekly budget"
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        "Step 2 — Identify which pages need to be built and edit what exists",
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        "Part 3 — Build priority"
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      "label": "Part 2 · Prompt 5",
      "pages": "46–50",
      "sections": 5,
      "words": 7647,
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        "Part 1 — Channel selection and priority",
        "Part 2 — Content piece distribution map",
        "Part 3 — Traffic building system",
        "Part 4 — Weekly distribution routine",
        "Part 5 — Distribution benchmarks and growth scorecard"
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      "file": "p2-06-tracking.md",
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      "group": "Marketing Funnel & Campaign",
      "label": "Part 2 · Prompt 6",
      "pages": "51–55",
      "sections": 6,
      "words": 7985,
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        "Part 1 — Funnel tracking system",
        "Part 2 — Tool setup guide",
        "Part 3 — UTM tracking system",
        "Part 4 — Weekly tracking routine",
        "Part 5 — Week-by-week implementation plan",
        "Part 6 — 90-day success dashboard"
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      "file": "p2-07-reporting.md",
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      "group": "Marketing Funnel & Campaign",
      "label": "Part 2 · Prompt 7",
      "pages": "56–58",
      "sections": 4,
      "words": 2097,
      "headings": [
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        "Part 2 — Monthly funnel review",
        "Part 3 — Investor and advisor update",
        "Part 4 — North star dashboard"
      ],
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        "Section 3 – Above-the-fold trust line",
        "Section 4 – CTA button text",
        "Section 5 – What happens on the demo",
        "Section 6 – Pain section",
        "Section 7 – How it works",
        "Section 8 – Differentiation section",
        "Section 9 – Testimonial block",
        "Section 10 – Logo bar caption",
        "Section 11 – Objection FAQ",
        "Section 12 – Privacy line",
        "Section 13 – Bottom repeat CTA"
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      "placeholderCount": 2
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      "label": "Library #0 · Prompt 2",
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        "Section 2 – Subheadline",
        "Section 3 – Date / time / format bar",
        "Section 4 – CTA button text",
        "Section 5 – Above-the-fold trust line",
        "Section 6 – Speaker block",
        "Section 7 – What you will learn",
        "Section 8 – Pain section",
        "Section 9 – Agenda block",
        "Section 10 – Social proof block",
        "Section 11 – Objection FAQ",
        "Section 12 – “Can't make it?” secondary CTA",
        "Section 13 – Privacy line",
        "Section 14 – Bottom repeat CTA"
      ],
      "placeholderCount": 6
    },
    {
      "id": "l0-03",
      "file": "l0-03-trial.md",
      "title": "Free Trial Page",
      "group": "Conversion Page Library #0",
      "label": "Library #0 · Prompt 3",
      "pages": "14–20",
      "sections": 15,
      "words": 1532,
      "headings": [
        "Section 1 – Headline",
        "Section 2 – Subheadline",
        "Section 3 – Primary CTA button text",
        "Section 4 – Trust strip",
        "Section 5 – Social proof bar",
        "Section 6 – The problem section",
        "Section 7 – What you get in the trial",
        "Section 8 – Up and running in 3 steps",
        "Section 9 – Social proof block",
```

```
        "Section 10 – Product visual caption",
        "Section 11 – Trust and security block",
        "Section 12 – Objection FAQ",
        "Section 13 – Pricing context line",
        "Section 14 – Bottom repeat CTA",
        "Section 15 – Privacy line"
      ],
      "placeholderCount": 10
    }
  ]
}
```

Source: Original files/Marketing workspace/prompt-library/complete-edition/working/manifest.json. Historical wording preserved; not a new approval or factual revalidation.

manifest.json

Technical appendix. This is an archived working file, not instructions to execute.


```
[
  {"id":"p1-01","file":"p1-01-business-market.md","title":"Describe Your Business &
Market","group":"Customer & Market Research","label":"Part 1 · Prompt 1","pages":"5-7","sections":7},
  {"id":"p1-02","file":"p1-02-mechanics-jtbd.md","title":"Map Your Product Mechanics &
JTBD","group":"Customer & Market Research","label":"Part 1 · Prompt 2","pages":"8-10","sections":6},
  {"id":"p1-03","file":"p1-03-bfg.md","title":"Build Your Features, Benefits &
Guarantees","group":"Customer & Market Research","label":"Part 1 · Prompt 3","pages":"11-
12","sections":6},
  {"id":"p1-04","file":"p1-04-uvps.md","title":"Generate Your UVPs & Map Your JTBD","group":"Customer
& Market Research","label":"Part 1 · Prompt 4","pages":"13-14","sections":6},
  {"id":"p1-05","file":"p1-05-competition.md","title":"Map Competition & Find Your Defensible
Position","group":"Customer & Market Research","label":"Part 1 · Prompt 5","pages":"15-
16","sections":6},
  {"id":"p1-06","file":"p1-06-review-intelligence.md","title":"Mine Competitor Reviews & Customer
Intelligence","group":"Customer & Market Research","label":"Part 1 · Prompt 6","pages":"17-
18","sections":6},
  {"id":"p1-07","file":"p1-07-user-research.md","title":"Design & Execute Your User
Research","group":"Customer & Market Research","label":"Part 1 · Prompt 7","pages":"19-
20","sections":8},
  {"id":"p1-08","file":"p1-08-problems.md","title":"Define the Real Problems You Are
Solving","group":"Customer & Market Research","label":"Part 1 · Prompt 8","pages":"21-
22","sections":6},
  {"id":"p1-09","file":"p1-09-avatar.md","title":"Build Your Customer Avatar","group":"Customer &
Market Research","label":"Part 1 · Prompt 9","pages":"23-25","sections":6},
  {"id":"p2-01","file":"p2-01-funnel.md","title":"Map Your Marketing Funnel
Architecture","group":"Marketing Funnel & Campaign","label":"Part 2 · Prompt 1","pages":"27-
31","sections":6},
  {"id":"p2-02","file":"p2-02-angles.md","title":"Define Your Marketing Angles","group":"Marketing
Funnel & Campaign","label":"Part 2 · Prompt 2","pages":"33-35","sections":6},
  {"id":"p2-03a","file":"p2-03a-campaign-script.md","title":"Build Your Campaign Concept & Video
Script","group":"Marketing Funnel & Campaign","label":"Part 2 · Prompt 3A","pages":"36-
38","sections":3},
  {"id":"p2-03b","file":"p2-03b-campaign-distribution.md","title":"Build Your Campaign
Distribution","group":"Marketing Funnel & Campaign","label":"Part 2 · Prompt 3B","pages":"39-
41","sections":5},
  {"id":"p2-04","file":"p2-04-funnel-pages.md","title":"Build Your Funnel Pages","group":"Marketing
Funnel & Campaign","label":"Part 2 · Prompt 4","pages":"42-45","sections":4},
  {"id":"p2-05","file":"p2-05-traffic.md","title":"Content Distribution & Traffic
Strategy","group":"Marketing Funnel & Campaign","label":"Part 2 · Prompt 5","pages":"46-
50","sections":5},
  {"id":"p2-06","file":"p2-06-tracking.md","title":"Tracking, Mapping &
Implementation","group":"Marketing Funnel & Campaign","label":"Part 2 · Prompt 6","pages":"51-
55","sections":6},
  {"id":"p2-07","file":"p2-07-reporting.md","title":"Reporting","group":"Marketing Funnel &
Campaign","label":"Part 2 · Prompt 7","pages":"56-58","sections":4},
  {"id":"l0-01","file":"l0-01-demo.md","title":"Book a Demo Page","group":"Conversion Page Library
#0","label":"Library #0 · Prompt 1","pages":"3-7","sections":13},
  {"id":"l0-02","file":"l0-02-webinar.md","title":"Webinar Registration Page","group":"Conversion Page
Library #0","label":"Library #0 · Prompt 2","pages":"8-13","sections":14},
  {"id":"l0-03","file":"l0-03-trial.md","title":"Free Trial Page","group":"Conversion Page Library
#0","label":"Library #0 · Prompt 3","pages":"14-20","sections":15}
]
```

Source: Original files/Marketing workspace/prompt-library/complete-edition/working/qa/check.json. Historical wording preserved; not a new approval or factual revalidation.

check.json

Technical appendix. This is an archived working file, not instructions to execute.


```
{
  "outputs": 20,
  "words": 67728,
  "headings": 139,
  "tables": 75,
  "searchCount": 5,
  "missingFiles": [],
  "errors": [],
  "qa": "/Users/toko/Desktop/MediMind Landing Page/marketing/prompt-library/complete-edition/working/qa"
}
```

Source: Original files/Marketing workspace/workshop/build_acquisition_sales_brief.py. Historical wording preserved; not a new approval or factual revalidation.

build_acquisition_sales_brief.py

Technical appendix. This is an archived working file, not instructions to execute.

```
"""Create the three-page Block 2 sales workshop brief and verify its page count."""
from pathlib import Path

from reportlab.lib import colors
from reportlab.lib.pagesizes import A4
from reportlab.lib.styles import ParagraphStyle
from reportlab.pdfbase import pdfmetrics
from reportlab.pdfbase.ttfonts import TTFont
from reportlab.platypus import SimpleDocTemplate, Paragraph, Spacer, Table, TableStyle, PageBreak
from pypdf import PdfReader

ROOT = Path(__file__).resolve().parents[2]
OUTPUT = ROOT / "output/pdf/MediMind - Acquisition and Sales Foundation.pdf"
OUTPUT.parent.mkdir(parents=True, exist_ok=True)
FONT_DIR = Path("/System/Library/Fonts/Supplemental")
for name, filename in (("Body", "Arial.ttf"), ("BodyBold", "Arial Bold.ttf"), ("Display",
"Georgia.ttf")):
    pdfmetrics.registerFont(TTFont(name, str(FONT_DIR / filename)))
pdfmetrics.registerFontFamily("Body", normal="Body", bold="BodyBold")
NAVY = colors.HexColor("#1a365d")
BLUE = colors.HexColor("#2b6cb0")
INK = colors.HexColor("#233248")
MUTED = colors.HexColor("#53657a")
LINE = colors.HexColor("#d7e2ef")
PALE = colors.HexColor("#edf5fc")
W, H = A4
M = 42
WIDTH = W - 2 * M


def p(text, size=10.2, leading=14, color=INK, bold=False):
    return Paragraph(text, ParagraphStyle("p", fontName="BodyBold" if bold else "Body",
                     fontSize=size, leading=leading, textColor=color, spaceAfter=0))


def heading(number, text):
    return [Spacer(1, 10), p(f"<font color='#2b6cb0'>{number}</font>  {text}", 13.2, 17, NAVY, True),
Spacer(1, 7)]


def card(title, body, fill=PALE):
    inner = [p(title, 11.5, 15, NAVY, True), Spacer(1, 7), p(body, 10.4, 14.4)]
    t = Table([[inner]], colWidths=[WIDTH])
    t.setStyle(TableStyle([("BACKGROUND", (0, 0), (-1, -1), fill),
                           ("LEFTPADDING", (0, 0), (-1, -1), 13),
                           ("RIGHTPADDING", (0, 0), (-1, -1), 13),
                           ("TOPPADDING", (0, 0), (-1, -1), 11),
                           ("BOTTOMPADDING", (0, 0), (-1, -1), 11)]))
    return t


def table(headers, rows, widths, size=9.6, leading=13):
    data = [[p(x, size, leading, NAVY, True) for x in headers]]
    data += [[p(x, size, leading) for x in row] for row in rows]
    t = Table(data, colWidths=widths, hAlign="LEFT")
    t.setStyle(TableStyle([("BACKGROUND", (0, 0), (-1, 0), PALE),
```

```
                           ("VALIGN", (0, 0), (-1, -1), "TOP"),
                           ("LEFTPADDING", (0, 0), (-1, -1), 8),
                           ("RIGHTPADDING", (0, 0), (-1, -1), 8),
                           ("TOPPADDING", (0, 0), (-1, -1), 5),
                           ("BOTTOMPADDING", (0, 0), (-1, -1), 5),
                           ("LINEBELOW", (0, 0), (-1, -1), .5, LINE)]))
    return t


TITLES = ["Acquisition & sales foundation", "Outreach that opens a conversation", "Turn a conversation
into a real test"]
SUBTITLES = ["A realistic first-month plan for hospitals beyond the anchor partner.",
             "Ready-to-adapt drafts. One clear next step; no pressure to switch.",
             "A 25-minute discovery call, followed by a clear decision and owner."]


def decorate(c, doc):
    page = doc.page
    c.setTitle("MediMind | Acquisition & Sales Foundation | Block 2")
    c.setAuthor("MediMind")
    c.setSubject("Lead generation, initial lead sources, outreach drafts and discovery script")
    c.setFillColor(NAVY)
    c.setFont("BodyBold", 17)
    c.drawString(M, H - 40, "MediMind")
    c.setFont("Body", 8.5)
    c.setFillColor(MUTED)
    c.drawRightString(W - M, H - 39, "BLOCK 2  /  02 OCTOBER 2026")
    c.setStrokeColor(LINE)
    c.line(M, H - 52, W - M, H - 52)
    c.setFont("Display", 23)
    c.setFillColor(NAVY)
    c.drawString(M, H - 87, TITLES[min(page - 1, 2)])
    c.setFont("Body", 9.8)
    c.setFillColor(MUTED)
    c.drawString(M, H - 108, SUBTITLES[min(page - 1, 2)])
    c.line(M, 39, W - M, 39)
    c.setFont("Body", 8)
    c.drawString(M, 25, "MediMind  |  Team working brief - proposed plan, not reported results")
    c.drawRightString(W - M, 25, f"{page} / 3")
    c.setFont("Body", 7)
    c.drawString(M, 14, "Basis: Block 2 workshop slide, founder offer and existing MediMind marketing
work.")


story = []
story += heading("01", "Lead generation plan: trust first, volume later")
story.append(p("<b>First buyer:</b> a Georgian private hospital/group leader with a recurring cross-
team problem and staff willing to test an alternative. Sell the hospital-wide vision; agree a
manageable first evaluation."))
story.append(Spacer(1, 9))
story.append(table(["Priority / channel", "Approach + weekly action"], [
    ["<b>1. Warm introductions</b>", "Ask clinicians, advisers and real professional contacts for a
specific introduction. Confirm the relationship; do not imply an endorsement."],
    ["<b>2. Targeted founder outreach</b>", "Contact a researched hospital leader by work email or
LinkedIn. Ask about one pain using the drafts on page 2."],
    ["<b>3. Follow-up + proof</b>", "Follow up personally with a relevant synthetic-data workflow
example. Skip paid ads, purchased lists and broad campaigns for now."],
], [139, WIDTH - 139]))
story.append(Spacer(1, 9))
story.append(p("<b>Budget:</b> 5 team hours/week: research, introductions, outreach, follow-up and
review - 1h each. Calls and engineering are additional. Founder owns conversations; a coworker can
maintain the list."))

story += heading("02", "Initial lead list: build 15 accounts, not 150 names")
story.append(p("Prioritize <b>private multi-profile hospitals and groups with an accessible
sponsor</b>. Rank warm access and a verified reason to talk above size. Several sites may share one
buying decision."))
story.append(Spacer(1, 8))
story.append(table(["Where to start", "What to verify before calling it a lead"], [
```

```
    ["Founder / clinician network", "Name the hospital, connector and intended buyer. A connector is
not automatically an internal champion."],
    ["Official hospital sites + professional profiles", "Confirm services, current leadership, buying
group and work contact route. Never infer software problems from a website."],
], [174, WIDTH - 174]))
story.append(Spacer(1, 8))
story.append(p("<b>Four research seeds, not qualified leads:</b> "
    "<link href='https://newhospitals.ge/en' color='#2b6cb0'>New Hospitals</link>; "
    "<link href='https://ahtbilisi.com/en/' color='#2b6cb0'>American Hospital Tbilisi</link>; "
    "<link href='https://cmchospital.ge/en' color='#2b6cb0'>Caucasus Medical Centre</link>; "
    "<link href='https://toduaclinic.ge/en' color='#2b6cb0'>Todua Clinic</link>. "
    "Official sites checked 2 October 2026. Their pain, interest, access and purchasing independence
are unconfirmed; deduplicate against existing relationships.", 9.4, 12.8))
story.append(Spacer(1, 8))
story.append(p("<b>One tracker row per buying organization:</b> hospital/group; source + date; fit
evidence; buyer; connector; possible champion; pain (hypothesis or confirmed); stage; owner; last
touch; next action + date.", 9.4, 12.8))
story.append(Spacer(1, 10))
story.append(card("30-day experiment - planning targets, not a forecast", "<b>Week 1:</b> verify 15
accounts and request 5 introductions. <b>Weeks 2-3:</b> send 10 tailored first messages, avoiding
duplicate approaches; follow up and hold discovery calls. <b>Week 4:</b> aim to scope one evaluation.
<br/><b>Learning targets:</b> 4 held discovery calls, 2 SQLs, 1 evaluation-ready hospital. Review what
blocks progress; a completed test may fall outside this month."))
story.append(PageBreak())

story += heading("03", "Outreach content: three usable drafts")
story.append(p("Send personally, in the recipient's preferred language. If you add a hospital-specific
opening, use only a fact you have checked. These are drafts for outreach, not messages that have been
sent.", 9.8, 13.5, MUTED))
story.append(Spacer(1, 12))
story.append(card("A. Warm-introduction request - to a real connector", "Do you know a hospital CEO or
operations leader whose team is doing too much work between disconnected systems?<br/>MediMind brings
that work into one tailored, AI-assisted system. We build a parallel version free for their team to
test, with no obligation to buy.<br/>Would you be comfortable introducing us for a 25-minute
conversation?"))
story.append(Spacer(1, 12))
story.append(card("B. Direct email - subject: Test the alternative before you switch", "Is your team
re-entering information, chasing updates or fixing gaps between hospital systems?<br/><br/>MediMind
brings your hospital into one tailored system with AI tools in daily workflows. We build an agreed
parallel version free, alongside your current system. Your team tests it before any obligation to buy.
<br/><br/>For our next 10 hospital clients, we match their current software price when they switch,
under agreed terms.<br/><br/>Could we spend 25 minutes next week choosing one workflow worth
testing?"))
story.append(Spacer(1, 12))
story.append(card("C. Short message - LinkedIn or an existing business chat", "When hospital systems
do not connect, staff become the connection. Does that sound familiar?<br/>We build a tailored, AI-
assisted parallel system free. Test it before any purchase commitment; your current system stays in
place.<br/>Open to a 25-minute call to see if this fits?"))
story += heading("", "A simple follow-up rhythm")
story.append(p("<b>Day 4:</b> \"Which would be more useful to improve first: repeated entry, chasing
patient information, or billing corrections? If none is a priority, no problem.\"<br/><b>Day 10:</b>
\"I will close the loop here. If a parallel comparison becomes useful later, reply and we can pick one
workflow.\"<br/>Stop after a decline or opt-out. If they respond, follow the agreed next step rather
than this schedule.", 10, 13.7))
story.append(Spacer(1, 10))
story.append(p("<b>Keep the offer accurate:</b> the next-ten-client offer is not a countdown. Record
eligibility, matched cost items, duration and development scope in the proposal. Do not promise a
launch date before readiness is checked, or proven revenue/clinical gains before they are measured.",
9, 12.2, MUTED))
story.append(PageBreak())

story += heading("04", "Discovery call script - diagnose before demonstrating")
story.append(table(["Time / purpose", "Questions to ask, in order"], [
    ["<b>0-2 min</b><br/>Set the frame", "\"Let's identify one problem worth testing in a free
parallel system. No decision to switch today. What would make this call useful?\""],
    ["<b>2-8 min</b><br/>Find the real pain", "\"Describe the last difficult handoff between teams.
Where did someone re-enter, call, wait or correct something? How often does it happen, and who deals
with it?\""],
    ["<b>8-12 min</b><br/>Find impact + urgency", "\"What did that cost in staff time, delays or
```

```
billing corrections? What have you tried? Why act now? What improvement would make a test worthwhile?
\""],
    ["<b>12-17 min</b><br/>Map the buying path", "\"Who approves software, organizes testing and
reviews clinical, finance and IT concerns? When does your contract renew? If the test works, how would
budget and approval happen?\""],
    ["<b>17-22 min</b><br/>Check fit + explain offer", "\"We build a separate test free while yours
stays in place, with no obligation to buy. Which workflow matters most? What cannot change? Who will
test it and approve data use?\""],
    ["<b>22-25 min</b><br/>Agree the next step", "\"Have I understood the problem, desired improvement
and constraints correctly? Shall we book a scoping session with your operations/clinical and IT
owners? Who joins, and when?\""],
], [104, WIDTH - 104], size=9.8, leading=13.4))
story.append(Spacer(1, 10))
story.append(p("<b>Example test:</b> compare one task on comparable cases in both systems. Measure
time, repeated entries and errors against a threshold agreed in advance. This does not prove clinical
outcomes or revenue gains.", 9.7, 13.2))

story += heading("", "After the call: advance, nurture or disqualify")
story.append(p("<b>MQL:</b> fits the target, a relevant contact engages, and discovery/another
meaningful next step is dated.<br/><b>SQL:</b> pain + reason to act, sponsor + approval path, internal
owner, credible budget path, feasible scope and a dated planning step are confirmed.<br/><b>Not ready:
</b> agree a specific follow-up trigger. <b>No fit:</b> close the lead with the reason; do not force a
demo.", 9.8, 13.4))
story.append(Spacer(1, 10))
story.append(card("The handoff: interest is not permission to build", "<b>SQL → Evaluation-ready:</b>
confirm scope, hospital testers, permitted data/access, security approvals, success criteria and
dates. <b>Then:</b> build + internal checks → hospital hands-on test → joint results review → explicit
buy / revise / stop decision. A purchase leads to a separately agreed rollout.<br/><b>Timing:</b> a
two-week build is a planning target only after readiness and feasibility are confirmed - not two weeks
from the first email."))
story.append(Spacer(1, 9))
story.append(p("<b>Measure the real milestone:</b> distinct new hospital buying organizations outside
the anchor partner completing their first hands-on parallel-system evaluation session in the last 30
days. Track introductions, held calls, SQLs, readiness, build delivery and stalled days to explain
that number.", 9.7, 13.2))

doc = SimpleDocTemplate(str(OUTPUT), pagesize=A4, leftMargin=M, rightMargin=M,
                        topMargin=120, bottomMargin=51)
doc.build(story, onFirstPage=decorate, onLaterPages=decorate)
reader = PdfReader(OUTPUT)
assert len(reader.pages) == 3, f"Expected 3 pages, got {len(reader.pages)}; shorten content before
publishing."
assert all(page.extract_text().strip() for page in reader.pages)
print(OUTPUT)
```

Source: Original files/Marketing workspace/workshop/build_core_positioning_brief.py. Historical wording preserved; not a new approval or factual revalidation.

build_core_positioning_brief.py

Technical appendix. This is an archived working file, not instructions to execute.

```
"""Render the short, shareable Block 1 workshop brief. No external data calls."""
from pathlib import Path

from reportlab.lib import colors
from reportlab.lib.enums import TA_LEFT
from reportlab.lib.pagesizes import A4
from reportlab.lib.styles import ParagraphStyle
from reportlab.pdfbase import pdfmetrics
from reportlab.pdfbase.ttfonts import TTFont
from reportlab.pdfgen import canvas
from reportlab.platypus import Paragraph, Table, TableStyle


ROOT = Path(__file__).resolve().parents[2]
OUTPUT = ROOT / "output/pdf/MediMind - Core Positioning and Customer Brief.pdf"
OUTPUT.parent.mkdir(parents=True, exist_ok=True)
FONT_DIR = Path("/System/Library/Fonts/Supplemental")
for name, filename in (("Body", "Arial.ttf"), ("BodyBold", "Arial Bold.ttf"), ("Display",
"Georgia.ttf")):
    pdfmetrics.registerFont(TTFont(name, str(FONT_DIR / filename)))
pdfmetrics.registerFontFamily("Body", normal="Body", bold="BodyBold")

NAVY = colors.HexColor("#1a365d")
BLUE = colors.HexColor("#2b6cb0")
INK = colors.HexColor("#233248")
MUTED = colors.HexColor("#53657a")
LINE = colors.HexColor("#d7e2ef")
PALE = colors.HexColor("#edf5fc")
GREEN = colors.HexColor("#22634d")
WIDTH, HEIGHT = A4
MARGIN = 42
CONTENT = WIDTH - 2 * MARGIN
C = canvas.Canvas(str(OUTPUT), pagesize=A4)
C.setTitle("MediMind | Core Positioning & Customer | Block 1")
C.setAuthor("MediMind")
C.setSubject("Workshop brief: value proposition, ideal customer, buyer personas, MQL and SQL")


def para(text, x, top, width, size=10.4, leading=14.2, color=INK, font="Body"):
    style = ParagraphStyle("p", fontName=font, fontSize=size, leading=leading,
                           textColor=color, alignment=TA_LEFT, spaceAfter=0)
    p = Paragraph(text, style)
    _, height = p.wrap(width, HEIGHT)
    p.drawOn(C, x, top - height)
    return top - height


def section(number, title, top):
    C.setFillColor(BLUE)
    C.roundRect(MARGIN, top - 20, 25, 20, 4, fill=1, stroke=0)
    C.setFillColor(colors.white)
    C.setFont("BodyBold", 9)
    C.drawCentredString(MARGIN + 12.5, top - 13, number)
    para(title, MARGIN + 35, top - 1, CONTENT - 35, 13.5, 17, NAVY, "BodyBold")
    return top - 30
```

```
def header(page, title, subtitle):
    C.setFillColor(NAVY)
    C.setFont("BodyBold", 18)
    C.drawString(MARGIN, HEIGHT - 44, "Medi")
    C.setFillColor(BLUE)
    C.drawString(MARGIN + pdfmetrics.stringWidth("Medi", "BodyBold", 18), HEIGHT - 44, "Mind")
    C.setFont("Body", 8.5)
    C.setFillColor(MUTED)
    C.drawRightString(WIDTH - MARGIN, HEIGHT - 42, "BLOCK 1  /  02 OCTOBER 2026")
    C.setStrokeColor(LINE)
    C.line(MARGIN, HEIGHT - 57, WIDTH - MARGIN, HEIGHT - 57)
    top = para(title, MARGIN, HEIGHT - 77, CONTENT, 26, 31, NAVY, "Display")
    top = para(subtitle, MARGIN, top - 10, CONTENT, 10.4, 14.2, MUTED)
    C.setStrokeColor(LINE)
    C.line(MARGIN, 39, WIDTH - MARGIN, 39)
    C.setFont("Body", 8)
    C.setFillColor(MUTED)
    C.drawString(MARGIN, 25, "MediMind  |  Team working brief - proposed definitions")
    C.drawRightString(WIDTH - MARGIN, 25, f"{page} / 2")
    return top - 23


def box(text, top, fill=PALE, size=11, leading=15, padding=14):
    p = Paragraph(text, ParagraphStyle("box", fontName="Body", fontSize=size, leading=leading,
textColor=NAVY))
    _, h = p.wrap(CONTENT - 2 * padding, HEIGHT)
    total = h + 2 * padding
    C.setFillColor(fill)
    C.roundRect(MARGIN, top - total, CONTENT, total, 7, fill=1, stroke=0)
    p.drawOn(C, MARGIN + padding, top - padding - h)
    return top - total


def cell(text, bold=False, size=9.5):
    return Paragraph(text, ParagraphStyle("cell", fontName="BodyBold" if bold else "Body",
                     fontSize=size, leading=12.9, textColor=NAVY if bold else INK))


top = header(1, "Core positioning & customer", "A clear offer, a focused buyer and a reason to choose
MediMind.")
top = section("01", "USP / value proposition - the one-liner", top)
top = box("<b>MediMind replaces disconnected hospital software with one tailored, AI-assisted system -
with a free parallel evaluation before you commit to buy.</b>", top, size=13, leading=18)
top = para("<b>The offer:</b> we build the parallel system free; the hospital tests its quality and
the improvement before deciding. No obligation to buy. For our next <b>10 hospital clients</b>, we
match their current software price when they switch. Ongoing development is included within agreed
scope.", MARGIN, top - 12, CONTENT, 10.2, 14)
top = para("<b>Today:</b> connected workflows and AI assistance. <b>Next:</b> the foundation for
validated, hospital-wide AI, with clinicians retaining control of medical decisions.", MARGIN, top -
8, CONTENT, 10.2, 14)

top = section("02", "Ideal customer profile (ICP) - who we pursue first", top - 14)
top = para("<b>Initial focus:</b> Georgian private multi-profile hospitals and hospital groups with
disconnected clinical and business software, repeated cross-team work, and a leadership reason to
improve it.", MARGIN, top, CONTENT)
top = para("<b>Strong fit:</b> a recurring problem we can demonstrate improvement on, an accessible
decision sponsor, a potential internal champion, and willingness to allocate staff time to an
evaluation. <b>Not the first priority:</b> organizations seeking only a standalone AI chatbot, with no
relevant pain, or outside a scope we can support.", MARGIN, top - 8, CONTENT, 10.1, 13.8)

top = section("03", "Buyer personas + customer value propositions", top - 14)
rows = [
    [cell("Person / role", True), cell("Pain they recognize", True), cell("Value we put in front of
them", True)],
    [cell("CEO / owner / group operator<br/><font color='#53657a'>Economic buyer</font>", True),
cell("Pressure to improve performance without making a disruptive, expensive mistake."),
cell("Evaluate a tailored hospital-wide replacement before buying; matched software pricing for the
next ten clients.")],
```

```
    [cell("Operations or clinical lead<br/><font color='#53657a'>Internal champion</font>", True),
cell("Staff repeat work and chase patient information across teams."), cell("Connected workflows and
AI assistance in daily work; staff test whether the new system makes the work better.")],
    [cell("Finance lead / CFO<br/><font color='#53657a'>Financial evaluator</font>", True),
cell("Billing gaps, corrections and reports that take too much work to reconcile."), cell("Connect
clinical activity with financial records and test a specific billing or reporting improvement.")],
    [cell("IT / data owner<br/><font color='#53657a'>Technical approver</font>", True),
cell("Migration risk, access concerns and the burden of supporting another system."), cell("A
separate, scoped evaluation while the existing system stays in place; agree controls and
responsibilities before rollout.")],
]
t = Table(rows, colWidths=[121, 158, CONTENT - 279])
t.setStyle(TableStyle([("BACKGROUND", (0, 0), (-1, 0), PALE), ("VALIGN", (0, 0), (-1, -1), "TOP"),
                       ("LEFTPADDING", (0, 0), (-1, -1), 9), ("RIGHTPADDING", (0, 0), (-1, -1), 9),
                       ("TOPPADDING", (0, 0), (-1, -1), 8), ("BOTTOMPADDING", (0, 0), (-1, -1), 8),
                       ("LINEBELOW", (0, 0), (-1, -1), .5, LINE)]))
_, h = t.wrap(CONTENT, HEIGHT)
t.drawOn(C, MARGIN, top - h)
top -= h
top = para("A <b>connector</b> opens the door. A <b>champion</b> mobilizes hospital colleagues. A
<b>buyer</b> authorizes the purchase. These may be different people.", MARGIN, top - 10, CONTENT, 9.1,
12.3, MUTED)
assert top > 48, f"Page 1 content exceeds safe area: {top}"
C.showPage()

top = header(2, "Which leads deserve attention?", "Working MQL and SQL definitions for a founder-led
hospital sale.")
top = section("04", "MQL + SQL - clear gates, not vague interest", top)

def lead_card(x, top, width, label, full_name, meaning, points):
    pad = 14
    blocks = [
        (label, 18, 22, NAVY, "BodyBold", 0),
        (full_name, 9, 12, MUTED, "Body", 4),
        (meaning, 11, 15, NAVY, "BodyBold", 11),
        ("<br/>".join(f"<b>{i + 1}.</b> {p}" for i, p in enumerate(points)), 10, 14, INK, "Body", 11),
    ]
    ps = []
    total = 2 * pad
    for text, size, leading, color, font, gap in blocks:
        p = Paragraph(text, ParagraphStyle("card", fontName=font, fontSize=size, leading=leading,
textColor=color))
        _, height = p.wrap(width - 2 * pad, HEIGHT)
        ps.append((p, height, gap))
        total += height + gap
    return (x, width, total, ps)

card_width = (CONTENT - 14) / 2
cards = [
    lead_card(MARGIN, top, card_width, "MQL", "Marketing-qualified lead", "A good-fit hospital that
has engaged and is ready for discovery.", [
        "Fits our target hospital profile and supportable scope.",
        "A relevant contact is identified and engaged.",
        "The contact agrees to discovery or another dated next step.",
    ]),
    lead_card(MARGIN + card_width + 14, top, card_width, "SQL", "Sales-qualified lead", "Discovery
confirms a credible opportunity to plan an evaluation.", [
        "A recurring pain and reason to act are confirmed.",
        "The buyer/sponsor and approval path are identified.",
        "An internal owner will coordinate the evaluation.",
        "A credible budget path exists if the test succeeds.",
        "Scope is feasible; evaluation planning has a date.",
    ]),
]
card_height = max(c[2] for c in cards)
for x, width, _, ps in cards:
    C.setFillColor(colors.HexColor("#f5f8fc"))
    C.roundRect(x, top - card_height, width, card_height, 7, fill=1, stroke=0)
    C.setStrokeColor(LINE)
```

```
    C.roundRect(x, top - card_height, width, card_height, 7, fill=0, stroke=1)
    cursor = top - 14
    for p, height, gap in ps:
        cursor -= gap
        p.drawOn(C, x + 14, cursor - height)
        cursor -= height
top -= card_height
top = para("<b>Does not count:</b> a page visit, content download, unaccepted introduction or friendly
reply with no meaningful next step. Missing SQL criteria stay in discovery or nurture; they do not
become a forecasted sale.", MARGIN, top - 12, CONTENT, 9.8, 13.3)

top = section("05", "Keep qualification separate from evaluation", top - 20)
labels = [("Target account", "Fit + route in"), ("MQL", "Discovery next"), ("SQL", "Plan the
evaluation"), ("Evaluation-ready", "Scope + team + approvals"), ("First real test", "Hospital staff
test it")]
gap = 10
node_width = (CONTENT - gap * 4) / 5
for i, (label, detail) in enumerate(labels):
    x = MARGIN + i * (node_width + gap)
    C.setFillColor(PALE if i < 4 else colors.HexColor("#eaf5ef"))
    C.roundRect(x, top - 51, node_width, 51, 5, fill=1, stroke=0)
    para(label, x + 7, top - 9, node_width - 14, 8.8, 11, NAVY, "BodyBold")
    para(detail, x + 7, top - 26, node_width - 14, 8, 10.5, MUTED)
    if i < 4:
        ax = x + node_width + 1
        C.setStrokeColor(BLUE)
        C.line(ax, top - 25, ax + 7, top - 25)
        C.line(ax + 4, top - 28, ax + 7, top - 25)
        C.line(ax + 4, top - 22, ax + 7, top - 25)
top -= 64
top = para("A SQL is <b>not</b> a granted-access milestone or a paying customer. Agree evaluation
scope, hospital participants, permitted data, success criteria and dates before building. Count one
buying organization once, not each contact or each site in a centrally purchasing group.", MARGIN,
top, CONTENT, 9.8, 13.3)
top = para("<b>Current priority metric:</b> new hospital buying organizations outside the anchor
partner completing their first hands-on parallel-system evaluation session in the last 30 days. SQLs
feed this metric; they are not the metric itself.", MARGIN, top - 9, CONTENT, 9.8, 13.3)

top = section("06", "What we should agree together", top - 19)
top = para("<b>1.</b> Confirm the target segment and strongest buyer pain. <b>2.</b> Approve these
MQL/SQL gates and one follow-up owner. <b>3.</b> Finalize the free-evaluation scope, next-ten-client
price-match terms and development boundaries.", MARGIN, top, CONTENT, 10.1, 14)
top = para("<b>Basis:</b> the supplied Block 1 workshop slide, MediMind marketing work and founder
offer. Buyer pains and these qualification rules are working hypotheses to validate; no clinical or
financial uplift is claimed as proven.", MARGIN, top - 15, CONTENT, 8.6, 11.5, MUTED)
assert top > 48, f"Page 2 content exceeds safe area: {top}"
C.showPage()
C.save()
print(OUTPUT)
```

Source: Original files/Marketing workspace/workshop/build_funnel_metrics_brief.py. Historical wording preserved; not a new approval or factual revalidation.

build_funnel_metrics_brief.py

Technical appendix. This is an archived working file, not instructions to execute.

```
"""Render the Block 3 funnel, dashboard and metric definitions as a short PDF."""
from pathlib import Path

from reportlab.lib import colors
from reportlab.lib.pagesizes import A4
from reportlab.lib.styles import ParagraphStyle
from reportlab.pdfbase import pdfmetrics
from reportlab.pdfbase.ttfonts import TTFont
from reportlab.platypus import SimpleDocTemplate, Paragraph, Spacer, Table, TableStyle, PageBreak
from pypdf import PdfReader

ROOT = Path(__file__).resolve().parents[2]
OUTPUT = ROOT / "output/pdf/MediMind - Funnel Metrics and Tracking.pdf"
OUTPUT.parent.mkdir(parents=True, exist_ok=True)
FONT_DIR = Path("/System/Library/Fonts/Supplemental")
for name, filename in (("Body", "Arial.ttf"), ("BodyBold", "Arial Bold.ttf"), ("Display",
"Georgia.ttf")):
    pdfmetrics.registerFont(TTFont(name, str(FONT_DIR / filename)))
pdfmetrics.registerFontFamily("Body", normal="Body", bold="BodyBold")
NAVY = colors.HexColor("#1a365d")
BLUE = colors.HexColor("#2b6cb0")
INK = colors.HexColor("#233248")
MUTED = colors.HexColor("#53657a")
LINE = colors.HexColor("#d7e2ef")
PALE = colors.HexColor("#edf5fc")
GREEN = colors.HexColor("#eaf5ef")
W, H = A4
M = 42
WIDTH = W - 2 * M


def p(text, size=10, leading=13.6, color=INK, bold=False):
    return Paragraph(text, ParagraphStyle("p", fontName="BodyBold" if bold else "Body",
                     fontSize=size, leading=leading, textColor=color, spaceAfter=0))


def heading(text):
    return [Spacer(1, 11), p(text, 12.8, 16.5, NAVY, True), Spacer(1, 7)]


def card(title, body, fill=PALE):
    t = Table([[[p(title, 11.5, 15, NAVY, True), Spacer(1, 6), p(body)]]], colWidths=[WIDTH])
    t.setStyle(TableStyle([("BACKGROUND", (0, 0), (-1, -1), fill),
                           ("LEFTPADDING", (0, 0), (-1, -1), 12),
                           ("RIGHTPADDING", (0, 0), (-1, -1), 12),
                           ("TOPPADDING", (0, 0), (-1, -1), 10),
                           ("BOTTOMPADDING", (0, 0), (-1, -1), 10)]))
    return t


def table(headers, rows, widths, size=9.6, leading=12.7):
    data = [[p(x, size, leading, NAVY, True) for x in headers]]
    data += [[p(x, size, leading) for x in row] for row in rows]
    t = Table(data, colWidths=widths, hAlign="LEFT")
    t.setStyle(TableStyle([("BACKGROUND", (0, 0), (-1, 0), PALE),
                           ("VALIGN", (0, 0), (-1, -1), "TOP"),
```

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                           ("LEFTPADDING", (0, 0), (-1, -1), 8),
                           ("RIGHTPADDING", (0, 0), (-1, -1), 8),
                           ("TOPPADDING", (0, 0), (-1, -1), 5),
                           ("BOTTOMPADDING", (0, 0), (-1, -1), 5),
                           ("LINEBELOW", (0, 0), (-1, -1), .5, LINE)]))
    return t


TITLES = ["Funnel, metrics & tracking", "A dashboard that tells us what to do", "Track the work. Keep
the rules simple."]
SUBTITLES = ["From finding the right hospital to a real evaluation and a buying decision.",
             "A proposed structure - no live CRM data or performance figures are assumed.",
             "One shared tracker, milestone dates and a short weekly review."]


def decorate(c, doc):
    page = doc.page
    c.setTitle("MediMind | Funnel, Metrics & Tracking | Block 3")
    c.setAuthor("MediMind")
    c.setSubject("Hospital acquisition funnel, dashboard structure, KPI definitions and tracking
rules")
    c.setFillColor(NAVY)
    c.setFont("BodyBold", 17)
    c.drawString(M, H - 40, "MediMind")
    c.setFont("Body", 8.5)
    c.setFillColor(MUTED)
    c.drawRightString(W - M, H - 39, "BLOCK 3  /  02 OCTOBER 2026")
    c.setStrokeColor(LINE)
    c.line(M, H - 52, W - M, H - 52)
    c.setFont("Display", 22)
    c.setFillColor(NAVY)
    c.drawString(M, H - 86, TITLES[min(page - 1, 2)])
    c.setFont("Body", 9.4)
    c.setFillColor(MUTED)
    c.drawString(M, H - 107, SUBTITLES[min(page - 1, 2)])
    c.line(M, 39, W - M, 39)
    c.setFont("Body", 8)
    c.drawString(M, 25, "MediMind  |  Team working brief - definitions and targets, not reported
results")
    c.drawRightString(W - M, 25, f"{page} / 3")
    c.setFont("Body", 7)
    c.drawString(M, 14, "Basis: supplied Block 3 slide, Blocks 1-2 and the founder's parallel-
evaluation offer.")


story = []
story += heading("01  The funnel: each step needs evidence")
story.append(card("Find the fit → earn trust → prove the alternative", "<b>Acquire:</b> Target + route
in → MQL → SQL<br/><b>Prove:</b> Evaluation-ready → Parallel delivered → First hands-on test<br/>
<b>Convert:</b> Results review → Purchase agreement → Approved rollout"))
story.append(Spacer(1, 10))
story.append(table(["Stage", "What must be true before recording it", "Owner"], [
    ["<b>1. Target + route in</b>", "Hospital/group fits our focus. Map buyer, possible connector and
contact route; direct outreach is also valid.", "Founder / researcher"],
    ["<b>2. Engaged / MQL</b>", "A relevant contact responds meaningfully and agrees to discovery or
another dated next step.", "Founder"],
    ["<b>3. Qualified / SQL</b>", "Discovery confirms pain, reason to act, sponsor, approval/budget
path, internal owner and feasible evaluation scope; next step dated.", "Founder"],
    ["<b>4. Evaluation-ready</b>", "Scope, testers, baseline, success criteria and dates agreed.
Required data/security approvals complete and approved data access works.", "Hospital owner + IT +
MediMind"],
    ["<b>5. Parallel delivered</b>", "Agreed scope passes internal checks. Test access and
instructions reach the named hospital evaluators; existing system stays in place.", "MediMind delivery
lead"],
    ["<b>6. First hands-on test</b>", "Hospital staff complete an agreed workflow in the parallel
system and feedback is recorded. A founder-only demo does not count.", "Hospital champion + founder"],
    ["<b>7. Results reviewed</b>", "Compare against the agreed baseline; record gaps and a dated buy,
revise or stop decision. A first test is not a passed evaluation.", "Buyer + evaluators"],
    ["<b>8. Purchase agreed</b>", "Authorized purchase agreement is signed, with scope, price and
```
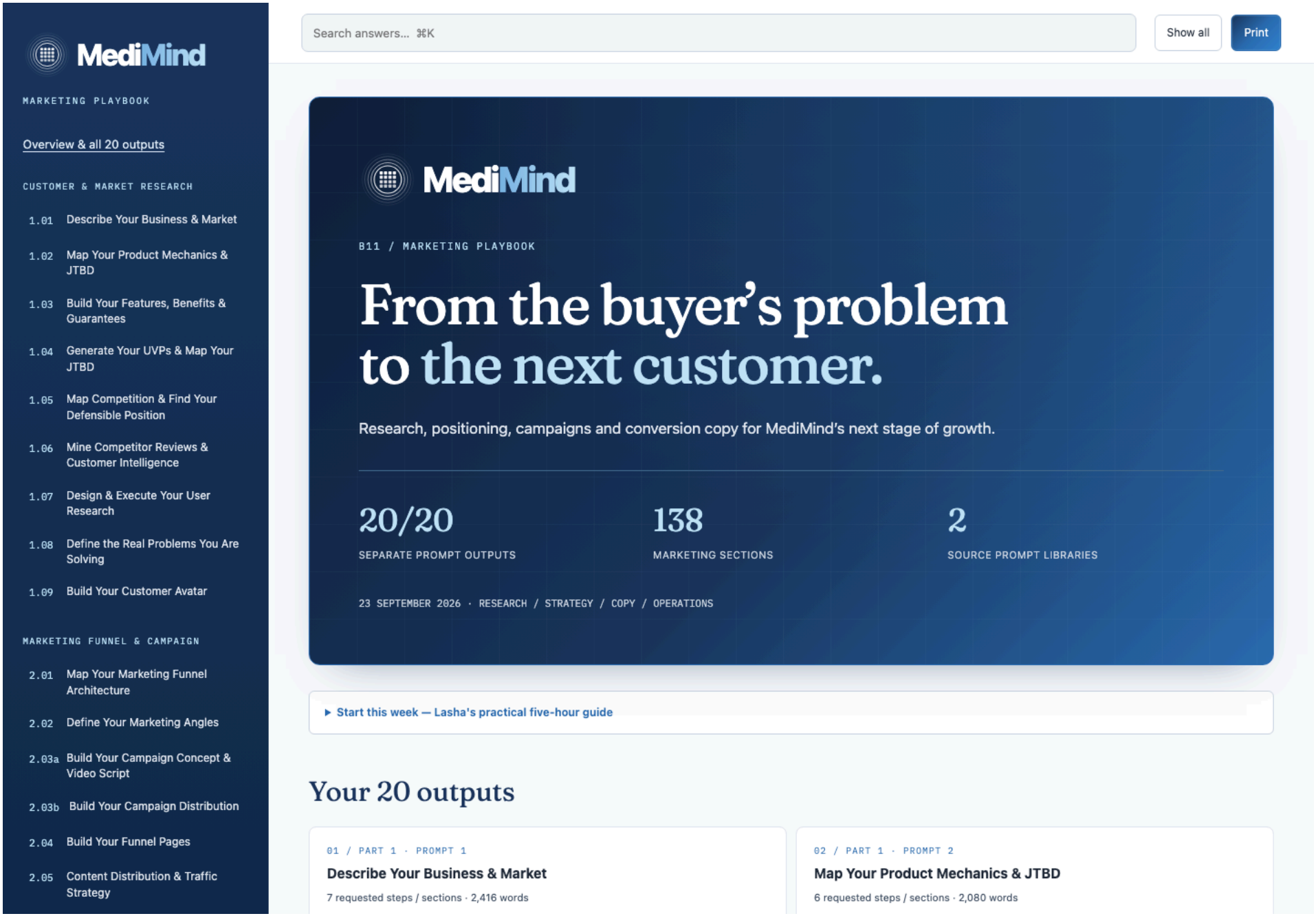

```
rollout terms. Invoice/payment is tracked separately.", "Buyer + founder"],
    ["<b>9. Approved rollout</b>", "Hospital accepts the go-live plan and agreed scope goes live.
Training, support and cutover are handled as a separate delivery plan.", "Hospital + delivery lead"],
], [118, WIDTH - 208, 90]))
story.append(Spacer(1, 10))
story.append(p("<b>MQL</b> = marketing-qualified lead. <b>SQL</b> = sales-qualified lead. A connector
opens the door; a champion organizes colleagues; a buyer approves spending. They need not be the same
person.", 9.4, 12.7))
story.append(Spacer(1, 9))
story.append(p("<b>Branches:</b> no fit → closed with reason; not now → nurture with a review date;
test gaps → revise and retest, or stop. Retests never create another first-test count. An introduction
alone is not an MQL.", 9.4, 12.7))
story.append(PageBreak())

story += heading("02  The one metric that matters right now")
story.append(card("New hospitals completing their first real parallel-system test", "Count distinct
new <b>hospital buying organizations outside the anchor partner</b> whose first hands-on test happened
in the last 30 days.<br/>Why this one: it shows a hospital moved beyond interest, our team delivered,
and its staff actually tried the alternative. It does <b>not</b> prove clinical benefit, a successful
full evaluation or a sale.", GREEN))
story += heading("03  Dashboard layout - review every Monday")
tiles = []
for title, subtitle in [("FIRST REAL TESTS", "Last 30 days - main metric"),
                        ("EVALUATION-READY", "First became ready in last 30 days"),
                        ("PARALLEL DELIVERED", "First delivery in last 30 days"),
                        ("OVERDUE ACTIONS", "Open accounts needing attention now")]:
    tiles.append([p(title, 9, 12, NAVY, True), Spacer(1, 4), p("Not recorded", 15, 19, BLUE, True),
p(subtitle, 8.5, 11.5, MUTED)])
grid = Table([[tiles[0], tiles[1]], [tiles[2], tiles[3]]], colWidths=[WIDTH / 2, WIDTH / 2])
grid.setStyle(TableStyle([("BACKGROUND", (0, 0), (-1, -1), PALE),
                          ("VALIGN", (0, 0), (-1, -1), "TOP"),
                          ("BOX", (0, 0), (-1, -1), 1, colors.white),
                          ("INNERGRID", (0, 0), (-1, -1), 5, colors.white),
                          ("LEFTPADDING", (0, 0), (-1, -1), 11),
                          ("RIGHTPADDING", (0, 0), (-1, -1), 11),
                          ("TOPPADDING", (0, 0), (-1, -1), 9),
                          ("BOTTOMPADDING", (0, 0), (-1, -1), 9)]))
story.append(grid)
story.append(Spacer(1, 8))
story.append(p("<b>Below the cards:</b> funnel by current stage → stuck-account list (blocker, owner,
next action/date) → outcomes (reviewed, revised, stopped, signed). Show last-updated time. Unknown is
not zero.", 9.6, 13))
story += heading("04  KPI definitions - tie every number to a stage")
story.append(table(["KPI / stage", "Definition and window", "Decision it supports"], [
    ["<b>First real tests</b><br/>Stage 6", "Distinct eligible organizations with first-test date in
the rolling 30-day window.", "Are we winning real evaluations?"],
    ["<b>New SQLs</b><br/>Stage 3", "Distinct organizations first meeting every SQL criterion in the
last 30 days.", "Is outreach finding a credible fit?"],
    ["<b>Evaluation-ready</b><br/>Stage 4", "Distinct organizations clearing all readiness gates in
the last 30 days; retain data-access date separately.", "Are hospital-side approvals moving?"],
    ["<b>Parallel delivered</b><br/>Stage 5", "Distinct first deliveries in the last 30 days. Also
report median calendar days from readiness to delivery; list open-build ages separately.", "Are we
keeping our side of the offer?"],
    ["<b>7-day test uptake</b><br/>Stage 5 → 6", "Of first deliveries at least 7 days old, % first
tested within 7 calendar days. Use a named delivery cohort; show n/N.", "Are hospital staff actually
trying it?"],
    ["<b>Overdue actions</b><br/>Any open stage", "Open organizations whose next-action date has
passed. Separately flag missing dates/owners. Snapshot, not a 30-day total.", "What needs attention
this week?"],
    ["<b>Reviewed / signed</b><br/>Stages 7 / 8", "Separate counts of first results reviews and signed
purchases in the last 30 days. Signed value is not cash collected.", "Do evaluations lead to a
decision?"],
], [110, 277, WIDTH - 387], size=9.3, leading=12.4))
story.append(PageBreak())

story += heading("05  One shared tracker is enough to start")
story.append(p("Use one row and stable ID per <b>buying organization</b>, not per contact or demo.
Centrally purchased sites count together. Keep the anchor partner and its expansion separate."))
story.append(Spacer(1, 9))
```

```
story.append(table(["Keep these fields", "What goes in them"], [
    ["<b>Account + route</b>", "ID; hospital/group; source (warm intro, direct outreach or inbound);
connector; buyer; champion; internal owner; anchor/existing flag."],
    ["<b>Current work</b>", "Current stage; active/nurture/closed status; pain and fit evidence;
blocker; next action + due date; responsible person."],
    ["<b>First milestone dates</b>", "MQL; SQL; approved data access verified; evaluation-ready;
delivery; first hands-on test; first results review; signed agreement; go-live."],
    ["<b>Evidence + decision</b>", "Link to call note, approval, delivery checklist or test feedback.
Record baseline, agreed success criteria, review outcome and reason for loss/delay."],
], [137, WIDTH - 137]))
story.append(Spacer(1, 9))
story.append(p("<b>Hospital commitment:</b> verify approved read-only backup access or the agreed data
route works. Access alone is <b>not</b> readiness or evaluation. Store approval references, never
credentials or patient data, in this tracker.", 9.7, 13.2))
story += heading("06  Counting rules that prevent misleading numbers")
story.append(p("<b>Dates:</b> latest 30 calendar dates, including today, in Asia/Tbilisi; timestamp
the report. Count each first milestone once; preserve dates when accounts pause.<br/><b>Do not mix:
</b> current-stage counts are a snapshot; milestone counts measure progress during a period.<br/>
<b>Conversion:</b> use deliveries from the same named week, not unrelated monthly totals. Once every
delivery has had 7 days, calculate tests within 7 days / all deliveries in that group. Show n/N, %,
and cohort dates. Zero denominator = N/A; immature groups = pending.", 9.8, 13.5))
story += heading("07  Timing: start the clock when the hospital is ready")
story.append(card("Proposed working targets — not a universal launch guarantee", "<b>Ready →
delivered:</b> aim for 14 calendar days after all readiness gates pass and scope/feasibility are
confirmed.<br/><b>Delivered → first hospital test:</b> aim for the next 7 calendar days. Book the
session before delivery.<br/><b>Combined goal:</b> first real test within 21 days of readiness. Track
elapsed time and blockers; never rush approvals to meet the date."))
story.append(Spacer(1, 9))
story.append(p("<b>Block 2's first-month targets:</b> 4 held calls, 2 SQLs, 1 ready hospital — not
reported results. A first test may fall next month. Establish a baseline before setting a recurring
volume target.", 9.6, 13))
story += heading("Weekly review: 15 minutes, three decisions")
story.append(p("<b>Founder:</b> update conversations and next steps. <b>Delivery lead:</b> verify
readiness, delivery and test evidence. <b>Tracker owner:</b> refresh the dashboard.<br/>Ask: Where did
progress stop? Who owns the next action and date? What did hospital testing prove — and what still
needs work? Record one corrective action for the biggest bottleneck.", 9.7, 13.2))

doc = SimpleDocTemplate(str(OUTPUT), pagesize=A4, leftMargin=M, rightMargin=M,
                        topMargin=120, bottomMargin=51)
doc.build(story, onFirstPage=decorate, onLaterPages=decorate)
reader = PdfReader(OUTPUT)
assert len(reader.pages) == 3, f"Expected 3 pages, got {len(reader.pages)}. Adjust layout before
publication."
assert all(page.extract_text().strip() for page in reader.pages)
print(OUTPUT)
```

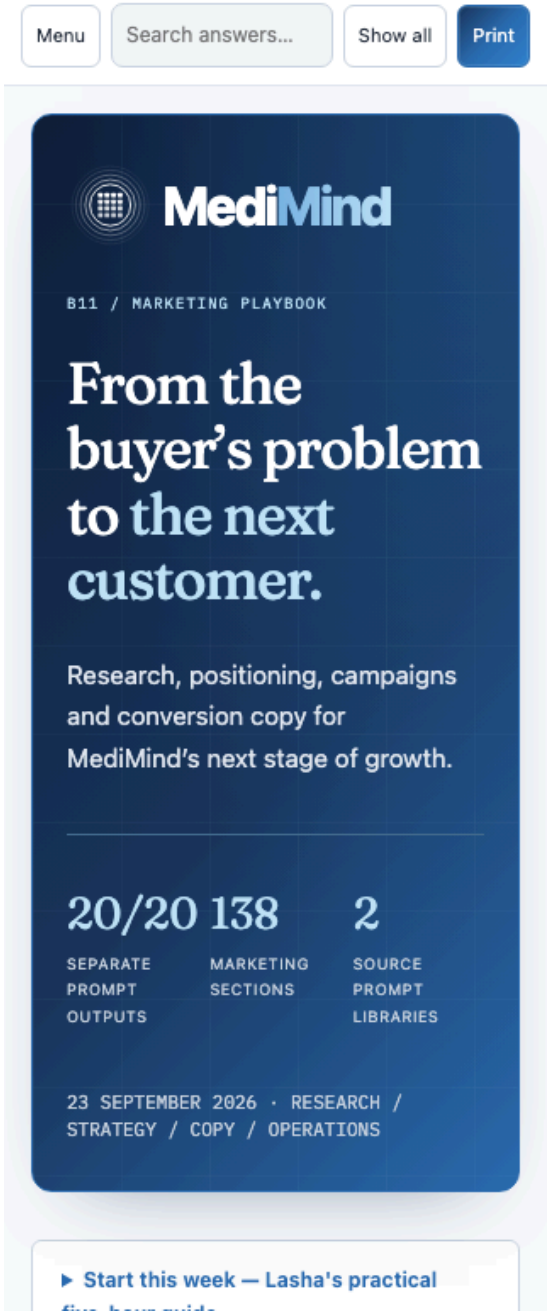
Archived visual QA 1/31

Original files/Marketing workspace/prompt-library/complete-edition/working/qa/brand-desktop.png



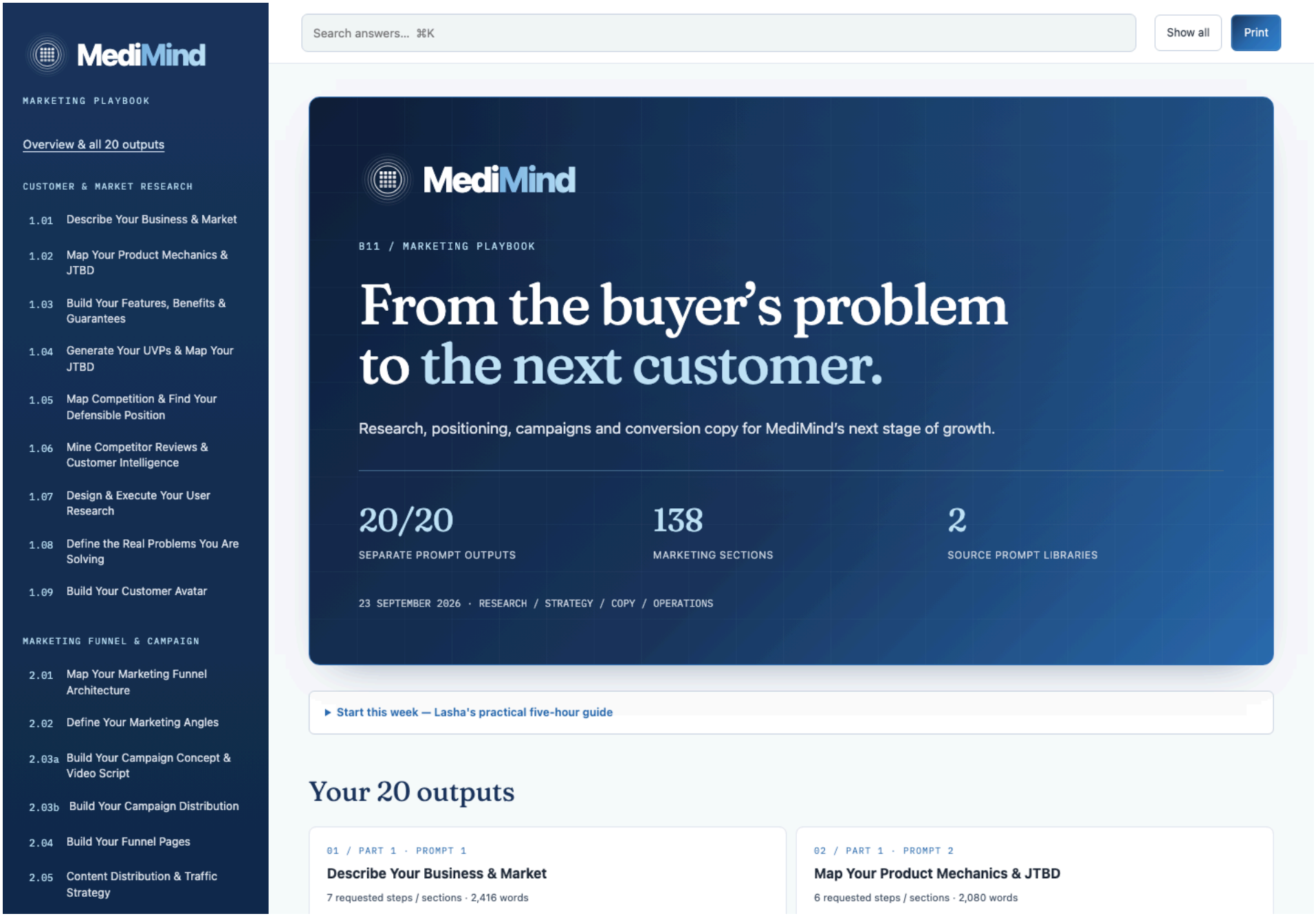
Archived visual QA 2/31

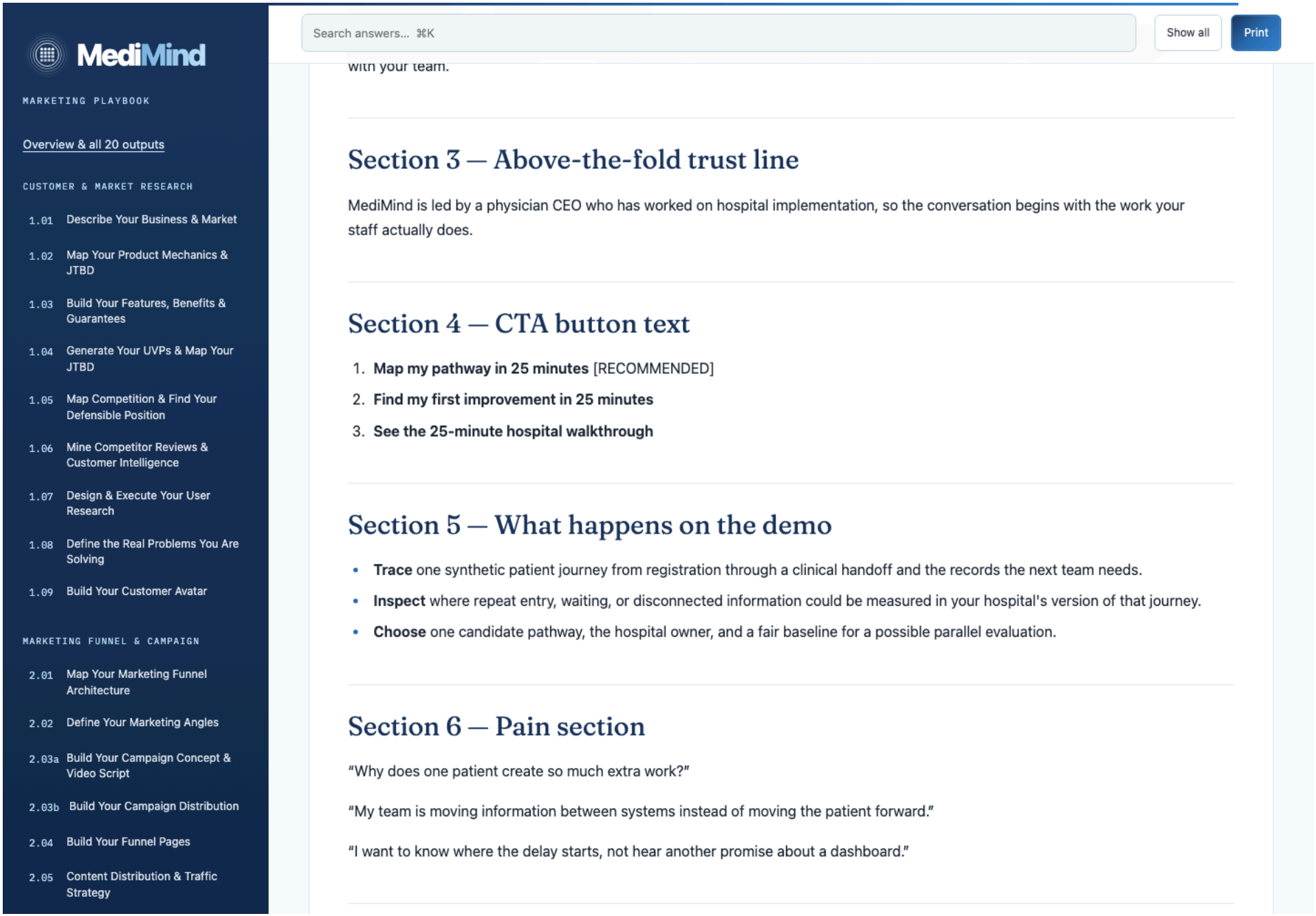
Original files/Marketing workspace/prompt-library/complete-edition/working/qa/brand-mobile-320.png



Archived visual QA 3/31

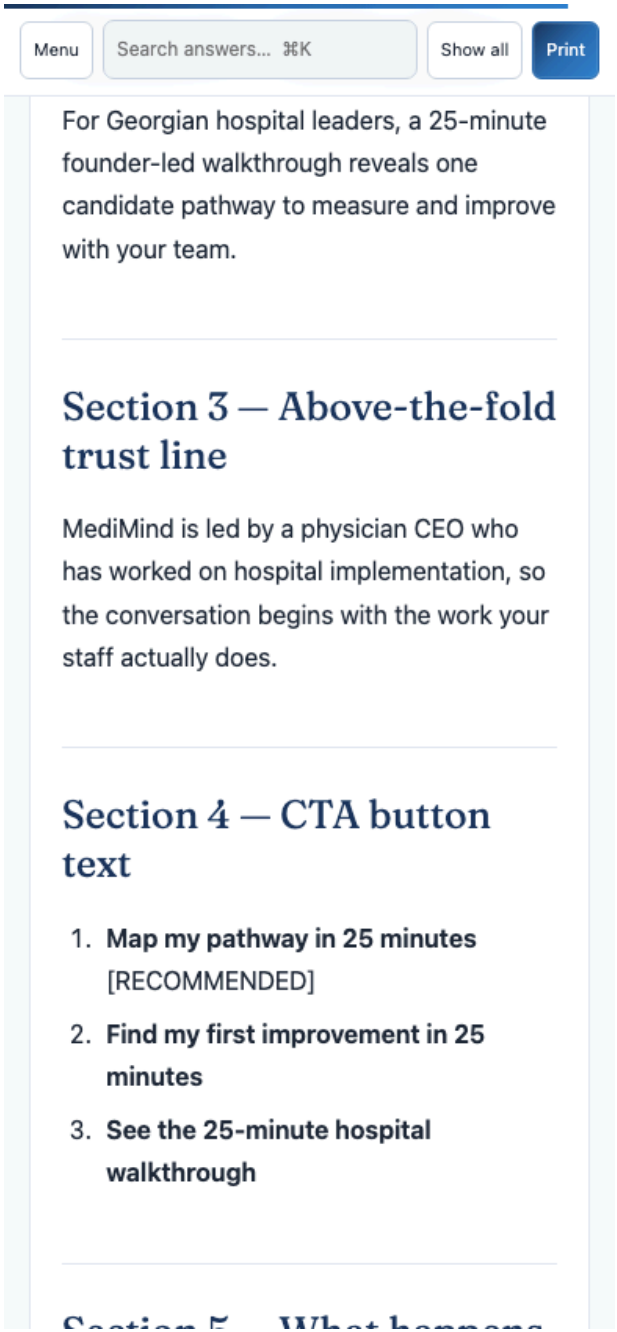
Original files/Marketing workspace/prompt-library/complete-edition/working/qa/desktop-overview.png




MediMind
MARKETING PLAYBOOK
Overview & all 20 outputs
CUSTOMER & MARKET RESEARCH
1.01 Describe Your Business & Market
1.02 Map Your Product Mechanics & JTBD
1.03 Build Your Features, Benefits & Guarantees
1.04 Generate Your UVPs & Map Your JTBD
1.05 Map Competition & Find Your Defensible Position
1.06 Mine Competitor Reviews & Customer Intelligence
1.07 Design & Execute Your User Research
1.08 Define the Real Problems You Are Solving
1.09 Build Your Customer Avatar
MARKETING FUNNEL & CAMPAIGN
2.01 Map Your Marketing Funnel Architecture
2.02 Define Your Marketing Angles
2.03a Build Your Campaign Concept & Video Script
2.03b Build Your Campaign Distribution
2.04 Build Your Funnel Pages
2.05 Content Distribution & Traffic Strategy
Search answers... ⌘K
Show all
Print
with your team.
Section 3 — Above-the-fold trust line
MediMind is led by a physician CEO who has worked on hospital implementation, so the conversation begins with the work your staff actually does.
Section 4 — CTA button text
1. Map my pathway in 25 minutes [RECOMMENDED]
2. Find my first improvement in 25 minutes
3. See the 25-minute hospital walkthrough
Section 5 — What happens on the demo
• Trace one synthetic patient journey from registration through a clinical handoff and the records the next team needs.
• Inspect where repeat entry, waiting, or disconnected information could be measured in your hospital's version of that journey.
• Choose one candidate pathway, the hospital owner, and a fair baseline for a possible parallel evaluation.
Section 6 — Pain section
"Why does one patient create so much extra work?"
"My team is moving information between systems instead of moving the patient forward."
"I want to know where the delay starts, not hear another promise about a dashboard."

Archived visual QA 5/31

Original files/Marketing workspace/prompt-library/complete-edition/working/qa/l0-01-mobile.png



2. **Send me the patient-pathway session details**
3. **Reserve my seat for the handoff workshop**

Section 5 — Above-the-fold trust line

Led by MediMind physician CEO Lasha Khoshtaria, who brings a clinician's view to hospital implementation questions.

Section 6 — Speaker block

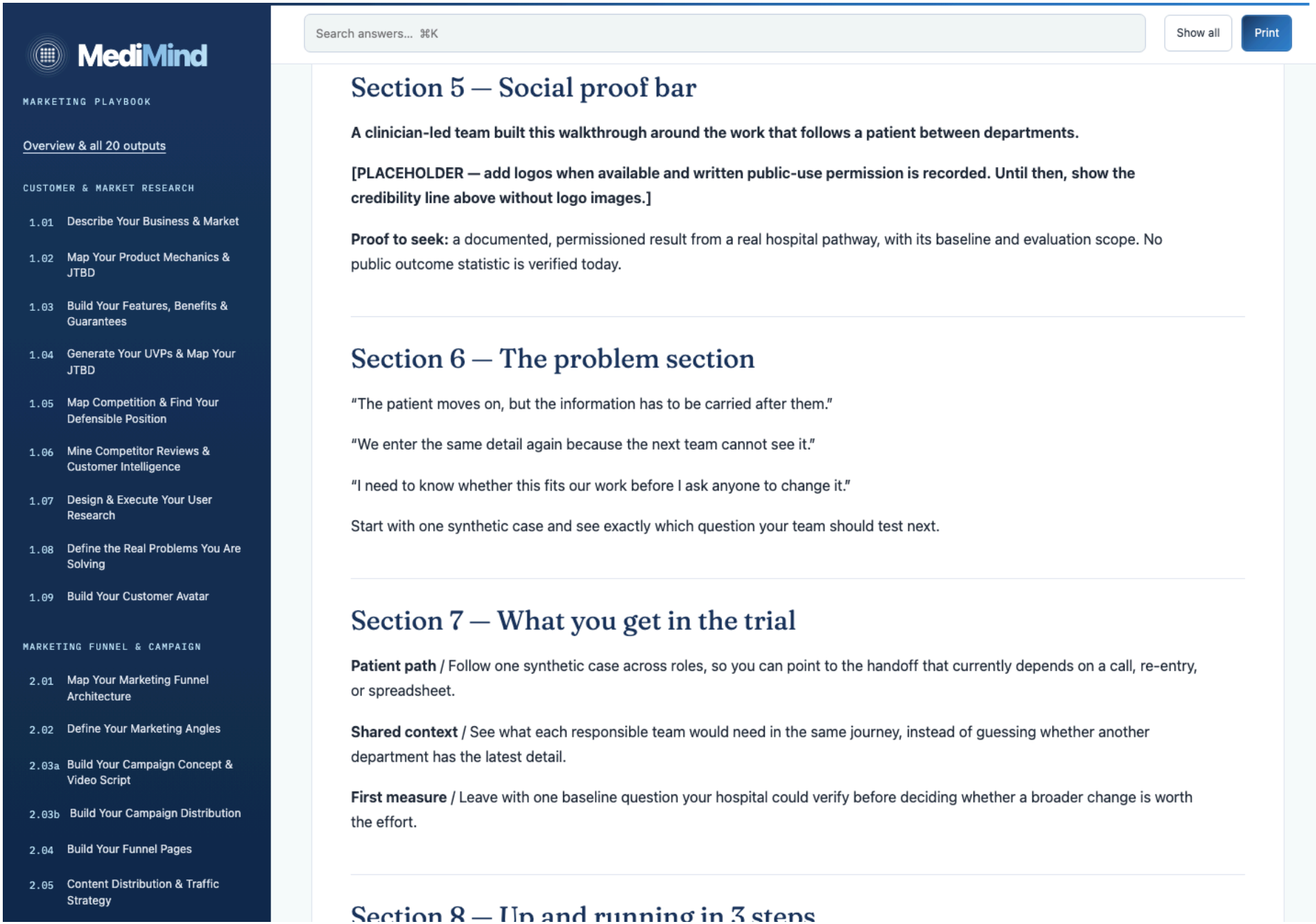
Lasha Khoshtaria — physician CEO, MediMind. Lasha brings a physician's view of the patient journey to the operational questions that follow it through a hospital. As a founder involved in hospital implementation, he can show how to turn a messy handoff into a specific workflow question that clinical, operations, and IT leaders can test together.

Relevant authority: physician and CEO of the company building the hospital operating system discussed in the session; no external affiliation is claimed.

Why this speaker: he can connect the clinical moment to the implementation choices the hospital would actually have to make, and distinguish a useful demonstration from a measured result.

Section 7 — What you will learn

- **Draw** a four-column map of one patient journey: action, responsible team, information needed, and where the handoff stalls.
- **Spot** repeated entry or reconciliation that looks small in each department but grows along the whole path.
- **Choose** a baseline measure, such as entries repeated per case or minutes from one handoff to the next, that your team can observe without claiming a patient outcome.
- **Set** a narrow evaluation boundary: the case type, owners, readiness checks, and evidence needed before considering a production change.


MediMind
MARKETING PLAYBOOK
Overview & all 20 outputs
CUSTOMER & MARKET RESEARCH
1.01 Describe Your Business & Market
1.02 Map Your Product Mechanics & JTBD
1.03 Build Your Features, Benefits & Guarantees
1.04 Generate Your UVPs & Map Your JTBD
1.05 Map Competition & Find Your Defensible Position
1.06 Mine Competitor Reviews & Customer Intelligence
1.07 Design & Execute Your User Research
1.08 Define the Real Problems You Are Solving
1.09 Build Your Customer Avatar
MARKETING FUNNEL & CAMPAIGN
2.01 Map Your Marketing Funnel Architecture
2.02 Define Your Marketing Angles
2.03a Build Your Campaign Concept & Video Script
2.03b Build Your Campaign Distribution
2.04 Build Your Funnel Pages
2.05 Content Distribution & Traffic Strategy
Search answers... ⌘K
Show all
Print
Section 5 — Social proof bar
A clinician-led team built this walkthrough around the work that follows a patient between departments.
[PLACEHOLDER — add logos when available and written public-use permission is recorded. Until then, show the credibility line above without logo images.]
Proof to seek: a documented, permissioned result from a real hospital pathway, with its baseline and evaluation scope. No public outcome statistic is verified today.
Section 6 — The problem section
"The patient moves on, but the information has to be carried after them."
"We enter the same detail again because the next team cannot see it."
"I need to know whether this fits our work before I ask anyone to change it."
Start with one synthetic case and see exactly which question your team should test next.
Section 7 — What you get in the trial
Patient path / Follow one synthetic case across roles, so you can point to the handoff that currently depends on a call, re-entry, or spreadsheet.
Shared context / See what each responsible team would need in the same journey, instead of guessing whether another department has the latest detail.
First measure / Leave with one baseline question your hospital could verify before deciding whether a broader change is worth the effort.
Section 8 — Up and running in 3 steps

Archived visual QA 8/31

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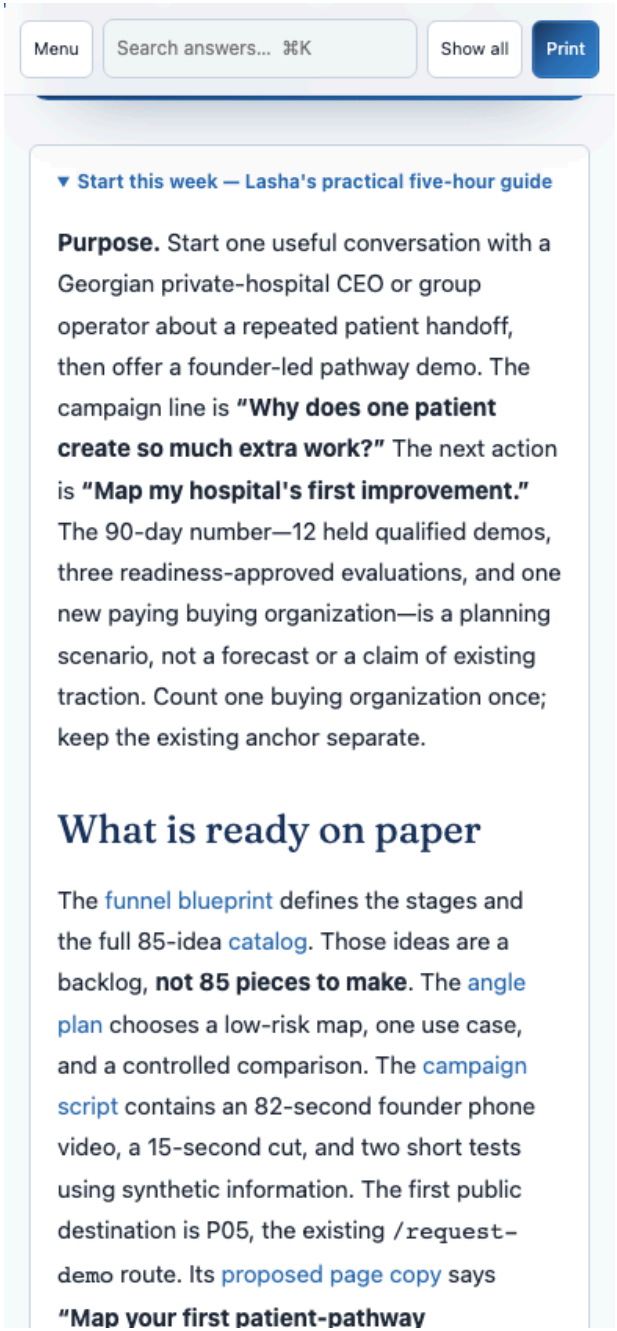
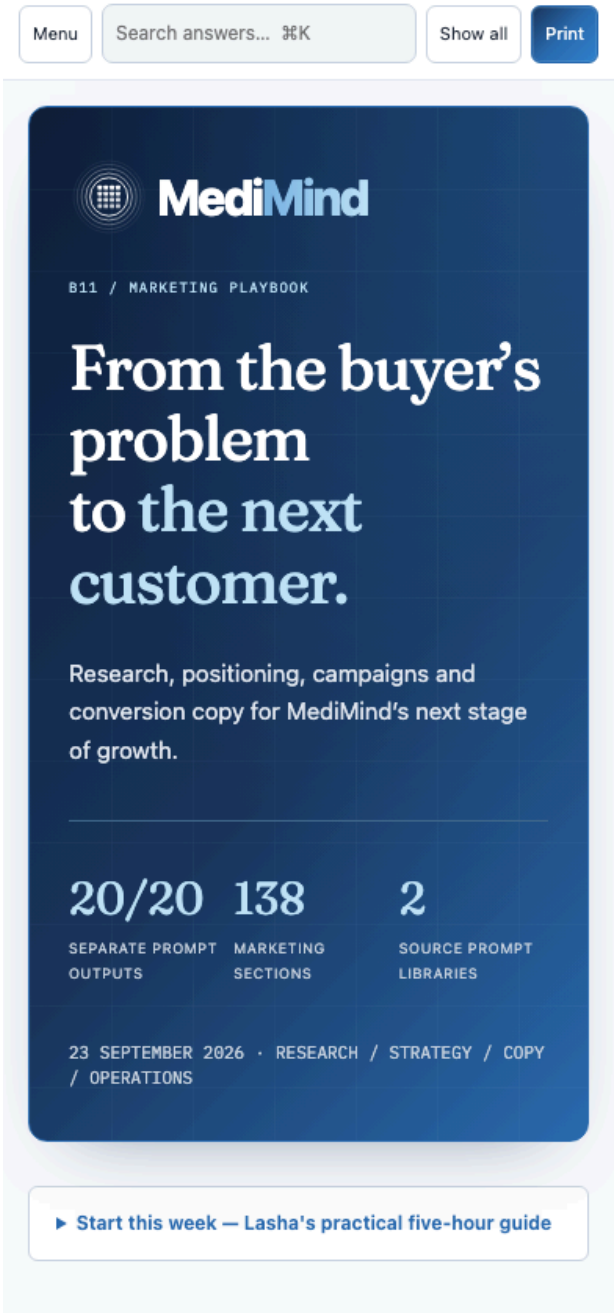


Archived visual QA 9/31

Original files/Marketing workspace/prompt-library/complete-edition/working/qa/mobile-overview.png



MediMind

MARKETING PLAYBOOK

Overview & all 20 outputs

CUSTOMER & MARKET RESEARCH

1.01 Describe Your Business & Market

1.02 Map Your Product Mechanics & JTBD

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1.04 Generate Your UVPs & Map Your JTBD

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MARKETING FUNNEL & CAMPAIGN

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2.03a Build Your Campaign Concept & Video Script

2.03b Build Your Campaign Distribution

2.04 Build Your Funnel Pages

2.05 Content Distribution & Traffic Strategy

Step 4 — Market sizing I can actually use

The defensible starting count is **232 hospitals and about 15,200 hospital beds in Georgia in 2025**, reported by Geostat in July 2026. This counts facilities, not purchasing organizations, qualified prospects, or buyers currently searching. It cannot be multiplied by a guessed annual contract value to produce a useful near-term market number.

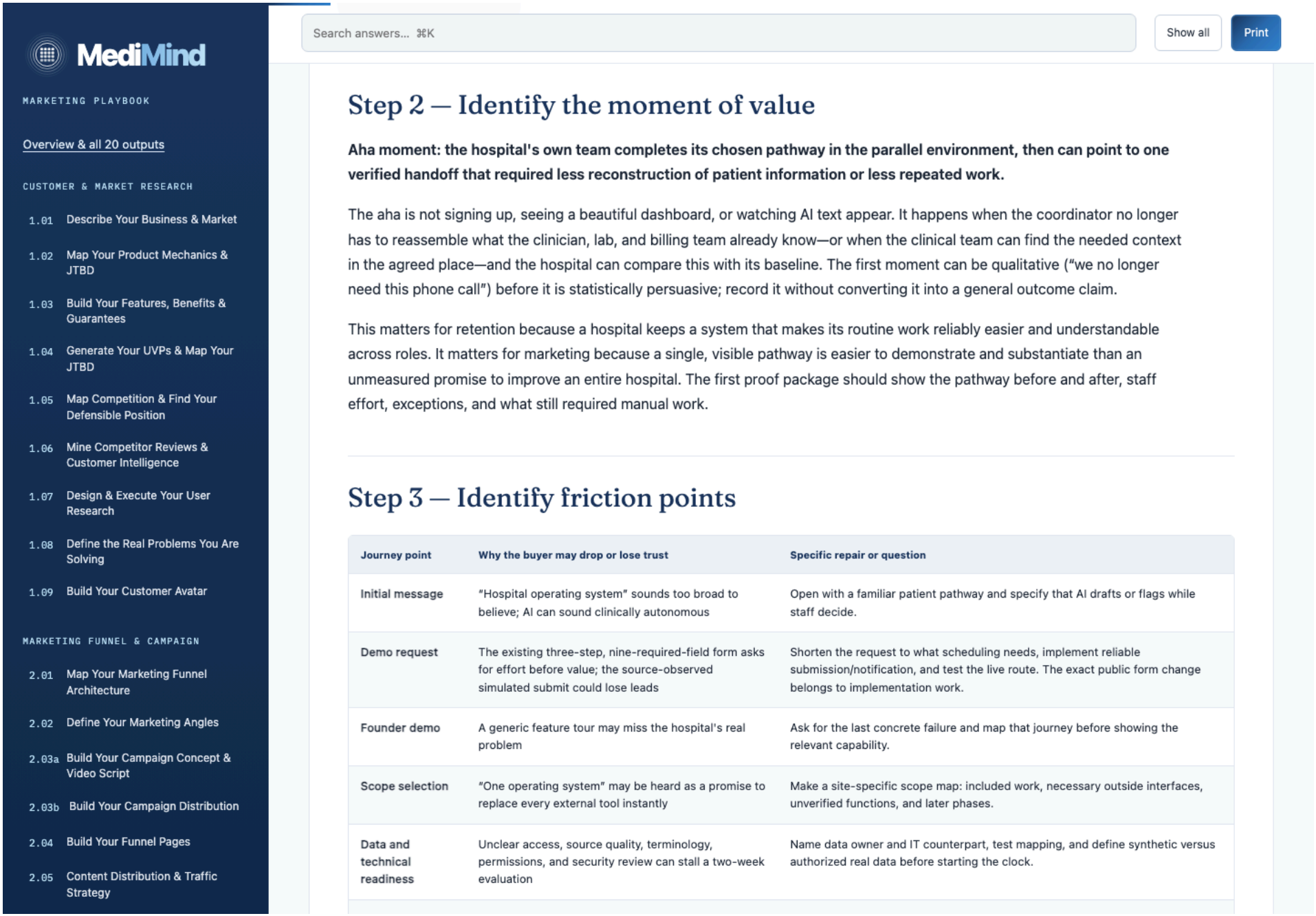
Build the reachable market from identifiable organizations, one decision unit at a time:

Filter	How to verify it	Count now
Georgian hospital facilities	Official annual hospital statistics	232
Distinct organizations that buy hospital software	Deduplicate ownership and purchasing authority, including chains	Unknown
Private, multi-profile organizations whose operational complexity fits the product	Review actual service mix and workflow ownership	Unknown
Reachable through a lawful, named route to a decision-maker	Record introduction, contact route, and last verification date	Unknown
Actively seeking a solution	Record recent trigger, alternatives, budget route, owner, and timeframe from a real buyer	Unknown
Able to evaluate during MediMind's available delivery window	Confirm access, counterpart staff, data readiness, scope, and delivery capacity	Unknown

Direct answer: A verified count of reachable businesses actively looking for a solution is unavailable. The answer is not 232 and not zero. Treat the number as a research output, not a slide assumption.

A useful first pass is a **15-organization research list**, including the anchor separately, and **8–10 conversations across at least four independent organizations**. These are proposed discovery targets, not an estimate of live demand. Publicly findable candidates in the prior research include Georgia Healthcare Group/Georgia Capital's healthcare operations, Aversi Clinic, and Vivo Medical Group; their existence does not imply interest or access. GeoHospitals belongs in an anchor/reference column, not the new-customer pipeline. A historical HelsiCore mention needs reconfirmation before it is treated as an active opportunity.

Named organization for the first research list	Why it is findable	Demand and reachability status
GeoHospitals	Founder-reported signed anchor agreement	Existing relationship; exclude from new-buyer count and verify approved reference scope



Step 2 — Identify the moment of value

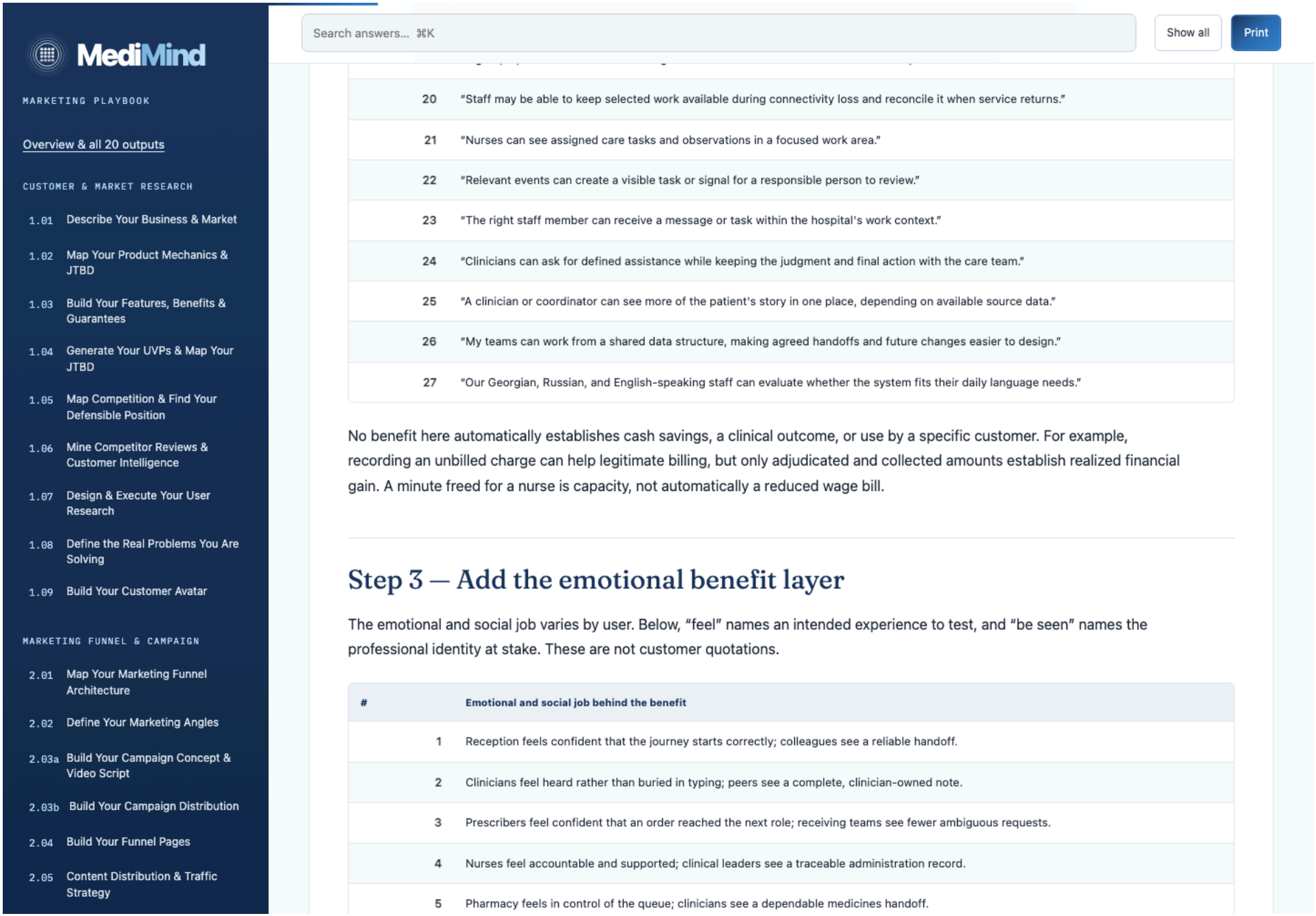
Aha moment: the hospital's own team completes its chosen pathway in the parallel environment, then can point to one verified handoff that required less reconstruction of patient information or less repeated work.

The aha is not signing up, seeing a beautiful dashboard, or watching AI text appear. It happens when the coordinator no longer has to reassemble what the clinician, lab, and billing team already know—or when the clinical team can find the needed context in the agreed place—and the hospital can compare this with its baseline. The first moment can be qualitative ("we no longer need this phone call") before it is statistically persuasive; record it without converting it into a general outcome claim.

This matters for retention because a hospital keeps a system that makes its routine work reliably easier and understandable across roles. It matters for marketing because a single, visible pathway is easier to demonstrate and substantiate than an unmeasured promise to improve an entire hospital. The first proof package should show the pathway before and after, staff effort, exceptions, and what still required manual work.

Step 3 — Identify friction points

Journey point	Why the buyer may drop or lose trust	Specific repair or question
Initial message	"Hospital operating system" sounds too broad to believe; AI can sound clinically autonomous	Open with a familiar patient pathway and specify that AI drafts or flags while staff decide.
Demo request	The existing three-step, nine-required-field form asks for effort before value; the source-observed simulated submit could lose leads	Shorten the request to what scheduling needs, implement reliable submission/notification, and test the live route. The exact public form change belongs to implementation work.
Founder demo	A generic feature tour may miss the hospital's real problem	Ask for the last concrete failure and map that journey before showing the relevant capability.
Scope selection	"One operating system" may be heard as a promise to replace every external tool instantly	Make a site-specific scope map: included work, necessary outside interfaces, unverified functions, and later phases.
Data and technical readiness	Unclear access, source quality, terminology, permissions, and security review can stall a two-week evaluation	Name data owner and IT counterpart, test mapping, and define synthetic versus authorized real data before starting the clock.



20	"Staff may be able to keep selected work available during connectivity loss and reconcile it when service returns."
21	"Nurses can see assigned care tasks and observations in a focused work area."
22	"Relevant events can create a visible task or signal for a responsible person to review."
23	"The right staff member can receive a message or task within the hospital's work context."
24	"Clinicians can ask for defined assistance while keeping the judgment and final action with the care team."
25	"A clinician or coordinator can see more of the patient's story in one place, depending on available source data."
26	"My teams can work from a shared data structure, making agreed handoffs and future changes easier to design."
27	"Our Georgian, Russian, and English-speaking staff can evaluate whether the system fits their daily language needs."

No benefit here automatically establishes cash savings, a clinical outcome, or use by a specific customer. For example, recording an unbilled charge can help legitimate billing, but only adjudicated and collected amounts establish realized financial gain. A minute freed for a nurse is capacity, not automatically a reduced wage bill.

Step 3 — Add the emotional benefit layer

The emotional and social job varies by user. Below, "feel" names an intended experience to test, and "be seen" names the professional identity at stake. These are not customer quotations.

#	Emotional and social job behind the benefit
1	Reception feels confident that the journey starts correctly; colleagues see a reliable handoff.
2	Clinicians feel heard rather than buried in typing; peers see a complete, clinician-owned note.
3	Prescribers feel confident that an order reached the next role; receiving teams see fewer ambiguous requests.
4	Nurses feel accountable and supported; clinical leaders see a traceable administration record.
5	Pharmacy feels in control of the queue; clinicians see a dependable medicines handoff.

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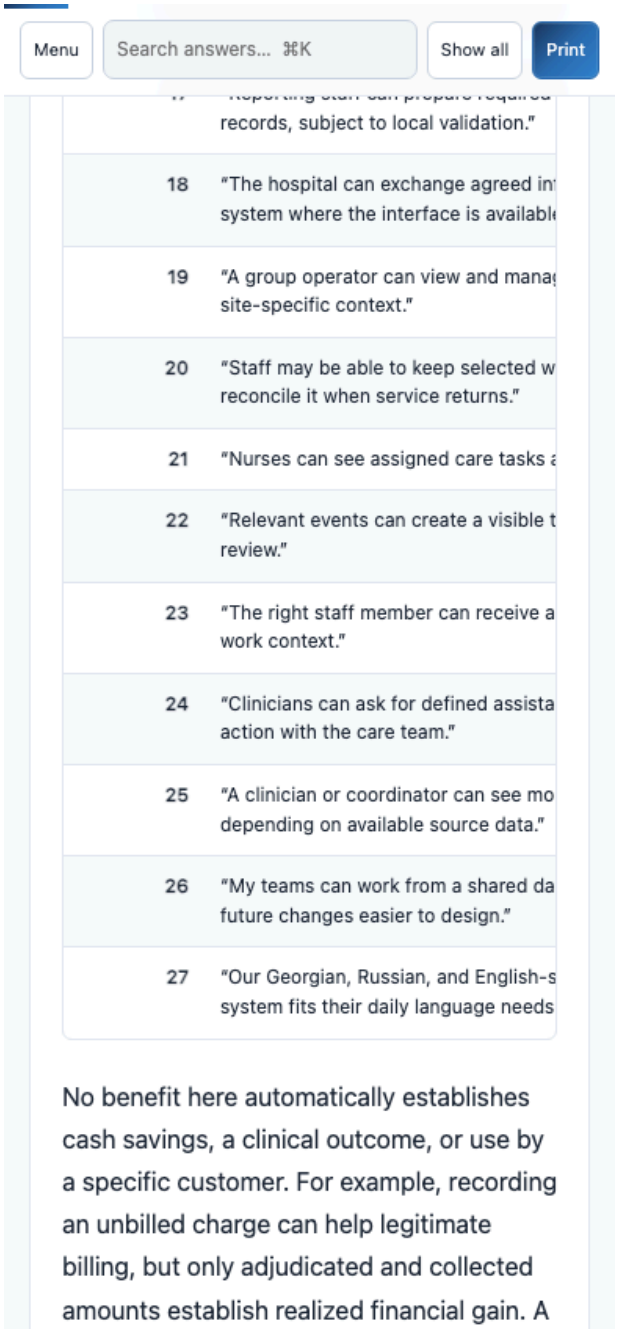

copy should invite firsthand evaluation early.

The right language shows a concrete mechanism and a controlled next step: **one patient → too many repeated handoffs → map the hospital's version → compare a connected path beside current work → decide from evidence.** Use the operating-system category, then plain language about the work people do. The FHIR R4 foundation can substantiate why data can share a structure, but should not replace the buyer's problem in the headline. The offer—matched current software price for eligible early partners, continuing development, three-business-day test target, and a readiness-dependent two-week parallel evaluation—supports the mechanism. These terms need written definitions. AI remains assistance under clinician review, with broader coordination a product direction.

Step 3 — Generate 10 UVPs as brand promises

Every line below speaks directly to the buyer about a valuable change. Wording that says “can” or “see whether” is intentional where performance has not yet been measured. Some lines suit a specific pathway or buying situation better than a universal homepage headline.

1. **See where one patient's journey creates repeat work—and test a more connected path with your own team.** This is the broad promise and the clearest link to the campaign idea.
2. **Judge a tailored hospital pathway beside your current system before you decide to switch.** This answers the most serious buying fear with the founder's proposed parallel evaluation.
3. **Give your teams one clearer patient story to work from across care, coordination, and business tasks.** This is the core connected-context promise, subject to available source data.
4. **Put your next hospital improvement into your team's hands to test while the need is still urgent.** This draws on the three-business-day request target; it does not promise clinical production in three days.
5. **Keep improving the system with your hospital instead of negotiating a separate development project for every need.** This expresses included continuing development; capacity and request scope require written terms.
6. **Start improving hospital work without first increasing the software price you already pay.** This speaks to eligible early partners under the founder's price-match offer, not total cost or a lifetime price lock.
7. **See the same agreed operating measures across your hospitals, with room for each site's work to differ.** This is the group-operator promise; data definitions and site access must be verified.
8. **Help clinicians spend less effort reconstructing the patient story before they make a decision.** This is a process hypothesis, not a claim of improved clinical outcomes.

Archived visual QA 15/31

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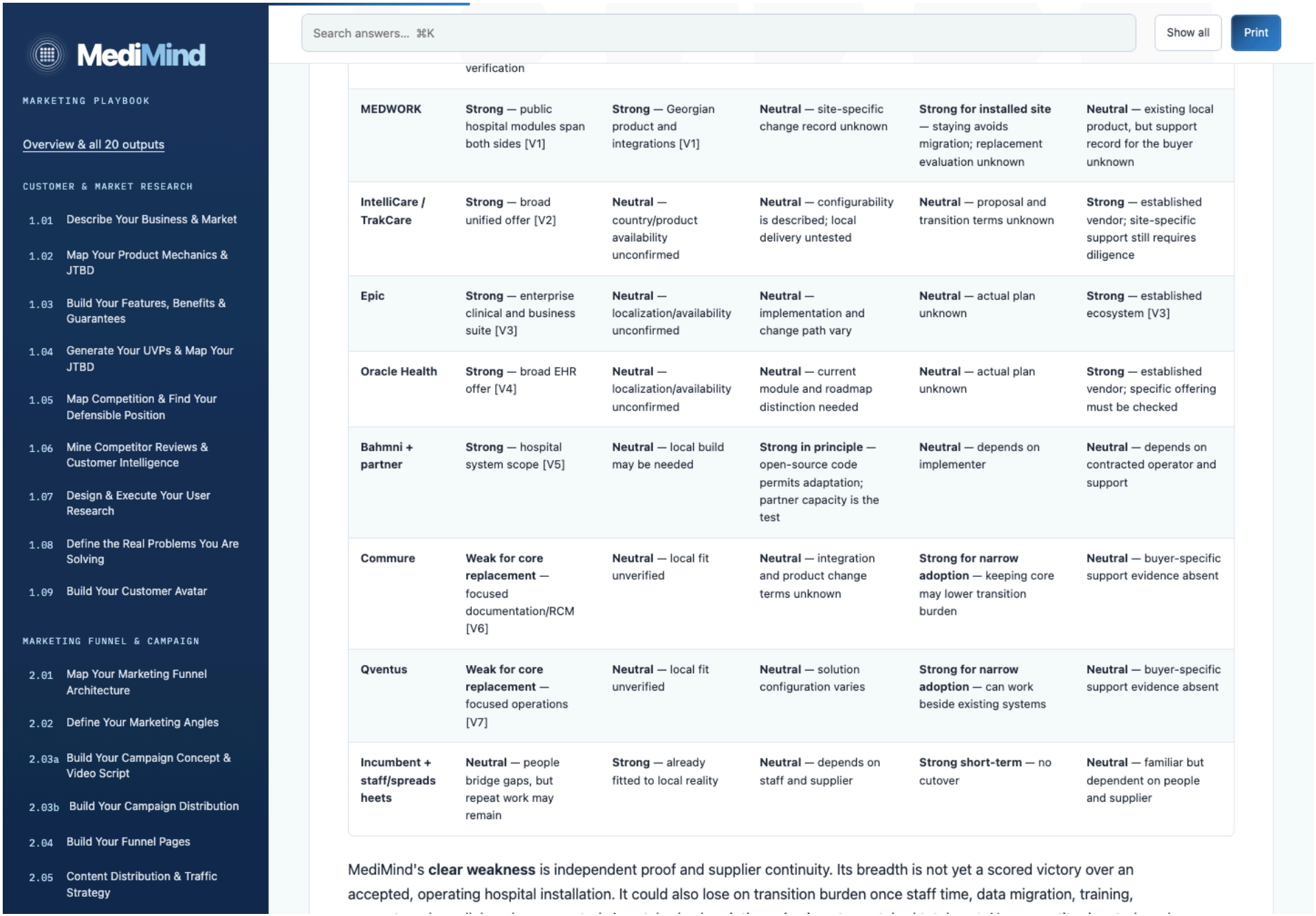

systems already deliver value; replacement risks losing it.

What customers hate. Task navigation or repeat entry (R1, R2, R9, R10); learning (R1, R3); site performance (R5); support/login (R7); coding data (R11). Causes and present-day product versions differ.

What customers wish existed. These are **inferred needs**, not explicit feature requests: less task friction (R1, R2, R9, R10), quicker support (R7), stable performance (R5), and correct local data (R11). R6 sought breadth and customization and moved **to** Epic.

Why customers leave. Only R6 states a switch rationale, and it is a switch **to Epic**. The inspected set provides no repeated reason for leaving Epic or Oracle. The requested "single most common reason" is therefore **unknown**. An unresolved current-vendor request remains a discovery hypothesis, not a review finding.

Step 3 — Extract exact customer language

The fragment library below is intentionally short. It contains **only words present on the linked live review pages**. These voices are largely U.S. or unspecified-market users; they should guide interview questions and provisional copy, not be attributed to Georgian hospital CEOs. Quotation budget is tracked across this and later files: no more than 25 verbatim words from either Capterra page in the entire edition.

Type	Exact fragment and source	What it points toward
Pain	"too many clicks" — Tabatha W., supervising nurse, Epic, 9 Apr 2026	Task navigation; she also strongly praised Epic overall.
Pain	"a lot of button pressing" — Kathleen B., patient services coordinator, Epic, 1 May 2025	Appointment-scheduling friction; she rated Epic positively.
Pain	"chart the same thing multiple times" — Brean W., registered nurse, Cerner PowerChart review, 5 Dec 2018	Duplicate charting in her older installation.
Desire	"one place" — Tabatha W., supervising nurse, Epic, 9 Apr 2026	An integrated workflow that people appreciate; it is already available in a competitor.
Desire	"well organized" — Tamara G., utilization-review nurse, Oracle listing, 2 Aug 2025	Accessible patient context; a competitor's strength.

Archived visual QA 17/31

Original files/Marketing workspace/prompt-library/complete-edition/working/qa/p1-06-mobile.png

Closing (3–5 minutes). Summarize back the incident and decision route; invite corrections. Ask for one non-sensitive process artifact, a safe observation opportunity, or the name/role of the person who sees a different part of the same handoff. Ask whether a brief findings summary would be useful. Do not ask for a purchase commitment in a research interview; a later commercial discussion needs its own invitation.

Step 5 — Build a documentation system

Use one folder per organization and one note per conversation, with an organization code rather than patient information. A simple spreadsheet is enough. Fields: date/language, role and site, recruiter relationship, consent/recording status, exact incident, observed versus reported facts, systems and people in order, current workaround, consequence and estimate basis, incumbent response, alternatives, buying trigger, decision participants, evaluation prerequisites, objections, direct quotation with permission flag, and the next evidence request. Keep a link to any authorized artifact, with access limits; do not duplicate sensitive material into the marketing workspace.

Within 24 hours, write a five-line debrief: the participant's job, the latest incident, the strongest evidence, the strongest counterevidence, and what this changes about the initial thesis. Tag statements as **observed**, **buyer-reported**, **team-reported**, **review-derived**, or **interpretation**. Keep a separate assumptions log with a confidence level and what would change it. Do not let a founder recollection become a verbatim buyer quote.

At the end of weeks 1 and 2, make a matrix of rows = organizations and columns = pathway, consequence, incumbent remedy, trigger, buyer authority, evaluation readiness, clinical/IT objections. Look for repeated patterns **across independent organizations**, not just repeated people within one network. Note negative cases explicitly. Summarize exact counts as `n/N` with role and source, not percentages or “most hospitals” from ten interviews. Saturation is assessed by whether new conversations still alter the decision map, not by reaching a fixed number.

Step 6 — Research action plan

Two-week target: 10–11 completed conversations as above, at least four independent buying organizations, plus one permissioned observation or de-identified pathway artifact if feasible. This is a proposed sprint, not scheduled interviews. The founder should allocate interview time separately from the shared five-hour-per-week marketing-production budget; the ten conversations themselves, preparation, and synthesis require extra founder time.

Independent private-hospital CEO or owner	"The same patient journey creates repeated handoffs, calls, and entries" is proposed wording, **not a quote**.	Clinical, scheduling, billing, and reporting steps may lack a shared, trustworthy handoff, or the workflow may be poorly designed even when the system is integrated.	The leader may fear spending scarce resources on software while staff remain frustrated, or may fear a failed replacement more than present inefficiency.	The same rework and delayed information may keep consuming staff capacity. The size of any financial or clinical effect needs measurement.
Hospital-network CEO or group operator	Site reports and patient pathways may be hard to compare or standardize across hospitals.	Different sites may use inconsistent definitions, contracts, systems, and practices. A shared database alone cannot resolve governance.	The leader may fear that growth is outrunning oversight and that a central change will alienate local clinical teams.	Expansion or improvement projects may require repeated reconciliation and site-by-site customization, if current remedies remain inadequate.
Clinician-owner or medically led hospital leader	Staff may spend attention navigating records, repeating documentation, or chasing results instead of patient work.	The information needed for a decision may be scattered, poorly presented, or delayed by local process.	The leader may fear that an AI or software initiative will erode clinical trust, obscure responsibility, or make care feel less personal.	Staff frustration and workarounds may persist; potential safety effects require clinical review, not a marketing inference.

Committee viewpoints sharpen the same stack rather than multiplying invented personas. Finance asks whether a charge was actually collected and whether total cost improved. Operations asks who does the extra work. Clinical leaders ask whether information is accurate and whether safeguards remain. IT asks who owns interfaces, access, backup, and exit. A purchase requires these jobs to align.

Step 3 — Identify the problem worth leading with

Lead with a measurable question: Why does one patient create so much extra work across your hospital? This is a proposed campaign hook, not a buyer quotation or proof that every hospital has the problem. It is specific enough to bring clinical and business work into one conversation and to start with a recent, observable pathway. It is broad enough for a CEO to invite the right committee members, while the first demo can narrow it to one task.

Its evidence status is **partially supported externally**: several first-person EHR users describe repeated charting or task navigation, and other users praise the benefits of connected records [P6]. Internal buyer notes independently name cross-department friction [I1]. What remains unproven is local urgency, economic weight, replace-versus-fix preference, and MediMind's actual net improvement. The honest message is an invitation to **map and measure the hospital's own pathway**, not a statement that MediMind has already eliminated the work.

For the first discovery call, ask for the last real patient journey that required duplicate entry, a call to another department, a

Step 3 — Write the avatar in their own voice

Clearly synthetic monologue (about 190 words). This is original creative writing. It is **not** a verbatim buyer quote and should never be attributed to a Georgian hospital leader. One short phrase about click burden is borrowed from a cited Epic staff review [R1] as a language cue, not evidence that a CEO said it. Real buyer language should replace this after interviews.

> I can see how hard our people work. What I cannot always see is why a single patient creates so many separate jobs for them. Someone checks a record, someone calls another team, and someone later has to make the numbers agree. I need to know whether that is an occasional exception or a pattern worth changing. I also need to know which part is software and which part is our process.
>
> I have no interest in putting care at risk for a prettier screen. Our current system may be frustrating in places, but staff know how to use it. A nurse saying there are "too many clicks" matters to me if it points to real time lost or missed information. I want my clinical and IT leads to inspect the same pathway with me and tell me what would make a test safe.
>
> Show me one improvement that stands up to comparison. Show me the work my team would have to do, the full cost, and the rule for stopping. If we can prove that first step, I can talk about a larger change with confidence.

The monologue avoids a fabricated patient incident, CEO quotation, recovery amount, or personal biography. The quoted three-word phrase is from Tabatha W., supervising nurse, 9 April 2026 and is the only reused review wording here.

Step 4 — Identify the avatar's watering holes

The following are **real, inspectable venues** where relevant material exists; whether this specific buyer visits them is **unverified**. The first research conversation should ask, "Which association, publication, colleague, event, or online channel actually influenced your last software decision?" Do not invent a Georgian hospital-CEO subreddit, private Slack, Discord, podcast habit, or group membership.

Channel	Verified venue or example	Practical use and validation question
Online	The Georgian Healthcare Association site describes a member network	A credible place to learn topics and seek permissioned

Archived visual QA 21/31

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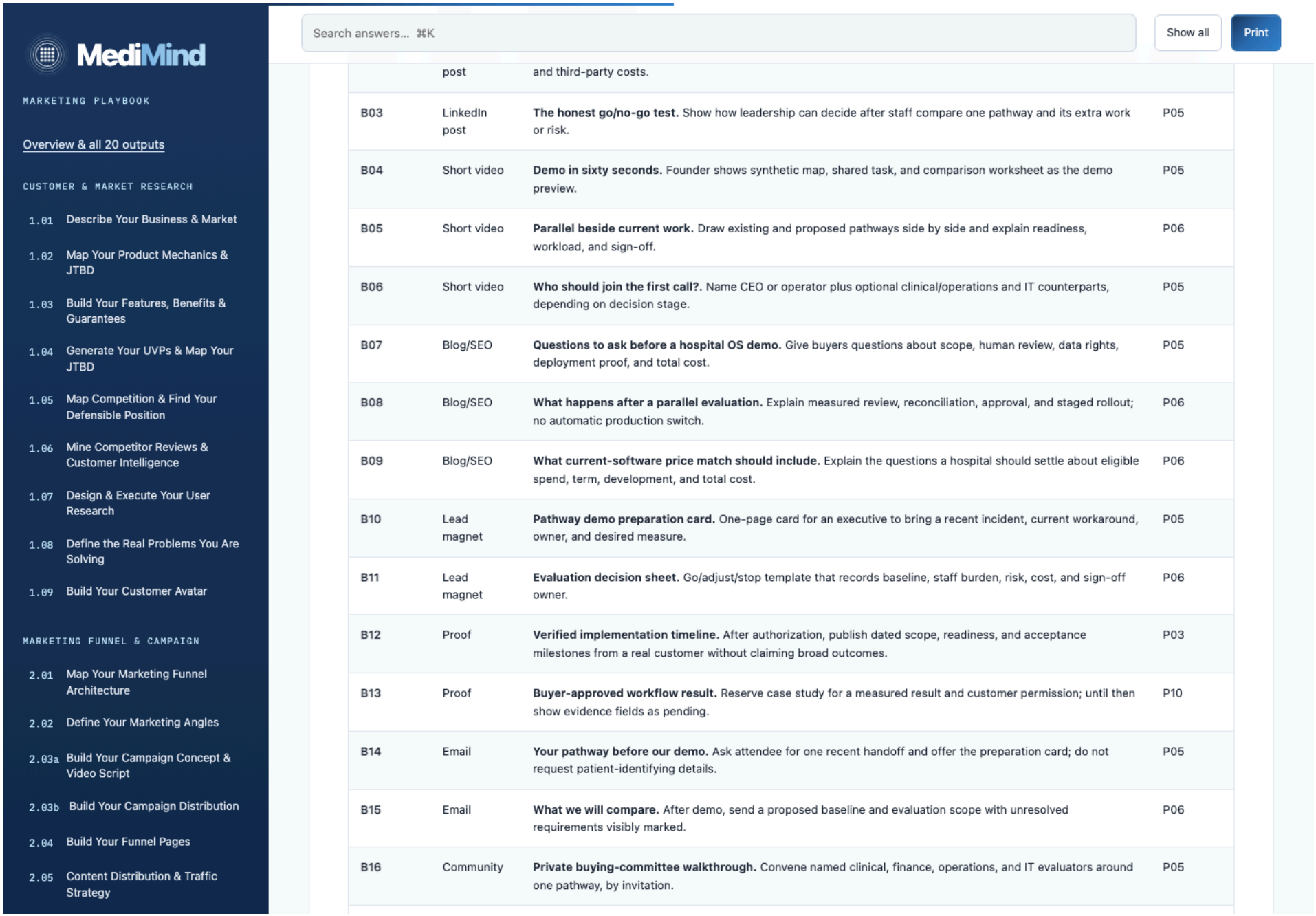
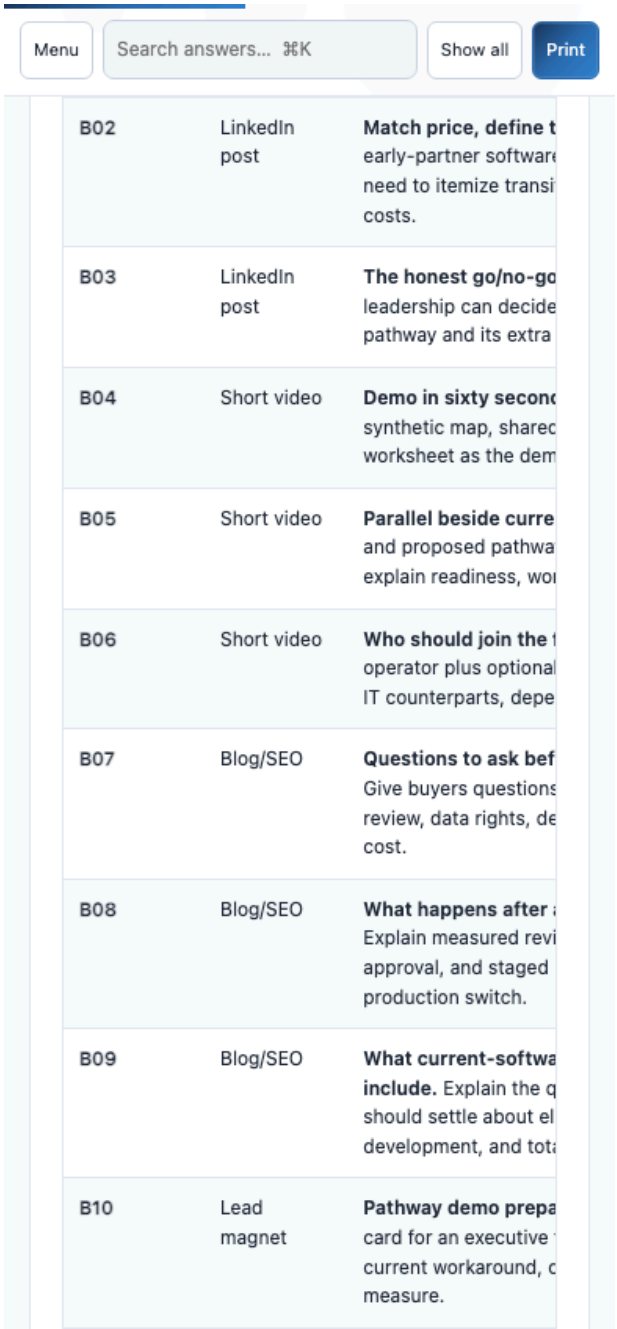


	post	and third-party costs.
B03	LinkedIn post	**The honest go/no-go test.** Show how leadership can decide after staff compare one pathway and its extra work or risk.
B04	Short video	**Demo in sixty seconds.** Founder shows synthetic map, shared task, and comparison worksheet as the demo preview.
B05	Short video	**Parallel beside current work.** Draw existing and proposed pathways side by side and explain readiness, workload, and sign-off.
B06	Short video	**Who should join the first call?.** Name CEO or operator plus optional clinical/operations and IT counterparts, depending on decision stage.
B07	Blog/SEO	**Questions to ask before a hospital OS demo.** Give buyers questions about scope, human review, data rights, deployment proof, and total cost.
B08	Blog/SEO	**What happens after a parallel evaluation.** Explain measured review, reconciliation, approval, and staged rollout; no automatic production switch.
B09	Blog/SEO	**What current-software price match should include.** Explain the questions a hospital should settle about eligible spend, term, development, and total cost.
B10	Lead magnet	**Pathway demo preparation card.** One-page card for an executive to bring a recent incident, current workaround, owner, and desired measure.
B11	Lead magnet	**Evaluation decision sheet.** Go/adjust/stop template that records baseline, staff burden, risk, cost, and sign-off owner.
B12	Proof	**Verified implementation timeline.** After authorization, publish dated scope, readiness, and acceptance milestones from a real customer without claiming broad outcomes.
B13	Proof	**Buyer-approved workflow result.** Reserve case study for a measured result and customer permission; until then show evidence fields as pending.
B14	Email	**Your pathway before our demo.** Ask attendee for one recent handoff and offer the preparation card; do not request patient-identifying details.
B15	Email	**What we will compare.** After demo, send a proposed baseline and evaluation scope with unresolved requirements visibly marked.
B16	Community	**Private buying-committee walkthrough.** Convene named clinical, finance, operations, and IT evaluators around one pathway, by invitation.

Archived visual QA 22/31

Original files/Marketing workspace/prompt-library/complete-edition/working/qa/p2-01-mobile.png



offline events	partner is confirmed.	MOFU	before promotion.

Priority 3 is the demo invitation framed as a controlled parallel comparison: "See your pathway beside today's work before deciding." The first three active angles therefore form one story: **map the problem → show a patient-pathway use case → compare safely**.

Part 3 — Angle-to-channel mapping

The table covers every selected "yes" angle. It specifies an initial asset and a countable success event. Channel use is proposed until the buyer's actual media and referral habits are observed. There is no paid campaign in the first month.

Selected angle and channel	First content/format	CTA	What success looks like
Low-risk CTA / founder LinkedIn	T01: post with a synthetic seven-handoff sketch and invitation to name the local version	Map my hospital's first improvement (P01)	A relevant operator requests or discusses a pathway map.
Low-risk CTA / existing professional introductions	One-to-one note offering T10 worksheet, only to a legitimate contact	Use the pathway worksheet (P01)	Recipient returns a real handoff or introduces its owner.
Use-case offer / founder phone video	T04: duplicate-entry trail using synthetic information	Map my hospital's first improvement (P01)	A relevant leader watches enough to ask about their own pathway; count replies, not views alone.
Use-case offer / website article	T07: how to map a patient handoff	Download the worksheet (P01)	Qualified downloads or a pathway question from a hospital.
Controlled comparison within low-risk CTA / founder follow-up	M10 readiness checklist after an interested response	Discuss an evaluation (P06)	Named clinical/operations and IT counterparts plus agreed next step.
Demo / existing `/request-demo` page	B04 synthetic demo preview, after real submission receipt is verified	Request a pathway demo (P05)	Held qualified demo, not a success-screen view.
Language/region / Georgian professional connections	Localized T01 with reviewed terminology, not automatic translation	Map my hospital's first improvement (P01)	A Georgian hospital operator recognizes and corrects the scenario.
Direct sales call / permissioned peer introduction	Short founder note about a recent handoff, not a generic pitch	Request a pathway demo (P05)	Relevant executive accepts a 25-minute conversation.

27–41s / mechanism	"So we start with one real pathway at your hospital. We map who needs to act, what information they need, and where the same work is repeated."	Whiteboard/paper diagram with three role labels and arrows; founder circles one repeated step. If a screenshot is used, switch only to synthetic demo data.	"Map one pathway."
41–57s / mechanism	"MediMind brings the clinical and business steps into one hospital operating system. Your team can compare that path beside the way it works today."	Show two paper lanes labeled "today" and "proposed"; reveal one shared task and patient context on a synthetic screen if available. Do not show a flashy dashboard unrelated to the example.	"Compare beside today's work."
57–69s / relief	"Then we measure the handoff, the staff effort, and the exceptions. Your clinicians and IT team help decide what is ready—and whether a wider change makes sense."	Founder places a simple three-column card: "work," "exceptions," "decision." A handwritten checkbox remains empty.	"Measure. Review. Decide."
69–72s / transition	"Bring me one handoff your hospital keeps repeating."	Direct to camera; keep the circled pathway visible.	"ONE HANDOFF"
72–82s / closing ten seconds	**"In 25 minutes, we'll map the first improvement worth testing. Map my hospital's first improvement."**	Hold a clean CTA card with the verified public route or a direct founder contact path.	"MAP MY HOSPITAL'S FIRST IMPROVEMENT"

The spoken script is about 155 words, leaving time for visual pauses. A fast read would undercut the tone; if the founder reaches the CTA too early, let the two-lane comparison remain on screen rather than adding claims. If the full cut runs past 90 seconds, cut the line about billing in the first act before accelerating speech.

Director's three instructions:

1. Keep the opening quiet and slightly puzzled; speak directly to one hospital operator, not to a crowd. Let the paper-card movement supply the interruption.
2. Make each cut coincide with one handoff. Use large, readable labels and high contrast; mobile viewers should understand the visual with sound off and captions on.
3. End on the founder's face and one CTA. Do not place an unverified customer logo, "25 hospitals live," a performance percentage, or an artificial deadline on screen.

15-second organic cut-down (same question-led campaign):

Time	Exact words	Visual
0–4s	"Why does one patient create so much extra work?"	Same three cards and blank patient card.

MediMind

MARKETING PLAYBOOK

Overview & all 20 outputs

CUSTOMER & MARKET RESEARCH

1.01 Describe Your Business & Market

1.02 Map Your Product Mechanics & JTBD

1.03 Build Your Features, Benefits & Guarantees

1.04 Generate Your UVPs & Map Your JTBD

1.05 Map Competition & Find Your Defensible Position

1.06 Mine Competitor Reviews & Customer Intelligence

1.07 Design & Execute Your User Research

1.08 Define the Real Problems You Are Solving

1.09 Build Your Customer Avatar

MARKETING FUNNEL & CAMPAIGN

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2.03a Build Your Campaign Concept & Video Script

2.03b Build Your Campaign Distribution

2.04 Build Your Funnel Pages

2.05 Content Distribution & Traffic Strategy

Email 4 — Truthful urgency, day 10; under 100 words

Subject: The handoff will repeat next month
Preview: Start with one measurable path.
Opening line: If the same pathway keeps creating extra work, waiting has a cost worth measuring.

Full body:

Hello [First name],

If staff repeat the same handoff next month, the calls and reconciliation will still need an owner. We cannot tell you the size of that cost without looking at your workflow. We can spend 25 minutes mapping one path and agreeing what your team should measure before any wider change.

CTA link text: Map my hospital's first improvement

No expiry date or limited place is claimed. If there is no permissioned email segment or working destination, these remain drafts.

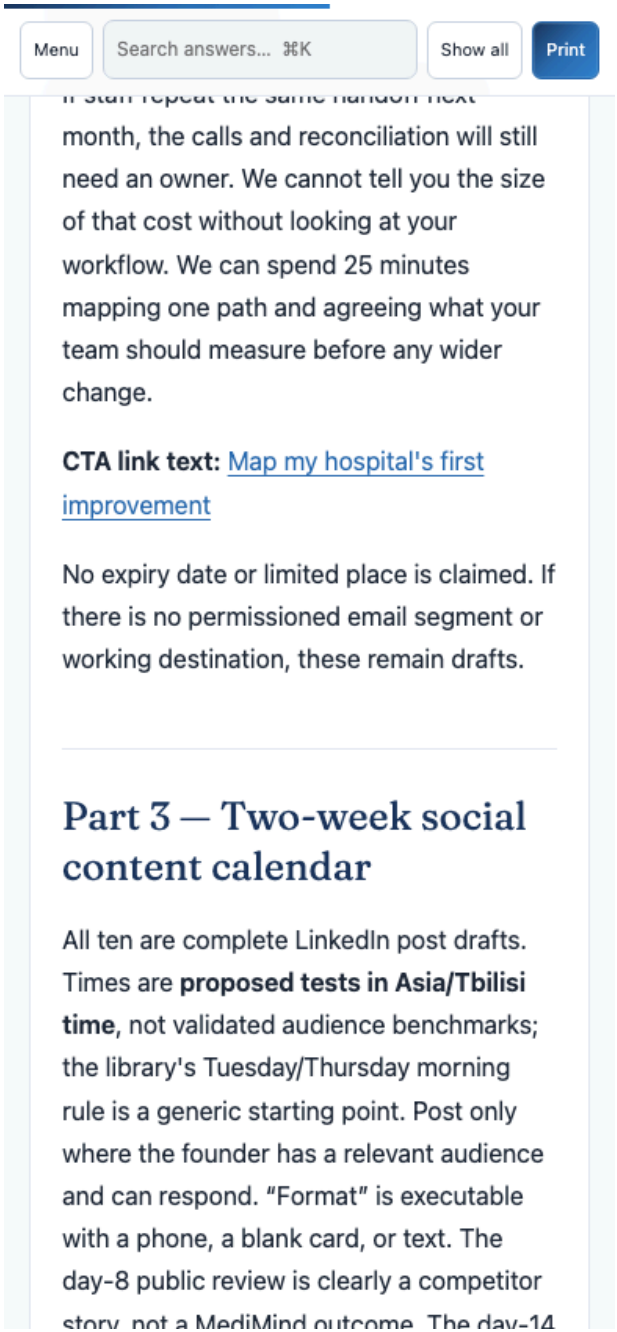
Part 3 — Two-week social content calendar

All ten are complete LinkedIn post drafts. Times are **proposed tests in Asia/Tbilisi time**, not validated audience benchmarks; the library's Tuesday/Thursday morning rule is a generic starting point. Post only where the founder has a relevant audience and can respond. "Format" is executable with a phone, a blank card, or text. The day-8 public review is clearly a competitor story, not a MediMind outcome. The day-14 close uses process proof because a buyer-approved result is unavailable.

Day / format / test time	Opening hook	Full post copy	CTA
1 / text + paper photo / 09:00	**A patient does not create the extra work. The handoffs do.**	"A doctor orders. A result comes back. The next person still has to ask whether it arrived. Finance still needs to reconcile what happened. Each team can be doing its job while the journey between them becomes another job. Before buying another dashboard, draw one real pathway and mark every call, copied value, and wait. What would your team find?"	None.
2 / text + three-card sketch / 09:00	**Why does one patient create so much extra work?**	"Try following one patient from order to result to bill. Where does someone look up the same detail? Where does a call replace a visible handoff? Where does the bill depend on a note another team cannot see yet? This is a question for a hospital's own	None.

Archived visual QA 26/31

Original files/Marketing workspace/prompt-library/complete-edition/working/qa/p2-03b-mobile.png



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Actual worksheet body, ready to place in a one-page PDF or on-page form: "Patient pathway: _____ (describe the kind of journey, not a person). Date observed: _____. (1) First request: who creates it, where is it recorded, and who needs it next? (2) Result or response: where does it arrive, who notices, and what happens if it is late? (3) Next clinical/operations decision: what facts must be visible, who confirms them, and what gets re-entered? (4) Financial/administrative close: what must reconcile, what does staff check manually, and what remains unknown? Circle each call, copied fact, wait, or recheck. Count the circles; note the staff time and any exception you can verify. Ask: which circle repeats enough to be worth fixing, and could the current supplier or a narrow change remove it?" This is a worksheet, not a diagnostic score. The P01 five-minute check uses these four boxes and returns the chosen pathway, not an invented maturity grade.

P02 `/hospital-os` — product overview for T12–T13, M03, M06, M09 / TOFU–MOFU

Visitor arrives from: a synthetic pathway visual, a mechanism post, or a FHIR explainer. **Job:** understand what MediMind does today, what needs local setup, and what is future direction. **Conversion event:** qualified move to a demo request.

> **Eyebrow:** One hospital pathway at a time
> **Headline:** Clinical and business work, in one path
> **Subheadline:** MediMind is a Georgian hospital operating system built around a shared FHIR R4 record and workflows that cross departments.
> **Opening:** A patient journey does not end at a chart. An order, result, task, medicine, bill, and report may pass through different teams. MediMind gives those teams a shared foundation so the next responsible person can work with relevant context. What the system can demonstrate or deploy at your hospital depends on the agreed modules, interfaces, data, and review.
> **How it works:** (1) Map the current pathway and people who own each step. (2) Show a proposed connected workflow using synthetic data. (3) Set a baseline and compare, with clinical and IT sign-off before any production change.
> **What FHIR R4 means:** It is a common structure for health information—like agreed labels on every handoff. It helps information move and be understood, but does not automatically solve interface, data-quality, or training work.
> **AI's role:** Defined assistance can help draft or surface information for human review. The future direction is broader coordination; no autonomous clinical decision or guaranteed patient outcome is claimed.
> **Scope note:** Clinical and business domains are described in the product map; ask which are demonstrated, configured, accepted, or planned for your institution. External payers, ministries, devices, and other systems still require appropriate interfaces.
> **Proof panel:** Founder-authored Medplum implementation account describes the architecture and a signed anchor-network agreement. It is not an independently measured outcome study or a statement that every facility is live.
> **CTA:** Bring one handoff; **map my hospital's first improvement** → P05.

Visual placement: Use the campaign's paper-card path or a clearly synthetic product screen with current capability labels. A

Archived visual QA 28/31

Original files/Marketing workspace/prompt-library/complete-edition/working/qa/p2-05-desktop.png

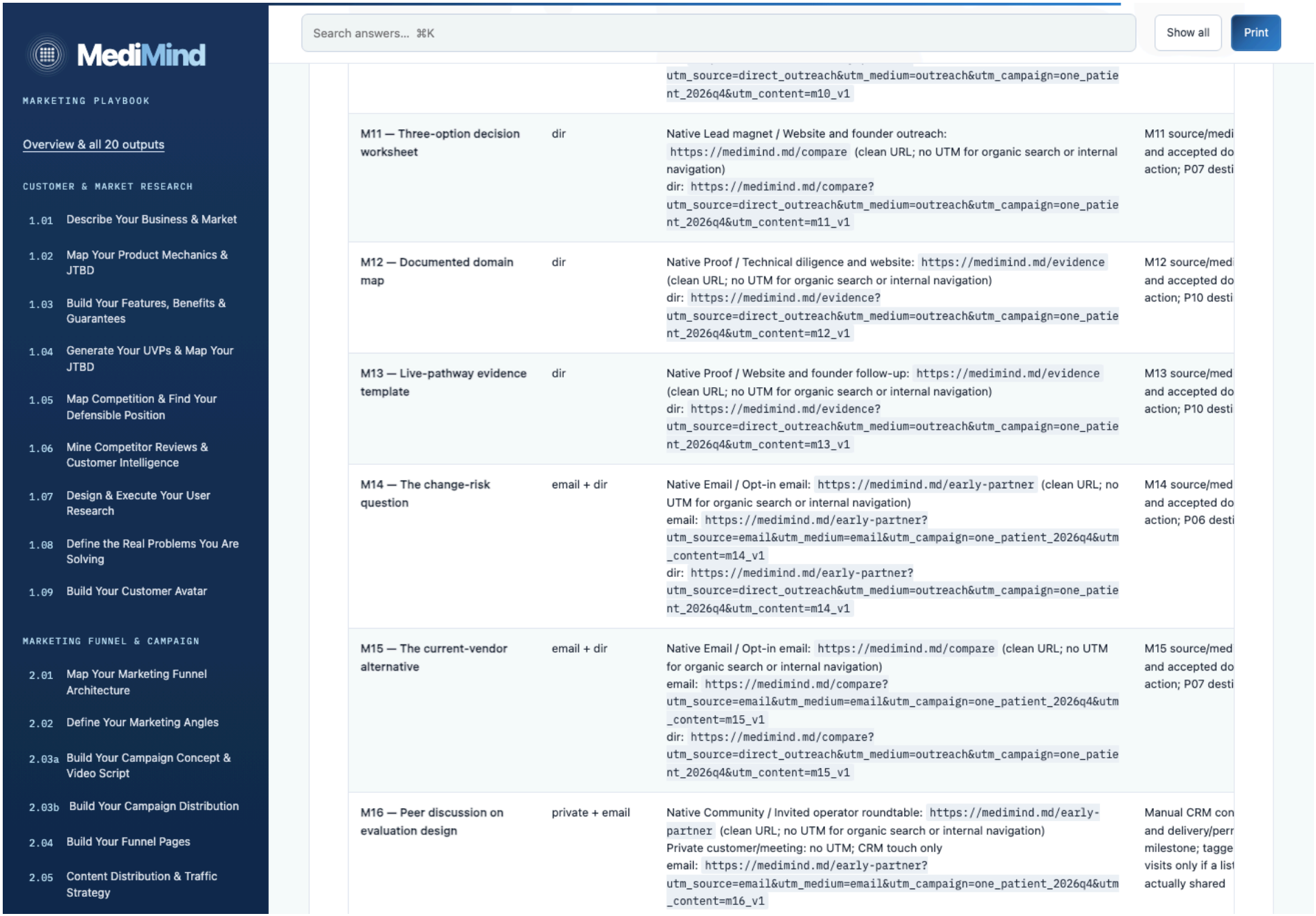
versus vendor change	Blog/SEO / W	comparison; after technical review. D → send the relevant row to one prospect; same hook; comparison; after discovery.
M09 — What FHIR R4 changes in hospital work	Blog/SEO / W	W → plain-language record analogy plus interface caveat; "Shared format still needs local integration"; product path; after technical review. LI → one diagram and caveat; same hook; path; following Thu.
M10 — Parallel-evaluation readiness checklist	Lead magnet / W and D	W → one-page owners/access/baseline/stop checklist; "Can your team test this fairly?"; download; when built. D → highlight missing owner field for interested account; same hook; review plan; after demo.
M11 — Three-option decision worksheet	Lead magnet / W and D	W → four-column comparison of burden/risk/cost/scope; "Compare actual alternatives"; worksheet; when built. D → prefill no customer data, ask buyer to score own options; same hook; compare; after discovery.
M12 — Documented domain map	Proof / technical diligence	W → verified scope matrix with deployed/accepted/pending states; "Which workflow is actually ready?"; inspect scope; only after module-by-module audit. D → send relevant slice to IT under review; same hook; diligence call; after request.
M13 — Live-pathway evidence template	Proof / W and founder follow-up	W → blank baseline/after template with outcome cells visibly pending; "What would convincing proof require?"; evidence method; after QA. D → customize measure names, not results; same hook; discuss evaluation; after demo.
M14 — The change-risk question	Email / E	E → one risk prompt and parallel-plan link; "What transition failure would stop this?"; evaluation plan; after opted-in comparison interest. D → use prospect's stated risk in reply; same hook; scope call; after response.
M15 — The current-vendor alternative	Email / E	E → two-column comparison question, no competitor caricature; "Can your supplier fix this across departments?"; comparison; after opt-in. D → one-to-one criteria sheet; same hook; compare; after discovery.
M16 — Peer discussion on evaluation design	Community / invited operator roundtable	Private roundtable → four questions, no claims or patient details; "What proof would justify change?"; readiness checklist; only with host/attendees confirmed. E → recap generic method to opt-ins; same hook; checklist; after event approval.
M17 — IT and clinical review session	Community / association or partner	Partner session → joint risk checklist, not a product deck; "What would clinical and IT each sign off?"; evaluation plan; after partner/date approval. W → publish generic FAQ from approved questions; same hook; plan; after review.

BOFU cards — B01–B17

ID — content piece	Original format / native home	Distribution: channel → adaptation; hook; CTA; timing

Archived visual QA 29/31

Original files/Marketing workspace/prompt-library/complete-edition/working/qa/p2-06-desktop.png



Archived visual QA 30/31

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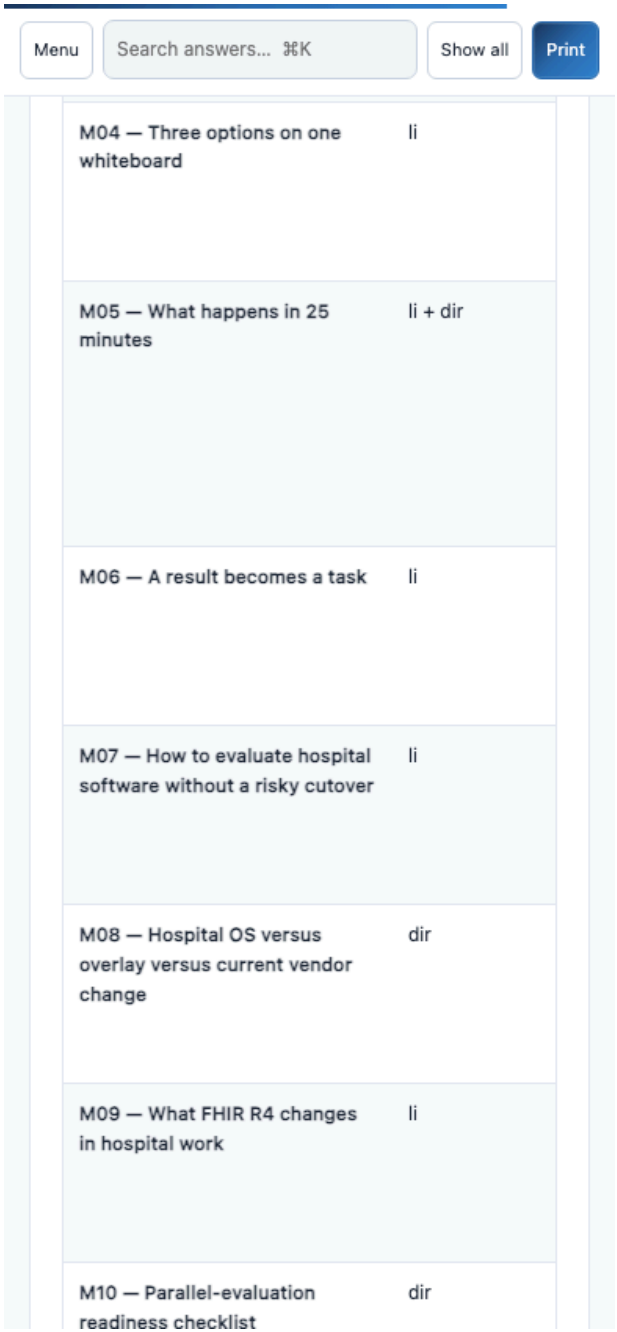

Part 2 — Monthly funnel review

Complete this four-part summary in 30 minutes using the already reconciled two-hour monthly tracking session. Month-over-month percentages use `([this]-[last])/[last]` only when last month is nonzero; otherwise write **N/A (new baseline)**. Compare qualified organizations, not duplicate contacts. For revenue, distinguish contracted recurring value from collected cash and do not count the anchor as a new close.

1. Month in numbers

Metric	Last month	This month	Change	Trend
Unique public-site visitors	[count or N/A]	[count]	[delta; % if denominator >0]	[↑/→/↓/N/A]
New accepted leads, distinct buying organizations	[count or N/A]	[count]	[delta; % if valid]	[↑/→/↓/N/A]
Held qualified demos, distinct buying organizations; guided trials separately	[demos] / [trials]	[demos] / [trials]	[deltas]	[↑/→/↓/N/A]
New paying buying organizations	[count]	[count]	[delta; % if valid]	[↑/→/↓/N/A]
Collected revenue / contracted MRR, labelled separately	[amount] / [MRR]	[amount] / [MRR]	[delta; % if valid]	[↑/→/↓/N/A]
Best content piece by held qualified-demo contribution	[ID or N/A]	[ID plus organization count]	[changed?]	[repeat/test]
Best channel by held qualified-demo contribution	[channel or N/A]	[channel plus organization count]	[changed?]	[repeat/test]

2. What worked

- **[Source/content ID]** generated **[number] held qualified demo(s) from distinct organizations** because **[buyer question or observed behavior]**; [repeat/expand one adaptation next month].
- **[Offer/page or follow-up change]** moved **[number]/[eligible denominator]** relevant organizations forward because **[observed reason]**; [keep or test once more].